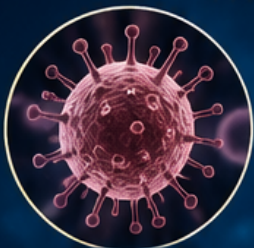


ALL ABOUT HUMAN DISEASES

A COMPLETE GUIDE TO UNDERSTANDING
THE CAUSES, SYMPTOMS, DIAGNOSIS AND TREATMENT
OF HUMAN DISEASES



INFECTIOUS
DISEASES



NEUROLOGICAL
DISORDERS



CARDIOVASCULAR
DISORDERS



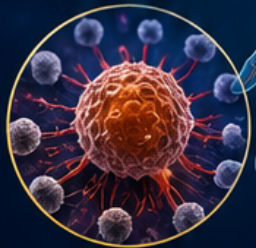
RESPIRATORY
DISEASES



METABOLIC
DISORDERS



MUSCULOSKELETAL
DISORDERS



CANCER



MENTAL
DISORDERS



CAUSES



SYMPTOMS



DIAGNOSIS



TREATMENT

AUTHOR

CHAMAN SINGH VERMA

All About Human Diseases

An Organ-Categorized Medical Reference

A Complete Catalog of Diseases and Disorders

Affecting Each Organ and System of the Human Body

Chaman Singh Verma

August 20, 2026

This comprehensive report catalogs human diseases organized by the primary organ or organ system affected. Each chapter provides an overview of the organ's anatomy and function, followed by detailed entries covering the most significant diseases—their etiology, pathophysiology, clinical presentation, diagnosis, and treatment. The report spans all major organ systems of the human body, from the brain and nervous system to the reproductive organs, providing a thorough medical reference for students, educators, and practitioners.

Contents

	Preface	xxiii
	Foreword	xxv
1	Abdominal Aortic Aneurysm	1
2	Abnormal Uterine Bleeding.....	5
3	Abscess	9
4	Acetaminophen (Paracetamol) Overdose	13
5	Achalasia.....	17
6	Achondroplasia.....	21
7	Acne Vulgaris	25
8	Acoustic Neuroma (Vestibular Schwannoma).....	29
9	Acromegaly	33
10	Actinic Keratosis	37
11	Acute/Short-Term Insomnia.....	41
12	Acute Bacterial Sinusitis.....	45
13	Acute Bronchitis	51
14	Acute Decompensated Heart Failure	55
15	Acute Gastritis.....	59
16	Acute Interstitial Nephritis	63
17	Acute Kidney Injury	67
18	Acute Lymphoblastic Leukemia.....	71
19	Acute Lymphoblastic Leukemia (ALL) in Children	77
20	Acute Myeloid Leukemia	83
21	Acute Otitis Media.....	89
22	Acute Otitis Media.....	93
23	Acute Pancreatitis.....	97
24	Acute Respiratory Distress Syndrome.....	101
25	Acute Stress Disorder	105
26	Addison's Disease	109
27	Adhesive Capsulitis.....	113
28	Adjustment Disorder	117
29	Adrenal Incidentaloma.....	121
30	Adult-Onset Still's Disease.....	125
31	Advanced Sleep-Wake Phase Disorder.....	129
32	African Trypanosomiasis (Sleeping Sickness)	133
33	Age-Related Bone Loss	137
34	Age-Related Cataract	141
35	Age-Related Macular Degeneration	145
36	Age-Related Macular Degeneration	149
37	Agoraphobia	153
38	Alcohol Intoxication and Withdrawal	157
39	Alcohol Use Disorder	161

40	Alcoholic Liver Disease	165
41	Alkaptonuria — Ochronosis.....	169
42	Allergic Fungal Rhinosinusitis (AFRS).....	173
43	Allergic Rhinitis.....	179
44	Alopecia Areata.....	183
45	Alpha-1 Antitrypsin Deficiency	187
46	Altitude Sickness.....	191
47	Alzheimer’s Disease	195
48	Ambiguous Genitalia / Disorders of Sex Development.....	199
49	Amblyopia	203
50	Amebiasis (Entamoeba histolytica).....	207
51	Amenorrhea (Primary, Secondary).....	211
52	Amyloidosis	215
53	Amyotrophic Lateral Sclerosis	219
54	Anal Fissure.....	223
55	Anaphylaxis.....	227
56	Angelman Syndrome	231
57	Angina Pectoris	235
58	Ankylosing Spondylitis	239
59	Anophthalmia and Microphthalmia	243
60	Anorexia Nervosa	247
61	Anthrax.....	251
62	Anti-GBM Disease	255
63	Antiphospholipid Syndrome	259
64	Antisocial Personality Disorder (ASPD)	263
65	Aortic Dissection	267
66	Aortic Regurgitation	271
67	Aortic Stenosis	275
68	Aortic Stenosis	279
69	Aphthous Ulcers (Canker Sores).....	283
70	Aplastic Anemia.....	287
71	Appendicitis.....	291
72	Arboviral Encephalitis.....	295
73	Arsenic Poisoning	299
74	Asbestosis.....	303
75	Ascites	307
76	Aspergillosis (Invasive)	311
77	Aspiration Pneumonia.....	315
78	Asplenia.....	319
79	Asplenia and Hyposplenism.....	323
80	Asthma	327
81	Asthma-COPD Overlap	331
82	Astigmatism.....	335
83	Atelectasis	339
84	Atopic Dermatitis	343
85	Atrial Fibrillation	347

86	Atrial Fibrillation in the Elderly	351
87	Atrial Flutter	355
88	Atrophic Vaginitis (Genitourinary Syndrome of Menopause).....	359
89	Attention-Deficit/Hyperactivity Disorder (ADHD)	363
90	Atypical Facial Pain	367
91	Autism Spectrum Disorder (ASD)	371
92	Autoimmune Hepatitis	375
93	Autoimmune Pancreatitis.....	381
94	Autoimmune Pancreatitis (AIP)	385
95	Autosomal Dominant Polycystic Kidney Disease	389
96	Bacterial Meningitis.....	393
97	Bacterial Vaginosis	397
98	Balanitis and Balanoposthitis.....	401
99	Barotrauma of the Ear	405
100	Barrett Esophagus	409
101	Bartholin Cyst	413
102	Basal Cell Carcinoma.....	417
103	Becker Muscular Dystrophy	423
104	Behçet's Disease.....	427
105	Bell's Palsy	431
106	Benign Paroxysmal Positional Vertigo	435
107	Benign Prostatic Hyperplasia.....	439
108	Benzodiazepine Overdose.....	443
109	Beta-Blocker Overdose	447
110	Binge-Eating Disorder.....	451
111	Bipolar II Disorder.....	455
112	Bipolar I Disorder.....	459
113	Bietti's Crystalline Dystrophy	463
114	Bladder Cancer	467
115	Blastomycosis	473
116	Blepharitis	477
117	Blepharospasm	481
118	Body Dysmorphic Disorder.....	485
119	Borderline Personality Disorder (BPD)	489
120	Botulism (Clostridium botulinum)	493
121	Brain Abscess	497
122	Brain Tumors in Children	503
123	Breast Cancer.....	507
124	Bronchiectasis.....	513
125	Bronchiolitis (RSV)	517
126	Bronchopulmonary Dysplasia (BPD).....	521
127	Brucellosis	525
128	Brugada Syndrome.....	529
129	Budd-Chiari Syndrome	533
130	Bulimia Nervosa	537
131	Bullous Pemphigoid	541

132	Bursitis	545
133	COVID-19	549
134	Cadmium Toxicity.....	553
135	Calcium Channel Blocker Overdose.....	557
136	Calcium Pyrophosphate Deposition Disease.....	561
137	Calcium and Phosphorus Disorders	565
138	Candida auris	569
139	Candidiasis (Invasive / Systemic)	573
140	Cannabis Toxicity and Cannabinoid Hyperemesis Syndrome.....	577
141	Carbon Monoxide Poisoning.....	581
142	Cardiac Tamponade	585
143	Carpal Tunnel Syndrome	589
144	Celiac Disease.....	593
145	Cellulitis	597
146	Central Pontine Myelinolysis	601
147	Central Post-Stroke Pain	605
148	Central Sleep Apnea.....	609
149	Cerebral Aneurysm.....	613
150	Cerebral Venous Thrombosis	617
151	Cerebral Visual Impairment	621
152	Cerumen Impaction	625
153	Cervical Cancer	629
154	Cervical Cancer	635
155	Cervicitis (Chlamydia, Gonorrhea).....	641
156	Cervicogenic Headache	645
157	Chagas Disease (American Trypanosomiasis).....	649
158	Chancroid.....	653
159	Charcot-Marie-Tooth Disease.....	657
160	Chickenpox	661
161	Chikungunya.....	667
162	Choanal Atresia.....	671
163	Cholecystitis	675
164	Cholelithiasis.....	679
165	Cholera (Vibrio cholerae).....	683
166	Chondrodermatitis Nodularis Helicis.....	689
167	Chronic Fatigue Syndrome	693
168	Chronic Gastritis.....	697
169	Chronic Hypertension in Pregnancy	701
170	Chronic Insomnia Disorder	705
171	Chronic Kidney Disease	709
172	Chronic Low Back Pain.....	713
173	Chronic Lymphocytic Leukemia	717
174	Chronic Neck Pain	723
175	Chronic Obstructive Pulmonary Disease	727
176	Chronic Otitis Media.....	731
177	Chronic Pancreatitis.....	735

178	Chronic Pelvic Pain	739
179	Chronic Rhinosinusitis.....	743
180	Chronic Traumatic Encephalopathy.....	747
181	Chronic Tubulointerstitial Nephritis	751
182	Cirrhosis	755
183	Cleft Lip and Palate.....	759
184	Clostridioides difficile Colitis	763
185	Cluster Headache	767
186	Coarctation of the Aorta	771
187	Coccidioidomycosis (Valley Fever)	775
188	Cogan’s Syndrome.....	779
189	Coloboma	783
190	Colorectal Adenoma.....	787
191	Colorectal Cancer	791
192	Color Blindness	797
193	Coma	801
194	Common Variable Immunodeficiency.....	805
195	Community-Acquired Pneumonia.....	809
196	Compartment Syndrome.....	813
197	Complex Regional Pain Syndrome	817
198	Complex Sleep Apnea	821
199	Concussion.....	825
200	Congenital Cataract	829
201	Congenital Diaphragmatic Hernia.....	833
202	Congenital Heart Disease (Overview)	837
203	Congenital Hip Dysplasia.....	841
204	Congenital Talipes Equinovarus (Clubfoot).....	845
205	Conjunctivitis	849
206	Constrictive Pericarditis.....	853
207	Contact Dermatitis.....	857
208	Convergence Insufficiency	861
209	Copper Deficiency and Excess	865
210	Coronary Artery Disease	869
211	Creutzfeldt-Jakob Disease (CJD)	873
212	Cri-du-Chat Syndrome	877
213	Crohn’s Disease	881
214	Croup (Laryngotracheobronchitis)	885
215	Cryoglobulinemic Vasculitis.....	889
216	Cryptococcosis	893
217	Cryptorchidism (Undescended Testis).....	897
218	Cushing Syndrome	901
219	Cushing’s Syndrome.....	905
220	Cyanide Toxicity	909
221	Cyclospora cayetanensis	913
222	Cyclothymic Disorder	917
223	Cystic Fibrosis	921

224	Cystitis	925
225	Cytomegalovirus (CMV)	929
226	Deep Vein Thrombosis	933
227	Delayed Sleep-Wake Phase Disorder	937
228	Delirium	941
229	Delusional Disorder	945
230	Dementia with Lewy Bodies	949
231	Dengue Fever	953
232	Dermatitis Herpetiformis	959
233	Dermatomyositis	963
234	Dermatomyositis	967
235	Deviated Nasal Septum.....	971
236	DiGeorge Syndrome.....	975
237	Diabetes Insipidus.....	979
238	Diabetic Ketoacidosis.....	983
239	Diabetic Nephropathy	987
240	Diabetic Peripheral Neuropathic Pain.....	991
241	Diabetic Retinopathy.....	995
242	Digoxin Toxicity.....	999
243	Dilated Cardiomyopathy.....	1003
244	Diphtheria	1007
245	Diphtheria (Corynebacterium diphtheriae).....	1011
246	Disseminated Intravascular Coagulation.....	1017
247	Dissociative Amnesia.....	1021
248	Dissociative Identity Disorder	1025
249	Diverticulitis	1029
250	Diverticulosis.....	1033
251	Down Syndrome (Trisomy 21)	1037
252	Drowning (Submersion Injury)	1041
253	Drug Hypersensitivity	1045
254	Dry Eye Disease.....	1049
255	Duchenne Muscular Dystrophy	1053
256	Ductal Carcinoma In Situ (DCIS)	1057
257	Duodenal Ulcer	1063
258	Dupuytren Contracture	1067
259	Dysbetalipoproteinemia	1071
260	Dysmenorrhea.....	1075
261	Dystonia	1079
262	Eastern Equine Encephalitis	1083
263	Ebola Virus Disease	1087
264	Eclampsia.....	1093
265	Ectopic Pregnancy	1097
266	Edwards Syndrome (Trisomy 18)	1101
267	Ehlers-Danlos Syndrome.....	1105
268	Ehrlichiosis.....	1109
269	Electrical Injury and Lightning Strike.....	1113

270	Empyema	1117
271	End-Stage Renal Disease	1121
272	Endometrial / Uterine Cancer	1125
273	Endometriosis	1131
274	Endometriosis	1135
275	Eosinophilic Esophagitis	1139
276	Eosinophilic Fasciitis (Shulman Syndrome)	1143
277	Eosinophilic Granulomatosis with Polyangiitis	1147
278	Epidermolysis Bullosa	1151
279	Epidural Hematoma	1155
280	Epiglottitis	1159
281	Epiglottitis	1163
282	Epilepsy	1167
283	Epistaxis	1171
284	Epstein-Barr Virus / Infectious Mononucleosis (Systemic Aspects)	1175
285	Erectile Dysfunction	1179
286	Erythema Multiforme	1183
287	Esophageal Atresia / Tracheoesophageal Fistula	1187
288	Esophageal Varices	1191
289	Essential Thrombocythemia	1195
290	Essential Tremor	1199
291	Esthesioneuroblastoma	1203
292	Ewing Sarcoma	1209
293	Ewing Sarcoma in Children	1215
294	Facioscapulohumeral Muscular Dystrophy	1221
295	Failure to Thrive	1225
296	Familial Adenomatous Polyposis	1229
297	Familial Combined Hyperlipidemia	1233
298	Familial Hypercholesterolemia	1237
299	Fanconi Syndrome	1241
300	Fat Necrosis of the Breast	1245
301	Febrile Seizures	1249
302	Fecal Incontinence	1253
303	Fibroadenoma	1257
304	Fibrocystic Changes	1261
305	Fibromyalgia	1265
306	Fibromyalgia	1269
307	Fibrous Dysplasia	1273
308	Fifth Disease (Erythema Infectiosum)	1277
309	Filariasis (Lymphatic Filariasis)	1281
310	Floaters and Vitreous Detachment	1285
311	Fluoride Excess — Fluorosis	1289
312	Focal Segmental Glomerulosclerosis	1293
313	Folate Deficiency Anemia	1297
314	Fractures	1301
315	Fragile X Syndrome	1305

316	Frailty	1309
317	Friedreich's Ataxia	1313
318	Frontotemporal Dementia	1317
319	Frontotemporal Dementia	1321
320	Fuchs' Endothelial Dystrophy.....	1325
321	Fungal Sinusitis	1329
322	Galactoceles.....	1333
323	Galactosemia.....	1337
324	Gas Gangrene (Clostridial Myonecrosis)	1341
325	Gastric Adenocarcinoma.....	1345
326	Gastric Lymphoma.....	1351
327	Gastric Ulcer.....	1357
328	Gastroesophageal Reflux Disease	1361
329	Gastrointestinal Stromal Tumor	1365
330	Gastroparesis.....	1369
331	Generalized Anxiety Disorder (GAD).....	1373
332	Genital Herpes (HSV).....	1377
333	Genital Warts (HPV)	1381
334	Gestational Diabetes Mellitus.....	1385
335	Gestational Diabetes Mellitus (Obstetric Focus).....	1389
336	Gestational Hypertension.....	1393
337	Gestational Trophoblastic Disease	1397
338	Giant Cell Arteritis	1401
339	Giardiasis	1405
340	Gingivitis and Periodontitis.....	1409
341	Glaucoma in Children	1413
342	Glioblastoma.....	1417
343	Glossopharyngeal Neuralgia	1423
344	Glycogen Storage Diseases.....	1427
345	Gonorrhea	1431
346	Gout.....	1435
347	Granuloma Annulare	1439
348	Granulomatosis with Polyangiitis	1443
349	Graves Disease	1447
350	Graves' Eye Disease	1451
351	Guillain-Barré Syndrome	1455
352	Gynecomastia	1459
353	HELLP Syndrome.....	1463
354	HIV/AIDS	1467
355	Haff Disease.....	1473
356	Hallucinogen Toxicity	1477
357	Hallux Valgus	1481
358	Hand, Foot, and Mouth Disease	1485
359	Hantavirus Pulmonary Syndrome.....	1489
360	Hashimoto Thyroiditis.....	1493
361	Head Lice (Pediculosis Capitis)	1497

362	Headache Attributed to Giant Cell Arteritis	1501
363	Headache Attributed to Intracranial Hypertension	1505
364	Headache Attributed to Intracranial Hypotension	1509
365	Headache Attributed to Sinusitis	1513
366	Headache in Brain Tumors	1517
367	Heart Block (AV Block)	1521
368	Heart Failure with Preserved Ejection Fraction	1525
369	Heart Failure with Preserved Ejection Fraction	1529
370	Heart Failure with Reduced Ejection Fraction	1533
371	Heat Stroke and Hyperthermia	1537
372	Helicobacter Pylori Infection	1541
373	Hemicrania Continua	1545
374	Hemolytic Anemias	1549
375	Hemolytic Uremic Syndrome (HUS)	1553
376	Hemophagocytic Lymphohistiocytosis and Macrophage Activation Syn- drome	1557
377	Hemophilia A	1561
378	Hemophilia B	1565
379	Hemorrhagic Stroke	1569
380	Hemorrhoids	1573
381	Henoch-Schönlein Purpura (IgA Vasculitis)	1577
382	Herniated Disc	1581
383	Hepatic Encephalopathy	1585
384	Hepatitis A	1589
385	Hepatitis B	1593
386	Hepatitis C	1597
387	Hepatitis D	1601
388	Hepatitis E	1605
389	Hepatoblastoma	1609
390	Hepatocellular Carcinoma	1615
391	Hereditary Angioedema	1621
392	Hereditary Hemochromatosis	1625
393	Hereditary Hemochromatosis	1629
394	Hereditary Spherocytosis	1633
395	Herpangina	1637
396	Herpes Simplex	1641
397	Herpes Zoster	1645
398	Hidradenitis Suppurativa	1649
399	Hip Fracture	1653
400	Hirschsprung Disease	1657
401	Histoplasmosis	1661
402	Hoarding Disorder	1665
403	Hodgkin Lymphoma	1669
404	Homocystinuria	1675
405	Hookworm / Soil-Transmitted Helminths	1679
406	Hordeolum (Stye) and Chalazion	1683

407	Human Metapneumovirus (hMPV)	1687
408	Human Metapneumovirus (hMPV)	1691
409	Huntington's Disease	1695
410	Hydrogen Sulfide Toxicity	1699
411	Hydronephrosis	1703
412	Hymenoptera Stings.....	1707
413	Hyperemesis Gravidarum.....	1711
414	Hyperopia.....	1715
415	Hyperosmolar Hyperglycemic State.....	1719
416	Hyperparathyroidism.....	1723
417	Hyperprolactinemia	1727
418	Hypersplenism	1731
419	Hypertension.....	1735
420	Hypertension in the Elderly	1739
421	Hyperthyroidism	1743
422	Hypertrophic Cardiomyopathy.....	1747
423	Hypertrophic Pyloric Stenosis	1751
424	Hypoglycemia	1755
425	Hypoparathyroidism.....	1759
426	Hypopituitarism.....	1763
427	Hypothermia.....	1767
428	Hypothyroidism	1771
429	Idiopathic Granulomatous Mastitis	1775
430	Idiopathic Hypersomnia	1779
431	Idiopathic Insomnia	1783
432	Idiopathic Intracranial Hypertension.....	1787
433	Idiopathic Pulmonary Fibrosis.....	1791
434	IgA Nephropathy.....	1795
435	IgA Vasculitis	1799
436	IgG4-Related Disease.....	1803
437	Illness Anxiety Disorder (Hypochondriasis)	1807
438	Immune Thrombocytopenic Purpura.....	1811
439	Impetigo	1815
440	Inclusion Body Myositis	1819
441	Indeterminate Colitis.....	1823
442	Infectious Mononucleosis	1827
443	Infective Endocarditis	1833
444	Inflammatory Breast Cancer.....	1837
445	Influenza.....	1843
446	Insomnia in the Context of Medical/Psychiatric Disorders.....	1849
447	Intraductal Papillary Mucinous Neoplasm (IPMN).....	1853
448	Intraductal Papilloma	1857
449	Intrauterine Growth Restriction	1861
450	Intraventricular Hemorrhage (IVH).....	1865
451	Intussusception.....	1869
452	Invasive Lobular Carcinoma (ILC)	1873

453	Inverted Papilloma	1879
454	Iodine Deficiency Disorders	1883
455	Iron Deficiency	1887
456	Iron Deficiency Anemia.....	1891
457	Iron Overdose	1895
458	Irregular Sleep-Wake Rhythm Disorder	1899
459	Irritable Bowel Syndrome	1903
460	Ischemic Cardiomyopathy	1907
461	Ischemic Stroke	1911
462	Jet Lag Disorder	1915
463	Juvenile Nasopharyngeal Angiofibroma (JNA)	1919
464	Kawasaki Disease.....	1923
465	Kawasaki Disease.....	1927
466	Keratitis	1931
467	Keratoconus.....	1935
468	Kidney Cancer (Renal Cell Carcinoma)	1939
469	Kikuchi-Fujimoto Disease.....	1945
470	Kleine-Levin Syndrome.....	1949
471	Klinefelter Syndrome (47,XXY)	1953
472	Kuru.....	1957
473	Kwashiorkor.....	1961
474	Kyphosis	1965
475	Labyrinthitis	1969
476	Lactose Intolerance.....	1973
477	Lambert-Eaton Myasthenic Syndrome (LEMS).....	1977
478	Large Bowel Obstruction	1981
479	Laryngitis	1985
480	Lateral Epicondylitis (Tennis Elbow).....	1989
481	Lead Poisoning (Plumbism).....	1993
482	Leishmaniasis	1997
483	Leprosy (Hansen's Disease).....	2001
484	Leptospirosis	2005
485	Lipoprotein(a) Disorders	2009
486	Lithium Toxicity	2013
487	Lobular Carcinoma In Situ (LCIS).....	2017
488	Long QT Syndrome	2023
489	Low Vision.....	2027
490	Ludwig Angina.....	2031
491	Lung Abscess.....	2035
492	Lung Cancer	2041
493	Lyme Disease (Borrelia burgdorferi)	2047
494	Lymphogranuloma Venereum (LGV).....	2051
495	Lynch Syndrome	2055
496	Lysosomal Storage Diseases.....	2059
497	Macular Edema	2063
498	Macular Hole	2067

499	Macular Pucker	2071
500	MDMA (Ecstasy) Toxicity.....	2075
501	Major Depressive Disorder (MDD)	2079
502	Malaria (Plasmodium spp.).....	2083
503	Male Breast Cancer	2089
504	Male Infertility.....	2095
505	Malignant Hyperthermia	2099
506	Malnutrition in the Elderly	2103
507	Mammalian Bites	2107
508	Mammary Duct Ectasia	2111
509	Maple Syrup Urine Disease.....	2115
510	Marasmic-Kwashiorkor	2119
511	Marasmus.....	2123
512	Marburg Virus	2127
513	Marfan Syndrome	2131
514	Marine Envenomation	2135
515	Massive Splenomegaly	2139
516	Mastitis (Puerperal).....	2143
517	Mastitis and Breast Abscess	2147
518	McArdle Disease	2153
519	Measles (Rubeola).....	2157
520	Meconium Aspiration Syndrome.....	2163
521	Medication Overuse Headache	2167
522	Medulloblastoma	2171
523	Melanoma.....	2177
524	Melasma	2183
525	Membranous Nephropathy	2187
526	Meningioma.....	2191
527	Mercury Toxicity.....	2195
528	Mesothelioma	2199
529	Metabolic Syndrome	2203
530	Metformin Overdose.....	2207
531	Methemoglobinemia.....	2211
532	Methyl Parathion and Other Organophosphate Insecticides.....	2215
533	Microscopic Polyangiitis	2219
534	Migraine	2223
535	Mild Cognitive Impairment.....	2227
536	Minimal Change Disease.....	2231
537	Mitral Regurgitation	2235
538	Mitral Stenosis.....	2239
539	Mitral Valve Prolapse	2243
540	Mixed Connective Tissue Disease	2247
541	Molluscum Contagiosum.....	2251
542	Morton Neuroma.....	2255
543	Motor Neuron Disease.....	2259
544	Mpox	2263

545	Mucopolysaccharidoses	2267
546	Mucormycosis	2271
547	Multiple Endocrine Neoplasia	2275
548	Multiple Gestation Complications	2279
549	Multiple Myeloma.....	2283
550	Multiple Myeloma (Bone Manifestations)	2289
551	Multiple Sclerosis	2295
552	Mumps	2299
553	Myasthenia Gravis	2305
554	Myelodysplastic Syndromes.....	2309
555	Myelofibrosis	2313
556	Myocardial Infarction	2317
557	Myocarditis	2321
558	Myofascial Pain Syndrome.....	2325
559	Myopia.....	2329
560	Myotonic Dystrophy.....	2333
561	NREM Parasomnias: Confusional Arousals	2337
562	NREM Parasomnias: Sleep-Related Eating Disorder	2341
563	NREM Parasomnias: Sleep Terrors	2345
564	NREM Parasomnias: Sleepwalking.....	2349
565	Narcissistic Personality Disorder.....	2353
566	Narcolepsy Type 1	2357
567	Narcolepsy Type 2	2361
568	Nasal Fracture	2365
569	Nasal Polyps	2369
570	Necrotizing Enterocolitis (NEC)	2373
571	Necrotizing Fasciitis	2377
572	Neonatal Abstinence Syndrome.....	2381
573	Neonatal Hypoglycemia	2385
574	Neonatal Jaundice / Hyperbilirubinemia / Kernicterus.....	2389
575	Neonatal Sepsis (Early-Onset and Late-Onset)	2393
576	Nephritic Syndrome	2397
577	Nephrolithiasis	2401
578	Nephrotic Syndrome	2405
579	Nesidioblastosis and Congenital Hyperinsulinism.....	2409
580	Neural Tube Defects.....	2413
581	Neuroblastoma	2417
582	Neurocysticercosis.....	2423
583	Neurofibromatosis Type 1	2427
584	New Daily Persistent Headache.....	2431
585	Non-24-Hour Sleep-Wake Rhythm Disorder	2435
586	Non-Alcoholic Fatty Liver Disease	2439
587	Non-Allergic Rhinitis.....	2443
588	Non-Gonococcal Urethritis (NGU).....	2447
589	Non-Hodgkin Lymphoma.....	2451
590	Normal Pressure Hydrocephalus.....	2457

591	Norovirus	2461
592	Nummular Dermatitis	2465
593	Obesity	2469
594	Obesity Hypoventilation Syndrome	2473
595	Obsessive-Compulsive Disorder (OCD).....	2477
596	Obstructive Sleep Apnea	2481
597	Obstructive Sleep Apnea	2485
598	Occipital Neuralgia.....	2489
599	Ocular Histoplasmosis Syndrome	2493
600	Oculocutaneous Albinism.....	2497
601	Onychomycosis (Tinea Unguium)	2501
602	Opioid Overdose.....	2505
603	Opioid Use Disorder.....	2509
604	Oral Candidiasis.....	2513
605	Oral Leukoplakia	2517
606	Oral Lichen Planus	2521
607	Oral Squamous Cell Carcinoma.....	2525
608	Organic Acidemias	2531
609	Organophosphate and Carbamate Insecticide Poisoning	2535
610	Osteoarthritis	2539
611	Osteogenesis Imperfecta	2543
612	Osteomyelitis.....	2547
613	Osteoporosis	2551
614	Osteosarcoma	2555
615	Osteosarcoma in Children	2561
616	Other Parasomnias: Exploding Head Syndrome.....	2567
617	Other Parasomnias: Sleep Enuresis	2571
618	Other Parasomnias: Sleep-Related Hallucinations	2575
619	Other Primary Headache Disorders.....	2579
620	Otitis Externa.....	2583
621	Otosclerosis	2587
622	Ovarian Cancer	2591
623	Ovarian Cancer	2597
624	Paget's Disease of Bone	2603
625	Paget's Disease of the Nipple	2607
626	Pancreatic Adenocarcinoma	2611
627	Pancreatic Exocrine Insufficiency	2617
628	Pancreatic Neuroendocrine Tumors.....	2621
629	Pancreatic Pseudocyst.....	2625
630	Panic Disorder / Panic Attacks.....	2629
631	Parkinson's Disease.....	2633
632	Paroxysmal Hemicrania	2637
633	Patau Syndrome (Trisomy 13)	2641
634	Patent Ductus Arteriosus (PDA) in Prematurity.....	2645
635	Pediatric Appendicitis.....	2649
636	Pelvic Inflammatory Disease.....	2653

637	Pelvic Inflammatory Disease.....	2657
638	Pemphigus Vulgaris	2661
639	Penile Cancer	2665
640	Penile Fracture.....	2671
641	Peptic Ulcer Bleeding	2675
642	Peptic Ulcer Disease.....	2679
643	Pericarditis.....	2683
644	Perinatal Asphyxia / Hypoxic-Ischemic Encephalopathy (HIE)	2687
645	Periodic Fever Syndromes	2691
646	Periodic Limb Movement Disorder.....	2695
647	Periodic Paralysis	2699
648	Peripheral Arterial Disease	2703
649	Peripheral Artery Disease	2707
650	Periventricular Leukomalacia (PVL)	2711
651	Persistent Depressive Disorder (Dysthymia)	2715
652	Persistent Pulmonary Hypertension of the Newborn (PPHN)	2719
653	Persistent Vegetative State	2723
654	Pertussis (Whooping Cough)	2727
655	Peyronie’s Disease.....	2731
656	Phantom Limb Pain.....	2735
657	Pharyngitis.....	2739
658	Phenylketonuria.....	2743
659	Pheochromocytoma	2747
660	Phimosis and Paraphimosis.....	2751
661	Phyllodes Tumor	2755
662	Pilonidal Sinus	2759
663	Pituitary Adenoma.....	2763
664	Placenta Accreta Spectrum.....	2767
665	Placenta Previa	2771
666	Placental Abruptio.....	2775
667	Plague (Yersinia pestis)	2779
668	Plantar Fasciitis	2783
669	Pleurisy	2787
670	Pneumoconiosis	2791
671	Pneumocystis jirovecii Pneumonia	2795
672	Pneumothorax	2799
673	Poliomyelitis	2803
674	Polyarteritis Nodosa.....	2807
675	Polycystic Ovary Syndrome.....	2811
676	Polycystic Ovary Syndrome.....	2815
677	Polycythemia Vera	2819
678	Polyhydramnios and Oligohydramnios	2823
679	Polymyalgia Rheumatica	2827
680	Polymyositis.....	2831
681	Porphyrias	2835
682	Portal Hypertension.....	2839

683	Post-Traumatic Headache	2843
684	Post-Traumatic Stress Disorder (PTSD)	2847
685	Postherpetic Neuralgia	2851
686	Postpartum Cardiomyopathy	2855
687	Postpartum Depression.....	2859
688	Postpartum Endometritis.....	2863
689	Postpartum Hemorrhage	2867
690	Postpartum Thyroiditis.....	2871
691	Prader-Willi Syndrome.....	2875
692	Preeclampsia.....	2879
693	Premature Ovarian Insufficiency	2883
694	Premature Rupture of Membranes.....	2887
695	Premenstrual Syndrome / Premenstrual Dysphoric Disorder	2891
696	Presbycusis	2895
697	Presbyopia	2899
698	Presbyopia and Cataracts.....	2903
699	Pressure Ulcers	2907
700	Preterm Birth.....	2911
701	Preterm Labor and Preterm Birth.....	2915
702	Priapism	2919
703	Primary Amebic Meningoencephalitis (Naegleria fowleri)	2923
704	Primary Angle-Closure Glaucoma	2927
705	Primary Biliary Cholangitis	2931
706	Primary Open-Angle Glaucoma.....	2935
707	Primary Sclerosing Cholangitis	2939
708	Propriospinal Myoclonus at Sleep Onset	2943
709	Prostate Cancer.....	2947
710	Psoriasis	2953
711	Psoriatic Arthritis.....	2957
712	Psoriatic Arthritis.....	2961
713	Psychophysiologic Insomnia	2965
714	Pterygium	2969
715	Pulmonary Embolism.....	2973
716	Pulmonary Hypertension	2977
717	Pyelonephritis.....	2981
718	REM Parasomnias: Nightmare Disorder	2985
719	REM Parasomnias: REM Sleep Behavior Disorder	2989
720	REM Parasomnias: Recurrent Isolated Sleep Paralysis	2993
721	Rabies	2997
722	Radiation Sickness (Acute Radiation Syndrome)	3001
723	Reactive Arthritis	3005
724	Recurrent Pregnancy Loss.....	3009
725	Relapsing Polychondritis	3013
726	Renal Osteodystrophy	3017
727	Renal Tubular Acidosis.....	3021
728	Respiratory Distress Syndrome (RDS).....	3025

729	Restless Legs Syndrome	3029
730	Restrictive Cardiomyopathy	3033
731	Retinal Detachment	3037
732	Retinal Vein Occlusion	3041
733	Retinitis Pigmentosa	3045
734	Retinoblastoma	3049
735	Retinopathy of Prematurity	3055
736	Retinopathy of Prematurity (ROP)	3059
737	Rh Incompatibility / Hemolytic Disease of the Newborn	3063
738	Rhabdomyolysis	3067
739	Rhabdomyosarcoma	3071
740	Rheumatic Heart Disease	3077
741	Rheumatoid Arthritis	3081
742	Rheumatoid Arthritis	3085
743	Rickets and Osteomalacia	3089
744	Rocky Mountain Spotted Fever	3093
745	Rosacea	3097
746	Rosai-Dorfman Disease	3101
747	Roseola (HHV-6 / Sixth Disease)	3105
748	Rotator Cuff Tendinopathy and Impingement	3109
749	Rotavirus	3113
750	Rubella	3117
751	SIADH	3121
752	SSRI Overdose and Serotonin Syndrome	3125
753	SUNCT and SUNA	3129
754	Salicylate (Aspirin) Overdose	3133
755	Sarcoidosis	3137
756	Sarcoidosis	3141
757	Sarcoidosis	3145
758	Sarcopenia	3149
759	Sarcopenic Obesity	3153
760	Scabies	3157
761	Scarlet Fever	3161
762	Schistosomiasis	3165
763	Schizoaffective Disorder	3169
764	Schizophrenia	3173
765	Schwannoma	3177
766	Scleritis	3181
767	Scoliosis	3185
768	Scorpion Stings	3189
769	Seborrheic Dermatitis	3193
770	Selenium Deficiency	3197
771	Sepsis and Septic Shock	3201
772	Septic Abortion	3207
773	Septic Arthritis	3211
774	Severe Combined Immunodeficiency	3217

775	Shift Work Sleep Disorder	3221
776	Short Bowel Syndrome	3225
777	Sick Sinus Syndrome	3229
778	Sickle Cell Disease.....	3233
779	Sideroblastic Anemia	3237
780	Silicosis	3241
781	Sleep-Related Bruxism	3245
782	Sleep-Related Rhythmic Movement Disorder.....	3249
783	Small Bowel Obstruction	3253
784	Snake Envenomation	3257
785	Social Anxiety Disorder (Social Phobia)	3261
786	Somatic Symptom Disorder.....	3265
787	Specific Phobias	3269
788	Spider Bites.....	3273
789	Splenic Abscess	3277
790	Splenic Artery Aneurysm.....	3283
791	Splenic Cyst.....	3287
792	Splenic Cysts.....	3291
793	Splenic Infarction	3295
794	Splenic Lymphoma	3299
795	Splenic Rupture	3305
796	Splenomegaly	3309
797	Spontaneous Abortion (Miscarriage).....	3313
798	Squamous Cell Carcinoma.....	3317
799	Stargardt Disease	3323
800	Stevens-Johnson Syndrome and Toxic Epidermal Necrolysis	3327
801	Stillbirth (Intrauterine Fetal Demise)	3331
802	Stimulant Overdose (Cocaine, Methamphetamine)	3335
803	Stimulant Use Disorder (Cocaine, Amphetamines)	3339
804	Strabismus	3343
805	Subdural Hematoma	3347
806	Sudden Sensorineural Hearing Loss	3351
807	Supraventricular Tachycardia.....	3355
808	Syphilis (Treponema pallidum)	3359
809	Systemic Lupus Erythematosus	3365
810	Systemic Lupus Erythematosus (Skin Manifestations)	3369
811	Systemic Sclerosis (Scleroderma)	3373
812	Takayasu Arteritis.....	3379
813	Tapeworm Infections (Taeniasis / Cysticercosis).....	3383
814	Temporal Lobe Epilepsy	3387
815	Temporomandibular Joint Disorders	3391
816	Tension-Type Headache	3395
817	Testicular Cancer.....	3399
818	Tetanus (Clostridium tetani).....	3405
819	Thalassemia.....	3411
820	Theophylline Overdose	3415

821	Thrombotic Thrombocytopenic Purpura	3419
822	Thyroid Nodules and Cancer	3423
823	Thyroiditis	3429
824	Tinea Infections	3433
825	Tinea Versicolor (Pityriasis Versicolor)	3439
826	Tinnitus	3443
827	Tonsillar Hypertrophy	3447
828	Tonsillitis	3451
829	Tourette Syndrome	3455
830	Toxoplasmosis	3459
831	Transient Ischemic Attack	3463
832	Trichomoniasis	3467
833	Tricuspid Regurgitation	3471
834	Tricyclic Antidepressant (TCA) Overdose	3475
835	Trigeminal Autonomic Cephalalgias	3479
836	Trigeminal Neuralgia	3483
837	Trigger Finger	3487
838	Triple-Negative Breast Cancer (TNBC)	3491
839	Tuberculosis	3497
840	Tuberculosis (Extrapulmonary)	3501
841	Turner Syndrome (45,X)	3507
842	Tympanic Membrane Perforation	3511
843	Type 1 Diabetes Mellitus	3515
844	Type 2 Diabetes Mellitus	3519
845	Typhoid Fever (Salmonella typhi)	3523
846	Ulcerative Colitis	3529
847	Undifferentiated Connective Tissue Disease	3533
848	Urea Cycle Disorders	3537
849	Urinary Incontinence	3541
850	Urinary Incontinence	3545
851	Urosepsis	3549
852	Urticaria	3555
853	Uterine Fibroids	3559
854	Uterine Rupture	3563
855	Usher Syndrome	3567
856	Uveitis	3571
857	Vaginal Cancer	3575
858	Vaginal Cancer	3581
859	Variant CJD (Mad Cow Disease)	3587
860	Varicella-Zoster (Systemic Aspects)	3591
861	Vasa Previa	3595
862	Vascular Dementia	3599
863	Ventricular Tachycardia	3603
864	Vertigo	3607
865	Vestibular Neuritis	3611
866	Viral Encephalitis	3615

867	Viral Exanthems	3621
868	Vitamin A Deficiency.....	3625
869	Vitamin B12 (Cobalamin) Deficiency	3629
870	Vitamin B12 Deficiency Anemia	3633
871	Vitamin B1 (Thiamine) Deficiency — Beriberi and Wernicke-Korsakoff Syndrome	3637
872	Vitamin B2 (Riboflavin) Deficiency — Ariboflavinosis.....	3641
873	Vitamin B3 (Niacin) Deficiency — Pellagra	3645
874	Vitamin B6 (Pyridoxine) Deficiency	3649
875	Vitamin B7 (Biotin) Deficiency	3653
876	Vitamin B9 (Folate) Deficiency	3657
877	Vitamin C Deficiency — Scurvy	3661
878	Vitamin D Deficiency — Rickets and Osteomalacia.....	3665
879	Vitamin E Deficiency	3669
880	Vitamin K Deficiency.....	3673
881	Vitiligo.....	3677
882	Vocal Cord Nodules	3681
883	Vocal Cord Paralysis	3685
884	Von Willebrand Disease	3689
885	Vulvar Cancer.....	3693
886	Vulvar Cancer.....	3699
887	Vulvovaginal Candidiasis.....	3705
888	Warts	3709
889	West Nile Virus	3713
890	Williams Syndrome.....	3717
891	Wilms Tumor (Nephroblastoma).....	3721
892	Wilson’s Disease.....	3727
893	Wound Infections and Surgical Site Infections.....	3731
894	Yellow Fever	3735
895	Zenker Diverticulum	3739
896	Zika Virus	3743
897	Zinc Deficiency.....	3747
898	Zollinger-Ellison Syndrome	3751
	Organ–Disease Dictionary.....	3755
	Quick Reference	3777
899	Classification Table.....	3781
	Prevalence of Diseases by Demographic Group	3787
	Abbreviations Used.....	3811
	Glossary of Medical Terms.....	3813
	References.....	3817

Preface

All About Human Diseases is a comprehensive, patient-oriented medical reference that catalogs 910 diseases and disorders affecting every organ and system of the human body. Each chapter follows a consistent 11-section structure designed to give readers a complete yet accessible understanding of each condition: Overview, Causes, Risk Factors, Signs and Symptoms, Progression and Stages, Complications, Diagnosis, Prevention, Treatment Options, Living with It, and Outlook.

This book is intended for students, educators, healthcare trainees, and anyone seeking to understand diseases in plain, jargon-free language. Medical terminology has been simplified throughout without sacrificing accuracy, making complex medical concepts approachable for a general audience.

The Organ–Disease Dictionary appendix provides an alternative navigation path, grouping every disease by the primary organ or organ system it affects. A comprehensive index with page references completes the volume.

Chaman Singh Verma

Foreword

The study of human disease is the study of human life itself. Each of the 910 entries in this volume represents not only a clinical entity but a story—of how the body responds to injury, infection, genetic mutation, and the wear of time. This book brings those stories together in a format that prioritizes clarity and compassion.

Medicine advances daily, but the fundamental need to understand what happens to our bodies and why remains constant. Whether you are a student beginning your medical journey, a practitioner refreshing your knowledge, or a patient or caregiver seeking reliable information, I hope this reference serves you well.

Chapter 1

Abdominal Aortic Aneurysm

Overview

An abdominal aortic aneurysm (AAA) is a permanent enlargement of the abdominal aorta, usually defined as an aortic diameter of at least 3 cm or more than 50% above the expected normal size. The aneurysm may enlarge silently over years. The principal danger is rupture, which causes life-threatening internal hemorrhage. Management is based on aneurysm size, growth rate, symptoms, operative risk, and anatomy.

Causes

AAA results from progressive weakening and remodeling of the aortic wall. Atherosclerotic inflammation, loss of elastic tissue, and degradation of structural proteins contribute to wall dilation. Less common causes include inherited connective-tissue disorders, vasculitis, infection, trauma, and prior aortic surgery.

Risk Factors

- Increasing age, especially older age in men
- Current or previous cigarette smoking
- Family history of AAA
- Hypertension and atherosclerotic cardiovascular disease
- Male sex and other vascular aneurysms
- Genetic connective-tissue disorders

Signs and Symptoms

Most AAAs are asymptomatic and are discovered during examination or imaging for another reason. Possible symptoms include persistent or deep abdominal, flank, or back pain and a pulsating abdominal mass. Sudden severe abdominal or back pain, fainting, low blood pressure, pallor, or collapse may indicate rupture and require emergency treatment.

Progression and Stages

Small aneurysms may remain stable and can be monitored with periodic ultrasound. Some enlarge gradually, while others expand more rapidly. Symptoms, rapid growth,

or a diameter at which rupture risk outweighs procedural risk generally prompt repair. Rupture can lead to shock and death within a short time.

Complications

- Aneurysm rupture and internal bleeding
- Thrombus formation with embolization to the legs or organs
- Compression of nearby abdominal structures
- Aortoenteric or aortocaval fistula, rarely
- Kidney, bowel, or limb ischemia

Diagnosis

Abdominal ultrasound is commonly used for detection and surveillance. Computed tomography angiography defines the aneurysm's size, extent, branch-vessel involvement, and suitability for repair. Magnetic resonance angiography may be used when CT contrast or radiation is unsuitable. Evaluation also includes cardiovascular risk assessment and preoperative testing.

Prevention

- Stop smoking and avoid second-hand smoke
- Control blood pressure, diabetes, and lipid abnormalities
- Maintain regular physical activity and a healthy weight
- Attend scheduled imaging surveillance
- Seek urgent care for sudden severe abdominal or back pain

Treatment Options

Small, asymptomatic AAAs are usually managed with surveillance and risk-factor reduction. Symptomatic, rapidly enlarging, or sufficiently large aneurysms require vascular-surgery assessment for elective repair. Endovascular aneurysm repair places a stent graft inside the aorta, whereas open repair replaces the diseased segment with a graft. A suspected rupture requires immediate resuscitation and emergency surgical or endovascular repair.

Living with It

Patients should keep imaging and vascular-surgery appointments, take prescribed cardiovascular medicines, stop smoking, and report new abdominal, flank, or back pain promptly. Physical activity is generally encouraged, but the treating clinician should guide activity when the aneurysm is large or symptomatic.

Outlook

The outlook is favorable for many patients when an AAA is detected before rupture and monitored or repaired at an appropriate time. Prognosis after rupture is substantially worse, making surveillance and rapid attention to warning symptoms essential.

Chapter 2

Abnormal Uterine Bleeding

Overview

- Abnormal uterine bleeding (AUB) is any deviation from normal menstrual volume, duration, or regularity. The PALM-COEIN classification system provides standardized terminology: structural causes include Polyp, Adenomyosis, Leiomyoma, Malignancy and abnormal increase in cells
- Non-structural causes include Coagulopathy, Ovulatory dysfunction, Endometrial, Iatrogenic, and Not yet classified.

Causes

This condition arises from structural abnormalities (PALM-COEIN classification: Polyps, Ovulatory dysfunction, Leiomyoma, Malignancy, Coagulopathy), endometrial dysfunction, or iatrogenic causes including hormonal contraception.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience abnormal menstrual bleeding pattern. Diagnosis includes complete blood count, serum hCG to exclude pregnancy, TSH, prolactin, and pelvic ultrasound
- Hysteroscopy and endometrial biopsy are indicated for persistent AUB. Treatment depends on the cause: hormonal therapy, tranexamic acid, NSAIDs, or surgical management.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of abnormal uterine bleeding require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 3

Abscess

Overview

A cutaneous abscess is a localized collection of pus in the dermis or subcutaneous tissue, often caused by *S. aureus* (including MRSA). This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of infection of a hair follicle (folliculitis progressing to furuncle to carbuncle), blocked sweat gland, or direct inoculation. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Most people recover well with adequate removal of fluid
- Recurrent abscesses should prompt evaluation for risk factors (diabetes, HIV, nasal *S. aureus* carriage, community MRSA).
- Close contact with infected individuals
- Compromised immune system
- Travel to endemic areas
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience a painful, fluctuant, erythematous nodule with surrounding induration. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of abscess require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification

- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

- Treatment typically involves incision and removal of fluid (I&D), which is the primary intervention
- Antibiotics are not routinely required for simple abscesses after adequate I&D but are indicated for surrounding cellulitis, immunocompromised patients, multiple lesions, or facial location.
- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 4

Acetaminophen (Paracetamol) Overdose

Overview

Acetaminophen overdose is the most common pharmaceutical overdose in the United States and the leading cause of acute liver failure. A toxic dose in adults is typically >7.5–10 g (or >150 mg/kg). This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of depletion of liver glutathione and accumulation of the toxic metabolite N-acetyl-p-benzoquinone imine (NAPQI), which covalently binds to hepatocyte proteins, causing centrilobular liver tissue death.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of acetaminophen (paracetamol) overdose require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Clinical features progress through four phases: Phase I (0–24 hours): nausea, vomiting, malaise, pallor
- Phase II (24–72 hours): right upper quadrant pain, elevated transaminases, PT/INR prolongation
- Phase III (72–96 hours): peak hepatotoxicity, liver encephalopathy, coagulopathy, jaundice, acute liver failure
- Phase IV (4 days–2 weeks): recovery or progression to death.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using serum acetaminophen level at >4 hours post-ingestion plotted on the Rumack-Matthew nomogram. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on N-acetylcysteine (NAC), which replenishes glutathione and provides substrate for non-toxic sulfate conjugation, most effective if started within 8–10 hours. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Your outlook depends on timely treatment
- The King's College Criteria guide liver transplantation decisions.

Chapter 5

Achalasia

Overview

Achalasia is a primary esophageal motility disorder characterized by impaired lower esophageal sphincter (LES) relaxation and absent esophageal peristalsis.

Causes

This condition results from progressive degeneration of inhibitory nitrenergic ganglion cells in the myenteric (Auerbach) plexus of the esophageal wall, leading to impaired lower esophageal sphincter relaxation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive difficulty swallowing to both solids and liquids, regurgitation of undigested food, chest pain, and weight loss.

Emergency warning signs

- Severe or worsening symptoms of achalasia require immediate medical evaluation

Progression and Stages

This condition happens because of autoimmune deterioration of inhibitory nitrenergic ganglion cells in the myenteric (Auerbach) plexus of the esophagus. The condition may

progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Your doctor confirms the diagnosis with esophageal manometry showing incomplete LES relaxation and aperistalsis
- Barium swallow demonstrates a characteristic bird's beak appearance of the distal esophagus. Management options include using small incisions and a camera Heller myotomy with fundoplication, peroral using a thin camera tube myotomy (POEM), pneumatic balloon dilation, and botulinum toxin injection into the LES.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good for symptom control, though patients have a slightly increased risk of esophageal squamous cell carcinoma. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 6

Achondroplasia

Overview

Achondroplasia is the most common form of disproportionate short stature, caused by a gain-of-function mutation in FGFR3 (most commonly G380R), with an incidence of 1 in 25,000 live births.

Causes

This condition happens because of constitutive activation of FGFR3, which negatively regulates endochondral ossification in the growth plate, leading to impaired long bone growth.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience rhizomelic limb shortening (proximal segments more affected), macrocephaly with frontal bossing, midface underdevelopment, trident hand, genu varum, thoracolumbar kyphosis, and foramen magnum stenosis. Intelligence is normal.

Emergency warning signs

- Severe or worsening symptoms of achondroplasia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Complications include cervicomedullary compression, hydrocephalus, spinal stenosis, and obesity.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical and radiographic findings with FGFR3 genetic confirmation. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on monitoring for hydrocephalus, decompression surgery for spinal stenosis, limb lengthening procedures (controversial), and vosoritide (CNP analog, FDA-approved in 2021) to improve growth velocity. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical

therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 7

Acne Vulgaris

Overview

Acne vulgaris is a chronic inflammatory disorder of the pilosebaceous unit, affecting up to 85% of adolescents and young adults.

Causes

This condition results from disruption of normal acne vulgaris function.

- This condition happens because of four pathogenic factors: follicular hyperkeratinization, increased sebum production (androgen-mediated), *Cutibacterium acnes* (formerly *Propionibacterium acnes*) colonization, and inflammation. Clinical lesions include open comedones (blackheads), closed comedones (whiteheads), papules, pustules, nodules, and cysts
- Acne predominantly affects the face, chest, and back.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of acne vulgaris require immediate medical evaluation

Progression and Stages

Severity is classified as mild (comedonal), moderate (papulopustular), or severe (nodulocystic, conglobata). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Isotretinoin requires iPLEDGE program enrollment due to teratogenicity. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is favorable
- Many patients improve with age.

Chapter 8

Acoustic Neuroma (Vestibular Schwannoma)

Overview

Vestibular schwannoma is a non-cancerous tumor arising from the Schwann cells of the vestibular portion of cranial nerve VIII and is the most common tumor of the cerebellopontine angle. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of neoplastic growth of Schwann cells. Bilateral tumors are pathognomonic of neurofibromatosis type 2. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience unilateral sensorineural hearing loss, ringing in the ears, and imbalance
- Larger tumors may compress adjacent structures causing facial numbness (CN V),

- facial weakness (CN VII), and brainstem compression.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of acoustic neuroma (vestibular schwannoma) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on MRI with gadolinium, which is the gold standard. Management options include observation with serial imaging (for small, slow-growing tumors), stereotactic radiosurgery, and microsurgical removal.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive for small tumors, with hearing preservation as a key goal. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 9

Acromegaly

Overview

Acromegaly is a chronic disorder caused by excessive growth hormone (GH) secretion, almost always from a pituitary somatotroph adenoma (95%), with GH stimulating liver production of insulin-like growth factor 1 (IGF-1) which mediates most of the somatic effects, and onset being insidious with a mean delay of 7–10 years between symptom onset and diagnosis.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience coarsening of facial features, prognathism (jaw protrusion), frontal bossing, enlarged hands and feet, soft tissue swelling, macroglossia, excessive sweating, deepening of the voice, arthropathy, carpal tunnel syndrome, high blood pressure, diabetes mellitus, sleep apnea, cardiomyopathy, and increased risk of colorectal polyps and cancer. Diagnosis shows elevated age- and sex-matched IGF-1 level and failure to suppress GH below 1 ng/mL after 75-g oral glucose tolerance test, with MRI showing a pituitary macroadenoma in most cases.

Emergency warning signs

- Severe or worsening symptoms of acromegaly require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include transsphenoidal surgery (first-line, curative in 80–90% of microadenomas), somatostatin analogs (octreotide LAR, lanreotide, pasireotide) for persistent disease, pegvisomant (GH receptor antagonist), and dopamine agonists (cabergoline) for mild cases. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is improved with early treatment, which reduces cardiovascular morbidity and mortality. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 10

Actinic Keratosis

Overview

Causes

This condition happens because of UV-induced keratinocyte dysplasia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of actinic keratosis require immediate medical evaluation

Progression and Stages

Actinic keratosis is a precancerous lesion caused by chronic UV radiation exposure, representing the earliest stage of the squamous cell carcinoma spectrum. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Biopsy may be required to exclude carcinoma. Management options depend on the number and location of lesions: freezing treatment (liquid nitrogen), topical 5-fluorouracil, imiquimod, ingenol mebutate, photodynamic therapy, and field therapy for extensive actinic damage.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed

- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with treatment
- Regular skin surveillance is recommended.

Chapter 11

Acute/Short-Term Insomnia

Overview

Acute insomnia is a sleep disturbance lasting less than three months, typically triggered by a specific stressor (acute medical illness, hospitalization, surgery, bereavement, work stress, jet lag). It is extremely common, affecting 30–50% of adults annually. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of transient hyperarousal and maladaptive behavioral responses (extended time in bed, napping, irregular sleep schedules). Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience difficulty initiating or maintaining sleep that is temporally linked to an identifiable stressor.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of acute/short-term insomnia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment focuses on sleep hygiene, stimulus control, and short-term pharmacotherapy limited to 1–2 weeks to prevent progression to chronic insomnia.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive
- Most cases resolve spontaneously when the stressor resolves.

Chapter 12

Acute Bacterial Sinusitis

Overview

Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

- Acute bacterial sinusitis is a bacterial infection of the paranasal sinuses lasting less than 4 weeks, most commonly following a viral upper respiratory infection
- The most frequent pathogens are *Streptococcus pneumoniae*, *Haemophilus influenzae*, and *Moraxella catarrhalis*.

Causes

This condition happens because of bacterial superinfection after viral mucosal inflammation impairs sinus removal of fluid. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of acute bacterial sinusitis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- You may experience facial pain or pressure, nasal obstruction, purulent rhinorrhea, and loss of smell
- To make the diagnosis, doctors look for persistent symptoms for more than 10 days without improvement, worsening after initial improvement, or severe symptoms with high fever and purulent discharge for 3–4 consecutive days.

Complications

- Most people recover well with appropriate antibiotic therapy
- Complications (orbital, intracranial) are rare but serious.
- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer

- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include amoxicillin-clavulanate as first-line therapy, with nasal saline irrigation, intranasal corticosteroids, and decongestants as adjunctive measures. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 13

Acute Bronchitis

Overview

- Acute bronchitis is an inflammation of the bronchial mucosa, most commonly caused by viral infections (rhinoviruses, coronaviruses, influenza, parainfluenza, adenovirus), accounting for approximately 90% of cases
- Bacterial causes include *Mycoplasma pneumoniae*, *Chlamydophila pneumoniae*, and *Bordetella pertussis*.

Causes

This condition happens because of infection of the bronchial mucosa leading to inflammation and increased mucus production.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience acute onset of cough (productive or nonproductive), often following an upper respiratory infection, with or without sputum production, chest discomfort, and low-grade fever
- The cough may persist for 2–3 weeks.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of acute bronchitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, with chest X-ray typically normal (helping to differentiate from pneumonia). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves symptomatic: rest, hydration, analgesics, cough suppressants for sleep-disturbing cough, and bronchodilators for wheezing
- Antibiotics are not indicated for most cases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well, with resolution in 1–3 weeks. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 14

Acute Decompensated Heart Failure

Overview

This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of precipitating factors including medication non-adherence, dietary indiscretion, myocardial reduced blood flow, arrhythmias, infections, and anemia. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience shortness of breath at rest, orthopnea, peripheral swelling, and signs of lung congestion. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of acute decompensated heart failure require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

- Acute decompensated heart failure is a sudden worsening of heart failure signs and symptoms, most commonly presenting as lung congestion with shortness of breath, orthopnea, and crackles on auscultation
- It is the leading cause of hospitalization in patients over 65.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this based on clinical assessment, chest imaging, and natriuretic peptide levels. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment focuses on intravenous loop diuretics for decongestion, vasodilators for hypertensive crisis, and inotropic agents for cardiogenic shock
- Non-invasive positive pressure ventilation reduces the need for intubation.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk

- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Your outlook depends on the underlying cause and response to therapy
- Early post-discharge follow-up and transition-of-care programs reduce readmission rates.
- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 15

Acute Gastritis

Overview

Acute gastritis is a self-limited inflammation of the gastric mucosa.

Causes

This condition results from disruption of normal acute gastritis function.

- Common causes include NSAIDs, alcohol, stress (in critically ill patients), and infections
- Stress-related mucosal disease (SRMD) occurs in critically ill patients, particularly those requiring mechanical ventilation or with coagulopathy.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience epigastric pain, nausea, vomiting, and hematemesis.

Emergency warning signs

- Severe or worsening symptoms of acute gastritis require immediate medical evaluation

Progression and Stages

This condition happens because of disruption of the gastric mucosal barrier by offending agents, typically affecting the fundus and body with potential progression to erosion or

ulceration. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on clinical presentation
- Endoscopy may be performed when indicated.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment focuses on removal of the offending agent and acid suppression with PPIs or H2 receptor antagonists
- Prophylaxis is recommended for high-risk ICU patients.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with resolution upon removal of the causative factor. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 16

Acute Interstitial Nephritis

Overview

Acute interstitial nephritis (AIN) is an immune-mediated inflammatory condition of the kidney tubules and interstitium, a common cause of AKI in hospitalized patients.

Causes

This condition results from disruption of normal acute interstitial nephritis function.

- The most common cause is drug reaction (antibiotics [penicillins, cephalosporins, fluoroquinolones], NSAIDs, PPIs, diuretics)
- Other causes include infections, autoimmune diseases, and idiopathic. This condition happens because of a hypersensitivity reaction to the offending agent.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- AKI with sterile pyuria and mild proteinuria should raise suspicion.
- Pain or discomfort in the affected area

Less common symptoms

- Clinical features may include fever, rash, eosinophilia, and eosinophiluria (the classic triad of fever, rash, and arthralgias is uncommon)

Emergency warning signs

- Severe or worsening symptoms of acute interstitial nephritis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for kidney biopsy showing lymphocytic infiltration, swelling, and tubulitis. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on discontinuation of the offending drug

- Corticosteroids are used for severe or delayed recovery.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable with early identification and drug withdrawal. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 17

Acute Kidney Injury

Overview

Acute kidney injury (AKI) is a rapid decline in kidney function over hours to days, resulting in accumulation of nitrogenous waste products (urea, creatinine) and inability to maintain fluid, electrolyte, and acid-base homeostasis. The KDIGO criteria define AKI as an increase in serum creatinine ≥ 0.3 mg/dL within 48 hours, increase in creatinine to ≥ 1.5 times baseline within 7 days, or urine volume < 0.5 mL/kg/h for 6 hours. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of the specific underlying cause leading to reduced GFR.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience oliguria or anuria, fluid overload, electrolyte disturbances, and metabolic acidosis.

Emergency warning signs

- Severe or worsening symptoms of acute kidney injury require immediate medical evaluation

Progression and Stages

Causes are classified as prerenal (decreased kidney perfusion: dehydration, heart failure, sepsis, NSAIDs), intrinsic (acute tubular tissue death from reduced blood flow or nephrotoxins, glomerulonephritis, interstitial nephritis, thrombotic microangiopathy), and postrenal (obstruction: kidney stones, enlarged prostate, pelvic malignancy). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on serum creatinine elevation, urine output criteria, urinalysis, and imaging. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on addressing the underlying cause, fluid and electrolyte management, and kidney replacement therapy (dialysis) for severe cases. Medications to manage

symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Mortality is significant, particularly in hospitalized patients. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 18

Acute Lymphoblastic Leukemia

Overview

Acute lymphoblastic leukemia is the most common childhood cancer (peak age 2–5 years), characterized by growth of immature lymphoid precursors (blasts) in the bone marrow, with B-cell ALL accounting for approximately 85% of childhood cases. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)

- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience bone marrow failure: anemia (fatigue, pallor), neutropenia (fever, infections), and thrombocytopenia (bleeding, bruising), along with bone pain, lymphadenopathy, hepatosplenomegaly, and testicular enlargement, with CNS involvement possibly presenting with headache, vomiting, or cranial nerve palsies. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of acute lymphoblastic leukemia require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases

to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using bone marrow biopsy showing >20% blasts, immunophenotyping (flow cytometry), cytogenetics [hyperdiploidy, t(12

- 21) ETV6-RUNX1—favorable
- T(9
- 22) BCR-ABL—unfavorable
- MLL rearrangements, Philadelphia chromosome-like ALL], and molecular studies.
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment typically involves risk-stratified and prolonged (2–3 years): induction (vincristine, corticosteroids, L-asparaginase, $\pm$ anthracyclines), consolidation, interim maintenance, delayed intensification, and maintenance therapy, with CNS-directed therapy (intrathecal chemotherapy, $\pm$ cranial radiation) essential. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Most people recover well in children, with cure rates exceeding 90% in developed countries, though adult ALL has a worse outlook (40–50% long-term survival), and CAR-T cell therapy (tisagenlecleucel) and blinatumomab are used for relapsed/refractory B-cell ALL. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 19

Acute Lymphoblastic Leukemia (ALL) in Children

Overview

This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

- ALL is the most common childhood cancer (25% of all pediatric malignancies) and the most common leukemia in children (see Chapter 10, Diseases of the Blood for comprehensive coverage). Peak incidence is 2–5 years. The majority of childhood ALL is B-cell precursor (B-ALL), and 25% have the t(12
- 21) translocation (ETV6-RUNX1 fusion) which confers a favorable outlook.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)

- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience pallor, fatigue, fever, bone pain, tiny red or purple spots, bruising, and hepatosplenomegaly. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of acute lymphoblastic leukemia (all) in children require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin

from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment usually involves risk-stratified chemotherapy with 5-year survival rates >90% in children. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and

depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 20

Acute Myeloid Leukemia

Overview

This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)

- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience bone marrow failure (anemia, neutropenia, thrombocytopenia) and symptoms of organ infiltration (hepatosplenomegaly, gum abnormal increase in cells, skin infiltration). Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of acute myeloid leukemia require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases

to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

To make the diagnosis, doctors look for bone marrow biopsy showing $\geq 20\%$ blasts, immunophenotyping, cytogenetics (t(15

- 17) APL—favorable
- Inv(16), t(8
- 21)—favorable
- Complex karyotype, monosomy 5/7—unfavorable), and molecular studies (FLT3, NPM1, CEBPA, IDH1/2, TP53 mutations).
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

- Treatment focuses on induction chemotherapy (cytarabine + anthracycline [“7+3”] or CPX-351 for secondary AML) followed by consolidation (cytarabine, allogeneic stem cell transplantation for intermediate/high-risk), with acute promyelocytic leukemia (APL, t(15
- 17), PML-RARA) treated with all-trans retinoic acid (ATRA) + arsenic trioxide with excellent cure rates (>90%).
- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies

(acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 21

Acute Otitis Media

Overview

Acute otitis media is a bacterial or viral infection of the middle ear that affects 80% of children by age 3.

Causes

This condition results from disruption of normal acute otitis media function.

- This condition happens because of Eustachian tube dysfunction secondary to upper respiratory infection, allowing bacterial or viral entry
- The most common pathogens are *Streptococcus pneumoniae*, *Haemophilus influenzae*, and *Moraxella catarrhalis*.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience ear pain, fever, irritability, and hearing loss
- Otoloscopic examination reveals a bulging, erythematous tympanic membrane with loss of landmarks and possible purulent effusion.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of acute otitis media require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical and otoscopic examination. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include analgesics and observation in mild cases, with amoxicillin as first-line antibiotic therapy for moderate-to-severe disease or treatment failure
- Recurrent acute otitis media may warrant myringotomy with tympanostomy tube placement.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 22

Acute Otitis Media

Overview

Acute otitis media (AOM) is the most common bacterial infection in children (see Chapter 24, Diseases of the Ear for comprehensive coverage). It affects 80% of children by age 3, with peak incidence at 6–24 months. Pathogens include *Streptococcus pneumoniae*, *Haemophilus influenzae* (nontypeable), and *Moraxella catarrhalis*.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience otalgia, fever, irritability, and bulging tympanic membrane with decreased mobility.

Emergency warning signs

- Severe or worsening symptoms of acute otitis media require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for acute onset of symptoms and middle ear effusion on otoscopy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include observation (watchful waiting for children >6 months with mild symptoms) or antibiotics (amoxicillin first-line). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 23

Acute Pancreatitis

Overview

- Acute pancreatitis is acute inflammation of the pancreas characterized by sudden onset of severe epigastric pain radiating to the back, often with nausea and vomiting. The two most common causes worldwide are gallstones (40%) and alcohol excess (30%)
- Other causes include hypertriglyceridemia (triglycerides >1000 mg/dL), medications (azathioprine, valproic acid, thiazides), hypercalcemia, trauma, ERCP, and genetic factors (PRSS1, SPINK1 mutations).

Causes

This condition happens because of premature activation of digestive enzymes within the pancreas, leading to autodigestion and inflammation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience characteristic abdominal pain, nausea, and vomiting.

Emergency warning signs

- Severe or worsening symptoms of acute pancreatitis require immediate medical evaluation

Progression and Stages

Severity is assessed using APACHE II, Ranson's criteria, and modified Marshall score. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Treatment options include aggressive IV fluid resuscitation (lactated Ringer's), pain control, early enteral nutrition, and monitoring for complications
- Infected pancreatic tissue death may require using a thin camera tube, through the skin, or surgical necrosectomy. Severe acute pancreatitis is marked by organ failure (persistent for >48 hours) and/or local complications (tissue death, pseudocyst, abscess).
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for two of three criteria: characteristic abdominal pain, serum amylase or lipase >3 times the upper limit of normal, and characteristic findings on imaging. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is favorable
- Most cases (80%) are mild and self-limiting.

Chapter 24

Acute Respiratory Distress Syndrome

Overview

Acute respiratory distress syndrome (ARDS) is a life-threatening form of acute lung injury characterized by diffuse alveolar damage, increased pulmonary vascular permeability, and severe hypoxemia. It typically develops within 6–72 hours of a direct or indirect lung insult and requires mechanical ventilation with positive end-expiratory pressure. ARDS carries a high mortality rate of 30–40% and is a major cause of respiratory failure in critically ill patients worldwide.

Classification

Cause Direct or indirect lung injury triggering systemic inflammation

Organ System Lungs/Respiratory

Pathophysiology Diffuse alveolar damage from inflammatory cascade leading to increased capillary permeability, surfactant dysfunction, alveolar flooding, and hyaline membrane formation

Duration Acute (days to weeks)

Onset Sudden (within 6–72 hours of inciting event)

Mode of Transmission Non-communicable

Clinical Course Rapid onset of respiratory failure requiring intensive care

Severity Severe (mild/moderate/severe based on PaO₂/FiO₂ ratio)

Extent of Disease Diffuse (bilateral lung involvement)

Age Group All ages

Epidemiology Incidence approximately 10% of ICU admissions; mortality 30–40%

ICD-11 Code CB00.0

Causes

Direct lung injuries causing ARDS include pneumonia (bacterial, viral, or fungal), aspiration of gastric contents, pulmonary contusion, near-drowning, and inhalation of toxic gases or smoke. Indirect injuries include sepsis (most common non-pulmonary cause), severe trauma with multiple transfusions, acute pancreatitis, burns, and drug overdoses. The common pathway involves activation of the innate immune system with neutrophil infiltration, cytokine release, and disruption of the alveolar-capillary barrier.

Risk Factors

- Sepsis (especially gram-negative bacteremia)
- Severe pneumonia (*Streptococcus pneumoniae*, influenza, SARS-CoV-2)
- Aspiration of gastric contents
- Major trauma or burns
- Acute pancreatitis
- Massive blood transfusion (transfusion-related acute lung injury, TRALI)
- Drug overdose (opioids, salicylates, tricyclic antidepressants)
- Near-drowning or smoke inhalation
- Severe acute pancreatitis

Signs and Symptoms

Early symptoms

- Early manifestations of acute respiratory distress syndrome may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Severe shortness of breath (dyspnea) with rapid onset
- Tachypnea and labored breathing
- Hypoxemia refractory to supplemental oxygen
- Rapid, shallow respirations
- Confusion or altered mental status from cerebral hypoxia
- Cyanosis (bluish discoloration of skin and mucous membranes)
- Crackles (rales) on lung auscultation
- Profound fatigue and respiratory muscle weakness
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of acute respiratory distress syndrome may occur

Emergency warning signs

- Severe or worsening symptoms of acute respiratory distress syndrome require immediate medical evaluation

Progression and Stages

ARDS progresses through three overlapping phases: the exudative phase (days 1–7) characterized by alveolar flooding with protein-rich fluid, hyaline membrane formation, and neutrophilic inflammation; the proliferative phase (days 7–21) with fibroblast proliferation and organization of exudates; and the fibrotic phase (after day 21) with collagen deposition and potential for irreversible pulmonary fibrosis. Severity is graded by the Berlin definition based on the PaO₂/FiO₂ ratio with PEEP $\geq$ 5 cmH₂O: mild (200–300 mmHg), moderate (100–200 mmHg), and severe (<100 mmHg).

Complications

- Ventilator-associated pneumonia
- Barotrauma and volutrauma (pneumothorax, pneumomediastinum)
- Multi-organ failure (renal, hepatic, cardiovascular)
- Pulmonary fibrosis in prolonged or severe cases
- Critical illness myopathy and polyneuropathy
- Post-intensive care syndrome (cognitive, psychiatric, and physical impairments)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis by Berlin criteria: acute onset (<1 week), bilateral opacities on chest imaging, respiratory failure not fully explained by cardiac failure or fluid overload
- Chest radiograph or CT scan showing bilateral diffuse infiltrates
- Arterial blood gas analysis measuring PaO₂/FiO₂ ratio for severity classification
- Echocardiogram to exclude cardiogenic pulmonary edema
- Bronchoalveolar lavage when infection or hemorrhage is suspected
- Lab tests including complete blood count, inflammatory markers, and cultures
- Imaging tests to monitor progression and complications

Prevention

- Early recognition and treatment of sepsis with appropriate antibiotics
- Lung-protective ventilation strategies in intubated patients
- Judicious fluid management and avoidance of fluid overload
- Early mobilization and oral care to reduce ventilator-associated complications
- Vaccination against respiratory pathogens (influenza, pneumococcus, SARS-CoV-2)
- Go for regular check-ups
- Follow your doctor's screening recommendations

Treatment Options

- Lung-protective mechanical ventilation with low tidal volumes (6 mL/kg ideal body weight)
- Positive end-expiratory pressure (PEEP) to maintain alveolar recruitment
- Prone positioning for severe ARDS to improve ventilation-perfusion matching
- Neuromuscular blockade (cisatracurium) in early severe ARDS
- Fluid-conservative strategy after initial resuscitation
- Corticosteroids (dexamethasone or methylprednisolone) in moderate-to-severe ARDS
- Extracorporeal membrane oxygenation (ECMO) for refractory hypoxemia
- Inhaled pulmonary vasodilators (nitric oxide, prostacyclin) as rescue therapy
- Treat underlying cause (antibiotics for infection, source control for sepsis)
- Lifestyle changes and self-care
- Surgery when needed

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups
- Keep learning about your condition

Outlook

Despite advances in critical care, ARDS carries a mortality of 30–40% and survivors often face long-term physical, cognitive, and psychiatric sequelae. Among those who survive, pulmonary function typically improves over 6–12 months, though residual restrictive or diffusion defects and impaired quality of life may persist.

Chapter 25

Acute Stress Disorder

Overview

Acute stress disorder has a variable prevalence (5–20% depending on trauma type). The condition describes the development of anxiety, dissociative, and other symptoms within 1 month of a traumatic event, with required symptom clusters similar to PTSD (intrusion, negative mood, dissociation, avoidance, and arousal) but with an emphasis on dissociative symptoms. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features last for 3–30 days after trauma
- If symptoms persist beyond 1 month, the diagnosis transitions to PTSD.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of acute stress disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include trauma-focused CBT initiated within the first month. Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics)

Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Approximately 50–80% of patients with ASD develop PTSD within 6 months.

Chapter 26

Addison's Disease

Overview

Addison's disease (primary adrenal insufficiency) is destruction or dysfunction of the adrenal cortex, leading to deficiency of cortisol, aldosterone, and adrenal androgens.

Causes

This condition results from disruption of normal addison's disease function.

- The most common cause in developed countries is autoimmune adrenalitis (often associated with other autoimmune diseases in APS type 2), while tuberculosis remains the most common cause worldwide
- Other causes include infections (CMV in HIV/AIDS, fungal), adrenal bleeding (Waterhouse-Friderichsen syndrome—meningococcal sepsis), metastatic disease, and drugs (ketoconazole, etomidate).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fatigue, weight loss, anorexia, nausea, vomiting, abdominal pain, hyperpigmentation (from elevated ACTH and MSH stimulating melanocytes, a hallmark of primary adrenal insufficiency), salt craving, hypotension, and orthostatic dizziness. Diagnosis shows low morning cortisol, elevated ACTH (primary), and failure to suppress cortisol with high-dose dexamethasone.

Emergency warning signs

- Severe or worsening symptoms of Addison's disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves glucocorticoid replacement (hydrocortisone 15–25 mg/day in divided doses) and mineralocorticoid replacement (fludrocortisone 0.05–0.2 mg/day).

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with appropriate replacement therapy and education about stress dosing. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 27

Adhesive Capsulitis

Overview

Adhesive capsulitis (frozen shoulder) is a chronic condition characterized by progressive pain and global restriction of passive and active range of motion of the glenohumeral joint, affecting 2–5% of the population.

Causes

This condition happens because of synovial inflammation followed by capsular thickening, pericapsular contracture, and dense fibrotic adhesions leading to severe loss of joint volume.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of adhesive capsulitis require immediate medical evaluation

Progression and Stages

Clinical progression follows three overlapping phases: freezing (painful, 2–9 months), frozen (stiffness predominates, 4–12 months), and thawing (gradual motion recovery,

5–24 months). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam based on equal restriction of active and passive external rotation
- Radiographs rule out glenohumeral osteoarthritis.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on physical therapy, intra-articular corticosteroid injections, oral NSAIDs, and manipulation under anesthesia or capsular release for recalcitrant cases. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 28

Adjustment Disorder

Overview

Adjustment disorder is one of the most commonly diagnosed psychiatric conditions in medical settings, with prevalence of 5–20% in outpatient mental health clinics and up to 12% in hospitalized medical patients.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Once the stressor is removed, symptoms typically resolve within 6 months.

Emergency warning signs

- Severe or worsening symptoms of adjustment disorder require immediate medical evaluation

Progression and Stages

You may experience the development of emotional or behavioral symptoms in response to an identifiable stressor within 3 months of onset, with marked distress that is out of proportion to the severity of the stressor, or significant functional impairment, not meeting criteria for another mental disorder. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function.

Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves primarily supportive psychotherapy, crisis intervention, and stress management. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well, with most cases resolving with removal of the stressor. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 29

Adrenal Incidentaloma

Overview

Adrenal incidentalomas are adrenal masses discovered incidentally on imaging performed for unrelated reasons, found in approximately 4% of CT scans. Most are non-cancerous, non-functioning cortical adenomas, but they may represent functioning tumors (cortisol-producing adenoma causing subclinical Cushing's syndrome, aldosterone-producing adenoma, pheochromocytoma) or rarely, adrenocortical carcinoma or metastases. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of incidental detection of adrenal masses. Evaluation requires hormonal workup (cortisol after 1 mg dexamethasone suppression test, plasma metanephrines, aldosterone/renin ratio) and imaging characteristics (size, density, enhancement pattern). Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of adrenal incidentaloma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include surgery for suspicious masses (>4 cm, HU >10, irregular borders, rapid growth), functioning tumors, and suspected malignancy, with surveillance for indeterminate masses. Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well for non-cancerous non-functioning adenomas.

Chapter 30

Adult-Onset Still's Disease

Overview

Adult-onset Still's disease is a rare systemic inflammatory disorder of unknown cause, considered the adult counterpart of systemic juvenile idiopathic arthritis (sJIA), characterized by a classic triad of high-spiking fevers, evanescent salmon-pink rash, and joint pain/arthritis, with an incidence of 0.16–0.4 per 100,000, a bimodal age distribution (15–25 and 36–46 years), and a slight female predominance.

Causes

This condition happens because of dysregulated innate immunity with activation of the inflammasome, leading to overproduction of IL-1, IL-6, IL-18, and other pro-inflammatory cytokines, with macrophage activation being a key feature.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience quotidian or double-quotidian fevers, an evanescent, salmon-pink, non-pruritic maculopapular rash (Koebner phenomenon), joint pain and arthritis (initially pauciarticular, may progress to severe destructive polyarthritis, particularly involving wrists and carpal bones—carpal ankylosis is characteristic), sore throat (intense, exudative, a highly distinctive feature), lymphadenopathy and splenomegaly, serositis, and muscle pain.

Emergency warning signs

- Severe or worsening symptoms of adult-onset still's disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using Yamaguchi criteria, with serum ferritin characteristically markedly elevated (often >5–10 times normal, >1000 ng/mL), with decreased glycosylated ferritin (<20%). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves stratified: NSAIDs for mild disease, corticosteroids for moderate disease, and IL-1 inhibitors (anakinra, canakinumab) or IL-6 inhibitors (tocilizumab) for refractory or severe disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Some patients achieve remission, while others have a chronic relapsing course.

Chapter 31

Advanced Sleep-Wake Phase Disorder

Overview

It is less common than DSWPD, affecting approximately 1% of the population, predominantly older adults (though familial cases with PER2 or PER3 gene mutations exist). This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience early evening sleepiness and early morning awakening.

Emergency warning signs

- Severe or worsening symptoms of advanced sleep-wake phase disorder require immediate medical evaluation

Progression and Stages

Advanced sleep-wake phase disorder (ASWPD) is a circadian rhythm sleep-wake disorder characterized by sleep-wake timing that is several hours earlier than desired, with

sleep onset between 6:00–9:00 PM and spontaneous awakening between 2:00–5:00 AM. This condition happens because of a short intrinsic circadian period (<24 hours) or an exaggerated phase-advance response to morning light. Treatment focuses on bright light therapy in the evening to phase-delay the circadian clock and low-dose melatonin upon early morning awakening. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using sleep diaries and actigraphy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment compliance. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 32

African Trypanosomiasis (Sleeping Sickness)

Overview

African trypanosomiasis (sleeping sickness) is a protozoan infection caused by *Trypanosoma brucei gambiense* (West African, 98% of cases, chronic) and *T. b. rhodesiense* (East African, acute), transmitted by the tsetse fly, with fewer than 2000 cases annually, endemic in sub-Saharan Africa. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of african trypanosomiasis (sleeping sickness) require immediate medical evaluation

Progression and Stages

You may experience hemolymphatic stage (Stage 1): chancre at the tsetse fly bite site, intermittent fever, headache, joint pain, generalized lymphadenopathy (Winterbottom's sign), and itching. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Meningoencephalitic stage (Stage 2): progressive confusion, personality changes, daytime somnolence, nighttime insomnia, headache, tremor, loss of coordination, and finally coma and death. The right treatment depends on staging: Stage 1 doctors typically treat it with pentamidine or suramin
- Stage 2 doctors typically treat it with nifurtimox-eflornithine combination therapy (NECT) or melarsoprol. The outlook is good with early treatment
- Stage 2 disease has higher mortality.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using microscopic identification of motile trypomastigotes in blood, lymph node aspirate, or CSF. Comprehensive medical history and physical examination. Laboratory tests to assess organ function and rule out other conditions. Imaging studies to visualize affected structures. Specialized testing depending on the affected system. Consultation with relevant specialists. Monitoring of disease progression over time.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 33

Age-Related Bone Loss

Overview

- It affects 10–20% of women over 50 and 4–5% of men
- Prevalence increases steeply with age.

Causes

This condition happens because of an imbalance between bone resorption and bone formation, driven by estrogen deficiency, secondary hyperparathyroidism, decreased mechanical loading, and reduced anabolic hormone levels.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of age-related bone loss require immediate medical evaluation

Progression and Stages

Osteoporosis is a systemic skeletal disorder characterized by low bone mass and microarchitectural deterioration of bone tissue, leading to increased fracture risk. The condition

may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using dual-energy X-ray absorptiometry (DXA): a T-score of ≤ -2.5 defines osteoporosis
- -1.0 to -2.5 defines osteopenia. The FRAX tool estimates 10-year fracture probability.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include calcium and vitamin D supplementation, weight-bearing exercise, fall prevention, bisphosphonates (first-line), denosumab, teriparatide, and romosozumab.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 34

Age-Related Cataract

Overview

Age-related cataract is an opacification of the crystalline lens and the leading cause of preventable blindness worldwide, resulting from cumulative oxidative damage to lens proteins. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age.

Causes

This condition happens because of aggregation of damaged lens proteins that scatter light. Degenerative processes involve progressive cellular dysfunction and tissue breakdown over time. Contributing factors include oxidative stress, inflammation, accumulated cellular damage, and reduced regenerative capacity.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience painless progressive blurring of vision, glare sensitivity, decreased color perception, and dimming of vision.

Emergency warning signs

- Severe or worsening symptoms of age-related cataract require immediate medical evaluation

Progression and Stages

Classification by location includes nuclear (yellowing and hardening of the lens nucleus), cortical (peripheral wedge-shaped opacities), and posterior subcapsular (opacity near the posterior capsule, often associated with steroid use). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using slit-lamp examination. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves surgical: phacoemulsification with implantation of an intraocular lens. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and

support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well, with cataract surgery being one of the most commonly performed and cost-effective surgical procedures globally. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 35

Age-Related Macular Degeneration

Overview

This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age.

Causes

This condition happens because of age-related degenerative changes in the retinal pigment epithelium and photoreceptors. Degenerative processes involve progressive cellular dysfunction and tissue breakdown over time. Contributing factors include oxidative stress, inflammation, accumulated cellular damage, and reduced regenerative capacity.

Risk Factors

- Advancing age
- Genetic predisposition
- Repetitive use or overuse (joints)
- Previous injuries
- Obesity
- Smoking and alcohol use
- Genetic predisposition and family history
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive central vision loss and distortion.

Emergency warning signs

- Severe or worsening symptoms of age-related macular degeneration require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Age-related macular deterioration is the leading cause of irreversible central vision loss in individuals over 50 in developed countries, affecting the macula. Early AMD is marked by drusen and pigmentary changes
- Advanced AMD manifests as either dry (atrophic) AMD with geographic shrinkage or wasting or wet (neovascular) AMD with choroidal neovascularization.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on fundus examination and optical coherence tomography. Treatment for wet AMD involves anti-VEGF intravitreal injections (ranibizumab, aflibercept, bevacizumab)
- Complement inhibitors are under investigation for dry AMD.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has improved significantly with anti-VEGF therapy for wet AMD. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 36

Age-Related Macular Degeneration

Overview

- Wet (exudative/neovascular) AMD is less common but more aggressive, driven by choroidal neovascularization. Treatment for dry AMD includes the AREDS2 formulation (vitamins C and E, lutein/zeaxanthin, zinc) for intermediate disease
- Wet AMD doctors typically treat it with intravitreal anti-VEGF injections.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of age-related macular degeneration require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Age-related macular deterioration (AMD) is the leading cause of irreversible central vision loss in adults over 65. Dry (nonexudative) AMD accounts for 85–90% of cases, characterized by drusen, retinal pigment epithelium shrinkage or wasting, and photoreceptor deterioration. outlook is guarded
- Low-vision rehabilitation is essential for all stages.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

In older adults, AMD severely impacts quality of life—loss of reading, face recognition, and driving independence—and is linked to depression and falls.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 37

Agoraphobia

Overview

Agoraphobia has a lifetime prevalence of 1–3%, with a mean age of onset in the late 20s. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of conditioned avoidance behaviors and interoceptive hypersensitivity, with Disease mechanism overlapping with panic disorder. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience marked fear or anxiety about being in situations from which escape might be difficult or help might not be available in the event of panic-like

symptoms, with typical feared situations including using public transportation, being in open spaces, being in enclosed spaces, standing in line or being in a crowd, or being outside of the home alone.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of agoraphobia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include CBT with in vivo exposure and SSRIs/SNRIs. Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with treatment. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 38

Alcohol Intoxication and Withdrawal

Overview

Alcohol (ethanol) is the most widely used psychoactive substance. Intoxication produces dose-dependent CNS depression: disinhibition, loss of coordination, slurred speech, involuntary eye movements, impaired judgment, and at higher levels, stupor, coma, and respiratory depression. Hypoglycemia (from inhibition of gluconeogenesis) is a critical consideration. Alcohol withdrawal syndrome occurs after reduction or cessation of chronic heavy alcohol use, typically starting 6–12 hours after last drink.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms can range from mild withdrawal (tremor, anxiety, tachycardia, high blood pressure) to delirium tremens (profound confusion, agitation, hallucinations, autonomic hyperactivity), with withdrawal seizures (generalized tonic-clonic) occurring 12–48 hours after cessation. Treatment: benzodiazepines are first-line, administered using a symptom-triggered protocol (CIWA-Ar scale)
- Severe DTs may require ICU admission.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of alcohol intoxication and withdrawal require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment for intoxication is supportive: monitoring, IV fluids, dextrose (if hypoglycemic), thiamine (100 mg IV) to prevent Wernicke encephalopathy, and consideration of co-ingestants.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 39

Alcohol Use Disorder

Overview

Alcohol use disorder is one of the most prevalent psychiatric disorders, with a 12-month prevalence of 5–8% and lifetime prevalence of 20–30% in the United States, more common in men but increasing in women.

Causes

This condition happens because of dysregulation of the brain reward system (mesolimbic dopamine pathway), with chronic alcohol use producing tolerance, withdrawal (GABA receptor downregulation and NMDA receptor upregulation), and sensitization of stress systems, with genetic factors contributing 50–60% of risk.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience at least 2 of 11 DSM-5-TR criteria occurring within a 12-month period, including alcohol taken in larger amounts or over longer periods than intended, persistent desire to cut down, craving, recurrent use resulting in failure to fulfill obligations, continued use despite problems, tolerance, and withdrawal.

Emergency warning signs

- Severe or worsening symptoms of alcohol use disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Medical complications include alcoholic hepatitis, cirrhosis, pancreatitis, Wernicke encephalopathy (confusion, loss of coordination, ophthalmoplegia—treated with IV thiamine), Korsakoff syndrome, alcoholic cardiomyopathy, peripheral neuropathy, and increased cancer risk.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment options include detoxification (benzodiazepines for withdrawal prevention), pharmacotherapy for relapse prevention (naltrexone, acamprosate, disulfiram), and psychosocial interventions (CBT, MET, 12-step facilitation). outlook: chronic relapsing condition
- 30–50% achieve sustained sobriety with treatment.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 40

Alcoholic Liver Disease

Overview

Alcoholic liver disease is a spectrum of liver injury due to excessive alcohol consumption that includes simple steatosis, alcoholic hepatitis, and cirrhosis. Simple steatosis is present in over 90% of heavy drinkers.

Causes

This condition results from disruption of normal alcoholic liver disease function.

- This condition happens because of direct hepatotoxicity of alcohol and its metabolites. Alcoholic hepatitis presents with jaundice, fever, hepatomegaly, and leukocytosis, carrying significant mortality (up to 40% in severe cases)
- Alcoholic cirrhosis develops in 10–20% of heavy drinkers.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of alcoholic liver disease require immediate medical evaluation

Progression and Stages

Your symptoms depend on the stage of disease. Management centers on complete alcohol abstinence, nutritional support, corticosteroids for severe alcoholic hepatitis, and liver transplantation for end-stage disease. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on history of significant alcohol intake (typically more than 50 g/day in men, more than 40 g/day in women for more than 5 years), clinical and laboratory findings, and exclusion of other causes. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with abstinence but poor with continued drinking. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 41

Alkaptonuria — Ochronosis

Overview

Alkaptonuria is a rare autosomal recessive disorder caused by deficiency of homogentisate 1,2-dioxygenase (HGD), the enzyme responsible for the breakdown of homogentisic acid (HGA) in the tyrosine degradation pathway. The incidence is approximately 1 in 250,000–1,000,000.

Causes

This condition happens because of accumulation of HGA which polymerizes to form a dark brown-black pigment that deposits in connective tissues throughout the body.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience homogentisic aciduria (urine turns dark brown/black upon exposure to air), ochronosis (bluish-black pigmentation of ears, nose, sclera, and skin), ochronotic arthropathy (progressive degenerative joint disease affecting the spine and large joints), and cardiovascular involvement including aortic valve stenosis and coronary artery calcium buildup.

Emergency warning signs

- Severe or worsening symptoms of alkaptonuria — ochronosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Treatment typically involves symptomatic: nitisinone reduces HGA production and slows disease progression but does not reverse existing ochronosis
- Pain management, physical therapy, and joint replacement surgery for advanced arthropathy.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using elevated urinary homogentisic acid and HGD gene sequencing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- The disease is progressive.

Chapter 42

Allergic Fungal Rhinosinusitis (AFRS)

Overview

Allergic fungal rhinosinusitis (AFRS) is a non-invasive, IgE-mediated hypersensitivity reaction to dematiaceous fungal elements (*Aspergillus*, *Bipolaris*, *Curvularia*) inhabiting the paranasal sinuses, accounting for 5–10% of chronic rhinosinusitis cases in immunocompetent young adults. Fungal spores elicit an intense type I and type IV inflammatory response characterized by eosinophilic recruitment, thick 'peanut-butter-like' allergic mucin production, and nasal polyposis. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience chronic nasal obstruction, loss of smell, facial pressure, and expanded sinus osteitis causing orbital or anterior cranial fossa erosion. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of allergic fungal rhinosinusitis (afrs) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Diagnosis relies on the Bent-Kuhn criteria: nasal polyposis, allergic mucin with Charcot-Leyden crystals and fungal hyphae on microscopy, characteristic hyperdense sinus opacification with central attenuation on CT, and positive anti-fungal IgE. Management requires functional using a thin camera tube sinus surgery (FESS) to evacuate allergic mucin followed by prolonged topical and systemic corticosteroid therapy. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or

antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 43

Allergic Rhinitis

Overview

Allergic rhinitis is an IgE-mediated inflammatory condition of the nasal mucosa that affects 10–30% of the global population.

Causes

This condition happens because of exposure to airborne allergens (pollens, dust mites, animal dander, molds) triggering IgE-mediated mast cell degranulation. Seasonal allergic rhinitis (hay fever) correlates with pollen seasons, while perennial allergic rhinitis is year-round.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sneezing, nasal congestion, rhinorrhea (clear, watery discharge), nasal itching, and nasal obstruction
- Ocular symptoms (itching, tearing, conjunctival injection) are common.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of allergic rhinitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by skin prick testing or specific serum IgE levels. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include allergen avoidance, intranasal corticosteroids (first-line for moderate-to-severe disease), second-generation oral antihistamines, leukotriene receptor antagonists, and allergen immunotherapy for refractory or severe cases. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated

Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate treatment, though the condition is chronic. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 44

Alopecia Areata

Overview

Alopecia areata is an autoimmune dermatological disorder characterized by non-scarring patchy hair loss affecting hair-bearing areas of the body, most commonly the scalp and beard area.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sudden onset of smooth, round or oval, circumscribed patches of hair loss without redness of the skin or scaling, featuring pathognomonic 'exclamation mark' hairs (short broken hairs tapering near the scalp surface) at active patch margins. Severe variants include alopecia totalis (complete loss of scalp hair) and alopecia universalis (complete loss of all scalp and body hair). Nail changes (pitting, trachyonychia) occur in 10–20% of patients.

Emergency warning signs

- Severe or worsening symptoms of alopecia areata require immediate medical evaluation

Progression and Stages

This condition happens because of CD8+ T cell-mediated autoimmune attack directed against hair follicles in the anagen (growth) phase, with collapse of hair follicle immune privilege. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam based on characteristic morphology and dermoscopy (trichoscopy). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include topical or intralesional corticosteroid injections (triamcinolone acetonide) for localized disease, topical minoxidil, immunotherapy, and oral Janus kinase (JAK) inhibitors (baricitinib, ritlecitinib) for severe diffuse disease. Medications to

manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Spontaneous regrowth occurs in localized disease, but relapse is common.

Chapter 45

Alpha-1 Antitrypsin Deficiency

Overview

Alpha-1 antitrypsin (AAT) deficiency is an autosomal codominant disorder caused by mutations in the SERPINA1 gene, most commonly the Pi*Z allele, leading to reduced serum levels and accumulation of misfolded AAT in the liver. The incidence is approximately 1 in 2,500–5,000 live births in Northern European populations. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of accumulation of polymerized AAT in hepatocytes causing liver disease and lack of functional AAT in the lungs permitting unchecked proteolytic destruction of alveolar walls. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Early symptoms

- Early-onset panacinar emphysema particularly in smokers (lung disease)

Common symptoms

- You may experience neonatal cholestasis, hepatitis, cirrhosis, and hepatocellular carcinoma (liver disease)
- And widespread panniculitis.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of alpha-1 antitrypsin deficiency require immediate medical evaluation

Progression and Stages

Treatment options include intravenous augmentation therapy (weekly infusions of pooled human plasma-derived AAT) to slow lung function decline. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Smoking cessation is critical
- Liver transplantation is curative for end-stage liver disease.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low serum AAT level, phenotyping (Pi*ZZ, Pi*SZ), and genotyping.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Avoidance of hepatotoxins is essential. Disorders of lipid metabolism encompass both primary (genetic) and secondary dyslipidemias that contribute substantially to the global burden of atherosclerotic cardiovascular disease. Genetic dyslipidemias result from mutations affecting lipoprotein production, processing, or clearance, while secondary causes include diabetes, hypothyroidism, nephrotic syndrome, cholestasis, and medications. Early identification and aggressive lipid-lowering therapy can substantially modify cardiovascular risk.

Chapter 46

Altitude Sickness

Overview

Altitude illness occurs at elevations above 2,500 m (8,000 ft) due to decreased barometric pressure and hypoxia.

Causes

This condition results from disruption of normal altitude sickness function.

- This condition happens because of compensatory hyperventilation, increased sympathetic tone, and lung and cerebral vasoconstriction and vasodilation
- Inadequate acclimatization leads to three syndromes. Acute mountain sickness (AMS) is the most common form: headache, nausea, dizziness, anorexia, fatigue, and sleep disturbance.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of altitude sickness require immediate medical evaluation

Progression and Stages

High-altitude cerebral swelling (HACE) is a life-threatening progression with loss of coordination, altered mental status, hallucinations, seizures, and coma. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors use the Lake Louise Score. Treatment: immediate descent, supplemental oxygen, dexamethasone. High-altitude lung swelling (HAPE) is non-cardiogenic lung swelling with cough, shortness of breath at rest, cyanosis, and crackles. Treatment: immediate descent, supplemental oxygen, nifedipine or phosphodiesterase-5 inhibitors. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment: acetazolamide for prophylaxis and mild treatment
- Descent for persistent symptoms.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with prompt descent. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 47

Alzheimer's Disease

Overview

Alzheimer's disease is a progressive neurodegenerative disorder and the most common cause of dementia, accounting for 60–70% of cases. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of the accumulation of amyloid-beta ($A\beta$) plaques between neurons and neurofibrillary tangles composed of hyperphosphorylated tau protein within neurons. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Advanced age
- Family history and genetic predisposition
- Head trauma or brain injury
- Vascular risk factors (hypertension, diabetes)
- Exposure to environmental toxins
- Chronic stress
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience insidious memory impairment, particularly of recent events, progressing to global thinking and memory decline, disorientation, language difficulties, and eventual loss of independence. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of alzheimer's disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical assessment, neuropsychological testing, neuroimaging showing hippocampal shrinkage or wasting and reduced glucose metabolism on PET, and cerebrospinal fluid biomarkers (A β 42, tau, phospho-tau). Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures

- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on symptomatic treatment with cholinesterase inhibitors (donepezil, rivastigmine, galantamine) and memantine
- Disease-modifying therapies targeting amyloid remain under investigation.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is progressive decline with modest benefit from available therapies. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 48

Ambiguous Genitalia / Disorders of Sex Development

Overview

Disorders of sex development (DSD) are congenital conditions in which the development of chromosomal, gonadal, or anatomic sex is atypical. Incidence is approximately 1 in 4500 live births.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience atypical genitalia at birth. Evaluation includes karyotype, hormone levels, imaging, and genitourinary anatomy assessment. Management requires a multidisciplinary team
- Pain or discomfort in the affected area

Emergency warning signs

- Salt-wasting congenital adrenal abnormal increase in cells is a life-threatening emergency requiring glucocorticoid and mineralocorticoid replacement.

Progression and Stages

DSD is classified by karyotype: 46,XX DSD (most commonly congenital adrenal abnormal increase in cells due to 21-hydroxylase deficiency), 46,XY DSD (androgen insensitivity syndrome, 5-alpha-reductase deficiency, gonadal dysgenesis), and sex chromosome DSD (Turner syndrome, Klinefelter syndrome, mixed gonadal dysgenesis). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the specific diagnosis. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 49

Amblyopia

Overview

- Amblyopia is reduced best-corrected visual acuity in one or both eyes resulting from abnormal visual development during the critical period of visual maturation (birth to approximately 8 years)
- It is the most common cause of monocular vision loss in children.

Causes

This condition happens because of strabismus, anisometropia, form deprivation (congenital cataract, ptosis), or bilateral high refractive error, leading to failure of normal neural pathway development in the amblyopic eye.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience reduced visual acuity that cannot be corrected with lenses.

Emergency warning signs

- Severe or worsening symptoms of amblyopia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on visual acuity testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include corrective lenses, patching or penalization (atropine drops) of the dominant eye, and treatment of the underlying cause. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with early intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 50

Amebiasis (*Entamoeba histolytica*)

Overview

Amebiasis is an intestinal and extraintestinal infection caused by *Entamoeba histolytica*, a protozoan parasite, with 50 million symptomatic cases and 100,000 deaths annually worldwide, particularly in the Indian subcontinent, sub-Saharan Africa, and Central/South America.

Causes

This condition happens because of cysts excysting in the terminal ileum, releasing trophozoites that colonize the large intestine, with *E. histolytica* trophozoites adhering to colonic mucin, secreting cysteine proteinases that degrade extracellular matrix, inducing host cell apoptosis, and invading the colonic epithelium.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience intestinal amebiasis: amoebic dysentery—bloody mucoid diarrhea (strawberry jelly stools), abdominal pain, tenesmus, and weight loss
- Extraintestinal amebiasis: amoebic liver abscess—most common extraintestinal form with fever, right upper quadrant pain, and “anchovy paste” pus.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of amebiasis (*entamoeba histolytica*) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using fresh stool microscopy (trophozoites with ingested RBCs), antigen detection, PCR, and serology. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include tinidazole or metronidazole followed by a luminal agent (paromomycin) to eradicate cysts. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 51

Amenorrhea (Primary, Secondary)

Overview

Amenorrhea is the absence of menstruation. Primary amenorrhea is the absence of menarche by age 15 with normal secondary sexual development, or by age 13 with no secondary sexual characteristics.

Causes

Causes include hypothalamic/pituitary disorders, gonadal disorders (Turner syndrome, Swyer syndrome, premature ovarian insufficiency), and outflow tract obstruction (imperforate hymen, Müllerian agenesis). Secondary amenorrhea is the absence of menstruation for three months or more in a woman with previously normal cycles

- The most common cause is pregnancy, with other causes including functional hypothalamic amenorrhea, PCOS, hyperprolactinemia, premature ovarian insufficiency, and Asherman syndrome. Diagnosis includes β -hCG, prolactin, TSH, FSH, and estrogen levels
- Progestin challenge test and MRI for pituitary lesions or outflow tract anomalies.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of amenorrhea (primary, secondary) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

The right treatment depends on the underlying cause. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical

therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 52

Amyloidosis

Overview

- Amyloidosis is a group of diseases characterized by the extracellular deposition of abnormal, insoluble fibrillar proteins (amyloid) in various tissues and organs, disrupting normal structure and function, with three main types clinically important: AL amyloidosis (immunoglobulin light chain—the most common systemic form, associated with plasma cell dyscrasia/multiple myeloma), AA amyloidosis (secondary/reactive serum amyloid A protein derived from chronic inflammation), and ATTR amyloidosis (transthyretin hereditary or wild-type/senile systemic amyloidosis). The incidence of AL amyloidosis is 3–5 per million, typically presenting at age 60–70.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Your symptoms depend on the organs involved: AL amyloidosis commonly presents with nephrotic syndrome (kidney amyloidosis—most common presentation, 70%), restrictive cardiomyopathy (heart failure with preserved ejection fraction, low-voltage QRS on ECG, thickened ventricular walls on echocardiography with a “speckled” appearance), hepatomegaly, peripheral and autonomic neuropathy, macroglossia (tongue enlargement—pathognomonic for AL), periorbital purple spots on the skin, and carpal tunnel syndrome.

Emergency warning signs

- Severe or worsening symptoms of amyloidosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Treatment for AL amyloidosis is anti-plasma cell therapy (similar to multiple myeloma: bortezomib-based regimens, autologous stem cell transplantation in selected patients)
- AA amyloidosis is treated by controlling the underlying inflammatory condition
- ATTR amyloidosis doctors typically treat it with tafamidis (stabilizes transthyretin, slows neuropathy and cardiomyopathy progression).

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for tissue biopsy with Congo red staining and apple-green birefringence under polarized light—the diagnostic gold standard. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Unfortunately, the outlook is often poor for AL amyloidosis without treatment but has improved significantly with modern therapies. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 53

Amyotrophic Lateral Sclerosis

Overview

About 10% of cases are familial, with mutations in SOD1, C9orf72, TARDBP, and FUS genes.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience focal muscle weakness, shrinkage or wasting, fasciculations, spasticity, and hyperreflexia
- Bulbar involvement causes slurred speech and difficulty swallowing.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of amyotrophic lateral sclerosis require immediate medical evaluation

Progression and Stages

Amyotrophic lateral sclerosis is a fatal motor neuron disease characterized by progressive deterioration of both upper and lower motor neurons in the motor cortex, brainstem, and spinal cord. This condition happens because of progressive motor neuron deterioration. Treatment focuses on riluzole and edaravone to modestly slow progression, non-invasive

ventilation, and multidisciplinary care. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis relies on the revised El Escorial criteria, supported by electromyography and exclusion of mimics. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Unfortunately, the outlook is often poor, with respiratory failure typically causing death within 3–5 years of onset. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 54

Anal Fissure

Overview

Anal fissure is a painful linear tear or ulceration in the distal anal canal lining extending from the dentate line to the anal verge.

Causes

This condition happens because of trauma caused by passage of hard, dry stool or severe diarrhea, precipitating internal anal sphincter spasm and reduced blood flow of the posterior anal midline (where 90% of fissures occur due to poor microvascular perfusion).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe, sharp, tearing pain during and after defecation, accompanied by bright red rectal bleeding on toilet paper or stool surface. Physical examination reveals a visible longitudinal fissure
- Chronic fissures feature a sentinel skin tag and hypertrophied anal papilla.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of anal fissure require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include stool softeners, high-fiber diet, warm sitz baths, and topical nitrates (0.2% nitroglycerin) or calcium channel blockers (2% diltiazem) to relax the sphincter
- Lateral internal sphincterotomy is reserved for chronic medically refractory fissures.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 55

Anaphylaxis

Overview

Anaphylaxis is a severe, life-threatening systemic hypersensitivity reaction (usually IgE-mediated) that occurs rapidly and can involve multiple organ systems.

Causes

This condition happens because of exposure to triggers including foods (nuts, shellfish, milk, eggs), insect stings (Hymenoptera), medications (penicillin, NSAIDs), latex, and contrast media.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience skin involvement (urticaria, angioedema, flushing in 80–90%), respiratory compromise (laryngeal swelling, bronchospasm), cardiovascular collapse (hypotension, distributive shock), digestive tract symptoms (nausea, vomiting, diarrhea), and neurological symptoms (anxiety, altered consciousness).

Emergency warning signs

- Severe or worsening symptoms of anaphylaxis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam: acute onset (minutes to hours) of skin/mucosal involvement plus respiratory compromise or hypotension after exposure to a likely allergen, with serum tryptase possibly elevated but not required for diagnosis. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves with intramuscular epinephrine (0.3–0.5 mg of 1:1000 in the mid-ant thigh, repeat every 5–15 minutes as needed), IV fluid resuscitation, H1 and H2 antihistamines, corticosteroids, and inhaled bronchodilators for bronchospasm. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications

and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with prompt epinephrine administration. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 56

Angelman Syndrome

Overview

Angelman syndrome is a neurogenetic disorder caused by loss of function of the maternal copy of the UBE3A gene on chromosome 15q11-q13. Mechanisms include maternal deletion (70%), paternal uniparental disomy (2–3%), imprinting defects (3–5%), and UBE3A mutations (10%). Incidence is 1 in 12,000–20,000. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe intellectual disability, loss of coordination, seizures, frequent laughter, happy demeanor, hand-flapping, and absent speech
- Microcephaly develops after age 2 years.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of angelman syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using methylation analysis of the 15q11-q13 region.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive, with antiepileptic drugs for seizures and physical/occupational therapy. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation

in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 57

Angina Pectoris

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Angina pectoris is chest discomfort caused by transient myocardial reduced blood flow without myocardial tissue death. Stable angina occurs predictably with exertion or emotional stress and is relieved by rest or nitroglycerin, reflecting fixed coronary atherosclerotic stenosis
- Unstable angina occurs unpredictably or at rest and represents acute coronary syndrome without troponin elevation
- Microvascular angina results from coronary microvascular dysfunction.

Causes

This condition happens because of an imbalance between myocardial oxygen supply and demand. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Treatment for stable angina includes antianginals (beta-blockers, calcium channel blockers, nitrates), risk factor modification, and revascularization when medical therapy is inadequate
- Unstable angina requires urgent antiplatelet therapy, anticoagulation, and early invasive strategy. Your outlook depends on the extent of coronary disease and risk

factor control.

- Advanced age
- Cigarette smoking
- Hypertension and diabetes
- Elevated cholesterol levels
- Family history of heart disease
- Obesity and physical inactivity
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience chest pressure or discomfort that may radiate to the left arm or jaw. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of angina pectoris require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis

causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Diagnosis typically involves ECG (may show ST depression during episodes), stress testing, and coronary angiography for definitive anatomic assessment. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques

- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 58

Ankylosing Spondylitis

Overview

Ankylosing spondylitis (AS) is a chronic inflammatory disease primarily affecting the axial skeleton (sacroiliac joints and spine), representing the prototypical spondyloarthropathy. It predominantly affects young men (20–40 years) and is strongly associated with HLA-B27 (present in 90% of patients). This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of immune-mediated inflammation of the entheses and axial joints. The hallmark is sacroiliitis presenting with inflammatory back pain (insidious onset, morning stiffness >30 minutes, improvement with activity, onset before age 40). In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking

- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- You may experience progressive spinal involvement leading to syndesmophyte formation, vertebral fusion ("bamboo spine"), and spinal rigidity
- Peripheral arthritis (hips, shoulders, knees), enthesitis (Achilles tendinitis, plantar fasciitis), anterior uveitis, and aortic regurgitation are associated features.
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of ankylosing spondylitis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use

- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Diagnosis typically involves clinical assessment, HLA-B27 testing, and imaging (MRI for early sacroiliitis, X-rays showing sacroiliac joint erosions and fusion, syndesmophytes). Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

- Treatment options include NSAIDs (first-line, used continuously), TNF inhibitors (adalimumab, etanercept, infliximab, certolizumab), and IL-17 inhibitors (secukinumab, ixekizumab) for inadequate response
- Physical therapy is essential.
- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways

- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

- The outlook varies from person to person
- Early treatment improves outcomes.

Chapter 59

Anophthalmia and Microphthalmia

Overview

- Anophthalmia is the complete absence of an eye, and microphthalmia is an abnormally small eye; both are congenital (present at birth)
- They are rare, affecting roughly 1 in 10,000 births, and may affect one or both eyes
- They are among the most severe congenital eye defects and require multidisciplinary care

Causes

These conditions happen because of disrupted development of the eye in early pregnancy, before the 8th week of gestation. Causes include genetic mutations, chromosomal abnormalities, infections during pregnancy (toxoplasmosis, rubella, cytomegalovirus), and exposure to toxins or certain medications. Many cases have no identifiable cause.

Risk Factors

- Family history of eye malformations
- Genetic syndromes (e.g., SOX2, OTX2 mutations; chromosomal anomalies)
- Maternal infections during early pregnancy (TORCH infections)
- Exposure to alcohol, retinoic acid, or certain medications in early pregnancy
- Advanced parental age (in some studies)

Signs and Symptoms

Common symptoms

- Small or absent eyeball, visible at birth
- Small, underdeveloped eyelids and eye socket (orbit)
- Absent or poorly functioning vision in the affected eye
- Shallow socket and small palpebral opening
- Associated facial, brain, or other systemic anomalies in syndromic cases

Emergency warning signs

- Any newborn with suspected eye malformation should be evaluated urgently to confirm the diagnosis and screen for associated anomalies

Progression and Stages

The condition is present at birth and is static in terms of the eyeball itself, but the orbit and facial bones fail to grow normally because the eye's growth stimulus is absent. Without treatment, the face becomes progressively asymmetric. Visual prognosis depends on whether the condition is bilateral and on associated brain or optic nerve abnormalities.

Complications

- Bilateral severe visual impairment or blindness
- Facial asymmetry and underdeveloped eye socket
- Prosis (drooping eyelid) and socket contraction
- Psychological and social impact on the child and family
- Associated systemic malformations (brain, heart, face) in syndromic forms

Diagnosis

- Clinical examination at birth
- Ultrasound, CT, or MRI of the eye and brain to assess the eyeball, optic nerve, and brain
- Genetic testing to identify an underlying cause
- Hearing assessment and evaluation for associated systemic anomalies
- Multidisciplinary assessment by an ophthalmologist, geneticist, and pediatrician

Prevention

- Prenatal ultrasound may detect severe cases before birth
- Genetic counseling for affected families
- Prevention of maternal infections (rubella vaccination before pregnancy)
- Avoidance of alcohol and harmful medications during pregnancy
- Maternal folic acid supplementation

Treatment Options

- Socket expansion with progressively larger conformers from infancy to promote orbital and facial growth
- Fitted prosthetic eye (ocular prosthesis) for cosmetic and developmental reasons
- Early referral for visual rehabilitation in bilateral cases

- Surgical socket reconstruction and eyelid procedures as the child grows
- Coordinated care for any associated systemic conditions
- Genetic counseling and support for the family

Living with It

- Begin socket expansion and prosthesis fitting early for best facial outcomes
- Work with a multidisciplinary team and early-intervention services
- Access visual rehabilitation and mobility training
- Seek peer support and counseling for the family
- Plan regular follow-up through childhood and adolescence

Outlook

The visual outcome depends on whether the condition is bilateral and on associated brain or optic nerve involvement. With early socket expansion and prosthetics, excellent cosmetic and facial-growth outcomes can be achieved. Children with bilateral involvement benefit greatly from early visual rehabilitation, and many lead full, independent lives.

Chapter 60

Anorexia Nervosa

Overview

Anorexia nervosa has a lifetime prevalence of 0.5–1% in women and 0.1–0.3% in men, with a 10:1 female-to-male ratio, and age of onset typically in adolescence (peak 15–19 years). This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of serotonergic and dopaminergic dysregulation, aberrant reward processing, and sociocultural factors (thin-ideal internalization). Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience restriction of energy intake relative to requirements leading to significantly low body weight, intense fear of gaining weight or becoming fat (even when underweight), and disturbance in self-perceived weight or shape, with subtypes including restricting type and binge-eating/purging type.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of anorexia nervosa require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Medical complications include bradycardia, hypotension, hypothermia, osteopenia/osteoporosis, lanugo, irregular heartbeats (prolonged QT interval), electrolyte disturbances, refeeding syndrome, and structural brain changes.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment options include medical stabilization, nutritional rehabilitation, psychotherapy (family-based treatment for adolescents, CBT-E for adults), and olanzapine for weight gain.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person: 50% recover, 30% improve, 20% remain chronic, with mortality of 5–10%. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 61

Anthrax

Overview

Anthrax is a zoonotic infection caused by *Bacillus anthracis*, a large, gram-positive, spore-forming, encapsulated rod, endemic in agricultural regions worldwide, acquired through contact with infected animals or animal products, and is a category A bioterrorism agent.

Causes

This condition happens because of spores entering through skin (cutaneous), inhalation (lung), or ingestion (digestive tract), germinating and producing three virulence factors: protective antigen (PA), lethal factor (LF), and swelling factor (EF), with LF being a metalloproteinase that cleaves MAPKKs, disrupting signaling and causing macrophage lysis and cytokine storm.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Early symptoms

- Inhalational anthrax: initial flu-like symptoms followed abruptly by respiratory failure, involving bleeding mediastinitis, pleural effusions, and shock

Common symptoms

- You may experience cutaneous anthrax (95% of natural cases): pruritic papule developing into a painless, ulcerated, black eschar surrounded by significant swelling (cancerous pustule)
- Digestive tract anthrax: abdominal pain, nausea, vomiting, bloody diarrhea, and

bowel perforation.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of anthrax require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using culture and Gram stain. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include anthrax antitoxin plus antibiotics (ciprofloxacin or doxycycline).
outlook: cutaneous < 1% mortality

- Inhalational 40–80%
- Digestive tract 25–60%.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 62

Anti-GBM Disease

Overview

Anti-glomerular basement membrane disease, also known as Goodpasture syndrome, is a rare, life-threatening small-vessel vasculitis characterized by autoantibodies directed against the non-collagenous domain of the $\alpha 3$ chain of type IV collagen ($\alpha 3(\text{IV})\text{NC1}$), found in the basement membranes of glomeruli and lung alveoli, with an incidence of 0.5–1 per million and a bimodal age distribution (young adults 20–30 years with male predominance, and older adults 60–70 years with female predominance). The condition involves anti-GBM antibodies binding to the target antigen in the GBM and alveolar basement membrane, activating complement and recruiting neutrophils, leading to crescentic glomerulonephritis and alveolar bleeding, with genetic susceptibility strongly associated with HLA-DRB1*1501.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience rapidly progressive glomerulonephritis (hematuria, proteinuria, oliguria, rising creatinine, crescentic GN on biopsy), with or without lung involvement (hemoptysis, shortness of breath, alveolar bleeding—seen in 60–80%, more common in smokers and young patients).

Emergency warning signs

- Severe or worsening symptoms of anti-gbm disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on detection of circulating anti-GBM antibodies by ELISA and kidney biopsy showing linear IgG deposition along the GBM on immunofluorescence (the pathognomonic finding). Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves urgent: high-dose corticosteroids combined with cyclophosphamide and plasmapheresis (daily for 7–14 days to remove pathogenic antibodies). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Your outlook depends on the degree of kidney impairment at presentation
- If dialysis-dependent at diagnosis, kidney recovery is rare.

Chapter 63

Antiphospholipid Syndrome

Overview

Antiphospholipid syndrome is an autoimmune thrombophilic disorder characterized by arterial or venous blood clot formation and/or pregnancy morbidity in the presence of persistent antiphospholipid antibodies: anticardiolipin antibodies, anti- β 2-glycoprotein I antibodies, and/or lupus anticoagulant, and may be primary (no associated disease) or secondary (associated with SLE). This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience deep vein blood clot formation, lung blockage by a blood clot, stroke, heart attack, recurrent pregnancy loss, livedo reticularis, thrombocytopenia, and cardiac valve disease, with catastrophic APS (CAPS) being a rare, life-threatening variant with widespread blood clot formation affecting multiple organs simultaneously.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of antiphospholipid syndrome require immediate medical evaluation

Progression and Stages

Your outlook depends on the site and severity of blood clot formation. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for at least one clinical criterion (blood clot formation or pregnancy morbidity) and one laboratory criterion (persistent antiphospholipid antibodies on two occasions ≥ 12 weeks apart).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves long-term anticoagulation with warfarin (INR target 2–3 for venous blood clot formation, 2–3 or 3–4 for arterial blood clot formation), with

DOACs not recommended for APS. Symptomatic management and supportive care
Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions)
Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications
Physical and occupational therapy Genetic counseling for family planning
Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 64

Antisocial Personality Disorder (ASPD)

Overview

Antisocial personality disorder has a prevalence of 0.5–3% in the general population, with a male-to-female ratio of 3:1, and is higher in prison populations (50–80%). This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of reduced amygdala volume and reactivity, ventromedial prefrontal cortex dysfunction, reduced autonomic responsivity to distress cues, and reduced gray matter volume in the orbitofrontal cortex and anterior insula, with genetic factors being strong (heritability 50–60%). Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a pervasive pattern of disregard for and violation of the rights of others, occurring since age 15, with at least three of the following: failure to conform to social norms, deceitfulness, impulsivity, irritability and aggressiveness, reckless disregard for safety, consistent irresponsibility, and lack of remorse, with the individual being at least 18 years old and having evidence of conduct disorder before age 15.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of antisocial personality disorder (aspd) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves challenging—no FDA-approved medications for ASPD, with pharmacotherapy targeting aggression and impulsivity (lithium, valproate, atypical antipsychotics). Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Unfortunately, the outlook is often poor
- Symptoms may attenuate with age but functional impairment persists.

Chapter 65

Aortic Dissection

Overview

Aortic dissection is a life-threatening medical emergency caused by a tear in the aortic intima that allows blood to surge into the aortic media, creating a false lumen. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of chronic high blood pressure, connective tissue disorders (Marfan syndrome, Ehlers-Danlos syndrome), or bicuspid aortic valve. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)

- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience sudden, severe, tearing or ripping chest pain radiating to the back (interscapular region), asymmetric blood pressure between arms, and neurological deficits. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of aortic dissection require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Stanford Type A (ascending aorta) requires immediate emergency surgical repair
- Stanford Type B (descending aorta) is managed medically with IV beta-blockers (esmolol, labetalol) to control heart rate and blood pressure unless complications occur.

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Diagnosis is confirmed urgently by CT angiography of the chest and abdomen or trans-esophageal echocardiography (TEE). Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Anti-hypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel)

Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

outlook is guarded, with high early mortality. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 66

Aortic Regurgitation

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Aortic regurgitation is incompetence of the aortic valve allowing diastolic backflow of blood from the aorta into the left ventricle
- Causes include bicuspid aortic valve, endocarditis, aortic root dilation (Marfan syndrome, aortic dissection, high blood pressure), and rheumatic disease.

Causes

This condition happens because of aortic valve leaflet or aortic root abnormalities. The chronic volume overload leads to left ventricular dilation and eccentric enlargement, eventually causing heart failure. Patients may be asymptomatic for years despite severe regurgitation. Clinical signs include a wide pulse pressure, water-hammer pulse, decrescendo diastolic murmur, and de Musset's sign. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus

- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of aortic regurgitation require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events

- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using echocardiography. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Treatment focuses on surgical aortic valve repair or replacement, indicated for symptomatic patients or when left ventricular ejection fraction falls below 50% or left ventricular end-systolic diameter exceeds 50 mm. Management includes lifestyle modifications, pharmacotherapy targeting the underlying mechanisms, and interventional procedures when indicated. Long-term adherence to treatment is essential for preventing disease progression. Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-

lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Most people recover well after surgical correction. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 67

Aortic Stenosis

Overview

Calcific (degenerative) aortic stenosis (AS) is the most common valvular heart disease in older adults, affecting 3–5% of those over 75. It is marked by progressive thickening, scarring, and calcium buildup of the aortic valve cusps, driven by lipid deposition, inflammation, and osteogenic differentiation of valvular interstitial cells. Once symptomatic (classic triad: shortness of breath, angina, syncope), outlook without intervention is poor (50% mortality at 2 years). In older adults, transcatheter aortic valve replacement (TAVR) has largely replaced surgical aortic valve replacement and is the standard of care for most elderly patients, with excellent outcomes and shorter recovery times. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease

- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of aortic stenosis require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection

- Chronic kidney disease from reduced perfusion

Diagnosis

History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk

- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 68

Aortic Stenosis

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Aortic stenosis is a narrowing of the aortic valve opening and the most common valvular disease in developed countries
- Degenerative calcific aortic stenosis is the most common cause in older adults, while bicuspid aortic valve (present in 1–2% of the population) is the most common cause in younger patients.

Causes

This condition happens because of progressive calcium buildup and restriction of the valve leaflets, causing pressure overload of the left ventricle, compensatory enlargement, and eventually heart failure. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease

- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience the classic triad of angina, syncope, and shortness of breath. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of aortic stenosis require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using echocardiography showing severe aortic stenosis (aortic valve area less than 1.0 cm², mean gradient greater than 40 mmHg). Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment typically involves aortic valve replacement with surgical (SAVR) or transcatheter (TAVR) approaches
- TAVR is increasingly preferred in intermediate- and high-risk patients.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)

- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Unfortunately, the outlook is often poor without intervention (mortality 50% at 2 years after symptom onset) but excellent after successful valve replacement. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 69

Aphthous Ulcers (Canker Sores)

Overview

- Aphthous ulcers are recurrent, painful, shallow ulcers of the oral mucosa that affect up to 25% of the population
- They are not contagious and are distinct from cold sores (herpes labialis).

Causes

This condition happens because of a multifactorial cause including local trauma, stress, nutritional deficiencies (iron, folate, B12), hormonal changes, and immune dysregulation. Clinical forms include minor aphthous (most common, small ulcers less than 1 cm, heal without scarring), major aphthous (larger, deeper ulcers, may scar), and herpetiform aphthous (multiple pinpoint ulcers that may coalesce).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of aphthous ulcers (canker sores) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves symptomatic, including topical anesthetics (lidocaine), antimicrobial rinses (chlorhexidine), topical corticosteroids (triamcinolone paste), and for severe cases, systemic agents (colchicine, thalidomide, apremilast). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good, with lesions healing spontaneously but frequently recurring. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 70

Aplastic Anemia

Overview

Aplastic anemia is a life-threatening bone marrow failure syndrome characterized by pancytopenia (anemia, leukopenia, thrombocytopenia) with a hypocellular bone marrow. The condition may be acquired (most common), inherited (Fanconi anemia, dyskeratosis congenita), or idiopathic, with acquired causes including autoimmune (T-cell mediated attack on hematopoietic stem cells), infections (hepatitis-associated aplastic anemia, parvovirus B19), drugs and toxins (chloramphenicol, chemotherapy agents, benzene), radiation, and pregnancy.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fatigue, pallor (anemia), recurrent infections (neutropenia), and bleeding (thrombocytopenia).

Emergency warning signs

- Severe or worsening symptoms of aplastic anemia require immediate medical evaluation

Progression and Stages

To make the diagnosis, doctors look for bone marrow biopsy showing markedly hypocellular marrow with increased fat cells, with severity classified as severe (marrow cellularity <25%, or 25–50% with <30% residual hematopoiesis and two of three cytopenias: ANC <500, platelets <20,000, reticulocytes <1%) or very severe (ANC <200). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Management options include allogeneic hematopoietic stem cell transplantation (curative, preferred for young patients with matched donors) and immunosuppressive therapy (antithymocyte globulin + cyclosporine, effective for older patients), with eltrombopag (thrombopoietin receptor agonist) showing efficacy in combination with immunosuppressive therapy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is guarded without treatment but improved with stem cell transplantation. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 71

Appendicitis

Overview

Appendicitis is inflammation of the vermiform appendix, the most common surgical emergency of the abdomen, with a lifetime risk of approximately 7–8%. It typically occurs between ages 10 and 30, with a slight male predominance.

Causes

The condition usually results from luminal obstruction of the appendix by a fecalith, lymphoid abnormal increase in cells, or rarely tumors, leading to increased intraluminal pressure, bacterial overgrowth, reduced blood flow, and perforation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the classic presentation of periumbilical pain that migrates to the right lower quadrant (McBurney's point), accompanied by anorexia, nausea, vomiting, and low-grade fever
- However, the classic The condition usually appears as present in only 50–60% of cases. Physical findings include tenderness at McBurney's point, guarding, and rebound tenderness. The Alvarado score (MANTRELS) aids clinical assessment.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of appendicitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Diagnosis typically involves laboratory findings of leukocytosis with left shift and CT abdomen/pelvis as the preferred imaging modality in adults (sensitivity and specificity >95%)
- Ultrasound may be used as first-line imaging in children and pregnant women.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves appendectomy (using small incisions and a camera preferred over open), with preoperative IV antibiotics
- Antibiotics alone may be considered for uncomplicated appendicitis in selected patients, though recurrence rates of 15–40% exist. Complicated appendicitis (gangrenous, perforated, with abscess or peritonitis) requires IV antibiotics and possibly interval appendectomy.
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with timely surgical intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 72

Arboviral Encephalitis

Overview

Arboviral encephalitis encompasses viral infections transmitted by arthropod vectors that cause brain inflammation, with major causes in North America including La Crosse encephalitis, St. Louis encephalitis, and Western equine encephalitis.

La Crosse Encephalitis: Caused by La Crosse virus (bunyavirus), transmitted by *Aedes triseriatus*. It is the most common cause of pediatric arboviral encephalitis in the United States (50–150 cases annually). This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)

- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Clinical features: incubation 5–15 days, with most infections being asymptomatic or mild febrile illness
- Neuroinvasive disease presents with fever, headache, seizures, altered mental status, and focal deficits. Case fatality rate is less than 1%.
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of arboviral encephalitis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes

- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

To diagnose arboviral encephalitis is by serology (IgM in CSF or serum by ELISA). Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

No specific antiviral therapy is available. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

Treatment typically involves supportive care.

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 73

Arsenic Poisoning

Overview

Arsenic is a naturally occurring metalloid found in groundwater, rice, and some industrial products. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of inorganic arsenic (trivalent arsenite) inhibiting pyruvate dehydrogenase, disrupting the Krebs's cycle and oxidative phosphorylation, and impairing DNA repair. Acute arsenic poisoning presents with severe digestive tract symptoms (vomiting, profuse watery diarrhea), hypotension, cardiac dysfunction, acute kidney injury, and death within 24 hours. Subacute/chronic toxicity presents with peripheral neuropathy, Mee's lines (transverse white bands on fingernails), diffuse hyperpigmentation, palmar-plantar hyperkeratosis, and squamous cell carcinoma.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of arsenic poisoning require immediate medical evaluation

Progression and Stages

Your outlook depends on severity and timing of treatment. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using 24-hour urine arsenic level. Treatment: dimercaprol (BAL) or succimer (DMSA) for acute symptomatic poisoning. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 74

Asbestosis

Overview

Asbestosis is a chronic interstitial lung disease caused by the inhalation of asbestos fibers, typically occurring after occupational exposure (shipbuilding, construction, insulation manufacturing) with a latency period of 20–30 years.

Causes

This condition happens because of macrophage phagocytosis of inhaled asbestos fibers, leading to chronic inflammation, fibrogenic growth factor release, and diffuse lung scarring.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive exertional shortness of breath, dry cough, end-inspiratory bibasilar crackles, and digital clubbing.

Emergency warning signs

- Severe or worsening symptoms of asbestosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on occupational history, high-resolution CT showing pleural plaques, lower lobe predominant reticular opacities, and asbestos bodies (ferruginous bodies) on bronchoalveolar lavage or lung biopsy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment typically involves supportive, including smoking cessation, oxygen therapy, and vaccination
- Patients are at high risk for bronchogenic carcinoma and cancerous mesothelioma.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is progressive and irreversible. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 75

Ascites

Overview

Causes

This condition happens because of portal high blood pressure (sinusoidal pressure greater than 12 mmHg) and splanchnic arterial vasodilation, leading to neurohormonal activation (RAAS, sympathetic nervous system, ADH) and sodium and water retention.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience abdominal distension and discomfort.

Emergency warning signs

- Severe or worsening symptoms of ascites require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Ascites is the accumulation of fluid in the peritoneal cavity and is the most common complication of decompensated cirrhosis. Your outlook depends on the response to treatment and underlying liver disease severity
- Spontaneous bacterial peritonitis is a serious complication.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using clinical examination (shifting dullness, fluid thrill) and abdominal ultrasound
- Diagnostic paracentesis shows a serum-ascites albumin gradient (SAAG) of 1.1 g/dL or greater, confirming portal high blood pressure.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include sodium restriction (less than 2 g/day), diuretics (spironolactone and furosemide), and large-volume paracentesis for tense ascites
- Refractory ascites may require transjugular intrahepatic portosystemic shunt (TIPS) or liver transplantation.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 76

Aspergillosis (Invasive)

Overview

Causes

This condition results from disruption of normal aspergillosis (invasive) function.

- This condition happens because of conidia (spores) being inhaled and reaching the alveoli
- In immunocompromised hosts, conidia germinate into hyphae that invade lung blood vessels (angioinvasion), causing blood clot formation, tissue death due to lack of blood supply, and involving bleeding tissue death.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience invasive lung aspergillosis (most common): fever refractory to antibiotics, cough, pleuritic chest pain, and shortness of breath, with chest CT showing nodules with halo sign and air crescent sign.

Emergency warning signs

- Severe or worsening symptoms of aspergillosis (invasive) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using culture, galactomannan (GM) antigen, and CT chest. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include voriconazole (IV or oral) as first-line, with isavuconazole as an alternative. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Unfortunately, the outlook is often poor without treatment
- Mortality 30–80%.

Chapter 77

Aspiration Pneumonia

Overview

Aspiration pneumonia is a pulmonary infection caused by the inhalation of oropharyngeal or gastric contents into the lower respiratory tract, leading to chemical pneumonitis and subsequent bacterial infection. It most commonly affects dependent lung segments, particularly the right lower lobe due to the more vertical orientation of the right main bronchus. Aspiration pneumonia is distinct from aspiration pneumonitis (Mendelson syndrome), which is a chemical injury without infection.

Classification

Cause Infectious (bacterial) following aspiration of colonized oropharyngeal material

Organ System Lungs/Respiratory

Pathophysiology Aspiration of oropharyngeal or gastric contents into lower airways causing chemical injury followed by bacterial infection with mixed aerobic and anaerobic flora

Duration Acute

Onset Sudden (chemical) to Gradual (infectious over days)

Mode of Transmission Non-communicable (aspiration of endogenous flora)

Clinical Course Variable; can progress rapidly to severe pneumonia or lung abscess

Severity Mild to Severe

Extent of Disease Localized (dependent lung segments, often right lower lobe)

Age Group Older adults, young children, and neurologically impaired patients

Epidemiology Accounts for 5–15% of community-acquired pneumonia cases; high incidence in nursing homes and hospitalized patients with dysphagia

ICD-11 Code CA22.0

Causes

Aspiration pneumonia results from the entry of bacteria-laden oropharyngeal secretions or gastric contents into the lower airways, typically in patients with impaired swallowing, reduced cough reflex, or decreased level of consciousness. The primary pathogens include anaerobic oral flora (*Peptostreptococcus*, *Bacteroides*, *Prevotella*, *Fusobacterium*) often

mixed with aerobic bacteria (*Streptococcus pneumoniae*, *Haemophilus influenzae*, *Staphylococcus aureus*). Hospital-acquired aspiration may involve more resistant organisms such as *Pseudomonas aeruginosa* and *Enterobacteriaceae*.

Risk Factors

- Dysphagia from neurologic disorders (stroke, Parkinson disease, dementia, amyotrophic lateral sclerosis)
- Decreased level of consciousness (alcohol or drug intoxication, general anesthesia, sedation, seizure)
- Impaired cough reflex due to neurologic injury or medications
- Nasogastric or endotracheal tubes compromising laryngeal function
- Gastroesophageal reflux disease with large-volume regurgitation
- Poor oral hygiene and periodontal disease leading to high bacterial load
- Advanced age with frailty and institutionalization

Signs and Symptoms

Early symptoms

- Early manifestations of aspiration pneumonia may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Cough productive of foul-smelling or purulent sputum
- Fever and rigors
- Dyspnea and tachypnea
- Pleuritic chest pain
- Crackles or bronchial breath sounds in affected lung zones
- Hypoxia with decreased oxygen saturation
- Confusion or altered mental status (especially in elderly)
- Tachycardia and signs of systemic inflammatory response
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of aspiration pneumonia may occur

Emergency warning signs

- Severe or worsening symptoms of aspiration pneumonia require immediate medical evaluation

Progression and Stages

Chemical pneumonitis from acidic gastric contents develops within hours of aspiration, presenting with acute dyspnea and hypoxemia even without infection. Bacterial aspiration pneumonia typically evolves over 1–3 days as aspirated organisms multiply in the lung parenchyma, producing a consolidated infection that appears as dependent-segment opacities on chest imaging. Untreated or inadequately treated infection can progress to necrotizing pneumonia, lung abscess formation, or empyema over the course of 7–14 days.

Complications

- Lung abscess (cavitary lesion from necrotizing infection)
- Empyema (purulent pleural effusion requiring drainage)
- Bronchopleural fistula
- Bacteremia with sepsis and septic shock
- Respiratory failure requiring mechanical ventilation
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical history of aspiration event or presence of risk factors (dysphagia, altered consciousness)
- Chest radiograph or CT showing opacities in dependent segments (right lower lobe, superior segments of lower lobes)
- Sputum gram stain and culture (aerobic and anaerobic)
- Blood cultures to identify bacteremia
- Complete blood count with leukocytosis and left shift
- Procalcitonin and C-reactive protein as inflammatory markers
- Imaging tests to evaluate for complications (cavitation, empyema)

Prevention

- Dysphagia screening and swallowing assessment in at-risk patients (post-stroke, elderly)
- Modified food textures and thickened liquids for patients with impaired swallowing
- Head-of-bed elevation to 30–45 degrees during and after feeding
- Good oral hygiene to reduce bacterial colonization of the oropharynx
- Proper tube feeding management with gastric residual volume monitoring

- Minimize sedative and antipsychotic medications in susceptible patients
- Go for regular check-ups
- Follow your doctor's screening recommendations

Treatment Options

- Empiric broad-spectrum antibiotics covering anaerobes and aerobes (beta-lactam/beta-lactamase inhibitor, clindamycin, or carbapenem for severe cases)
- Targeted antibiotic therapy based on culture sensitivity results
- Supplemental oxygen and respiratory support as needed
- Chest physiotherapy and postural drainage to mobilize secretions
- Source control: dental treatment for periodontal disease if present
- Tube feeding modifications or percutaneous gastrostomy for severe dysphagia
- Lifestyle changes and self-care
- Surgery when needed (drainage of empyema, lung resection for abscess)

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups
- Keep learning about your condition

Outlook

With prompt diagnosis and appropriate antibiotic therapy, most patients with aspiration pneumonia recover fully, though recovery may be prolonged in elderly or debilitated patients. Mortality ranges from 5–20% depending on the severity of infection, comorbidities, and the patient's functional status.

Chapter 78

Asplenia

Overview

Asplenia is the absence of splenic function, which may be congenital (asplenia syndrome, often associated with complex congenital heart disease and situs abnormalities) or acquired (splenectomy, sickle cell disease, celiac disease, inflammatory bowel disease).

Causes

This condition happens because of anatomical absence or functional impairment of the spleen, which normally clears encapsulated bacteria through opsonin-dependent phagocytosis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience increased risk for overwhelming post-splenectomy infection (OPSI), a fulminant sepsis with 50–70% mortality.

Emergency warning signs

- Severe or worsening symptoms of asplenia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on history of splenectomy or conditions causing functional asplenia, confirmed by the presence of Howell-Jolly bodies on blood smear or absence of splenic uptake on nuclear medicine scan. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment focuses on pneumococcal, meningococcal, and Hib vaccination, prophylactic antibiotics (penicillin V or amoxicillin), and patient education about infection risks.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is good with preventive measures
- OPSI carries high mortality. Functional asplenia (impaired splenic function without anatomical absence) occurs in sickle cell disease (autosplenectomy from repeated splenic tissue death due to lack of blood supply), celiac disease, and inflammatory bowel disease.

Chapter 79

Asplenia and Hyposplenism

Overview

Asplenia (absence of splenic tissue) and hyposplenism (impaired splenic reticuloendothelial function) result from surgical splenectomy, congenital anomalies (Ivemark syndrome), or autosplenectomy secondary to repeated microvascular tissue death due to lack of blood supply in sickle cell anemia. The spleen filters encapsulated bacteria, senescent erythrocytes, and intraerythrocytic inclusions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience vulnerability to fulminant encapsulated bacterial sepsis (*Streptococcus pneumoniae*, *Neisseria meningitidis*, *Haemophilus influenzae* type b) and capnocytophaga infections following dog bites. Peripheral blood smears demonstrate Howell-Jolly bodies (nuclear remnants), Pappenheimer bodies, Heinz bodies, and target cells.

Emergency warning signs

- Severe or worsening symptoms of asplenia and hyposplenism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with blood smear morphology and 99mTc-labeled heat-damaged RBC radionuclide scintigraphy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include mandatory immunizations (pneumococcal, meningococcal, Hib, annual influenza), prophylactic oral penicillin in children, and immediate empiric parenteral antibiotics for febrile episodes. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up

care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 80

Asthma

Overview

- Asthma is a heterogeneous chronic inflammatory disease of the airways characterized by wheezing, breathlessness, chest tightness, and cough, particularly at night or in the early morning
- Triggers include allergens, respiratory infections, exercise, cold air, and irritants.

Causes

This condition happens because of IgE-mediated mast cell degranulation, eosinophilic infiltration, goblet cell abnormal increase in cells, smooth muscle enlargement, and airway remodeling, leading to variable airflow obstruction and airway hyperresponsiveness.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience episodic wheezing, breathlessness, chest tightness, and cough. Diagnosis is supported by variable airflow limitation on spirometry (reversibility of more than 12% and 200 mL in FEV₁ after bronchodilator), peak flow variability, and bronchoprovocation testing. Management follows stepwise therapy: inhaled corticosteroids (cornerstone of controller therapy), long-acting beta-agonists, leukotriene receptor antagonists, long-acting muscarinic antagonists, and biologic therapies (anti-IgE, anti-IL5, anti-IL4R α) for severe eosinophilic asthma.

Emergency warning signs

- Severe or worsening symptoms of asthma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate controller therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 81

Asthma-COPD Overlap

Overview

Asthma-COPD overlap is a condition in which patients share features of both asthma and COPD, including persistent airflow limitation, airway inflammation that is both eosinophilic and neutrophilic, and significant exacerbation burden.

Causes

This condition happens because of overlapping pathophysiological mechanisms of both asthma and COPD, with greater morbidity than either disease alone.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience persistent airflow limitation with variability and significant exacerbation burden.

Emergency warning signs

- Severe or worsening symptoms of asthma-copd overlap require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Management combines inhaled corticosteroids with long-acting bronchodilators
- Biologic therapies are considered for severe disease.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is worse than for asthma or COPD alone. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 82

Astigmatism

Overview

Astigmatism is a refractive error caused by irregular curvature of the cornea or lens, resulting in different refractive powers in different meridians.

Causes

This condition happens because of irregular corneal or lenticular curvature.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience blurred or distorted vision at both near and distance. Most astigmatism is corneal and regular.

Emergency warning signs

- Severe or worsening symptoms of astigmatism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on refraction testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on correction with cylindrical lenses (glasses or contact lenses) or refractive surgery
- Irregular astigmatism may require rigid gas-permeable contact lenses or scleral lenses.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider

- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate correction. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 83

Atelectasis

Overview

Atelectasis is the collapse or incomplete expansion of lung parenchyma, leading to impaired gas exchange.

Causes

This condition happens because of airway obstruction by mucus plugs, foreign bodies, or tumors (resorptive atelectasis)

- Pleural effusion or pneumothorax (compression atelectasis)
- Or loss of lung surfactant (microatelectasis). It is the most common cause of early postoperative fever (within 24–48 hours).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience tachypnea, cough, hypoxemia, and decreased breath sounds over the affected region.

Emergency warning signs

- Severe or worsening symptoms of atelectasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using chest radiograph showing opacification with volume loss and tracheal deviation toward the affected side. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on incentive spirometry, chest physiotherapy, early postoperative ambulation, and therapeutic bronchoscopy for persistent mucus plugging. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well when treated promptly. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 84

Atopic Dermatitis

Overview

Atopic dermatitis (AD, eczema) is a chronic, relapsing inflammatory skin disease characterized by intense itching, xerosis, and eczematous lesions with a characteristic distribution. It affects 10–20% of children and 3–7% of adults.

Causes

This condition results from disruption of normal atopic dermatitis function.

- This condition happens because of epidermal barrier dysfunction (filaggrin mutations in 30–50% of cases), immune dysregulation (Th2-predominant inflammation), and microbial dysbiosis (colonization with *Staphylococcus aureus*)
- The "atopic march" links AD to food allergy, allergic rhinitis, and asthma.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms vary depending on age: infantile AD involves the face and extensor surfaces
- Childhood AD affects flexural areas (antecubital and popliteal fossae)
- Adult AD presents with lichenified plaques on flexural surfaces.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of atopic dermatitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include regular emollient use, topical corticosteroids (first-line for flares), topical calcineurin inhibitors (tacrolimus, pimecrolimus for sensitive areas), and for moderate-to-severe disease, biologic therapies (dupilumab [anti-IL-4R α], tralokinumab [anti-IL-13]) and JAK inhibitors (abrocitinib, baricitinib). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- The condition tends to improve with age.

Chapter 85

Atrial Fibrillation

Overview

Atrial fibrillation is the most common sustained irregular heartbeat, affecting 2–4% of the adult population and increasing in prevalence with age. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of disorganized electrical activity in the atria, leading to irregular and often rapid ventricular response. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension and diabetes
- Elevated cholesterol levels
- Family history of heart disease
- Obesity and physical inactivity
- Hypertension (high blood pressure)
- Diabetes mellitus

- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of atrial fibrillation require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- You may experience palpitations, fatigue, shortness of breath, and decreased exercise tolerance
- Complications include thromboembolism (stroke), tachycardia-mediated cardiomy-

- opathy, and impaired quality of life.
- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using ECG showing absent P waves, irregularly irregular R-R intervals, and variable QRS morphology. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment focuses on rate control (beta-blockers, calcium channel blockers, digoxin) and rhythm control (antiarrhythmic drugs, cardioversion, catheter ablation)

- The CHA₂DS₂-VASc score guides anticoagulation decisions, with direct oral anticoagulants preferred over warfarin for non-valvular atrial fibrillation.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 86

Atrial Fibrillation in the Elderly

Overview

Atrial fibrillation (AF) prevalence increases from 2% in adults under 65 to 15–20% in those over 80. Geriatric-specific management challenges include higher thromboembolic risk, increased bleeding risk, kidney impairment affecting DOAC dosing, high rates of polypharmacy, and increased risk of falls complicating anticoagulation decisions. Rate control is the default strategy in older adults, reserving rhythm control for symptomatic patients despite adequate rate control. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity

- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of atrial fibrillation in the elderly require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

Anticoagulation for stroke prevention is indicated for most older adults with AF, using DOACs in preference to warfarin due to lower intracranial bleeding risk.

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk

- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 87

Atrial Flutter

Overview

Atrial flutter is a supraventricular arrhythmia characterized by a rapid, regular atrial rate of 250–350 beats per minute with a characteristic sawtooth pattern on electrocardiogram. It arises from a macro-reentrant circuit, most commonly in the right atrium around the tricuspid annulus (cavotricuspid isthmus-dependent flutter). This condition shares risk factors with atrial fibrillation and often coexists with or alternates between the two arrhythmias.

Classification

Cause Reentrant electrical circuit in the atria

Organ System Heart

Pathophysiology Macro-reentrant circuit, typically cavotricuspid isthmus-dependent, producing continuous atrial depolarization

Duration Acute or Chronic (paroxysmal, persistent, or permanent)

Onset Sudden

Mode of Transmission Non-communicable

Clinical Course Can be paroxysmal, persistent, or permanent; often progresses to atrial fibrillation

Severity Mild to Moderate

Extent of Disease Localized (right atrial conduction system)

Age Group Older adults (peak prevalence after age 60)

Epidemiology Prevalence approximately 0.1% in the general population; increases sharply with age

ICD-11 Code BC93.0

Causes

Atrial flutter is caused by a macro-reentrant circuit within the atrial myocardium, most commonly involving the cavotricuspid isthmus in the right atrium. Conditions that increase atrial pressure or cause atrial enlargement, such as hypertension, heart failure, valvular heart disease, and pulmonary disease, predispose to the development of flutter. Post-cardiac surgery and hyperthyroidism are also well-established triggers.

Risk Factors

- Advanced age
- Hypertension with left atrial enlargement
- Heart failure with reduced or preserved ejection fraction
- Chronic obstructive pulmonary disease (COPD)
- Hyperthyroidism
- Obesity and obstructive sleep apnea
- Prior cardiac surgery (especially valve or coronary bypass surgery)

Signs and Symptoms

Early symptoms

- Early manifestations of atrial flutter may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
 - Palpitations described as a rapid, regular pounding in the chest
 - Shortness of breath, especially with exertion
 - Fatigue and reduced exercise tolerance
 - Dizziness or lightheadedness
 - Syncope or near-syncope with rapid ventricular rates
 - Chest discomfort or pressure
 - Anxiety or sensation of unease
 - Pain or discomfort in the affected area
 - Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of atrial flutter may occur

Emergency warning signs

- Severe or worsening symptoms of atrial flutter require immediate medical evaluation

Progression and Stages

Atrial flutter often presents as paroxysmal episodes that may spontaneously convert to sinus rhythm or progress to persistent flutter requiring cardioversion. Over time, many patients develop concurrent atrial fibrillation (flutter-fib syndrome) as atrial remodeling

progresses. Chronic atrial flutter with poorly controlled ventricular rates can lead to tachycardia-induced cardiomyopathy and worsening heart failure.

Complications

- Thromboembolic events including ischemic stroke (similar risk profile to atrial fibrillation)
- Tachycardia-induced cardiomyopathy from persistently rapid ventricular rates
- Progression to atrial fibrillation (flutter-fib)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- 12-lead ECG showing characteristic sawtooth flutter waves (typically negative in II, III, aVF)
- Holter or event monitoring for paroxysmal episodes
- Echocardiogram to assess atrial size and structural heart disease
- Laboratory tests including thyroid function and electrolytes
- Transesophageal echocardiogram (TEE) to exclude left atrial thrombus before cardioversion
- Imaging tests as needed

Prevention

- Treat underlying conditions such as hypertension, heart failure, and hyperthyroidism
- Maintain a healthy weight and manage sleep apnea
- Avoid excessive alcohol consumption
- Anticoagulation therapy based on CHA₂DS₂-VASc score to prevent stroke
- Catheter ablation as preventive curative treatment
- Go for regular check-ups
- Follow your doctor's screening recommendations

Treatment Options

- Rate control with beta-blockers or calcium channel blockers
- Electrical or pharmacologic cardioversion to restore sinus rhythm
- Catheter ablation (cavotricuspid isthmus ablation) with high success rate
- Anticoagulation (warfarin or direct oral anticoagulants) for stroke prevention
- Antiarrhythmic drugs (flecainide, propafenone, amiodarone) for rhythm control
- Lifestyle changes and self-care
- Surgery when needed (maze procedure in selected cases)

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups
- Keep learning about your condition

Outlook

The prognosis for patients with atrial flutter is generally favorable with appropriate management. Catheter ablation of typical atrial flutter has a success rate of 90–95% and is considered first-line definitive therapy in suitable candidates.

Chapter 88

Atrophic Vaginitis (Genitourinary Syndrome of Menopause)

Overview

Atrophic vaginitis, now termed genitourinary syndrome of menopause (GSM), is a chronic progressive condition resulting from estrogen deficiency after menopause, affecting up to 50% of postmenopausal women. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of estrogen withdrawal leading to thinning of the vaginal epithelium, decreased vaginal secretions, loss of rugae, shortening and narrowing of the vaginal canal, and changes in the vulvar and lower urinary tract tissues. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience vaginal dryness, burning, itching, dyspareunia, and urinary symptoms (frequency, urgency, recurrent UTIs).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of atrophic vaginitis (genitourinary syndrome of menopause) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam: pale, thin, smooth vaginal mucosa with loss of rugae, elevated vaginal pH (>5.0), and maturation index showing predominance of parabasal cells on cytology.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include vaginal moisturizers and lubricants for mild symptoms, and low-dose vaginal estrogen therapy for moderate to severe symptoms. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Most people recover well with treatment, significantly improving quality of life and sexual function.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 89

Attention-Deficit/Hyperactivity Disorder (ADHD)

Overview

Attention-deficit/hyperactivity disorder has a worldwide prevalence of 5–7% in children and 2.5–3% in adults, more common in boys in childhood (3:1 ratio) but approaching parity in adulthood. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of striatal and prefrontal dopaminergic and noradrenergic dysregulation, with reduced brain volume in prefrontal cortex, caudate, and cerebellum, delayed cortical maturation, and high genetic heritability (75–80%). Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a persistent pattern of inattention, hyperactivity, and impulsivity that interferes with functioning, with three presentations: predominantly inattentive, predominantly hyperactive-impulsive, and combined presentation, with onset before age 12.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of attention-deficit/hyperactivity disorder (adhd) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, based on detailed developmental history, collateral reports, and behavior rating scales.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include pharmacotherapy (stimulants as first-line: methylphenidate and amphetamine formulations
- Non-stimulants as second-line: atomoxetine, guanfacine, clonidine) and psychosocial interventions (behavioral parent training, CBT, executive function coaching). outlook: 50–65% of children continue to have impairing symptoms into adulthood.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 90

Atypical Facial Pain

Overview

Persistent idiopathic facial pain (PIFP, formerly atypical facial pain) is a persistent facial pain that does not fit the diagnostic criteria for any other cranial neuralgia or headache disorder.

Causes

This condition happens because of poorly understood Disease mechanism, possibly involving central sensitization and dysfunction of descending pain modulatory systems.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience daily or nearly continuous deep, poorly localized, aching or burning pain that may cross the midline.

Emergency warning signs

- Severe or worsening symptoms of atypical facial pain require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for exclusion of dental and neurological causes after thorough investigation including imaging. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves multimodal: TCAs, SNRIs, gabapentin, pregabalin, and thinking and memory behavioral therapy. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Invasive procedures are generally not recommended.

Chapter 91

Autism Spectrum Disorder (ASD)

Overview

Autism spectrum disorder has a prevalence of 1–2% worldwide, with a male-to-female ratio of 3–4:1. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of dysregulation of synaptic pruning and neural connectivity, an imbalance of excitation/inhibition (particularly GABAergic and glutamatergic systems), aberrant brain growth, and alterations in frontotemporal, frontoparietal, and interhemispheric connectivity, with strong genetic factors (heritability 80–90%). Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Social communication deficits include reduced social-emotional reciprocity, deficits in nonverbal communicative behaviors, and deficits in developing relationships. Restricted, repetitive patterns manifest as stereotyped speech/gait, insistence on sameness, highly restricted fixated interests, and hyper- or hyporeactivity to sensory input. Intellectual disability is present in 30–50%, and language impairment in 25–30%.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of autism spectrum disorder (asd) require immediate medical evaluation

Progression and Stages

You may experience persistent deficits in social communication and social interaction across multiple contexts, and restricted, repetitive patterns of behavior, interests, or activities, with three levels of severity requiring support. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include early intensive behavioral intervention (EIBI/ABA), social skills training, speech and language therapy, and pharmacotherapy for comorbid symptoms (atypical antipsychotics for irritability/aggression, SSRIs for anxiety/depression). Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is highly variable
- Early intervention significantly improves functional outcomes.

Chapter 92

Autoimmune Hepatitis

Overview

Autoimmune hepatitis is a chronic inflammatory liver disease characterized by autoimmune destruction of hepatocytes, predominantly affecting women. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

- This condition happens because of autoimmune-mediated hepatocyte destruction. Type 1 (positive ANA or anti-smooth muscle antibodies) is more common and occurs at any age
- Type 2 (positive anti-LKM-1 antibodies) is rarer and typically affects children and young women.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Symptoms can range from asymptomatic elevation of aminotransferases to acute hepatitis or fulminant failure. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of autoimmune hepatitis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

To make the diagnosis, doctors look for elevated transaminases, hypergammaglobulinemia (IgG), characteristic autoantibodies, and liver biopsy showing interface hepatitis with plasma cell infiltration. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or

antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment focuses on corticosteroids (prednisone) combined with azathioprine to induce and maintain remission. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Treatment aims to control inflammation, manage symptoms, and prevent disease flares. A step-up approach is often used, starting with symptomatic treatments and progressing to immunosuppressive therapies as needed. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended

- Gradually resume normal activities as symptoms improve

Outlook

The outlook is good with treatment, although lifelong immunosuppression is often necessary. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 93

Autoimmune Pancreatitis

Overview

Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions.

- Autoimmune pancreatitis (AIP) is a form of chronic pancreatitis caused by autoimmune inflammation of the pancreatic parenchyma. Type 1 AIP is linked to IgG4-related disease and involves other organs (biliary strictures, retroperitoneal scarring, salivary gland enlargement)
- Type 2 AIP is limited to the pancreas and is linked to inflammatory bowel disease.

Causes

This condition happens because of autoimmune-mediated inflammation. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Clinical features often mimic pancreatic cancer with painless jaundice, weight loss, and a pancreatic mass. Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of autoimmune pancreatitis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Doctors diagnose this based on the international consensus diagnostic criteria (ICDC): imaging (sausage-shaped pancreas on CT, delayed enhancement), serology (elevated IgG4 in type 1), histology (lymphoplasmacytic infiltrate with storiform scarring), and response to corticosteroids. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Treatment typically involves with oral prednisone, with rituximab for relapsing type 1 AIP. Treatment aims to control inflammation, manage symptoms, and prevent disease flares. A step-up approach is often used, starting with symptomatic treatments and progressing to immunosuppressive therapies as needed. Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

- The outlook is generally positive
- Unlike pancreatic cancer, AIP responds dramatically to corticosteroids.

Chapter 94

Autoimmune Pancreatitis (AIP)

Overview

Type 1 AIP affects predominantly older males and presents with painless obstructive jaundice, weight loss, and a diffuse 'sausage-shaped' enlargement of the pancreas with a capsule-like rim on CT/MRI, frequently mimicking pancreatic ductal adenocarcinoma. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of autoimmune pancreatitis (aip) require immediate medical evaluation

Progression and Stages

Autoimmune pancreatitis (AIP) is a distinct form of chronic pancreatitis driven by autoimmune inflammation, classified into Type 1 (IgG4-related disease, systemic involvement with dense IgG4+ plasma cell infiltration and storiform scarring) and Type 2 (pancreas-specific, neutrophilic phlebotomy lesions without IgG4 systemic disease). Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy

- Lymphoma risk in some autoimmune conditions

Diagnosis

Diagnosis relies on the International Consensus Diagnostic Criteria (ICDC): elevated serum IgG4 levels ($> 2\times$ normal), characteristic parenchymal enlargement, main pancreatic duct narrowing, and IgG4+ plasma cells on core biopsy. AIP demonstrates dramatic responsiveness to systemic corticosteroid therapy (prednisone), avoiding unnecessary pancreaticoduodenectomy (Whipple procedure). Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring

for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Autoimmune diseases are generally chronic conditions requiring long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 95

Autosomal Dominant Polycystic Kidney Disease

Overview

Autosomal dominant polycystic kidney disease (ADPKD) is the most common hereditary kidney disease, affecting 1 in 400–1000 individuals. It is caused by mutations in PKD1 (chromosome 16, 85% of cases) or PKD2 (chromosome 4, 15% of cases).

Causes

This condition happens because of defective polycystin proteins leading to progressive bilateral kidney cyst formation, massive kidney enlargement, declining GFR, and ESRD typically by the fifth to seventh decade.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience flank pain, hematuria, high blood pressure, and abdominal distension
- Extrarenal manifestations include liver cysts (most common), intracranial berry aneurysms (5–10%), cardiac valvular abnormalities, and colonic diverticulosis.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of autosomal dominant polycystic kidney disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on imaging (CT or MRI showing ≥ 3 cysts per kidney) and genetic testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include BP control (ACE inhibitors/ARBs), salt restriction, adequate hydration, and tolvaptan (vasopressin V2 receptor antagonist) to slow cyst growth and kidney function decline. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Screening of first-degree relatives is recommended.

Chapter 96

Bacterial Meningitis

Overview

Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

- Bacterial meningitis is an acute, life-threatening infection of the leptomeninges and cerebrospinal fluid
- The most common causative organisms vary by age: *Streptococcus pneumoniae* and *Neisseria meningitidis* in adults, *Group B Streptococcus* and *Escherichia coli* in neonates, and *Haemophilus influenzae* type b (in unvaccinated populations).

Causes

This condition happens because of bacterial invasion of the subarachnoid space. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience severe headache, high fever, neck stiffness (nuchal rigidity), photophobia, altered mental status, and petechial rash (meningococcal disease)
- Kernig's and Brudzinski's signs may be positive.
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of bacterial meningitis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)

- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

To make the diagnosis, doctors look for lumbar puncture showing elevated opening pressure, turbid CSF with neutrophilic pleocytosis, elevated protein, low glucose, and positive Gram stain and culture. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)

- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

- Treatment focuses on empiric treatment with ceftriaxone plus vancomycin, which should not be delayed
- Dexamethasone improves outcomes in pneumococcal meningitis.
- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Your outlook depends on the causative organism and timeliness of treatment. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 97

Bacterial Vaginosis

Overview

Bacterial vaginosis (BV) is a gynecologic infection and the most common cause of vaginal discharge in reproductive-age women, affecting 20–50% depending on population. It is a polymicrobial dysbiosis characterized by a reduction in hydrogen peroxide-producing *Lactobacillus* species and overgrowth of anaerobic bacteria. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience a thin, homogenous, gray-white vaginal discharge with a fishy odor (especially after intercourse or menses), but 50% of women are asymptomatic. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of bacterial vaginosis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment

- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using Amsel criteria (≥ 3 of: thin white discharge, pH >4.5 , positive whiff test, clue cells on saline wet mount) or Nugent score. BV in pregnancy is linked to preterm labor, PROM, and postpartum endometritis. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures

- Travel precautions and pre-travel consultations

Treatment Options

Treatment typically involves metronidazole (500 mg BID for 7 days or 2 g single dose) or clindamycin cream. Recurrence is common. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 98

Balanitis and Balanoposthitis

Overview

- Balanitis is inflammation of the glans penis
- Balanoposthitis additionally involves the foreskin, and is common in uncircumcised men.

Causes

This condition happens because of infectious (*Candida albicans*—most common, bacterial, STIs), irritant (soap, detergent), or skin (lichen planus, psoriasis, contact dermatitis) factors, with diabetes mellitus predisposing to candida balanitis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience redness of the skin, swelling, itching, discharge, dysuria, and tender inguinal lymphadenopathy.

Emergency warning signs

- Severe or worsening symptoms of balanitis and balanoposthitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Swab for KOH preparation (candida), bacterial culture, and STI screening.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include improved hygiene, topical antifungal (clotrimazole, miconazole) for candida, topical antibiotic/steroid combinations, and addressing underlying causes. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 99

Barotrauma of the Ear

Overview

Ear barotrauma is tissue injury to the tympanic membrane and middle ear cavity caused by an uncompensated pressure differential between ambient atmospheric pressure and the air-filled middle ear space, occurring during rapid altitude changes (scuba diving descent, commercial aviation, blast injuries). This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of Eustachian tube dysfunction (occlusion due to upper respiratory infection, mucosal swelling, or allergic rhinitis) preventing equalizer air flow into the middle ear. During ambient pressure increases, negative middle ear pressure draws the tympanic membrane inward, precipitating mucosal hyperemia, subepithelial bleeding, middle ear effusion, or tympanic membrane rupture.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms often appear as sudden severe ear pain (otalgia), a sensation of ear fullness/pressure, conductive hearing loss, ringing in the ears, and occasionally feeling of spinning or otorrhea (bloody ear discharge). Otoscopy demonstrates tympanic membrane retraction, hemotympanum, or perforation.

Emergency warning signs

- Severe or worsening symptoms of barotrauma of the ear require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment focuses on prevention (Valsalva maneuver, chewing, oral/nasal decongestants prior to flight/dive)
- Acute traumatic perforations usually heal spontaneously, with surgical tympanoplasty reserved for persistent non-healing tears.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 100

Barrett Esophagus

Overview

Barrett esophagus is a metaplastic condition of the distal esophagus resulting from chronic gastroesophageal reflux disease (GERD).

Causes

This condition happens because of chronic acid and bile exposure inducing intestinal metaplasia, where normal stratified squamous epithelium is replaced by non-ciliated columnar epithelium with goblet cells.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features mirror those of GERD (heartburn, regurgitation), though some patients are asymptomatic.

Emergency warning signs

- Severe or worsening symptoms of barrett esophagus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Treatment options include high-dose proton pump inhibitors (PPIs) and using a thin camera tube surveillance to screen for dysplasia
- Using a thin camera tube radiofrequency ablation or mucosal removal is indicated for high-grade dysplasia.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for upper endoscopy showing salmon-pink mucosa extending proximal to the gastroesophageal junction, with histopathological confirmation of intestinal goblet cells. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 101

Bartholin Cyst

Overview

Bartholin cyst (and Bartholin abscess) results from fluid accumulation secondary to obstruction of the main duct of the greater vestibular (Bartholin) gland located at the 4 and 8 o'clock positions of the posterior vaginal introitus.

Causes

This condition happens because of ductal occlusion caused by local inflammation, trauma, thickened mucus, or prior infection (*Neisseria gonorrhoeae*, *Chlamydia trachomatis*, *Escherichia coli*). Uninfected cysts present as painless, unilateral labial swellings. Secondary bacterial infection causes a Bartholin abscess, characterized by rapid onset of severe localized vulvar pain, dyspareunia, redness of the skin, and a fluctuant mass.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of Bartholin cyst require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam based on characteristic anatomic location
- Biopsy is indicated in women > 40 years to rule out rare Bartholin gland carcinoma. Treatment for asymptomatic small cysts is conservative
- Symptomatic cysts or abscesses require incision and removal of fluid with Word catheter placement or marsupialization to maintain ductal patency.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 102

Basal Cell Carcinoma

Overview

Basal cell carcinoma (BCC) is the most common skin cancer worldwide, arising from basal cells of the epidermis. It is strongly associated with cumulative ultraviolet radiation exposure and predominantly affects fair-skinned individuals. BCC is locally invasive but rarely metastasizes. Clinical subtypes include nodular (most common, pearly papule with telangiectasia and central ulceration—"rodent ulcer"), superficial (erythematous, scaly plaque, often on the trunk), and morpheaform (sclerosing, whitish, scar-like). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity

- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of basal cell carcinoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- The right treatment depends on subtype, size, and location: surgical removal, Mohs micrographic surgery (for high-risk locations such as the face, with highest cure rate of 99%), electrodesiccation and curettage (for superficial, low-risk lesions), and topical therapy (5-fluorouracil, imiquimod) for superficial BCC
- Vismodegib and sonidegib (Hedgehog pathway inhibitors) are used for advanced or

metastatic BCC.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this based on biopsy. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and

depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

- Most people recover well
- Metastasis is extremely rare.

Chapter 103

Becker Muscular Dystrophy

Overview

Becker muscular dystrophy (BMD) is an X-linked recessive disorder caused by in-frame mutations in the dystrophin gene, producing a truncated but partially functional dystrophin protein. It affects approximately 1 in 18,000–30,000 males. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of dystrophin deficiency that is less severe than DMD. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Cardiomyopathy is a significant cause of morbidity and mortality, often presenting before significant skeletal muscle weakness.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of becker muscular dystrophy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- You may experience proximal muscle weakness with later onset (late childhood to early adulthood) and slower progression compared to DMD
- Patients may remain ambulatory into their 30s or beyond.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on genetic testing and muscle biopsy.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves similar to DMD, with emphasis on cardiac surveillance and management. Symptomatic management and supportive care Enzyme replacement

therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is better than DMD, with longer ambulation and survival.

Chapter 104

Behçet's Disease

Overview

- Behçet's disease is a chronic, relapsing inflammatory disorder causing inflammation of blood vessels (vasculitis) throughout the body
- It classically presents with a triad of recurrent mouth ulcers, genital ulcers, and eye inflammation (uveitis)
- It is most common along the historic "Silk Road" (Turkey, Middle East, East Asia) and can affect any organ system

Causes

The exact cause is unknown, but the disease is thought to involve an abnormal immune response triggered by infection or other environmental factors in genetically susceptible people. The HLA-B51 gene is strongly associated with the disease. The immune system attacks the body's own blood vessels, causing inflammation, thrombosis, and tissue damage.

Risk Factors

- Ethnic background (Turkish, Middle Eastern, East Asian ancestry)
- HLA-B51 gene carriage
- Male sex (associated with more severe eye disease)
- Young adulthood, typically ages 20–40
- Family history of the disease

Signs and Symptoms

Common symptoms

- Recurrent painful mouth ulcers (aphthous ulcers)
- Painful genital ulcers
- Eye inflammation: uveitis, redness, pain, blurred vision, floaters
- Skin lesions, including acne-like bumps and nodules
- Joint pain and swelling (arthritis)
- Fatigue and fever

Emergency warning signs

- Sudden vision loss or severe eye pain requires urgent evaluation, as eye involvement can cause blindness
- Sudden headache, weakness, or confusion may indicate nervous system involvement
- Chest pain, shortness of breath, or coughing up blood may indicate vascular involvement

Progression and Stages

The disease follows a relapsing-remitting course with unpredictable flares and periods of improvement. It tends to be most severe in young men. Ocular disease, if untreated, causes progressive inflammation that can lead to scarring, glaucoma, and blindness. Vascular complications such as venous thrombosis and arterial aneurysm can occur and are potentially life-threatening.

Complications

- Permanent vision loss and blindness
- Central nervous system involvement (meningoencephalitis, stroke)
- Venous thrombosis, including deep vein and cerebral venous thrombosis
- Aneurysm or rupture of major arteries
- Gastrointestinal ulceration and bleeding
- Chronic joint damage and disability

Diagnosis

- Clinical diagnosis based on the characteristic pattern of symptoms (recurrent oral ulcers plus at least two of: genital ulcers, eye inflammation, skin lesions, or positive pathergy test)
- Pathergy test (skin response to a small needle prick)
- HLA-B51 genetic testing as a supporting clue
- Eye examination with slit lamp and fundoscopy
- Blood tests for inflammation (ESR, CRP) and imaging as indicated
- Exclusion of other causes of uveitis and vasculitis

Prevention

- There is no known prevention for Behçet's disease
- Early recognition and treatment reduce the risk of irreversible organ damage
- Regular ophthalmological follow-up to detect silent eye inflammation
- Managing cardiovascular risk factors

Treatment Options

- Topical or oral corticosteroids for mouth and genital ulcers
- Colchicine for mucosal and joint symptoms
- Immunosuppressants (azathioprine, cyclosporine, methotrexate) for moderate to severe disease
- Biologic agents (infliximab, adalimumab, secukinumab, interferon) for refractory or sight-threatening disease
- Anticoagulation for thrombosis (with caution) alongside immunosuppression
- Regular multidisciplinary care (ophthalmology, rheumatology, neurology)

Living with It

- Follow the treatment plan and attend regular specialist follow-up
- Protect vision with regular eye examinations and prompt reporting of symptoms
- Manage flares with the help of the care team
- Seek support from patient organizations
- Maintain a healthy lifestyle and manage stress

Outlook

With modern immunosuppressive and biologic therapy, the outlook has improved substantially. Many patients achieve good control, but the disease is lifelong and relapsing. Eye involvement remains the most serious complication; early aggressive treatment significantly reduces the risk of blindness.

Chapter 105

Bell's Palsy

Overview

Bell's palsy is an acute, idiopathic, unilateral facial nerve (cranial nerve VII) paralysis of lower motor neuron type and is the most common cause of acute facial palsy. The condition is thought to result from reactivation of herpes simplex virus type 1. Onset is typically abrupt, with complete facial weakness developing over hours to days. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience inability to close the affected eye, drooping mouth, hyperacusis, and loss of taste on the anterior two-thirds of the tongue. Headache and facial pain

Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of bell's palsy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, after excluding causes such as Ramsay Hunt syndrome, Lyme disease, and stroke. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Management with oral corticosteroids (prednisone) started within 72 hours of onset significantly improves outcomes

- Eye protection is essential to prevent corneal damage.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive, with most patients recovering fully within weeks to months. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 106

Benign Paroxysmal Positional Vertigo

Overview

non-cancerous paroxysmal positional feeling of spinning is the most common cause of peripheral feeling of spinning, resulting from displaced otoliths from the utricle into the semicircular canals, most commonly the posterior canal.

Causes

This condition happens because of movement of calcium carbonate crystals within the semicircular canals.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience brief episodes (less than 1 minute) of severe feeling of spinning triggered by head movement, with characteristic involuntary eye movements.

Emergency warning signs

- Severe or worsening symptoms of benign paroxysmal positional vertigo require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on the Dix-Hallpike test, which reproduces feeling of spinning and reveals torsional-upbeating involuntary eye movements. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on canalith repositioning maneuvers (Epley maneuver for posterior canal BPPV), which are highly effective. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well, although recurrence occurs in 20–50% of cases. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 107

Benign Prostatic Hyperplasia

Overview

non-cancerous prostatic abnormal increase in cells is the most common non-cancerous tumor in men, characterized by growth of stromal and epithelial cells in the transition zone of the prostate, causing urethral obstruction, affecting over 50% of men by age 60 and up to 90% by age 85.

Causes

This condition happens because of age-related hormonal changes driving prostatic growth.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience lower urinary tract symptoms (LUTS): urinary frequency, urgency, nocturia, weak stream, hesitancy, straining, incomplete emptying, and dribbling, with acute urinary retention possibly occurring.

Emergency warning signs

- Severe or worsening symptoms of benign prostatic hyperplasia require immediate medical evaluation

Progression and Stages

The right treatment depends on severity: watchful waiting for mild symptoms, alpha-blockers (tamsulosin, alfuzosin) for smooth muscle relaxation, 5-alpha-reductase inhibitors (finasteride, dutasteride) for gland shrinkage in larger prostates, and surgical intervention (TURP, holmium laser enucleation) for refractory cases. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves digital rectal examination (DRE) showing smooth, symmetrically enlarged prostate, American Urological Association Symptom Score (AUASS), PSA, urinalysis, post-void residual volume, and uroflowmetry. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 108

Benzodiazepine Overdose

Overview

Benzodiazepine overdose is a pharmaceutical overdose involving positive allosteric modulators of GABA_A receptors. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of excessive GABAergic inhibition producing CNS depression.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience drowsiness, slurred speech, loss of coordination, and coma
- Respiratory depression is mild in pure benzodiazepine overdose but is synergistic with ethanol, opioids, and other CNS depressants.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of benzodiazepine overdose require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Urine drug screens have limited utility.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Treatment typically involves supportive care (airway protection, respiratory support)
- Flumazenil, a GABA_A receptor antagonist, is reserved for pure benzodiazepine overdose with severe respiratory depression and is contraindicated in chronic benzodiazepine users or mixed ingestions due to risk of seizures. The outlook is generally positive with supportive care.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 109

Beta-Blocker Overdose

Overview

- Beta-blocker overdose (propranolol most lethal, also atenolol, metoprolol, sotalol) is a pharmaceutical overdose causing bradycardia, hypotension, and decreased myocardial contractility. Propranolol also has sodium channel blockade contributing to QRS widening and seizures
- Sotalol causes QT prolongation and torsades de pointes.

Causes

This condition happens because of beta-adrenergic receptor blockade.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bradyarrhythmias, low cardiac output, lung swelling, hypoglycemia, seizures, and altered mental status.

Emergency warning signs

- Severe or worsening symptoms of beta-blocker overdose require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include IV fluids, atropine, glucagon (5–10 mg IV bolus, then 1–10 mg/hour infusion) which increases cyclic AMP via non-adrenergic activation of adenylate cyclase, high-dose insulin euglycemia therapy, vasopressors, and calcium salts. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 110

Binge-Eating Disorder

Overview

Binge-eating disorder has a lifetime prevalence of 2–3% in the general population and is the most common eating disorder, more common in women but with a narrower sex ratio than anorexia and bulimia (3:2 female-to-male). This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of altered reward sensitivity (dopamine dysfunction in striatum), impulsivity, and emotional dysregulation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience recurrent episodes of binge eating (loss-of-control eating of objectively large amounts of food) without regular compensatory behaviors, associated with eating much more rapidly than normal, eating until uncomfortably full, eating large amounts when not physically hungry, eating alone due to embarrassment, and feeling disgusted/depressed/guilty afterward, occurring at least once a week for 3 months. BED is strongly associated with obesity (60–80% are obese) and metabolic syndrome.

Emergency warning signs

- Severe or worsening symptoms of binge-eating disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include CBT-E, IPT, and lisdexamfetamine (FDA-approved for moderate-to-severe BED). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is better than for anorexia or bulimia
- 50–70% achieve remission with structured treatment.

Chapter 111

Bipolar II Disorder

Overview

This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of similar pathophysiological mechanisms as Bipolar I. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience patients spending significantly more time in depressive episodes than hypomanic episodes, often leading to underdiagnosis.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of bipolar ii disorder require immediate medical evaluation

Progression and Stages

Bipolar II disorder has a lifetime prevalence of approximately 0.5%, characterized by one or more hypomanic episodes (distinct period of elevated, expansive, or irritable mood lasting at least 4 consecutive days, plus similar symptoms as mania but of lesser severity, without marked functional impairment or psychosis) and one or more major depressive episodes. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for the absence of full manic episodes, distinguishing it from Bipolar I.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include mood stabilizers (lithium, lamotrigine, valproate) and atypical antipsychotics (quetiapine, which is FDA-approved for bipolar II depression), with careful use of antidepressants with mood stabilizer coverage. Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person but generally better than Bipolar I. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 112

Bipolar I Disorder

Overview

Bipolar I disorder has a lifetime prevalence of approximately 1% worldwide, with equal distribution across sexes. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of dysregulation of intracellular signaling pathways (GSK-3, PKC, protein kinase A) and circadian rhythm disruption, with strong genetic factors (80–90% heritability). Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience at least one lifetime manic episode lasting at least 1 week (or any duration if hospitalization is required), characterized by elevated, expansive, or irritable mood plus increased goal-directed activity or energy, with at least three of the following (four if mood is only irritable): inflated self-esteem or grandiosity, decreased need for sleep, pressured speech, flight of ideas or racing thoughts, distractibility, psychomotor agitation, and excessive involvement in high-risk activities.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of bipolar i disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using clinical interview using DSM-5-TR criteria. Treatment for acute mania includes a mood stabilizer plus an atypical antipsychotic or benzodiazepine
- Lithium remains the gold-standard mood stabilizer with demonstrated antisuicidal effects, with other first-line treatments including valproate and atypical antipsychotics (olanzapine, quetiapine, risperidone, aripiprazole).
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system

- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with most patients requiring long-term maintenance therapy. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 113

Bietti's Crystalline Dystrophy

Overview

- Bietti's crystalline dystrophy (BCD) is a rare inherited eye disease causing progressive degeneration of the retina and accumulation of crystal-like deposits
- It is caused by mutations in the CYP4V2 gene, which plays a role in fat (lipid) metabolism
- It is more common in people of East Asian descent and typically begins in young adulthood

Causes

This condition happens because of mutations in the CYP4V2 gene, inherited in an autosomal recessive pattern. The gene normally helps the body process certain fatty acids; when it is defective, lipids accumulate abnormally in the retina and cornea, forming crystalline deposits and causing progressive death of retinal cells, particularly in the pigment epithelium and choroid.

Risk Factors

- Having two copies of a CYP4V2 mutation (one from each parent)
- East Asian (especially Chinese and Japanese) ancestry
- Family history of the condition
- Consanguineous parents increase the risk of recessive disease

Signs and Symptoms

Common symptoms

- Night blindness (difficulty seeing in dim light)
- Progressive loss of peripheral (side) vision
- Blurred central vision in later stages
- Glare and light sensitivity
- Crystals visible in the retina on eye examination, sometimes with similar deposits in the cornea

Emergency warning signs

- Rapidly worsening vision requires prompt evaluation to document progression and rule out treatable causes

Progression and Stages

Symptoms usually begin between ages 20 and 40 and progress slowly over decades. Night blindness and peripheral field loss appear first, followed by gradual constriction of the visual fields. Central vision is often preserved until later stages, but eventually significant visual impairment develops. The rate of progression varies between individuals.

Complications

- Severe visual field constriction (tunnel vision)
- Central vision loss in advanced disease
- Difficulty driving, reading, and working in dim light
- Impaired mobility and independence
- Psychological impact and reduced quality of life

Diagnosis

- Dilated fundus examination showing the characteristic retinal crystalline deposits and pigmentary changes
- Optical coherence tomography (OCT) showing retinal atrophy
- Electroretinography (ERG) to assess retinal function
- Visual field testing (perimetry)
- Corneal examination for crystals (at the limbus)
- Genetic testing to confirm CYP4V2 mutations

Prevention

- Genetic counseling for affected families
- Carrier testing and prenatal testing where the family mutation is known
- Regular monitoring to detect and treat complications
- Use of UV-protective eyewear
- Avoidance of smoking

Treatment Options

- There is currently no cure for Bietti's crystalline dystrophy
- Gene replacement therapy is under investigation in clinical trials
- Low-vision aids and rehabilitation

- Vitamin and dietary supplements are sometimes suggested but their benefit is unproven
- Treatment of any coexisting conditions such as cataracts
- Participation in research registries and trials

Living with It

- Work with low-vision services for magnification and mobility training
- Use assistive technology such as screen readers and talking devices
- Plan activities around remaining vision, especially for night travel
- Join support networks for inherited retinal disease
- Stay informed about advances in gene therapy research

Outlook

Bietti's crystalline dystrophy is progressive and currently incurable, and it gradually reduces vision over decades. The pace varies, and many people retain useful vision well into middle age. Research into gene therapy offers cautious hope for slowing or halting the disease in the future.

Chapter 114

Bladder Cancer

Overview

Bladder cancer is the fourth most common cancer in men, with 550,000 new cases/year worldwide. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

- Urothelial (transitional cell) carcinoma accounts for >90%
- Squamous cell carcinoma (3–5%) and adenocarcinoma (1–2%) are less common. This condition happens because of multifocal field cancerization with mutations in FGFR3 (low-grade) and p53/Rb (high-grade, CIS).

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)

- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience gross, painless hematuria (most common), microscopic hematuria, and irritative voiding symptoms with CIS. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of bladder cancer require immediate medical evaluation

Progression and Stages

Recovery varies by stage: 5-year survival is 96% for Ta, 70–90% for T1, 50–60% for T2, 30–50% for T3, and <15% for metastatic. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant

metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this based on cystoscopy (gold standard) with biopsy/removal, urine cytology, FISH (UroVysion), and CT urography. Staging distinguishes non-muscle-invasive (Ta, T1, CIS) from muscle-invasive (T2+) disease. Treatment for NMIBC includes TURBT with risk-adapted intravesical therapy (mitomycin C, BCG)

- BCG-unresponsive NMIBC requires radical cystectomy or pembrolizumab. MIBC doctors typically treat it with neoadjuvant cisplatin-based chemotherapy followed by radical cystectomy with urinary diversion
- Trimodal therapy is an option for patients unfit for cystectomy. Metastatic disease doctors typically treat it with platinum-based chemotherapy, checkpoint inhibitors, FGFR inhibitors (erdafitinib for FGFR3-mutant), and antibody-drug conjugates (enfortumab vedotin, sacituzumab govitecan).
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging

- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 115

Blastomycosis

Overview

Blastomycosis is a systemic fungal infection caused by *Blastomyces dermatitidis*, a dimorphic fungus endemic to the Ohio and Mississippi River Valleys and the Great Lakes region, with 1–2 cases per 100,000 population in endemic areas.

Causes

This condition happens because of conidia being inhaled, converting to broad-based budding yeasts in the alveoli, and disseminating hematogenously.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience lung blastomycosis: acute pneumonia or chronic pneumonia (resembles TB or lung cancer)
- Extrapulmonary blastomycosis: skin is the most common site (verrucous or ulcerative lesions), bone (lytic lesions, osteomyelitis), and genitourinary (prostatitis, epididymitis).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of blastomycosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using culture, direct examination of KOH wet mount (large, broad-based budding yeasts), and antigen testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include itraconazole for mild-moderate lung disease and liposomal amphotericin B for severe, disseminated, or CNS disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 116

Blepharitis

Overview

Blepharitis is a chronic inflammatory condition affecting the eyelid margins, characterized by redness, crusting, and irritation. It is one of the most common ophthalmic conditions encountered in clinical practice, affecting people of all ages. The condition is often associated with meibomian gland dysfunction, seborrheic dermatitis, and rosacea.

Classification

Cause Inflammatory / Infectious

Organ System Eyes

Pathophysiology Meibomian gland dysfunction with bacterial overgrowth and inflammation of eyelid margins

Duration Chronic (months to years)

Onset Gradual

Mode of Transmission Non-communicable

Clinical Course Chronic, relapsing-remitting

Severity Mild to Moderate

Extent of Disease Localized

Age Group All ages

Epidemiology Very common; estimated prevalence 37–47% in ophthalmology patients

ICD-11 Code 9A61.0

Causes

Anterior blepharitis involves inflammation at the base of the eyelashes, often from bacterial colonization (*Staphylococcus aureus*, *S. epidermidis*) or seborrheic dermatitis. Posterior blepharitis results from meibomian gland dysfunction with altered lipid secretion, creating an environment that promotes bacterial growth and inflammation. Demodex mite infestation contributes in some cases.

Risk Factors

- Seborrheic dermatitis
- Rosacea
- Meibomian gland dysfunction
- Contact lens wear
- Dry eye syndrome
- Advanced age
- Poor eyelid hygiene
- Diabetes mellitus

Signs and Symptoms

Early symptoms

- Early manifestations of blepharitis may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Chronic redness and swelling of the eyelid margins
- Crusting, flaking, and collarettes at the base of eyelashes
- Itching, burning, and foreign body sensation
- Tearing and dry eye symptoms
- Photophobia and blurred vision
- Misdirected or loss of eyelashes
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of blepharitis may occur

Emergency warning signs

- Severe or worsening symptoms of blepharitis require immediate medical evaluation

Progression and Stages

Blepharitis follows a chronic relapsing-remitting course with exacerbations triggered by stress, poor hygiene, or underlying skin conditions. Without consistent management, inflammation can lead to structural changes in the eyelid margin, including scarring, lash

loss (madarosis), and trichiasis.

Complications

- Recurrent hordeola and chalazia
- Dry eye syndrome from tear film disruption
- Conjunctival injection and keratitis
- Corneal ulceration and scarring (rare)
- Trichiasis (inward-turning lashes)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis by slit-lamp examination of eyelid margins
- Assessment of meibomian gland expression and secretion quality
- Lid margin swab for culture in refractory cases
- Demodex mite evaluation by lash epilation and microscopy
- Lab tests to check for underlying conditions

Prevention

- Daily eyelid hygiene with warm compresses and lid scrubs
- Manage underlying skin conditions (seborrhea, rosacea)
- Regular eyelid massage to express meibomian glands
- Avoid eye makeup contamination and replace products regularly
- Go for regular check-ups so any problems are caught early

Treatment Options

- Warm compresses applied daily for 5–10 minutes followed by lid massage
- Eyelid margin scrubs with diluted baby shampoo or commercial lid wipes
- Topical antibiotic ointments (erythromycin, bacitracin) for anterior blepharitis
- Topical cyclosporine or corticosteroids for inflammatory component
- Oral doxycycline or azithromycin for meibomian gland dysfunction
- Omega-3 fatty acid supplementation
- Artificial tears for associated dry eye
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider

- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Blepharitis is a chronic condition that typically requires long-term management rather than cure. With consistent eyelid hygiene and appropriate treatment, most patients achieve adequate symptom control. The condition tends to recur and requires ongoing maintenance therapy to prevent complications and preserve ocular surface health.

Chapter 117

Blepharospasm

Overview

- Blepharospasm is an involuntary, forceful, and often progressive closing of the eyelids caused by abnormal contraction of the eyelid muscles
- It is a form of focal dystonia and can range from occasional increased blinking to severe, functionally disabling spasms
- It typically begins in middle age and affects women more than men

Causes

The cause of primary (essential) blepharospasm is unknown, but it is thought to involve abnormal signal processing in the basal ganglia of the brain. It can be worsened by bright light, stress, fatigue, and eye strain. Secondary blepharospasm can follow eye disease, dry eyes, certain medications, stroke, or other neurological disorders.

Risk Factors

- Female sex
- Age between 40 and 60 years
- Family history of dystonia
- Eye irritation, dry eyes, or light sensitivity
- Stress and fatigue
- Neurological conditions (Parkinson disease, stroke, multiple sclerosis)

Signs and Symptoms

Common symptoms

- Increased frequency of blinking
- Involuntary, forceful eyelid closure that is difficult to stop
- Sensitivity to bright light (photophobia)
- Dry or irritated eyes
- Spasms that worsen with stress, fatigue, or concentration
- In severe cases, functional blindness because the eyes cannot be opened

Emergency warning signs

- Sudden eyelid drooping, double vision, or facial weakness may indicate a more serious neurological problem and requires evaluation

Progression and Stages

Blepharospasm often starts with increased blinking and eye irritation and progresses over months to years to more forceful and frequent spasms. It may spread to involve other facial muscles. The severity fluctuates and can be triggered by specific activities. It is not progressive to a life-threatening degree, but it can become functionally disabling.

Complications

- Functional visual impairment (inability to keep eyes open)
- Difficulty driving, reading, and working
- Social embarrassment and anxiety
- Depression and reduced quality of life
- Injury from falls during spasm episodes
- Side effects of treatment

Diagnosis

- Clinical diagnosis based on history and observation of the spasms
- Neurological examination to rule out other movement disorders
- Eye examination to exclude eye surface disease or ocular irritation
- Imaging (MRI) in atypical or secondary cases
- Electromyography (EMG) may support the diagnosis

Prevention

- Primary blepharospasm is not preventable
- Managing eye irritation and dry eyes
- Reducing stress and fatigue
- Using tinted lenses for light sensitivity
- Early recognition and referral to a neurologist

Treatment Options

- Botulinum toxin injections into the eyelid muscles are the first-line and most effective treatment
- Artificial tears and lubricants for associated dry eyes
- Photochromic or FL-41 tinted lenses for light sensitivity

- Oral medications (benzodiazepines, anticholinergics) in some cases with limited benefit
- Biofeedback, relaxation, and sensory "tricks" (e.g., touching the face, humming) to ease spasms
- Surgical myectomy (removal of eyelid muscles) in severe, refractory cases

Living with It

- Schedule botulinum toxin injections regularly as recommended
- Use tinted lenses and sunglasses outdoors
- Manage stress through relaxation and adequate sleep
- Seek support from patient organizations
- Discuss functional accommodations with employers

Outlook

Blepharospasm is not life-threatening, and botulinum toxin injections provide effective relief for the vast majority of patients, with repeat treatments every 3–4 months. Although there is no cure, most people maintain good function and quality of life with treatment.

Chapter 118

Body Dysmorphic Disorder

Overview

Body dysmorphic disorder has a prevalence of 1.7–2.4% in the general population, with higher rates in dermatology and cosmetic surgery settings (8–15%).

Causes

This condition happens because of distorted body image perception.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a preoccupation with one or more perceived defects or flaws in physical appearance that are not observable or appear only slightly noticeable to others, with repetitive behaviors (mirror checking, excessive grooming, skin picking, reassurance seeking) or mental acts (comparing appearance with others) in response to the appearance concerns, with the preoccupation being time-consuming (at least 1 hour per day). Common concerns involve the skin, hair, nose, eyes, teeth, and weight or muscle size (in muscle dysmorphia).

Emergency warning signs

- Severe or worsening symptoms of body dysmorphic disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include SSRIs at high doses (similar to OCD) and CBT tailored for BDD. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is chronic without treatment
- Cosmetic procedures generally do not help.

Chapter 119

Borderline Personality Disorder (BPD)

Overview

Borderline personality disorder has a prevalence of 1–2.5% in the general population and 10–20% in outpatient psychiatric settings. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of a hyperreactive amygdala (heightened emotional sensitivity), impaired prefrontal regulation of emotion, reduced hippocampal and amygdala volumes, and altered oxytocin and endogenous opioid systems, with adverse childhood experiences being strongly associated. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a pervasive pattern of instability in interpersonal relationships, self-image, affect, and marked impulsivity, with DSM-5-TR criteria requiring at least five of the following: frantic efforts to avoid real or imagined abandonment, unstable and intense interpersonal relationships (alternating between idealization and devaluation—splitting), marked identity disturbance, impulsivity, recurrent suicidal behavior or non-suicidal self-injury (NSSI), affective instability, chronic feelings of emptiness, inappropriate anger, and transient stress-related paranoid ideation or dissociative symptoms.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of borderline personality disorder (bpd) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Treatment options include dialectical behavior therapy (DBT) as first-line psychotherapy, mentalization-based treatment (MBT), and symptom-targeted pharmacotherapy (SSRIs, mood stabilizers, low-dose atypical antipsychotics). outlook: 80% achieve symptomatic remission over 10-year follow-up
- Suicide risk is 5–10%.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 120

Botulism (*Clostridium botulinum*)

Overview

Botulism is a rare, potentially fatal neuroparalytic disease caused by botulinum neurotoxin (BoNT) produced by *Clostridium botulinum*, with approximately 200 cases per year in the US, including foodborne, infant botulism (most common form in US), wound botulism (injection drug users), and iatrogenic forms.

Causes

This condition happens because of BoNT (a zinc-dependent metalloproteinase) cleaving SNARE proteins (SNAP-25, synaptobrevin, syntaxin) at the presynaptic neuromuscular junction, preventing acetylcholine release and causing flaccid paralysis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience symmetric, descending flaccid paralysis beginning with cranial nerve palsies (diplopia, slurred speech, dysphonia, difficulty swallowing, ptosis) and progressing to descending symmetric weakness of neck, trunk, and extremities, with no sensory deficits and no fever.

Emergency warning signs

- Severe or worsening symptoms of botulism (*clostridium botulinum*) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, confirmed by detection of BoNT in serum, stool, or food (mouse bioassay or PCR). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment options include equine heptavalent botulism antitoxin (HBAT) for adults or human botulinum immune globulin (BabyBIG) for infants, with supportive care including mechanical ventilation for respiratory failure. outlook: mortality < 5% with modern intensive care.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 121

Brain Abscess

Overview

Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

- A brain abscess is a focal collection of pus within the brain parenchyma, resulting from hematogenous spread, contiguous infection (sinusitis, otitis media, dental infection), or direct inoculation (trauma, neurosurgery)
- Common organisms include *Streptococcus* species, *Staphylococcus aureus*, and anaerobes.

Causes

This condition happens because of bacterial infection leading to focal suppuration within brain tissue. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience headache, fever, weakness, numbness, or problems with movement, seizures, and altered consciousness. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of brain abscess require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock

- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this based on MRI with contrast showing a ring-enhancing lesion with surrounding swelling and mass effect. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification

- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment focuses on targeted intravenous antibiotics (guided by culture when possible) and, for larger abscesses (greater than 2.5 cm), neurosurgical removal by suction or removal. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

The outlook has improved significantly with modern imaging and treatment. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 122

Brain Tumors in Children

Overview

This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

- Brain tumors are the most common solid tumors in children and the second most common pediatric malignancy after leukemia. They differ significantly from adult brain tumors in histology and location. Medulloblastoma is an embryonal tumor of the cerebellum, most common in children aged 3–8 years, presenting with symptoms of increased intracranial pressure, loss of coordination, and cranial nerve deficits
- Treatment usually involves surgical removal, craniospinal radiation, and chemotherapy, with 5-year survival of approximately 70–80%. Ependymoma arises from ependymal cells lining the ventricles, most commonly in the posterior fossa
- Treatment typically involves maximal safe surgical removal followed by focal radiation, with 5-year survival of 50–70%. Brainstem glioma, most commonly diffuse intrinsic pontine glioma (DIPG), is a devastating high-grade glioma of the pons in children aged 5–10 years presenting with rapidly progressive cranial neuropathies, loss of coordination, and long-tract signs
- Radiation provides temporary palliation, and median survival is less than 1 year.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Headache and facial pain
- Weakness or paralysis of muscles
- Numbness, tingling, or abnormal sensations
- Vision changes including blurred or double vision
- Difficulty with balance and coordination
- Cognitive changes (memory loss, confusion, difficulty concentrating)
- Speech and language difficulties
- Seizures or involuntary movements
- Dizziness and vertigo
- Fatigue and sleep disturbances
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of brain tumors in children require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph

node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Pilocytic astrocytoma is the most common glioma in children, a low-grade (WHO Grade I) tumor typically occurring in the cerebellum, optic pathway, hypothalamus, and brainstem
- Complete surgical removal is curative, with 90–95% survival.

Complications

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 123

Breast Cancer

Overview

Breast cancer is the most commonly diagnosed cancer worldwide (2.3 million new cases annually) and the second leading cause of cancer-related death in women. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

The majority of breast cancers (70–80%) are invasive ductal carcinoma (not otherwise specified), with other types including invasive lobular carcinoma (10–15%), inflammatory breast cancer, Paget’s disease of the nipple, and male breast cancer (<1%). This condition happens because of cancerous transformation of breast epithelial cells. Screening: mammography is recommended biennially for women ages 50–74 (USPSTF) or annually starting at age 40 (American College of Radiology), with earlier and more intensive screening for high-risk individuals including breast MRI. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience a painless breast mass, skin changes, nipple discharge, and axillary lymphadenopathy. Diagnosis: diagnostic mammography with ultrasound
- Core needle biopsy for histopathological evaluation. Molecular subtyping by immunohistochemistry: estrogen receptor (ER), progesterone receptor (PR), HER2, and Ki-67, with four molecular subtypes: luminal A (ER+/PR+, HER2-, low Ki-67—best outlook), luminal B (ER+, HER2+ or high Ki-67), HER2-enriched (ER-, PR-, HER2+), and triple-negative/basal-like (ER-, PR-, HER2—worst outlook). Staging uses the TNM system.
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of breast cancer require immediate medical evaluation

Progression and Stages

The right treatment depends on stage, molecular subtype, and patient factors: Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Radiation therapy: Standard after breast-conserving surgery; post-mastectomy radiation for tumors >5 cm, node-positive disease, or close margins. **Systemic therapy:** Endocrine therapy (tamoxifen for premenopausal; aromatase inhibitors for postmenopausal) for ER+ disease; anti-HER2 therapy (trastuzumab, pertuzumab, T-DM1, T-DXd) for HER2+ disease; chemotherapy (anthracyclines, taxanes) for high-risk disease; CDK4/6 inhibitors for advanced ER+ disease; PARP inhibitors for BRCA-mutated tumors; immune checkpoint inhibitors for triple-negative disease with PD-L1 expression.

- **Surgery:** Breast-conserving surgery (lumpectomy) with sentinel lymph node biopsy, or mastectomy (simple, modified radical, or radical). Oncoplastic techniques optimize cosmetic outcomes.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type

(e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 124

Bronchiectasis

Overview

- Bronchiectasis is a permanent, abnormal dilation of the bronchi and bronchioles caused by chronic inflammation and recurrent infection
- Etiologies include cystic scarring, primary ciliary dyskinesia, immunodeficiency, prior severe respiratory infections, and idiopathic causes.

Causes

This condition happens because of impaired mucociliary clearance, mucus stasis, chronic bacterial infection, and progressive airway destruction.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience chronic productive cough with copious purulent sputum, recurrent respiratory infections, hemoptysis, and progressive shortness of breath.

Emergency warning signs

- Severe or worsening symptoms of bronchiectasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using high-resolution CT showing tram-track opacities, signet ring sign, and dilated bronchi. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment options include airway clearance techniques, targeted antibiotics for exacerbations and chronic suppression, bronchodilators, and Treatment for underlying causes
- Macrolide prophylaxis reduces exacerbation frequency.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause and frequency of exacerbations. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 125

Bronchiolitis (RSV)

Overview

Bronchiolitis is the most common lower respiratory tract infection in infants, caused most frequently by respiratory syncytial virus (RSV). It affects nearly all children by age 2, with peak incidence at 2–6 months.

Causes

This condition happens because of inflammation, swelling, and tissue death of the bronchiolar epithelium, leading to airway obstruction, air trapping, and atelectasis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience coryza, cough, tachypnea, wheezing, crackles, and respiratory distress
- Apnea is a presenting sign in young infants (<2 months).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of bronchiolitis (rsv) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- RSV rapid antigen testing or PCR confirms the cause.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Palivizumab (RSV monoclonal antibody) is used for prophylaxis in high-risk infants.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves primarily supportive: supplemental oxygen, nasal suctioning, and hydration. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Most people recover well with supportive care.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 126

Bronchopulmonary Dysplasia (BPD)

Overview

Bronchopulmonary dysplasia (BPD) is a chronic lung disease of prematurity, defined as the need for supplemental oxygen or respiratory support at 36 weeks postmenstrual age. Incidence is 30–50% in extremely preterm infants.

Causes

This condition happens because of the combined effects of lung immaturity, mechanical ventilation, oxygen toxicity, and inflammation, leading to impaired alveolarization and dysregulated vascular development.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience persistent oxygen requirement, tachypnea, retractions, and wheezing.

Emergency warning signs

- Severe or worsening symptoms of bronchopulmonary dysplasia (bpd) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- The outlook varies from person to person
- Long-term complications include reactive airway disease, lung high blood pressure, and increased risk of respiratory infections.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include optimizing nutrition (high-calorie feeds), fluid restriction, diuretics, bronchodilators, and systemic corticosteroids in severe cases. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-

care measures Physical therapy and rehabilitation Surgical intervention when indicated
Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 127

Brucellosis

Overview

Brucellosis is a zoonotic infection caused by gram-negative coccobacilli of the genus *Brucella*, endemic in the Mediterranean basin, Middle East, Central Asia, Africa, and Latin America, with 500,000 new cases annually, and transmission through ingestion of unpasteurized dairy products or direct contact with infected animal tissues.

Causes

This condition happens because of *Brucella* surviving and replicating within macrophages by inhibiting phagolysosome fusion and producing a type IV secretion system that modulates the host immune response.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience acute brucellosis (80%): undulant fever (evening fever spikes that wax and wane), profuse sweating, chills, malaise, joint pain, muscle pain, and back pain, with focal disease including spondylitis, sacroiliitis, peripheral arthritis, epididymo-orchitis, neurobrucellosis, and endocarditis (leading cause of mortality).

Emergency warning signs

- Severe or worsening symptoms of brucellosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using blood culture, bone marrow culture, serology, and PCR. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include doxycycline plus rifampin for 6 weeks (standard), with longer therapy for focal disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well for most cases
- Mortality < 1% except for endocarditis (up to 20%).

Chapter 128

Brugada Syndrome

Overview

- Brugada syndrome is an inherited sodium channelopathy (SCN5A mutation most common) characterized by an ECG pattern of coved ST-segment elevation in the right precordial leads (V1–V2) and an increased risk of ventricular fibrillation and sudden cardiac death, particularly during sleep or at rest
- It is more common in men and in Southeast Asian populations.

Causes

This condition happens because of mutations in cardiac sodium channels leading to abnormal repolarization. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience syncope, nocturnal agitated resuscitation, or sudden cardiac death
- Patients may be asymptomatic.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of brugada syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on ECG findings, with provocation testing using sodium channel blockers (ajmaline, flecainide) to unmask the pattern when necessary.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on implantable cardioverter-defibrillator placement for high-risk patients
- Fever and certain drugs should be avoided as they can exacerbate the ECG abnormality.

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well in low-risk patients and improved with implantable cardioverter-defibrillator in high-risk patients.

Chapter 129

Budd-Chiari Syndrome

Overview

Budd-Chiari syndrome is a liver venous outflow obstruction at any level from the small liver veins to the junction of the inferior vena cava and right atrium. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition results from disruption of normal budd-chiari syndrome function.

- The most common cause is myeloproliferative tumors (particularly polycythemia vera)
- Other causes include antiphospholipid syndrome, paroxysmal nocturnal hemoglobinuria, and oral contraceptive use. This condition happens because of obstruction of liver venous outflow, leading to liver congestion, sinusoidal high blood pressure, and hepatocyte reduced blood flow.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms can range from asymptomatic to fulminant liver failure
- Classic features include acute abdominal pain, hepatomegaly, and ascites.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of budd-chiari syndrome require immediate medical evaluation

Progression and Stages

Your outlook depends on the cause and severity. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using Doppler ultrasound, CT, or MR venography showing absent or abnormal liver venous flow.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Treatment options include anticoagulation, catheter-directed thrombolysis, TIPS for decompression, and liver transplantation for progressive liver failure. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 130

Bulimia Nervosa

Overview

Bulimia nervosa has a lifetime prevalence of 1–1.5% in women and 0.5% in men, with an age of onset in late adolescence to early adulthood. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of altered reward circuitry, deficits in inhibitory control, and interoceptive dysfunction. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of bulimia nervosa require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Medical complications include electrolyte disturbances (hypokalemic hypochloremic metabolic alkalosis), Russell's sign, parotid gland enlargement, dental erosion, Mallory-Weiss tears, esophageal rupture, and ipecac cardiomyopathy.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

You may experience recurrent episodes of binge eating (eating a large amount of food in a discrete period with a sense of loss of control) followed by recurrent inappropriate compensatory behaviors to prevent weight gain (self-induced vomiting, laxative/diuretic misuse, fasting, excessive exercise), occurring at least once a week for 3 months, with self-evaluation unduly influenced by body shape and weight.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include CBT-E (first-line), IPT (second-line), and SSRI pharmacotherapy (high-dose fluoxetine 60 mg/day is FDA-approved). outlook: 50–60% achieve recovery
- Relapse rates are moderate.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 131

Bullous Pemphigoid

Overview

Bullous pemphigoid (BP) is the most common autoimmune blistering disease, predominantly affecting the elderly (peak incidence 70–80 years).

Causes

This condition happens because of IgG autoantibodies targeting BP180 and BP230, components of hemidesmosomes at the dermal-epidermal junction, causing subepidermal blister formation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience tense blisters on an erythematous or urticarial base, itching, and predominance on the trunk and flexural surfaces
- Pain or discomfort in the affected area

Less common symptoms

- Mucosal involvement is uncommon (10–30%).

Emergency warning signs

- Severe or worsening symptoms of bullous pemphigoid require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves skin biopsy showing subepidermal split with eosinophilic infiltrate, direct immunofluorescence showing linear IgG and C3 at the basement membrane zone, and anti-BP180/BP230 antibodies by ELISA. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include potent topical corticosteroids (clobetasol, first-line for localized and generalized disease), systemic corticosteroids for extensive disease, and steroid-sparing agents (doxycycline, azathioprine, mycophenolate, rituximab). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-

care measures Physical therapy and rehabilitation Surgical intervention when indicated
Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable but may be chronic. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 132

Bursitis

Overview

Bursitis is inflammation of a bursa, a synovial fluid-filled sac that reduces friction between bones and overlying soft tissues (tendons, muscles, skin). The most commonly affected bursae include the subacromial (shoulder), olecranon (elbow), prepatellar (knee—"housemaid's knee"), and trochanteric (hip—"weaver's bottom").

Causes

This condition happens because of mechanical irritation or infection of the bursa.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience localized pain, swelling, tenderness, and sometimes redness of the skin and warmth over the affected bursa
- The overlying joint typically has a full range of motion, distinguishing bursitis from arthritis. Septic bursitis is more common in the olecranon and prepatellar bursae, often caused by *S. aureus*, presenting with more intense inflammation, fever, and systemic symptoms. Diagnosis is primarily clinical
- Ultrasound can confirm bursal fluid collection. removal by suction of bursal fluid is indicated when septic bursitis is suspected. Treatment for aseptic bursitis includes rest, ice, NSAIDs, and corticosteroid injection
- Septic bursitis requires bursal removal by suction, incision and removal of fluid if needed, and antibiotics.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of bursitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 133

COVID-19

Overview

Coronavirus disease 2019 (COVID-19) is an infectious disease caused by severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2), first identified in Wuhan, China in December 2019 and declared a pandemic by the WHO in March 2020. SARS-CoV-2 is a betacoronavirus that uses the angiotensin-converting enzyme 2 receptor for cell entry.

Causes

This condition happens because of viral infection causing a spectrum of illness from asymptomatic infection to severe pneumonia with acute respiratory distress syndrome, multi-organ failure, and death. Transmission occurs primarily through respiratory droplets and aerosols, with an incubation period of 2–14 days.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of covid-19 require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

You may experience fever, cough, fatigue, muscle pain, loss of smell or ageusia, headache, and shortness of breath

- Severe disease features hypoxemia, bilateral lung infiltrates, lymphopenia, elevated inflammatory markers, and cytokine storm
- Complications include ARDS, thromboembolism, myocarditis, acute kidney injury, neurological manifestations, and multisystem inflammatory syndrome in children.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using RT-PCR testing or rapid antigen tests. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Your outlook depends on age, comorbidities, and disease severity
- Vaccination has significantly reduced severe disease and death.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- The right treatment depends on severity: supportive care for mild disease
- Dexamethasone, remdesivir, baricitinib, tocilizumab, and anticoagulation for hospitalized patients
- Long COVID is marked by persistent symptoms lasting weeks to months after acute infection.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 134

Cadmium Toxicity

Overview

Cadmium is a heavy metal used in batteries, pigments, electroplating, and fertilizers, with a biological half-life of 15–30 years.

Causes

This condition results from disruption of normal cadmium toxicity function.

- This condition happens because of cadmium accumulating in the kidneys and bone, impairing kidney tubular reabsorption and causing osteomalacia. Acute inhalation exposure causes chemical pneumonitis and lung swelling. Chronic exposure causes kidney tubular damage, Itai-Itai disease (severe osteomalacia, bone pain, and fractures with kidney tubular dysfunction), high blood pressure, and lung cancer. Diagnosis: 24-hour urine and blood cadmium levels. Treatment: removal from exposure
- Chelation is generally ineffective and BAL is contraindicated.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cadmium toxicity require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on chronicity of exposure. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 135

Calcium Channel Blocker Overdose

Overview

Calcium channel blocker (CCB) overdose (verapamil, diltiazem, nifedipine, amlodipine, and other dihydropyridines) is a highly lethal pharmaceutical overdose. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of CCBs inhibiting L-type calcium channels in cardiac myocytes, vascular smooth muscle, and pancreatic beta cells, leading to negative chronotropy, dromotropy, and inotropy with vasodilation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience hypotension, bradycardia (or reflex tachycardia with dihydropyridines), atrioventricular block, junctional rhythms, asystole, hyperglycemia, metabolic acidosis, and altered mental status.

Emergency warning signs

- Severe or worsening symptoms of calcium channel blocker overdose require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include aggressive fluid resuscitation, calcium salts to overcome calcium channel blockade, and high-dose insulin euglycemia therapy (the most effective treatment). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is guarded
- Vasopressors and ECMO may be needed.

Chapter 136

Calcium Pyrophosphate Deposition Disease

Overview

Calcium pyrophosphate deposition disease (CPPD, formerly pseudogout) is a crystal-induced arthropathy resulting from the deposition of calcium pyrophosphate dihydrate (CPP) crystals in articular cartilage (chondrocalcinosis) and synovium. The condition occurs predominantly in elderly individuals (> 60 years) and is linked to hyperparathyroidism, hemochromatosis, hypomagnesemia, and hypothyroidism.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience acute monoarthritis or oligoarthritis resembling gout, most commonly affecting the knee, wrist, or symphysis pubis, with swelling, warmth, severe pain, and redness of the skin.

Emergency warning signs

- Severe or worsening symptoms of calcium pyrophosphate deposition disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Your doctor confirms the diagnosis with synovial fluid removal by suction showing weakly positively birefringent, rhomboid-shaped CPP crystals under polarized light microscopy
- Radiographs demonstrate chondrocalcinosis (linear calcium buildup of articular cartilage). Treatment for acute attacks includes intra-articular corticosteroid injection, oral NSAIDs, or colchicine.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good for acute control. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 137

Calcium and Phosphorus Disorders

Overview

- Calcium and phosphorus disorders encompass a group of mineral disorders resulting from dysregulation of calcium and phosphorus homeostasis, which is tightly regulated by parathyroid hormone (PTH), vitamin D, and calcitonin. Hypocalcemia (total calcium less than 8.5 mg/dL or ionized calcium less than 4.2 mg/dL) results from vitamin D deficiency, hypoparathyroidism, chronic kidney disease, hypomagnesemia, and medications
- You may experience neuromuscular irritability (perioral paresthesias, Chvostek sign, Trousseau sign, carpopedal spasm, tetany), prolonged QT interval, seizures, and laryngospasm. Hypercalcemia (total calcium greater than 10.5 mg/dL) is most commonly due to primary hyperparathyroidism or malignancy
- You may experience “bones, stones, groans, and psychiatric overtones”—bone pain, nephrolithiasis, polyuria, polydipsia, constipation, nausea, confusion, and shortened QT interval
- Severe hypercalcemia (greater than 13 mg/dL) is a medical emergency requiring IV fluids, calcitonin, bisphosphonates, and treatment of the underlying cause. Hypophosphatemia (phosphate less than 2.5 mg/dL) causes muscle weakness, rhabdomyolysis, and impaired bone mineralization
- Hyperphosphatemia (phosphate greater than 4.5 mg/dL) is most common in chronic kidney disease, causing secondary hyperparathyroidism, vascular calcium buildup, and kidney osteodystrophy.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of calcium and phosphorus disorders require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 138

Candida auris

Overview

Causes

This condition happens because of *C. auris* forming biofilms on medical devices, with mechanisms of antifungal resistance including mutations in ERG11 (azoles), FKS1 (echinocandins), and overexpression of efflux pumps.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience colonization often being asymptomatic, with invasive disease including candidemia, wound infections, and deep-seated candidiasis.

Emergency warning signs

- Severe or worsening symptoms of candida auris require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for MALDI-TOF mass spectrometry or DNA sequencing, as standard biochemical methods misidentify *C. auris*. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include echinocandins as first-line, with central line removal essential. outlook: mortality 30–60%. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 139

Candidiasis (Invasive / Systemic)

Overview

Invasive candidiasis encompasses candidemia (bloodstream infection) and deep-seated candidiasis (abscesses, endocarditis, endophthalmitis, osteomyelitis, meningitis), and is the most common healthcare-associated fungal infection, with *Candida* species being the fourth most common cause of bloodstream infections in hospitalized patients.

Causes

This condition happens because of *Candida* invading through breach of mucocutaneous barriers, adhering to endothelium, and forming biofilms on medical devices.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience candidemia: fever refractory to broad-spectrum antibiotics, hypotension, and sepsis, often without localizing signs.

Emergency warning signs

- Severe or worsening symptoms of candidiasis (invasive / systemic) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using blood culture and (1,3)- β -D-glucan (BDG) assay. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include echinocandins (caspofungin, micafungin, anidulafungin) as first-line, with central line removal being essential. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is guarded, with significant mortality in ICU patients. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 140

Cannabis Toxicity and Cannabinoid Hyperemesis Syndrome

Overview

Cannabis is the most commonly used illicit substance worldwide. Acute intoxication produces euphoria, relaxation, impaired short-term memory, altered judgment, conjunctival injection, tachycardia, and increased appetite. Severe effects include acute psychosis, panic attacks, and in high doses (especially edibles), severe sedation, loss of coordination, and hypotension. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cannabis toxicity and cannabinoid hyperemesis syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Cannabis hyperemesis syndrome (CHS) is a complication of chronic heavy cannabis use, characterized by cyclic episodes of severe nausea, vomiting, and abdominal pain, relieved by compulsive hot showers or baths.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam. Treatment: cessation of cannabis is curative
- For acute episodes, hot showers, topical capsaicin cream, benzodiazepines, haloperidol, metoclopramide, and ondansetron.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with cannabis abstinence.

Chapter 141

Carbon Monoxide Poisoning

Overview

Carbon monoxide (CO) is an odorless, colorless gas produced by incomplete combustion of carbon-based fuels. This condition results from acute injury or exposure to harmful agents.

Causes

This condition results from disruption of normal carbon monoxide poisoning function.

- This condition happens because of CO binding to hemoglobin with 240 times greater affinity than oxygen, forming carboxyhemoglobin (COHb), which shifts the oxygen dissociation curve to the left, impairing oxygen delivery and utilization
- CO also binds to myoglobin and cytochrome c oxidase.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: headache (most common), nausea, dizziness, confusion, shortness of breath, blurred vision, loss of consciousness, and seizures
- Severe poisoning causes coma, respiratory failure, cardiovascular collapse, and death. nervous system sequelae include acute encephalopathy and delayed neuropsychiatric syndrome (DNS).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of carbon monoxide poisoning require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using COHb level. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment: 100% normobaric oxygen
- Hyperbaric oxygen therapy (HBOT) is indicated for severe poisoning and may prevent DNS.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with prompt treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 142

Cardiac Tamponade

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Cardiac tamponade is the accumulation of pericardial fluid under pressure, causing compression of the heart and impaired cardiac filling, leading to reduced cardiac output and obstructive shock
- Causes include malignancy (most common in developed countries), trauma, uremia, infection, aortic dissection, and post-procedural causes.

Causes

This condition happens because of rapid or excessive pericardial fluid accumulation. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)

- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience Beck's triad (hypotension, muffled heart sounds, and jugular venous distension) and pulsus paradoxus. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of cardiac tamponade require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using echocardiography, which shows pericardial effusion with diastolic collapse of the right atrium and right ventricle. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Treatment typically involves emergent pericardiocentesis or surgical pericardiotomy. Management includes lifestyle modifications, pharmacotherapy targeting the underlying mechanisms, and interventional procedures when indicated. Long-term adherence to treatment is essential for preventing disease progression. Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and throm-

thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Your outlook depends on the underlying cause and timeliness of intervention. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 143

Carpal Tunnel Syndrome

Overview

Carpal tunnel syndrome is the most common entrapment neuropathy, caused by compression of the median nerve as it passes through the carpal tunnel in the wrist. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of mechanical compression of the median nerve within the carpal tunnel. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Family history of the genetic condition
- Consanguineous parents (for recessive disorders)
- Advanced parental age (for new mutations)
- Carrier status in parents
- Ethnic background (some conditions)
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of carpal tunnel syndrome require immediate medical evaluation

Progression and Stages

You may experience numbness and tingling in the median nerve distribution (thumb, index, middle, and radial half of the ring finger), nocturnal pain, and weakness of the thenar muscles in advanced cases. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on clinical examination (Phalen's test, Tinel's sign) and nerve conduction studies confirming delayed median nerve conduction across the wrist. Management begins with wrist splinting (especially at night), ergonomic modifications, and corticosteroid injection
- Surgical carpal tunnel release is indicated for severe or refractory cases.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate treatment.

Chapter 144

Celiac Disease

Overview

Celiac disease is an autoimmune enteropathy triggered by ingestion of gluten (gliadin and glutenin proteins found in wheat, barley, and rye) in genetically susceptible individuals (HLA-DQ2 or HLA-DQ8). Prevalence is approximately 1% worldwide.

Causes

This condition happens because of an immune response causing villous shrinkage or wasting, crypt abnormal increase in cells, and intraepithelial lymphocytosis in the small intestinal mucosa, primarily affecting the duodenum and jejunum.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features vary widely: classical symptoms in children include chronic diarrhea, failure to thrive, and abdominal distension
- In adults, diarrhea, weight loss, iron deficiency anemia, and osteoporosis. Dermatitis herpetiformis is the cutaneous manifestation. Many patients are asymptomatic or have extraintestinal manifestations (fatigue, neurological symptoms, liver disease, infertility).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of celiac disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- To make the diagnosis, doctors look for serological testing (anti-tissue transglutaminase IgA, anti-endomysial antibodies) and duodenal biopsy showing villous shrinkage or wasting, crypt abnormal increase in cells, and increased intraepithelial lymphocytes
- HLA testing helps exclude the diagnosis.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves a strict lifelong gluten-free diet, which leads to mucosal recovery and symptom resolution. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with dietary compliance. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 145

Cellulitis

Overview

Cellulitis is a bacterial infection of the dermis and subcutaneous tissue, most commonly caused by *Streptococcus pyogenes* (group A strep) and *Staphylococcus aureus*. Predisposing factors include skin barrier disruption (fissures, insect bites, surgical wounds), swelling, chronic venous insufficiency, and immunosuppression. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of bacterial entry through disrupted skin barriers. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience a well-demarcated area of redness of the skin, warmth, swelling, tenderness, and sometimes lymphangitis and regional lymphadenopathy
- The lower extremities are most commonly affected.
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of cellulitis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

Complications include abscess formation, necrotizing fasciitis (a surgical emergency), bacteremia, and sepsis.

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage

- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Wound cultures and blood cultures may guide therapy in severe cases.
- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include oral antibiotics (dicloxacillin, cephalexin, or clindamycin for MRSA coverage) for mild cases and IV antibiotics (ceftriaxone, vancomycin, or

daptomycin) for severe disease. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- Most people recover well with appropriate antibiotic therapy
- Recurrent cellulitis may require prophylactic antibiotics.

Chapter 146

Central Pontine Myelinolysis

Overview

Central pontine myelinolysis, also known as osmotic demyelination syndrome, is a non-inflammatory demyelinating condition of the pons caused by overly rapid correction of hyponatremia.

Causes

This condition happens because of osmotic stress leading to oligodendrocyte damage and demyelination.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience spastic quadriparesis, pseudobulbar palsy (difficulty swallowing, slurred speech, emotional lability), and in severe cases, locked-in syndrome.

Emergency warning signs

- Severe or worsening symptoms of central pontine myelinolysis require immediate medical evaluation

Progression and Stages

The outlook varies from person to person, with recovery being partial or absent depending on severity. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on MRI showing a characteristic symmetric T2-hyperintense lesion in the central basis pontis. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves primarily preventive: serum sodium should be corrected at a rate not exceeding 8–10 mmol/L in 24 hours
- Treatment typically involves supportive once established.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 147

Central Post-Stroke Pain

Overview

Central post-stroke pain (CPSP, Dejerine-Roussy syndrome) is a central neuropathic pain condition arising from a cerebrovascular lesion (ischemic or involving bleeding) affecting the spinothalamic tract or thalamocortical projections. It occurs in 1–12% of stroke survivors, with onset typically within 1–6 months after the stroke. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of the lesion in the somatosensory pathway. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience burning, aching, pricking, and freezing sensations contralateral to the lesion, with prominent allodynia and hyperalgesia to cold stimuli. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of central post-stroke pain require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam with confirmatory neuroimaging identifying the lesion in the appropriate somatosensory pathway. First-line pharmacotherapy includes amitriptyline and lamotrigine
- Gabapentin, pregabalin, and duloxetine are also used.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Opioids are generally ineffective for central neuropathic pain.

Chapter 148

Central Sleep Apnea

Overview

Central sleep apnea (CSA) is a sleep-related breathing disorder characterized by recurrent episodes of absent or reduced respiratory effort during sleep due to instability of the central respiratory control system, without airway obstruction. CSA affects 5–10% of patients referred for sleep studies and is frequently comorbid with heart failure and neurological conditions. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of increased chemosensitivity (high loop gain), prolonged circulation time (Cheyne-Stokes respiration in heart failure), hypobaric hypoxia-induced hyperventilation and hypocapnia (high-altitude periodic breathing), opioid-induced respiratory depression, or brainstem lesions.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fragmented sleep, nocturnal shortness of breath, and excessive daytime sleepiness.

Emergency warning signs

- Severe or worsening symptoms of central sleep apnea require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for polysomnography demonstrating five or more central apneas or hypopneas per hour with no evidence of obstruction. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on treatment of underlying conditions, supplemental oxygen, positive airway pressure (CPAP, bilevel, or adaptive servo-ventilation), acetazolamide, and gradual acclimatization for high-altitude periodic breathing. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and

follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Your outlook depends on the underlying cause
- Optimization of cardiac function is essential in heart failure patients.

Chapter 149

Cerebral Aneurysm

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- A cerebral aneurysm is a focal dilation of a cerebral artery, most commonly at bifurcation sites in the Circle of Willis
- Saccular (berry) aneurysms are the most prevalent type.

Causes

This condition happens because of weakness in the arterial wall. Most are asymptomatic and discovered incidentally, but rupture causes subarachnoid bleeding with high mortality (approximately 40%). Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension and diabetes
- Elevated cholesterol levels
- Family history of heart disease
- Obesity and physical inactivity
- Family history and genetic predisposition
- Head trauma or brain injury

- Vascular risk factors (hypertension, diabetes)
- Exposure to environmental toxins
- Chronic stress
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of cerebral aneurysm require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

- Doctors diagnose this using CT angiography, MR angiography, or digital subtraction angiography. Treatment for unruptured aneurysms may involve observation or prophylactic treatment depending on size, location, and patient factors
- Ruptured aneurysms require urgent surgical clipping or endovascular coiling.
- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Anti-hypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

The outlook is good for unruptured aneurysms but poor following rupture. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 150

Cerebral Venous Thrombosis

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Cerebral venous blood clot formation is a blood clot formation of the dural venous sinuses or cerebral veins that leads to impaired venous removal of fluid, increased intracranial pressure, and potential venous tissue death due to lack of blood supply
- It accounts for less than 1% of all strokes but disproportionately affects young adults and women, particularly those using oral contraceptives or during pregnancy and the puerperium.

Causes

This condition happens because of thrombotic occlusion of cerebral veins or dural sinuses. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease

- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Clinical features vary widely and may include headache, seizures, weakness, numbness, or problems with movement, papilledema, and altered consciousness. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of cerebral venous thrombosis require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events

- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

To make the diagnosis, doctors look for CT or MR venography showing a filling defect in the venous sinuses. Management consists of anticoagulation (even in the presence of involving bleeding venous tissue death due to lack of blood supply), Treatment for raised intracranial pressure, and treatment of seizures. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid. History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

In most cases, the outlook is favorable with prompt diagnosis and treatment. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 151

Cerebral Visual Impairment

Overview

- Cerebral (or cortical) visual impairment (CVI) is reduced vision caused by damage to the brain's visual processing areas rather than to the eyes themselves
- It is the leading cause of visual impairment in children in developed countries
- Vision loss ranges from mild to severe and often coexists with other neurological conditions

Causes

This condition happens because of damage to the visual pathways or visual cortex of the brain. In children, common causes include prematurity and perinatal hypoxia, periventricular leukomalacia, intraventricular hemorrhage, infection during pregnancy (e.g., cytomegalovirus, toxoplasmosis), hydrocephalus, and traumatic brain injury. In adults, stroke, head trauma, infection, and brain tumors are the main causes.

Risk Factors

- Premature birth and low birth weight
- Hypoxic-ischemic encephalopathy at birth
- Hydrocephalus and shunted cerebrospinal fluid
- Seizures (epilepsy)
- Cerebral palsy and developmental delay
- Stroke, trauma, or brain tumors in adults

Signs and Symptoms

Common symptoms

- Visual acuity poorer than expected for the eye exam findings
- Difficulty recognizing faces or objects
- Preferential attention to moving or brightly colored objects
- Difficulty seeing in cluttered or visually busy environments
- Variable vision that changes with fatigue, light, or novelty
- Reduced visual fields, often lower visual field loss

Emergency warning signs

- Sudden loss of vision with other neurological symptoms such as weakness, confusion, or headache requires urgent evaluation

Progression and Stages

CVI is not progressive when caused by a static brain injury; however, the functional vision often improves over time, especially in children, as the brain matures and with rehabilitation. In adults with stroke or degenerative disease, the course depends on the underlying cause. Recovery can continue for months to years after injury.

Complications

- Delayed motor, cognitive, and language development in children
- Difficulty with learning and school performance
- Increased risk of falls and accidents
- Social isolation and behavioral challenges
- Overlapping sensory and neurological disabilities

Diagnosis

- Ophthalmological examination showing normal or near-normal eye structure
- Neuroimaging (MRI) to identify brain lesions
- Visual evoked potentials (VEP) to assess visual pathway function
- Functional vision assessment by a specialist team
- EEG and neurological evaluation for associated seizures

Prevention

- Preventing prematurity and optimizing neonatal care
- Prompt treatment of hydrocephalus and seizures
- Prevention of head injury and infection
- Stroke prevention measures in adults (blood pressure, cholesterol, healthy lifestyle)

Treatment Options

- Early intervention and developmental programs for children
- Specialized educational support and visual stimulation training
- Environmental modifications: reducing clutter, enhancing contrast and lighting
- Multidisciplinary rehabilitation (vision, occupational, and physical therapy)
- Treatment of the underlying neurological condition
- Assistive technology and accommodations in school and at home

Living with It

- Provide predictable, simplified visual environments
- Use familiar objects, routines, and verbal cues
- Involve teachers, therapists, and family in a consistent approach
- Regularly reassess functional vision, as it can improve over time
- Seek support networks for families affected by CVI

Outlook

The outlook is variable. Many children with CVI make significant visual gains with early intervention and support, particularly when the underlying injury is static. Functional vision and quality of life often improve substantially with rehabilitation and appropriate educational strategies.

Chapter 152

Cerumen Impaction

Overview

Cerumen impaction is an accumulation of earwax that causes symptoms or obscures the tympanic membrane, affecting approximately 6% of the general population and up to 30% of the elderly.

Causes

This condition happens because of overproduction of cerumen or from use of cotton swabs and hearing aids that push cerumen deeper into the canal.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience hearing loss, aural fullness, ringing in the ears, and otalgia.

Emergency warning signs

- Severe or worsening symptoms of cerumen impaction require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on otoscopic examination. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Most people recover well with proper removal
- Avoidance of cotton swabs is recommended for prevention.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on manual extraction with a curette, irrigation with warm water, or cerumenolytic agents (dilute hydrogen peroxide, mineral oil, glycerin). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 153

Cervical Cancer

Overview

Treatment for abnormal screening results follows the ASCCP risk-based management consensus guidelines. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of cervical cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome

(rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Cervical cancer screening with Pap smear and HPV co-testing remains the cornerstone of prevention. HPV vaccination (targeting ages 11–12, with catch-up through age 26) has the potential to nearly eliminate cervical cancer globally. Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and

consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian

- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 154

Cervical Cancer

Overview

Cervical cancer is the fourth most common cancer in women worldwide, with approximately 600,000 new cases and 340,000 deaths annually, disproportionately affecting low- and middle-income countries. Nearly all cases (99%) are caused by persistent infection with high-risk human papillomavirus (HPV) types 16 and 18 (responsible for 70% of cases). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)

- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience abnormal vaginal bleeding, postcoital bleeding, and pelvic pain. Cervical screening involves Pap smear (cytology) and/or HPV DNA testing, with abnormal results evaluated by colposcopy and biopsy. Pre-invasive lesions (cervical intraepithelial neoplasia, CIN) are treated by excisional procedures (LEEP, cold knife conization). Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of cervical cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment for early-stage disease is radical hysterectomy with pelvic lymph node dissection or chemoradiation
- Advanced disease doctors typically treat it with concurrent chemoradiation and adjuvant chemotherapy.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Most people recover well with early detection, and prevention through HPV vaccination and screening has dramatically reduced incidence. Avoid tobacco use and limit alcohol

consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies

(acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 155

Cervicitis (Chlamydia, Gonorrhea)

Overview

Cervicitis is inflammation of the uterine cervix, most commonly caused by *Chlamydia trachomatis* and *Neisseria gonorrhoeae*. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience mucopurulent endocervical discharge, cervical friability, and intermenstrual or postcoital bleeding
- Many infections are asymptomatic. Untreated cervicitis can ascend to cause PID, tubal factor infertility, ectopic pregnancy, and neonatal infection. Screening with NAAT is recommended for sexually active women under 25. Treatment for chlamydia: azithromycin 1 g single dose or doxycycline 100 mg BID for 7 days
- Treatment for gonorrhea: ceftriaxone 500 mg IM single dose. Partner treatment and test-of-cure are indicated.
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of cervicitis (chlamydia, gonorrhea) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes

- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

The outlook is generally positive with treatment. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 156

Cervicogenic Headache

Overview

Cervicogenic headache is a secondary headache originating from bony or soft tissue structures of the cervical spine. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of convergence of cervical afferents (C1–C3) with trigeminal afferents in the trigeminocervical complex. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience unilateral pain starting in the neck or occiput and spreading to the frontotemporal region, precipitated or worsened by specific neck movements or sustained head postures, with reduced range of motion and tenderness over

upper cervical facets or occipital nerve. Diagnosis is supported by temporary relief with anesthetic block of the C2–3 nerve root or greater occipital nerve. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cervicogenic headache require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include physical therapy, occipital nerve block, radiofrequency ablation, and optimization of ergonomics. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person and depends on the underlying pathology. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 157

Chagas Disease (American Trypanosomiasis)

Overview

Chagas disease is a protozoan infection caused by *Trypanosoma cruzi*, transmitted by reduviid (triatomine) bugs (“kissing bugs”), with 6–8 million infected, primarily in Latin America, with increasing cases in the US, Europe, and Japan due to migration.

Causes

This condition happens because of metacyclic trypomastigotes entering the skin or mucous membranes, invading macrophages and other nucleated cells, and transforming into amastigotes, with the parasite having a tropism for cardiac muscle, smooth muscle, and autonomic ganglia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of chagas disease (american trypanosomiasis) require immediate medical evaluation

Progression and Stages

You may experience acute phase (6–8 weeks, often asymptomatic): Romãna’s sign (unilateral, painless palpebral swelling and conjunctivitis), fever, lymphadenopathy, hepatosplenomegaly, and occasionally acute myocarditis. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Indeterminate phase: asymptomatic, positive serology
- Chronic phase (10–30 years after infection, 30% progress): Chagas cardiomyopathy—the most common and serious manifestation, megaesophagus, and megacolon.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using microscopic identification of trypomastigotes in blood (acute) or serology (chronic). Comprehensive medical history and physical examination. Laboratory tests to assess organ function and rule out other conditions. Imaging studies to visualize affected structures. Specialized testing depending on the affected system. Consultation with relevant specialists. Monitoring of disease progression over time.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment options include benznidazole or nifurtimox for acute and congenital disease, with supportive care for chronic cardiomyopathy.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is good for acute disease
- Chronic cardiomyopathy has significant morbidity.

Chapter 158

Chancroid

Overview

Chancroid is a sexually transmitted infection caused by *Haemophilus ducreyi*, a fastidious gram-negative coccobacillus, most common in resource-limited settings, and a cofactor for HIV transmission due to genital ulceration.

Causes

This condition happens because of *H. ducreyi* entering through microabrasions during sexual contact and producing a cytolethal distending toxin that causes epithelial cell tissue death.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 4–10 days, with painful, non-indurated genital ulcers with ragged, undermined borders and a purulent, necrotic base, and painful inguinal lymphadenopathy occurring in 50% of cases.

Emergency warning signs

- Severe or worsening symptoms of chancroid require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, with Gram stain showing gram-negative coccobacilli in “school of fish” arrangement. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include azithromycin, ceftriaxone, ciprofloxacin, or erythromycin. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 159

Charcot-Marie-Tooth Disease

Overview

Charcot-Marie-Tooth disease (CMT) is the most common hereditary peripheral neuropathy, affecting 1 in 2,500 individuals.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience slowly progressive distal muscle weakness and shrinkage or wasting (beginning in peroneal muscles, leading to foot drop and steppage gait), pes cavus, hammer toes, distal sensory loss, and diminished or absent deep tendon reflexes. Upper extremity involvement occurs later, producing intrinsic hand muscle weakness and claw hand deformity.

Emergency warning signs

- Severe or worsening symptoms of charcot-marie-tooth disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

- It comprises a genetically heterogeneous group of disorders classified by nerve conduction velocity and inheritance pattern: CMT1 (demyelinating, NCV <38 m/s, autosomal dominant, most commonly PMP22 duplication), CMT2 (axonal, NCV >38 m/s, autosomal dominant, MFN2 most common), CMTX (X-linked, GJB1 mutations), and CMT4 (autosomal recessive). In most cases, the outlook is good
- Most patients remain ambulatory with normal life expectancy, though the severity varies widely by subtype.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical examination, nerve conduction studies, and genetic testing (next-generation sequencing panels). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive: physical and occupational therapy, ankle-foot orthoses for foot drop, surgical correction of foot deformities, and pain Treatment for neuropathic pain. Medications to manage symptoms and address underlying mechanisms

Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 160

Chickenpox

Overview

Chickenpox (varicella) is a highly contagious primary infection caused by varicella-zoster virus (VZV, human herpesvirus 3), predominantly affecting unvaccinated children. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of airborne respiratory droplet transmission or direct contact with vesicular fluid, with viral replication in nasopharyngeal lymph nodes followed by viremic dissemination to skin and viscera. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience a brief viral prodrome (fever, malaise, pharyngitis) followed by a characteristic intensely pruritic maculopapular rash that rapidly evolves into clear vesicles on an erythematous base ('dewdrops on a rose petal'), which rupture and crust over 48 hours. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of chickenpox require immediate medical evaluation

Progression and Stages

Lesions appear in successive crops over 3–5 days, resulting in lesions at all stages of development simultaneously across the body. The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Complications include secondary bacterial skin infections (*Staphylococcus aureus*, *Streptococcus pyogenes*), cerebellar loss of coordination, pneumonia (more severe in adults), and encephalitis
- VZV remains dormant in dorsal root ganglia and may reactivate as herpes zoster later in life.
- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam, confirmed by Tzanck smear (multinucleated giant cells) or PCR of lesion fluid. Management in healthy children is supportive (antipruritics, cool compresses)
- Aspirin is strictly contraindicated due to Reye syndrome)
- Oral acyclovir or valacyclovir is indicated for adolescents, adults, and immunocompromised patients.
- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Live attenuated varicella vaccine provides strong protection. Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring

long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 161

Chikungunya

Overview

Chikungunya is a mosquito-borne viral illness caused by chikungunya virus (CHIKV), an alphavirus transmitted by *Aedes aegypti* and *A. albopictus*, with outbreaks in Africa, Asia, the Indian Ocean islands, Europe, and the Americas.

Causes

This condition happens because of CHIKV infecting fibroblasts, epithelial cells, and monocytes, activating immune cells that produce cytokines mediating acute fever and severe joint pain, with the virus persisting in macrophages and synovial tissue, contributing to chronic arthropathy.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 2–6 days, with abrupt onset of high fever, severe polyarthralgia (symmetric, involving hands, wrists, ankles, knees, and spine), maculopapular rash on trunk and limbs, and muscle pain, with chronic joint pain (> 3 months) occurring in 30–60% of patients.

Emergency warning signs

- Severe or worsening symptoms of chikungunya require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using RT-PCR (first 5 days of illness) and serology. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive (rest, hydration, acetaminophen for fever, NSAIDs for joint pain after dengue is excluded). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is good for acute disease
- Chronic joint pain can be debilitating.

Chapter 162

Choanal Atresia

Overview

Choanal atresia is a congenital obstruction of the posterior nasal airway caused by a bony (most common), membranous, or mixed atresia plate, and may be associated with CHARGE syndrome.

Causes

This condition happens because of failure of the nasobuccal membrane to rupture during embryonic development.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of choanal atresia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Bilateral choanal atresia presents at birth with cyclic cyanosis (worsening with feeding, improving with crying) and severe respiratory distress
- Unilateral choanal atresia may present later with unilateral nasal obstruction and rhinorrhea.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with inability to pass a nasogastric tube and by CT imaging. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves surgical transnasal or transpalatal repair, ideally in the neonatal period for bilateral disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good following surgical repair. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 163

Cholecystitis

Overview

Acute calculous cholecystitis results from persistent impaction of a gallstone in the cystic duct, causing bile stasis, mucosal reduced blood flow, chemical irritation, and secondary bacterial superinfection (*Escherichia coli*, *Klebsiella*, *Enterococcus*). Acalculous cholecystitis occurs in critically ill patients (burns, trauma, major surgery, prolonged parenteral nutrition).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe steady right upper quadrant (RUQ) or epigastric pain radiating to the right scapula/shoulder, fever, leukocytosis, and a positive Murphy sign (inspiratory arrest on deep palpation of the RUQ). Diagnosis is supported by abdominal ultrasonography showing gallbladder wall thickening (> 4 mm), pericholecystic fluid, and sonographic Murphy sign
- HIDA scan confirms cystic duct obstruction if ultrasound is equivocal.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cholecystitis require immediate medical evaluation

Progression and Stages

Cholecystitis is inflammation of the gallbladder wall, classified as acute calculous (90% of cases), acalculous (10%), or chronic. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Complications include gallbladder gangrene, perforation, and emphysematous cholecystitis.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include NPO status, IV fluids, IV broad-spectrum antibiotics, and early using small incisions and a camera cholecystectomy (within 24–72 hours). Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 164

Cholelithiasis

Overview

Cholelithiasis is the presence of solid concretions (gallstones) within the gallbladder lumen, affecting 10–15% of the adult population. Most gallstones (> 80%) are asymptomatic. Symptomatic cholelithiasis presents as biliary colic: episodic right upper quadrant or epigastric dull pain precipitated by fatty meal ingestion, lasting 1–5 hours, without fever or peritoneal signs.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cholelithiasis require immediate medical evaluation

Progression and Stages

Gallstones are classified as cholesterol stones (80%, radiolucent, resulting from bile supersaturated with cholesterol, gallbladder hypomotility, and excess bilirubin) or pigment stones (20%, radiopaque, divided into black stones from chronic hemolysis/cirrhosis and brown stones from biliary tract infection). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that

may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using abdominal ultrasound. Asymptomatic stones require no treatment
- Symptomatic biliary colic is managed with elective using small incisions and a camera cholecystectomy.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well following surgery. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 165

Cholera (*Vibrio cholerae*)

Overview

This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of colonization of the small intestine and secretion of cholera toxin (CT), an A-B subunit enterotoxin that ADP-ribosylates Gs protein, constitutively activating adenylate cyclase, increasing cAMP, and causing massive chloride and water secretion into the intestinal lumen. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience sudden onset of profuse, painless, watery diarrhea (rice-water stools), vomiting, and rapidly progressing dehydration leading to hypovolemic shock, metabolic acidosis, and kidney failure within hours in severe cases. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of cholera (*vibrio cholerae*) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this based on stool culture (TCBS agar) or rapid dipstick test in endemic settings. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Cholera is an acute diarrheal disease caused by *Vibrio cholerae* serogroups O1 and O139, with 1.3–4 million cases and 21,000–143,000 deaths annually worldwide, primarily in developing countries with poor water sanitation. Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions

and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment focuses on aggressive rehydration with oral rehydration solution (WHO-ORS) for mild to moderate dehydration, intravenous Ringer's lactate for severe dehydration, and antibiotics (doxycycline, azithromycin, or ciprofloxacin) to reduce duration and volume of diarrhea. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- Most people recover well with prompt rehydration (mortality $< 1\%$)
- Without treatment, mortality exceeds 50%.

Chapter 166

Chondrodermatitis Nodularis Helicis

Overview

Chondrodermatitis nodularis helicis is a painful, inflammatory condition affecting the cartilage of the pinna, most commonly the helix or antihelix.

Causes

This condition happens because of necrotizing chondritis with overlying epidermal changes, often triggered by chronic pressure during sleep.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a small, exquisitely tender nodule with overlying crusting or ulceration, frequently on the side most affected by sleeping pressure.

Emergency warning signs

- Severe or worsening symptoms of chondrodermatitis nodularis helicis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical examination and histopathology. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include pressure relief (using a donut-shaped pillow), topical corticosteroids, and intralesional corticosteroid injection
- Refractory cases may require surgical removal.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is good with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 167

Chronic Fatigue Syndrome

Overview

Chronic fatigue syndrome/myalgic encephalomyelitis (CFS/ME) is a complex, debilitating multisystem illness affecting 0.2–0.4% of the population, with a female predominance. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

The cause is unknown but is likely multifactorial, involving post-infectious triggers (EBV, enterovirus, Q fever, Ross River virus), immune dysregulation, neuroendocrine dysfunction, and genetic predisposition, with proposed mechanisms including chronic immune activation, hypothalamic-pituitary-adrenal axis hypofunction, mitochondrial dysfunction, and central sensitization. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features (CDC/Institute of Medicine 2015 diagnostic criteria) include substantial reduction in previous activity level persisting >6 months, post-exertional malaise (PEM), unrefreshing sleep, thinking and memory impairment (brain fog), and orthostatic intolerance, along with additional symptoms such as chronic pain, sore throat, tender lymphadenopathy, and flu-like malaise.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of chronic fatigue syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam (no biomarkers), requiring ruling out other causes.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include symptom management with pacing/activity management (avoiding PEM), and symptom-specific treatments such as sleep hygiene, low-dose naltrexone, and SSRIs. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with approximately 5% recovering fully and most patients experiencing a fluctuating course.

Chapter 168

Chronic Gastritis

Overview

- Chronic gastritis is marked by chronic inflammatory infiltrate in the gastric mucosa. Two main types exist: Type A (autoimmune) gastritis involves the fundus and body and is linked to pernicious anemia and increased risk of gastric carcinoid tumors
- Type B (atrophic) gastritis involves the antrum and is the most common form worldwide.

Causes

This condition happens because of autoimmune destruction of parietal cells with anti-parietal cell and anti-intrinsic factor antibodies (Type A) or *H. pylori* infection (Type B). Long-standing *H. pylori* gastritis can progress through atrophic gastritis, intestinal metaplasia, dysplasia, and gastric adenocarcinoma (Correa cascade).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience dyspepsia and potential symptoms of vitamin B12 deficiency in Type A.

Emergency warning signs

- Severe or worsening symptoms of chronic gastritis require immediate medical evaluation

Progression and Stages

Doctors diagnose this based on endoscopy with biopsy using the Sydney system for grading. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on *H. pylori* eradication therapy for Type B and vitamin B12 supplementation for Type A. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the subtype and the presence of precancerous changes. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 169

Chronic Hypertension in Pregnancy

Overview

Chronic high blood pressure in pregnancy is defined as high blood pressure predating pregnancy or diagnosed before 20 weeks of gestation. It affects 1–5% of pregnancies. Women with chronic high blood pressure are at increased risk of superimposed preeclampsia (25%), placental abruption, preterm birth, and fetal growth restriction. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity

- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of chronic hypertension in pregnancy require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment options include antihypertensives (labetalol, nifedipine, methyldopa) for BP $\geq$ 160/100 mmHg
- ACE inhibitors and ARBs are contraindicated in pregnancy due to fetotoxicity.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

In most cases, the outlook is favorable with appropriate management. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 170

Chronic Insomnia Disorder

Overview

Chronic insomnia disorder is a sleep disorder defined by persistent difficulty initiating sleep, maintaining sleep, or early morning awakening with inability to return to sleep, despite adequate opportunity for sleep, occurring at least three nights per week for at least three months, with significant daytime impairment (fatigue, mood disturbance, thinking and memory dysfunction, or reduced performance). The prevalence is 6–10% in the general population, with females affected 1.5–2 times more frequently than males. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of hyperarousal of the hypothalamic-pituitary-adrenal axis (elevated cortisol, ACTH), increased high-frequency EEG activity during sleep, activation of the sympathetic nervous system, and maladaptive conditioning (negative associations with the sleep environment). Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Family history of mental illness
- Trauma or adverse childhood experiences
- Chronic stress
- Substance use
- Social isolation
- Underlying medical conditions

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Medication use

Signs and Symptoms

Common symptoms

- You may experience difficulty falling asleep, frequent awakenings, early morning awakening, and daytime impairment.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of chronic insomnia disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam based on ICSD-3 and DSM-5 criteria, supplemented by sleep diaries and actigraphy
- Polysomnography is not routinely indicated but may evaluate comorbid sleep disorders.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system

- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on thinking and memory behavioral therapy for insomnia (CBT-I) as first-line therapy—stimulus control, sleep restriction, thinking and memory restructuring, and sleep hygiene education—with pharmacotherapy (melatonin, ramelteon, orexin receptor antagonists, benzodiazepine receptor agonists, trazodone, doxepin) as adjunctive treatment. Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with CBT-I
- Long-term outcomes favor CBT-I over medication alone.

Chapter 171

Chronic Kidney Disease

Overview

Chronic kidney disease (CKD) is abnormalities of kidney structure or function present for more than 3 months, with health implications.

Causes

This condition results from disruption of normal chronic kidney disease function.

- The most common causes are diabetes mellitus (leading cause in developed countries) and high blood pressure
- Other causes include glomerulonephritis, polycystic kidney disease, and chronic obstruction. This condition happens because of progressive loss of nephrons leading to declining GFR.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of chronic kidney disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- CKD is staged by the glomerular filtration rate (GFR): Stage 1 (GFR ≥ 90 with kidney damage markers), Stage 2 (GFR 60–89), Stage 3a (GFR 45–59), Stage 3b (GFR 30–44), Stage 4 (GFR 15–29), and Stage 5 (GFR < 15 , end-stage kidney disease, ESRD). Treatment options include BP control (target $< 130/80$ with ACE inhibitors/ARBs), glycemic control, SGLT2 inhibitors (dapagliflozin, empagliflozin), dietary modification, and avoidance of nephrotoxins
- Kidney replacement therapy is required for Stage 5 CKD. Your outlook depends on stage and progression rate
- Cardiovascular disease is the leading cause of death in CKD.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on GFR estimation and markers of kidney damage (albuminuria, abnormal imaging, abnormal histology). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 172

Chronic Low Back Pain

Overview

Chronic low back pain (cLBP) is pain in the lower back persisting for ≥ 3 months. It is the leading cause of years lived with disability worldwide, with a lifetime prevalence of 60–80%. Most cases are non-specific (no identifiable structural pathology).

Causes

This condition happens because of a biopsychosocial interaction of biological factors (structural pathology, inflammation), psychological factors (catastrophizing, fear-avoidance beliefs, depression), and social factors (occupational demands, disability compensation).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pain in the lower back that may radiate to the legs (sciatica), with or without neurological deficits.

Emergency warning signs

- Severe or worsening symptoms of chronic low back pain require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Red flags for serious pathology include age >50 or <20, recent trauma, unexplained weight loss, fever, night pain, history of cancer, and progressive neurological deficits. Initial Treatment options include education, activity modification, NSAIDs, and muscle relaxants
- First-line pharmacotherapy is NSAIDs, with TCAs, SNRIs, and gabapentinoids as adjuncts
- Non-pharmacological treatments include physical therapy, manual therapy, acupuncture, thinking and memory behavioral therapy, and graded activity programs
- Interventional procedures include epidural steroid injections, facet joint injections, and spinal cord stimulation.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Most cases improve with conservative management.

Chapter 173

Chronic Lymphocytic Leukemia

Overview

Chronic lymphocytic leukemia is the most common leukemia in adults in Western countries, with a median age at diagnosis of 70 years, characterized by accumulation of mature-appearing but functionally incompetent B lymphocytes in the blood, bone marrow, lymph nodes, and spleen. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)

- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of chronic lymphocytic leukemia require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

You may experience generalized lymphadenopathy, hepatosplenomegaly, fatigue, weight loss, night sweats, and autoimmune complications (autoimmune hemolytic anemia, immune thrombocytopenia), with many patients being asymptomatic and diagnosed incidentally on routine CBC showing lymphocytosis ($>5000/\mu\text{L}$ B lymphocytes). Treatment typically involves indicated for symptomatic or bulky disease, progressive cytopenias, or autoimmune complications, with options including BTK inhibitors (ibrutinib, acalabrutinib, zanubrutinib), BCL-2 inhibitor (venetoclax) plus obinutuzumab, and FCR (fludarabine, cyclophosphamide, rituximab) for young patients with mutated IGHV. Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using flow cytometry showing clonal B cells co-expressing CD5, CD19, CD20 (dim), CD23, with prognostic markers including IGHV mutation status (mutated = favorable, unmutated = unfavorable), TP53 mutations/deletions (very unfavorable), del(13q) (favorable), trisomy 12, and del(11q). Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel

- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support

- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

In most cases, the outlook is favorable, with CLL being incurable but highly treatable and many patients having prolonged survival. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 174

Chronic Neck Pain

Overview

Chronic neck pain has a point prevalence of 10–15% and a lifetime prevalence of 50–70%. It is more common in women and office workers.

Causes

This condition happens because of mechanical neck pain (poor posture, prolonged computer use), cervical spondylosis, cervical radiculopathy, or myofascial pain.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pain in the neck that may radiate to the shoulders or arms.

Emergency warning signs

- Severe or worsening symptoms of chronic neck pain require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam. Management parallels chronic low back pain: physical therapy, manual therapy, exercise, NSAIDs, and muscle relaxants
- Cervical epidural steroid injections and selective nerve root blocks are used for radicular pain.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 175

Chronic Obstructive Pulmonary Disease

Overview

Causes

This condition results from disruption of normal chronic obstructive pulmonary disease function.

- This condition happens because of chronic inflammation leading to airway remodeling and alveolar destruction
- COPD encompasses chronic bronchitis (persistent cough and sputum production) and emphysema (destruction of alveolar walls).

Risk Factors

- Chronic obstructive lung disease is a common, preventable, and treatable disease characterized by persistent respiratory symptoms and airflow limitation due to airway or alveolar abnormalities, usually caused by significant exposure to noxious particles or gases
- Cigarette smoking is the predominant risk factor (approximately 80% of cases).
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive shortness of breath, chronic cough, sputum production, wheezing, and chest tightness.

Emergency warning signs

- Severe or worsening symptoms of chronic obstructive pulmonary disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Doctors diagnose this using spirometry showing post-bronchodilator FEV₁/FVC ratio less than 0.70
- GOLD classification stages severity by FEV₁. Treatment options include smoking cessation (most important intervention), bronchodilators, inhaled corticosteroids for frequent exacerbators, lung rehabilitation, long-term oxygen therapy for chronic hypoxemic failure, and lung volume reduction surgery or lung transplantation for advanced disease. Your outlook depends on disease severity, exacerbation frequency, and smoking cessation.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 176

Chronic Otitis Media

Overview

Chronic otitis media is a chronic or recurrent perforation of the tympanic membrane with or without active middle ear infection.

Causes

This condition happens because of persistent mucosal inflammation or squamous disease (cholesteatoma), which is an abnormal accumulation of keratinizing squamous epithelium that can erode the ossicles, facial nerve, and bony labyrinth.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of chronic otitis media require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- You may experience persistent or recurrent otorrhea through a perforated tympanic membrane, hearing loss, and occasional feeling of spinning
- Cholesteatoma may cause conductive hearing loss, facial nerve palsy, and intracranial complications.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on otoscopic examination and temporal bone CT. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves primarily surgical (tympanoplasty, mastoidectomy). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the extent of disease and success of surgical intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 177

Chronic Pancreatitis

Overview

Chronic pancreatitis is progressive, irreversible morphological changes of the pancreatic parenchyma leading to impairment of exocrine and endocrine function.

Causes

This condition results from disruption of normal chronic pancreatitis function.

- The most common cause in Western countries is chronic alcohol use (70–80%)
- Other causes include hereditary pancreatitis (PRSS1 mutations), cystic scarring (CFTR mutations), autoimmune pancreatitis, tropical pancreatitis, and idiopathic. This condition happens because of repeated episodes of pancreatic injury leading to scarring, parenchymal shrinkage or wasting, ductal strictures, and calcifications.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience chronic epigastric pain (often worse after meals), steatorrhea (exocrine insufficiency), diabetes mellitus (endocrine insufficiency), and weight loss.

Emergency warning signs

- Severe or worsening symptoms of chronic pancreatitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on CT scan showing pancreatic calcifications, ductal dilatation, and parenchymal shrinkage or wasting

- MRCP for ductal morphology
- And functional testing (fecal elastase, secretin stimulation).
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include pancreatic enzyme replacement therapy (PERT), pain management (analgesics, using a thin camera tube removal of fluid of ductal strictures, celiac plexus block), insulin for diabetes, and total pancreatectomy with islet autotransplantation for refractory cases. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person with progressive functional decline. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 178

Chronic Pelvic Pain

Overview

Chronic pelvic pain (CPP) is persistent or recurrent pain in the pelvic region lasting ≥ 6 months in women (and ≥ 3 –6 months in men). In women, estimated prevalence is 15–25%.

Causes

This condition results from disruption of normal chronic pelvic pain function.

- This condition happens because of various causes: in women, endometriosis, interstitial cystitis, chronic pelvic inflammatory disease, pelvic adhesions, adenomyosis, and pelvic floor myofascial pain
- In men, chronic prostatitis and pudendal neuralgia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience persistent or recurrent pelvic pain.

Emergency warning signs

- Severe or worsening symptoms of chronic pelvic pain require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for a multidisciplinary approach including gynecological, urological, gastroenterological, and pain medicine evaluation. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves directed at the identified cause

- Pelvic floor physical therapy is highly effective for pelvic floor myofascial dysfunction
- Pharmacotherapy includes NSAIDs, gabapentinoids, TCAs, and hormonal therapy for endometriosis.
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 179

Chronic Rhinosinusitis

Overview

Causes

This condition results from disruption of normal chronic rhinosinusitis function.

- This condition happens because of persistent mucosal inflammation, often with bacterial biofilms
- CRSwNP involves type 2 inflammation with eosinophilic infiltration and IL-5/IL-13 pathways.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience nasal obstruction, facial pain or pressure, purulent nasal discharge, and hyposmia.

Emergency warning signs

- Severe or worsening symptoms of chronic rhinosinusitis require immediate medical evaluation

Progression and Stages

Chronic rhinosinusitis is a symptomatic inflammation of the paranasal sinuses persisting for 12 weeks or longer, classified into CRS with nasal polyps (CRSwNP) and CRS without

nasal polyps (CRSsNP). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for using a thin camera tube or CT evidence of sinus inflammation. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include nasal saline irrigation, intranasal corticosteroids, and antibiotics for acute exacerbations

- Functional using a thin camera tube sinus surgery (FESS) is indicated for refractory disease
- Biologic therapies (dupilumab, omalizumab) have shown efficacy for CRSwNP.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

The outlook varies from person to person, with many patients requiring long-term management.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 180

Chronic Traumatic Encephalopathy

Overview

Chronic traumatic encephalopathy is a progressive neurodegenerative disease associated with repetitive head impacts, found predominantly in contact sport athletes, military personnel exposed to blast injuries, and victims of repeated physical abuse. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of accumulation of hyperphosphorylated tau protein in neurons and astrocytes, typically in a perivascular distribution at the depths of cortical sulci.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience behavioral and mood changes (depression, apathy, irritability, impulsivity), thinking and memory impairment (progressive dementia), and motor abnormalities (parkinsonism)
- Symptoms typically manifest years to decades after the repetitive head trauma. Diagnosis is currently only confirmed postmortem by neuropathological examination.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of chronic traumatic encephalopathy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive, as no disease-modifying treatment exists. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications

and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is progressive and incurable
- The discovery of chronic traumatic encephalopathy has led to significant changes in sports safety protocols.

Chapter 181

Chronic Tubulointerstitial Nephritis

Overview

Chronic tubulointerstitial nephritis is gradual tubular shrinkage or wasting and interstitial scarring.

Causes

Causes include chronic analgesic use (NSAIDs, combined analgesics), lithium therapy, heavy metals (lead, cadmium), hyperuricemia, sarcoidosis, and chronic infections. This condition happens because of prolonged exposure to nephrotoxic agents or chronic inflammation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience slowly progressive CKD, often with inability to concentrate urine (polyuria, nocturia) and mild proteinuria.

Emergency warning signs

- Severe or worsening symptoms of chronic tubulointerstitial nephritis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- The outlook varies from person to person
- Progression to ESRD may occur with continued exposure.

Complications

Treatment focuses on removal of the offending agent and Treatment for CKD complications.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using kidney biopsy showing tubular shrinkage or wasting, interstitial scarring, and chronic inflammatory infiltrate. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 182

Cirrhosis

Overview

This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age.

Causes

This condition happens because of chronic liver injury due to viral hepatitis (B, C), alcohol, NAFLD/NASH, autoimmune hepatitis, primary biliary cholangitis, primary sclerosing cholangitis, or hereditary hemochromatosis. Degenerative processes involve progressive cellular dysfunction and tissue breakdown over time. Contributing factors include oxidative stress, inflammation, accumulated cellular damage, and reduced regenerative capacity.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cirrhosis require immediate medical evaluation

Progression and Stages

Cirrhosis is the end stage of chronic liver disease characterized by diffuse scarring and formation of regenerative nodules that distort the liver architecture and vasculature, and is the 12th leading cause of death worldwide. Your symptoms depend on the stage and complications. Your outlook depends on the stage and complications, with transplantation offering the main treatment. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Cirrhosis may be compensated (no complications) or decompensated (development of ascites, variceal bleeding, liver encephalopathy, or jaundice). Portal high blood pressure drives many complications including esophageal and gastric varices, ascites, hepatorenal syndrome, and splenomegaly with hypersplenism.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using liver biopsy, imaging (ultrasound, CT, MRI), and elastography (FibroScan)
- The MELD score predicts mortality and guides transplant listing.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include treatment of the underlying cause (antivirals, alcohol abstinence, weight loss), diuretics for ascites, beta-blockers or band ligation for variceal prophylaxis, lactulose and rifaximin for liver encephalopathy, and liver transplantation for end-stage disease.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 183

Cleft Lip and Palate

Overview

Cleft lip and palate are among the most common congenital anomalies (1 per 700 live births), resulting from failure of fusion of the maxillary and medial nasal processes (cleft lip) or the palatal shelves (cleft palate).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience feeding difficulties, speech abnormalities, hearing loss, and dental anomalies.

Emergency warning signs

- Severe or worsening symptoms of cleft lip and palate require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam at birth. Treatment requires a multidisciplinary team approach: surgical repair (cheiloplasty at 3–6 months, palatoplasty at 9–18 months), speech therapy, orthodontic intervention, and Treatment for associated ear issues. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate multidisciplinary care. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 184

Clostridioides difficile Colitis

Overview

Clostridioides difficile colitis is an infectious inflammation of the colon caused by overgrowth of toxin-producing Clostridioides difficile bacteria, most commonly following antibiotic use. It is the leading cause of antibiotic-associated diarrhea and healthcare-associated infectious diarrhea, with approximately 500,000 infections annually in the United States.

Classification

Cause Infectious

Organ System Gastrointestinal

Pathophysiology Disruption of normal gut flora by antibiotics -> C. difficile overgrowth
-> toxin A and B production -> colonic epithelial damage and pseudomembrane formation

Duration Acute

Onset Sudden

Mode of Transmission Fecal-oral (spore-mediated)

Clinical Course Variable; may recur

Severity Mild to Severe

Extent of Disease Localized to colon

Age Group Adults (especially elderly)

Epidemiology Highest incidence in hospitalized elderly patients; nosocomial spread

ICD-11 Code 1A04

Causes

C. difficile colitis occurs when antibiotic therapy disrupts the normal colonic microbiome, allowing C. difficile spores to germinate and produce toxins A (enterotoxin) and B (cytotoxin), which cause direct colonic epithelial damage, inflammation, and formation of characteristic pseudomembranes. Clindamycin, fluoroquinolones, cephalosporins, and broad-spectrum penicillins are the most common antibiotic culprits. This condition develops from a combination of genetic and environmental factors that disrupt normal body processes. When these normal processes are disrupted, it leads to the symptoms and

signs of the condition. Several factors can work together to trigger or make the condition worse. Knowing the specific cause helps your doctor choose the most effective treatment.

Risk Factors

- Recent or current antibiotic use
- Advanced age (over 65 years)
- Hospitalization or long-term care facility residence
- Immunocompromised state
- Proton pump inhibitor use

Signs and Symptoms

Early symptoms

- Early manifestations of *clostridioides difficile* colitis may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Watery diarrhea occurring 3–15 times daily, often with a characteristic foul odor
- Abdominal cramping and diffuse lower abdominal tenderness
- Fever, nausea, and malaise
- Leukocytosis with left shift on laboratory studies
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of *clostridioides difficile* colitis may occur

Emergency warning signs

- Severe or worsening symptoms of *clostridioides difficile* colitis require immediate medical evaluation

Progression and Stages

Disease severity ranges from mild self-limited diarrhea to severe fulminant colitis. Severe disease is marked by elevated white blood cell count ($>15,000$ cells/mcL) and/or elevated serum creatinine (>1.5 times baseline), while fulminant colitis features toxic megacolon, colonic perforation, shock, or peritonitis requiring intensive care.

Complications

- Toxic megacolon with risk of colonic perforation
- Fulminant colitis requiring colectomy
- Recurrent *C. difficile* infection (20–30% after first episode)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Stool testing for *C. difficile* toxins (toxin A/B enzyme immunoassay) or glutamate dehydrogenase (GDH) antigen
- Nucleic acid amplification test (NAAT) via PCR for toxin genes
- Lab tests to check how your organs are working
- Imaging tests

Prevention

- Judicious antibiotic stewardship to minimize unnecessary use
- Strict hand hygiene with soap and water (alcohol-based sanitizers do not kill spores)
- Go for regular check-ups so any problems are caught early
- Follow your doctor's screening recommendations

Treatment Options

- Vancomycin orally (125 mg four times daily) or fidaxomicin (200 mg twice daily) for initial episodes
- Metronidazole reserved for mild disease when vancomycin and fidaxomicin are unavailable
- Fecal microbiota transplantation for multiply recurrent *C. difficile* infection
- Lifestyle changes and self-care
- Surgery when needed (colectomy for fulminant disease)

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Most patients with *C. difficile* colitis recover completely with appropriate antibiotic therapy, though recurrence rates remain significant. Your outlook depends on the specific condition, how advanced it is when found, and how well it responds to treatment. Getting treated early generally leads to better results. Many people manage this condition effectively with the right treatment. Regular check-ups are important to monitor your condition and adjust treatment as needed.

Chapter 185

Cluster Headache

Overview

Cluster headache is a trigeminal autonomic cephalalgia (TAC) characterized by strictly unilateral, severe to very severe pain in the orbital, supraorbital, temporal, or any combination of these areas, lasting 15–180 minutes if untreated, with attacks occurring from once every other day to 8 times per day. Prevalence is approximately 0.1%, with a male predominance (3:1). This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of activation of the posterior hypothalamic gray matter and the trigeminovascular system. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience unilateral burning, boring, or stabbing pain with ipsilateral autonomic features (conjunctival injection, lacrimation, nasal congestion, rhinorrhea, miosis, ptosis, eyelid swelling) and restlessness or agitation during attacks. Episodic cluster headache occurs in bouts with remission periods
- Chronic cluster headache has attacks for >1 year without remission.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cluster headache require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam based on ICHD-3 criteria. Acute Treatment options include subcutaneous sumatriptan, intranasal zolmitriptan, and high-flow oxygen. Transitional therapy uses short-course oral corticosteroids or greater occipital nerve block. Preventive therapy includes verapamil, lithium, topiramate, and valproic acid. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system

- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is chronic but attacks can be prevented during bouts with appropriate therapy. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 186

Coarctation of the Aorta

Overview

Coarctation of the Aorta is a congenital cardiac anomaly characterized by a discrete constriction of the aortic lumen, most commonly located distal to the origin of the left subclavian artery near the insertion of the ductus arteriosus (juxtaductal).

Causes

This condition happens because of ectopic ductal tissue contraction or impaired embryonic aortic arch development.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience upper extremity high blood pressure with delayed, weak, or absent femoral pulses and a lower extremity blood pressure gradient ($> 10 - -20$ mmHg lower in legs)
- Patients may present with headache, cold feet, leg claudication, or heart failure. A continuous systolic murmur is best heard over the back. Chest radiograph demonstrates rib notching (due to collateral intercostal arterial enlargement) and the 'figure-3' sign on the aorta.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of coarctation of the aorta require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with echocardiography or cardiac MRI. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on surgical removal with end-to-end anastomosis or balloon angioplasty with stenting. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good following early intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 187

Coccidioidomycosis (Valley Fever)

Overview

Coccidioidomycosis (Valley fever) is a systemic fungal infection caused by *Coccidioides immitis* and *C. posadasii*, dimorphic fungi endemic to the Sonoran Desert (southwestern US, northern Mexico, and parts of Central and South America), with 150,000 infections annually in the US, and incidence increasing due to climate change.

Causes

This condition happens because of inhalation of arthroconidia leading to conversion into spherules containing endospores in the lung, with a robust Th1 response controlling infection.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of coccidioidomycosis (valley fever) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

You may experience acute lung: 60% asymptomatic, 40% develop Valley fever (influenza-like illness with fever, cough, chest pain, joint pain, and redness of the skin nodosum)

- Chronic lung: persistent cough, hemoptysis, cavities
- Disseminated coccidioidomycosis: skin, bones/joints, and CNS (coccidioidal meningitis—the most feared complication).
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using culture, serology, and histopathology. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include fluconazole for mild-moderate disease and liposomal amphotericin B for severe disseminated disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well for acute lung disease
- Meningitis requires lifelong therapy.

Chapter 188

Cogan's Syndrome

Overview

Cogan's syndrome is a rare, chronic inflammatory disorder characterized by non-syphilitic interstitial keratitis and vestibuloauditory symptoms reminiscent of Ménière's disease/hearing loss, associated with systemic vasculitis, typically affecting young adults (median age 25–40 years), with equal sex distribution. The Disease mechanism is poorly understood but likely involves a T-cell-mediated and humoral autoimmune response directed against a common antigen in the cornea and inner ear, with approximately 50–70% of patients having systemic vasculitis that can involve large, medium, or small vessels. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience ocular involvement (interstitial keratitis—the hallmark, presenting with eye pain, photophobia, blurred vision, conjunctival injection), vestibuloauditory involvement (acute-onset Ménière-like attacks of feeling of spinning, nausea, vomiting, ringing in the ears, and fluctuating sensorineural hearing loss that can

progress to bilateral profound deafness), and systemic vasculitis (constitutional symptoms, skin nodules or purple spots on the skin, neuropathy, arthritis).

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cogan's syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, based on ocular and vestibuloauditory manifestations in the absence of syphilis or other infection.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves with corticosteroids (high-dose systemic and topical eye), with methotrexate, azathioprine, cyclophosphamide, rituximab, and TNF inhibitors used for refractory or severe disease. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person: up to 50% of patients develop significant visual or hearing impairment.

Chapter 189

Coloboma

Overview

- Coloboma is a congenital eye malformation in which part of the eye tissue is missing, caused by incomplete closure of the embryonic eye (optic fissure) during development
- It most often affects the iris ("keyhole pupil"), but can involve the retina, optic nerve, lens, or eyelid
- It affects roughly 1 in 10,000 births and may occur in one or both eyes

Causes

This condition happens because of failure of the optic fissure to close fully during early fetal development (around the 5th–7th week of gestation). It can be inherited in autosomal dominant, recessive, or X-linked patterns, or arise from new mutations. It is also associated with chromosomal abnormalities and syndromes such as CHARGE syndrome and cat-eye syndrome.

Risk Factors

- Family history of coloboma
- Genetic syndromes (CHARGE syndrome, cat-eye syndrome)
- Chromosomal abnormalities
- Certain genetic mutations (e.g., PAX2, PAX6, CHD7)
- Exposure to some medications or toxins in early pregnancy

Signs and Symptoms

Common symptoms

- Iris coloboma: a keyhole or cat-eye shaped pupil
- Retinal or optic nerve coloboma: reduced vision, visual field defects
- Light sensitivity (photophobia) with iris involvement
- May be associated with a notch in the eyelid (eyelid coloboma)
- Some people have no visual symptoms if the defect is peripheral

Emergency warning signs

- Sudden visual change, flashes, floaters, or a shadow in the vision requires urgent evaluation, as coloboma increases the risk of retinal detachment

Progression and Stages

Coloboma is a structural condition present from birth and is generally stable. Visual function depends on which part of the eye is affected and its size. Complications such as retinal detachment, glaucoma, or cataract can develop later and require monitoring. Associated systemic anomalies in syndromic cases may dominate the clinical picture.

Complications

- Retinal detachment (increased risk)
- Glaucoma
- Cataract formation
- Strabismus (misaligned eyes) and amblyopia (lazy eye)
- Reduced vision or blindness in the affected eye
- Associated systemic abnormalities in syndromic forms

Diagnosis

- Eye examination confirming the characteristic missing tissue
- Slit-lamp examination for iris, lens, and anterior chamber involvement
- Dilated fundus examination for retinal and optic nerve coloboma
- Optical coherence tomography (OCT) and fundus photography
- Genetic testing and evaluation for associated syndromes
- Ultrasound or imaging of the eye if the view is obstructed

Prevention

- Coloboma is congenital and largely not preventable
- Genetic counseling for affected families
- Prenatal diagnosis in known familial cases
- Early eye examination after birth to detect and manage complications
- Avoidance of known teratogenic exposures in pregnancy

Treatment Options

- Cosmetic contact lenses or surgery for iris coloboma
- Correction of refractive error and treatment of amblyopia and strabismus
- Monitoring and treatment of glaucoma and cataract

- Timely treatment of retinal detachment, which is more challenging in coloboma
- Low-vision support for significant visual impairment
- Management of associated systemic conditions in syndromic cases

Living with It

- Attend regular eye examinations to monitor for complications
- Protect the eyes and report sudden visual changes promptly
- Use sunglasses for light sensitivity
- Seek support for cosmetic or functional concerns
- Coordinate care with a multidisciplinary team in syndromic cases

Outlook

Coloboma is stable, and most people retain good vision when the macula and optic nerve are not involved. With monitoring and treatment of complications, the long-term outlook is generally good, though retinal detachment remains a lifelong risk that requires vigilance.

Chapter 190

Colorectal Adenoma

Overview

Colorectal adenoma is a non-cancerous epithelial tumor that is a precursor to colorectal cancer through the adenoma-carcinoma sequence. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades.

Causes

Most adenomas are asymptomatic and detected through screening colonoscopy. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Genetic predisposition and family history
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of colorectal adenoma require immediate medical evaluation

Progression and Stages

It is classified as tubular, tubulovillous, or villous based on histology, and by the degree of dysplasia (low-grade or high-grade). Larger adenomas (>1 cm), villous histology, and high-grade dysplasia carry higher cancerous potential. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on colonoscopy with histopathological evaluation. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Treatment focuses on using a thin camera tube removal (polypectomy) with surveillance colonoscopy intervals based on polyp number, size, and histology. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with complete removal. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 191

Colorectal Cancer

Overview

This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

- Colorectal cancer (CRC) is the third most commonly diagnosed cancer worldwide and the second leading cause of cancer-related death. Approximately 70% of CRCs arise from adenomatous polyps via the chromosomal instability pathway (APC/KRAS/TP53 mutations)
- Lynch syndrome (hereditary nonpolyposis colorectal cancer, HNPCC) accounts for 2–4% of cases due to mismatch repair gene mutations.

Causes

This condition happens because of accumulated genetic and epigenetic alterations driving cancerous transformation. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes

- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Symptoms vary depending on location
- Right-sided colon cancers tend to present with iron deficiency anemia and occult GI bleeding, while left-sided cancers present with altered bowel habits, obstruction, and hematochezia.
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of colorectal cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment for localized disease is surgical removal with lymphadenectomy, with adjuvant chemotherapy (FOLFOX, CapeOX) for stage III disease
- Neoadjuvant chemoradiation is standard for locally advanced rectal cancer. Your outlook depends on stage at diagnosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

- Doctors diagnose this based on colonoscopy with biopsy
- Screening methods include colonoscopy (gold standard), fecal occult blood testing (FIT), and CT colonography. Staging uses TNM classification with CT, MRI (rectal cancer), and PET. Metastatic CRC doctors typically treat it with systemic chemotherapy, targeted therapy (bevacizumab [anti-VEGF], cetuximab/panitumumab [anti-EGFR] for RAS wild-type tumors), immunotherapy (pembrolizumab for MSI-high/dMMR tumors), and metastasectomy for selected patients.
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing

infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications

- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 192

Color Blindness

Overview

- Color blindness (color vision deficiency) is a reduced ability to distinguish certain colors, most commonly red and green
- It affects about 8% of men and 0.5% of women worldwide (roughly 1 in 12 men)
- Complete color blindness (achromatopsia) is very rare; most people with the condition see colors, just differently

Causes

Most color blindness is inherited in an X-linked pattern, caused by mutations in genes on the X chromosome (OPN1LW and OPN1MW) that encode red and green cone photopigments. Because it is X-linked, it is much more common in males. Acquired color vision deficiency can result from aging, diabetes, glaucoma, retinal disease, certain medications, or chemical exposure.

Risk Factors

- Male sex (X-linked inheritance)
- Family history of color blindness
- Eye diseases such as glaucoma, age-related macular degeneration, or diabetic retinopathy
- Certain medications and occupational chemical exposures

Signs and Symptoms

Common symptoms

- Difficulty distinguishing red from green, or blue from yellow
- Trouble reading color-coded charts, maps, and traffic signals
- Colors appear duller or muted
- Difficulty identifying ripe fruit or matching clothing
- Many people are unaware they have the condition until formally tested

Emergency warning signs

- New onset of color vision problems in an adult may signal retinal or optic nerve disease and warrants evaluation

Progression and Stages

Inherited color blindness is present from birth and remains stable throughout life. Acquired deficiencies may progress if the underlying disease progresses. Severity ranges from mild anomalous trichromacy (three cone types present but shifted) to complete dichromacy (one cone type missing).

Complications

- Difficulty in certain careers (pilot, electrician, police, medicine in some roles)
- Safety concerns in tasks relying on color signals
- Frustration in daily life and possible anxiety, especially in children
- Underlying disease progression in acquired cases

Diagnosis

- Ishihara color plates (most common screening test)
- Farnsworth–Munsell 100 hue test to characterize the deficiency
- Anomaloscopy to precisely classify the defect
- Genetic testing in selected cases
- Eye examination to rule out acquired causes when onset is new

Prevention

- Inherited color blindness cannot be prevented
- Managing underlying eye diseases may prevent acquired cases
- Genetic counseling for families with known inheritance patterns

Treatment Options

- There is no cure for inherited color blindness
- Special lenses and filters (including tinted contact lenses) that enhance color discrimination in some people
- Gene therapy is under investigation for specific forms
- Practical accommodations: labeling, use of apps that identify colors, memorizing signal order
- Treatment of the underlying condition in acquired cases

Living with It

- Use smartphone apps and assistive technology for color identification
- Arrange clothing and labels using consistent patterns
- Inform teachers and employers about appropriate accommodations
- Children may benefit from early testing and reassurance

Outlook

Color blindness is a stable, lifelong trait that does not affect general health or life expectancy. With accommodations, most people live full and productive lives with only minor practical limitations.

Chapter 193

Coma

Overview

Coma is a state of profound unconsciousness in which the patient cannot be aroused and does not respond to external stimuli. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition results from disruption of normal coma function.

- This condition happens because of dysfunction of the ascending reticular activating system in the brainstem or bilateral cerebral hemispheres
- Common causes include traumatic brain injury, stroke, intracranial bleeding, status epilepticus, metabolic encephalopathy, drug intoxication, infections, and hypoxic-ischemic brain injury.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience unresponsiveness to external stimuli with eyes closed. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of coma require immediate medical evaluation

Progression and Stages

Recovery varies widely depending on the cause and severity of injury. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical assessment using the Glasgow Coma Scale (score 3–15), urgent neuroimaging, and laboratory studies. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Management targets the underlying cause, with supportive measures including airway management, intracranial pressure monitoring, and prevention of secondary brain injury.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 194

Common Variable Immunodeficiency

Overview

Common variable immunodeficiency is the most common symptomatic primary antibody deficiency, affecting approximately 1 in 25,000–50,000. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of common variable immunodeficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Complications include bronchiectasis (from recurrent infections), autoimmune cytopenias, granulomatous disease, lymphoproliferation, and increased risk of digestive tract and lymphoid malignancies.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for exclusion of other causes of hypogammaglobulinemia and is typically made in the second to fourth decade of life. You may experience recurrent infections.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

The condition is marked by hypogammaglobulinemia (decreased IgG and IgA, with or without decreased IgM), impaired vaccine antibody responses, and increased susceptibility to bacterial infections, particularly sinopulmonary infections (pneumonia, sinusitis, bronchitis).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person but improved with immunoglobulin replacement.

Chapter 195

Community-Acquired Pneumonia

Overview

Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

- Community-acquired pneumonia is an acute infection of the lung parenchyma acquired outside of a hospital setting
- Common pathogens include *Streptococcus pneumoniae* (most common typical bacterial cause), *Haemophilus influenzae*, atypical organisms (*Mycoplasma pneumoniae*, *Chlamydophila pneumoniae*, *Legionella pneumophila*), and respiratory viruses.

Causes

This condition happens because of microbial infection of the alveolar spaces leading to consolidation. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience fever, productive cough, shortness of breath, pleuritic chest pain, and malaise. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of community-acquired pneumonia require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- Doctors diagnose this based on chest X-ray showing consolidation
- The CURB-65 score assesses severity and guides disposition. Your outlook depends on severity and patient factors.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)

- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include empiric antibiotics: amoxicillin or macrolides for outpatient mild disease

- Respiratory fluoroquinolones or beta-lactam plus macrolide for inpatient disease
- Severe disease requires ICU admission and combination therapy.
- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 196

Compartment Syndrome

Overview

Compartment syndrome is elevated pressure within a closed fascial compartment, compromising tissue perfusion and leading to reduced blood flow and tissue death of muscle and nerves. Acute compartment syndrome is a surgical emergency most commonly caused by fractures (tibial fractures most common), crush injuries, reperfusion injury, or tight casts. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pain out of proportion to the injury, worsened by passive stretching of the affected muscles, with tense, swollen compartments
- The five P's (pulselessness, tingling or numbness, paralysis, pallor, poikilothermia) are late signs.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of compartment syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical examination and compartment pressure measurement (pressures >30 mmHg or within 30 mmHg of diastolic pressure indicate need for fasciotomy).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves emergent fasciotomy to decompress all affected compartments
- Delayed treatment leads to irreversible muscle tissue death, contracture (Volkmann's

ischemic contracture), and potentially limb loss.

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with early fasciotomy but poor if Treatment typically involves delayed.

Chapter 197

Complex Regional Pain Syndrome

Overview

- Complex regional pain syndrome (CRPS) is a painful condition characterized by disproportionate pain, sensory changes, vasomotor dysfunction, sudomotor dysfunction, and trophic changes in a limb. CRPS Type I occurs without a definable nerve lesion, typically after trauma or immobilization
- CRPS Type II occurs with a definable major nerve injury. The incidence is 5–26 per 100,000 person-years, with a female-to-male ratio of 3–4:1.

Causes

This condition happens because of multifactorial Disease mechanism including peripheral and central sensitization, sympathetic nervous system dysregulation, neurogenic inflammation, and altered somatosensory processing. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features are organized into four domains: sensory (allodynia, hyperalgesia), vasomotor (temperature asymmetry, skin color changes), sudomotor/swelling, and motor/trophic (decreased range of motion, weakness, tremor, dystonia).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of complex regional pain syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam using the Budapest diagnostic criteria.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves most effective when initiated early and includes a multidisciplinary approach with physical therapy, pharmacotherapy (gabapentinoids, TCAs, bisphosphonates), and interventional procedures (sympathetic nerve blocks, spinal cord stimulation). Symptomatic management and supportive care Enzyme replacement therapy

for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Early treatment improves outcomes but up to 20% of patients have persistent disability.

Chapter 198

Complex Sleep Apnea

Overview

Complex sleep apnea (treatment-emergent central sleep apnea) is a sleep-related breathing disorder defined by the emergence or persistence of CSA during PAP therapy in patients initially diagnosed with OSA. It occurs in 5–15% of patients initiating CPAP therapy, particularly those with cardiovascular disease, opioid use, or high loop gain physiology. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of PAP-induced elimination of the obstructive component, unmasking a latent central ventilatory instability.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience persistent respiratory events despite adequate PAP therapy for OSA.

Emergency warning signs

- Severe or worsening symptoms of complex sleep apnea require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for polysomnography on PAP demonstrating central events. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on continued PAP adherence, as many cases resolve spontaneously within 3–6 months
- Persistent cases may require switching to adaptive servo-ventilation or bilevel PAP with a backup rate.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 199

Concussion

Overview

- Concussion is a mild traumatic brain injury caused by a direct blow to the head or a biomechanical force transmitted to the brain, resulting in a transient disturbance of neurological function
- It is the most common form of traumatic brain injury.

Causes

This condition happens because of a complex neurometabolic cascade involving ionic flux, neurotransmitter release, impaired cerebral blood flow regulation, and axonal injury, without macrostructural damage visible on conventional neuroimaging. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Doctors usually diagnose this based on your symptoms and a physical exam
- CT head is indicated to exclude intracranial bleeding in patients with risk factors (age greater than 65, anticoagulant use, focal neurological signs, persistent vomiting, worsening headache).
- Advanced age
- Family history and genetic predisposition
- Head trauma or brain injury
- Vascular risk factors (hypertension, diabetes)
- Exposure to environmental toxins
- Chronic stress
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience headache, dizziness, confusion, amnesia, visual disturbances, sensitivity to light and noise, difficulty concentrating, and emotional lability
- Loss of consciousness occurs in fewer than 10% of cases.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of concussion require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- The outlook is generally positive, although post-concussion syndrome (persistent symptoms beyond 4 weeks) occurs in 10–30% of patients
- Second impact syndrome is a rare but potentially fatal complication.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Treatment focuses on physical and thinking and memory rest for 24–48 hours, followed by a graduated return-to-activity protocol. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 200

Congenital Cataract

Overview

Congenital cataract is an opacification of the lens present at or shortly after birth, affecting 1–6 per 10,000 live births. The condition may be idiopathic, hereditary (autosomal dominant most common), or associated with systemic conditions (Down syndrome, congenital rubella, galactosemia, Lowe syndrome, persistent fetal vasculature). This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of abnormal lens development or metabolic insult in utero. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Degenerative processes involve progressive cellular dysfunction and tissue breakdown over time. Contributing factors include oxidative stress, inflammation, accumulated cellular damage, and reduced regenerative capacity.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience lens opacity present at birth or in early childhood.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of congenital cataract require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using slit-lamp examination.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment focuses on urgent surgical removal, ideally within the first few weeks of life for unilateral cases, followed by postoperative visual rehabilitation with patching and corrective lenses. Your outlook depends on timing of intervention and compliance with visual rehabilitation.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 201

Congenital Diaphragmatic Hernia

Overview

Congenital diaphragmatic hernia (CDH) is the herniation of abdominal contents into the thoracic cavity through a defect in the diaphragm, most commonly the left posterolateral foramen of Bochdalek. Incidence is 1 per 2500 live births. CDH causes lung underdevelopment and lung high blood pressure, which determine outlook. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features at birth include respiratory distress, scaphoid abdomen, and decreased breath sounds on the affected side.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of congenital diaphragmatic hernia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with a nasogastric tube in the chest on radiograph.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include immediate endotracheal intubation (avoid bag-mask ventilation), nasogastric decompression, and gentle ventilation strategies
- Surgical repair is performed after physiologic stabilization. Overall survival is approximately 70–80%.
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 202

Congenital Heart Disease (Overview)

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Congenital heart disease (CHD) is the most common congenital anomaly, affecting approximately 1% of live births. CHD encompasses a wide spectrum of structural cardiac defects (see Chapter 12, Diseases of the Heart for comprehensive coverage). Septal defects (ASD, VSD, AVSD) allow left-to-right shunting, leading to lung overcirculation and, over time, lung high blood pressure and Eisenmenger syndrome
- Small defects may close spontaneously, while larger defects require surgical or transcatheter closure. Tetralogy of Fallot is the most common cyanotic CHD, consisting of VSD, overriding aorta, right ventricular outflow tract obstruction, and right ventricular enlargement
- Surgical repair is performed in infancy. Transposition of the great arteries creates parallel circulations incompatible with life without mixing
- The arterial switch operation is performed in the first week of life. Coarctation of the aorta presents with high blood pressure in the upper extremities and diminished femoral pulses
- Treatment typically involves surgical removal or balloon angioplasty. Hypoplastic left heart syndrome requires staged surgical palliation (Norwood, Glenn, Fontan procedures) or heart transplantation.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats
- Sudden weakness or numbness (stroke symptoms)

Emergency warning signs

- Severe or worsening symptoms of congenital heart disease (overview) require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal

medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 203

Congenital Hip Dysplasia

Overview

Developmental dysplasia of the hip (DDH) encompasses a spectrum from mild acetabular dysplasia to frank dislocation of the femoral head from the acetabulum. Incidence is 1–3 per 1000 live births. Clinical screening includes the Ortolani and Barlow maneuvers. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of congenital hip dysplasia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Your doctor confirms the diagnosis with ultrasound (for infants <6 months) or radiography (for older infants). Treatment for infants under 6 months is a Pavlik harness to maintain hip flexion and abduction
- For older infants or failed Pavlik treatment, closed or open reduction with a spica cast is required.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care
Enzyme replacement therapy for certain conditions
Gene therapy (emerging for select conditions)
Dietary modifications (e.g., phenylketonuria)
Regular monitoring for associated complications
Physical and occupational therapy
Genetic counseling for family planning
Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with early treatment
- Untreated DDH leads to osteoarthritis and limb-length discrepancy.

Chapter 204

Congenital Talipes Equinovarus (Clubfoot)

Overview

Congenital talipes equinovarus (clubfoot) is a congenital deformity of the foot characterized by four components: equinus, varus, adductus, and cavus. Incidence is 1 per 1000 live births, more common in males (2:1), and 50% are bilateral. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

The cause is multifactorial. Diagnosis is primarily clinical at birth. The Ponseti method is the standard of care: serial manipulation and long-leg casting for 6–8 weeks, followed by through the skin Achilles tenotomy, and then a foot abduction brace. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of congenital talipes equinovarus (clubfoot) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with adherence to bracing
- Surgical release is reserved for resistant or neglected cases.

Chapter 205

Conjunctivitis

Overview

Viral conjunctivitis, most commonly caused by adenovirus, results from viral infection of the conjunctiva.

Causes

Bacterial conjunctivitis results from bacterial infection (*S. aureus*, *S. pneumoniae*, *H. influenzae*, *N. gonorrhoeae*, *C. trachomatis*).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of viral conjunctivitis include acute onset, watery discharge, and preauricular lymphadenopathy
- Bacterial conjunctivitis presents with purulent discharge and eyelids matted shut
- Allergic conjunctivitis features bilateral itching, tearing, and stringy mucus.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of conjunctivitis require immediate medical evaluation

Progression and Stages

Conjunctivitis is an inflammation of the conjunctiva and the most common eye disease worldwide, classified as infectious (viral, bacterial), allergic, or chemical or irritant. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, with Gram stain and culture used for hyperacute or bacterial cases. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves etiologic: supportive for viral, topical antibiotics for bacterial, and topical antihistamine or mast cell stabilizers for allergic
- Gonococcal conjunctivitis requires intramuscular ceftriaxone.

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with appropriate treatment
- Untreated gonococcal conjunctivitis can lead to corneal perforation within 24 hours.

Chapter 206

Constrictive Pericarditis

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Constrictive pericarditis is a condition in which the pericardium becomes thickened, fibrotic, and calcified, restricting diastolic filling of the heart
- Common causes include prior cardiac surgery, radiation therapy, tuberculosis, idiopathic pericarditis, and autoimmune disease.

Causes

This condition happens because of chronic inflammation leading to pericardial scarring and restriction. The Disease mechanism mimics restrictive cardiomyopathy but with ventricular interdependence and exaggerated respiratory variation in filling. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease

- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience signs of right heart failure (swelling, ascites, hepatomegaly, elevated jugular venous pressure with Kussmaul's sign). Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of constrictive pericarditis require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events

- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Diagnosis typically involves echocardiography, cardiac MRI (pericardial thickening and late enhancement), and cardiac catheterization (equalization of diastolic pressures). Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Treatment typically involves pericardiectomy, which is curative but carries significant surgical morbidity. Management includes lifestyle modifications, pharmacotherapy targeting the underlying mechanisms, and interventional procedures when indicated. Long-term adherence to treatment is essential for preventing disease progression. Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE

inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Most people recover well after successful pericardiectomy. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 207

Contact Dermatitis

Overview

Contact dermatitis is inflammation of the skin caused by direct contact with an irritant or allergen. Irritant contact dermatitis (ICD) accounts for 80% of cases and is caused by direct chemical damage to the skin (soaps, detergents, solvents, acids). Allergic contact dermatitis (ACD) is a type IV delayed hypersensitivity reaction requiring prior sensitization to the allergen. Common allergens include nickel, poison ivy/oak/urushiol, fragrances, preservatives, rubber chemicals, and hair dyes.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pruritic, erythematous, vesicular or bullous dermatitis in the distribution of contact.

Emergency warning signs

- Severe or worsening symptoms of contact dermatitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on history and patch testing for ACD. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include identification and avoidance of the causative agent, topical corticosteroids, and in severe cases, systemic corticosteroids. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with avoidance of the offending agent. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 208

Convergence Insufficiency

Overview

- Convergence insufficiency is an eye coordination disorder in which the eyes fail to turn inward sufficiently when looking at a near object
- It affects roughly 5% of children and adults and is a common cause of eye strain and reading difficulty
- It is not a vision-threatening condition but can significantly affect schoolwork and daily near tasks

Causes

This condition happens because of impaired coordination between the eye muscles that control inward turning (convergence). The underlying cause is often unclear; it may be related to the brain's ability to control binocular alignment. It is more common after head injury or concussion, and can coexist with other binocular vision problems.

Risk Factors

- Family history of eye coordination problems
- Head trauma or concussion
- Prolonged near work (reading, screens)
- Neurodevelopmental conditions such as attention-deficit/hyperactivity disorder
- Fatigue and stress

Signs and Symptoms

Common symptoms

- Eye strain, fatigue, and headache with near work
- Blurred or double vision when reading
- Words appear to move or "swim" on the page
- Squinting, rubbing eyes, or closing one eye while reading
- Poor concentration and reading comprehension, avoiding near tasks

Emergency warning signs

- Sudden onset of double vision or headache after head injury requires urgent medical evaluation

Progression and Stages

Symptoms typically appear in childhood or adolescence and are often first noticed when school reading demands increase. The condition tends to persist if untreated. It does not worsen vision or damage the eyes, but symptoms can fluctuate with fatigue, stress, and the amount of near work.

Complications

- Academic underperformance and avoidance of reading
- Chronic headaches and eye strain
- Reduced attention and self-esteem in children
- Difficulty with prolonged near work in adults (screen-based occupations)

Diagnosis

- Comprehensive eye examination with assessment of binocular function
- Near point of convergence testing
- Cover test to detect eye misalignment
- Measurement of convergence amplitudes
- Evaluation of accommodative function

Prevention

- Convergence insufficiency is largely not preventable
- Regular eye examinations for children, especially with reading complaints
- Prompt evaluation after concussion or head injury
- Ergonomic practices for near work

Treatment Options

- Vision therapy (office-based orthoptic exercises) is the most effective treatment, improving convergence in most cases
- Home-based pencil push-up exercises as a first-line option
- Prismatic glasses (base-in) for symptom relief in some patients
- Regular monitoring and retraining of eye coordination
- Treatment of coexisting accommodative disorders

Living with It

- Practice prescribed eye exercises regularly and consistently
- Take frequent breaks from near work and screens
- Ensure good lighting and posture when reading
- Teachers and parents should be informed about accommodations

Outlook

The outlook is very good. With vision therapy, most people achieve normal convergence and resolution or significant improvement of symptoms. Untreated, the condition persists but does not damage the eyes or vision.

Chapter 209

Copper Deficiency and Excess

Overview

Copper deficiency is a rare mineral deficiency disorder that occurs after gastric bypass surgery, in malabsorption, prolonged parenteral nutrition, excessive zinc intake (zinc induces metallothionein which blocks intestinal copper absorption), and in Menkes disease. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of deficiency of copper, an essential trace element serving as a cofactor for cytochrome c oxidase, superoxide dismutase, lysyl oxidase, dopamine β -hydroxylase, and ceruloplasmin. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience microcytic hypochromic anemia (refractory to iron therapy), neutropenia, myeloneuropathy (spastic gait, sensory loss of coordination, peripheral neuropathy mimicking B12 deficiency), and pancytopenia.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of copper deficiency and excess require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low serum copper and low ceruloplasmin.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on oral copper 2–5 mg/kg/day as copper sulfate. Menkes disease (kinky hair disease) is an X-linked recessive disorder of ATP7A causing impaired intestinal copper absorption, presenting in infancy with progressive neurodegeneration,

seizures, hypotonia, failure to thrive, characteristic brittle, depigmented, kinky hair (pili torti), and tortuous cerebral arteries

- Subcutaneous copper-histidine may improve outcomes if started early, but Unfortunately, the outlook is often poor. Wilson disease (copper excess) is covered in Chapter 8.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 210

Coronary Artery Disease

Overview

Coronary artery disease (CAD), also called coronary heart disease or ischemic heart disease, occurs when plaque narrows or blocks the arteries supplying the heart muscle. Reduced blood flow may cause stable angina, while sudden plaque disruption and clot formation can cause an acute coronary syndrome, including myocardial infarction. CAD may be present without symptoms until a serious event occurs.

Causes

The usual cause is atherosclerosis. Injury or dysfunction of the arterial lining permits lipid accumulation and inflammation, producing plaques that narrow the coronary arteries. Plaque rupture can trigger platelet activation and thrombosis. Less common causes include coronary spasm, spontaneous coronary artery dissection, and coronary microvascular disease.

Risk Factors

- Smoking, hypertension, diabetes, and abnormal blood lipids
- Increasing age and family history of premature cardiovascular disease
- Obesity, physical inactivity, and an unhealthy diet
- Chronic kidney disease and inflammatory disorders
- Previous stroke, peripheral artery disease, or other atherosclerotic disease

Signs and Symptoms

Typical angina is pressure, tightness, heaviness, or discomfort in the chest brought on by exertion or emotional stress and relieved by rest. Discomfort may spread to the arm, shoulder, neck, jaw, or back. Shortness of breath, fatigue, nausea, or sweating may occur, particularly in older adults, women, and people with diabetes. New, severe, prolonged, or rest pain may indicate a heart attack and requires emergency assessment.

Progression and Stages

Early atherosclerosis may be silent. As plaque enlarges, exertion can exceed the heart's blood supply and produce stable angina. Plaque rupture may abruptly reduce flow and cause unstable angina or myocardial infarction. Chronic ischemia can contribute to ischemic cardiomyopathy and heart failure.

Complications

- Myocardial infarction and sudden cardiac arrest
- Heart failure and ischemic cardiomyopathy
- Ventricular or atrial arrhythmias
- Cardiogenic shock
- Stroke and peripheral arterial disease as manifestations of systemic atherosclerosis

Diagnosis

Assessment begins with the symptom pattern, medical and family history, physical examination, and cardiovascular risk estimation. Tests may include an electrocardiogram, blood tests including cardiac troponin when acute ischemia is suspected, echocardiography, stress testing, coronary CT angiography, and coronary angiography. The choice depends on symptoms, urgency, pretest probability, and comorbidities.

Prevention

- Do not smoke; offer cessation support when needed
- Follow a heart-healthy eating pattern and exercise regularly
- Control blood pressure, diabetes, and lipid abnormalities
- Maintain a healthy weight and sleep adequately
- Take preventive medicines when prescribed

Treatment Options

Treatment may include lifestyle change, lipid-lowering therapy, antiplatelet therapy when indicated, antianginal medicines, and medicines for blood pressure or diabetes. Acute coronary syndromes require emergency treatment. Revascularization with percutaneous coronary intervention or coronary artery bypass grafting is considered when symptoms or coronary anatomy warrant it. Cardiac rehabilitation supports recovery and long-term risk reduction.

Living with It

Patients should follow the prescribed medication plan, attend follow-up visits, maintain regular activity appropriate to their condition, and learn the warning signs of myocardial infarction. Sudden chest pressure, shortness of breath, cold sweating, fainting, or pain radiating to the arm or jaw warrants emergency services.

Outlook

CAD is chronic, but progression and cardiovascular events can often be reduced through smoking cessation, risk-factor control, evidence-based medicines, and revascularization when appropriate. Prognosis depends on the extent of disease, ventricular function, associated conditions, and adherence to treatment.

Chapter 211

Creutzfeldt-Jakob Disease (CJD)

Overview

Creutzfeldt-Jakob disease is a rapidly progressive, fatal neurodegenerative disease caused by prions—misfolded, protease-resistant isoforms (PrP^{Sc}) of the normal cellular prion protein (PrP^{C}), with an incidence of 1–2 per million annually worldwide, median age at onset 60–65 years, with sporadic CJD (sCJD) accounting for 85–90% of cases, familial CJD (10%) linked to PRNP mutations, and iatrogenic CJD (<1%) from contaminated surgical instruments.

Causes

This condition happens because of PrP^{Sc} converting native PrP^{C} into the misfolded conformation through a template-directed process, leading to exponential accumulation of PrP^{Sc} aggregates, causing vacuolation (spongiform change), neuronal loss, and astrocytosis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of creutzfeldt-jakob disease (cjd) require immediate medical evaluation

Progression and Stages

You may experience rapidly progressive dementia (weeks to months), myoclonus (startle-sensitive, characteristic), cerebellar loss of coordination, pyramidal and extrapyramidal signs, visual disturbances, and akinetic mutism in the terminal stage, with death typically occurring within 1 year of symptom onset. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical criteria, EEG showing periodic sharp wave complexes (PSWCs), CSF RT-QuIC (specific and sensitive), and MRI showing T2/FLAIR hyperintensity in the basal ganglia and thalamus. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment typically involves palliative.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is uniformly fatal. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 212

Cri-du-Chat Syndrome

Overview

Cri-du-chat syndrome is a chromosomal disorder caused by deletion of the short arm of chromosome 5 (5p-), with an incidence of 1 in 15,000–50,000 live births. The name derives from the characteristic high-pitched, cat-like cry in infancy due to laryngeal underdevelopment. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience microcephaly, round face, widely spaced eyes, epicanthal folds, low-set ears, hypotonia, severe intellectual disability, and congenital heart disease. Most cases are de novo deletions.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cri-du-chat syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using karyotype or chromosomal microarray.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive with early intervention therapies. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 213

Crohn's Disease

Overview

Crohn's disease (CD) is a chronic, relapsing inflammatory bowel disease that can affect any part of the digestive tract from mouth to anus, though the terminal ileum and colon are most commonly involved. Peak incidence occurs in the second to fourth decades of life, with a bimodal distribution. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of an interplay of genetic susceptibility (over 240 identified loci, with NOD2/CARD15 being the most significant), environmental factors (smoking doubles the risk), and dysregulated immune response, characterized by transmural inflammation, skip lesions, and non-caseating granulomas. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking

- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- You may experience chronic diarrhea (often non-bloody), abdominal pain (typically right lower quadrant), weight loss, fatigue, and perianal disease (fistulas, skin tags, abscesses)
- Extraintestinal manifestations include arthritis, redness of the skin nodosum, pyoderma gangrenosum, uveitis, and primary sclerosing cholangitis.
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of crohn's disease require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use

- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Diagnosis typically involves colonoscopy with biopsies, MR enterography or CT enterography, and serological markers (CRP, fecal calprotectin). Management follows a step-up or top-down approach: 5-aminosalicylates for mild colonic disease, corticosteroids for moderate flares (not for maintenance), immunomodulators (azathioprine, methotrexate), and biologic therapies (anti-TNF [infliximab, adalimumab], anti-integrin [vedolizumab], anti-IL12/23 [ustekinumab], anti-IL23 [risankizumab]). Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild

disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

- The outlook varies from person to person
- Surgery is required in approximately 50% of patients within 10 years but is not curative.

Chapter 214

Croup (Laryngotracheobronchitis)

Overview

Croup is a viral infection of the upper airway causing subglottic swelling and obstruction, most commonly caused by parainfluenza virus types 1 and 3. It predominantly affects children 6 months to 3 years of age.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience abrupt onset of a barking “seal-like” cough, inspiratory stridor, hoarseness, and respiratory distress, often worse at night.

Emergency warning signs

- Severe or worsening symptoms of croup (laryngotracheobronchitis) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Neck radiograph may show the “steeple sign” (subglottic narrowing).
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include a single dose of oral or intramuscular dexamethasone (0.6 mg/kg) for mild to moderate croup and nebulized epinephrine for moderate to severe croup. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider

- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 215

Cryoglobulinemic Vasculitis

Overview

Mixed cryoglobulinemia (types II and III) represents 80% of cases and is predominantly HCV-associated. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- You may experience palpable purple spots on the skin (the most common feature, recurrent, involving lower extremities), arthralgias, peripheral neuropathy (symmetric, often painful), membranoproliferative glomerulonephritis (proteinuria, hematuria, high blood pressure, progressive kidney dysfunction), and less commonly, Raynaud's phenomenon, skin ulcers, and digital gangrene. Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of cryoglobulinemic vasculitis require immediate medical evaluation

Progression and Stages

Cryoglobulinemic vasculitis is a small-vessel vasculitis caused by immune complexes of cryoglobulins (immunoglobulins that precipitate at temperatures $<37^{\circ}\text{C}$ and dissolve on rewarming) deposited in vessel walls, classified into three types: type I (monoclonal, associated with lymphoproliferative disorders), type II (mixed monoclonal and polyclonal, most commonly associated with hepatitis C virus—HCV), and type III (polyclonal, associated with autoimmune diseases such as SLE or Sjögren's syndrome). Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

To make the diagnosis, doctors look for detection of serum cryoglobulins (blood collected and transported at 37°C), low complement levels (C4 markedly decreased), positive rheumatoid factor, and evidence of HCV infection. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
Corticosteroids for rapid control of inflammation
Disease-modifying antirheumatic drugs (DMARDs)
Biologic therapies targeting specific immune pathways
Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
Antimalarial drugs (hydroxychloroquine) for mild disease
Physical and occupational therapy to maintain function
Pain management for chronic pain
Lifestyle modifications including diet and stress reduction
Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Your outlook depends on the underlying cause and organ involvement. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 216

Cryptococcosis

Overview

Cryptococcosis is a systemic fungal infection caused by *Cryptococcus neoformans* (primarily in immunocompromised) and *Cryptococcus gattii* (emerging, affects immunocompetent individuals), with 250,000 cases annually worldwide and 180,000 deaths, mostly in HIV/AIDS (CD4 < 100), and cryptococcal meningitis being the leading cause of meningitis in HIV-infected adults in sub-Saharan Africa.

Causes

This condition happens because of inhalation of basidiospores, with the polysaccharide capsule being the major virulence factor, inhibiting phagocytosis and suppressing T cell responses.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience cryptococcal meningitis: subacute headache, fever, nausea, and altered mental status, with elevated intracranial pressure being a major cause of morbidity.

Emergency warning signs

- Severe or worsening symptoms of cryptococcosis require immediate medical evaluation

Progression and Stages

The outlook has improved with modern antifungal therapy but remains poor in advanced HIV/AIDS. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using serum cryptococcal antigen (CrAg) and CSF analysis (India ink stain, CrAg). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include induction therapy with liposomal amphotericin B plus flucytosine, followed by consolidation and maintenance with fluconazole. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 217

Cryptorchidism (Undescended Testis)

Overview

Cryptorchidism is the absence of one or both testes from the scrotum, occurring in 3% of full-term and 30% of preterm male newborns.

Causes

This condition happens because of failure of testicular descent. Spontaneous descent occurs in most cases within the first 3–6 months of life.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cryptorchidism (undescended testis) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using physical examination
- Ultrasound or laparoscopy may be used for nonpalpable testes.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves surgical orchiopexy, optimally performed between 6–18 months of age, to preserve fertility, reduce the risk of testicular torsion, and allow for scrotal examination. Undescended testes are associated with impaired spermatogenesis and increased risk of testicular malignancy
- Early orchiopexy does not eliminate the cancer risk but facilitates self-examination.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed

- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 218

Cushing Syndrome

Overview

Cushing syndrome is a clinical state resulting from prolonged exposure to excess systemic glucocorticoids. Exogenous administration of glucocorticoids is the most common cause. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Endogenous Cushing syndrome results from Cushing disease (ACTH-secreting pituitary adenoma, 70% of endogenous cases), ectopic ACTH secretion (eg., small cell lung carcinoma), or primary adrenal adenoma/carcinoma. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience central obesity, moon facies, buffalo hump, abdominal purple striae, proximal muscle weakness, skin fragility, high blood pressure, hyperglycemia, and osteopenia.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cushing syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves initial screening with 24-hour urinary free cortisol, late-night salivary cortisol, or 1-mg overnight dexamethasone suppression test, followed by plasma ACTH measurement to differentiate ACTH-dependent from ACTH-independent causes.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves targeted: surgical removal of pituitary or adrenal tumors, discontinuation of exogenous steroids, or medical therapy (ketoconazole, metyrapone). Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good following successful surgical cure.

Chapter 219

Cushing's Syndrome

Overview

- Cushing's syndrome results from chronic exposure to excess cortisol, regardless of the cause, with exogenous glucocorticoid use (iatrogenic) being the most common cause overall. Endogenous causes are divided into ACTH-dependent (70–80%): Cushing's disease (pituitary adenoma secreting ACTH, 70%) and ectopic ACTH syndrome (small cell lung cancer, bronchial carcinoid)
- And ACTH-independent (20–30%): adrenal adenoma, adrenal carcinoma, and bilateral adrenal abnormal increase in cells.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience central (truncal) obesity, moon facies, dorsocervical fat pad ("buffalo hump"), supraclavicular fat pads, thin extremities, purple striae (>1 cm wide), easy bruising, proximal muscle weakness, high blood pressure, hyperglycemia/diabetes, osteoporosis, menstrual irregularity, hirsutism, and psychiatric disturbances.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of Cushing's syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves confirmation of hypercortisolism (24-hour urinary free cortisol, late-night salivary cortisol, or 1 mg overnight dexamethasone suppression test) followed by determination of the cause (plasma ACTH, high-dose dexamethasone suppression test, CRH stimulation test, MRI pituitary, CT adrenals, inferior petrosal sinus sampling).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

The right treatment depends on the cause: transsphenoidal surgery for Cushing's disease, surgical removal of adrenal tumors, chemotherapy for adrenal carcinoma, and ketoconazole or metyrapone for medical management. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with successful treatment.

Chapter 220

Cyanide Toxicity

Overview

Cyanide is a rapidly acting poison from industrial sources, smoke inhalation, and certain plants.

Causes

This condition happens because of cyanide binding to cytochrome c oxidase (complex IV of the mitochondrial electron transport chain), inhibiting cellular respiration and causing histotoxic hypoxia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: rapid onset (seconds to minutes) of altered mental status, seizures, coma, trismus, hyperventilation, hypotension, bradycardia, ventricular arrhythmias, and cardiovascular collapse
- Classic sign is bright red venous blood.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cyanide toxicity require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam, especially in smoke inhalation victims
- Elevated lactate suggests cyanide toxicity. Management: hydroxocobalamin (5 g IV) is first-line, binding cyanide to form nontoxic cyanocobalamin
- Sodium thiosulfate provides substrate for rhodanese to convert cyanide to thiocyanate.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with rapid treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 221

Cyclospora cayetanensis

Overview

Cyclosporiasis is a diarrheal illness caused by *Cyclospora cayetanensis*, a coccidian protozoan parasite, with outbreaks associated with imported fresh produce from endemic regions (Central and South America, the Caribbean, Southeast Asia).

Causes

This condition happens because of ingested sporulated oocysts releasing sporozoites in the small intestine that invade enterocytes, causing villous blunting, crypt abnormal increase in cells, and inflammation of the duodenum and jejunum.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 1–14 days, with prolonged, relapsing watery diarrhea (explosive, non-bloody), nausea, anorexia, weight loss, abdominal cramps, and profound fatigue, with average illness duration of 5–7 weeks if untreated.

Emergency warning signs

- Severe or worsening symptoms of cyclospora cayetanensis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using modified Ziehl-Neelsen acid-fast stain of stool or PCR. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include trimethoprim-sulfamethoxazole (TMP-SMX). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 222

Cyclothymic Disorder

Overview

Cyclothymic disorder has a lifetime prevalence of 0.5–1%, characterized by at least 2 years (1 year in children and adolescents) of numerous periods of hypomanic symptoms that do not meet full criteria for a hypomanic episode and numerous periods of depressive symptoms that do not meet full criteria for a major depressive episode, with symptoms present at least half the time. The condition is often described as a temperamental or personality disposition with mood instability.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience chronic mood instability without meeting full criteria for hypomanic or depressive episodes.

Emergency warning signs

- Severe or worsening symptoms of cyclothymic disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include mood stabilizers (lithium, lamotrigine) and psychotherapy (IPSRT, CBT). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with up to 30% of patients eventually developing Bipolar I or Bipolar II disorder. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 223

Cystic Fibrosis

Overview

Cystic scarring is an autosomal recessive disorder caused by mutations in the CFTR gene on chromosome 7, affecting approximately 1 in 2,500 Caucasian individuals. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience chronic cough, recurrent infections, and growth failure.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cystic fibrosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

This condition happens because of a defective chloride channel leading to thick, viscous secretions in multiple organs. Lung manifestations are the primary cause of morbidity and mortality, including chronic obstructive lung disease with bronchiectasis, chronic *Pseudomonas aeruginosa* colonization, and progressive respiratory failure

- Pancreatic insufficiency causes malabsorption and failure to thrive
- Other complications include CF-related diabetes, liver disease, sinusitis, and male infertility.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using newborn screening, sweat chloride testing (greater than 60 mmol/L), and CFTR genotyping.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include CFTR modulators that target the underlying protein defect, airway clearance, inhaled mucolytics, and antimicrobials. Symptomatic management and

supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has improved dramatically, with median survival of approximately 50 years in developed countries.

Chapter 224

Cystitis

Overview

Cystitis (lower urinary tract infection) is one of the most common bacterial infections, particularly in women (50–60% of women experience at least one episode). *Escherichia coli* is the causative organism in 80–90% of cases.

Causes

This condition happens because of bacterial colonization and inflammation of the bladder mucosa.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience dysuria, urinary frequency, urgency, suprapubic pain, and hematuria.

Emergency warning signs

- Severe or worsening symptoms of cystitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on urinalysis and urine culture. Treatment for uncomplicated cystitis in non-pregnant women includes nitrofurantoin, trimethoprim-sulfamethoxazole, or fosfomycin

- Recurrent cystitis may require prophylactic antibiotics, behavioral modifications, and cranberry products
- Complicated cystitis (in males, pregnant women, or with structural abnormalities) requires broader-spectrum antibiotics and evaluation for underlying causes.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate antibiotic therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 225

Cytomegalovirus (CMV)

Overview

Cytomegalovirus is a beta-herpesvirus that infects 60–90% of the adult population worldwide, generally asymptomatic in immunocompetent hosts but causes significant disease in immunocompromised patients and congenitally infected neonates, and is the most common congenital infection (0.5–1% of live births).

Causes

This condition happens because of CMV infecting epithelial cells, endothelial cells, fibroblasts, and monocytes, with primary infection followed by lifelong latency and reactivation occurring with immunosuppression.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Your symptoms depend on the host: immunocompetent host presents with mononucleosis-like syndrome
- Immunocompromised host (HIV with CD4 < 50, transplant recipients) presents with CMV retinitis, CMV pneumonitis, CMV colitis, and CMV esophagitis
- Congenital CMV presents with sensorineural hearing loss (most common sequela), microcephaly, intracranial calcifications, chorioretinitis, and hepatosplenomegaly.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of cytomegalovirus (cmv) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using PCR (quantitative) of blood, urine, CSF, or BAL fluid.
Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include ganciclovir or valganciclovir for immunocompromised patients and congenital CMV. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the host immune status. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 226

Deep Vein Thrombosis

Overview

This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension and diabetes
- Elevated cholesterol levels
- Family history of heart disease
- Obesity and physical inactivity
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience unilateral leg swelling, pain (often described as a dull ache or cramping), warmth, redness of the skin, and palpable cord along the affected vein, though many DVTs are asymptomatic. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of deep vein thrombosis require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

The outlook is good with treatment, though complications include post-thrombotic syndrome (chronic venous insufficiency in 20–50% of patients) and chronic thromboembolic lung high blood pressure.

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Diagnosis typically involves clinical probability assessment (Wells score), D-dimer testing (high negative predictive value), and compression ultrasonography (first-line imaging, showing non-compressibility of the affected vein). Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Treatment typically involves anticoagulation: initial therapy with low-molecular-weight heparin (enoxaparin), fondaparinux, or direct oral anticoagulants (DOACs—rivaroxaban, apixaban), followed by long-term anticoagulation for 3–6 months (first unprovoked DVT) or longer depending on risk of recurrence. Management includes lifestyle modifications, pharmacotherapy targeting the underlying mechanisms, and interventional procedures when indicated. Long-term adherence to treatment is essential for preventing disease

progression. Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 227

Delayed Sleep-Wake Phase Disorder

Overview

Prevalence is 1–3% in adults and 7–16% in adolescents, making it the most common circadian rhythm disorder. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe difficulty falling asleep at conventional times but normal sleep once initiated when allowed to follow the endogenous schedule.

Emergency warning signs

- Severe or worsening symptoms of delayed sleep-wake phase disorder require immediate medical evaluation

Progression and Stages

Delayed sleep-wake phase disorder (DSWPD) is a circadian rhythm sleep-wake disorder characterized by a stable delay of the sleep-wake phase relative to the desired or required schedule, with sleep onset typically after 1:00–3:00 AM and wake times in the late morning or early afternoon. This condition happens because of an abnormally long intrinsic

circadian period, reduced phase-advance capacity, or a hypo-responsive phase-advance mechanism to morning light. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical history, sleep diaries, actigraphy, and dim-light melatonin onset measurement. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include timed bright light therapy in the morning, low-dose melatonin (0.5–3 mg) administered 5–6 hours before target bedtime, and consistent sleep-wake scheduling
- Chronotherapy may be considered in refractory cases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment compliance. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 228

Delirium

Overview

Delirium is an acute, fluctuating disturbance in attention, awareness, and thinking, caused by an underlying medical condition, intoxication, or medication side effect. It is a medical emergency affecting 10–30% of hospitalized older adults and up to 50% of postoperative patients. Delirium is linked to increased mortality, longer hospital stays, functional decline, and incident dementia. Three subtypes exist: hyperactive (agitation, restlessness), hypoactive (withdrawal, lethargy, reduced speech—often missed or misdiagnosed as depression), and mixed. The Confusion Assessment Method (CAM) is the most widely used diagnostic tool: acute onset and fluctuating course, plus inattention, plus either disorganized thinking or altered level of consciousness. Precipitating factors include infection, electrolyte disturbance, surgery, psychoactive medications, pain, sleep deprivation, and urinary retention. Pharmacologic treatment (haloperidol or atypical antipsychotics) is reserved for severe agitation that threatens safety.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of delirium require immediate medical evaluation

Progression and Stages

Predisposing factors include advanced age, dementia, sensory impairment, polypharmacy, and multiple comorbidities. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Prevention is paramount (the Hospital Elder Life Program—HELP).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Delirium is linked to increased mortality and functional decline.

Chapter 229

Delusional Disorder

Overview

Delusional disorder has a lifetime prevalence of 0.2%, with a later age of onset (typically 40–60 years) compared to schizophrenia.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience one or more delusions that persist for at least 1 month, without meeting full criteria for schizophrenia (hallucinations, if present, are not prominent and are related to the delusional theme)
- Negative symptoms are absent
- Functioning is not markedly impaired). Common subtypes include persecutory (most common), jealous, erotomanic (de Clérambault syndrome), grandiose, and somatic.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of delusional disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include antipsychotics (low-dose). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is better overall than schizophrenia. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 230

Dementia with Lewy Bodies

Overview

Dementia with Lewy bodies (DLB) is a progressive neurodegenerative disorder accounting for 5–15% of dementia cases. Core You may experience fluctuating thinking with pronounced variations in attention and alertness, recurrent well-formed visual hallucinations (often detailed, involving people or animals), and parkinsonism (bradykinesia, rigidity, gait disorder). REM sleep behavior disorder (RBD) is a supportive feature, often predating dementia onset by years. In the geriatric population, DLB patients are extremely sensitive to neuroleptics (neuroleptic sensitivity reaction—severe parkinsonism, sedation, autonomic instability, increased mortality), which should be avoided. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of dementia with lewy bodies require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Treatment options include cholinesterase inhibitors (rivastigmine is FDA-approved for DLB), avoidance of anticholinergic medications, and low-dose levodopa for parkinsonism. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is progressive. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 231

Dengue Fever

Overview

Dengue fever is a mosquito-borne viral illness caused by dengue virus (DENV 1–4), a flavivirus transmitted by *Aedes aegypti* mosquitoes, with 400 million infections and 100 million symptomatic cases annually, and 40,000 deaths, endemic in tropical and subtropical regions. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system
- Travel to endemic areas
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Healthcare exposure (nosocomial infections)

- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of dengue fever require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- You may experience incubation of 3–14 days, with classic dengue fever presenting with sudden onset high fever, severe headache (retro-orbital), joint pain and muscle pain (“breakbone fever”), maculopapular rash, lymphadenopathy, and leukopenia
- Severe dengue (dengue involving bleeding fever/dengue shock syndrome) presents with plasma leakage, severe thrombocytopenia, mucosal bleeding, and shock during the critical phase at defervescence.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using NS1 antigen test, IgM and IgG serology, and RT-PCR. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification

- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment typically involves supportive (acetaminophen for fever, avoiding NSAIDs and aspirin due to bleeding risk, fluid resuscitation). Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Most people recover well for classic dengue
- Severe dengue has significant mortality without prompt supportive care.
- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 232

Dermatitis Herpetiformis

Overview

Dermatitis herpetiformis (DH) is the cutaneous manifestation of celiac disease, characterized by intensely pruritic, grouped vesicles and papules on an erythematous base, symmetrically distributed on extensor surfaces (elbows, knees, buttocks, back).

Causes

This condition results from disruption of normal dermatitis herpetiformis function.

- This condition happens because of IgA antibody deposition at the dermal papillae, triggered by gluten ingestion
- Nearly all patients have underlying celiac disease, though digestive tract symptoms may be absent.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience intensely pruritic, grouped vesicles on extensor surfaces.

Emergency warning signs

- Severe or worsening symptoms of dermatitis herpetiformis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on skin biopsy showing granular IgA deposition at the dermal papillae. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include a strict gluten-free diet and dapsone (which rapidly controls skin symptoms)
- Sulfapyridine is an alternative for patients intolerant to dapsone.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with dietary compliance. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 233

Dermatomyositis

Overview

Dermatomyositis is an idiopathic inflammatory myopathy with characteristic skin manifestations, discussed in Chapter ???. In addition to proximal muscle weakness, it presents with heliotrope rash, Gottron's papules, V-sign, shawl sign, and mechanic's hands. It is mediated by complement attack on intramuscular blood vessels (perifascicular shrinkage or wasting on biopsy). Adults with dermatomyositis have an increased risk of underlying malignancy, necessitating age-appropriate cancer screening.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of dermatomyositis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 234

Dermatomyositis

Overview

Dermatomyositis is an idiopathic inflammatory myopathy with characteristic skin manifestations, affecting both children and adults.

Causes

This condition happens because of autoimmune-mediated inflammation of skin and muscle. Cutaneous features include heliotrope rash (violaceous discoloration of the eyelids), Gottron's papules (erythematous to violaceous papules overlying the knuckles), V-sign (erythematous rash on the anterior neck and chest), shawl sign (erythematous rash over shoulders and upper back), mechanic's hands (rough, cracked skin on the fingers), and nail fold capillary changes. Proximal muscle weakness is the hallmark of myopathy.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience both skin and muscle manifestations
- Dermatomyositis in adults carries an increased risk of underlying malignancy (ovarian, lung, GI), necessitating age-appropriate cancer screening.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of dermatomyositis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves CK levels, myositis-specific antibodies (anti-Mi-2, anti-MDA5, anti-TIF1- γ), muscle MRI, EMG, and muscle biopsy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include corticosteroids, steroid-sparing agents (methotrexate, azathioprine, mycophenolate), IVIG, and rituximab. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Malignancy screening is essential in adults.

Chapter 235

Deviated Nasal Septum

Overview

Nasal septal deviation is a misalignment of the cartilaginous or bony septum from the midline that is present to some degree in most individuals.

Causes

This condition happens because of developmental or traumatic factors.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience unilateral or bilateral nasal obstruction, which may be worsened by mucosal swelling during allergic rhinitis or upper respiratory infections
- Deviation may cause recurrent epistaxis, facial pain, and contribute to obstructive sleep apnea.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of deviated nasal septum require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using anterior rhinoscopy and CT imaging. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on septoplasty for severe symptomatic deviation, which may be combined with turbinate reduction for concurrent turbinate enlargement. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well following surgical correction. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 236

DiGeorge Syndrome

Overview

DiGeorge syndrome (22q11.2 deletion syndrome, velocardiofacial syndrome) is a developmental disorder caused by a microdeletion on chromosome 22q11.2, resulting in developmental abnormalities derived from the third and fourth pharyngeal arches. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of digeorge syndrome require immediate medical evaluation

Progression and Stages

You may experience thymic underdevelopment or aplasia (T-cell immunodeficiency of varying severity), hypoparathyroidism (hypocalcemia), congenital heart defects (tetralogy of Fallot, interrupted aortic arch, truncus arteriosus, ventricular septal defect), characteristic facies (broad nasal bridge, small ears, micrognathia), palatal abnormalities (velopharyngeal insufficiency, cleft palate), and learning difficulties, with the severity of immunodeficiency varying widely. Your outlook depends on the severity of associated anomalies. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with FISH or chromosomal microarray showing 22q11.2 deletion. Management addresses individual manifestations: cardiac surgery, calcium/vitamin D supplementation for hypocalcemia, thymic transplantation for complete aplasia, and developmental/educational support.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 237

Diabetes Insipidus

Overview

Diabetes insipidus is a condition of impaired water homeostasis caused by deficient production (central DI) or action (nephrogenic DI) of antidiuretic hormone (ADH/vasopressin). This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition results from disruption of normal diabetes insipidus function.

- Central DI results from decreased ADH secretion due to pituitary/hypothalamic disease (tumors, surgery, trauma, infiltrative disease, idiopathic), while nephrogenic DI results from kidney resistance to ADH (lithium use is the most common acquired cause)
- Genetic forms include X-linked aquaporin-2 mutations).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience polyuria (output >3 L/day, often 5–20 L/day), polydipsia, nocturia, and dilute urine (specific gravity <1.005 , osmolality <300 mOsm/kg), with

- severe dehydration and hypernatremia possibly occurring if water intake is restricted.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of diabetes insipidus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves the water deprivation test, with desmopressin administration differentiating central DI (improved urine concentration) from nephrogenic DI (no response). Treatment for central DI is desmopressin (DDAVP) replacement, while Treatment for nephrogenic DI includes thiazide diuretics, low-salt diet, and avoidance of nephrotoxic agents.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with appropriate management.

Chapter 238

Diabetic Ketoacidosis

Overview

Diabetic ketoacidosis (DKA) is a life-threatening metabolic emergency characterized by hyperglycemia (glucose >250 mg/dL), metabolic acidosis (pH <7.3 , bicarbonate <18 mEq/L), and ketosis (elevated ketones in blood and urine). It occurs predominantly in T1DM but can occur in T2DM under significant physiological stress. Precipitants include insulin non-compliance, infections, new-onset diabetes, heart attack, and medications.

Causes

This condition happens because of absolute or relative insulin deficiency counteracted by counter-regulatory hormones (glucagon, cortisol, catecholamines), leading to lipolysis, ketone body production (beta-hydroxybutyrate, acetoacetate), and osmotic diuresis causing dehydration and electrolyte depletion.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience polyuria, polydipsia, nausea, vomiting, abdominal pain, Kussmaul respirations (deep, rapid breathing), and a fruity breath odor.

Emergency warning signs

- Severe or worsening symptoms of diabetic ketoacidosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on blood glucose, arterial blood gas, serum ketones, and anion gap calculation. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on aggressive IV fluid resuscitation, continuous IV insulin infusion, and careful potassium replacement. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with prompt treatment
- Mortality is low in well-managed cases.

Chapter 239

Diabetic Nephropathy

Overview

Diabetic nephropathy is the most common cause of ESRD worldwide, affecting 30–40% of patients with diabetes mellitus (both T1DM and T2DM).

Causes

This condition happens because of hyperglycemia-induced metabolic and hemodynamic changes leading to glomerular hyperfiltration, mesangial expansion, basement membrane thickening, and ultimately nodular glomerulosclerosis (Kimmelstiel-Wilson nodules).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of diabetic nephropathy require immediate medical evaluation

Progression and Stages

Clinical progression includes initial hyperfiltration (increased GFR), microalbuminuria (moderately increased albuminuria), macroalbuminuria (heavily increased albuminuria),

declining GFR, and ESRD. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on persistent albuminuria in the setting of diabetes. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include optimal glycemic control, RAAS blockade (ACE inhibitors or ARBs), SGLT2 inhibitors, BP control (target <130/80), and finerenone (non-steroidal mineralocorticoid receptor antagonist). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 240

Diabetic Peripheral Neuropathic Pain

Overview

Diabetic peripheral neuropathy (DPN) is a neuropathic pain condition affecting approximately 50% of patients with diabetes mellitus, with 15–25% of these patients experiencing neuropathic pain. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bilateral, symmetric, stocking-glove distribution of burning, tingling, shooting, and electric sensations, often worse at night, with reduced sensation, absent ankle reflexes, and allodynia. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive

changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of diabetic peripheral neuropathic pain require immediate medical evaluation

Progression and Stages

This condition happens because of hyperglycemia-induced metabolic pathways (polyol pathway flux, advanced glycation end products, oxidative stress, protein kinase C activation) leading to axonal deterioration, demyelination, and loss of small and large nerve fibers. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. First-line pharmacotherapy includes duloxetine, pregabalin, gabapentin, and TCAs.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Outlook is chronic
- Pain management does not reverse the underlying neuropathy but improves quality of life

- Glycemic control is essential for prevention and slowing progression.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 241

Diabetic Retinopathy

Overview

Causes

This condition happens because of chronic hyperglycemia damaging retinal capillaries, causing vascular occlusion, non-perfusion, microaneurysms, hemorrhages, hard exudates, and cotton-wool spots (non-proliferative DR).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of diabetic retinopathy require immediate medical evaluation

Progression and Stages

Your outlook depends on glycemic control and stage at diagnosis. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Diabetic retinopathy is a microvascular complication of diabetes mellitus and a leading cause of blindness in working-age adults.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves dilated fundus examination and fundus photography. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include optimal glycemic control, anti-VEGF intravitreal injections for diabetic macular swelling, focal or grid laser photocoagulation, and panretinal photocoagulation for proliferative disease
- Regular screening is recommended for all diabetic patients.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 242

Digoxin Toxicity

Overview

Digoxin toxicity is a pharmaceutical overdose involving a cardiac glycoside that inhibits $\text{Na}^+/\text{K}^+-\text{ATPase}$, increasing intracellular calcium and enhancing cardiac contractility. Toxicity occurs with therapeutic excess (most common, especially with kidney impairment, hypokalemia, hypomagnesemia, hypercalcemia, or drug interactions) or acute overdose.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience digestive tract (nausea, vomiting, anorexia), neurological (visual disturbances—yellow-green halos, blurred vision, confusion), and cardiac manifestations including any bradyarrhythmia, atrial tachycardia with block, accelerated junctional rhythm, and bidirectional ventricular tachycardia (pathognomonic)
- Hyperkalemia is a marker of severe acute toxicity.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of digoxin toxicity require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with ECG findings and serum digoxin level. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include digoxin immune Fab (Digibind) as the definitive antidote for life-threatening arrhythmias or hyperkalemia. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with timely administration of Fab fragments. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 243

Dilated Cardiomyopathy

Overview

Dilated cardiomyopathy is a condition characterized by ventricular chamber dilation and systolic dysfunction in the absence of coronary artery disease, valvular disease, or hypertensive heart disease. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Causes include familial or genetic factors (mutations in titin, lamin A/C, and other cytoskeletal genes in 30–50%), viral myocarditis, alcohol, toxins (doxorubicin), peripartum cardiomyopathy, and tachycardia-mediated cardiomyopathy. This condition happens because of impaired myocardial contractility leading to ventricular dilation. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol

- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience heart failure symptoms. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of dilated cardiomyopathy require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia

- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

- Doctors diagnose this based on echocardiography showing a dilated left ventricle with reduced ejection fraction
- Endomyocardial biopsy may be indicated for suspected myocarditis.
- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)

- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Management follows guideline-directed medical therapy for heart failure with reduced ejection fraction
- Anticoagulation may be considered for high thromboembolic risk
- Genetic counseling and family screening are recommended.
- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Your outlook depends on the underlying cause and response to therapy. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 244

Diphtheria

Overview

Diphtheria is a serious bacterial infection caused by *Corynebacterium diphtheriae* that produces an exotoxin causing tissue death and systemic toxicity. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of infection with toxigenic strains of *C. diphtheriae*. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience a tough, gray-white pseudomembrane on the pharynx and tonsils, neck swelling (“bull neck”), fever, and toxic appearance
- The exotoxin can cause myocarditis and neuropathy.
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of diphtheria require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment

- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using culture on tellurite agar and toxin detection. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

The outlook has been dramatically improved by vaccination with diphtheria toxoid (as part of DTaP/Tdap), which has made the disease rare in developed countries. Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures

- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include diphtheria antitoxin and antibiotics (penicillin or erythromycin). Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 245

Diphtheria (*Corynebacterium diphtheriae*)

Overview

This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of *C. diphtheriae* colonizing the nasopharynx or skin and producing diphtheria toxin, a potent ADP-ribosyltransferase that inactivates elongation factor 2 (EF-2), inhibiting protein synthesis and causing cell tissue death, with toxin causing local tissue destruction (pseudomembrane formation) and systemic effects (myocarditis, neuritis). Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions

- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of diphtheria (*corynebacterium diphtheriae*) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

You may experience respiratory diphtheria: sore throat, difficulty swallowing, low-grade fever, and a characteristic grayish-white, thick, adherent pseudomembrane over the tonsils, pharynx, or larynx, with airway obstruction being the primary local risk, and systemic complications including myocarditis and neuropathy.

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, with Gram stain and culture confirmation. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Diphtheria is a toxin-mediated disease caused by *Corynebacterium diphtheriae*, a gram-positive bacillus, with sporadic cases in high-income countries and outbreaks persisting in regions with low vaccination coverage, transmission by respiratory droplets or contact with infected skin lesions. Hand hygiene with soap and water or alcohol-based sanitizer

Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include diphtheria antitoxin (equine) administered immediately and antibiotics (erythromycin or penicillin G). outlook: mortality 5–10%, lower with antitoxin. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery

- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 246

Disseminated Intravascular Coagulation

Overview

Disseminated intravascular coagulation is an acquired, life-threatening coagulation disorder characterized by systemic activation of coagulation, leading to widespread microvascular blood clot formation and consumption of platelets and clotting factors, resulting in a paradoxical bleeding tendency.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience widespread bleeding (from IV sites, surgical wounds, mucosal surfaces), microvascular blood clot formation (organ dysfunction), and hemodynamic instability.

Emergency warning signs

- Severe or worsening symptoms of disseminated intravascular coagulation require immediate medical evaluation

Progression and Stages

Your outlook depends on the underlying cause and severity. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

The condition is always secondary to an underlying condition: sepsis (most common cause), trauma, malignancy, obstetric complications (placental abruption, amniotic fluid blockage by a blood clot), transfusion reactions, and severe liver disease.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical findings and laboratory evidence: thrombocytopenia, prolonged PT and aPTT, low fibrinogen, elevated D-dimer and fibrin degradation products, and schistocytes on peripheral smear, with the ISTH DIC score aiding diagnosis. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Management targets the underlying cause, with supportive care including platelet and FFP transfusions for active bleeding, cryoprecipitate for fibrinogen <100 mg/dL, and heparin for thrombotic predominant DIC.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 247

Dissociative Amnesia

Overview

Dissociative amnesia has a prevalence of 1–3% in the general population.

Causes

This condition happens because of stress-induced disruption of hippocampal and prefrontal memory encoding and retrieval processes, typically associated with trauma (sexual assault, combat, natural disaster, interpersonal violence).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience inability to recall important autobiographical information, usually of a traumatic or stressful nature, that is inconsistent with ordinary forgetting, with the amnesia possibly being localized, selective, generalized, or systematized.

Emergency warning signs

- Severe or worsening symptoms of dissociative amnesia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, with neuroimaging and EEG to rule out organic amnesia. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include psychotherapy (psychodynamic, CBT, hypnotherapy) focused on recovery from trauma and integration. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good for localized and selective amnesia—memory typically returns spontaneously or with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 248

Dissociative Identity Disorder

Overview

Dissociative identity disorder has a prevalence of 1–1.5% in the general population, though it is frequently underdiagnosed. The condition typically arises from severe, repetitive, early childhood trauma before age 6–9, when the child’s personality is still developing and dissociation serves as a psychological defense, with evidence of functional differences between identities on neuroimaging.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience disruption of identity involving two or more distinct personality states (alter identities), each with its own relatively enduring pattern of perceiving, relating to, and thinking about the environment and self, accompanied by recurrent gaps in the recall of everyday events, important personal information, and/or traumatic events.

Emergency warning signs

- Severe or worsening symptoms of dissociative identity disorder require immediate medical evaluation

Progression and Stages

Treatment typically involves long-term, phase-oriented treatment focusing on safety/stabilization, trauma processing, and identity integration. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview (SCID-D). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Treatment aims for improved cooperation between identities or integration.

Chapter 249

Diverticulitis

Overview

Diverticulitis is inflammation or infection of a diverticulum, occurring when a diverticulum becomes obstructed by fecal material, leading to microperforation and localized inflammation.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of uncomplicated diverticulitis include left lower quadrant pain, fever, leukocytosis, and altered bowel habits
- Complicated diverticulitis may involve abscess formation, perforation with peritonitis, fistula formation (colovesical fistula most common), or stricture.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of diverticulitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on CT abdomen/pelvis with contrast, which demonstrates pericolic fat stranding, wall thickening, and abscess. Treatment for uncomplicated diverticulitis includes antibiotics (fluoroquinolones + metronidazole, or amoxicillin-clavulanate) and a liquid diet progressing to a high-fiber diet
- Complicated diverticulitis requires hospitalization, IV antibiotics, and through the skin removal of fluid for abscesses.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is good with appropriate treatment
- Elective colectomy is considered after two or more episodes of complicated diverticulitis or for immunocompromised patients.

Chapter 250

Diverticulosis

Overview

Diverticulosis is the presence of small, bulging pouches (diverticula) in the wall of the colon, most commonly in the sigmoid colon. It affects over 50% of individuals over 60 in Western countries.

Causes

This condition happens because of increased intraluminal pressure at weak points in the colonic wall where vasa recta penetrate the circular muscle layer, and is strongly associated with low-fiber diets, obesity, and aging.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience mostly asymptomatic presentation, discovered incidentally on colonoscopy or CT.

Emergency warning signs

- Severe or worsening symptoms of diverticulosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on colonoscopy or imaging. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include high-fiber diets and stool softeners for prevention of complications.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Most patients remain asymptomatic.

Chapter 251

Down Syndrome (Trisomy 21)

Overview

- Down syndrome is the most common chromosomal disorder, with an incidence of 1 in 700 live births. It is caused by trisomy 21 (94%), Robertsonian translocation (4%), or mosaic trisomy 21 (2%)
- Risk increases with maternal age.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience flat facial profile, upslanting palpebral fissures, epicanthal folds, Brushfield spots, single transverse palmar crease, hypotonia, joint hyperlaxity, short stature, intellectual disability (mild to moderate), and congenital heart disease (50%). Associated conditions include hearing loss, hypothyroidism, celiac disease, duodenal atresia, and increased risk of leukemia. Prenatal screening includes first-trimester nuchal translucency and maternal serum markers.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of down syndrome (trisomy 21) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Your doctor confirms the diagnosis with karyotype or FISH. Management requires lifelong multidisciplinary care
- Life expectancy has increased to approximately 60 years.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 252

Drowning (Submersion Injury)

Overview

Drowning is respiratory impairment from submersion or immersion in a liquid medium. It is a leading cause of unintentional injury death worldwide, particularly in children under 5 years. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of initial breath-holding and panic, laryngospasm (10–15% dry drowning—no removal by suction), then removal by suction of water, surfactant washout, lung swelling, and hypoxemia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: respiratory distress, coughing, tachypnea, cyanosis, foam from mouth/nose, altered mental status, seizures, cardiac arrest
- The primary determinant of the outcome is duration of anoxia.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of drowning (submersion injury) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using history of submersion, clinical presentation, chest X-ray, and arterial blood gas. Management: immediate rescue, CPR if pulseless, airway management, positive pressure ventilation, supplemental oxygen, and rewarming for hypothermic victims. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is better in children and those with submersion time <10 minutes and rapid resuscitation. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 253

Drug Hypersensitivity

Overview

Drug hypersensitivity reactions are adverse drug reactions with an immunologic basis, representing approximately 5–10% of all adverse drug reactions. The condition may be immediate (IgE-mediated, occurring within 1–6 hours: urticaria, angioedema, anaphylaxis) or delayed (T-cell mediated, occurring hours to days after exposure: maculopapular exanthema, DRESS syndrome, Stevens-Johnson syndrome, toxic epidermal necrolysis, acute generalized exanthematous pustulosis).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Common culprits include beta-lactam antibiotics, sulfonamides, anticonvulsants, and NSAIDs.

Emergency warning signs

- Severe or worsening symptoms of drug hypersensitivity require immediate medical evaluation

Progression and Stages

Your symptoms depend on the type and severity of reaction. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle

symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves detailed clinical history, skin testing (for IgE-mediated reactions), drug provocation tests, and lymphocyte transformation tests (for delayed reactions). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include drug avoidance, acute treatment of reactions, and in some cases, desensitization protocols for essential medications. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable with avoidance of the offending drug. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 254

Dry Eye Disease

Overview

Dry eye disease is a multifactorial disorder of the ocular surface characterized by loss of homeostasis of the tear film, affecting approximately 5–30% of the global population, with higher prevalence in the elderly, women, and those in arid or windy environments.

Causes

This condition happens because of tear film instability due to aqueous-deficient (reduced lacrimal gland production, as in Sjögren's syndrome, aging, or medication side effects) or evaporative causes (meibomian gland dysfunction, lid abnormalities, or environmental factors).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience dryness, grittiness, burning, foreign body sensation, photophobia, and blurred vision
- Signs include reduced tear break-up time (less than 10 seconds), Schirmer's test less than 5 mm in 5 minutes, corneal staining, and meibomian gland dropout.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of dry eye disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by tear osmolarity testing and inflammatory markers. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves stepwise: artificial tears, punctal plugs, topical cyclosporine A or lifitegrast, secretagogues, warm compresses and lid hygiene, and environmental modifications. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and

support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with most patients achieving symptom control with appropriate management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 255

Duchenne Muscular Dystrophy

Overview

Duchenne muscular dystrophy (DMD) is the most common and severe childhood muscular dystrophy, affecting approximately 1 in 3500–5000 live-born males. It is an X-linked recessive disorder caused by mutations in the dystrophin gene (DMD) on Xp21, leading to absence of the dystrophin protein essential for sarcolemmal integrity. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive proximal muscle weakness beginning between ages 2–5, with Gowers' sign (using hands to "climb up" the thighs when rising from the floor), waddling gait, toe walking, and calf pseudohypertrophy
- By age 10–12, most patients lose ambulation. Cardiomyopathy and respiratory failure develop in adolescence.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of duchenne muscular dystrophy require immediate medical evaluation

Progression and Stages

Treatment options include corticosteroids (deflazacort, prednisone) to slow progression, cardiac management (ACE inhibitors, beta-blockers), respiratory support, and newer therapies including exon-skipping agents (eteplirsen, golodirsen, viltolarsen, casimersen for specific mutations) and gene therapy (delandistrogene moxeparvovec). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on elevated CK (10–100 times normal), genetic testing showing dystrophin mutations, and muscle biopsy showing dystrophic changes with absent dystrophin on immunostaining.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is severe, with most patients losing ambulation by age 10–12 and survival into the third decade with respiratory support.

Chapter 256

Ductal Carcinoma In Situ (DCIS)

Overview

Ductal carcinoma in situ (DCIS) is a non-invasive neoplastic growth of epithelial cells confined within the mammary ductal-lobular system without penetration of the basement membrane, accounting for 20–25% of screen-detected breast malignancies. Driven by localized genetic mutations and hormonal stimulation, DCIS is categorized by architectural patterns (comedo, cribriform, papillary, solid), with comedo DCIS presenting central high-grade tissue death and dystrophic calcium buildup. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)

- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of ductal carcinoma in situ (dcis) require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Diagnosis is established via stereotactic or ultrasound-guided core needle biopsy. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

The condition is overwhelmingly asymptomatic and detected on routine screening mammography as microcalcifications (clustered, pleomorphic, or linear branching). Avoid tobacco use and limit alcohol consumption. Maintain a healthy weight through diet and exercise. Eat a balanced diet rich in fruits, vegetables, and whole grains. Protect skin from excessive sun exposure and avoid tanning beds. Get vaccinated against cancer-causing infections (HPV, hepatitis B). Participate in recommended cancer screening programs. Know your family history and consider genetic testing if indicated. Avoid exposure to known carcinogens in the workplace and environment.

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

- Treatment focuses on breast-conserving surgery (lumpectomy) with negative margins (> 2 mm) followed by radiation therapy and adjuvant tamoxifen/aromatase inhibitors for ER-positive lesions
- Total mastectomy is indicated for widespread or multicentric disease.
- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team. Manage treatment side effects with supportive medications. Maintain adequate nutrition with help from a dietitian. Stay physically active within your energy limits. Join cancer support groups for emotional support and shared experiences. Seek counseling or mental health support for anxiety and depression. Communicate openly with your healthcare team about symptoms. Plan for work and financial considerations during treatment. Consider complementary therapies.

(acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Most people recover well, with 20-year disease-specific survival exceeding 98%. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 257

Duodenal Ulcer

Overview

Duodenal ulcer is the most common type of peptic ulcer, occurring predominantly in the first part of the duodenum (duodenal bulb). *Helicobacter pylori* is the primary cause in 90–95% of cases, with NSAIDs as the second most common cause.

Causes

This condition happens because of acid hypersecretion due to loss of negative feedback from somatostatin-producing D-cells infected by *H. pylori*.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience epigastric pain relieved by food and antacids ("hunger pain"), nocturnal pain, and bloating.

Emergency warning signs

- Severe or worsening symptoms of duodenal ulcer require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on upper endoscopy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on *H. pylori* eradication (triple or quadruple therapy for 14 days), PPI therapy, and smoking cessation. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive
- Unlike gastric ulcers, duodenal ulcers rarely undergo cancerous transformation.

Chapter 258

Dupuytren Contracture

Overview

Dupuytren contracture is a progressive fibroproliferative disorder of the palmar fascia resulting in nodule formation and fixed flexion deformities of the digits.

Causes

This condition happens because of abnormal myofibroblast growth and excessive collagen type III deposition, leading to thickening and shortening of the palmar aponeurosis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features begin with painless palmar skin pitting and subcutaneous nodules (most commonly over the 4th and 5th rays), progressing to dense fascial cords and fixed flexion contractures at the metacarpophalangeal (MCP) and proximal interphalangeal (PIP) joints (positive Tabletop test).

Emergency warning signs

- Severe or worsening symptoms of dupuytren contracture require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include observation for mild non-functional cases, collagenase *Clostridium histolyticum* injections, needle aponeurotomy, or subtotal palmar fasciectomy for functional impairment ($> 30^\circ$ MCP contracture). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 259

Dysbetalipoproteinemia

Overview

Dysbetalipoproteinemia (type III hyperlipoproteinemia, remnant removal disease) is a disorder of chylomicron and VLDL remnant clearance caused by homozygosity for apolipoprotein E2 (apoE2/E2 genotype) in conjunction with a second metabolic hit (obesity, diabetes, hypothyroidism, or estrogen deficiency). Only 1–5% of individuals with the apoE2/E2 genotype develop the full phenotype, which is otherwise rare (prevalence approximately 1 in 10,000).

Causes

This condition happens because of impaired liver clearance of remnant lipoproteins due to poor binding of apoE2 to the LDL receptor.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience both cholesterol and triglyceride elevation (typically in equal proportion, greater than 300 mg/dL), tuberoeruptive xanthomas, and palmar crease xanthomas (pathognomonic).

Emergency warning signs

- Severe or worsening symptoms of dysbetalipoproteinemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using ultracentrifugation showing a broad β -band, VLDL cholesterol-to-total triglyceride ratio greater than 0.30, and apoE genotyping (E2/E2). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment options include lifestyle modification
- Statins and fibrates are synergistic and often normalize lipids.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 260

Dysmenorrhea

Overview

Dysmenorrhea is painful menstrual cramps, the most common gynecologic complaint, affecting 50–80% of reproductive-age women. Primary dysmenorrhea occurs without organic pelvic pathology, typically beginning within a few years of menarche, caused by excessive endometrial prostaglandin production leading to myometrial hypercontractility and uterine reduced blood flow.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience cramping lower abdominal pain radiating to the back and thighs, beginning just before or at the onset of menstruation and lasting 1–3 days. Secondary dysmenorrhea is due to underlying pathology (endometriosis, adenomyosis, uterine fibroids, pelvic inflammatory disease).

Emergency warning signs

- Severe or worsening symptoms of dysmenorrhea require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Secondary dysmenorrhea requires pelvic ultrasound to identify underlying causes. Treatment of primary dysmenorrhea includes NSAIDs and combined oral contraceptives.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 261

Dystonia

Overview

Dystonia is a movement disorder characterized by sustained or intermittent muscle contractions causing abnormal, often repetitive, movements, postures, or both, with a prevalence of 1 in 3,000. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of dysfunction of the basal ganglia-thalamo-cortical motor circuit, with genetic forms caused by mutations in genes including TOR1A (DYT1, early-onset generalized), THAP1 (DYT6, mixed phenotype), GCH1 (DYT5, dopa-responsive dystonia), and SGCE (DYT11, myoclonus-dystonia). Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms vary depending on distribution: focal (most common, eg., cervical dystonia/spasmodic torticollis, blepharospasm, oromandibular dystonia, laryngeal dystonia/spasmodic dysphonia, writer's cramp), segmental (involving two contiguous body regions), multifocal, hemidystonia, or generalized. Dystonic movements are typically directional, patterned, and often action-specific, worsened by stress and fatigue, and may be alleviated by sensory tricks (*geste antagoniste*). Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of dystonia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with genetic testing for early-onset or familial cases. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function

- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include oral medications (trihexyphenidyl, benzodiazepines, baclofen, tetrabenazine), botulinum toxin injections (first-line for focal dystonia), deep brain stimulation of the globus pallidus internus (GPi-DBS, most effective for generalized and cervical dystonia), and levodopa trial for suspected dopa-responsive dystonia. outlook: focal dystonia is usually non-progressive after initial spread
- Generalized dystonia may progress over years but stabilize with treatment.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 262

Eastern Equine Encephalitis

Overview

Eastern equine encephalitis is a rare but severe mosquito-borne viral disease caused by Eastern equine encephalitis virus (EEEV), an alphavirus, and is the most severe arboviral encephalitis in North America, with 5–10 human cases annually in the United States and a case fatality rate of 30–50%.

Causes

This condition happens because of EEEV crossing the blood-brain barrier and infecting neurons, causing neuronal tissue death, perivascular cuffing, and intense neutrophilic inflammation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of eastern equine encephalitis require immediate medical evaluation

Progression and Stages

You may experience an incubation of 4–10 days, with a systemic phase (sudden fever, chills, headache, muscle pain, malaise) followed by an encephalitic phase (altered mental status, seizures, cranial nerve palsies, and focal deficits), with rapid progression to coma and respiratory failure. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using EEEV-specific IgM in CSF or serum by ELISA. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Treatment typically involves supportive care. outlook: 30–50% mortality
- Survivors have a high rate (50–80%) of severe nervous system sequelae.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 263

Ebola Virus Disease

Overview

Ebola virus disease is a severe, often fatal involving bleeding fever caused by Ebola virus, with five species: Zaire (most virulent), Sudan, Bundibugyo, Taï Forest, and Reston, with the 2014–2016 West Africa outbreak being the largest (28,646 cases, 11,323 deaths), and transmission through direct contact with blood, secretions, or bodily fluids of infected humans or animals. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of the virus entering through mucosal surfaces or breaks in skin, infecting dendritic cells, macrophages, and monocytes, spreading to lymph nodes, liver, spleen, and adrenal glands, causing profound immunosuppression and release of pro-inflammatory cytokines (cytokine storm), with endothelial cell dysfunction, DIC, and multi-organ failure. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)

- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of ebola virus disease require immediate medical evaluation

Progression and Stages

You may experience an incubation of 2–21 days, with prodrome (sudden fever, severe headache, muscle pain, malaise), digestive tract phase (severe watery diarrhea, nausea, vomiting, abdominal pain), and systemic phase (multi-organ failure, coagulopathy, hypotension/shock, and nervous system involvement). The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection.

Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using RT-PCR of blood. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Treatment options include supportive care and two FDA-approved monoclonal antibody therapies: mAb114 (ansuvimab) and REGN-EB3. outlook: Zaire species 50–80% mortality without treatment
- 33–50% with supportive care and monoclonal antibodies.
- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 264

Eclampsia

Overview

Eclampsia is a hypertensive disorder of pregnancy defined as the occurrence of generalized tonic-clonic seizures in a woman with preeclampsia that cannot be attributed to other causes. It occurs in 1 in 2000 deliveries in developed countries and more frequently in resource-limited settings. Seizures may occur antepartum (38%), intrapartum (18%), or postpartum (44%). This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience generalized tonic-clonic seizures in the setting of preeclampsia.

Emergency warning signs

- Severe or worsening symptoms of eclampsia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves a medical emergency: magnesium sulfate (4–6 g IV loading dose followed by 1–2 g/hour infusion) is the first-line anticonvulsant, superior to diazepam or phenytoin
- Airway protection, seizure precautions, and antihypertensive therapy are essential. The main treatment typically involves delivery after maternal stabilization.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with prompt treatment
- Magnesium toxicity doctors typically treat it with calcium gluconate.

Chapter 265

Ectopic Pregnancy

Overview

Ectopic pregnancy occurs when a fertilized ovum implants outside the uterine cavity, most commonly in the fallopian tube (95%), but also in the cervix, ovary, abdominal cavity, or previous cesarean scar. The incidence is 1–2% of all pregnancies. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

This condition happens because of impaired tubal function or anatomy preventing normal embryo transport.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience amenorrhea, vaginal bleeding, and unilateral lower abdominal pain
- Ruptured ectopic pregnancy presents with sudden severe pain, syncope, involving bleeding shock, and peritonitis.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of ectopic pregnancy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using serum β -hCG and transvaginal ultrasound (empty uterine cavity, adnexal mass, free fluid)
- A discriminatory zone of β -hCG >2000 IU/L with no intrauterine pregnancy suggests ectopic pregnancy.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- The right treatment depends on stability: stable patients with small ectopic may be treated with methotrexate
- Unstable patients require using small incisions and a camera salpingectomy or salpingotomy.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with early diagnosis
- Ruptured ectopic pregnancy is life-threatening.

Chapter 266

Edwards Syndrome (Trisomy 18)

Overview

Edwards syndrome is a chromosomal disorder caused by trisomy 18, with an incidence of 1 in 5000 live births. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe intellectual disability, growth restriction, low-set malformed ears, micrognathia, prominent occiput, clenched fists with overlapping fingers, rocker-bottom feet, and congenital heart disease. Median survival is less than 1 year
- Only 5–10% survive to age 1 year.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of edwards syndrome (trisomy 18) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using karyotype.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment typically involves supportive and palliative.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 267

Ehlers-Danlos Syndrome

Overview

Ehlers-Danlos syndrome (EDS) is a heterogeneous group of connective tissue disorders caused by defects in collagen synthesis and processing, with a combined prevalence of 1 in 5,000. The most common subtype, hypermobile EDS (hEDS, 90%), has unknown genetic basis. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition results from disruption of normal ehlers-danlos syndrome function.

- Classical EDS (cEDS) results from COL5A1/A2 mutations causing abnormal type V collagen
- Vascular EDS (vEDS) results from COL3A1 mutations causing abnormal type III collagen.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms vary depending on subtype: hEDS presents with joint hypermobility, skin hyperextensibility, easy bruising, chronic pain, and recurrent dislocations
- CEDS features velvety, hyperextensible skin, atrophic scars, and easy bruising
- VEDS presents with thin, translucent skin, characteristic facial appearance (prominent eyes, thin nose, lobeless ears), and life-threatening arterial, intestinal, or uterine rupture.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of ehlers-danlos syndrome require immediate medical evaluation

Progression and Stages

Doctors diagnose this based on Villefranche classification and genetic testing. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on symptomatic treatment: physical therapy for joint instability, pain management, wound care, and cardiovascular surveillance in vEDS. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is normal for hEDS and cEDS
- VEDS has a median survival of 50 years, primarily due to arterial rupture.

Chapter 268

Ehrlichiosis

Overview

Ehrlichiosis is a tick-borne illness caused by obligate intracellular bacteria of the family Anaplasmataceae, with human monocytic ehrlichiosis (HME) caused by *Ehrlichia chaffeensis*, transmitted by the Lone Star tick (*Amblyomma americanum*), with over 1500 cases annually in the United States.

Causes

This condition happens because of *E. chaffeensis* infecting monocytes and macrophages, forming intracytoplasmic microcolonies (morulae), and inhibiting phagolysosomal fusion.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 5–14 days, with fever, severe headache, muscle pain, malaise, nausea, and rigors, with rash being less common (30%) than in RMSF, and laboratory findings of leukopenia, thrombocytopenia, and elevated liver transaminases being characteristic.

Emergency warning signs

- Severe or worsening symptoms of ehrlichiosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using PCR of blood (high sensitivity in the first week). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include doxycycline. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with treatment
- Mortality 1–3%.

Chapter 269

Electrical Injury and Lightning Strike

Overview

Electrical injuries from high-voltage ($>1,000$ V) and low-voltage ($<1,000$ V, typically household 110–240 V) sources cause damage via thermal burns, direct cellular injury, and electroporation of cell membranes. Current follows the path of least resistance: nerves and blood vessels are most affected, followed by muscle, then skin and bone. This condition results from acute injury or exposure to harmful agents.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: cutaneous burns (entry/exit wounds—may be deceptively small), muscle tissue death, rhabdomyolysis, compartment syndrome, irregular heartbeats, neurological injury, and fractures. Lightning strike produces a massive direct current (DC) shock of up to 30 million volts and 30,000 amps. Clinical features: cardiac arrest, respiratory arrest, confusion, amnesia, keraunographic markings, tympanic membrane rupture, and cataracts.

Emergency warning signs

- Severe or worsening symptoms of electrical injury and lightning strike require immediate medical evaluation

Progression and Stages

Management: advanced cardiac life support, fluid resuscitation, surgical fasciotomy for compartment syndrome, and wound care. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using history and physical examination. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Lightning strike patients in cardiac arrest may be salvageable with prolonged resuscitation.

Chapter 270

Empyema

Overview

Causes

This condition happens because of bacterial infection spreading to the pleural space.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience persistent fever despite appropriate antibiotic therapy for pneumonia, pleuritic chest pain, and shortness of breath.

Emergency warning signs

- Severe or worsening symptoms of empyema require immediate medical evaluation

Progression and Stages

Treatment options include chest tube removal of fluid, intrapleural fibrinolytics (alteplase and DNase), VATS decortication for advanced disease, and appropriate antibiotics. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Empyema is the accumulation of purulent fluid within the pleural space, most commonly as a complication of pneumonia, thoracic surgery, or trauma
- It progresses through exudative, fibrinopurulent, and organizational stages.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using chest X-ray, ultrasound, and diagnostic thoracentesis showing purulent fluid with low pH, low glucose, high LDH, and positive Gram stain or culture. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with prompt removal of fluid and appropriate antibiotics. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 271

End-Stage Renal Disease

Overview

Causes

This condition happens because of progressive nephron loss from any cause of CKD.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience uremic syndrome: nausea, vomiting, anorexia, itching, pericarditis, encephalopathy, peripheral neuropathy, and restless legs syndrome.

Emergency warning signs

- Severe or worsening symptoms of end-stage renal disease require immediate medical evaluation

Progression and Stages

End-stage kidney disease (ESRD) is the final stage of CKD (GFR <15 mL/min/1.73 m²) where the kidneys can no longer sustain life without kidney replacement therapy. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on GFR criteria and clinical manifestations. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Post-transplant immunosuppression (tacrolimus, mycophenolate, prednisone) carries risks of infection and malignancy. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Treatment focuses on dialysis (hemodialysis or peritoneal dialysis) or kidney transplantation
- Kidney transplantation is the treatment of choice for eligible patients, offering superior survival and quality of life.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Living donor transplantation provides the best outcomes, while dialysis has a 5-year survival of approximately 40–50%.

Chapter 272

Endometrial / Uterine Cancer

Overview

Endometrial cancer is the most common gynecologic malignancy in developed countries. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Two broad histologic types exist: type I (endometrioid adenocarcinoma, 80%, estrogen-driven, good outlook) and type II (serous carcinoma, clear cell carcinoma, not estrogen-driven, poor outlook). Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle

- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Clinical The condition usually appears as postmenopausal bleeding (90% present with bleeding). Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of endometrial / uterine cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs.

Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment for early-stage disease is total hysterectomy with bilateral salpingo-oophorectomy
- Progestin therapy may be used for fertility-sparing management. In most cases, the outlook is good: 5-year survival is 95% for stage I disease.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using endometrial biopsy. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status

- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 273

Endometriosis

Overview

Endometriosis is the most common cause of chronic pelvic pain and a leading cause of infertility. Beyond first-line therapies (NSAIDs, combined oral contraceptives), surgical removal of deeply infiltrating endometriosis may be necessary for pain refractory to medical management. GnRH agonists or antagonists with add-back therapy are used for moderate to severe disease. Dienogest and norethindrone acetate are effective progestin-only options. Treatment of endometriosis-associated infertility includes using small incisions and a camera removal followed by IVF.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of endometriosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 274

Endometriosis

Overview

Endometriosis is defined by the presence of endometrial-like tissue outside the uterus, affecting approximately 10% of reproductive-age women, with the most common sites being the ovaries (endometriomas—“chocolate cysts”), uterosacral ligaments, pelvic peritoneum, and rectovaginal septum. The pathogenesis is debated, with Sampson’s theory of retrograde menstruation being the most widely accepted.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience chronic pelvic pain (dysmenorrhea, dyspareunia, chronic pelvic pain), infertility (present in 30–50% of women with endometriosis), and cyclic symptoms. Diagnosis is typically made by laparoscopy with visualization of endometriotic implants and histological confirmation, with MRI useful for deep infiltrating endometriosis.

Emergency warning signs

- Severe or worsening symptoms of endometriosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include NSAIDs for pain, combined oral contraceptives, progestins, GnRH agonists/antagonists with add-back therapy, and surgical removal/ablation of endometriotic lesions. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with recurrence possible after treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 275

Eosinophilic Esophagitis

Overview

Eosinophilic esophagitis (EoE) is a chronic, immune-mediated inflammatory condition of the esophagus characterized by symptoms of esophageal dysfunction and histologic evidence of eosinophil-predominant inflammation. It affects approximately 1 in 2,000 individuals, with a strong male predominance and peak incidence in children and adults aged 20–40.

Classification

Cause Allergic/Immune

Organ System Gastrointestinal/Esophagus

Pathophysiology Th2-mediated allergic response to food or environmental allergens
-> eosinophilic infiltration and degranulation in esophageal mucosa -> epithelial injury, fibrosis, and stricture formation

Duration Chronic

Onset Gradual

Mode of Transmission Non-communicable (immune-mediated)

Clinical Course Chronic, relapsing-remitting

Severity Mild to Severe

Extent of Disease Localized (esophagus)

Age Group Children and Adults

Epidemiology Male predominance (3:1); increasing incidence in developed countries

ICD-11 Code DC20.0

Causes

Eosinophilic esophagitis arises from a type 2 helper T-cell (Th2)-driven allergic inflammatory response to food allergens (most commonly milk, wheat, egg, soy, and peanuts) or aeroallergens in genetically susceptible individuals. The resulting eosinophil infiltration into the esophageal epithelium releases cytotoxic granule proteins that damage tissue, promote fibrosis, and cause dysmotility. This condition develops from a combination of genetic and environmental factors that disrupt normal body processes. When these normal

processes are disrupted, it leads to the symptoms and signs of the condition. Several factors can work together to trigger or make the condition worse. Knowing the specific cause helps your doctor choose the most effective treatment.

Risk Factors

- Atopic conditions (asthma, allergic rhinitis, atopic dermatitis, food allergies)
- Male sex (3:1 male-to-female ratio)
- Family history of eosinophilic esophagitis
- Living in a cold or dry climate (environmental aeroallergen exposure)
- Season of birth (spring/summer linked to higher risk)

Signs and Symptoms

Early symptoms

- Early manifestations of eosinophilic esophagitis may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- In adults: dysphagia (difficulty swallowing) and food impaction, often with solid foods
- In children: abdominal pain, vomiting, regurgitation, nausea, and failure to thrive
- Heartburn or chest pain that does not respond to proton pump inhibitors
- The esophagus may appear with rings (trachealization), longitudinal furrows, white exudates, or strictures on endoscopy
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of eosinophilic esophagitis may occur

Emergency warning signs

- Severe or worsening symptoms of eosinophilic esophagitis require immediate medical evaluation

Progression and Stages

Untreated eosinophilic esophagitis progresses from an inflammatory phenotype (edema, exudates, furrows) to a fibrostenotic phenotype (rings, strictures, narrowing) over years, driven by chronic eosinophilic inflammation. Disease activity fluctuates with allergen exposure, and progression is faster in patients with persistent untreated inflammation.

Complications

- Esophageal food impaction requiring emergency endoscopic removal
- Esophageal stricture formation requiring dilation
- Spontaneous esophageal perforation (Boerhaave syndrome, rare)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Upper endoscopy with esophageal biopsies showing ≥ 15 eosinophils per high-power field after an 8-week PPI trial to exclude PPI-responsive esophageal eosinophilia
- Clinical history of esophageal dysfunction (dysphagia, food impaction)
- Lab tests to check how your organs are working
- Imaging tests

Prevention

- Avoidance of identified trigger foods (guided by allergy testing or elimination diets)
- Management of concurrent atopic conditions
- Go for regular check-ups so any problems are caught early
- Follow your doctor's screening recommendations

Treatment Options

- Topical swallowed corticosteroids (fluticasone or budesonide slurry) to reduce esophageal eosinophilic inflammation
- Dietary elimination therapy (six-food elimination diet, targeted elimination based on allergy testing, or elemental diet)
- Proton pump inhibitors as first-line therapy for patients with PPI-responsive esophageal eosinophilia
- Lifestyle changes and self-care
- Surgery when needed (endoscopic dilation for strictures)

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Eosinophilic esophagitis is a chronic condition requiring long-term management, but most patients achieve symptom control with dietary therapy or topical corticosteroids. Your outlook depends on the specific condition, how advanced it is when found, and how well it responds to treatment. Getting treated early generally leads to better results. Many people manage this condition effectively with the right treatment. Regular check-ups are important to monitor your condition and adjust treatment as needed.

Chapter 276

Eosinophilic Fasciitis (Shulman Syndrome)

Overview

Eosinophilic fasciitis is a rare, fibrosing disorder of the fascia characterized by symmetrical, painful swelling and induration of the skin and subcutaneous tissue, with peripheral eosinophilia, with an incidence of approximately 1 per million, equal sex distribution, and typically affecting adults 40–60 years of age. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

The cause is unknown, but triggers include vigorous exercise or trauma (in up to 50%), infections, and medications (statins, antitubercular drugs), with Disease mechanism involving inflammation and thickening of the deep fascia, infiltration by lymphocytes, eosinophils, macrophages, and plasma cells, leading to collagen deposition and scarring. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience acute or subacute onset of symmetrical, painful swelling and induration of the arms and legs (sparing the hands, feet, and face—a distinguishing feature from systemic sclerosis), with a characteristic “peau d’orange” or cobblestone appearance, and the groove sign (linear depression overlying superficial veins) being a pathognomonic finding, with joint contractures developing due to fascial scarring.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of eosinophilic fasciitis (shulman syndrome) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for a deep incisional biopsy including skin, subcutaneous tissue, fascia, and muscle, showing fasciitis with a mixed inflammatory infiltrate and often eosinophils, with peripheral eosinophilia ($>5\%$ or absolute count $>500/\mu\text{L}$) present in 70–90%.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves with corticosteroids (prednisone 0.5–1 mg/kg/day), with methotrexate, mycophenolate, and azathioprine used as steroid-sparing agents. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Most patients improve with corticosteroids, but joint contractures may persist.

Chapter 277

Eosinophilic Granulomatosis with Polyangiitis

Overview

Eosinophilic granulomatosis with polyangiitis is a rare ANCA-associated vasculitis characterized by asthma, eosinophilia, and necrotizing vasculitis of small and medium-sized vessels with extravascular eosinophilic granulomas, with an incidence of 1–3 per million, equal sex distribution, and typically presenting in the fourth to fifth decade. The condition involves a Th2-driven immune response with prominent eosinophil activation and degranulation, ANCA (typically MPO-ANCA, positive in 30–50%, more commonly in patients with kidney and nervous system involvement), and eosinophilic infiltration of tissues.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of eosinophilic granulomatosis with polyangiitis require immediate medical evaluation

Progression and Stages

Clinical features evolve through three phases: prodromal (asthma, allergic rhinitis, often years before systemic disease), eosinophilic (peripheral eosinophilia $>1500/\mu\text{L}$, eosinophilic pneumonia, gastroenteritis), and vasculitic (systemic vasculitis with mononeuritis multiplex—the most common vasculitic manifestation, purple spots on the skin, glomerulonephritis, cardiomyopathy, and skin nodules), with cardiac involvement (eosinophilic myocarditis, coronary arteritis, heart failure) being the leading cause of death. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on the 2022 ACR/EULAR criteria. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves stratified: patients without poor prognostic factors (FFS five-factor score=0) receive corticosteroids alone
- Patients with poor prognostic factors (FFS ≥ 1) receive corticosteroids plus cyclophosphamide or rituximab.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Your outlook depends on cardiac involvement and kidney function
- 5-year survival exceeds 90%.

Chapter 278

Epidermolysis Bullosa

Overview

Epidermolysis bullosa (EB) is a group of inherited blistering disorders characterized by mechanical fragility of the skin and mucous membranes, with an overall prevalence of 1 in 50,000.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience blister formation at sites of minor friction or trauma, ranging from mild acral blistering (EBS) to widespread blistering, scarring, milia, nail dystrophy, and mitten-hand deformities from repeated scarring (severe DEB). Extracutaneous involvement includes oral, esophageal, and laryngeal blistering, corneal erosions, anemia, growth retardation, and increased risk of aggressive squamous cell carcinoma in severe DEB.

Emergency warning signs

- Severe or worsening symptoms of epidermolysis bullosa require immediate medical evaluation

Progression and Stages

This condition happens because of mutations in genes encoding structural proteins of the skin, classified by the level of blister formation within the basement membrane zone: EB simplex (EBS, keratin 5/14 mutations, intraepidermal cleavage), junctional EB (JEB, laminin 332/collagen XVII mutations, lamina lucida cleavage), and dystrophic EB (DEB, collagen VII mutations, sublamina densa cleavage). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical features, skin biopsy for immunofluorescence mapping, transmission electron microscopy, and genetic testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment typically involves multidisciplinary: gentle skin care, wound dressings (non-adherent), prevention and treatment of infection, nutritional support, esophageal dilation for strictures, physical therapy for contractures, and surveillance for squamous cell carcinoma.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Emerging therapies include gene therapy (beremagene geperpavec for DEB), protein replacement therapy, and cell-based therapies. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Recovery varies widely: EBS has normal life expectancy
- Severe JEB and recessive DEB cause significant morbidity and mortality from infection, malnutrition, and aggressive squamous cell carcinoma.

Chapter 279

Epidural Hematoma

Overview

An epidural hematoma is a traumatic intracranial bleeding accumulating between the inner skull table and the dura mater, most commonly resulting from skull fracture crossing the pterion with laceration of the middle meningeal artery.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of epidural hematoma require immediate medical evaluation

Progression and Stages

You may experience transient loss of consciousness at injury followed by a lucid interval (lasting hours), after which neurological deterioration ensues with severe headache, contralateral hemiparesis, ipsilateral dilated and unreactive pupil (due to uncal herniation with CN III compression), and Cushing triad (high blood pressure, bradycardia, irregular respiration). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate

stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using non-contrast head CT showing a convex, biconvex (lens-shaped) hyperdense collection that does not cross cranial suture lines. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Management requires urgent neurosurgical surgical opening of the skull and hematoma evacuation to prevent fatal brain herniation.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well if evacuated promptly before herniation. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 280

Epiglottitis

Overview

Epiglottitis is a rapidly progressive, life-threatening inflammation of the epiglottis and supraglottic structures.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience abrupt onset of high fever, severe sore throat, drooling, muffled voice (“hot potato” voice), tripod positioning, and inspiratory stridor.

Emergency warning signs

- Severe or worsening symptoms of epiglottitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Lateral neck radiograph shows the “thumbprint sign” (enlarged epiglottis). Airway Treatment typically involves the priority: immediate establishment of a secured airway (endotracheal intubation in the operating room). Blood and epiglottic cultures are obtained, and intravenous antibiotics (ceftriaxone or cefotaxime) are initiated.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Historically caused by *Haemophilus influenzae* type b (Hib), its incidence has declined dramatically due to the Hib vaccine.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with prompt airway management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 281

Epiglottitis

Overview

Epiglottitis (supraglottitis) is a rapidly progressive, life-threatening cellulitis and swelling of the epiglottis and surrounding supraglottic structures (arytenoids, aryepiglottic folds), leading to catastrophic upper airway obstruction. Primary bacterial invasion causes massive supraglottic swelling, narrowing the laryngeal inlet.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features present with acute onset of high fever, severe sore throat, odynophagia, muffled 'hot potato' voice, drooling, and respiratory distress. Patients characteristically assume a 'tripod position' (sitting upright, leaning forward, neck hyperextended, chin thrust forward) to optimize airway caliber. Direct visualization with a tongue depressor is strictly contraindicated due to risk of precipitating complete laryngospasm. Lateral neck X-ray reveals a swollen epiglottis ('thumbprint sign'). Management mandates immediate airway stabilization in an operating room via endotracheal intubation or tracheostomy, followed by parenteral broad-spectrum antibiotics (ceftriaxone plus vancomycin).

Emergency warning signs

- Severe or worsening symptoms of epiglottitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Historically caused by *Haemophilus influenzae* type b (Hib) in young children, widespread Hib vaccination has shifted epidemiology toward older adults and non-typeable *H. influenzae*, *Streptococcus pneumoniae*, *Staphylococcus aureus*, and Beta-hemolytic streptococci.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 282

Epilepsy

Overview

- Epilepsy is a chronic neurological disorder characterized by recurrent, unprovoked seizures resulting from abnormal excessive or synchronous neuronal activity in the brain
- It is defined by two or more unprovoked seizures occurring more than 24 hours apart, or one unprovoked seizure with a high recurrence risk.

Causes

This condition happens because of genetic, structural (eg., cortical dysplasia, tumor, stroke), infectious, metabolic, or immune causes. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Your symptoms depend on seizure type. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of epilepsy require immediate medical evaluation

Progression and Stages

Seizure classification distinguishes focal (partial) from generalized seizures, with further subtypes including absence, tonic-clonic, myoclonic, atonic, and tonic seizures. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis relies on clinical history, EEG showing epileptiform discharges, and neuroimaging. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on antiseizure medications selected based on seizure type and syndrome classification
- Drug-resistant epilepsy may be treated with epilepsy surgery, vagus nerve stimulation, or responsive neurostimulation.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with many patients achieving seizure control with appropriate therapy. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 283

Epistaxis

Overview

Epistaxis is a nosebleed that affects approximately 60% of the population at some point, with most bleeds originating from Kiesselbach's plexus on the anterior nasal septum (anterior epistaxis).

Causes

This condition happens because of local trauma or systemic factors such as dry air, digital trauma, allergic rhinitis, nasal foreign bodies, anticoagulant use, high blood pressure, coagulopathy, and hereditary involving bleeding telangiectasia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bleeding from the anterior or posterior nasal cavity
- Pain or discomfort in the affected area

Less common symptoms

- Posterior bleeds from the sphenopalatine artery are less common but more severe.

Emergency warning signs

- Severe or worsening symptoms of epistaxis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on clinical examination. Anterior epistaxis is managed with direct pressure, chemical burning to seal tissue (silver nitrate), or anterior nasal packing
- Posterior epistaxis requires posterior packing, using a thin camera tube sphenopalatine artery ligation, or embolization.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good, with most cases resolving with simple measures. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 284

Epstein-Barr Virus / Infectious Mononucleosis (Systemic Aspects)

Overview

Epstein-Barr virus is a gamma-herpesvirus that infects > 90% of the global population, with primary infection causing infectious mononucleosis (IM) in adolescents and young adults, and EBV being associated with several malignancies: Burkitt lymphoma, Hodgkin lymphoma, post-transplant lymphoproliferative disorder, nasopharyngeal carcinoma, and gastric carcinoma. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of EBV infecting B lymphocytes via CD21 (CR2) receptor, leading to polyclonal B cell growth, with the host T cell response producing the atypical lymphocytosis characteristic of IM. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience an incubation of 30–50 days, with the classical triad of fever, pharyngitis (severe, exudative), and cervical lymphadenopathy, along with profound fatigue, splenomegaly, hepatomegaly, and rash (especially with ampicillin/amoxicillin administration).
Fever and chills
Fatigue and malaise
Localized pain and inflammation at infection site
Swelling, redness, and warmth (local infection)
Cough and respiratory symptoms (respiratory infections)
Nausea, vomiting, and diarrhea (gastrointestinal infections)
Headache and body aches
Skin rash or lesions
Enlarged lymph nodes
Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of Epstein-Barr virus / infectious mononucleosis (systemic aspects) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

Systemic complications include airway obstruction, splenic rupture, hemolytic anemia, thrombocytopenia, myocarditis, and encephalitis.

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using atypical lymphocytosis, Monospot test, and EBV-specific antibodies. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification

- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment typically involves supportive. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most people recover well for acute IM. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 285

Erectile Dysfunction

Overview

Erectile dysfunction is the persistent inability to achieve or maintain an erection sufficient for satisfactory sexual performance, affecting approximately 50% of men between ages 40 and 70 to varying degrees.

Causes

This condition happens because of impaired vascular, neurological, or endocrine function.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience difficulty achieving or maintaining erections. Evaluation includes sexual history, hormonal testing (testosterone, prolactin, TSH), and cardiovascular risk assessment. The first-choice treatment typically involves PDE5 inhibitors (sildenafil, tadalafil, vardenafil, avanafil), with second-line options including vacuum erection devices and intracavernosal injection therapy, and third-line being penile prosthesis implantation.

Emergency warning signs

- Severe or worsening symptoms of erectile dysfunction require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with appropriate management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 286

Erythema Multiforme

Overview

Erythema multiforme (EM) is an acute, usually self-limited, hypersensitivity reaction of the skin and mucous membranes, most commonly triggered by infections, particularly herpes simplex virus. It is characterized by target lesions with a distinctive three-zone appearance. EM is classified as EM minor (typical target lesions, minimal mucosal involvement) and EM major (extensive mucosal involvement).

Classification

Cause Hypersensitivity / Immune

Organ System Skin

Pathophysiology Immune-mediated reaction with cytotoxic T-cell response directed against keratinocytes expressing viral antigens

Duration Acute (1–6 weeks)

Onset Sudden

Mode of Transmission Non-communicable

Clinical Course Acute, self-limited

Severity Mild to Moderate

Extent of Disease Widespread

Age Group Young adults (20–40 years)

Epidemiology Incidence 1–5 per million per year

ICD-11 Code EB01.0

Causes

Herpes simplex virus (HSV-1 and HSV-2) infection is the most common trigger, accounting for 50–70% of cases, particularly recurrent EM. Other triggers include *Mycoplasma pneumoniae* infection, other viral infections (EBV, VZV, CMV, hepatitis viruses), medications (particularly NSAIDs, sulfonamides, anticonvulsants, antibiotics), and rarely vaccines. The reaction involves HSV DNA fragments in keratinocytes triggering a T-cell-mediated immune response.

Risk Factors

- Recurrent herpes simplex virus infection
- Recent viral or bacterial infection (especially *Mycoplasma pneumoniae*)
- HLA subtype DQ3 (genetic susceptibility)
- Medication use
- Immunosuppression

Signs and Symptoms

Early symptoms

- Early manifestations of erythema multiforme may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
 - Sudden onset of red papules that evolve into target lesions with three zones
 - Classic target lesions have a central blister or crust (dark), middle pale edematous ring, and outer red rim
 - Lesions are acraly distributed: hands, feet, extensor surfaces of arms and legs
 - Mucosal involvement in EM major: oral, ocular, genital erosions
 - Lesions may be pruritic or burning
 - Systemic symptoms: low-grade fever, malaise, headache
 - Pain or discomfort in the affected area
 - Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of erythema multiforme may occur

Emergency warning signs

- Severe or worsening symptoms of erythema multiforme require immediate medical evaluation

Progression and Stages

Lesions develop rapidly over 1–3 days and evolve from macules to papules to target lesions. New lesions may continue to appear for 3–5 days. The eruption typically lasts 1–6 weeks and resolves spontaneously without scarring. Recurrent EM, usually triggered by HSV, tends to follow a milder, shorter course with each episode.

Complications

- Secondary bacterial infection of eroded skin
- Post-inflammatory hyperpigmentation or hypopigmentation
- Ocular complications (conjunctivitis, keratitis, symblepharon) in EM major
- Esophageal or genital strictures from mucosal involvement
- Recurrent episodes (HSV-associated)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on characteristic target lesions and acral distribution
- Skin biopsy of early lesion shows interface dermatitis with necrotic keratinocytes
- Direct immunofluorescence to differentiate from autoimmune blistering diseases
- HSV PCR or serology to identify viral trigger
- Mycoplasma serology in children

Prevention

- Suppressive antiviral therapy (acyclovir, valacyclovir) for HSV-associated recurrent EM
- Avoid known medication triggers
- Prompt treatment of HSV outbreaks
- Go for regular check-ups so any problems are caught early

Treatment Options

- Supportive care and symptomatic treatment for mild EM minor
- Topical corticosteroids for pruritus and inflammation
- Oral antihistamines for pruritus
- Systemic corticosteroids for severe or extensive disease (controversial)
- Antiviral therapy (acyclovir) if active HSV infection present
- Oral antibiotics if Mycoplasma is the trigger
- Wound care for eroded skin and mucosal lesions
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help

- Keep learning about your condition and how to manage it

Outlook

EM minor is a self-limited condition with an excellent prognosis, typically resolving within 2–6 weeks without scarring. EM major may require hospitalization for supportive care but also has a good prognosis in most cases. Recurrent HSV-associated EM can be effectively managed with suppressive antiviral therapy. Mortality is rare and usually related to underlying disease or complications.

Chapter 287

Esophageal Atresia / Tracheoesophageal Fistula

Overview

Esophageal atresia (EA) with or without tracheoesophageal fistula (TEF) is a congenital anomaly with incidence of 1 per 3500 live births. The most common type (86%) is EA with distal TEF (Gross Type C).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical presentation includes polyhydramnios (prenatal), excessive drooling, coughing, choking, and cyanosis with feeding. Failure to pass a nasogastric tube beyond 8–10 cm from the gums is diagnostic. Radiography shows a coiled nasogastric tube in the proximal esophageal pouch and gas in the stomach (if distal TEF is present). Associated anomalies include the VACTERL association.

Emergency warning signs

- Severe or worsening symptoms of esophageal atresia / tracheoesophageal fistula require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves surgical: primary repair with division of the fistula and end-to-end esophageal anastomosis. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well for isolated EA/TEF. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 288

Esophageal Varices

Overview

Esophageal varices are dilated submucosal veins in the lower third of the esophagus resulting from portal high blood pressure, most commonly caused by liver cirrhosis.

Causes

This condition results from disruption of normal esophageal varices function.

- This condition happens because of collateral venous circulation routing portal blood flow from the left gastric (coronary) vein into the systemic azygos vein due to elevated intrahepatic vascular resistance. Unruptured varices are asymptomatic
- Rupture causes catastrophic upper digestive tract bleeding presenting with painless hematemesis, melena, hypovolemic shock, and altered consciousness.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of esophageal varices require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using upper digestive tract endoscopy showing tortuous blue submucosal vessels. Treatment for acute variceal bleeding requires hemodynamic resuscitation, IV octreotide/somatostatin (to reduce splanchnic blood flow), prophylactic IV ceftriaxone, and urgent using a thin camera tube band ligation (EBL)
- Transjugular intrahepatic portosystemic shunt (TIPS) is indicated for refractory bleeding. Mortality per bleeding episode is 15–20%.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Primary prophylaxis includes non-selective beta-blockers (nadolol, propranolol) or elective using a thin camera tube band ligation.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 289

Essential Thrombocythemia

Overview

- Essential thrombocythemia is a myeloproliferative tumor characterized by sustained thrombocytosis (platelet count $>450,000/\mu\text{L}$) with increased megakaryocyte growth, in the absence of other myeloproliferative tumors. The condition is driven by JAK2 V617F (50%), CALR (25–30%), or MPL (3–5%) mutations
- Approximately 10% are triple-negative.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features may include microvascular symptoms (erythromelalgia, headache, visual disturbances), blood clot formation (arterial or venous), or bleeding (with very high platelet counts $>1,000,000/\mu\text{L}$ due to acquired von Willebrand syndrome), though many patients are asymptomatic.

Emergency warning signs

- Severe or worsening symptoms of essential thrombocythemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on sustained thrombocytosis with the characteristic mutation profile. Treatment for low-risk ET involves observation with low-dose aspirin
- High-risk ET (age >60 or history of blood clot formation) requires cytoreduction with hydroxyurea or anagrelide.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 290

Essential Tremor

Overview

Essential tremor is one of the most common movement disorders, affecting 1% of the general population and 5% of individuals over 65 years. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of abnormal oscillatory activity in the cerebello-thalamo-cortical circuit, with genetic heterogeneity (autosomal dominant inheritance in familial cases, ETM1/ETM2 loci). Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bilateral, symmetric postural and kinetic tremor of the upper limbs (6–12 Hz), which may also involve the head (titubation), voice, chin, or trunk.

Tremor is typically absent at rest, worsens with goal-directed movement, and is exacerbated by stress, fatigue, and caffeine. Alcohol consumption temporarily reduces tremor amplitude in 50–70% of patients. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of essential tremor require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, based on the Movement Disorder Society consensus criteria and exclusion of other causes (Parkinson disease, dystonic tremor, enhanced physiologic tremor, drug-induced tremor). Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include The first-choice treatment with propranolol or primidone, alternative agents (topiramate, gabapentin, benzodiazepines), and for medication-refractory cases, deep brain stimulation of the ventral intermediate nucleus (VIM) of the thalamus or focused ultrasound thalamotomy. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is non-cancerous but slowly progressive
- Disability from tremor may be significant.

Chapter 291

Esthesioneuroblastoma

Overview

Esthesioneuroblastoma (olfactory neuroblastoma) is a rare, cancerous neuroectodermal tumor originating from the specialized olfactory neuroepithelium lining the cribriform plate, superior nasal cavity, and upper nasal septum. The tumor exhibits a bimodal age distribution (10–20 and 50–60 years) and is marked by slow, locally invasive growth with erosion into the anterior cranial fossa, orbits, and paranasal sinuses. Histopathology shows small round blue cells forming Homer-Wright pseudorosettes with positive neuroendocrine markers (synaptophysin, chromogranin, S100 at tumor margins). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity

- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Staging uses the Kadish system. Management requires multimodality therapy comprising craniofacial or using a thin camera tube endonasal removal combined with adjuvant radiotherapy and chemotherapy. 5-year survival ranges from 60 to 80%. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of esthesioneuroblastoma require immediate medical evaluation

Progression and Stages

Symptoms often appear as unilateral nasal obstruction, recurrent epistaxis, loss of smell, and advanced orbital symptoms (proptosis, diplopia). Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original

location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies

- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay

physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 292

Ewing Sarcoma

Overview

This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

- Ewing sarcoma is the second most common primary bone malignancy in children and adolescents, arising from primitive neuroectodermal cells. The classic translocation t(11
- 22)(q24
- Q12) results in the EWS-FLI1 fusion gene. It typically affects the diaphysis of long bones (femur, tibia, fibula) and the pelvis.

Causes

This condition happens because of the EWS-FLI1 fusion gene driving oncogenesis. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes

- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience bone pain, swelling, and a mass, with systemic symptoms (fever, weight loss) that may mimic infection. X-rays show a permeative lytic lesion with "onion skin" periosteal reaction. MRI and CT chest are essential for staging. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of ewing sarcoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with

possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Your doctor confirms the diagnosis with biopsy with molecular testing for the translocation. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging

- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment options include neoadjuvant chemotherapy (vincristine, doxorubicin, cyclophosphamide alternating with ifosfamide/etoposide), local control with surgery and/or radiation therapy, and adjuvant chemotherapy. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease

- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

- The outlook is generally positive for localized disease
- The 5-year survival rate is approximately 70%.

Chapter 293

Ewing Sarcoma in Children

Overview

This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

- Ewing sarcoma is the second most common cancerous bone tumor in children and adolescents (see Chapter 26, Diseases of the Musculoskeletal System for comprehensive coverage). Peak incidence is 10–20 years. It arises in the diaphysis of long bones and flat bones (pelvis, ribs). The hallmark is a t(11
- 22)(q24
- Q12) translocation producing the EWSR1-FLI1 fusion gene.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)

- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of ewing sarcoma in children require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin

from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment options include chemotherapy, surgical removal, and/or radiation therapy. Survival for localized disease is 70–75%. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 294

Facioscapulohumeral Muscular Dystrophy

Overview

Facioscapulohumeral muscular dystrophy (FSHD) is an autosomal dominant myopathy caused by contraction of D4Z4 repeats on chromosome 4q35, leading to inappropriate expression of the DUX4 gene. It has a prevalence of approximately 1 in 8000–20,000. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Early symptoms

- You may experience initial involvement of facial muscles (inability to fully close the eyes, difficulty whistling, transverse smile) and scapular stabilizers ("scapular winging"), followed by humeral and pelvic girdle muscles

Common symptoms

- Asymmetric involvement is common. Hearing loss and retinal telangiectasias (Coats disease) may occur.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of facioscapulohumeral muscular dystrophy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on genetic testing.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include physical therapy and orthopedic interventions (scapular bracing/fusion) and respiratory monitoring
- There is no curative treatment.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with slowly progressive weakness but usually normal life expectancy.

Chapter 295

Failure to Thrive

Overview

Failure to thrive (FTT) in older adults is a geriatric syndrome of unintentional weight loss (>5% in 6–12 months), decreased appetite, poor nutrition, and functional decline, often accompanied by social withdrawal and depression. It is multifactorial, resulting from medical (malignancy, dementia, depression, infection, hyperthyroidism, malabsorption, difficulty swallowing), social (isolation, poverty, caregiver loss), and psychological (bereavement, delirium) causes.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience weight loss, poor appetite, functional decline, and social withdrawal.

Emergency warning signs

- Severe or worsening symptoms of failure to thrive require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with identification of underlying causes. Treatment targets underlying causes, nutritional supplementation, appetite stimulants (megestrol acetate, mirtazapine), and social support. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 296

Familial Adenomatous Polyposis

Overview

Familial adenomatous polyposis (FAP) is an autosomal dominant condition caused by germline mutations in the APC gene on chromosome 5q21, characterized by the development of hundreds to thousands of adenomatous polyps throughout the colon and rectum, typically beginning in adolescence. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades.

Causes

This condition happens because of APC gene mutation leading to uncontrolled cell growth. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the development of polyps in adolescence
- Without treatment, CRC develops in virtually 100% of patients by age 40.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of familial adenomatous polyposis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on colonoscopy showing innumerable polyps and genetic testing. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on prophylactic proctocolectomy with ileal pouch-anal anastomosis (IPAA) recommended in the late teens to early twenties. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with prophylactic surgery
- Attenuated FAP (AFAP) has fewer polyps (10–100) and later-onset CRC. Gardner syndrome is a variant with extraintestinal manifestations (osteomas, desmoid tumors, congenital enlargement of retinal pigment epithelium).

Chapter 297

Familial Combined Hyperlipidemia

Overview

Familial combined hyperlipidemia (FCHL) is the most common genetic dyslipidemia, with a prevalence of 1–2% in the general population and up to 10–20% in patients with premature coronary artery disease. It is a complex oligogenic disorder characterized by overproduction of apolipoprotein B-100 and very-low-density lipoprotein (VLDL) by the liver. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience premature coronary artery disease, obesity, insulin resistance, and the absence of tendon xanthomas (distinguishing it from FH). The lipid phenotype is variable: affected individuals may have elevated total cholesterol and/or elevated triglycerides. Diagnosis is made in individuals with elevated apolipoprotein B (greater than 120 mg/dL), elevated LDL-C and/or triglycerides, and a family history of premature cardiovascular disease.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of familial combined hyperlipidemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include lifestyle modification, statins as first-line therapy, with the addition of fibrates for hypertriglyceridemia.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling
Pharmacotherapy to correct metabolic abnormalities
Hormone replacement therapy when indicated
Regular monitoring of metabolic parameters
Weight management programs
Exercise prescription
Management of associated conditions
Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment.

Chapter 298

Familial Hypercholesterolemia

Overview

Familial hypercholesterolemia (FH) is an autosomal codominant disorder of LDL metabolism caused by loss-of-function mutations in the LDLR gene (LDL receptor, 90% of cases), APOB (apolipoprotein B-100), or gain-of-function mutations in PCSK9. It is the most common monogenic lipid disorder, with a prevalence of 1 in 250 for heterozygous FH (HeFH) and 1 in 300,000 for homozygous FH (HoFH).

Causes

This condition happens because of impaired liver clearance of LDL particles.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience tendon xanthomas (Achilles, extensor tendons of the hands), tuberous xanthomas, xanthelasmas, corneal arcus before age 45, and premature atherosclerotic cardiovascular disease
- Homozygous FH presents in childhood with LDL-C greater than 500 mg/dL, widespread xanthomas, and coronary atherosclerosis before age 20.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of familial hypercholesterolemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical criteria (Dutch Lipid Clinic Network or Simon Broome criteria) supported by genetic testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include high-dose statins with ezetimibe as first-line for HeFH

- PCSK9 inhibitors for inadequate response
- For HoFH, all available agents and LDL apheresis are needed.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with early diagnosis and treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 299

Fanconi Syndrome

Overview

Fanconi syndrome is generalized dysfunction of the proximal kidney tubule, leading to impaired reabsorption of glucose, amino acids, phosphate, bicarbonate, uric acid, and other solutes. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Causes include hereditary disorders (cystinosis, Wilson's disease, galactosemia), drugs (ifosfamide, outdated tetracycline, aminoglycosides), multiple myeloma, and heavy metal poisoning. This condition happens because of impaired proximal tubular transport mechanisms. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience metabolic acidosis (proximal RTA), hypophosphatemic rickets/osteomalacia, polyuria, and growth retardation in children.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of fanconi syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on laboratory findings of generalized proximal tubular dysfunction. Management addresses the underlying cause and provides supplementation of lost solutes (phosphate, vitamin D, bicarbonate).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause.

Chapter 300

Fat Necrosis of the Breast

Overview

Fat tissue death of the breast is a non-cancerous, non-enzymatic inflammatory reaction resulting from focal adipocyte destruction following blunt trauma, breast surgery, core needle biopsy, or radiation therapy. Disrupted adipocytes release neutral triglycerides that undergo enzymatic saponification by tissue lipases, precipitating a foreign-body granulomatous response dominated by lipid-laden macrophages (foam cells), multinucleated giant cells, and peripheral fibrous tissue scarring.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a solitary, firm, irregular, tethered breast mass occasionally accompanied by skin dimpling, nipple retraction, or bruising, closely mimicking invasive ductal carcinoma.

Emergency warning signs

- Severe or worsening symptoms of fat necrosis of the breast require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with mammography (showing lipid cysts with eggshell calcifications or spiculated masses) and core needle biopsy when cancerous features cannot be excluded. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves conservative with patient reassurance following definitive diagnosis
- Symptomatic oil cysts may be aspirated.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 301

Febrile Seizures

Overview

Febrile seizures are the most common seizures in childhood, occurring in 2–5% of children between 6 months and 5 years of age. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition results from disruption of normal febrile seizures function.

- This condition happens because of fever (temperature greater than 38 degrees Celsius) associated with an extracranial infection, in the absence of central nervous system infection or defined electrolyte imbalance. Simple febrile seizures are generalized, last less than 15 minutes, and do not recur within 24 hours
- Complex febrile seizures are focal, prolonged (greater than 15 minutes), or recurrent.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience generalized or focal seizure activity triggered by fever. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of febrile seizures require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, after excluding central nervous system infection. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment typically involves supportive
- Antipyretics prevent recurrence only partially, and prophylactic anticonvulsants are generally not recommended.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well, with most children outgrowing febrile seizures by age 6. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 302

Fecal Incontinence

Overview

Fecal incontinence (FI)—involuntary loss of solid or liquid stool—affects 10–20% of community-dwelling older adults and up to 50% of nursing home residents.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of fecal incontinence require immediate medical evaluation

Progression and Stages

Causes include impaired anal sphincter function (obstetric trauma, surgical injury, age-related deterioration), reduced rectal sensation (diabetes, spinal cord injury, fecal impaction with overflow), diarrhea, and thinking and memory/mobility impairment. Mark's (Vaizey) score to quantify severity. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors use the St. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Addressing constipation and fecal impaction in institutionalized patients is critical. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment options include dietary modification (fiber supplementation for formed stool, avoidance of dietary triggers), scheduled toilet routines, pelvic floor rehabilitation, an-

tidiarrheal medications, anal plugs, and surgical options for severe refractory cases.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 303

Fibroadenoma

Overview

Fibroadenomas are the most common non-cancerous breast tumors in young women (ages 15–35). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades.

Causes

This condition happens because of estrogen-dependent non-cancerous tumors composed of fibrous and glandular tissue. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features typically include a solitary, firm, mobile, well-circumscribed, rubbery mass—often described as a “breast mouse” due to its mobility—which can grow during pregnancy or hormonal stimulation.

Emergency warning signs

- Severe or worsening symptoms of fibroadenoma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using ultrasound (well-circumscribed, hypoechoic, oval mass with horizontal orientation) and core needle biopsy to confirm. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include surgical removal for giant fibroadenomas or those causing symptoms

- Small, asymptomatic fibroadenomas may be observed.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 304

Fibrocystic Changes

Overview

Fibrocystic changes are the most common non-cancerous breast condition, affecting up to 60% of women at some point. Histologically, it encompasses a spectrum of findings: cysts (microcystic and macrocystic), scarring, adenosis, and epithelial abnormal increase in cells.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience breast pain and nodularity.

Emergency warning signs

- Severe or worsening symptoms of fibrocystic changes require immediate medical evaluation

Progression and Stages

The condition is marked by breast pain (mastalgia), tenderness, nodularity, and thickening, often fluctuating with the menstrual cycle, and is hormone-dependent, typically worsening in the luteal phase. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced

stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical breast examination, mammography (for women over 40), and ultrasound (to evaluate dominant cysts). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include reassurance, well-fitting support bras, NSAIDs, and for severe symptoms, oral contraceptives or tamoxifen. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well, with no cancerous potential. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 305

Fibromyalgia

Overview

Fibromyalgia is a chronic pain condition characterized by widespread musculoskeletal pain, fatigue, sleep disturbance, thinking and memory dysfunction ("fibro fog"), and mood disorders, affecting 2–4% of the population, predominantly women.

Causes

This condition happens because of central sensitization (amplified pain processing in the CNS), altered neurotransmitter levels (substance P, serotonin), and abnormal functional connectivity in brain pain networks.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience chronic widespread pain, tender points (18 defined anatomic sites historically), fatigue, and sleep disturbance.

Emergency warning signs

- Severe or worsening symptoms of fibromyalgia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Doctors diagnose this based on the 2010 ACR criteria using a widespread pain index and symptom severity scale
- It is a clinical To diagnose exclusion with no specific laboratory tests.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves multimodal: exercise (most evidence-based intervention), thinking and memory-behavioral therapy, pharmacotherapy (pregabalin, duloxetine, milnacipran, amitriptyline, tramadol), and patient education. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures

Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Symptoms are chronic but functional status can improve with multimodal management.

Chapter 306

Fibromyalgia

Overview

See Chapter ??, Section 305. From a pain medicine perspective, fibromyalgia is the prototypical nociplastic pain condition involving central sensitization, dysfunctional descending pain modulation, and aberrant connectivity within the default mode network and salience network. Management emphasizes a multimodal approach: pharmacotherapy (duloxetine 60–120 mg/day, milnacipran 100–200 mg/day, pregabalin 150–600 mg/day—the three FDA-approved agents), graded aerobic exercise, thinking and memory behavioral therapy, sleep hygiene, and patient education. Opioids are ineffective and should be avoided.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of fibromyalgia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 307

Fibrous Dysplasia

Overview

- Fibrous dysplasia is a non-cancerous bone disorder where normal bone is replaced by fibrous tissue and immature woven bone, caused by postzygotic activating mutations in the GNAS gene. It may be monostotic (single bone, 70–80%) or polyostotic (multiple bones)
- The ribs, femur, tibia, and skull are commonly affected.

Causes

This condition happens because of GNAS mutation leading to abnormal bone formation. McCune-Albright syndrome is a variant combining polyostotic fibrous dysplasia with café-au-lait spots and endocrine hyperfunction (precocious puberty).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bone pain, deformity, or pathologic fracture.

Emergency warning signs

- Severe or worsening symptoms of fibrous dysplasia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using X-ray showing ground-glass appearance and thinning of the cortex. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves symptomatic: observation, bisphosphonates for pain, and surgical correction of deformities or fractures. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is good
- Cancerous transformation is rare.

Chapter 308

Fifth Disease (Erythema Infectiosum)

Overview

Fifth disease, also known as erythema infectiosum, is a common childhood viral exanthem caused by parvovirus B19. It is characterized by a distinctive slapped cheek rash followed by a lacy reticular rash on the trunk and extremities. The infection is typically mild and self-limited in children but can cause significant complications in adults, immunocompromised individuals, and during pregnancy.

Classification

Cause Infectious (Viral)

Organ System Pediatric / Infectious

Pathophysiology Parvovirus B19 infection causing lytic infection of erythroid progenitor cells

Duration Acute (1–3 weeks)

Onset Sudden

Mode of Transmission Respiratory droplets; bloodborne; vertical (mother to fetus)

Clinical Course Acute, self-limited

Severity Mild

Extent of Disease Systemic

Age Group Children (peak 4–10 years)

Epidemiology Very common; 40–60% of adults seropositive; epidemic cycles every 3–6 years

ICD-11 Code 1D60.0

Causes

Parvovirus B19, a small single-stranded DNA virus, infects erythroid progenitor cells via the P blood group antigen (globoside) receptor, leading to temporary arrest of red blood cell production. The rash and joint symptoms are immune-mediated, appearing after the development of IgM antibodies. Transmission occurs through respiratory secretions, blood products, and vertically from mother to fetus.

Risk Factors

- Age 4–10 years (peak incidence)
- Contact with infected children (parents, teachers, healthcare workers)
- Outdoor activities and school attendance
- Immunosuppression (chronic hemolytic anemia, HIV, transplant)
- Pregnancy (risk of fetal hydrops)
- Lack of prior exposure (seronegative individuals)

Signs and Symptoms

Early symptoms

- Early manifestations of fifth disease (*erythema infectiosum*) may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Prodrome: low-grade fever, headache, malaise, myalgia (mild or absent in children)
- Slapped cheek rash: bright red, confluent erythema on both cheeks, sparing the nose and perioral area
- Reticular rash: lacy, erythematous maculopapular rash on trunk and extremities
- Rash may recur with heat, exercise, or sun exposure for several weeks
- Arthralgia and arthritis (more common in adults, especially women)
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of fifth disease (*erythema infectiosum*) may occur

Emergency warning signs

- Severe or worsening symptoms of fifth disease (*erythema infectiosum*) require immediate medical evaluation

Progression and Stages

After an incubation period of 4–14 days, the prodromal phase (if present) lasts 2–3 days. The slapped cheek rash appears first, followed 1–4 days later by the reticular rash that

spreads to the trunk and limbs. The rash typically lasts 1–3 weeks but may recur with triggers. Arthralgia can persist for weeks to months, particularly in adults.

Complications

- Aplastic crisis in patients with hemolytic anemias (sickle cell disease, hereditary spherocytosis)
- Chronic pure red cell aplasia in immunocompromised patients
- Hydrops fetalis and fetal loss in pregnancy (infection in first 20 weeks)
- Chronic arthropathy in adults
- Myocarditis and hepatitis (rare)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on characteristic slapped cheek rash
- Parvovirus B19 IgM serology confirms recent infection
- PCR for parvovirus B19 DNA in immunocompromised or fetal cases
- Complete blood count showing reticulocytopenia during aplastic crisis
- Prenatal ultrasound for fetal hydrops evaluation

Prevention

- Good hand hygiene and respiratory etiquette
- Exclusion from school during prodromal phase (before rash appears; no longer infectious once rash develops)
- Pregnant healthcare workers should avoid contact with known cases
- No vaccine available currently
- Go for regular check-ups so any problems are caught early

Treatment Options

- Supportive care for uncomplicated cases
- Antipyretics for fever and joint pain (acetaminophen, ibuprofen, NSAIDs)
- Blood transfusion for aplastic crisis
- Intravenous immunoglobulin (IVIG) for chronic infection in immunocompromised patients
- Intrauterine transfusion for severe fetal anemia and hydrops
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

The prognosis for fifth disease in healthy children is excellent, with full recovery without sequelae. Adults may experience prolonged arthralgia but generally recover fully. The most serious complications occur in patients with underlying hemolytic anemias (aplastic crisis), immunocompromised states, and during pregnancy. With appropriate management, outcomes are generally good even in these higher-risk groups.

Chapter 309

Filariasis (Lymphatic Filariasis)

Overview

Lymphatic filariasis (elephantiasis) is a mosquito-borne nematode infection caused by *Wuchereria bancrofti* (90% of cases), *Brugia malayi*, and *B. timori*, with 120 million infected, primarily in tropical and subtropical regions, and is the second leading cause of permanent disability worldwide.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience asymptomatic microfilaremia (most infected individuals), acute filarial fevers (adenolymphangitis with painful lymphadenitis, fever, chills), and chronic manifestations (lymphedema of the leg, scrotal hydrocele, elephantiasis).

Emergency warning signs

- Severe or worsening symptoms of filariasis (lymphatic filariasis) require immediate medical evaluation

Progression and Stages

This condition happens because of third-stage filariform larvae (L3) being inoculated by mosquito bite, migrating to the lymphatics, and maturing into adult worms in lymphatic vessels and lymph nodes, with the host immune response to dying adult worms causing

granulomatous inflammation and lymphatic damage. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using microscopic detection of microfilariae on thick blood smear or antigen detection. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include diethylcarbamazine (DEC) and albendazole. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- The outlook is good with early treatment
- Chronic lymphedema requires long-term care.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 310

Floaters and Vitreous Detachment

Overview

- Floaters are small shadows, specks, threads, or cobweb-like shapes that drift across the field of vision as the eye moves
- Posterior vitreous detachment (PVD) is the age-related separation of the vitreous gel from the retina, and is the most common cause of sudden onset of floaters
- PVD occurs in most people by age 80 and is usually a normal part of aging rather than a disease

Causes

Floaters and vitreous detachment happen because of age-related liquefaction (syneresis) and shrinkage of the vitreous gel. As the gel shrinks, it pulls away from the retina; the strands and debris cast shadows on the retina that are perceived as floaters. Secondary causes include trauma, high myopia, intraocular inflammation (uveitis), bleeding into the vitreous (vitreous hemorrhage), and previous eye surgery such as cataract extraction.

Risk Factors

- Age over 50 years
- Nearsightedness (high myopia)
- Eye trauma or prior eye surgery
- Intraocular inflammation (uveitis)
- Diabetes with retinal disease
- Family history of early vitreous detachment

Signs and Symptoms

Common symptoms

- Seeing spots, specks, threads, or cobweb shapes that drift when the eye moves
- Flashes of light (photopsia), often in the peripheral vision, especially in dim lighting
- Sudden shower of many new floaters
- A shadow or "curtain" across part of the vision (a retinal emergency)

Emergency warning signs

- Sudden appearance of many new floaters
- Flashes of light that persist
- A dark curtain or veil covering part of the visual field
- Sudden loss of vision — these may indicate a retinal tear or detachment and require immediate eye examination

Progression and Stages

The vitreous detachment is a single event that usually occurs over weeks to months. Floaters are most noticeable when they first appear and typically become less bothersome over time as the debris settles and the brain adapts. The critical danger window is in the weeks after PVD begins, when vitreous traction can cause a retinal tear.

- Vitreous syneresis (gel liquefaction) precedes detachment
- Complete PVD separates the gel from the retina
- Incomplete or anomalous PVD may leave traction on the retina, leading to tears, hemorrhage, or detachment

Complications

- Retinal tear (tractional)
- Retinal detachment
- Vitreous hemorrhage
- Epiretinal membrane formation
- Persistent visually disturbing floaters affecting daily function

Diagnosis

- Dilated fundus examination with scleral depression to detect tears
- Slit-lamp biomicroscopy to inspect the vitreous
- B-scan ultrasound when the vitreous is too hazy to see the retina
- Optical coherence tomography (OCT) if macular pathology is suspected
- Urgent referral when new floaters, flashes, or visual field loss are present

Prevention

- Posterior vitreous detachment cannot be prevented; it is a normal aging change
- Prompt examination for sudden onset of floaters or flashes to allow early treatment of any tear
- Protect the eyes from trauma
- Control underlying conditions such as diabetes

Treatment Options

- No treatment is needed for uncomplicated PVD and benign floaters; symptoms usually improve with time
- Laser retinopexy or cryotherapy to seal retinal tears
- Vitrectomy surgery for dense, persistent, or visually disabling floaters
- Surgery (scleral buckle, vitrectomy) if retinal detachment develops
- Regular follow-up monitoring during the acute phase

Living with It

- Most floaters become less noticeable as the brain learns to ignore them
- Follow up as directed, especially in the weeks after symptoms begin
- Seek urgent care if new flashes, more floaters, or a shadow appear
- Stay informed about your condition

Outlook

The outlook is excellent for uncomplicated posterior vitreous detachment; most people experience harmless floaters that fade in prominence. Prompt treatment of retinal tears prevents most detachments. When retinal detachment occurs, timely surgery usually restores vision, though outcomes depend on how quickly treatment is delivered.

Chapter 311

Fluoride Excess — Fluorosis

Overview

Fluorosis is a mineral excess disorder caused by chronic excessive intake of fluoride, typically from drinking water with high fluoride content (greater than 1.5 ppm), but also from dental products, fluoride supplements, and industrial exposure. It is endemic in regions of India, China, Africa, and the Middle East.

Causes

This condition happens because of fluoride incorporation into hydroxyapatite crystals of bones and teeth.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of fluoride excess — fluorosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- You may experience dental fluorosis (chalky white or yellow-brown mottling, pitting, and staining of teeth, occurring when excess fluoride is ingested during tooth development) and skeletal fluorosis (osteosclerosis, bone and joint pain, ligamentous calcium buildup, periosteal bone formation, and spinal rigidity mimicking ankylosing spondylitis)
- Advanced stages lead to crippling deformities, kyphosis, and nervous system complications from spinal canal stenosis).

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on history of high fluoride exposure, radiographic findings (osteosclerosis, periosteal bone apposition, interosseous membrane calcium buildup), and elevated urinary fluoride. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment focuses on prevention: defluoridation of drinking water, use of alternative water sources, and avoidance of fluoride-containing products in endemic areas. The outlook is good with prevention
- No specific treatment exists for established disease.
- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 312

Focal Segmental Glomerulosclerosis

Overview

- Focal segmental glomerulosclerosis (FSGS) is a pattern of glomerular injury characterized by segmental sclerosis in some (focal) glomeruli. It is a common cause of nephrotic syndrome in adults. The condition may be primary (idiopathic) or secondary (to obesity, HIV infection, sickle cell disease, drug abuse, or adaptive hyperfiltration)
- Podocyte injury is the central pathological event.

Causes

Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience proteinuria (often nephrotic-range), hematuria, high blood pressure, and progressive kidney insufficiency.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of focal segmental glomerulosclerosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using kidney biopsy, which must include sufficient glomeruli to demonstrate the focal pattern. Treatment for primary FSGS includes corticosteroids (6 months), calcineurin inhibitors for steroid-resistant cases, and rituximab
- Secondary FSGS requires treatment of the underlying cause.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics)

Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- APOL1 gene variants are associated with increased risk, particularly in individuals of African ancestry.

Chapter 313

Folate Deficiency Anemia

Overview

Folate deficiency anemia is a megaloblastic anemia that is clinically indistinguishable from B12 deficiency anemia but without the neurological manifestations. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of folate (vitamin B9) deficiency, with causes including poor dietary intake (alcoholism, inadequate fruit/vegetable consumption), malabsorption (celiac disease, tropical sprue), increased demand (pregnancy, hemolytic anemias), and medications (methotrexate, trimethoprim, phenytoin). Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fatigue, pallor, and glossitis.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of folate deficiency anemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using macrocytic anemia, low serum folate, and elevated homocysteine (with normal methylmalonic acid, distinguishing it from B12 deficiency).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on oral folic acid supplementation (1 mg daily) and addressing the underlying cause. Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription

Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment.

Chapter 314

Fractures

Overview

A fracture is a break in the continuity of bone. Common fracture types include distal radius (Colles' fracture), hip (intertrochanteric or subcapital femoral neck), vertebral compression, scaphoid, and clavicle fractures. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of mechanical force exceeding bone strength.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pain, deformity, swelling, and loss of function.

Emergency warning signs

- Severe or worsening symptoms of fractures require immediate medical evaluation

Progression and Stages

Fractures are classified as open (compound, with skin disruption) or closed (simple), and by pattern (transverse, oblique, spiral, comminuted, greenstick, impacted, avulsion). The condition may progress through different stages of severity over time. Early stages often

involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- In most cases, the outlook is good
- Hip fractures in the elderly are orthopedic emergencies requiring surgical fixation within 24–48 hours to reduce mortality and complications.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using X-rays (at least two views)
- CT for complex fractures, MRI for occult fractures and ligamentous injury.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment aims for reduction (realigning fracture fragments), fixation (maintaining alignment during healing), and rehabilitation

- Closed reduction with casting is appropriate for stable, minimally displaced fractures
- Open reduction with internal fixation (ORIF) is indicated for displaced, unstable, or intra-articular fractures.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 315

Fragile X Syndrome

Overview

Fragile X syndrome is the most common inherited cause of intellectual disability and the most common monogenic cause of autism spectrum disorder. It is caused by a CGG trinucleotide repeat expansion (>200 repeats) in the FMR1 gene on the X chromosome, leading to hypermethylation and gene silencing. Incidence is 1 in 4000 males and 1 in 8000 females. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features in males include intellectual disability, long narrow face, prominent ears, large testes (macroorchidism, postpubertal), joint hypermobility, autistic behaviors, attention deficits, and anxiety. Females may have mild intellectual disability or learning difficulties. Premutation carriers (55–200 repeats) are at risk of FXTAS and FXPOI.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of fragile x syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using FMR1 DNA testing.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive and includes behavioral therapies, educational interventions, and pharmacotherapy for associated conditions. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling

for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 316

Frailty

Overview

- Frailty is a geriatric syndrome of increased vulnerability to stressors resulting from age-related decline in physiological reserve across multiple organ systems. It affects a significant proportion of older adults. Two major conceptual frameworks guide frailty assessment. The **Fried Frailty Phenotype** (2001) defines frailty by the presence of three or more of five criteria: unintentional weight loss (>10 lb in one year), self-reported exhaustion, weakness (low grip strength), slow gait speed, and low physical activity
- Pre-frailty is defined as one or two criteria. The **Frailty Index** (Rockwood and Mitnitski) accumulates deficits across multiple domains as a ratio of deficits present to total deficits assessed (typically 30–70 items).

Causes

This condition happens because of age-related decline in physiologic reserve, with sarcopenia as a core biological substrate.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience unintentional weight loss, exhaustion, weakness, slow gait speed, and low physical activity.

Emergency warning signs

- Severe or worsening symptoms of frailty require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- The outlook varies from person to person
- Frailty is a dynamic process that can improve or worsen over time.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors use the Fried Frailty Phenotype (three or more of five criteria) or the Frailty Index (>0.25 indicates frailty). Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include resistance and aerobic exercise, protein-calorie supplementation, vitamin D, reduction of polypharmacy, and comprehensive geriatric assessment (CGA). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 317

Friedreich's Ataxia

Overview

Friedreich's loss of coordination is the most common hereditary loss of coordination, caused by homozygous GAA trinucleotide repeat expansion in the FXN gene encoding frataxin, with a prevalence of 1 in 50,000. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of frataxin deficiency, leading to mitochondrial iron accumulation, oxidative stress, and impaired energy metabolism, primarily affecting the dorsal root ganglia, posterior columns, corticospinal tracts, and cerebellum. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive gait and limb loss of coordination, slurred speech, areflexia, loss of vibratory and proprioceptive sensation, extensor plantar responses, scoliosis, pes cavus, hypertrophic cardiomyopathy, and increased risk of diabetes mellitus. Onset is typically in childhood or adolescence (mean age 10–15 years). Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of friedreich's ataxia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical criteria and confirmed by FXN genetic testing. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures

- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on symptomatic treatment: physical therapy for balance and mobility, occupational therapy, speech therapy, and orthopedic Treatment for scoliosis. Cardiac monitoring is essential (echocardiography, ECG). Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is progressive
- Most patients lose ambulation 10–15 years after symptom onset, with mean survival to age 40–50, primarily due to cardiomyopathy.

Chapter 318

Frontotemporal Dementia

Overview

- Frontotemporal dementia encompasses a group of neurodegenerative disorders selectively affecting the frontal and temporal lobes
- Onset typically occurs between ages 40 and 65, making it a common cause of early-onset dementia.

Causes

This condition happens because of accumulation of tau, TDP-43, or FUS proteins. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience behavioral variant FTD with personality changes, disinhibition, apathy, and executive dysfunction
- Language variants include semantic dementia (progressive loss of word meaning) and non-fluent or agrammatic primary progressive aphasia.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of frontotemporal dementia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on MRI showing disproportionate frontal and temporal lobe shrinkage or wasting. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive, focusing on behavioral management, speech therapy, and caregiver support. Neurological treatment focuses on symptom manage-

ment, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is progressive, with no disease-modifying treatment available. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 319

Frontotemporal Dementia

Overview

Frontotemporal dementia (FTD) is a group of neurodegenerative disorders characterized by progressive dysfunction of the frontal and temporal lobes. Although typically presenting at a younger age (45–65 years), FTD can occur in older adults. The behavioral variant (bvFTD) presents with early and progressive changes in personality, social conduct, and executive function—disinhibition, apathy, loss of empathy, compulsive behaviors, and hyperorality. Language variants include non-fluent/agrammatic primary progressive aphasia (PPA) and semantic variant PPA. Approximately 30–50% of FTD is familial, with mutations in MAPT (tau), GRN (progranulin), and C9orf72. There is no disease-modifying therapy. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of frontotemporal dementia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Treatment typically involves symptomatic and supportive, focusing on behavior management (SSRIs for disinhibition), caregiver support, and addressing safety and legal concerns. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is progressive. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 320

Fuchs' Endothelial Dystrophy

Overview

- Fuchs' endothelial corneal dystrophy is a bilateral, progressive disorder of the corneal endothelium characterized by guttae, endothelial cell loss, and corneal swelling
- It is autosomal dominant and more common in women.

Causes

This condition happens because of progressive endothelial cell dysfunction and loss.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of fuchs' endothelial dystrophy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

- Early disease is asymptomatic
- As the disease progresses, You may experience morning blurring that improves through the day, glare, and reduced visual acuity
- Advanced disease causes persistent corneal swelling and bullous keratopathy.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using slit-lamp examination and specular microscopy showing reduced endothelial cell density. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with surgical intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 321

Fungal Sinusitis

Overview

Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

- Fungal sinusitis encompasses a spectrum of conditions ranging from non-invasive forms (fungal ball, allergic fungal sinusitis) to life-threatening invasive forms (acute invasive fungal rhinosinusitis). Non-invasive forms result from fungal colonization or allergic reaction
- Invasive forms occur in immunocompromised patients (diabetes with ketoacidosis, neutropenia, transplant recipients) and are caused by Mucorales or *Aspergillus* species.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)

- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Symptoms of acute invasive disease include rapidly progressive facial pain, nasal congestion, black necrotic eschar, and extension to the orbit and brain. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of fungal sinusitis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)

- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

To diagnose invasive disease requires tissue biopsy. Treatment for invasive disease involves aggressive surgical removal of dead tissue, systemic antifungal therapy (amphotericin B), and reversal of immunosuppression. outlook for invasive disease is poor with high mortality. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use

- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 322

Galactocele

Overview

A galactocele is a non-cancerous retention cyst filled with milk or milky fluid, occurring predominantly in lactating women during or immediately following weaning.

Causes

This condition happens because of obstruction of a lactiferous duct (secondary to inflammation, inspissated milk secretions, or external compression), leading to proximal ductal dilation and accumulation of fat-rich milk elements.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a painless, smooth, mobile, well-circumscribed subareolar mass that may fluctuate in size with breastfeeding. Ultrasound demonstrates a well-marginated oval lesion with acoustic enhancement and a characteristic fat-fluid level.

Emergency warning signs

- Severe or worsening symptoms of galactocele require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors confirm the diagnosis through clinical examination, sonography, and fine-needle removal by suction yielding milky fluid, which is both diagnostic and therapeutic. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on needle removal by suction for symptomatic relief
- Infected galactoceles require antibiotic coverage and removal of fluid.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 323

Galactosemia

Overview

Galactosemia is an autosomal recessive disorder of galactose metabolism caused most commonly by deficiency of galactose-1-phosphate uridylyltransferase (GALT), with an incidence of approximately 1 in 30,000–60,000 live births.

Causes

This condition happens because of accumulation of galactose-1-phosphate and galactitol when galactose from dietary lactose cannot be converted to glucose.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of galactosemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Clinical features present after the initiation of milk feeding with poor feeding, vomiting, diarrhea, failure to thrive, hypoglycemia, jaundice, hepatomegaly, ascites, and liver dysfunction
- *Escherichia coli* neonatal sepsis is a frequent and life-threatening complication
- Cataracts develop due to galactitol accumulation in the lens. outlook is guarded despite treatment
- Long-term complications including speech apraxia, thinking and memory deficits, loss of coordination, tremor, and premature ovarian insufficiency are common.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using newborn screening (elevated total galactose), red blood cell GALT enzyme activity (absent or severely reduced), and genetic testing of the GALT gene. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on immediate elimination of dietary lactose and galactose (soy-based or elemental formula) for life. Medications to manage symptoms and address underlying

mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 324

Gas Gangrene (Clostridial Myonecrosis)

Overview

Gas gangrene is a rapidly progressive, life-threatening infection of skeletal muscle caused by *Clostridium perfringens* (80% of cases) and other clostridial species, with low incidence but high mortality (20–60%), occurring post-traumatically or spontaneously (associated with colorectal carcinoma, neutropenic colitis).

Causes

This condition happens because of spores germinating in devitalized, hypoxic tissue and producing potent exotoxins, including alpha-toxin (lecithinase, phospholipase C) causing hemolysis, platelet aggregation, increased vascular permeability, and direct myonecrosis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sudden onset of severe pain at the wound site, marked swelling, tenderness, and crepitus on palpation, with skin progressing from pale to bronze to purplish-black with involving bleeding bullae, and systemic toxicity including tachycardia, tachypnea, hypotension, fever, and kidney failure.

Emergency warning signs

- Severe or worsening symptoms of gas gangrene (clostridial myonecrosis) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, with Gram stain showing gram-positive rods and radiographs showing gas in soft tissues. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Management requires urgent surgical exploration with extensive removal of dead tissue, high-dose penicillin G plus clindamycin, and aggressive supportive care. outlook: mortality is 100% without treatment
- With treatment, 20–60%.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 325

Gastric Adenocarcinoma

Overview

Gastric adenocarcinoma is the fifth most common cancer worldwide and the fourth leading cause of cancer-related death. The two histological subtypes are intestinal (associated with *H. pylori*, chronic atrophic gastritis, intestinal metaplasia, and dietary factors) and diffuse (associated with CDH1 mutations, affecting younger patients with worse outlook). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of multifactorial carcinogenesis involving *H. pylori* infection, high-salt diet, smoking, family history, and pernicious anemia. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)

- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of gastric adenocarcinoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment focuses on using a thin camera tube mucosal removal or surgical gastrectomy with lymphadenectomy for early-stage disease

- Perioperative chemotherapy (FLOT regimen), targeted therapy (trastuzumab for HER2-positive disease), and immunotherapy (nivolumab for PD-L1-positive tumors) for advanced disease. Unfortunately, the outlook is often poor for advanced disease but favorable for early-stage disease with appropriate treatment.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this based on upper endoscopy with biopsy, with staging using TNM classification, CT imaging, and laparoscopy for peritoneal assessment. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology

- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 326

Gastric Lymphoma

Overview

Gastric lymphoma is the most common extranodal lymphoma. Mucosa-associated lymphoid tissue (MALT) lymphoma accounts for the majority and is strongly associated with *H. pylori* infection. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

- This condition happens because of *H. pylori*-driven lymphoproliferation
- High-grade (diffuse large B-cell) lymphoma may arise from low-grade MALT lymphoma or develop de novo.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)

- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience epigastric pain, weight loss, and GI bleeding. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of gastric lymphoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment focuses on *H. pylori* eradication therapy for low-grade MALT lymphoma
- Antibiotic-refractory or high-grade disease requires chemotherapy (R-CHOP) and/or radiation.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this based on endoscopy with deep biopsy and immunohistochemistry. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and

depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

The outlook is generally positive for low-grade MALT lymphoma, which may regress completely with eradication therapy alone. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 327

Gastric Ulcer

Overview

Gastric ulcer is a mucosal defect extending through the muscularis mucosae into the deeper layers of the gastric wall, most commonly located along the lesser curvature and antrum. *Helicobacter pylori* infection is present in 60–80% of cases and NSAID use (including aspirin and low-dose aspirin) in most others.

Causes

This condition happens because of mucosal injury caused by *H. pylori* infection, NSAID-induced prostaglandin inhibition, and less commonly Zollinger-Ellison syndrome, Crohn's disease, or CMV infection in immunocompromised patients.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience epigastric pain aggravated by food (distinguishing it from duodenal ulcers, which improve with food), nausea, vomiting, and early satiety.

Emergency warning signs

- Severe or worsening symptoms of gastric ulcer require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- In most cases, the outlook is good with treatment
- Complications include bleeding (most common), perforation, gastric outlet obstruction, and rarely cancerous transformation.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on upper digestive tract endoscopy with biopsy to exclude malignancy (all gastric ulcers should be biopsied). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on *H. pylori* eradication when positive, cessation of NSAIDs, and proton pump inhibitors (PPIs) for 8 weeks with repeat endoscopy to confirm healing. Medications

to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 328

Gastroesophageal Reflux Disease

Overview

Gastroesophageal reflux disease (GERD) is a chronic condition characterized by symptoms of heartburn and regurgitation caused by reflux of gastric contents into the esophagus, occurring when the lower esophageal sphincter (LES) relaxes inappropriately or transiently. It affects approximately 20–30% of adults in Western countries weekly.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Diagnosis is primarily clinical
- A therapeutic trial of proton pump inhibitors (PPIs) is both diagnostic and therapeutic. Upper endoscopy is indicated for alarm features (difficulty swallowing, weight loss, GI bleeding, anemia) or symptoms persisting despite PPI therapy. Ambulatory pH monitoring (with impedance) is the gold standard for confirming abnormal acid exposure.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of gastroesophageal reflux disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- You may experience pyrosis (heartburn), regurgitation of acid or food, difficulty swallowing, odynophagia, chronic cough, laryngitis, and asthma-like symptoms
- Complications include erosive esophagitis, peptic stricture, Barrett's esophagus (intestinal metaplasia of the esophageal mucosa, a precursor to esophageal adenocarcinoma), and esophageal adenocarcinoma.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on lifestyle modifications (weight loss, elevation of the head of the bed, avoidance of trigger foods and late meals), antacids and alginate agents for mild symptoms,

H2 receptor antagonists for nocturnal symptoms, and PPIs (omeprazole, lansoprazole, esomeprazole) as the mainstay of therapy for moderate-to-severe disease. using small incisions and a camera Nissen fundoplication or magnetic sphincter augmentation (LINX device) are surgical options for refractory disease or patient preference to discontinue medical therapy. using a thin camera tube therapies (transoral incisionless fundoplication, radiofrequency ablation) are alternative options. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is good with appropriate medical or surgical management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 329

Gastrointestinal Stromal Tumor

Overview

digestive tract stromal tumor (GIST) is the most common mesenchymal tumor of the GI tract, arising from interstitial cells of Cajal. The stomach is the most common location (60–70% of GISTs). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of activating mutations in the KIT or PDGFRA receptor tyrosine kinase genes. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity

- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Symptoms can range from incidental discovery to GI bleeding, abdominal pain, or a palpable mass. Symptoms vary depending on the severity and extent of the condition
- Pain or discomfort in the affected area
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of gastrointestinal stromal tumor require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment focuses on surgical removal for localized disease
- Imatinib (a tyrosine kinase inhibitor) is used for advanced, unresectable, or metastatic disease and as adjuvant therapy for high-risk removed tumors.

Complications

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this based on endoscopy or imaging, confirmed by biopsy showing KIT (CD117) or DOG1 positivity. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

The outlook has been significantly improved with targeted tyrosine kinase inhibitor therapy. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 330

Gastroparesis

Overview

Gastroparesis is delayed gastric emptying in the absence of mechanical obstruction. It affects up to 5% of diabetic patients, and also occurs post-surgically (particularly after vagotomy or fundoplication), idiopathically, or due to medications (opioids, anticholinergics) and connective tissue disorders (scleroderma).

Causes

This condition happens because of impaired gastric motility due to autonomic neuropathy (diabetes), vagal nerve injury, or medication effects.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience nausea, vomiting of undigested food hours after eating, early satiety, postprandial fullness, and weight loss.

Emergency warning signs

- Severe or worsening symptoms of gastroparesis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on gastric emptying scintigraphy (4-hour study showing >10% retention at 4 hours). Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on dietary modification (small, frequent, low-fat, low-fiber meals), prokinetic agents (metoclopramide or domperidone), erythromycin (motilin receptor agonist), and gastric electrical stimulation (Enterra therapy) for refractory cases
- Metoclopramide carries a black box warning for tardive dyskinesia.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with symptomatic improvement achievable but cure uncommon. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 331

Generalized Anxiety Disorder (GAD)

Overview

Generalized anxiety disorder has a lifetime prevalence of 5–9% and is twice as common in women. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of hyperactivation of the amygdala, prefrontal cortex dysfunction, reduced GABAergic tone, elevated norepinephrine activity, and serotonergic dysregulation. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience excessive, uncontrollable worry about multiple domains for at least 6 months, occurring more days than not, with associated symptoms including

restlessness, easy fatiguability, difficulty concentrating, irritability, muscle tension, and sleep disturbance, with at least three of these required for diagnosis.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of generalized anxiety disorder (gad) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using clinical interview
- The GAD-7 scale is a validated screening tool.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include pharmacotherapy (SSRIs and SNRIs are first-line, with buspirone as an alternative) and psychotherapy (CBT is first-line). Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Many patients respond to treatment but symptoms may be chronic.

Chapter 332

Genital Herpes (HSV)

Overview

Genital herpes is caused by herpes simplex virus type 2 (HSV-2, most cases) and type 1 (HSV-1, increasing proportion). In the obstetric context, the primary concern is neonatal herpes transmission during vaginal delivery in women with active primary or recurrent genital lesions. The risk of neonatal transmission is 25–60% with primary infection during the third trimester and less than 2% with recurrent infection. Management at onset of labor: if active lesions or prodromal symptoms are present, cesarean delivery is recommended. For women with a history of recurrent genital herpes, suppressive acyclovir or valacyclovir is given from 36 weeks to delivery. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of genital herpes (hsv) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)

- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 333

Genital Warts (HPV)

Overview

- Genital warts are caused by low-risk human papillomavirus types 6 and 11. In the obstetric context, genital warts may increase in size and number during pregnancy. Treatment options in pregnancy include freezing treatment, trichloroacetic acid (TCA), or surgical removal
- Podophyllin, podofilox, and imiquimod are contraindicated. Cesarean delivery is reserved for cases where warts cause mechanical obstruction of the birth canal.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of genital warts (hpv) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 334

Gestational Diabetes Mellitus

Overview

Gestational diabetes mellitus (GDM) is glucose intolerance first recognized during pregnancy, affecting 2–10% of pregnancies. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of insufficient insulin secretion to overcome pregnancy-related insulin resistance mediated by placental hormones (human placental lactogen, cortisol, progesterone). Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Family history
- Obesity and sedentary lifestyle
- Poor dietary habits
- Advancing age
- Hormonal imbalances
- Certain medications
- Genetic predisposition and family history
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features are typically asymptomatic
- Screening is performed at 24–28 weeks gestation using the 75-g oral glucose tolerance test.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of gestational diabetes mellitus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Complications for the mother include preeclampsia and increased cesarean delivery rate
- Fetal/neonatal complications include macrosomia, shoulder dystocia, neonatal hypoglycemia, and long-term risk of obesity and T2DM.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on the oral glucose tolerance test.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include medical nutrition therapy, exercise, and insulin therapy when glycemic targets are not met with lifestyle measures
- Metformin may be used in select cases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is favorable
- GDM usually resolves postpartum, but women with GDM have a 50% lifetime risk of developing T2DM and should undergo periodic screening.

Chapter 335

Gestational Diabetes Mellitus (Obstetric Focus)

Overview

Gestational diabetes mellitus (GDM) is a pregnancy-related condition requiring coordinated obstetric care given its implications for pregnancy outcomes. Universal screening is performed at 24–28 weeks using a 75-g oral glucose tolerance test (OGTT). Poor glycemic control in GDM is linked to an increased risk of preeclampsia, polyhydramnios, preterm labor, and cesarean delivery. Fetal surveillance includes ultrasound monitoring for macrosomia and amniotic fluid index. Intrapartum Treatment options include close monitoring of maternal glucose to maintain normoglycemia, reducing the risk of neonatal hypoglycemia. Postpartum testing (75-g OGTT at 6–12 weeks) is essential as up to 50% of women with GDM develop type 2 diabetes within 10 years. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of gestational diabetes mellitus (obstetric focus) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 336

Gestational Hypertension

Overview

Gestational high blood pressure is a hypertensive disorder of pregnancy defined as new-onset high blood pressure (systolic BP ≥ 140 mmHg or diastolic BP ≥ 90 mmHg) after 20 weeks of gestation in the absence of proteinuria or other features of preeclampsia. It affects 6–8% of pregnancies. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of pregnancy-induced vasospasm and endothelial dysfunction. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol

- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience elevated blood pressure after 20 weeks without proteinuria. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of gestational hypertension require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events

- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

To make the diagnosis, doctors look for new-onset high blood pressure after 20 weeks in the absence of proteinuria or end-organ dysfunction. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment options include close monitoring of blood pressure and fetal surveillance
- Antihypertensive therapy is initiated for persistent BP $\geq 160/105$ mmHg using labetalol, nifedipine, or methyldopa.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)

- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

- The outlook is generally positive
- Blood pressure typically normalizes by 12 weeks postpartum, though approximately 25% of women progress to preeclampsia.

Chapter 337

Gestational Trophoblastic Disease

Overview

Gestational trophoblastic disease (GTD) encompasses a spectrum of disorders arising from abnormal trophoblast growth, ranging from non-cancerous hydatidiform mole to cancerous choriocarcinoma. Incidence is 1 in 1000 pregnancies in Western countries. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience vaginal bleeding, exaggerated uterine size for dates, hyperemesis gravidarum, and elevated β -hCG. Ultrasonography shows a characteristic “snowstorm” pattern in complete mole.

Emergency warning signs

- Severe or worsening symptoms of gestational trophoblastic disease require immediate medical evaluation

Progression and Stages

Hydatidiform mole is classified as complete (46,XX, entirely paternal genome, risk of cancerous transformation 15–20%) or partial (69,XXY, triploid, risk 1–5%). The condition

may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves suction dilation and curettage with serial β -hCG monitoring. Gestational trophoblastic neoplasia (GTN) is diagnosed when β -hCG plateaus or rises after molar evacuation

- Low-risk GTN doctors typically treat it with single-agent methotrexate or actinomycin D
- High-risk GTN requires multi-agent chemotherapy.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well, with cure rates exceeding 90% for low-risk and 80–85% for high-risk disease. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 338

Giant Cell Arteritis

Overview

Giant cell arteritis is the most common primary systemic vasculitis, characterized by granulomatous inflammation of large and medium-sized arteries, with a predilection for the cranial branches of the aorta, particularly the temporal artery, exclusively affecting individuals over 50 years of age (peak incidence 70–80 years), with a female predominance of 2–3:1, and being more common in Northern European descendants.

Causes

This condition happens because of a Th1- and Th17-driven inflammatory response targeting the vessel wall, with CD4+ T-cell infiltration, multinucleated giant cells, intimal abnormal increase in cells, and luminal occlusion, with IL-6 being a key driver.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience new-onset severe headache (the most common symptom, often temporal), scalp tenderness, jaw claudication (pathognomonic), visual disturbances (amaurosis fugax, diplopia, anterior ischemic optic neuropathy leading to irreversible blindness in 15–20% if untreated), polymyalgia rheumatica symptoms in 40–60%, fever, weight loss, and an elevated ESR, with aortic involvement (aortitis, aortic aneurysm, aortic dissection) occurring in 10–20%.

Emergency warning signs

- Severe or worsening symptoms of giant cell arteritis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for a high index of suspicion: ESR is almost always markedly elevated (>50 – 100 mm/hr), CRP is elevated, and temporal artery biopsy (TAB, at least 3–5 cm in length) shows granulomatous inflammation with multinucleated giant cells. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves with high-dose corticosteroids (prednisone 40–60 mg/day or IV methylprednisolone if vision threatened), with tocilizumab (anti-IL-6 receptor) approved for GCA and used as a steroid-sparing agent. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with prompt treatment, though delayed treatment risks irreversible blindness. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 339

Giardiasis

Overview

Giardiasis is a protozoan diarrheal illness caused by the flagellate *Giardia lamblia*, the most common intestinal parasitic infection in the United States and a leading cause of waterborne outbreaks globally, with 300 million cases annually worldwide.

Causes

This condition happens because of cysts excysting in the duodenum, releasing trophozoites that adhere to enterocytes via a ventral sucking disk, causing enterocyte damage, microvillus shrinkage or wasting, and increased intestinal permeability.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 1–3 weeks, with acute giardiasis presenting with watery, foul-smelling diarrhea (sulfuriferous), abdominal cramps, bloating, flatulence, and steatorrhea, and chronic giardiasis presenting with persistent diarrhea, weight loss, malabsorption, and fatigue.

Emergency warning signs

- Severe or worsening symptoms of giardiasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using microscopy of stool for cysts and trophozoites or antigen detection. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include tinidazole or metronidazole. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 340

Gingivitis and Periodontitis

Overview

Gingivitis is an inflammation of the gingiva without attachment loss, representing the earliest and most common form of periodontal disease, caused by dental plaque biofilm accumulation. Periodontitis extends beyond the gingiva with loss of connective tissue attachment and alveolar bone, leading to pocket formation, gingival recession, tooth mobility, and eventual tooth loss.

Causes

This condition happens because of bacterial plaque accumulation and the host inflammatory response.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience red, swollen, bleeding gingiva for gingivitis
- Periodontitis additionally presents with pocket formation, gingival recession, and tooth mobility.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of gingivitis and periodontitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on clinical examination and probing depths. Treatment for gingivitis involves oral hygiene instruction and professional cleaning
- Periodontitis Treatment usually involves scaling and root planing, surgical intervention, and systemic antibiotics for aggressive disease.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well for gingivitis with proper hygiene
- Periodontitis requires long-term maintenance.

Chapter 341

Glaucoma in Children

Overview

Primary congenital glaucoma is a condition resulting from developmental abnormality of the trabecular meshwork (goniodysgenesis), causing elevated intraocular pressure from birth or early childhood. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of abnormal development of the aqueous outflow pathways. Degenerative processes involve progressive cellular dysfunction and tissue breakdown over time. Contributing factors include oxidative stress, inflammation, accumulated cellular damage, and reduced regenerative capacity. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience epiphora, blepharospasm, photophobia, buphthalmos, and corneal swelling
- Juvenile open-angle glaucoma presents in older children and adolescents.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of glaucoma in children require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on examination under anesthesia and tonometry. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Your outlook depends on early intervention to prevent irreversible optic nerve damage and amblyopia.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on early surgical intervention (goniotomy or trabeculotomy)
- Secondary childhood glaucoma may result from aphakia, trauma, uveitis, or steroids.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 342

Glioblastoma

Overview

This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

- This condition happens because of rapid infiltrative growth with microvascular growth and areas of tissue death
- The IDH-wildtype molecular profile is characteristic.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)

- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Your symptoms depend on tumor location but typically include progressive headache, seizures, weakness, numbness, or problems with movement, and thinking and memory changes. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of glioblastoma require immediate medical evaluation

Progression and Stages

Glioblastoma is a WHO grade 4 astrocytoma and the most common and aggressive primary cancerous brain tumor in adults. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this based on MRI showing a ring-enhancing lesion with surrounding swelling and mass effect. Management follows the Stupp protocol: maximal safe surgical removal followed by concurrent temozolomide chemotherapy and radiotherapy, then adjuvant temozolomide

- MGMT promoter methylation status predicts response to temozolomide
- Tumor-treating fields (TTFields) have shown incremental survival benefit.
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs

Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian

- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Unfortunately, the outlook is often poor despite aggressive treatment, with median survival of approximately 15 months. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 343

Glossopharyngeal Neuralgia

Overview

Glossopharyngeal neuralgia is a rare neuropathic pain disorder characterized by paroxysmal, severe, stabbing pain in the posterior pharynx, tonsillar fossa, base of tongue, and deep ear (cranial nerves IX and X). It may be caused by neurovascular compression of the glossopharyngeal nerve. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience attacks triggered by swallowing, coughing, talking, or yawning, which can be associated with bradycardia, syncope, or asystole due to vagal activation.

Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of glossopharyngeal neuralgia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include carbamazepine or oxcarbazepine as first-line
- Surgical options include microvascular decompression or through the skin radiofrequency rhizotomy.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 344

Glycogen Storage Diseases

Overview

- Glycogen storage diseases (GSDs) are a group of autosomal recessive disorders (except GSD IX, X-linked) resulting from defects in glycogen synthesis, degradation, or glycolysis, leading to abnormal glycogen accumulation. Von Gierke disease (GSD I) is caused by glucose-6-phosphatase deficiency, presenting with severe fasting hypoglycemia, hepatomegaly, growth retardation, lactic acidosis, hyperuricemia, hypertriglyceridemia, and a rounded, doll-like facies
- Treatment usually involves continuous nasogastric infusion of uncooked cornstarch. Pompe disease (GSD II) is caused by acid maltase deficiency, a lysosomal storage disorder presenting in infants with hypertrophic cardiomyopathy, macroglossia, hypotonia (floppy baby), and rapidly progressive weakness
- Enzyme replacement therapy (alglucosidase alfa) improves survival. Cori disease (GSD III) is caused by debranching enzyme deficiency, causing hypoglycemia, hepatomegaly, myopathy, and possible cardiomyopathy. McArdle disease (GSD V) is myophosphorylase deficiency (see Chapter 16, Muscles).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of glycogen storage diseases require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 345

Gonorrhea

Overview

Gonorrhea is a sexually transmitted infection caused by the gram-negative diplococcus *Neisseria gonorrhoeae*. It primarily infects the mucous membranes of the urogenital tract, rectum, pharynx, and conjunctivae. Untreated gonorrhea can lead to serious reproductive complications and facilitate HIV transmission.

Classification

Cause Infectious (bacterial)

Organ System Infectious/Reproductive

Pathophysiology *N. gonorrhoeae* attaches to columnar epithelial cells via pili and Opa proteins, triggers local inflammation with neutrophil infiltration, and can evade immune clearance through antigenic variation

Duration Acute (weeks if treated); chronic if untreated

Onset Sudden (2–14 days after exposure)

Mode of Transmission Sexual contact (vaginal, anal, oral); perinatal

Clinical Course Acute urethritis or cervicitis with purulent discharge; may progress to ascending infection or become asymptomatic

Severity Mild to moderate; severe with disseminated disease

Extent of Disease Localized (urogenital, rectal, pharyngeal); systemic in disseminated gonococcal infection

Age Group Sexually active adolescents and young adults (15–29 years)

Epidemiology Second most commonly reported bacterial STI worldwide; highest incidence in 15–24-year-old women and 20–29-year-old men

ICD-11 Code AA00.0

Causes

Gonorrhea is caused by infection with *Neisseria gonorrhoeae*, an aerobic gram-negative bacterium that colonizes mucosal surfaces. The bacterium penetrates columnar and transitional epithelium, triggering an intense neutrophilic inflammatory response that produces the characteristic purulent discharge.

Risk Factors

- Unprotected vaginal, anal, or oral sex
- Multiple sexual partners or a partner with multiple partners
- Age 15–29 years
- Previous or concurrent STI (especially chlamydia)
- Inconsistent condom use
- Sex work or transactional sex
- Substance use associated with high-risk sexual behavior
- Men who have sex with men (MSM)

Signs and Symptoms

Early symptoms

- Early manifestations of gonorrhea may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Purulent urethral discharge (men)
- Dysuria and urinary urgency
- Cervicitis with mucopurulent discharge (women)
- Intermenstrual bleeding or menorrhagia
- Anal discharge, itching, or pain (rectal infection)
- Sore throat (pharyngeal infection, often asymptomatic)
- Conjunctivitis with purulent discharge (ocular infection)
- Testicular pain or swelling (epididymitis)
- Lower abdominal or pelvic pain
- Many infections (especially in women) are asymptomatic

Less common symptoms

- Atypical or less common presentations of gonorrhea may occur

Emergency warning signs

- Severe or worsening symptoms of gonorrhea require immediate medical evaluation

Progression and Stages

Incubation is typically 2–8 days. Acute infection presents with urethritis or cervicitis; without treatment, the infection can ascend to cause epididymitis in men and pelvic inflammatory disease (PID) in women. Hematogenous dissemination occurs in 1–3% of cases, causing dermatitis, tenosynovitis, and septic arthritis.

Complications

- Pelvic inflammatory disease (PID) with tubal scarring
- Infertility (tubal factor in women; epididymal obstruction in men)
- Ectopic pregnancy
- Disseminated gonococcal infection (DGI) with dermatitis and arthritis
- Neonatal ophthalmia (ophthalmia neonatorum) from perinatal exposure
- Increased HIV acquisition and transmission risk
- Fitz-Hugh–Curtis syndrome (perihepatitis)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Nucleic acid amplification test (NAAT) of urine, urethral, cervical, rectal, or pharyngeal swab
- Gram stain of urethral discharge showing gram-negative intracellular diplococci (men)
- Culture with antimicrobial susceptibility testing (if treatment failure suspected)
- Test for concurrent chlamydia, HIV, and syphilis
- Pelvic examination in women with suspected PID
- Lab tests
- Imaging tests (pelvic ultrasound for suspected PID)

Prevention

- Consistent and correct condom use
- Routine STI screening for sexually active young women and MSM
- Partner notification and treatment (expedited partner therapy)
- Avoid sexual contact until treatment is complete
- Regular check-ups

Treatment Options

- Ceftriaxone 500 mg intramuscularly as a single dose (first-line)
- Treatment for concurrent chlamydia (doxycycline 100 mg twice daily for 7 days)

- Test of cure for pharyngeal infections
- Sexual partners should be treated empirically
- Lifestyle changes

Living with It

- Follow treatment plan
- Regular appointments
- Adjust daily habits
- Support groups
- Keep learning

Outlook

With appropriate antibiotic treatment, uncomplicated gonorrhea is completely curable and symptoms resolve within days. However, reinfection is common without partner treatment and behavior change. Antimicrobial resistance (especially extended-spectrum cephalosporin resistance) is a growing global concern that may complicate future treatment.

Chapter 346

Gout

Overview

Gout is the most common inflammatory arthropathy, caused by deposition of monosodium urate crystals in joints and soft tissues due to chronic hyperuricemia (serum urate >6.8 mg/dL, the saturation point). It predominantly affects middle-aged men and postmenopausal women. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition results from disruption of normal gout function.

- Acute gouty arthritis classically presents as sudden, severe monoarthritis of the first metatarsophalangeal joint (podagra), with intense pain, redness of the skin, warmth, and swelling
- Recurrent attacks may involve multiple joints. Chronic tophaceous gout causes tophi (urate deposits) in joints, cartilage, and soft tissues.

Risk Factors

- Family history
- Obesity and sedentary lifestyle
- Poor dietary habits
- Advancing age
- Hormonal imbalances
- Certain medications
- Genetic predisposition and family history
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of gout require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on synovial fluid analysis showing needle-shaped, negatively birefringent crystals under polarized light microscopy. Treatment for acute attacks includes NSAIDs, colchicine (most effective within 12 hours of onset), or corticosteroids
- Long-term urate-lowering therapy (ULT) with allopurinol or febuxostat (xanthine oxidase inhibitors) or probenecid (uricosuric) is indicated for recurrent gout, tophi, or joint damage, targeting serum urate <6 mg/dL.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with effective urate-lowering therapy.

Chapter 347

Granuloma Annulare

Overview

Granuloma annulare (GA) is a benign, chronic inflammatory skin condition characterized by annular (ring-shaped) papules or plaques, most commonly on the extremities. It is a self-limited dermatosis of unknown cause that affects people of all ages, with a predilection for children and young adults. The condition is not contagious and typically resolves spontaneously over months to years.

Classification

Cause Idiopathic / Inflammatory

Organ System Skin

Pathophysiology Lymphohistiocytic granulomatous inflammation with necrobiotic collagen degeneration

Duration Chronic (months to years)

Onset Gradual

Mode of Transmission Non-communicable

Clinical Course Chronic, self-limited

Severity Mild

Extent of Disease Localized to Generalized

Age Group Children and Young Adults (peak <30 years)

Epidemiology Prevalence 0.1–0.4%; female predominance 2:1

ICD-11 Code EE71.0

Causes

The exact cause of granuloma annulare is unknown. The condition is thought to represent a delayed-type hypersensitivity reaction with T-cell-mediated immune response. Associations have been reported with diabetes mellitus, thyroid disease, dyslipidemia, and malignancy (particularly in generalized GA). Trauma, sun exposure, and viral infections have been proposed as triggers in susceptible individuals.

Risk Factors

- Female sex
- Diabetes mellitus (particularly generalized GA)
- Thyroid disease
- Dyslipidemia
- Malignancy (generalized GA in older adults)
- HIV infection
- Family history (rare)
- UV exposure (possibly)

Signs and Symptoms

Early symptoms

- Early manifestations of granuloma annulare may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Firm, smooth, flesh-colored to erythematous papules arranged in an annular (ring) configuration
- Most commonly on the dorsal hands, feet, wrists, and ankles
- Central clearing with raised border (classic presentation)
- Generally asymptomatic; mild pruritus in some cases
- No scale, no ulceration
- Variants: localized (most common), generalized, subcutaneous, perforating, patch
- Difficulty performing daily activities (cosmetic concern)

Less common symptoms

- Atypical or less common presentations of granuloma annulare may occur

Emergency warning signs

- Severe or worsening symptoms of granuloma annulare require immediate medical evaluation

Progression and Stages

Localized GA typically persists for 1–2 years before spontaneous resolution, though it may last longer. Generalized GA tends to have a more chronic course lasting 3–5 years or

more. Recurrence occurs in up to 40% of cases. Subcutaneous GA (common in children) usually resolves within 1–2 years. The condition does not cause permanent scarring.

Complications

- Cosmetic concern and psychosocial impact
- Large or disseminated lesions
- Association with underlying systemic disease (diabetes, malignancy in generalized form)
- Recurrence after resolution
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on characteristic annular morphology and distribution
- Skin biopsy shows palisading granulomas with necrobiotic collagen and mucin deposition
- Biopsy may be needed to differentiate from tinea corporis, sarcoidosis, erythema migrans
- Blood glucose or HbA1c testing to screen for diabetes (generalized GA)
- Lipid panel and thyroid function tests as clinically indicated

Prevention

- No established prevention measures
- Manage underlying conditions (diabetes, thyroid disease)
- Sun protection may help in photosensitive cases
- Go for regular check-ups so any problems are caught early

Treatment Options

- Observation and reassurance (many cases resolve spontaneously)
- High-potency topical corticosteroids under occlusion
- Intralesional corticosteroid injections for localized lesions
- Topical calcineurin inhibitors (tacrolimus, pimecrolimus)
- Phototherapy (PUVA, narrowband UVB) for generalized GA
- Systemic therapies for refractory generalized GA (hydroxychloroquine, dapsone, methotrexate, TNF inhibitors)
- Cryotherapy for small lesions
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Granuloma annulare is a benign, self-limited condition with an excellent prognosis. Approximately 50% of localized cases resolve within 2 years. Generalized GA tends to persist longer but eventually resolves in most cases. Treatment accelerates clearance but does not prevent recurrence. The condition does not cause permanent tissue damage or increase mortality, though underlying disease associations should be evaluated in generalized cases.

Chapter 348

Granulomatosis with Polyangiitis

Overview

Granulomatosis with polyangiitis is an ANCA-associated necrotizing granulomatous vasculitis affecting small and medium-sized vessels, with a predilection for the respiratory tract and kidneys, with an incidence of approximately 2–12 per million, equal sex distribution, and typically presenting in the fourth to sixth decade. The condition involves c-ANCA (cytoplasmic) directed against proteinase 3 (PR3), leading to neutrophil activation, degranulation, and respiratory burst, causing endothelial injury and necrotizing inflammation. The classic triad involves the upper respiratory tract (sinusitis, epistaxis, nasal crusting, saddle nose deformity, subglottic stenosis), lower respiratory tract (cough, hemoptysis, lung nodules and cavitary lesions, alveolar bleeding), and kidney (rapidly progressive glomerulonephritis with crescents—pauci-immune on immunofluorescence), with other features including ocular involvement (scleritis, episcleritis, orbital pseudotumor), cutaneous purple spots on the skin, mononeuritis multiplex, and arthralgias.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of granulomatosis with polyangiitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for histologic confirmation (nasal, endobronchial, or kidney biopsy showing granulomatous inflammation, multinucleated giant cells, and necrotizing vasculitis) or serologic evidence of PR3-ANCA (positive in >90% of active generalized GPA). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves with high-dose corticosteroids combined with rituximab or cyclophosphamide for induction of remission, with maintenance therapy using rituximab (scheduled infusions every 6 months) or methotrexate/azathioprine. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-

care measures Physical therapy and rehabilitation Surgical intervention when indicated
Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has improved dramatically with modern therapy, with 5-year survival exceeding 80%. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 349

Graves Disease

Overview

Graves disease is an autoimmune disorder and the most common cause of hyperthyroidism (60–80% of cases), occurring predominantly in women aged 20–50.

Causes

This condition happens because of thyroid-stimulating immunoglobulin (TSI/TSHR autoantibodies) binding to and activating the TSH receptor, stimulating unregulated thyroid hormone synthesis and diffuse thyroid abnormal increase in cells.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience heat intolerance, weight loss despite increased appetite, tachycardia, palpitations, fine tremor, diffuse non-tender goiter with bruit, and specific extrathyroidal manifestations: Graves ophthalmopathy (exophthalmos, proptosis, lid lag due to orbital fibroblast TSHR expression) and pretibial myxedema.

Emergency warning signs

- Severe or worsening symptoms of graves disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with suppressed TSH, elevated free T4 and T3, positive TSI antibodies, and diffuse increased radioactive iodine uptake (RAIU). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include antithyroid drugs (methimazole, propylthiouracil), radioactive iodine ablation, or thyroidectomy, along with beta-blockers (propranolol) for symptom control. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and

support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 350

Graves' Eye Disease

Overview

- Graves' eye disease, also called thyroid eye disease or Graves' orbitopathy, is an autoimmune inflammation of the tissues behind the eye
- It affects about 25–50% of people with Graves' disease (overactive thyroid) and can occur with any thyroid condition
- Symptoms range from mild dryness and eye prominence to double vision and, rarely, sight-threatening optic nerve compression

Causes

This condition happens because of an autoimmune reaction in which antibodies stimulate fibroblasts and immune cells in the eye socket (orbit), causing inflammation, swelling, and fat and muscle enlargement. The expanding tissue pushes the eye forward (proptosis). Smoking, thyroid dysfunction, and radiation therapy for thyroid disease can worsen the condition.

Risk Factors

- Graves' disease (hyperthyroidism)
- Female sex and middle age
- Cigarette smoking (strongly increases severity)
- Radioactive iodine treatment that causes fluctuating thyroid hormone levels
- Family history of autoimmune thyroid disease
- High thyroid antibody levels

Signs and Symptoms

Common symptoms

- Bulging eyes (proptosis)
- Dry, gritty, or watery eyes
- Redness, puffiness, and swelling around the eyes
- Sensitivity to light and double vision (diplopia)
- Difficulty fully closing the eyelids
- Pain behind the eyes worsened by eye movement

Emergency warning signs

- Sudden vision loss, severe double vision, or intense eye pain
- Inability to close the eyes causing corneal exposure
- Signs of optic nerve compression, such as reduced color vision or vision loss — these require urgent specialist care

Progression and Stages

The disease follows an active (inflammatory) phase lasting 6 months to 2 years, followed by a stable (inactive) phase. Progression ranges from mild to severe. The majority of cases are mild, but a minority progress to moderate or severe disease with significant proptosis, diplopia, or optic neuropathy. Treatment in the active phase aims to reduce inflammation; rehabilitation is deferred to the stable phase.

Complications

- Optic nerve compression leading to vision loss
- Corneal exposure, dryness, and ulceration
- Permanent double vision
- Disfiguring proptosis
- Facial and psychological distress from appearance changes

Diagnosis

- Clinical examination, including measurement of eye protrusion (exophthalmometry) and eye movement
- Thyroid function tests (TSH, free T4, T3) and thyroid antibodies (TSI, TRAb)
- Orbital CT or MRI to assess tissue swelling and muscle involvement
- Visual acuity, color vision, and visual field testing to check for optic nerve compression
- Optical coherence tomography (OCT) of the optic nerve in severe cases

Prevention

- Quit smoking, as this is the strongest modifiable risk factor
- Stable control of thyroid hormone levels
- Avoidance of hypothyroidism after thyroid treatment
- Early referral to an eye specialist when eye symptoms develop

Treatment Options

- Selenium supplements and lubricating eye drops for mild disease
- Corticosteroids (oral or intravenous) for active moderate-to-severe inflammation

- Immunomodulators (e.g., rituximab, tocilizumab) or teprotumumab for active disease
- Orbital radiation therapy in selected cases
- Surgery for stable disease: orbital decompression for proptosis, strabismus surgery for diplopia, eyelid surgery
- Management of thyroid status by an endocrinologist

Living with It

- Protect eyes with sunglasses and lubricants, especially outdoors
- Elevate the head while sleeping to reduce periorbital swelling
- Seek support for appearance concerns and connect with support groups
- Attend coordinated endocrinology and ophthalmology follow-up

Outlook

Most cases of Graves' eye disease are mild and self-limited, and the active phase eventually burns out. With modern therapy, severe disease is usually controlled, and surgery restores function and appearance in most. Vision loss from optic neuropathy is permanent if untreated but is rare when managed promptly.

Chapter 351

Guillain-Barré Syndrome

Overview

- Guillain-Barré syndrome is an acute, immune-mediated polyneuropathy characterized by rapidly progressive, symmetric limb weakness with areflexia
- It often follows a respiratory or digestive tract infection by 1–4 weeks, with *Campylobacter jejuni*, cytomegalovirus, Epstein-Barr virus, and Zika virus being common triggers.

Causes

This condition happens because of an autoimmune attack on peripheral nerves triggered by preceding infection. The most common variant is acute inflammatory demyelinating polyneuropathy (AIDP), while acute motor axonal neuropathy (AMAN) is more prevalent in Asia. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience rapidly progressive symmetric weakness, areflexia, and possible autonomic dysfunction (tachycardia, blood pressure lability) and respiratory muscle weakness requiring ventilatory support.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of guillain-barré syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical features and CSF analysis showing albuminocytological dissociation (elevated protein with normal cell count).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include intravenous immunoglobulin (IVIg) or plasmapheresis. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational

therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person, with most patients recovering
- 5–10% die and 10–20% have significant residual disability.

Chapter 352

Gynecomastia

Overview

Gynecomastia is non-cancerous enlargement of breast tissue in males, caused by an imbalance of estrogen and androgen effects, present in up to 70% of adolescent males (physiologic gynecomastia, typically resolving within 1–2 years) and up to 65% of men over 50. Pathological causes include medications (spironolactone, finasteride, cimetidine, chemotherapy, exogenous testosterone/steroids, cannabinoids), liver disease, hypogonadism, hyperthyroidism, Klinefelter syndrome, and obesity (pseudogynecomastia). This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a palpable, firm, disk-shaped tissue beneath the areola. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of gynecomastia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical examination and evaluation of hormonal levels. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include addressing reversible causes, observation for physiologic or medication-induced gynecomastia, and surgical mastectomy for persistent, symptomatic cases. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 353

HELLP Syndrome

Overview

It occurs in 0.5–0.9% of pregnancies and in 10–20% of women with severe preeclampsia. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of microvascular endothelial damage leading to platelet activation, microangiopathic hemolytic anemia, and hepatocellular tissue death. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience right upper quadrant or epigastric pain (65%), nausea and vomiting (50%), and headache (30%). The Tennessee classification requires all three criteria: hemolysis (LDH >600 U/L, schistocytes on smear, bilirubin >1.2 mg/dL), elevated liver enzymes (AST $\geq$ 70 U/L), and low platelets (<100,000/ μ L).

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hellp syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

HELLP syndrome is a serious complication of severe preeclampsia characterized by Hemolysis, Elevated Liver enzymes, and Low Platelets.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves delivery, which usually results in disease resolution

- Corticosteroids may improve platelet count temporarily

- Plasma exchange may be considered for persistent postpartum HELLP.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with delivery
- Differential diagnosis includes acute fatty liver of pregnancy, TTP, and HUS.

Chapter 354

HIV/AIDS

Overview

Human immunodeficiency virus (HIV) infection is a progressive immune deficiency caused by HIV-1 (global pandemic) or HIV-2 (West Africa), ultimately leading to acquired immunodeficiency syndrome (AIDS) if untreated. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of targeting of CD4+ T lymphocytes, macrophages, and dendritic cells, with transmission occurring through sexual contact, blood-borne exposure, and vertical transmission (mother to child). Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Clinical features follow a natural history including acute retroviral syndrome (fever, rash, lymphadenopathy, muscle pain 2–4 weeks after infection), clinical latency (chronic infection with asymptomatic period lasting years), and AIDS (CD4 count <200 cells/ μL or presence of AIDS-defining opportunistic infections and malignancies).
Fever and chills
Fatigue and malaise
Localized pain and inflammation at infection site
Swelling, redness, and warmth (local infection)
Cough and respiratory symptoms (respiratory infections)
Nausea, vomiting, and diarrhea (gastrointestinal infections)
Headache and body aches
Skin rash or lesions
Enlarged lymph nodes
Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of hiv/aids require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using fourth-generation HIV antigen/antibody combination immunoassay, with confirmatory testing using HIV-1/HIV-2 differentiation immunoassay and HIV RNA viral load. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Treatment typically involves with antiretroviral therapy (ART), typically a combination of three or more drugs from different classes (NRTIs, NNRTIs, integrase inhibitors, protease inhibitors, entry inhibitors), with goals of viral suppression (undetectable viral load), immune reconstitution (CD4 recovery), and prevention of transmission (U=U: Undetectable = Untransmittable). Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices

including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

The outlook has been transformed by ART, with near-normal life expectancy for adherent patients. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 355

Haff Disease

Overview

Haff disease is a rare syndrome of rhabdomyolysis occurring within 24 hours of consuming certain freshwater or marine fish or crustaceans.

Causes

The cause is an unknown toxin (possibly a heat-stable palytoxin-like compound from algae/dinoflagellates that accumulates in the food chain).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: sudden onset of severe muscle pain, muscle tenderness, weakness, dark urine (myoglobinuria), and elevated serum creatine kinase (CK, often >10,000 IU/L) within 6–24 hours of fish consumption, without fever, nervous system deficits, or rash.

Emergency warning signs

- Severe or worsening symptoms of haff disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with CK elevation, urinalysis (heme-positive without RBCs), and history of recent fish consumption. Management: aggressive IV fluids (target urine output >200 mL/hour), monitoring for acute kidney injury and compartment syndrome, and correction of electrolyte abnormalities. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Most patients recover within days without long-term sequelae if treated promptly.

Chapter 356

Hallucinogen Toxicity

Overview

LSD (lysergic acid diethylamide) and psilocybin are serotonin 5-HT_{2A} receptor agonists producing altered perception, synesthesias, depersonalization, emotional lability, and hallucinations (typically visual). Physiological effects include dilated pupils, tachycardia, high blood pressure, hyperthermia, and diaphoresis. Acute adverse events include bad trips (anxiety, panic, dysphoria), psychosis, flashbacks, and serotonin syndrome. Phencyclidine (PCP) is a dissociative anesthetic acting as an NMDA receptor antagonist. Toxicity produces involuntary eye movements (horizontal, vertical, or rotatory—diagnostic clue), high blood pressure, tachycardia, hyperthermia, psychosis, violent behavior, rhabdomyolysis, and seizures. Large-volume fluid resuscitation for rhabdomyolysis.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hallucinogen toxicity require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment: benzodiazepines for agitation, antipsychotics for psychosis (caution: haloperidol may lower seizure threshold), and supportive care.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 357

Hallux Valgus

Overview

Hallux valgus is a structural deformity of the first ray of the foot characterized by lateral deviation of the hallux (great toe) and medial displacement of the first metatarsal head.

Causes

This condition happens because of biomechanical instability of the first metatarsophalangeal (MTP) joint, often exacerbated by constrictive footwear, ligamentous laxity, flat feet (pes planus), or genetic predisposition.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a painful exostosis (bunion) over the medial eminence of the first metatarsal head, lateral crowding of second and third toes, metatarsalgia, and difficulty fitting shoes.

Emergency warning signs

- Severe or worsening symptoms of hallux valgus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Your doctor confirms the diagnosis with weight-bearing foot radiographs showing a first MTP angle $> 15^\circ$ and an intermetatarsal angle $> 9^\circ$. Management begins with wide-toebox shoes, orthotics, and night splints
- Surgical osteotomy and soft tissue balancing are indicated for severe refractory pain.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 358

Hand, Foot, and Mouth Disease

Overview

Hand, foot, and mouth disease (HFMD) is a highly contagious pediatric viral exanthem caused by enteroviruses, most commonly Coxsackievirus A16 and Enterovirus 71.

Causes

This condition happens because of fecal-oral or oral-oral transmission with primary viral replication in pharyngeal and digestive tract lymphoid tissue followed by viremic cutaneous and mucosal dissemination.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience low-grade fever, malaise, and sore throat followed by painful vesiculoulcerative enanthem on the buccal mucosa, tongue, and soft palate, and a non-pruritic, non-tender maculopapular or oval vesicular exanthem on the palms of the hands, soles of the feet, and buttocks.

Emergency warning signs

- Severe or worsening symptoms of hand, foot, and mouth disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Treatment typically involves supportive with oral fluid hydration and analgesics (aspirin is strictly avoided)
- Severe neurological complications (aseptic meningitis, brainstem encephalitis) may rarely occur with Enterovirus 71.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam based on characteristic lesion distribution. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 359

Hantavirus Pulmonary Syndrome

Overview

Hantavirus lung syndrome is a severe, often fatal respiratory illness caused by hantaviruses, with the primary cause in North America being Sin Nombre virus (SNV), with 800 cases reported in the Americas since 1993, mortality 30–40%, and transmission through inhalation of aerosolized excreta from infected rodents. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of the virus infecting endothelial cells (particularly in the lungs), causing increased vascular permeability without direct cytotoxicity, mediated by CD8+ T cell activation and cytokine release disrupting endothelial barrier function. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hantavirus pulmonary syndrome require immediate medical evaluation

Progression and Stages

You may experience an incubation of 9–35 days, with prodrome (3–5 days): fever, muscle pain, headache, and digestive tract symptoms, followed by rapid onset of cardiopulmonary phase: cough, shortness of breath, and respiratory failure from lung swelling, often within 24 hours, with hypotension and shock being common. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam in endemic areas, confirmed by serology.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment typically involves aggressive supportive care (mechanical ventilation, vasopressors, ECMO in severe cases). outlook: mortality 30–40%.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 360

Hashimoto Thyroiditis

Overview

Hashimoto thyroiditis (chronic lymphocytic thyroiditis) is an autoimmune disease and the most common cause of hypothyroidism in iodine-sufficient regions, affecting women 10 times more frequently than men. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of cell-mediated cytotoxicity (CD8+ T cells) and anti-thyroid peroxidase (anti-TPO) and anti-thyroglobulin autoantibodies, leading to lymphocytic infiltration, germinal center formation, Hurthle cell metaplasia, and progressive destruction of thyroid follicles. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- You may experience cold intolerance, weight gain, fatigue, constipation, dry skin, bradycardia, muscle weakness, and a non-tender firm goiter. Patients may experience transient hyperthyroidism (hashitoxicosis) early in the disease due to follicular destruction. Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of hashimoto thyroiditis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease

activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Your doctor confirms the diagnosis with elevated serum TSH, low free T4, and high titers of anti-TPO antibodies. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Treatment typically involves lifelong hormone replacement with levothyroxine, adjusted to maintain TSH within normal limits. Treatment aims to control inflammation, manage symptoms, and prevent disease flares. A step-up approach is often used, starting with symptomatic treatments and progressing to immunosuppressive therapies as needed. Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Most people recover well with proper dosing. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 361

Head Lice (Pediculosis Capitis)

Overview

- Head lice is infestation of the scalp by *Pediculus humanus capitis*, common in school-age children (6–12 million cases/year in the US). Transmission occurs through direct head-to-head contact
- Fomite transmission is less common.

Causes

This condition happens because of louse infestation with eggs (nits) laid at the hair base that hatch in 7–10 days and mature in 2–3 weeks.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience scalp itching (from hypersensitivity to louse saliva), visible nits (0.8 mm, yellow-white, firmly attached to the hair shaft), and less commonly live lice.

Emergency warning signs

- Severe or worsening symptoms of head lice (pediculosis capitis) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using visual inspection with a fine-toothed comb

- Wood's lamp shows nits as pale blue. The first-choice treatment typically involves permethrin 1% or pyrethrins (OTC), applied for 10 minutes and repeated in 7–10 days
- Second-line options include malathion 0.5% lotion, benzyl alcohol 5%, spinosad 0.9%, and ivermectin 0.5% lotion
- Oral ivermectin is used for refractory cases. Wet combing is a non-chemical option. All close contacts should be checked
- Environmental measures include washing bedding in hot water.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with appropriate treatment
- No-nit policies in schools are not recommended by the AAP/CDC.

Chapter 362

Headache Attributed to Giant Cell Arteritis

See Chapter ??, Section [338](#).

Overview

This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of headache attributed to giant cell arteritis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 363

Headache Attributed to Intracranial Hypertension

Overview

Idiopathic intracranial high blood pressure (IIH, pseudotumor cerebri) is a secondary headache disorder characterized by elevated intracranial pressure without identifiable cause. The annual incidence is 0.5–3 per 100,000, rising to 12–20 per 100,000 in obese women of childbearing age. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of impaired CSF absorption at the arachnoid granulations or elevated cerebral venous sinus pressure. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking

- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of headache attributed to intracranial hypertension require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- You may experience daily, diffuse, pressure-like headache worse in the morning or with Valsalva, transient visual obscurations, pulsatile ringing in the ears, and papilledema
- Visual loss is the most serious complication.
- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

To make the diagnosis, doctors look for elevated opening pressure on lumbar puncture (>25 cm H₂O) with normal CSF composition, normal neuroimaging, and no alternative cause. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid. History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose

- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment options include weight loss, acetazolamide, and serial ophthalmological monitoring
- Refractory cases may require optic nerve sheath fenestration or CSF shunting.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

- The outlook varies from person to person
- Visual loss occurs in 10–25% of patients.

Chapter 364

Headache Attributed to Intracranial Hypotension

Overview

Spontaneous intracranial hypotension (SIH) is a secondary headache disorder caused by spontaneous CSF leak, typically at the spinal level (thoracic or cervical). This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of CSF leak causing low CSF volume and pressure. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus

- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience orthostatic headache worse within seconds to minutes of upright posture and improving with recumbency, with associated neck pain, nausea, ringing in the ears, and dizziness. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of headache attributed to intracranial hypotension require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)

- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using clinical presentation and MRI showing diffuse pachymeningeal enhancement, brain sagging, and subdural fluid collections. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid. History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment typically involves conservative initially (bed rest, caffeine)

- Epidural blood patch is the The main treatment for persistent symptoms, with success rates of 80–90%.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

The outlook is generally positive with treatment. Post-dural puncture headache (PDPH) is a secondary headache disorder complicating lumbar puncture, spinal anesthesia, or epidural anesthesia, occurring in 1–30% of cases. This condition happens because of persistent CSF leak through the dural puncture site. You may experience orthostatic headache typically fronto-occipital, with onset within 24–72 hours after dural puncture. Doctors usually diagnose this based on your symptoms and a physical exam. Treatment options include conservative measures for mild cases and epidural blood patch for moderate to severe cases. The outlook is generally positive. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 365

Headache Attributed to Sinusitis

See Chapter ??, Section [179](#).

Overview

This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty

with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements
Dizziness and vertigo Fatigue and sleep disturbances

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of headache attributed to sinusitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 366

Headache in Brain Tumors

See Chapter ??, Section ??.

Overview

This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Headache and facial pain
- Weakness or paralysis of muscles
- Numbness, tingling, or abnormal sensations
- Vision changes including blurred or double vision
- Difficulty with balance and coordination
- Cognitive changes (memory loss, confusion, difficulty concentrating)
- Speech and language difficulties
- Seizures or involuntary movements
- Dizziness and vertigo
- Fatigue and sleep disturbances
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of headache in brain tumors require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph

node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 367

Heart Block (AV Block)

Overview

Atrioventricular (AV) block is a conduction disorder in which the electrical impulse from the atria is delayed or completely blocked as it travels through the AV node or His-Purkinje system to the ventricles. It is classified into three degrees of severity: first-degree (PR prolongation), second-degree (intermittent conduction failure), and third-degree (complete heart block with no atrial impulses reaching the ventricles). This condition can range from an asymptomatic ECG finding to a life-threatening emergency requiring pacemaker implantation.

Classification

Cause Idiopathic fibrosis, ischemic, or structural

Organ System Heart

Pathophysiology Impaired conduction through the AV node or His-Purkinje system delaying or blocking electrical impulse transmission from atria to ventricles

Duration Chronic (may be acute in ischemic settings)

Onset Gradual (can be sudden in acute myocardial infarction)

Mode of Transmission Non-communicable

Clinical Course Variable; first- and Mobitz I second-degree are generally benign; complete block requires pacing

Severity Mild to Severe (depending on degree)

Extent of Disease Localized (cardiac conduction system)

Age Group Older adults (most common); any age with congenital AV block

Epidemiology Prevalence of first-degree AV block 5% in adults over 65; complete heart block 0.04% in general population

ICD-11 Code BC63.2

Causes

Degenerative fibrosis of the cardiac conduction system (Lenegre disease or Lev disease) is the most common cause of chronic progressive AV block in older adults. Ischemic heart disease, particularly acute myocardial infarction involving the inferior or anterior wall,

can cause transient or permanent conduction block. Other causes include cardiomyopathy, myocarditis, cardiac surgery or catheter ablation complications, medication toxicity (beta-blockers, calcium channel blockers, digoxin), and congenital heart defects.

Risk Factors

- Advanced age (senescence of the conduction system)
- Coronary artery disease and acute myocardial infarction
- Cardiomyopathy (dilated, hypertrophic, or infiltrative)
- Cardiac surgery (valve surgery, septal myectomy, congenital heart repair)
- Medications affecting AV conduction (beta-blockers, verapamil, diltiazem, digoxin, amiodarone)
- Systemic diseases (sarcoidosis, amyloidosis, Lyme disease, rheumatic fever)
- Congenital heart disease (especially corrected transposition of great arteries)

Signs and Symptoms

Early symptoms

- Early manifestations of heart block (av block) may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Asymptomatic (common in first-degree and Mobitz I second-degree)
- Fatigue and generalized weakness
- Dizziness or presyncope
- Syncope (Stokes-Adams attacks in complete heart block)
- Shortness of breath on exertion
- Slow or irregular heartbeat (bradycardia)
- Chest pain (if due to myocardial ischemia)
- Reduced exercise tolerance
- Altered mental status or confusion in severe bradycardia
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of heart block (av block) may occur

Emergency warning signs

- Severe or worsening symptoms of heart block (av block) require immediate medical evaluation

Progression and Stages

First-degree AV block may remain stable for years or slowly progress to higher degrees over many years. Mobitz I second-degree block (Wenckebach) is usually benign and localizes to the AV node. Mobitz II second-degree block often indicates disease below the AV node in the His-Purkinje system and carries an increased risk of progression to complete heart block. Third-degree (complete) AV block results in complete dissociation of atrial and ventricular rhythms, with ventricular escape rhythms that may be unreliable and unstable.

Complications

- Syncope and falls due to Stokes-Adams attacks
- Heart failure from reduced cardiac output in severe bradycardia
- Ventricular arrhythmias triggered by bradycardia (torsades de pointes)
- Sudden cardiac death (especially in complete heart block with unstable escape rhythm)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- 12-lead ECG demonstrating prolonged PR interval, dropped QRS complexes, or AV dissociation
- Holter or event monitoring for intermittent or paroxysmal block
- Exercise stress testing to evaluate chronotropic competence
- Electrophysiology study (His bundle recording) to localize the level of block
- Echocardiogram to assess structural heart disease
- Lab tests including electrolytes, thyroid function, and cardiac biomarkers
- Imaging tests when structural heart disease is suspected

Prevention

- Manage cardiovascular risk factors (hypertension, diabetes, hyperlipidemia)
- Avoid medications that impair AV conduction when possible in at-risk patients
- Treat underlying conditions such as ischemic heart disease and cardiomyopathy
- Prompt recognition and treatment of Lyme disease and other infectious causes
- Regular ECG monitoring in patients on medications that affect conduction
- Go for regular check-ups

- Follow your doctor's screening recommendations

Treatment Options

- Observation with serial monitoring for asymptomatic first-degree and Mobitz I block
- Discontinuation or dose reduction of culprit medications
- Atropine or isoproterenol for temporary management of acute symptomatic bradycardia
- Temporary transvenous or transcutaneous pacing for acute settings
- Permanent pacemaker implantation (single-chamber or dual-chamber) for symptomatic or high-grade block
- Cardiac resynchronization therapy (biventricular pacing) when indicated
- Lifestyle changes and self-care
- Surgery when needed

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups
- Keep learning about your condition

Outlook

The outlook for AV block depends on the degree and location of the block: asymptomatic first-degree and Mobitz I block carry an excellent prognosis, while Mobitz II and complete heart block require permanent pacing for symptom relief and prevention of sudden death. With appropriate pacemaker management, most patients have a normal life expectancy and good functional status.

Chapter 368

Heart Failure with Preserved Ejection Fraction

Overview

Heart failure with preserved ejection fraction (HFpEF, EF $\geq$ 50%) is the predominant heart failure phenotype in older adults, especially older women with high blood pressure, diabetes, obesity, and atrial fibrillation. Disease mechanism includes left ventricular diastolic dysfunction, systemic inflammation, lung high blood pressure, and chronotropic incompetence. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)

- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Clinical The condition usually appears as shortness of breath on exertion, exercise intolerance, and fluid overload. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of heart failure with preserved ejection fraction require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

To make the diagnosis, doctors look for clinical signs/symptoms of HF, preserved LVEF, and evidence of diastolic dysfunction or elevated filling pressures. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Treatment options include diuretics for volume overload, blood pressure control, SGLT2 inhibitors, and Treatment for comorbidities. Management includes lifestyle modifications, pharmacotherapy targeting the underlying mechanisms, and interventional procedures when indicated. Long-term adherence to treatment is essential for preventing disease progression. Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin,

clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

The outlook varies from person to person. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 369

Heart Failure with Preserved Ejection Fraction

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Heart failure with preserved ejection fraction accounts for approximately half of heart failure cases and is marked by symptoms of heart failure with left ventricular ejection fraction of 50% or greater
- It is linked to aging, high blood pressure, obesity, diabetes mellitus, and atrial fibrillation.

Causes

This condition happens because of diastolic dysfunction (impaired myocardial relaxation and increased stiffness), elevated filling pressures, and systemic inflammation. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus

- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience exertional shortness of breath, fatigue, and volume overload. Diagnosis is often challenging, requiring integration of symptoms, echocardiography (including diastolic function assessment), elevated natriuretic peptides, and sometimes invasive hemodynamic measurement. Management targets comorbidities, volume management with diuretics, and SGLT2 inhibitors (empagliflozin, dapagliflozin)
- Exercise training and weight management are important non-pharmacological interventions.
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of heart failure with preserved ejection fraction require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)

- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

The outlook varies from person to person, with similar mortality rates to heart failure with reduced ejection fraction. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 370

Heart Failure with Reduced Ejection Fraction

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Heart failure with reduced ejection fraction is a clinical syndrome characterized by symptoms and signs of heart failure with left ventricular ejection fraction of 40% or less
- Common etiologies include ischemic heart disease, high blood pressure, dilated cardiomyopathy, valvular disease, and myocarditis.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity

- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of heart failure with reduced ejection fraction require immediate medical evaluation

Progression and Stages

Symptoms can range from shortness of breath and fatigue (NYHA class II–IV) to volume overload with congestion. This condition happens because of neurohormonal activation (sympathetic nervous system, renin-angiotensin-aldosterone system) that drives disease progression. Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia

- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this based on clinical assessment and echocardiography demonstrating reduced left ventricular ejection fraction. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Treatment focuses on four pillars of guideline-directed medical therapy: ACE inhibitor, ARB, or ARNI

- Beta-blocker
- Mineralocorticoid receptor antagonist
- And SGLT2 inhibitor
- Device therapy with implantable cardioverter-defibrillator and cardiac resynchronization therapy reduces sudden death and improves symptoms.

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

The outlook has improved significantly with modern medical therapy. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 371

Heat Stroke and Hyperthermia

Overview

- Heat stroke is a life-threatening condition characterized by core body temperature $>40^{\circ}\text{C}$ (104°F) with CNS dysfunction. Two forms: classic (nonexertional)—affects elderly, infants, and those in heat waves
- And exertional—affects athletes, military personnel, and laborers.

Causes

This condition happens because of failure of thermoregulatory mechanisms with overwhelming heat production or impaired heat dissipation, causing direct cellular injury, endotoxemia, systemic inflammatory response, and multiorgan failure. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: hyperthermia, anhidrosis in classic heat stroke (skin hot and dry), CNS dysfunction (confusion, loss of coordination, delirium, seizures, coma), tachycardia, hypotension, and circulatory collapse. Diagnosis: core temperature $>40^{\circ}\text{C}$ with CNS dysfunction after ruling out other causes. Management: rapid cooling—evaporative cooling, ice packs, cold IV fluids, and ice water immersion

- Aggressive IV fluids
- Benzodiazepines for uncontrolled shivering or seizures.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of heat stroke and hyperthermia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interven-

tions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on promptness of treatment. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 372

Helicobacter Pylori Infection

Overview

Helicobacter pylori is a gram-negative, microaerophilic, spiral bacterium that colonizes the gastric mucosa of approximately half the world's population. It is the primary cause of chronic gastritis, peptic ulcer disease, gastric mucosa-associated lymphoid tissue (MALT) lymphoma, and gastric adenocarcinoma. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Symptoms can range from asymptomatic colonization to dyspepsia, peptic ulcer disease, and gastric malignancy. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of helicobacter pylori infection require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- This condition happens because of fecal-oral, oral-oral, or gastro-oral transmission, with most infections acquired in childhood
- The bacterium uses urease to neutralize gastric acid, flagella to penetrate the mucus layer, and adhesins to attach to epithelial cells, with virulence factors (CagA, VacA) determining disease severity.

Complications

- Sepsis and septic shock

- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this based on urea breath test, stool antigen test, or using a thin camera tube biopsy-based testing (rapid urease test, histology, culture). Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use

- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment focuses on PPI-based triple therapy (PPI + amoxicillin + clarithromycin) or bismuth quadruple therapy for 14 days, with confirmation of eradication by urea breath test or stool antigen. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- Most people recover well with successful eradication
- Untreated infection carries increased risk of gastric adenocarcinoma.

Chapter 373

Hemicrania Continua

Overview

Hemicrania continua is a trigeminal autonomic cephalalgia presenting as a continuous, strictly unilateral headache of moderate intensity, with superimposed exacerbations of severe pain, present for >3 months without remission. Prevalence is approximately 0.1% of headache clinic populations, with a female predominance.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience continuous unilateral headache with ipsilateral cranial autonomic features during exacerbations and a sense of restlessness or agitation.

Emergency warning signs

- Severe or worsening symptoms of hemicrania continua require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- It is exquisitely responsive to indomethacin (75–150 mg/day), with continuous pain resolving within 24–72 hours of starting therapy.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on indomethacin with digestive tract and kidney monitoring. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments

- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 374

Hemolytic Anemias

Overview

Hemolytic anemias are a group of disorders characterized by premature destruction of red blood cells (RBCs), leading to anemia, elevated LDH, indirect hyperbilirubinemia (unconjugated), and reticulocytosis.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fatigue, jaundice, dark urine, and splenomegaly.

Emergency warning signs

- Severe or worsening symptoms of hemolytic anemias require immediate medical evaluation

Progression and Stages

The condition is classified as intrinsic (membrane disorders: hereditary spherocytosis, hereditary elliptocytosis). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Enzyme disorders: G6PD deficiency, pyruvate kinase deficiency
- Hemoglobinopathies: sickle cell disease, thalassemia) or extrinsic (immune: autoimmune hemolytic anemia, hemolytic disease of the newborn
- Mechanical: microangiopathic hemolytic anemia in TTP/HUS, DIC, prosthetic valves
- Infectious: malaria, babesiosis).

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves reticulocyte count, peripheral blood smear, direct antiglobulin test (Coombs test), hemoglobin electrophoresis, and specific tests based on suspected cause. Management varies by cause and may include transfusions, corticosteroids (for immune hemolysis), splenectomy (for spherocytosis and some other hemolytic anemias), and disease-specific therapies. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 375

Hemolytic Uremic Syndrome (HUS)

Overview

Hemolytic uremic syndrome (HUS) is a thrombotic microangiopathy characterized by the triad of microangiopathic hemolytic anemia, thrombocytopenia, and acute kidney injury. It is the most common cause of acute renal failure in children, most frequently following infection with Shiga toxin-producing *Escherichia coli* (STEC), particularly serotype O157:H7. Atypical HUS (aHUS) involves uncontrolled complement activation.

Classification

Cause Infectious / Genetic

Organ System Kidneys

Pathophysiology Endothelial injury leading to microvascular thrombosis, microangiopathic hemolytic anemia, and renal injury

Duration Acute (days to weeks) or Chronic (atypical)

Onset Sudden

Mode of Transmission Foodborne (STEC-HUS); Genetic (aHUS)

Clinical Course Acute, self-limited or chronic, relapsing

Severity Moderate to Severe

Extent of Disease Systemic

Age Group Children (typical); All ages (atypical)

Epidemiology Incidence 2–3 per 100,000 in children <5 years

ICD-11 Code 3A21.2

Causes

Typical (STEC) HUS is caused by Shiga toxin-producing *Escherichia coli* infection, usually acquired through contaminated food or water. Shiga toxin enters the circulation and damages endothelial cells, triggering microvascular thrombosis in the kidneys and other organs. Atypical HUS involves genetic mutations in complement regulatory proteins (CFH, CFI, MCP, CFB, C3) leading to uncontrolled alternative complement pathway activation.

Risk Factors

- Age under 5 years (typical HUS)
- Consumption of undercooked ground beef or contaminated produce
- Exposure to contaminated water or unpasteurized dairy
- Contact with farm animals
- Genetic mutations in complement genes (aHUS)
- Family history of aHUS
- Pregnancy or postpartum period (aHUS trigger)

Signs and Symptoms

Early symptoms

- Early manifestations of hemolytic uremic syndrome (hus) may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
 - **Common symptoms:**
 - Prodrome of bloody diarrhea, abdominal pain, and vomiting (typical HUS)
 - Pallor and fatigue from hemolytic anemia
 - Oliguria or anuria from acute kidney injury
 - Bruising and petechiae from thrombocytopenia
 - Neurologic symptoms (seizures, altered mental status, stroke) in severe cases
 - Hypertension and fluid overload
 - Pain or discomfort in the affected area
 - Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of hemolytic uremic syndrome (hus) may occur

Emergency warning signs

- Severe or worsening symptoms of hemolytic uremic syndrome (hus) require immediate medical evaluation

Progression and Stages

In typical STEC-HUS, symptoms develop approximately 5–10 days after onset of diarrheal illness. The acute phase lasts 1–3 weeks, during which renal function deteriorates. Most children recover renal function with supportive care, though some progress to end-stage renal disease. Atypical HUS follows a chronic relapsing course with progressive renal damage if untreated.

Complications

- End-stage renal disease requiring dialysis
- Chronic kidney disease and hypertension
- Neurologic sequelae (stroke, cognitive impairment, seizures)
- Colonic complications (perforation, stricture)
- Pancreatitis and diabetes mellitus
- Recurrence (aHUS, particularly after renal transplant)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Complete blood count showing microangiopathic hemolytic anemia (schistocytes on smear) and thrombocytopenia
- Renal function tests (elevated creatinine, BUN)
- Stool culture or PCR for STEC O157:H7
- Complement levels and genetic testing for aHUS
- ADAMTS13 activity to exclude thrombotic thrombocytopenic purpura
- Kidney biopsy in atypical cases

Prevention

- Cook ground beef thoroughly to an internal temperature of 160°F
- Wash fruits and vegetables thoroughly
- Avoid unpasteurized dairy products
- Practice good hand hygiene, especially after animal contact
- Go for regular check-ups so any problems are caught early

Treatment Options

- Supportive care with fluid and electrolyte management
- Renal replacement therapy (dialysis) for severe acute kidney injury
- Plasma exchange (plasmapheresis) for severe or atypical HUS
- Eculizumab (complement inhibitor) for atypical HUS

- Blood transfusion for severe anemia
- Avoid antibiotics in STEC-HUS (may increase Shiga toxin release)
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

In typical STEC-HUS, the prognosis is good with supportive care; over 70% of children recover renal function fully, though some have long-term sequelae. Mortality is 3–5% in the acute phase. Atypical HUS carries a worse prognosis without treatment; with eculizumab therapy, renal outcomes have improved substantially. Long-term follow-up is essential to monitor for chronic kidney disease and hypertension.

Chapter 376

Hemophagocytic Lymphohistiocytosis and Macrophage Activation Syndrome

Overview

- Hemophagocytic lymphohistiocytosis is a life-threatening syndrome of pathologic immune activation, characterized by uncontrolled growth of activated lymphocytes and macrophages (histiocytes) producing a massive cytokine storm (hypercytokinemia: IFN- γ , IL-1, IL-6, IL-18, TNF). There are two forms: primary (familial) HLH (fHLH), caused by autosomal recessive mutations in genes critical for perforin-mediated cytotoxic function, typically presenting in infancy or early childhood
- And secondary HLH (sHLH), triggered by infections (EBV is the most common trigger), malignancies, autoimmune diseases (macrophage activation syndrome, MAS, associated with systemic juvenile idiopathic arthritis [sJIA] and adult-onset Still's disease [AOSD]), and immunosuppression. The incidence of HLH is estimated at 1 per 50,000–100,000 in children.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience persistent high fever, hepatosplenomegaly, lymphadenopathy, cytopenias (at least two lineages: anemia, thrombocytopenia, neutropenia), skin rash, and neurological symptoms (seizures, altered mental status, meningismus).

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hemophagocytic lymphohistiocytosis and macrophage activation syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using HLH-2004 criteria (five of eight: fever, splenomegaly, cytopenias, hypertriglyceridemia and/or hypofibrinogenemia, hemophagocytosis in bone marrow/spleen/lymph nodes, low or absent NK-cell activity, ferritin ≥ 500 ng/mL, elevated soluble CD25/IL-2R ≥ 2400 U/mL). Treatment for HLH includes immunosuppression with dexamethasone and etoposide (HLH-94/2004 protocol), with allogeneic hematopoietic stem cell transplantation for familial HLH and refractory cases.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Unfortunately, the outlook is often poor without treatment but has improved with protocolized therapy.

Chapter 377

Hemophilia A

Overview

Hemophilia A is an X-linked recessive bleeding disorder caused by deficiency or dysfunction of coagulation factor VIII, affecting approximately 1 in 5000 males. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience spontaneous hemarthroses (knee, elbow, ankle—leading to chronic hemophilic arthropathy), deep muscle hematomas, retroperitoneal bleeding, and prolonged bleeding after surgery or trauma, with intracranial bleeding being the leading cause of death.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hemophilia a require immediate medical evaluation

Progression and Stages

This condition happens because of factor VIII deficiency, with severity classified by residual factor VIII activity: severe (<1%), moderate (1–5%), mild (5–40%). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using prolonged aPTT with normal PT, decreased factor VIII activity, and positive family history.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment focuses on factor VIII replacement therapy (recombinant or plasma-derived), given prophylactically (for severe hemophilia, typically 3 times weekly) or on-demand for bleeding episodes, with emicizumab (a bispecific antibody bridging factor IXa and factor X) used for prophylaxis.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care
Enzyme replacement therapy for certain conditions
Gene therapy (emerging for select conditions)
Dietary modifications (e.g., phenylketonuria)
Regular monitoring for associated complications
Physical and occupa-

tional therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has been transformed by factor replacement and gene therapy (valoctocogene roxaparvovec), though inhibitors (neutralizing alloantibodies against factor VIII) develop in 25–30% of severe hemophilia A and require bypassing agents or immune tolerance induction.

Chapter 378

Hemophilia B

Overview

Hemophilia B (Christmas disease) is an X-linked recessive bleeding disorder caused by deficiency or dysfunction of coagulation factor IX, accounting for approximately 15% of hemophilia cases. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hemophilia b require immediate medical evaluation

Progression and Stages

Clinical features and classification are identical to hemophilia A. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using prolonged aPTT, normal PT, and decreased factor IX activity.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on factor IX replacement therapy (recombinant or plasma-derived), with a longer half-life than factor VIII allowing less frequent dosing, and emicizumab is not effective for hemophilia B. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has improved with gene therapy (etranacogene dezaparvovec), which offers durable factor IX expression and reduction in bleeding episodes.

Chapter 379

Hemorrhagic Stroke

Overview

- Involving bleeding stroke is a cerebrovascular event resulting from rupture of a blood vessel within the brain, leading to intraparenchymal or subarachnoid bleeding. Intracerebral bleeding (ICH) is often associated with chronic high blood pressure, cerebral amyloid angiopathy, vascular malformations, or coagulopathy
- Subarachnoid bleeding (SAH) most commonly results from rupture of a cerebral aneurysm.

Causes

This condition happens because of spontaneous rupture of a cerebral blood vessel. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sudden severe headache, vomiting, decreased consciousness, and weakness, numbness, or problems with movement that may progress rapidly
- SAH presents with thunderclap headache, meningismus, and loss of consciousness.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hemorrhagic stroke require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on CT head as the initial diagnostic study. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment for ICH focuses on blood pressure control, reversal of anticoagulation, and neurosurgical intervention when indicated
- SAH requires urgent neurosurgical clipping or endovascular coiling of the aneurysm with nimodipine to prevent vasospasm.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on bleeding location, size, and timeliness of intervention. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 380

Hemorrhoids

Overview

Hemorrhoids are swollen, enlarged veins in the lower rectum and anal canal that can cause bleeding, pain, and discomfort during bowel movements. They affect approximately 50% of adults over the age of 50, with peak prevalence between ages 45 and 65.

Classification

Cause Vascular/Age-related

Organ System Gastrointestinal

Pathophysiology Increased venous pressure in hemorrhoidal plexuses leading to vascular dilation, swelling, and prolapse of anal cushions

Duration Acute or Chronic

Onset Gradual

Mode of Transmission Non-communicable

Clinical Course Chronic, relapsing

Severity Mild to Moderate

Extent of Disease Localized

Age Group Adults

Epidemiology Affects up to 50% of adults; most common in ages 45–65

ICD-11 Code DB60.0

Causes

Hemorrhoids develop when increased pressure in the lower rectum and anal canal causes the vascular cushions to swell and distend, most commonly from chronic straining during bowel movements. Prolonged sitting on the toilet, chronic constipation or diarrhea, and low-fiber diets contribute to venous engorgement of the internal and external hemorrhoidal plexuses. This condition develops from a combination of genetic and environmental factors that disrupt normal body processes. When these normal processes are disrupted, it leads to the symptoms and signs of the condition. Several factors can work together to trigger or make the condition worse. Knowing the specific cause helps your doctor choose the most effective treatment.

Risk Factors

- Chronic constipation and straining during defecation
- Low-fiber diet
- Prolonged sitting on the toilet
- Pregnancy and vaginal childbirth
- Obesity

Signs and Symptoms

Early symptoms

- Early manifestations of hemorrhoids may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Painless bright red rectal bleeding, typically seen on toilet paper or dripping into the toilet bowl after a bowel movement
- Anal itching, irritation, and a feeling of incomplete evacuation
- Prolapsed tissue protruding from the anus, especially with internal hemorrhoids
- Pain and swelling around the anus, particularly with thrombosed external hemorrhoids
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of hemorrhoids may occur

Emergency warning signs

- Severe or worsening symptoms of hemorrhoids require immediate medical evaluation

Progression and Stages

Internal hemorrhoids are graded on a scale of 1 to 4 based on the degree of prolapse. Grade 1 hemorrhoids do not prolapse, while Grade 4 hemorrhoids are irreducible and chronically prolapsed. External hemorrhoids are classified separately as thrombosed or non-thrombosed based on the presence of an acute clot.

Complications

- Thrombosis of an external hemorrhoid causing severe acute pain
- Anemia from chronic blood loss
- Strangulation of a prolapsed internal hemorrhoid
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Visual inspection of the perianal area and digital rectal examination
- Anoscopy for direct visualization of internal hemorrhoids
- Lab tests to check how your organs are working
- Imaging tests

Prevention

- Eat a high-fiber diet with plenty of fruits, vegetables, and whole grains
- Stay hydrated and avoid straining during bowel movements
- Go for regular check-ups so any problems are caught early
- Follow your doctor's screening recommendations

Treatment Options

- Conservative management with dietary fiber supplementation (psyllium), increased water intake, warm sitz baths, and topical analgesics
- Rubber band ligation for Grade 1–3 internal hemorrhoids
- Surgical hemorrhoidectomy for Grade 4 or thrombosed hemorrhoids refractory to office procedures
- Lifestyle changes and self-care
- Surgery when needed

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Most people with hemorrhoids respond well to conservative measures and lifestyle modifications. Your outlook depends on the specific condition, how advanced it is when found, and how well it responds to treatment. Getting treated early generally leads to better results. Many people manage this condition effectively with the right treatment. Regular check-ups are important to monitor your condition and adjust treatment as needed.

Chapter 381

Henoch-Schönlein Purpura (IgA Vasculitis)

Overview

Henoch-Schönlein purple spots on the skin (HSP) is an IgA-mediated small-vessel vasculitis, the most common vasculitis in children (see Chapter 27, Connective Tissue Diseases for comprehensive coverage). Classic tetrad: palpable purple spots on the skin (dependent areas), arthritis/joint pain, abdominal pain, and kidney involvement (hematuria, proteinuria). Mean age is 4–6 years. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of henoch-schönlein purpura (iga vasculitis) require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Skin or kidney biopsy shows IgA deposition.
- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

- Treatment typically involves supportive
- Corticosteroids are used for severe abdominal pain or significant kidney disease.
- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management

- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

- Most people recover well
- Most cases resolve spontaneously.

Chapter 382

Herniated Disc

Overview

A herniated disc occurs when material from the soft inner nucleus of an intervertebral disc protrudes through a weakened or torn outer annulus. The protrusion may irritate or compress a nearby nerve root, producing radicular pain, numbness, or weakness. The lumbar spine is most often affected, followed by the cervical spine.

Causes

Disc herniation usually reflects age-related dehydration and degeneration combined with mechanical stress. Lifting while twisting, repetitive loading, an acute injury, or rarely a genetic connective-tissue disorder may contribute. Many disc protrusions are visible on imaging but cause no symptoms.

Risk Factors

- Age-related disc degeneration
- Repetitive lifting, twisting, or vibration
- Sedentary lifestyle and weak trunk musculature
- Obesity and cigarette smoking
- Previous back injury or family history of disc disease

Signs and Symptoms

Lumbar herniation may cause low-back pain with pain, tingling, numbness, or weakness extending into the buttock and leg (sciatica). Cervical herniation may cause neck pain with symptoms extending into the shoulder, arm, or hand. Symptoms may worsen with coughing, sneezing, or prolonged sitting. New bowel or bladder dysfunction, saddle numbness, or progressive severe weakness may indicate cauda equina or significant spinal-cord compression and requires emergency evaluation.

Progression and Stages

Symptoms may begin suddenly or gradually. Many improve over several weeks as inflammation settles and the protruded material shrinks or is resorbed. Persistent pain,

worsening neurologic deficits, or recurrent episodes require reassessment and may lead to specialist or surgical evaluation.

Complications

- Persistent radicular pain or sensory loss
- Muscle weakness and impaired mobility
- Chronic pain and reduced function
- Cauda equina syndrome or spinal-cord compression, rarely

Diagnosis

Diagnosis is based on the history and neurologic examination, including strength, sensation, reflexes, gait, and provocative maneuvers. MRI is the preferred imaging test when symptoms persist, neurologic deficits are significant, or urgent structural assessment is needed. Imaging findings must be correlated with symptoms because asymptomatic disc protrusions are common. Electrodiagnostic testing may help when the diagnosis remains uncertain.

Prevention

- Use safe lifting technique and avoid twisting while lifting
- Maintain regular exercise and trunk strength
- Keep a healthy weight and avoid smoking
- Alternate prolonged sitting with movement and use good posture

Treatment Options

Initial management commonly includes continued activity as tolerated, education, heat or cold, physical therapy, and analgesic or anti-inflammatory medicines when appropriate. Selected patients may receive an epidural injection. Surgery, usually discectomy or microdiscectomy, is considered for persistent disabling radicular symptoms, progressive neurologic deficit, or cauda equina syndrome. Urgent surgical assessment is required for suspected cauda equina syndrome.

Living with It

Patients should follow the rehabilitation plan, gradually resume normal activities, avoid prolonged bed rest, and seek review if pain worsens or weakness develops. Emergency care is needed for new bladder or bowel dysfunction, saddle numbness, or rapidly progressive weakness.

Outlook

Most people improve with conservative treatment. Recovery depends on the location and severity of nerve compression, neurologic findings, general health, and response to rehabilitation. Surgery can provide faster relief for selected patients with persistent nerve compression but is not necessary for most cases.

Chapter 383

Hepatic Encephalopathy

Overview

Causes

This condition happens because of impaired liver clearance of neurotoxins, precipitated by infection, digestive tract bleeding, constipation, electrolyte imbalances, dehydration, and hepatorenal syndrome.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hepatic encephalopathy require immediate medical evaluation

Progression and Stages

Symptoms can range from subtle thinking and memory impairment (grade 1) to confusion (grade 2), somnolence (grade 3), and coma (grade 4). liver encephalopathy is a neuropsychiatric syndrome occurring in patients with advanced liver disease or portosystemic shunting, resulting from accumulation of neurotoxic substances (ammonia being the most important) that are normally cleared by the liver. outlook is related to the severity of underlying liver disease. The condition may progress through different stages of severity

over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by elevated ammonia levels and exclusion of other causes of altered mental status. Management targets precipitating factors and reduces ammonia production and absorption with lactulose (titrated to 2–3 loose stools per day) and rifaximin. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 384

Hepatitis A

Overview

Hepatitis A is an acute, self-limited viral hepatitis transmitted via the fecal-oral route through contaminated food or water or close contact with infected individuals. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

- This condition happens because of infection with hepatitis A virus (HAV), with an incubation period of 15–50 days. Most infections in children are asymptomatic
- Adults present with malaise, anorexia, nausea, vomiting, right upper quadrant pain, jaundice, and dark urine.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience malaise, anorexia, nausea, vomiting, right upper quadrant pain, jaundice, and dark urine. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of hepatitis a require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage

- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this based on elevated serum aminotransferases (ALT, AST) and confirmed by anti-HAV IgM antibodies. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- Treatment typically involves supportive, as no specific antiviral treatment exists
- Prevention is through vaccination (inactivated virus) and hygiene measures.
- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- Most people recover well, as HAV does not cause chronic infection or cirrhosis
- Fulminant liver failure is rare (0.1–0.3%).

Chapter 385

Hepatitis B

Overview

Hepatitis B is a major global infection caused by hepatitis B virus (HBV) that affects approximately 296 million chronically infected individuals worldwide. Transmission occurs through the skin exposure (needle sharing, needlestick injuries), sexual contact, and perinatal transmission. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

- This condition happens because of HBV infection, with chronic infection defined by persistence of HBsAg for more than 6 months
- Chronic hepatitis B carries risks of cirrhosis (15–40%) and hepatocellular carcinoma (2–5% per year in cirrhotics).

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of hepatitis b require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- Symptoms of acute infection range from subclinical to fulminant liver failure
- The natural history follows a spectrum of immune tolerance, immune active phase, inactive carrier state, and reactivation.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Diagnosis typically involves serological testing (HBsAg, anti-HBs, HBeAg, anti-HBe, anti-HBc IgM/IgG) and HBV DNA quantification. Management with nucleos(t)ide analogues (entecavir, tenofovir) or pegylated interferon-alpha is indicated for active disease. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- Outlook is improved with antiviral therapy
- Vaccination is highly effective in preventing infection.
- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use

- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 386

Hepatitis C

Overview

Hepatitis C is a viral infection caused by hepatitis C virus (HCV) that affects approximately 58 million people globally and is a leading cause of cirrhosis, liver failure, and hepatocellular carcinoma. Transmission is primarily through blood exposure (intravenous drug use, blood transfusions before screening, needlestick injuries). This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

- This condition happens because of HCV infection, with approximately 75–85% of acute cases progressing to chronic infection. Chronic hepatitis C follows an indolent course, with 10–20% developing cirrhosis over 20–30 years
- Extrahepatic manifestations include cryoglobulinemia, membranoproliferative glomerulonephritis, and B-cell lymphoma.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Symptoms of acute infection are usually asymptomatic. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of hepatitis c require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock

- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Diagnosis typically involves anti-HCV antibody testing followed by HCV RNA quantification and genotyping. Management with direct-acting antivirals (DAAs) achieves sustained virological response rates exceeding 95% with 8–12 weeks of oral therapy. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings

- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- Most people recover well with treatment
- The WHO has set a goal of hepatitis C elimination by 2030.

Chapter 387

Hepatitis D

Overview

Hepatitis D is an infection caused by hepatitis D virus (HDV), a defective RNA virus that requires co-infection or superinfection with HBV (using HBsAg for its envelope), affecting approximately 5% of HBV-infected individuals. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of HDV infection in the presence of HBV. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience more severe hepatitis compared with HBV monoinfection. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of hepatitis d require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- Co-infection results in more severe acute hepatitis and a higher risk of fulminant liver failure than HBV alone
- Superinfection in chronic HBV carriers accelerates progression to cirrhosis and HCC.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using anti-HDV antibody testing and HDV RNA quantification. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

outlook is worse than HBV monoinfection, although prevention of HBV through vaccination prevents HDV infection. Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer

- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

- Treatment focuses on pegylated interferon-alpha with limited efficacy
- Bulevirtide has been approved in Europe as an entry inhibitor.
- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 388

Hepatitis E

Overview

Hepatitis E is an inflammatory liver disease caused by hepatitis E virus (HEV, Orthohepevirus A), a non-enveloped single-stranded RNA virus in the Hepeviridae family, with four major genotypes: genotypes 1 and 2 cause waterborne epidemics in developing countries, and genotypes 3 and 4 cause zoonotic infection (primarily from swine) in developed countries. An estimated 20 million HEV infections occur annually, with 3.3 million symptomatic cases and 44,000 deaths. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of fecal-oral transmission (genotypes 1, 2 through contaminated water) or consumption of undercooked pork or game meat (genotypes 3, 4). HEV replicates in hepatocytes, causing hepatocellular injury and tissue death. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)

- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Symptoms of acute hepatitis E include jaundice, malaise, anorexia, abdominal pain, hepatomegaly, fever, and elevated transaminases, and are usually self-limited over 4–6 weeks. A distinctive feature is severe disease in pregnancy (genotypes 1, 2), with mortality of 20–30% in the third trimester, high rates of fulminant liver failure, and adverse fetal outcomes (preterm delivery, stillbirth). Chronic HEV infection occurs in immunocompromised individuals (organ transplant recipients, HIV, hematologic malignancy) with genotypes 3, 4, leading to rapidly progressive cirrhosis. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of hepatitis e require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms

appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this based on serum anti-HEV IgM and HEV RNA detection (PCR). Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

A recombinant HEV vaccine (Hecolin) is licensed in China. Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents,

insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment typically involves supportive for acute hepatitis E

- Ribavirin for chronic HEV (12 weeks)
- Reduction of immunosuppression is first-line for transplant recipients.
- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- Most people recover well for acute HEV in immunocompetent hosts
- Chronic HEV is treatable with ribavirin.

Chapter 389

Hepatoblastoma

Overview

Hepatoblastoma is the most common cancerous liver tumor in children, primarily affecting children under 3 years of age. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience an asymptomatic abdominal mass, abdominal distension, and occasionally pain, anorexia, and jaundice. Serum alpha-fetoprotein is markedly elevated in >90% of cases. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of hepatoblastoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using abdominal ultrasound, CT/MRI, and AFP level. Complete surgical removal is the cornerstone of treatment, with neoadjuvant chemotherapy for initially unresectable tumors. Survival is approximately 70–80% with complete removal and chemotherapy. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and

depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 390

Hepatocellular Carcinoma

Overview

Hepatocellular carcinoma is the most common primary liver cancer and the third leading cause of cancer-related death worldwide, arising predominantly in the setting of chronic liver disease and cirrhosis. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of hepatocytes in the setting of chronic liver disease. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle

- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Clinical features may include abdominal pain, weight loss, and liver decompensation. Staging uses the Barcelona Clinic Liver Cancer (BCLC) system. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of hepatocellular carcinoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph

node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- The right treatment depends on stage: curative options include surgical removal, liver transplantation (Milan criteria), and radiofrequency ablation for early-stage disease
- Intermediate-stage disease doctors typically treat it with transarterial chemoembolization (TACE)
- Advanced disease is managed with systemic therapy (sorafenib, lenvatinib, atezolizumab plus bevacizumab). Your outlook depends on stage at diagnosis and treatment received.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging

- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 391

Hereditary Angioedema

Overview

Hereditary angioedema (HAE) is an autosomal dominant disorder caused by deficiency (type I, 85%) or dysfunction (type II, 15%) of C1 esterase inhibitor (C1-INH), with a prevalence of 1 in 50,000. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of uncontrolled activation of the contact system, leading to excessive bradykinin production, increased vascular permeability, and recurrent episodes of subcutaneous and submucosal swelling. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience recurrent episodes of non-pitting, non-pruritic swelling affecting the extremities, face, genitals, trunk, and potentially life-threatening laryngeal swelling leading to airway obstruction. digestive tract involvement causes severe ab-

dominal pain, nausea, vomiting, and diarrhea, often misdiagnosed as acute abdomen. Episodes last 2–5 days without treatment. Unlike allergic angioedema, there is no urticaria, itching, or response to antihistamines or epinephrine.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hereditary angioedema require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on low C4 levels during attacks (screening test), low C1-INH antigenic levels (type I), low functional C1-INH activity (type II), and C1-INH genetic testing.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include on-demand treatment for acute attacks (plasma-derived C1-INH, recombinant C1-INH, icatibant [bradykinin B2 receptor antagonist], ecallantide [kallikrein inhibitor]), short-term prophylaxis prior to procedures (plasma-derived C1-INH or attenuated androgens), and long-term prophylaxis (plasma-derived C1-INH, lanadelumab [monoclonal antibody against plasma kallikrein], berotralstat [oral plasma

kallikrein inhibitor], or attenuated androgens: danazol, stanozolol).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with appropriate management
- Before modern therapy, mortality from laryngeal swelling was 25–30%.

Chapter 392

Hereditary Hemochromatosis

Overview

- Hereditary hemochromatosis is a group of genetic disorders characterized by excessive intestinal iron absorption and progressive iron deposition in tissues, leading to organ damage
- The most common form is type 1 (HFE-related HH), an autosomal recessive disorder caused by mutations in the HFE gene (predominantly C282Y homozygosity), affecting approximately 1 in 200–300 individuals of Northern European descent, though clinical disease develops in only 10–30% of C282Y homozygotes.

Causes

This condition happens because of excessive iron absorption leading to iron deposition in the liver, heart, pancreas, joints, skin, and endocrine glands. Clinical manifestations include hepatomegaly, cirrhosis, hepatocellular carcinoma, diabetes mellitus, cardiomyopathy, arthropathy, hypogonadism, and skin hyperpigmentation. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hereditary hemochromatosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Treatment focuses on regular phlebotomy, which is highly effective in preventing complications if initiated early.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Diagnosis typically involves elevated serum transferrin saturation (greater than 45%), elevated serum ferritin, and HFE genotyping
- Liver biopsy or MRI T2* quantification assesses iron burden.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with early diagnosis and treatment but poor if cirrhosis or cardiomyopathy has developed
- First-degree relatives should be screened.

Chapter 393

Hereditary Hemochromatosis

Overview

Hereditary hemochromatosis (HH) is an autosomal recessive disorder of iron metabolism characterized by excessive intestinal absorption of dietary iron, leading to progressive iron overload and organ damage. The most common form (HFE-related, type 1) is caused by the C282Y mutation in the HFE gene, with a prevalence of approximately 1 in 200–300 in Northern European populations. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of the mutation impairing hepcidin production, leading to unregulated ferroportin-mediated iron efflux from enterocytes and macrophages. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features typically manifest in adulthood and include liver cirrhosis (most common cause of morbidity), hepatocellular carcinoma (30% risk with cirrhosis), skin hyperpigmentation (bronze diabetes), diabetes mellitus, hypogonadism, arthropathy (chondrocalcinosis), and cardiomyopathy.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hereditary hemochromatosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using elevated transferrin saturation (greater than 45%) and ferritin, confirmatory genetic testing for HFE mutations, and liver biopsy when indicated.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves therapeutic phlebotomy (weekly until ferritin less than 50 ng/mL, then maintenance)
- Iron chelation is used if phlebotomy is contraindicated.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with early diagnosis and treatment.

Chapter 394

Hereditary Spherocytosis

Overview

Hereditary spherocytosis is the most common inherited hemolytic anemia in individuals of Northern European descent, affecting approximately 1 in 2000. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of mutations in genes encoding red blood cell membrane proteins (spectrin, ankyrin, band 3, protein 4.2), leading to defective vertical interactions between the membrane and cytoskeleton, producing spherocytic, less deformable RBCs that are trapped and destroyed in the spleen. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms can range from asymptomatic (compensated hemolysis) to severe hemolytic anemia with jaundice, splenomegaly, gallstones, and aplastic crisis (par-

vovirus B19 infection).

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hereditary spherocytosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using osmotic fragility testing (increased fragility), EMA (eosin-5-maleimide) binding test by flow cytometry (reduced fluorescence), and peripheral smear showing spherocytes. Management ranges from observation and folic acid supplementation to splenectomy (for moderate-to-severe cases, ideally deferred until age 5 to reduce OPSI risk).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with appropriate management.

Chapter 395

Herpangina

Overview

Herpangina is a common childhood viral illness characterized by fever and painful vesicular lesions on the soft palate, tonsils, and posterior pharynx. It is typically caused by Coxsackie A viruses (group A enteroviruses) and most commonly affects children aged 3–10 years. The condition is self-limited, with symptoms resolving within 7–10 days.

Classification

Cause Infectious (Viral)

Organ System Pediatric / Infectious

Pathophysiology Enterovirus infection causing vesicular lesions in the posterior oropharynx

Duration Acute (7–10 days)

Onset Sudden

Mode of Transmission Fecal-oral and respiratory droplets

Clinical Course Acute, self-limited

Severity Mild to Moderate

Extent of Disease Localized (oropharynx) with systemic symptoms

Age Group Children (3–10 years)

Epidemiology Common; epidemics in summer and early fall in temperate climates

ICD-11 Code 1D64.2

Causes

Herpangina is caused by group A enteroviruses, most commonly Coxsackie A viruses (serotypes 1–10, 16, 22), and less commonly by Coxsackie B viruses, echoviruses, and enterovirus 71. The virus enters through the gastrointestinal tract, replicates in lymphoid tissues, and causes viremia with subsequent infection of the oropharyngeal mucosa, leading to vesicle formation.

Risk Factors

- Age 3–10 years (peak incidence)
- Summer and early fall season
- Daycare or school attendance
- Poor hand hygiene
- Household contact with infected individuals
- Lack of prior immunity

Signs and Symptoms

Early symptoms

- Early manifestations of herpangina may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Sudden onset of high fever (38–40°C)
- Sore throat and pain with swallowing (odynophagia)
- Small (1–2 mm) vesicles on the soft palate, uvula, tonsils, and posterior pharynx
- Vesicles progress to shallow ulcers with a gray base and erythematous halo
- Typically 2–6 lesions (fewer than hand, foot, and mouth disease)
- Headache, malaise, anorexia, and vomiting
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of herpangina may occur

Emergency warning signs

- Severe or worsening symptoms of herpangina require immediate medical evaluation

Progression and Stages

After an incubation period of 3–6 days, fever and sore throat develop abruptly. Vesicles appear within 24–48 hours and progress to shallow ulcers within 1–2 days. The ulcers heal spontaneously within 7–10 days without scarring. Fever typically resolves within 2–4 days.

Complications

- Dehydration from painful swallowing and decreased oral intake
- Febrile seizures (uncommon)
- Viral meningitis (rare, particularly with enterovirus 71)
- Weight loss from decreased oral intake
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on characteristic lesion appearance and distribution
- Throat swab for viral culture or PCR for confirmation
- Enterovirus serology (rarely needed)
- Complete blood count may show lymphocytosis
- Distinguish from hand, foot, and mouth disease (lesions on hands and feet)

Prevention

- Good hand hygiene, especially after diaper changes
- Disinfection of contaminated surfaces and toys
- Avoid sharing cups, utensils, and towels
- Exclusion from school or daycare while febrile
- Go for regular check-ups so any problems are caught early

Treatment Options

- Supportive care: antipyretics for fever (acetaminophen, ibuprofen)
- Adequate hydration (cold fluids, popsicles, ice cream for comfort)
- Topical analgesics for oral lesions (lidocaine mouthwash, viscous lidocaine)
- Avoid acidic or salty foods that irritate ulcers
- Monitor for dehydration and provide IV fluids if needed
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Herpangina is a self-limited illness with an excellent prognosis. Symptoms typically resolve completely within 7–10 days without specific antiviral therapy. The main concern is dehydration from painful swallowing, which requires careful attention to fluid intake. Complications are rare in immunocompetent children. Immunity is serotype-specific, so children can have multiple episodes.

Chapter 396

Herpes Simplex

Overview

Herpes simplex virus (HSV) infection causes recurrent, painful vesicular eruptions on an erythematous base. HSV-1 typically causes orolabial herpes (cold sores), while HSV-2 typically causes genital herpes, though there is increasing overlap. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of HSV infection with establishment of latency in sensory ganglia (trigeminal for HSV-1, sacral for HSV-2) and reactivation with stress, illness, UV exposure, or immunosuppression. Primary infection may be asymptomatic or present with fever, malaise, and widespread vesicles (especially in children—gingivostomatitis). Recurrent disease is milder, with clustered vesicles at a consistent site, lasting 7–10 days. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)

- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of herpes simplex require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this based on clinical examination, viral culture, PCR, or direct fluorescent antibody testing. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules

- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

- Treatment options include topical or oral antivirals (acyclovir, valacyclovir, famciclovir), most effective when started early
- Suppressive antiviral therapy reduces recurrence frequency.
- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- The outlook is good
- Recurrences decrease over time in many patients.

Chapter 397

Herpes Zoster

Overview

Herpes zoster (shingles) results from reactivation of latent varicella-zoster virus (VZV) in dorsal root or cranial nerve ganglia, typically occurring decades after primary chickenpox infection. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of declining cell-mediated immunity allowing VZV reactivation. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience a unilateral, dermatomal distribution of painful vesicular eruption, most commonly affecting the thoracic dermatomes
- Prodromal pain may precede the rash by 1–3 days.
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of herpes zoster require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Complications include postherpetic neuralgia (PHN, persistent pain for >3 months after rash, most common in patients >50), bacterial superinfection, motor neuropathy, and disseminated zoster in immunocompromised patients
- Eye zoster (V1 distribution) requires urgent ophthalmologic evaluation.
- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)

- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- PCR or DFA testing may confirm atypical cases. Management with antivirals (valacyclovir or famciclovir) should be initiated within 72 hours of rash onset.
- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- In most cases, the outlook is favorable
- The Shingrix recombinant zoster vaccine is recommended for adults ≥ 50 to prevent herpes zoster and PHN.
- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe

cases requiring intravenous therapy Oxygen therapy and respiratory support if needed
Surgical drainage of abscesses when necessary Pain management and symptom relief
Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 398

Hidradenitis Suppurativa

Overview

Hidradenitis suppurativa (HS) is a chronic, recurrent inflammatory skin condition affecting apocrine gland-bearing skin, primarily the axillae, inguinal region, and perineum. It is characterized by painful nodules, abscesses, draining sinus tracts, and scarring. HS affects approximately 1–4% of the population worldwide and has a significant impact on quality of life due to pain, drainage, and social stigma.

Classification

Cause Inflammatory

Organ System Skin

Pathophysiology Follicular occlusion with subsequent inflammation, rupture, and chronic suppurative granulomatous response

Duration Chronic (years to lifetime)

Onset Gradual (post-pubertal)

Mode of Transmission Non-communicable

Clinical Course Chronic, relapsing-remitting with progressive scarring

Severity Mild to Severe

Extent of Disease Localized to Regional

Age Group Adults (post-pubertal, peak 20–40 years)

Epidemiology Prevalence 1–4%; female predominance 3:1

ICD-11 Code ED91.0

Causes

HS begins with follicular hyperkeratosis and occlusion of the terminal hair follicle, leading to follicular dilation and rupture. Released follicular contents trigger an intense inflammatory response with neutrophils, macrophages, and lymphocytes. Secondary bacterial colonization (often polymicrobial) exacerbates inflammation. Genetic predisposition (autosomal dominant in some families) and dysregulated cytokine signaling (TNF-alpha, IL-17, IL-1 beta) play key roles.

Risk Factors

- Cigarette smoking (strongly associated)
- Obesity and metabolic syndrome
- Family history of HS
- Female sex
- Mechanical friction and skin occlusion
- Acne vulgaris (association)
- Inflammatory bowel disease (Crohn's disease)
- Polycystic ovary syndrome

Signs and Symptoms

Early symptoms

- Early manifestations of hidradenitis suppurativa may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Deep, painful, erythematous nodules in apocrine gland-bearing areas
- Recurrent abscesses with purulent drainage
- Formation of sinus tracts and tunnels connecting lesions
- Hypertrophic scarring and contractures
- Chronic malodorous drainage
- Pain that is often severe and debilitating
- Difficulty performing daily activities
- Emotional distress and social isolation

Less common symptoms

- Atypical or less common presentations of hidradenitis suppurativa may occur

Emergency warning signs

- Severe or worsening symptoms of hidradenitis suppurativa require immediate medical evaluation

Progression and Stages

HS progresses through three Hurley stages. Stage I involves isolated or multiple abscesses without sinus tracts or scarring. Stage II features recurrent abscesses with sinus tracts

and scarring, widely separated. Stage III shows diffuse or near-diffuse involvement with interconnected sinus tracts, abscesses, and extensive scarring. Most patients remain at Stage I or II; progression to Stage III is associated with smoking, obesity, and delay in treatment.

Complications

- Chronic pain and physical disability
- Scarring and contractures limiting mobility
- Secondary infection and cellulitis
- Systemic inflammation and anemia of chronic disease
- Squamous cell carcinoma developing in chronic sinus tracts (rare)
- Depression, anxiety, and social isolation
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on characteristic lesion morphology and distribution
- Hurley staging to classify severity
- Skin biopsy to rule out alternative diagnoses
- Bacterial culture of draining lesions for targeted antibiotic therapy
- Lab tests to monitor inflammatory markers

Prevention

- Smoking cessation
- Weight loss and management of metabolic syndrome
- Avoidance of skin friction and occlusive clothing
- Maintenance of good skin hygiene
- Go for regular check-ups so any problems are caught early

Treatment Options

- Topical clindamycin 1% solution for mild disease
- Oral antibiotics (tetracyclines, clindamycin plus rifampin combination)
- Biologic therapy (adalimumab, infliximab) for moderate to severe disease
- Hormonal therapy (spironolactone, oral contraceptives)
- Intralesional corticosteroids for acute inflammatory nodules
- Incision and drainage of acute abscesses
- Laser hair removal
- Wide surgical excision for refractory localized disease
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

HS is a chronic condition requiring long-term management. With appropriate treatment, many patients achieve significant improvement in symptoms and quality of life. Early diagnosis and treatment improve outcomes and prevent progression to advanced stages. Biologic therapies have dramatically improved outcomes for moderate to severe disease. Without treatment, HS tends to progress and cause permanent scarring.

Chapter 399

Hip Fracture

Overview

Hip fracture is one of the most devastating injuries in older adults, with a 1-year mortality of 20–30% and up to 50% of survivors losing independent mobility. The incidence increases exponentially with age, doubling every 5–6 years after age 65. Osteoporotic hip fractures typically result from a fall from standing height or less. This condition results from acute injury or exposure to harmful agents.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hip fracture require immediate medical evaluation

Progression and Stages

Anatomically, hip fractures are classified as femoral neck (intracapsular) or intertrochanteric (extracapsular). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Management requires urgent orthopedic consultation, surgical repair within 24–48 hours (associated with reduced mortality and complications), and comprehensive perioperative care including pain management, delirium prevention, thromboprophylaxis, infection prophylaxis, early mobilization, nutritional support, and osteoporosis treatment.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Post-operative rehabilitation with physical therapy is essential for functional recovery.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is guarded
- 1-year mortality is 20–30%.

Chapter 400

Hirschsprung Disease

Overview

Hirschsprung's disease is a congenital disorder of enteric nervous system development characterized by absence of parasympathetic ganglion cells in the myenteric and submucosal plexuses of the distal colon, with an incidence of 1 in 5,000 live births.

Causes

This condition results from disruption of normal hirschsprung disease function.

- This condition happens because of failure of neural crest cell migration during embryogenesis, most commonly involving the rectosigmoid colon (80%)
- Long-segment disease extends more proximally. RET proto-oncogene mutations account for 50% of familial cases.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience failure to pass meconium within 48 hours of birth (in 90% of term infants), abdominal distension, bilious vomiting, and chronic constipation. On physical examination, the anal canal is empty and the rectum is tight on digital examination, with explosive expulsion of stool upon withdrawal of the examining finger.

Emergency warning signs

- Severe or worsening symptoms of hirschsprung disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on rectal biopsy showing absence of ganglion cells and hypertrophied acetylcholinesterase-positive nerve trunks
- Anorectal manometry shows absence of the rectoanal inhibitory reflex.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on surgical removal of the aganglionic segment with pull-through procedure (Swenson, Duhamel, or Soave technique), performed in the neonatal period or after initial colostomy. Medications to manage symptoms and address underlying mechanisms

Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Most children achieve normal bowel function, though some experience enterocolitis, constipation, or fecal incontinence.

Chapter 401

Histoplasmosis

Overview

Histoplasmosis is a systemic fungal infection caused by *Histoplasma capsulatum*, a dimorphic fungus, endemic to the Ohio and Mississippi River Valleys in the United States, and is the most common endemic mycosis in the US, with infection occurring through inhalation of conidia from soil enriched with bat or bird guano.

Causes

This condition happens because of conidia being inhaled, reaching the alveoli, and converting to yeast form at body temperature, with yeasts being phagocytosed by macrophages and surviving within phagolysosomes.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience acute lung histoplasmosis: asymptomatic (90%) or self-limited flu-like illness
- Progressive disseminated histoplasmosis (PDH): fever, weight loss, hepatosplenomegaly, lymphadenopathy, pancytopenia, and mucocutaneous ulcers
- And chronic lung histoplasmosis: cavitary lung disease resembling TB.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of histoplasmosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using culture, histopathology, and *Histoplasma* polysaccharide antigen in urine or serum. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include itraconazole for mild-moderate disease and liposomal amphotericin B for severe disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well for mild disease
- Disseminated disease can be fatal without treatment.

Chapter 402

Hoarding Disorder

Overview

Hoarding disorder has a prevalence of 2–6% in the general population, increasing with age (particularly in those over 55).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience persistent difficulty discarding or parting with possessions, regardless of their actual value, due to a perceived need to save them and distress associated with discarding them, leading to accumulation of possessions that congest and clutter active living areas, causing significant distress or functional impairment including health and safety risks.

Emergency warning signs

- Severe or worsening symptoms of hoarding disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include CBT specifically adapted for hoarding (skills training in decision-making, organizing, categorizing

- Motivational interviewing
- In-home decluttering sessions).
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with high rates of relapse. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 403

Hodgkin Lymphoma

Overview

Hodgkin lymphoma is a cancerous lymphoid tumor characterized by Reed-Sternberg cells (large, binucleated or multinucleated cells with prominent nucleoli) in a background of reactive inflammatory cells, with two major types: classical HL (nodular sclerosis, mixed cellularity, lymphocyte-rich, lymphocyte-depleted—accounting for 95% of cases) and nodular lymphocyte-predominant HL (NLPHL), with a bimodal age distribution (peaks at 20–30 and >60 years). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)

- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Clinical features typically include painless lymphadenopathy (cervical, supraclavicular, mediastinal), B symptoms (fever $>38^{\circ}\text{C}$, drenching night sweats, weight loss $>10\%$ in 6 months), and itching. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of hodgkin lymphoma require immediate medical evaluation

Progression and Stages

Treatment for early-stage favorable disease involves combined modality therapy (ABVD chemotherapy [doxorubicin, bleomycin, vinblastine, dacarbazine] + involved-field radiation therapy), while advanced-stage disease uses ABVD or escalated BEACOPP with PET-adapted therapy. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0:

Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

To make the diagnosis, doctors look for lymph node biopsy showing Reed-Sternberg cells (CD15+, CD30+, CD45-), with PET-CT used for staging (Lugano classification). Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)

- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies

- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

- Most people recover well, with cure rates exceeding 80%
- Brentuximab vedotin and checkpoint inhibitors (nivolumab, pembrolizumab) are used for relapsed/refractory disease.

Chapter 404

Homocystinuria

Overview

Homocystinuria (classic form) is an autosomal recessive disorder caused by deficiency of cystathionine β -synthase (CBS), the enzyme that converts homocysteine to cystathionine in the transsulfuration pathway. The incidence is approximately 1 in 200,000–300,000 live births.

Causes

This condition happens because of accumulation of homocysteine and methionine, producing a characteristic clinical phenotype resembling Marfan syndrome.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience tall, thin build, ectopia lentis (downward lens dislocation), myopia, intellectual disability (variable), skeletal abnormalities (scoliosis, pectus excavatum, arachnodactyly, genu valgum), osteoporosis, and a high risk of thromboembolism (the most life-threatening feature).

Emergency warning signs

- Severe or worsening symptoms of homocystinuria require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Doctors diagnose this using elevated total plasma homocysteine (greater than 100 $\mu\text{mol/L}$), elevated methionine, and low cystine
- Newborn screening enables early detection.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on pyridoxine for responsive patients

- Pyridoxine-nonresponsive patients require methionine restriction, betaine supplementation, and B12/folate supplementation
- Antiplatelet therapy reduces thromboembolic risk.

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with early treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 405

Hookworm / Soil-Transmitted Helminths

Overview

Causes

This condition happens because of hookworm larvae penetrating skin, migrating through the circulation to the lungs, ascending the trachea, and maturing in the small intestine, with adult worms attaching to intestinal mucosa and feeding on blood.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience hookworm: cutaneous larva currens, iron deficiency anemia (from chronic blood loss), fatigue, pallor, and growth retardation
- *Ascaris*: heavy infection causes abdominal pain, malnutrition, and intestinal obstruction
- *Trichuris*: dysentery syndrome (bloody, mucoid stools, tenesmus, rectal prolapse).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hookworm / soil-transmitted helminths require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using stool microscopy with Kato-Katz method. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Soil-transmitted helminths include hookworm (*Ancylostoma duodenale*, *Necator americanus*), roundworm (*Ascaris lumbricoides*), and whipworm (*Trichuris trichiura*), constituting the most common infections of humans globally, with 1.5 billion people infected, endemic in tropical and subtropical regions with poor sanitation.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include albendazole or mebendazole. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical

therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 406

Hordeolum (Stye) and Chalazion

Overview

Hordeolum (stye) and chalazion are common eyelid lesions affecting the meibomian or Zeis glands. A hordeolum is an acute bacterial infection, while a chalazion is a chronic sterile lipogranulomatous inflammation from gland obstruction. These conditions affect people of all ages and are among the most frequent ophthalmic complaints in primary care.

Classification

Cause Infectious / Inflammatory

Organ System Eyes

Pathophysiology Bacterial infection (hordeolum) or duct obstruction with lipogranulomatous inflammation (chalazion)

Duration Acute (hordeolum) to Chronic (chalazion)

Onset Sudden (hordeolum) or Gradual (chalazion)

Mode of Transmission Autoinoculation

Clinical Course Acute, self-limited or chronic, resolving

Severity Mild

Extent of Disease Localized

Age Group All ages

Epidemiology Very common; affects all ages; more frequent in adults

ICD-11 Code 9A60.0 / 9A60.1

Causes

A hordeolum results from acute bacterial infection of the eyelid glands, most commonly *Staphylococcus aureus*, causing localized abscess formation. A chalazion develops when the meibomian gland duct becomes obstructed, leading to extrusion of gland contents into surrounding tissue and a chronic lipogranulomatous inflammatory response. Poor eyelid hygiene and chronic blepharitis are predisposing factors.

Risk Factors

- Chronic blepharitis
- Poor eyelid hygiene
- Contact lens use
- Recurrent styes
- Diabetes mellitus
- Seborrheic dermatitis
- Rosacea
- Immunosuppression

Signs and Symptoms

Early symptoms

- Early manifestations of hordeolum (stye) and chalazion may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Hordeolum presents as an acute, painful, erythematous nodule at the eyelid margin with localized swelling and tenderness
- Chalazion presents as a firm, painless, slowly growing nodule within the eyelid, away from the margin
- External hordeolum points at the lid margin; internal hordeolum points on the conjunctival surface
- Tearing, foreign body sensation, and photophobia
- Blurred vision from mass effect on the cornea
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of hordeolum (stye) and chalazion may occur

Emergency warning signs

- Severe or worsening symptoms of hordeolum (stye) and chalazion require immediate medical evaluation

Progression and Stages

An untreated hordeolum typically suppurates and drains spontaneously within 3–5 days. A chalazion may persist for weeks to months, gradually enlarging before eventually resolving or requiring intervention. Chronic or recurrent lesions warrant evaluation for underlying systemic conditions.

Complications

- Preseptal or orbital cellulitis (rare, from extension of infection)
- Recurrent hordeola or chalazia
- Corneal astigmatism from large chalazion
- Scarring or lid deformity
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on history and physical examination
- Slit-lamp examination to differentiate hordeolum from chalazion
- Lid eversion to assess internal lesions
- Biopsy for atypical or recurrent lesions to rule out sebaceous cell carcinoma
- Lab tests to check for underlying conditions

Prevention

- Maintain good eyelid hygiene with warm compresses
- Avoid touching or rubbing the eyes
- Treat underlying blepharitis or rosacea
- Replace eye makeup regularly
- Go for regular check-ups so any problems are caught early

Treatment Options

- Warm compresses applied 4–6 times daily for 10–15 minutes promote drainage
- Gentle lid massage after warm compresses
- Topical antibiotic ointments (bacitracin, erythromycin) for hordeolum
- Incision and curettage for persistent chalazion
- Intralesional corticosteroid injection for large chalazia
- Oral antibiotics for associated cellulitis
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Both hordeolum and chalazion carry an excellent prognosis with appropriate treatment. Most hordeola resolve within one week, while chalazia may take several weeks to months. Recurrence is common, especially in patients with underlying blepharitis or rosacea, but complications are rare with proper management.

Chapter 407

Human Metapneumovirus (hMPV)

Overview

Human metapneumovirus is a single-stranded RNA virus (family Paramyxoviridae) first identified in 2001, causing acute respiratory tract infections across all ages, with the highest burden in young children, older adults, and immunocompromised patients, accounting for 5–15% of hospitalized respiratory illnesses in children.

Causes

This condition happens because of hMPV infecting ciliated epithelial cells, causing syncytia formation, ciliary dysfunction, and epithelial sloughing, with the virus inhibiting type I interferon signaling.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 3–6 days, with presentation nearly indistinguishable from RSV: infants and young children present with coryza, cough, fever, wheezing, bronchiolitis, and pneumonia
- Older adults and immunocompromised present with severe lower respiratory tract infection.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of human metapneumovirus (hmpv) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using RT-PCR of nasopharyngeal swab or BAL. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment typically involves supportive care.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well for immunocompetent hosts. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 408

Human Metapneumovirus (hMPV)

Overview

Human metapneumovirus (hMPV) is a paramyxovirus closely related to respiratory syncytial virus (RSV), discovered in 2001, yet ubiquitous—virtually all children seroconvert by age 5.

Causes

This condition happens because of hMPV infecting respiratory epithelium causing syncytia formation, ciliary dysfunction, airway obstruction, and inflammation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms can range from mild URI to severe lower respiratory tract disease (bronchiolitis, croup, pneumonia)
- Severe disease is most common in premature infants, those with cardiopulmonary disease, and immunocompromised children.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of human metapneumovirus (hmpv) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using multiplex PCR. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment typically involves supportive care
- No licensed vaccine or specific antiviral therapy is available.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well for immunocompetent children. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 409

Huntington's Disease

Overview

Huntington's disease is an autosomal dominant neurodegenerative disorder caused by a CAG trinucleotide repeat expansion in the huntingtin (HTT) gene on chromosome 4. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a triad of motor dysfunction (chorea, dystonia, bradykinesia), thinking and memory decline (executive dysfunction, dementia), and psychiatric disturbances (depression, irritability, psychosis). Headache and facial pain Weakness

or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of huntington's disease require immediate medical evaluation

Progression and Stages

This condition happens because of mutant huntingtin protein causing progressive deterioration of neurons, particularly in the caudate nucleus and putamen (striatum). Onset typically occurs between ages 30 and 50, with disease severity correlating inversely with the number of CAG repeats. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with genetic testing. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Treatment focuses on symptomatic treatment with tetrabenazine or deutetabenazine for chorea, antipsychotics, and antidepressants
- Genetic counseling is essential.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is progressive and fatal, with no curative treatment available. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 410

Hydrogen Sulfide Toxicity

Overview

Hydrogen sulfide (H_2S) is a colorless gas with a characteristic rotten egg odor (rapid olfactory fatigue occurs, rendering odor unreliable), encountered in sewers, manure pits, oil and gas drilling, and pulp mills.

Causes

This condition happens because of H_2S inhibiting cytochrome c oxidase (identical mechanism to cyanide), causing histotoxic hypoxia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Chronic low-level exposure causes headache, fatigue, olfactory dysfunction, and thinking and memory impairment.
- Pain or discomfort in the affected area

Emergency warning signs

- Clinical features: acute exposure causes conjunctivitis, respiratory tract irritation, lung swelling, loss of consciousness (knockdown—immediate collapse), seizures, and death

Progression and Stages

Your outlook depends on severity of exposure. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Management: immediate removal from exposure, nitrite therapy to induce methemoglobinemia, hyperbaric oxygen, and supportive care.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 411

Hydronephrosis

Overview

Hydronephrosis is the dilation of the renal pelvis and calyces caused by obstruction of urine outflow from the kidney. It can be unilateral or bilateral, acute or chronic, and affects patients of all ages from prenatal to elderly. The condition may be asymptomatic or lead to progressive renal damage if the underlying obstruction is not relieved.

Classification

Cause Obstructive / Anatomic

Organ System Kidneys

Pathophysiology Urinary tract obstruction leading to increased intrapelvic pressure, tubular dilation, and eventual renal parenchymal damage

Duration Acute (hours to days) or Chronic (months to years)

Onset Gradual or Sudden

Mode of Transmission Non-communicable

Clinical Course Variable depending on cause and duration

Severity Mild to Severe

Extent of Disease Localized (unilateral or bilateral)

Age Group All ages

Epidemiology Common; found in 1–5% of autopsies; congenital in 1 in 100 births

ICD-11 Code GB00.0

Causes

Hydronephrosis results from any obstruction along the urinary tract, from the ureteropelvic junction to the urethra. Common causes include ureteropelvic junction obstruction (congenital), ureteral stones, benign prostatic hyperplasia, pelvic or retroperitoneal tumors, posterior urethral valves (neonates), and neurogenic bladder. Vesicoureteral reflux can also produce pelvicalyceal dilation without obstruction.

Risk Factors

- Nephrolithiasis (kidney stones)
- Benign prostatic hyperplasia
- Pelvic malignancies (prostate, bladder, cervical, ovarian, colorectal)
- Pregnancy (physiologic hydronephrosis)
- Congenital anomalies (UPJ obstruction, posterior urethral valves)
- Neurogenic bladder
- Previous pelvic surgery or radiation
- Spinal cord injury

Signs and Symptoms

Early symptoms

- Early manifestations of hydronephrosis may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Flank pain or discomfort, often radiating to the groin
- Acute obstruction causes severe colicky pain (ureteral colic)
- Nausea and vomiting with acute obstruction
- Reduced urine output or anuria (bilateral or solitary kidney)
- Urinary tract infections and fever
- Hematuria (especially with stones)
- Hypertension from activation of the renin-angiotensin system
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of hydronephrosis may occur

Emergency warning signs

- Severe or worsening symptoms of hydronephrosis require immediate medical evaluation

Progression and Stages

If obstruction is unrelieved, progressive dilation leads to tubular atrophy, interstitial fibrosis, and irreversible nephron loss. Acute complete obstruction causes rapid renal damage within days. Chronic partial obstruction allows for slow, insidious loss of renal function over months to years. The extent of recovery after relief depends on the severity and duration of obstruction.

Complications

- Obstructive uropathy and hydronephrotic atrophy
- Acute kidney injury or chronic kidney disease
- Urinary tract infections and pyelonephritis
- Urolithiasis
- Hypertension
- Sepsis from infected obstructed kidney (pyonephrosis)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Renal ultrasound is the initial imaging modality of choice
- CT urography or MRI to identify level and cause of obstruction
- Voiding cystourethrogram (VCUG) for suspected reflux or posterior urethral valves
- Renal scintigraphy (MAG3) to assess functional significance
- Blood tests (creatinine, BUN) to evaluate renal function
- Urinalysis and culture to exclude infection

Prevention

- Maintain hydration to reduce stone formation
- Regular urologic screening for patients with known risk factors
- Prompt treatment of urinary tract infections
- Prenatal ultrasound screening for congenital anomalies
- Go for regular check-ups so any problems are caught early

Treatment Options

- Relief of obstruction by ureteral stenting or nephrostomy tube
- Ureteroscopy with laser lithotripsy for stones
- Pyeloplasty for ureteropelvic junction obstruction
- Transurethral resection of prostate (TURP) for BPH
- Valve ablation for posterior urethral valves

- Antibiotics for associated urinary tract infection
- Lifestyle changes and self-care
- Surgery when needed

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

The prognosis depends on the cause, severity, and duration of obstruction. Acute obstruction relieved promptly carries an excellent prognosis with full recovery of renal function. Chronic or severe obstruction can lead to irreversible renal damage. Long-term monitoring after relief of obstruction is essential to detect recurrence and manage residual renal impairment.

Chapter 412

Hymenoptera Stings

Overview

Hymenoptera (bees, wasps, hornets, fire ants) cause local pain, redness of the skin, and swelling from venom components. The major concern is immediate hypersensitivity (IgE-mediated anaphylaxis) in sensitized individuals. Anaphylaxis presents with urticaria, angioedema, bronchospasm, laryngeal swelling, hypotension, and cardiovascular collapse occurring within minutes to 1 hour after sting.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hymenoptera stings require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam. Treatment for anaphylaxis: intramuscular epinephrine (0.3–0.5 mg, 1:1,000, anterolateral thigh) is first-line
- Adjunctive includes antihistamines, corticosteroids, and IV fluids. Venom immunotherapy (VIT) for patients with a history of anaphylaxis provides 80–98% protection from future reactions.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with prompt treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 413

Hyperemesis Gravidarum

Overview

Hyperemesis gravidarum (HG) is severe, persistent nausea and vomiting during pregnancy causing weight loss ($>5\%$ of prepregnancy weight), dehydration, electrolyte abnormalities, and ketonuria. It affects 0.5–2% of pregnancies, distinguishing it from the more common (70–80%) mild nausea and vomiting of pregnancy. The Disease mechanism involves elevated hCG, estrogen, and progesterone, with a potential genetic predisposition (mutation in GDF15 gene).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe nausea and vomiting leading to weight loss and dehydration.

Emergency warning signs

- Severe or worsening symptoms of hyperemesis gravidarum require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include non-pharmacologic measures (dietary modifications, ginger, acupressure), antiemetics (doxylamine/pyridoxine combination as first-line, ondansetron, metoclopramide, promethazine), IV fluid and electrolyte repletion, and thiamine supplementation to prevent Wernicke encephalopathy.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with treatment
- Severe cases requiring parenteral nutrition are rare.

Chapter 414

Hyperopia

Overview

Hyperopia is a refractive error that occurs when the axial length of the eye is too short or the refractive power is insufficient, causing light to focus behind the retina.

Causes

This condition happens because of insufficient refractive power relative to axial length. Mild hyperopia may be accommodated for, especially in young patients.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience blurred near vision, eye strain, headaches, and in children, esotropia and amblyopia.

Emergency warning signs

- Severe or worsening symptoms of hyperopia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on refraction testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment focuses on correction with convex (plus) lenses
- Severe hyperopia in childhood requires early correction to prevent amblyopia and strabismus.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with correction. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 415

Hyperosmolar Hyperglycemic State

Overview

Hyperosmolar hyperglycemic state (HHS) is a metabolic emergency primarily affecting T2DM patients, characterized by extreme hyperglycemia (glucose >600 mg/dL), profound dehydration, hyperosmolality (serum osmolality >320 mOsm/kg), and minimal or absent ketosis.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe dehydration, altered mental status, and neurological deficits (seizures, focal deficits).

Emergency warning signs

- Severe or worsening symptoms of hyperosmolar hyperglycemic state require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on blood glucose, serum osmolality, and absence of significant ketosis. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

This condition happens because of sufficient residual insulin to prevent ketogenesis but not to control glucose.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on aggressive fluid resuscitation (initially normal saline), insulin infusion (at lower doses than DKA), and electrolyte replacement. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is more guarded than DKA
- Mortality is higher (up to 20%).

Chapter 416

Hyperparathyroidism

Overview

Hyperparathyroidism is marked by excessive secretion of parathyroid hormone (PTH). Primary hyperparathyroidism (most common) results from a parathyroid adenoma (85%), abnormal increase in cells (10–15%), or rarely carcinoma (<1%), causing hypercalcemia which may be asymptomatic or symptomatic: “bones, stones, abdominal groans, and psychic moans” (bone pain/pathologic fractures, kidney stones, abdominal pain/pancreatitis/constipation, depression/thinking and memory dysfunction). Secondary hyperparathyroidism occurs in CKD due to phosphate retention and decreased vitamin D activation. Tertiary hyperparathyroidism occurs after prolonged secondary hyperparathyroidism with autonomous PTH secretion. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hyperparathyroidism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To diagnose primary disease shows elevated calcium with elevated or inappropriately normal PTH.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves parathyroidectomy for symptomatic patients or those meeting criteria (calcium >1 mg/dL above normal, reduced bone density, kidney stones, age <50 , or kidney impairment). Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular

monitoring of metabolic parameters Weight management programs Exercise prescription
Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with surgical cure.

Chapter 417

Hyperprolactinemia

Overview

Hyperprolactinemia is the most common pituitary hormone excess disorder, characterized by elevated serum prolactin levels.

Causes

Causes include prolactin-secreting pituitary adenomas (prolactinomas, most common), medications (antipsychotics, metoclopramide, domperidone, verapamil), hypothalamic/pituitary stalk compression, primary hypothyroidism (increased TRH stimulates prolactin), chronic kidney failure, and chest wall trauma.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features in women include galactorrhea, oligomenorrhea or amenorrhea, infertility, and decreased libido. In men: decreased libido, erectile dysfunction, infertility, gynecomastia, and galactorrhea (rare). Mass effects from macroprolactinomas (>1 cm) include headache, visual field defects (bitemporal hemianopia), and hypopituitarism.

Emergency warning signs

- Severe or worsening symptoms of hyperprolactinemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on confirmed elevated serum prolactin (macroprolactin excluded), MRI pituitary with contrast to identify adenoma, and thyroid function tests to exclude hypothyroidism. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include dopamine agonists (cabergoline preferred over bromocriptine due to better efficacy and tolerability) as first-line therapy for symptomatic hyperprolactinemia and prolactinomas, and transsphenoidal surgery for dopamine agonist-resistant or apoplectic macroprolactinomas. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and

rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Most patients achieve normal prolactin levels with dopamine agonist therapy.

Chapter 418

Hypersplenism

Overview

Hypersplenism is a syndrome of splenic overactivity resulting in cytopenias (one or more of anemia, leukopenia, thrombocytopenia) due to excessive sequestration and destruction of blood cells. It is always secondary to an underlying condition causing splenomegaly.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience cytopenias with corresponding symptoms (anemia, infection risk, bleeding tendency). Diagnosis is suggested by cytopenias, splenomegaly, and bone marrow showing adequate or hypercellular hematopoiesis.

Emergency warning signs

- Severe or worsening symptoms of hypersplenism require immediate medical evaluation

Progression and Stages

Your outlook depends on the underlying condition and severity of cytopenias. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves directed at the underlying cause
- Splenectomy may be considered for severe, symptomatic cytopenias refractory to medical management, though it increases the risk of overwhelming post-splenectomy infection (OPSI).
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider

- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 419

Hypertension

Overview

high blood pressure is defined as a sustained elevation of systemic arterial blood pressure (systolic ≥ 130 mmHg or diastolic ≥ 80 mmHg per ACC/AHA guidelines). Essential (primary) high blood pressure accounts for 90–95% of cases and results from complex interactions between genetic factors, kidney sodium retention, sympathetic nervous system hyperreactivity, and vascular smooth muscle enlargement. Secondary high blood pressure (5–10% of cases) results from kidney parenchymal disease, renovascular high blood pressure, primary aldosteronism, obstructive sleep apnea, or endocrine tumors. Most patients are asymptomatic until end-organ damage occurs. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease

- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of hypertension require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection

- Chronic kidney disease from reduced perfusion

Diagnosis

To make the diagnosis, doctors look for average blood pressure measurements from ≥ 2 occasions. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

Treatment options include lifestyle modification (DASH diet, sodium restriction, exercise) and first-line pharmacotherapy (ACE inhibitors/ARBs, thiazide diuretics, calcium channel blockers).

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary

artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Most people recover well with early control. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 420

Hypertension in the Elderly

Overview

High blood pressure affects over 70% of adults aged 65 and older. The predominant form in older adults is isolated systolic high blood pressure (ISH), defined as systolic BP ≥ 130 mmHg with diastolic BP < 80 mmHg, resulting from age-related arterial stiffening. ISH confers significant risk for stroke, coronary artery disease, heart failure, and thinking and memory decline. The SPRINT trial demonstrated that intensive systolic BP lowering to < 120 mmHg reduced cardiovascular events and mortality in adults over 75, though with increased risk of orthostatic hypotension, electrolyte abnormalities, and acute kidney injury. Current guidelines recommend a target of $< 130/80$ mmHg for most older adults, but treatment decisions should be individualized based on frailty, falls risk, polypharmacy, and life expectancy. First-line agents include thiazide diuretics, calcium channel blockers, and ACE inhibitors/ARBs. Initial doses should be low with gradual up-titration. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking

- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of hypertension in the elderly require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)

- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 421

Hyperthyroidism

Overview

Hyperthyroidism (thyrotoxicosis) is a state of excessive thyroid hormone production, affecting approximately 1–2% of the population. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

The most common cause is Graves' disease (an autoimmune condition with thyroid-stimulating immunoglobulins [TSI] that activate the TSH receptor, causing diffuse goiter, ophthalmopathy, and dermopathy), with other causes including toxic multinodular goiter, toxic adenoma (Plummer's disease), subacute thyroiditis (painful, viral), silent thyroiditis, postpartum thyroiditis, and exogenous thyroid hormone ingestion. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience weight loss, heat intolerance, palpitations, anxiety, tremor, diarrhea, menstrual irregularities, warm moist skin, lid retraction, and diffuse goiter. Diagnosis shows suppressed TSH with elevated free T4 and/or T3. Management options include antithyroid drugs (methimazole—preferred, or propylthiouracil [PTU]—used in first trimester of pregnancy and thyroid storm), radioactive iodine (I-131) ablation, and thyroidectomy, with beta-blockers (propranolol) providing symptomatic relief.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hyperthyroidism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with appropriate treatment.

Chapter 422

Hypertrophic Cardiomyopathy

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Hypertrophic cardiomyopathy is the most common inherited cardiovascular disease, affecting 1 in 500 individuals, characterized by asymmetric left ventricular enlargement (typically interventricular septum of 15 mm or greater) in the absence of other loading conditions
- It is a leading cause of sudden cardiac death in young athletes.

Causes

This condition happens because of mutations in sarcomere proteins (beta-myosin heavy chain, myosin-binding protein C being most common). The Disease mechanism involves diastolic dysfunction, dynamic left ventricular outflow tract obstruction (present in 70%), and myocardial reduced blood flow. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol

- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience exertional shortness of breath, chest pain, syncope, and palpitations. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of hypertrophic cardiomyopathy require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events

- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using echocardiography and cardiac MRI. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Treatment options include beta-blockers, calcium channel blockers, and mavacamten for obstructive disease

- Septal ablation or myectomy is for refractory obstruction
- Implantable cardioverter-defibrillator is placed for high-risk patients.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)

- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

In most cases, the outlook is good with appropriate management, though sudden cardiac death risk requires stratification. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 423

Hypertrophic Pyloric Stenosis

Overview

Hypertrophic pyloric stenosis (HPS) is progressive thickening of the pyloric muscle causing gastric outlet obstruction. It occurs in 2–4 per 1000 live births, more commonly in males (4:1) and firstborn infants.

Causes

This condition happens because of enlargement and abnormal increase in cells of the pyloric circular muscle.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience presentation at 3–6 weeks of age with non-bilious, projectile vomiting after feeds
- Infants remain hungry after vomiting.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hypertrophic pyloric stenosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using physical examination (palpable olive-shaped mass in the right upper quadrant) and ultrasound (thickened pyloric muscle). Metabolic alkalosis with hypochloremia and hypokalemia results from loss of gastric acid. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves pyloromyotomy (Ramstedt procedure) after correction of fluid and electrolyte abnormalities. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up

care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 424

Hypoglycemia

Overview

Hypoglycemia is a blood glucose level below 70 mg/dL, with clinically significant hypoglycemia occurring below 54 mg/dL (Level 2) and severe hypoglycemia (Level 3) being any event requiring external assistance.

Causes

This condition happens because of an imbalance between glucose-lowering therapy and glucose supply.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience adrenergic symptoms (tremor, palpitations, sweating, anxiety) and neuroglycopenic symptoms (confusion, blurred vision, seizures, loss of consciousness).

Emergency warning signs

- Severe or worsening symptoms of hypoglycemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

It is the most common acute complication of diabetes treatment (insulin and sulfonylureas).

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on the Whipple triad: symptoms of hypoglycemia, low measured glucose, and resolution of symptoms with glucose correction. Treatment for mild-moderate hypoglycemia is with rapid-acting oral glucose (15–20 g)
- Severe hypoglycemia requires IV dextrose or glucagon injection.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is excellent with prompt treatment
- Recurrent hypoglycemia may cause hypoglycemia unawareness, requiring adjustment of insulin regimens.

Chapter 425

Hypoparathyroidism

Overview

Hypoparathyroidism is marked by deficient PTH production, leading to hypocalcemia and hyperphosphatemia. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

The most common cause is accidental damage to or removal of the parathyroid glands during thyroid surgery (iatrogenic hypoparathyroidism), with other causes including autoimmune hypoparathyroidism (in isolation or as part of APS-1, AIRE gene mutation) and genetic causes such as DiGeorge syndrome (22q11.2 deletion). Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of hypocalcemia include perioral and digital paresthesias, carpopedal spasm, tetany (Chvostek's sign—facial muscle twitch with facial nerve tapping)

- Trousseau’s sign—carpopedal spasm with blood pressure cuff inflation), seizures, and irregular heartbeats (prolonged QT).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hypoparathyroidism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low PTH with hypocalcemia and hyperphosphatemia.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves oral calcium supplementation (calcium carbonate or citrate) and active vitamin D analogs (calcitriol). Dietary modifications and nutritional counseling
Pharmacotherapy to correct metabolic abnormalities
Hormone replacement therapy when indicated
Regular monitoring of metabolic parameters
Weight management programs
Exercise prescription
Management of associated conditions
Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with appropriate supplementation.

Chapter 426

Hypopituitarism

Overview

Hypopituitarism is deficiency in one or more pituitary hormones, with causes including pituitary adenomas (most common), craniopharyngiomas, meningiomas, pituitary surgery/radiation, Sheehan syndrome (postpartum pituitary tissue death from bleeding), traumatic brain injury, infiltrative diseases (sarcoidosis, histiocytosis), infections, and autoimmune hypophysitis, with hormones lost in a characteristic order: GH > LH/FSH > TSH > ACTH > prolactin. Clinical manifestations depend on which hormones are deficient: GH deficiency (decreased muscle mass, increased fat), gonadotropin deficiency (amenorrhea, erectile dysfunction, decreased libido), ACTH deficiency (secondary adrenal insufficiency—without hyperpigmentation and with preserved aldosterone), TSH deficiency (central hypothyroidism), and ADH deficiency (central diabetes insipidus).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hypopituitarism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for dynamic endocrine testing (insulin tolerance test, ACTH stimulation test, GnRH stimulation test) and MRI of the pituitary. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves hormone replacement for each deficient axis, with hydrocortisone for ACTH deficiency (must be replaced before thyroid hormone to avoid adrenal crisis). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and

support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with appropriate replacement. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 427

Hypothermia

Overview

Causes

Causes include environmental exposure, cold-water immersion, alcohol intoxication, hypothyroidism, hypoglycemia, and trauma. This condition happens because of initial sympathetic activation (shivering, vasoconstriction, tachycardia) transitioning to progressive depression of all organ systems as temperature drops.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: mild hypothermia: intact shivering, confusion, loss of coordination, slurred speech
- Moderate hypothermia: cessation of shivering, progressive bradycardia, hyporeflexia, paradoxical undressing, obtundation
- Severe hypothermia: coma, apnea, ventricular arrhythmias, asystole. The classic ECG finding is the Osborn wave (J wave).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hypothermia require immediate medical evaluation

Progression and Stages

Hypothermia is defined as core temperature below 35°C (95°F), classified as mild (32–35°C), moderate (28–32°C), and severe (<28°C). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using core temperature measurement. Management: passive external rewarming for mild hypothermia

- Active external rewarming for moderate hypothermia
- Active internal rewarming (warmed IV fluids, ECMO) for severe hypothermia.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate rewarming. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 428

Hypothyroidism

Overview

Hypothyroidism is a state of deficient thyroid hormone production, affecting approximately 2% of the population, with higher prevalence in women, the elderly, and iodine-deficient regions. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition results from disruption of normal hypothyroidism function.

- This condition happens because of various causes, with the most common in iodine-sufficient areas being Hashimoto's thyroiditis (chronic autoimmune thyroiditis, characterized by lymphocytic infiltration and destruction of the thyroid gland with positive anti-TPO and anti-thyroglobulin antibodies), while iodine deficiency is the most common cause worldwide
- Other causes include post-thyroidectomy or post-radioiodine therapy, external beam radiation, medications (lithium, amiodarone, tyrosine kinase inhibitors), central (pituitary/hypothalamic) hypothyroidism, and congenital hypothyroidism.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fatigue, weight gain, cold intolerance, constipation, dry skin, hair loss, bradycardia, delayed relaxation of deep tendon reflexes, and depression, with severe hypothyroidism (myxedema coma) being a medical emergency with hypothermia, altered consciousness, and cardiovascular collapse.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of hypothyroidism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using elevated TSH (primary hypothyroidism) and low free T4.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves levothyroxine (synthetic T4) replacement, dosed to normalize TSH. Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate replacement therapy.

Chapter 429

Idiopathic Granulomatous Mastitis

Overview

Idiopathic granulomatous mastitis (IGM) is a rare, chronic, non-caseating inflammatory breast disease affecting parous women of childbearing age, typically within five years of lactation. The underlying Disease mechanism involves an autoimmune delayed-type hypersensitivity reaction directed against extravasated proteinaceous luminal secretions from damaged ductal epithelium.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms often appear as a rapidly enlarging, painful, hard, irregular breast mass frequently accompanied by overlying skin redness of the skin, nipple retraction, ulceration, and chronic sterile sinus tract removal of fluid, closely mimicking breast cancer or acute bacterial abscess.

Emergency warning signs

- Severe or worsening symptoms of idiopathic granulomatous mastitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for core needle biopsy demonstrating non-caseating granulomatous inflammation centered on breast lobules, with negative bacterial, fungal, and mycobacterial cultures. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include systemic corticosteroids (prednisone), immunosuppressive therapy (methotrexate), and conservative local care
- Surgical removal is reserved for refractory cases due to high recurrence rates (20 – 50%).
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 430

Idiopathic Hypersomnia

Overview

Idiopathic hypersomnia is a disorder of excessive daytime sleepiness despite normal or prolonged nighttime sleep, with prolonged sleep inertia (severe difficulty awakening, confusion, grogginess, sometimes referred to as sleep drunkenness) and non-restorative naps. The prevalence is approximately 5–10 per 100,000.

Causes

This condition happens because of unknown Disease mechanism, with hypotheses including altered GABA-A receptor function, increased endogenous sleep-promoting substances, or dysfunction of wake-promoting circuits.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience excessive daytime sleepiness, prolonged sleep inertia, and non-restorative naps.

Emergency warning signs

- Severe or worsening symptoms of idiopathic hypersomnia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for MSLT showing mean sleep latency ≤ 8 minutes with fewer than two SOREMs, or total 24-hour sleep time of >660 minutes on polysomnography or actigraphy, with exclusion of narcolepsy, sleep-disordered breathing, insufficient sleep syndrome, and other causes. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include wake-promoting agents (modafinil, armodafinil, pitolisant, solriamfetol), sodium oxybate, and clarithromycin. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and

follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is chronic
- Response to Treatment typically involves often less robust than in narcolepsy.

Chapter 431

Idiopathic Insomnia

Overview

Idiopathic insomnia is a lifelong form of insomnia with onset in infancy or early childhood, without identifiable triggers, and in the absence of medical, psychiatric, or environmental causes. It is rare, representing less than 5% of insomnia cases. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of a hypothesized primary abnormality in the sleep-wake regulatory system, such as dysfunction of the VLPO or an imbalance between sleep-promoting and arousal-promoting systems. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience objectively reduced total sleep time on polysomnography and persistent hyperarousal.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of idiopathic insomnia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, with polysomnography used to rule out other sleep disorders.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on CBT-I as first-line therapy, though efficacy may be limited
- Pharmacotherapy (hypnotics, sedating antidepressants) is often required long-term.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is persistence across the lifespan. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 432

Idiopathic Intracranial Hypertension

Overview

- Idiopathic intracranial hypertension (IIH), formerly called pseudotumor cerebri, is a condition of raised pressure within the skull in the absence of a brain tumor or hydrocephalus
- It most commonly affects young, overweight women
- It causes headache and visual disturbances; untreated, it can lead to permanent vision loss

Causes

The cause is unknown, but the raised intracranial pressure is thought to result from impaired absorption of cerebrospinal fluid. It is strongly associated with obesity and weight gain. Certain medications (tetracyclines, retinoids such as vitamin A, lithium, growth hormone) and conditions such as polycystic ovary syndrome and venous sinus thrombosis can trigger or mimic it.

Risk Factors

- Female sex and childbearing age
- Obesity, especially recent weight gain
- Use of certain medications (tetracycline antibiotics, isotretinoin)
- Excess vitamin A intake
- Endocrine disorders such as polycystic ovary syndrome
- Cerebral venous sinus thrombosis

Signs and Symptoms

Common symptoms

- Headache that is often worse in the morning or with lying down
- Pulsatile tinnitus (hearing a whooshing sound in sync with the pulse)
- Transient visual obscurations (brief episodes of blurred or grayed vision, often on standing)
- Double vision (usually from sixth nerve palsy)
- Pulsing visual flashes

Emergency warning signs

- Sudden, severe, or progressive vision loss requires urgent evaluation, as it can be permanent
- Worsening headache with confusion or vomiting requires emergency assessment

Progression and Stages

Without treatment, intracranial pressure remains elevated and can progressively damage the optic nerves, causing gradual visual field loss and eventually blindness. With treatment and weight loss, the pressure usually normalizes and vision stabilizes or improves. The disease can recur, especially with weight regain.

Complications

- Optic nerve damage (optic atrophy) and permanent vision loss
- Visual field defects and constriction
- Chronic headaches and reduced quality of life
- Sixth nerve palsy causing double vision
- Recurrence of symptoms after treatment

Diagnosis

- Lumbar puncture showing elevated opening pressure with normal cerebrospinal fluid
- Ophthalmoscopy showing papilledema (swelling of the optic disc)
- Visual field testing and formal perimetry
- MRI/MRV of the brain to exclude tumors, hydrocephalus, or venous sinus thrombosis
- Optical coherence tomography (OCT) of the optic nerve
- Diagnosis requires normal imaging, elevated pressure, and characteristic symptoms

Prevention

- Weight management and prevention of obesity
- Cautious use of medications known to trigger IIH
- Prompt neurological and ophthalmological evaluation of new headaches with visual symptoms
- Regular eye examinations for those with risk factors

Treatment Options

- Weight loss (even 5–10%) is a cornerstone of management
- Carbonic anhydrase inhibitors (acetazolamide) to reduce cerebrospinal fluid production

- Diuretics (e.g., furosemide) in some cases
- Repeat lumbar punctures for temporary relief
- Optic nerve sheath fenestration or cerebrospinal fluid shunt surgery when vision is threatened
- Discontinuation of any causative medications

Living with It

- Follow the weight loss plan and treatment regimen
- Attend regular ophthalmological monitoring of the optic nerves
- Report any new or worsening visual symptoms immediately
- Manage headaches with the help of a specialist
- Connect with support networks for chronic headache conditions

Outlook

With early diagnosis and treatment, vision loss can usually be prevented and headache improved. Weight loss is associated with the best long-term outcomes. Untreated, the condition carries a real risk of permanent blindness, which is why regular visual monitoring is essential.

Chapter 433

Idiopathic Pulmonary Fibrosis

Overview

Idiopathic lung scarring is a chronic, progressive, fibrosing interstitial pneumonia of unknown cause, occurring predominantly in older males. The pathology is marked by usual interstitial pneumonia pattern: patchy, subpleural, basal-predominant scarring with honeycombing and fibroblastic foci. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age.

Causes

This condition happens because of aberrant wound healing and fibroblast growth in the lung parenchyma. Degenerative processes involve progressive cellular dysfunction and tissue breakdown over time. Contributing factors include oxidative stress, inflammation, accumulated cellular damage, and reduced regenerative capacity.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience insidious-onset exertional shortness of breath and dry cough
- Velcro-like crackles are heard on auscultation.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of idiopathic pulmonary fibrosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Treatment options include antifibrotic therapies (pirfenidone and nintedanib) that slow disease progression
- Lung transplantation is the only intervention shown to improve survival.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for HRCT showing UIP pattern and exclusion of known causes

- Lung biopsy may be needed when HRCT is inconclusive
- Lung function tests show a restrictive pattern with reduced DLCO.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Unfortunately, the outlook is often poor, with median survival of 3–5 years. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 434

IgA Nephropathy

Overview

IgA nephropathy (Berger's disease) is the most common primary glomerulonephritis worldwide, with higher prevalence in East Asia and Southern Europe.

Causes

This condition happens because of deposition of galactose-deficient IgA1 in the mesangium, triggering an immune response, mesangial growth, and glomerular injury.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience episodic gross hematuria (often concurrent with upper respiratory infections—"synpharyngitic hematuria"), microscopic hematuria, or proteinuria
- Some present with nephrotic syndrome or rapidly progressive glomerulonephritis.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of iga nephropathy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- To make the diagnosis, doctors look for kidney biopsy showing mesangial IgA deposition on immunofluorescence
- The Oxford Classification (MEST-C scoring) guides outlook.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Neflgentan (sodium-glucose cotransporter 2 inhibitor) is an emerging therapy. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- The right treatment depends on severity: RAAS blockade for proteinuria, SGLT2 inhibitors, and immunosuppression (corticosteroids) for high-risk patients
- Supportive care includes BP control and smoking cessation.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Up to 30% progress to ESRD within 20 years.

Chapter 435

IgA Vasculitis

Overview

IgA vasculitis is a small-vessel vasculitis characterized by IgA-dominant immune complex deposition in vessel walls, affecting the skin, joints, digestive tract, and kidneys, and is the most common vasculitis in children, with an incidence of 3–26 per 100,000 in children versus 0.1–1.8 per 100,000 in adults, with a median age at presentation of 4–6 years in children. The condition involves IgA1 deposition in small vessels with complement activation and leukocytoclasia, with triggers including upper respiratory tract infections. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- You may experience palpable purple spots on the skin (non-thrombocytopenic, symmetrically distributed over the lower extremities and buttocks—the universal feature), arthritis/arthralgias (oligoarticular, non-erosive, typically of lower extremity joints), digestive tract involvement (colicky abdominal pain, nausea, vomiting, intussusception, digestive tract bleeding), and kidney involvement (IgA nephropathy—hematuria, proteinuria, high blood pressure, more severe in adults). Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of iga vasculitis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation

- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by skin or kidney biopsy showing leukocytoclastic vasculitis with predominant IgA deposition on immunofluorescence. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

- Treatment typically involves supportive for mild cases
- Corticosteroids are used for significant digestive tract involvement, orchitis, or severe arthritis.
- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways

- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Most people recover well in children (complete recovery in 95%), with relapses in 30%. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 436

IgG4-Related Disease

Overview

IgG4-related disease is a fibroinflammatory condition characterized by tumefactive (mass-like) lesions, lymphoplasmacytic infiltration rich in IgG4-positive plasma cells, storiform scarring, and often elevated serum IgG4 levels, affecting virtually any organ with the most common sites being the pancreas (autoimmune pancreatitis type 1), salivary glands (sialadenitis, Mikulicz disease), orbits (dacryoadenitis, orbital pseudotumor), retroperitoneum (retroperitoneal scarring), thyroid (Riedel thyroiditis), kidneys (tubulointerstitial nephritis), lymph nodes, lungs, and biliary tree, with an incidence of 1–5 per 100,000, a male predominance (3:1), and typically presenting in the sixth decade.

Causes

This condition happens because of a complex interplay of genetic susceptibility, adaptive and innate immune activation, with a Th2-skewed immune response, T follicular helper cells, and overproduction of IL-4, IL-5, IL-13, and TGF- β , driving IgG4 class switching, eosinophilia, and scarring.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features are organ-dependent but commonly include painless swelling of affected organs, obstructive symptoms (jaundice from pancreatic/biliary involvement, hydronephrosis from retroperitoneal scarring), and constitutional symptoms (fatigue, weight loss, low-grade fever).

Emergency warning signs

- Severe or worsening symptoms of ige4-related disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for histologic confirmation: characteristic histopathology (dense lymphoplasmacytic infiltrate with >10 IgG4+ plasma cells per high-power field, IgG4+/IgG+ ratio $>40\%$, storiform scarring, obliterative phlebitis). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves with corticosteroids (first-line, highly effective), with rituximab, azathioprine, mycophenolate, and methotrexate used for steroid-refractory or relapsing disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is good but relapses are common (30–50%) during steroid tapering. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 437

Illness Anxiety Disorder (Hypochondriasis)

Overview

Illness anxiety disorder has a prevalence of 1–5% in the general population, with equal distribution across sexes. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of interoceptive amplification, catastrophic misinterpretation, and memory biases for threat-related health information. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience preoccupation with having or acquiring a serious illness, with somatic symptoms being absent or mild, and high levels of anxiety about health, with the care-seeking type seeking repeated medical evaluations and the care-avoidant type avoiding medical care due to fear of discovering serious illness.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of illness anxiety disorder (hypochondriasis) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include CBT with psychoeducation, thinking and memory restructuring, behavioral experiments, and response prevention. outlook: chronic fluctuating course.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 438

Immune Thrombocytopenic Purpura

Overview

Immune thrombocytopenic purple spots on the skin is an autoimmune disorder characterized by isolated thrombocytopenia (platelet count $<100,000/\mu\text{L}$) due to antibody-mediated platelet destruction and impaired megakaryopoiesis.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience tiny red or purple spots, purple spots on the skin, mucosal bleeding (epistaxis, gingival bleeding, menorrhagia), and in severe cases, intracranial bleeding (rare but life-threatening). Diagnosis is one of exclusion: isolated thrombocytopenia on CBC with normal WBC and hemoglobin, normal peripheral smear (except for thrombocytopenia), and exclusion of other causes. The first-choice treatment options include corticosteroids (prednisone or dexamethasone) and IVIG or anti-D immunoglobulin for rapid platelet increase, with second-line therapies including thrombopoietin receptor agonists (romiplostim, eltrombopag, avatrombopag), rituximab, splenectomy, and fostamatinib (SYK inhibitor).

Emergency warning signs

- Severe or worsening symptoms of immune thrombocytopenic purpura require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

The condition may be primary (idiopathic) or secondary (associated with SLE, antiphospholipid syndrome, infections [HIV, H. pylori], drugs, lymphoproliferative disorders, or vaccination).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is good, with TPO receptor agonists having largely replaced splenectomy as the preferred second-line therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 439

Impetigo

Overview

Impetigo is a highly contagious superficial bacterial skin infection, most common in children aged 2–5 years.

Causes

This condition results from disruption of normal impetigo function.

- This condition happens because of inoculation of bacteria (*Staphylococcus aureus* or *Streptococcus pyogenes*) into minor skin abrasions, insect bites, or areas of dermatitis. It occurs in two forms: non-bullous impetigo (70% of cases), caused by *S. aureus* or *S. pyogenes*, presenting with honey-colored crusted erosions typically on the face and extremities
- And bullous impetigo, caused by exfoliative toxin-producing *S. aureus*, presenting with flaccid, fluid-filled bullae that rupture leaving thin, brown crusts.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of impetigo require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Complications are rare but include cellulitis, lymphangitis, and post-streptococcal glomerulonephritis (from *S. pyogenes*).

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam. Treatment for localized disease is topical mupirocin or retapamulin ointment
- Oral antibiotics (cephalexin, flucloxacillin, or dicloxacillin) are indicated for widespread disease, poor response to topical therapy, or lymphadenopathy.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Most people recover well with treatment
- Adequate hygiene is essential to prevent spread.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 440

Inclusion Body Myositis

Overview

- Inclusion body myositis (IBM) is the most common acquired myopathy in adults over 50 and the most common inflammatory myopathy in men. It is marked by slowly progressive, asymmetric muscle weakness affecting both proximal and distal muscles, with characteristic involvement of wrist and finger flexors (difficulty gripping objects) and quadriceps (difficulty rising from chairs and frequent falls)
- Difficulty swallowing occurs in 30–40%.

Causes

This condition happens because of a combination of inflammatory and degenerative mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of inclusion body myositis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on muscle biopsy showing endomysial inflammation, rimmed vacuoles, and intracellular amyloid deposits
- CK is only mildly elevated.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment typically involves largely supportive, with physical therapy to prevent falls and maintain function
- The disease is refractory to immunosuppressive therapy.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is slowly progressive with significant functional impairment over time. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 441

Indeterminate Colitis

Overview

It accounts for 5–15% of IBD cases. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of overlapping pathological features of both Crohn's disease and ulcerative colitis. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Clinical features are those of colonic inflammation without clear distinguishing characteristics. Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of indeterminate colitis require immediate medical evaluation

Progression and Stages

Indeterminate colitis is inflammatory bowel disease confined to the colon where features of both Crohn's disease and ulcerative colitis are present, and definitive classification is not possible even after pathological review. Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy

- Lymphoma risk in some autoimmune conditions

Diagnosis

Management follows the same principles as UC and CD, with treatment tailored to the predominant clinical phenotype. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation

- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

The outlook varies from person to person and depends on the clinical course. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 442

Infectious Mononucleosis

Overview

Infectious mononucleosis is an acute lymphoproliferative disorder caused by Epstein-Barr virus (EBV, human herpesvirus 4), primarily affecting adolescents and young adults. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of EBV infecting B lymphocytes, triggering a robust CD8+ T-cell response that produces the characteristic atypical lymphocytosis and many of the clinical manifestations, with transmission through saliva (“kissing disease”) and an incubation period of 4–6 weeks. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)

- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience the classic triad of fever, pharyngitis (with severe exudative tonsillitis), and lymphadenopathy (typically posterior cervical), along with fatigue (often profound and prolonged), hepatosplenomegaly, maculopapular rash (10–15%, but up to 90% if amoxicillin or ampicillin is given), periorbital swelling, and palatal tiny red or purple spots. Diagnosis is suggested by lymphocytosis (>50% lymphocytes, with >10% atypical lymphocytes), positive heterophile antibody test (Monospot), and EBV-specific serology (anti-VCA IgM for acute infection, anti-VCA IgG for past infection, anti-EBNA for past infection).
Fever and chills
Fatigue and malaise
Localized pain and inflammation at infection site
Swelling, redness, and warmth (local infection)
Cough and respiratory symptoms (respiratory infections)
Nausea, vomiting, and diarrhea (gastrointestinal infections)
Headache and body aches
Skin rash or lesions
Enlarged lymph nodes
Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of infectious mononucleosis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

In most cases, the outlook is excellent, though splenic rupture is a rare but life-threatening complication.

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Detailed history including exposure, travel, and vaccination status
Physical examination focusing on the affected system
Complete blood count with differential
Microbiological culture of blood, sputum, urine, or wound
PCR and nucleic acid amplification tests for rapid pathogen detection
Antigen detection tests
Serological testing for antibodies (IgM, IgG)
Imaging studies (chest X-ray, CT, ultrasound)
Gram stain and microscopy of clinical specimens
Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer
Vaccination according to recommended schedules
Safe food handling and water purification
Use of personal protective equipment in healthcare settings
Safe sex practices including condom use
Mosquito avoidance (nets, repellents, insecticides)
Respiratory etiquette (covering coughs and sneezes)
Isolation of infected individuals when indicated
Prophylactic antibiotics or antivirals for high-risk exposures
Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings

- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment typically involves symptomatic: rest, hydration, analgesics, and antipyretics, with corticosteroids considered for impending airway obstruction, severe thrombocytopenia, or hemolytic anemia. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global

burden of infectious diseases.

Chapter 443

Infective Endocarditis

Overview

- Infective endocarditis is an infection of the endocardial surface of the heart, most commonly the heart valves
- Acute infective endocarditis is caused by virulent organisms (*Staphylococcus aureus*) and progresses rapidly, while subacute disease is caused by less virulent organisms (viridans group streptococci).

Causes

This condition happens because of microbial infection of damaged or abnormal endocardium. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system
- Travel to endemic areas
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Advanced age
- Cigarette smoking
- Hypertension and diabetes
- Elevated cholesterol levels
- Family history of heart disease

- Obesity and physical inactivity
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fever, chills, night sweats, malaise, weight loss, new or changing cardiac murmur, splinter hemorrhages, Osler's nodes, Janeway lesions, Roth spots, and splenomegaly. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of infective endocarditis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for modified Duke criteria: major criteria (positive blood cultures, evidence of endocardial involvement on echocardiography) and minor criteria (predisposing condition, fever, vascular phenomena, immunologic phenomena). Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on prolonged intravenous antibiotics (4–6 weeks) based on culture and sensitivity
- Surgical intervention is indicated for heart failure, uncontrolled infection, large vegetations, or perivalvular abscess.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the causative organism, patient factors, and timeliness of intervention. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 444

Inflammatory Breast Cancer

Overview

Inflammatory breast cancer is a rare, aggressive form of breast cancer (2–5% of all breast cancers) characterized by rapid onset of breast swelling, redness of the skin, warmth, swelling (“peau d’orange”—orange peel appearance of the skin), and often a palpable mass, resulting from dermal lymphatic invasion by tumor cells. The condition is more common in younger women and African American women, with triple-negative and HER2-positive subtypes being more common than in typical breast cancer. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)

- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of inflammatory breast cancer require immediate medical evaluation

Progression and Stages

Doctors usually diagnose this based on your symptoms and a physical exam, usually at an advanced stage (Stage IIIB or higher). Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more

advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment typically involves neoadjuvant chemotherapy (anthracycline- and taxane-based) followed by mastectomy and post-mastectomy radiation. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

The outlook has improved but remains worse than other breast cancers, with 5-year survival approximately 40–50%. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 445

Influenza

Overview

This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of viral infection of the respiratory epithelium. Transmission occurs via respiratory droplets and aerosols, with an incubation period of 1–4 days. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)

- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of influenza require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- Influenza is an acute respiratory infection caused by influenza viruses (types A and B), belonging to the Orthomyxoviridae family
- Influenza A viruses are further classified by hemagglutinin and neuraminidase surface antigens, with antigenic drift causing seasonal epidemics and antigenic shift potentially causing pandemics.

Complications

- You may experience abrupt onset of fever, chills, muscle pain, headache, malaise, nonproductive cough, sore throat, and nasal congestion
- Complications include viral or secondary bacterial pneumonia, myocarditis, pericarditis, encephalopathy, myositis, and Reye's syndrome (in children given aspirin).
- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using rapid influenza diagnostic tests, molecular assays (RT-PCR), or viral culture. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- Most people recover well for most patients
- Annual influenza vaccination is the most effective preventive measure.
- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules

- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include neuraminidase inhibitors (oseltamivir, zanamivir) and cap-dependent endonuclease inhibitor (baloxavir marboxil), most effective when initiated within 48 hours of symptom onset. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 446

Insomnia in the Context of Medical/Psychiatric Disorders

Overview

Insomnia comorbid with medical or psychiatric conditions is a sleep disorder that represents the most common presentation of insomnia in clinical practice. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of the underlying medical or psychiatric disorder and its effects on sleep regulation. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Family history of mental illness
- Trauma or adverse childhood experiences
- Chronic stress
- Substance use
- Social isolation
- Underlying medical conditions
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures

- Medication use

Signs and Symptoms

Common symptoms

- You may experience difficulty initiating or maintaining sleep in the presence of a comorbid condition.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of insomnia in the context of medical/psychiatric disorders require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, with identification of the underlying medical or psychiatric disorder.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on treatment of the underlying condition, CBT-I adapted for the specific comorbidity, and cautious use of hypnotics mindful of drug-disease and drug-drug interactions
- Depression with comorbid insomnia may benefit from sedating antidepressants (trazodone, mirtazapine, amitriptyline).
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the course of the underlying condition. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 447

Intraductal Papillary Mucinous Neoplasm (IPMN)

Overview

Intraductal papillary mucinous tumor (IPMN) is a mucin-producing epithelial precursor lesion arising within the main pancreatic duct or branch ducts, characterized by intraductal papillary growth, mucin hypersecretion, and progressive cystic ductal ectasia. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)

- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Symptoms often appear as recurrent acute pancreatitis (due to mucin plugging of the ampulla of Vater) or incidental pancreatic cysts on cross-sectional imaging. Mucin extruding from a gaping 'fish-mouth' ampulla on esophagogastroduodenoscopy is pathognomonic. Symptoms vary depending on the severity and extent of the condition Pain or discomfort in the affected area
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of intraductal papillary mucinous neoplasm (ipmn) require immediate medical evaluation

Progression and Stages

IPMNs carry variable cancerous potential, progressing through low-, intermediate-, and high-grade dysplasia to invasive IPMN-associated adenocarcinoma. Classified by ductal location into main-duct (high cancerous risk, > 60%), branch-duct (lower risk), and mixed-type. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment typically involves dictated by Sendai/Fukuoka guidelines: main-duct IPMNs (> 5 mm duct) undergo surgical removal, whereas branch-duct IPMNs are managed conservatively or removed based on high-risk stigmata (solid nodule, main duct dilation, jaundice). Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 448

Intraductal Papilloma

Overview

Intraductal papilloma is a non-cancerous, focal neoplastic growth of the lactiferous ductal epithelium lining a fibrovascular core, representing the most common cause of spontaneous pathological nipple discharge. Solitary intraductal papillomas arise within major subareolar lactiferous ducts in women aged 30–50 years, whereas multiple papillomas originate in peripheral terminal duct lobular units and confer an increased risk of subsequent breast carcinoma. Torsion of the delicate fibrovascular stalk causes localized reduced blood flow, bleeding, and epithelial sloughing.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms often appear as unilateral, spontaneous, bloody (sanguineous) or serous single-duct nipple discharge, occasionally accompanied by a small subareolar palpable nodule.

Emergency warning signs

- Severe or worsening symptoms of intraductal papilloma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors confirm the diagnosis through galactography (ductography), ultrasound, and core needle or microdochectomy biopsy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on complete surgical removal of the involved duct (microdochectomy) to rule out concurrent ductal carcinoma in situ. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 449

Intrauterine Growth Restriction

Overview

Intrauterine growth restriction (IUGR), also termed fetal growth restriction (FGR), is defined as an estimated fetal weight below the 10th percentile for gestational age, often subclassified as symmetric (early onset, involving all biometric parameters) and asymmetric (late onset, with head sparing). It affects 5–10% of pregnancies.

Causes

Causes include maternal factors (high blood pressure, preeclampsia, chronic kidney disease, malnutrition, smoking, alcohol, illicit drugs), placental factors (abnormal placentation, tissue death due to lack of blood supply, velamentous cord insertion), and fetal factors (aneuploidy, congenital infections, multiple gestation).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience subnormal fetal growth on serial ultrasound.

Emergency warning signs

- Severe or worsening symptoms of intrauterine growth restriction require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Diagnosis relies on accurate gestational dating and serial ultrasound measurement of fetal biometry, estimated fetal weight, and amniotic fluid volume
- Umbilical artery Doppler velocimetry identifies fetuses at highest risk.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include antenatal surveillance and delivery when fetal maturity is achieved or when the risks of continued in utero existence outweigh neonatal risks. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause and gestational age at delivery. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 450

Intraventricular Hemorrhage (IVH)

Overview

Intraventricular bleeding (IVH) is bleeding into the germinal matrix and ventricular system of the brain, occurring almost exclusively in preterm infants (<32 weeks gestation). Incidence is 15–25% in very low birth weight infants. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of fragility of the highly vascularized germinal matrix, which is susceptible to rupture from fluctuations in cerebral blood flow.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features may include apnea, bradycardia, seizures, bulging fontanel, hypotension, and anemia.

Emergency warning signs

- Severe or worsening symptoms of intraventricular hemorrhage (ivh) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using cranial ultrasound. IVH is graded on a scale of I–IV. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment typically involves supportive
- Prevention is paramount and includes antenatal corticosteroids, delayed cord clamping, and avoidance of rapid volume expansion or hypotension.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the grade of bleeding. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 451

Intussusception

Overview

Intussusception is the telescoping of one segment of the intestine into an adjacent segment, most commonly ileocecal. It is the most common cause of intestinal obstruction in children aged 3 months to 3 years. The classic triad (not always present) is colicky abdominal pain, vomiting, and red-currant jelly stools (late sign). A sausage-shaped mass may be palpable in the right upper quadrant. Ultrasound demonstrates a target sign or pseudokidney sign.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of intussusception require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves non-surgical reduction by air enema under fluoroscopic guidance, which is successful in 80–95% of cases. Surgical reduction is required for perforation, peritonitis, or failed pneumatic reduction. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with early treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 452

Invasive Lobular Carcinoma (ILC)

Overview

Invasive lobular carcinoma (ILC) is the second most common histological subtype of invasive breast cancer, accounting for 10–15% of all cases. Pathophysiologically, ILC is marked by loss of cellular adhesion mediated by somatic mutation or promoter hypermethylation of the E-cadherin gene (CDH1). Histologically, dyscohesive tumor cells infiltrate the stroma in a single-file, linear array ('Indian file' pattern) surrounding non-neoplastic ducts without eliciting a marked desmoplastic stroma. Metastatic dissemination exhibits a unique tropism for the peritoneum, retroperitoneum, digestive tract, leptomeninges, and ovaries. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes

- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of invasive lobular carcinoma (ilc) require immediate medical evaluation

Progression and Stages

Consequently, ILC frequently presents as an ill-defined thickening or subtle induration rather than a discrete palpable mass, leading to delayed mammographic detection and higher rates of multifocality, bilaterality, and advanced stage at presentation. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ

of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using core biopsy. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)

- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment options include surgical removal, radiation, and long-term endocrine therapy, as most ILCs are strongly ER/PR-positive. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies

- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

outlook is comparable to invasive ductal carcinoma, though late recurrences are more frequent. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 453

Inverted Papilloma

Overview

Inverted papilloma (Schneiderian papilloma) is a non-cancerous but locally aggressive sinonasal epithelial tumor characterized by inward growth (endophytic inversion) of neoplastic epithelium into the underlying stroma rather than outward exophytic growth. Originating predominantly from the lateral nasal wall or maxillary sinus ostium in males aged 40–70 years, it is strongly associated with high-risk human papillomavirus (HPV 16/18) infection. Key pathological features include high recurrence rates (20 – 30%) and a 5–10% risk of synchronous or metachronous transformation into squamous cell carcinoma.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience unilateral nasal obstruction, epistaxis, and pressure pain. CT reveals a soft tissue mass with focal hyperostosis indicating the bony site of origin. Management requires complete using a thin camera tube endonasal removal with drilling of the attachment site.

Emergency warning signs

- Severe or worsening symptoms of inverted papilloma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 454

Iodine Deficiency Disorders

Overview

Iodine deficiency disorders are a group of mineral deficiency disorders affecting nearly 2 billion people, particularly in mountainous regions (Himalayas, Andes, Alps), flood-prone river valleys, and areas with iodine-depleted soils. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition results from disruption of normal iodine deficiency disorders function.

- This condition happens because of deficiency of iodine, an essential component of thyroid hormones (thyroxine, T4
- Triiodothyronine, T3) required for normal growth, development, and metabolism.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of iodine deficiency disorders require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- You may experience goiter (compensatory thyroid enlargement), cretinism (nervous system cretinism with intellectual disability, deaf-mutism, spastic diplegia)
- Myxedematous cretinism with hypothyroidism, growth retardation, coarse facial features), hypothyroidism in older children and adults (fatigue, cold intolerance, weight gain, constipation, dry skin, bradycardia), and pregnancy complications including miscarriage, stillbirth, congenital anomalies, and impaired neurocognitive development.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low urinary iodine concentration (less than 100 µg/L), elevated TSH, low T4, and goiter on palpation or ultrasound.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment focuses on universal salt iodization programs for prevention
- Iodine supplementation (oral potassium iodide 100–200 mg daily) for mild deficiency and levothyroxine for hypothyroidism.
- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with correction of deficiency.

Chapter 455

Iron Deficiency

Overview

Iron deficiency is the most common nutritional deficiency worldwide and the leading cause of anemia, affecting approximately 25% of the global population. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition results from disruption of normal iron deficiency function.

- This condition happens because of inadequate dietary iron intake, poor bioavailability (non-heme iron from plant sources), increased demands (pregnancy, growth, menstruation), and chronic blood loss (digestive tract bleeding, menorrhagia, hookworm)
- Iron is essential for hemoglobin synthesis, myoglobin, and numerous enzymes involved in electron transport and DNA synthesis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features manifest as microcytic hypochromic anemia including fatigue, pallor, weakness, shortness of breath on exertion, tachycardia, headache, and epithe-

lial changes including koilonychia (spoon nails), glossitis, angular stomatitis, and pagophagia (pica for ice).

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of iron deficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low serum ferritin (gold standard), low serum iron, low transferrin saturation, elevated total iron-binding capacity (TIBC), elevated soluble transferrin receptor, and peripheral smear showing microcytic, hypochromic RBCs with pencil-shaped cells and target cells.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on oral iron therapy (ferrous sulfate 325 mg three times daily, equivalent to 195 mg elemental iron per day) as first-line, with response monitored by reticulocytosis at 3–7 days and hemoglobin rise of 1 g/dL per 2–3 weeks

- Parenteral iron is used for intolerance, severe deficiency, malabsorption, or chronic kidney disease
- Treatment of the underlying cause is mandatory.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment.

Chapter 456

Iron Deficiency Anemia

Overview

Iron deficiency anemia is a nutritional deficiency disorder that affects approximately 1.2 billion people worldwide, making it the most common nutritional deficiency globally. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of insufficient iron for hemoglobin synthesis, leading to microcytic, hypochromic anemia, with causes including chronic blood loss (menstruation, GI bleeding from peptic ulcers, colorectal cancer, or NSAID use), inadequate dietary intake (vegetarians/vegans), and increased demand (pregnancy, growth). Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fatigue, pallor, shortness of breath on exertion, pica (craving for non-food items like ice or clay), koilonychia (spoon nails), angular cheilitis, and

glossitis.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of iron deficiency anemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using CBC showing microcytic anemia (low MCV, MCH, MCHC), low serum ferritin (most specific for iron deficiency), low serum iron, elevated TIBC, and low transferrin saturation, with peripheral smear showing hypochromic microcytes, pencil cells, and target cells.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on identifying and treating the underlying cause, and iron supplementation (oral ferrous sulfate 325 mg daily, taken with vitamin C to enhance absorption), with IV iron used for intolerance, malabsorption, or chronic kidney disease. Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate treatment.

Chapter 457

Iron Overdose

Overview

- Iron overdose is a leading cause of poisoning death in children under 6 years due to accidental ingestion of prenatal iron supplements. Toxic dose: elemental iron >20 mg/kg
- Severe toxicity >60 mg/kg.

Causes

This condition happens because of free iron catalyzing hydroxyl radical formation via the Fenton reaction, causing lipid peroxidation, mitochondrial dysfunction, and metabolic acidosis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of iron overdose require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Clinical stages: Stage 1 (0–6 hours): nausea, vomiting, abdominal pain, diarrhea, hematemesis, hematochezia
- Stage 2 (6–24 hours): apparent recovery
- Stage 3 (12–48 hours): metabolic acidosis, seizures, coma, shock, coagulopathy, liver tissue death, lung swelling
- Stage 4 (2–4 weeks): digestive tract obstruction from strictures.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using serum iron level and abdominal X-ray. Treatment: aggressive IV fluids, deferoxamine (IV iron chelator) for metabolic acidosis, shock, or iron level >500 mcg/dL, and whole-bowel irrigation for retained tablets. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 458

Irregular Sleep-Wake Rhythm Disorder

Overview

Irregular sleep-wake rhythm disorder (ISWRD) is a circadian rhythm sleep-wake disorder presenting as a fragmented, disorganized sleep-wake pattern with at least three distinct sleep episodes within a 24-hour period, without a clear circadian rhythm. It is most common in patients with neurodegenerative disorders (Alzheimer disease, dementia), traumatic brain injury, intellectual disability, and in institutionalized elderly populations. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience multiple sleep episodes throughout the day and night with no dominant sleep period.

Emergency warning signs

- Severe or worsening symptoms of irregular sleep-wake rhythm disorder require immediate medical evaluation

Progression and Stages

This condition happens because of deterioration of the SCN or reduced photic input. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for actigraphy or sleep diaries over at least 7 days. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on increasing daytime bright light exposure, structured daytime activities and social schedules, evening melatonin, and avoidance of nighttime light
- Combination therapy (bright light plus melatonin plus structured activity) is most effective.
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person and depends on the underlying condition. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 459

Irritable Bowel Syndrome

Overview

Irritable bowel syndrome (IBS) is a chronic functional digestive tract disorder characterized by recurrent abdominal pain associated with altered bowel habits, in the absence of structural or biochemical abnormalities. It affects approximately 10–15% of the global population and is more common in women. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of visceral hypersensitivity, altered digestive tract motility, gut-brain axis dysfunction, intestinal dysbiosis, low-grade mucosal inflammation, and post-infectious triggers. IBS is subtyped by predominant stool pattern: IBS with constipation (IBS-C), IBS with diarrhea (IBS-D), IBS with mixed bowel habits (IBS-M), and unsubtyped IBS (IBS-U). Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience recurrent abdominal pain, on average at least 1 day per week in the last 3 months, associated with defecation, change in stool frequency, or change in stool form.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of irritable bowel syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on the Rome IV criteria
- Alarm features (rectal bleeding, nocturnal symptoms, unintentional weight loss, family history of colorectal cancer, iron deficiency anemia, onset after age 50) warrant further investigation to exclude organic disease.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves multimodal: dietary modification (low FODMAP diet, soluble fiber supplementation), antispasmodics (hyoscine, dicyclomine, peppermint oil), laxatives for IBS-C (linaclotide, lubiprostone, plecanatide), antidiarrheals for IBS-D (loperamide, eluxadoline, rifaximin), neuromodulators (tricyclic antidepressants, SSRIs), and gut-directed psychological therapies (hypnotherapy, thinking and memory-behavioral therapy). Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Symptoms are chronic but can be managed with multimodal therapy.

Chapter 460

Ischemic Cardiomyopathy

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Ischemic cardiomyopathy is myocardial dysfunction resulting from chronic reduced blood flow or prior heart attack, leading to progressive ventricular dilation and heart failure
- It is the most common cause of heart failure worldwide.

Causes

This condition happens because of myocyte loss, scarring, and adverse ventricular remodeling following coronary artery disease. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity

- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience symptoms of heart failure: shortness of breath on exertion, orthopnea, paroxysmal nocturnal shortness of breath, fatigue, and peripheral swelling. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of ischemic cardiomyopathy require immediate medical evaluation

Progression and Stages

Treatment options include guideline-directed medical therapy (ACE inhibitors, ARBs, ARNI, beta-blockers, mineralocorticoid receptor antagonists, SGLT2 inhibitors, and diuretics) Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

- Coronary revascularization may improve outcomes in selected patients
- Advanced therapies include cardiac resynchronization therapy, implantable cardioverter-defibrillators, and cardiac transplantation.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this based on echocardiography showing reduced left ventricular ejection fraction (less than 40%), regional wall motion abnormalities, and mitral regurgitation. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Your outlook depends on the extent of myocardial dysfunction and response to therapy. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 461

Ischemic Stroke

Overview

Ischemic stroke is a focal brain tissue death due to lack of blood supply caused by occlusion of a cerebral artery by a blood clot or blood clot that travels, accounting for approximately 85% of all strokes. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of thrombotic or embolic arterial occlusion leading to focal brain reduced blood flow and tissue death due to lack of blood supply. Common etiologies include atherosclerotic large-vessel disease, cardioembolism (eg., atrial fibrillation), small-vessel occlusive disease (lacunar stroke), and cryptogenic causes. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Treatment options include intravenous thrombolysis with alteplase (within 4.5 hours) and mechanical thrombectomy for large-vessel occlusions (up to 24 hours in selected patients)
- Secondary prevention includes antithrombotic therapy, statins, and risk factor modification.
- Advanced age

- Cigarette smoking
- Hypertension and diabetes
- Elevated cholesterol levels
- Family history of heart disease
- Obesity and physical inactivity
- Family history and genetic predisposition
- Head trauma or brain injury
- Vascular risk factors (hypertension, diabetes)
- Exposure to environmental toxins
- Chronic stress
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Your symptoms depend on the vascular territory affected: middle cerebral artery occlusion causes contralateral hemiparesis, hemisensory loss, and aphasia (if dominant hemisphere)
- Posterior circulation strokes produce visual field defects, loss of coordination, and crossed deficits.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of ischemic stroke require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries

- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves urgent non-contrast CT to exclude bleeding, followed by CT angiography and perfusion imaging. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on infarct size, location, and timeliness of intervention. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 462

Jet Lag Disorder

Overview

Jet lag disorder is a temporary circadian misalignment caused by rapid travel across multiple time zones.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms include difficulty initiating or maintaining sleep at the destination bedtime, daytime fatigue, impaired thinking and memory performance, digestive tract disturbances, and general malaise. This condition happens because of the circadian system adapting at a rate of approximately one time zone per day. You may experience sleep disturbance and daytime impairment following rapid transmeridian travel.

Emergency warning signs

- Severe or worsening symptoms of jet lag disorder require immediate medical evaluation

Progression and Stages

Severity correlates with the number of time zones crossed (typically three or more) and direction of travel (eastward travel is more difficult because it requires phase-advancing,

which is harder than phase-delaying). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include controlled light exposure (seek morning light when traveling east, evening light when traveling west), melatonin (0.5–5 mg at destination bedtime), and short-term use of hypnotics for sleep initiation. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive
- Symptoms resolve as the circadian system adapts.

Chapter 463

Juvenile Nasopharyngeal Angiofibroma (JNA)

Overview

Juvenile nasopharyngeal angiofibroma (JNA) is a non-cancerous, non-encapsulated, highly vascular, locally invasive tumor arising almost exclusively in adolescent males (ages 10–25 years). Originating near the sphenopalatine foramen, JNA consists of thin-walled blood vessels lacking smooth muscle coats embedded in a fibrous stroma, causing intense bleeding upon minimal manipulation. The tumor expands locally into the nasopharynx, pterygopalatine fossa, orbits, and middle cranial fossa.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Biopsy is strictly contraindicated due to catastrophic bleeding risk.

Emergency warning signs

- Severe or worsening symptoms of juvenile nasopharyngeal angiofibroma (jna) require immediate medical evaluation

Progression and Stages

Symptoms often appear as the classic triad of recurrent painless unilateral epistaxis, unyielding nasal obstruction, and a nasopharyngeal mass, with advanced cases causing facial asymmetry ('frog-face' deformity). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors confirm the diagnosis through contrast CT, MRI, and digital subtraction angiography. Management requires preoperative transarterial embolization followed by complete surgical removal. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 464

Kawasaki Disease

Overview

Kawasaki disease is an acute, self-limited, systemic vasculitis of medium-sized arteries, primarily affecting children under 5 years of age, with a predilection for the coronary arteries, with the highest incidence in Japan (300 per 100,000 children <5 years), and approximately 20–25 per 100,000 in North America and Europe.

Causes

The cause is unknown but suspected to involve an infectious trigger in genetically susceptible individuals, leading to an intense innate and adaptive immune response, with Disease mechanism involving systemic inflammation, endothelial activation, and infiltration of the vessel wall by neutrophils, lymphocytes, and macrophages, with consequent arteritis that can lead to coronary artery aneurysm formation and blood clot formation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of kawasaki disease require immediate medical evaluation

Progression and Stages

Clinical To make the diagnosis, doctors look for fever for at least 5 days plus four of five principal features: bilateral conjunctival injection (non-exudative), oral mucosal changes (strawberry tongue, erythematous lips, pharyngeal redness of the skin), polymorphous rash (usually truncal), extremity changes (redness of the skin and swelling of hands/feet in acute phase, periungual desquamation in subacute phase), and cervical lymphadenopathy (>1.5 cm, typically unilateral). Treatment typically involves intravenous immunoglobulin (IVIG, 2 g/kg single dose) plus high-dose aspirin in the acute phase, reduced to low-dose antiplatelet dose after defervescence. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Complications of coronary arteritis include coronary artery aneurysms (occur in 25% of untreated KD, reduced to $<5\%$ with treatment), heart attack, arrhythmias, and sudden death.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Echocardiography is essential at diagnosis and follow-up.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment, with coronary artery aneurysm rates reduced to <5%. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 465

Kawasaki Disease

Overview

Kawasaki disease is an acute, self-limited vasculitis of unknown cause that predominantly affects children under 5 years (see Chapter 27, Connective Tissue Diseases for comprehensive coverage). Diagnostic criteria include fever for ≥ 5 days plus four of five features: bilateral conjunctival injection, oral mucosal changes, polymorphous rash, extremity changes, and cervical lymphadenopathy (≥ 1.5 cm).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of kawasaki disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

The most serious complication is coronary artery aneurysms, which develop in 20–25% of untreated cases.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves intravenous immunoglobulin (IVIG, 2 g/kg) and high-dose aspirin. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with prompt treatment
- The risk of coronary aneurysms is reduced to less than 5%.

Chapter 466

Keratitis

Overview

Keratitis is an inflammation of the cornea that may be infectious or non-infectious. Infectious keratitis can be bacterial, viral (herpes simplex), fungal, or amoebic (*Acanthamoeba*). Bacterial keratitis, the most common type, results from corneal infection, often associated with contact lens use or trauma.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pain, redness, tearing, decreased vision, and a corneal infiltrate or ulcer
- Herpes simplex keratitis may present as epithelial (dendritic ulcer) or stromal disease.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of keratitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves corneal scraping for Gram stain, culture, and sensitivity. Management requires frequent topical antimicrobial agents appropriate for the suspected organism. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Your outlook depends on the causative organism and timeliness of treatment
- Corneal scarring may require transplantation.

Chapter 467

Keratoconus

Overview

- Keratoconus is a progressive, non-inflammatory ectatic disorder of the cornea characterized by progressive thinning and steepening, resulting in irregular astigmatism and reduced visual acuity
- Onset is typically in adolescence or early adulthood.

Causes

This condition happens because of structural weakening of the corneal stroma.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive visual distortion and decreased visual acuity
- Slit-lamp findings include Munson's sign, Vogt's striae, and corneal iron deposition.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of keratoconus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Management progresses from spectacles and rigid gas-permeable contact lenses to corneal crosslinking to halt progression, intrastromal corneal ring segments, and ultimately keratoplasty in advanced cases. The outlook varies from person to person
- Corneal crosslinking effectively halts progression.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on corneal topography and tomography. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 468

Kidney Cancer (Renal Cell Carcinoma)

Overview

kidney cell carcinoma (RCC) accounts for 2–3% of adult malignancies (400,000 cases/year worldwide). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Clear cell RCC (75%) is the most common subtype, followed by papillary (15%), chromophobe (5%), and collecting duct carcinoma. This condition happens because of VHL inactivation leading to HIF accumulation, VEGF overexpression, and angiogenesis in clear cell RCC. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer

- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience the triad of hematuria, flank pain, and palpable abdominal mass (now uncommon)
- 60% are found incidentally on imaging)
- Paraneoplastic syndromes include erythrocytosis (EPO), hypercalcemia (PTHrP), high blood pressure (renin), and Stauffer syndrome (reversible liver dysfunction).
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of kidney cancer (renal cell carcinoma) require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional

lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment options include partial nephrectomy (nephron-sparing for T1a, T1b tumors), radical nephrectomy for larger tumors, and ablative therapies for small tumors in elderly patients
- Advanced disease doctors typically treat it with tyrosine kinase inhibitors (sunitinib, pazopanib, cabozantinib), immunotherapy (nivolumab + ipilimumab, pembrolizumab + axitinib), mTOR inhibitors (everolimus, temsirolimus), and HIF-2 α inhibitors (belzutifan for VHL-associated RCC). Recovery varies by stage: 5-year survival is 93% for localized, 72% regional, and 14% metastatic.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

- Doctors diagnose this based on CT abdomen with contrast (heterogeneous, enhancing kidney mass), MRI for cystic/complex lesions, and kidney mass biopsy for small tumors
- Staging uses the TNM system.
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)

- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 469

Kikuchi-Fujimoto Disease

Overview

Kikuchi-Fujimoto disease (histiocytic necrotizing lymphadenitis) is a rare, non-cancerous, self-limiting inflammatory disorder of lymph nodes, affecting predominantly young Asian females under 40 years of age. Driven by an aberrant T-cell-mediated immune response to viral triggers (EBV, CMV, HHV-6, HIV) or autoimmune stimuli, histopathology reveals localized cortical and paracortical apoptotic tissue death surrounded by abundant histiocytes, plasmacytoid monocytes, and CD8+ T-lymphocytes, with a characteristic absence of neutrophils.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms often appear as painful or tender cervical lymphadenopathy (most commonly posterior cervical chain), accompanied by low-grade fever, night sweats, fatigue, weight loss, and occasional erythematous skin rashes, closely mimicking lymphoma, tuberculosis, or systemic lupus erythematosus.

Emergency warning signs

- Severe or worsening symptoms of kikuchi-fujimoto disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for excisional lymph node biopsy to distinguish from cancerous lymphoma. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves supportive with analgesics and NSAIDs
- Severe systemic symptoms respond rapidly to a short course of corticosteroids. Spontaneous resolution occurs within 1 to 4 months with minimal recurrence risk (< 5%).
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 470

Kleine-Levin Syndrome

Overview

Kleine-Levin syndrome (recurrent hypersomnia) is a rare disorder characterized by recurrent episodes of severe hypersomnia (16–20 hours of sleep per day), lasting days to weeks, with onset typically during adolescence. Prevalence is estimated at 1–2 per million, with a male predominance (2:1). This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of unknown Disease mechanism, with proposed mechanisms including autoimmune or parainfectious inflammation in the hypothalamus. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience recurrent episodes of severe hypersomnia accompanied by hyperphagia, hypersexuality, mood changes, thinking and memory impairment, and

sometimes psychotic features

- Between episodes, patients have normal sleep, thinking, and function.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of kleine-levin syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam using ICSD-3 criteria.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Management during episodes includes stimulants (modafinil, methylphenidate) for sleepiness, lithium for episode prevention, and mood stabilizers (valproate, lamotrigine).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive
- The disease typically resolves spontaneously over 8–14 years.

Chapter 471

Klinefelter Syndrome (47,XXY)

Overview

Klinefelter syndrome is a chromosomal disorder caused by the presence of an extra X chromosome (47,XXY), with an incidence of 1 in 650 live-born males. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience tall, eunuchoid habitus, gynecomastia, small testes, azoospermia (infertility), sparse body hair, and low testosterone. Intellectual development is usually normal but verbal and language delays are common.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of klinefelter syndrome (47, xxy) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using karyotype.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include testosterone replacement therapy beginning at puberty, breast reduction surgery for gynecomastia, and assisted reproductive technology for infertility. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 472

Kuru

Overview

Kuru is a prion disease historically endemic among the Fore people of Papua New Guinea, transmitted through ritualistic endocannibalism, with peak incidence in the 1950s–1960s, and the last known case reported in 2009. The disease was the first human prion disease shown to be transmissible (to chimpanzees, by Carleton Gajdusek, 1966).

Causes

This condition happens because of the same mechanism as other prion diseases, with the incubation period being inversely related to the dose of infectious material and age at exposure.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of kuru require immediate medical evaluation

Progression and Stages

You may experience three stages: ambulant (tremor and progressive loss of coordination, “kuru” means “to shiver” in Fore), sedentary (severe loss of coordination, slurred speech, loss of independent ambulation), and terminal (complete motor incapacity, primitive reflexes, incontinence, and mutism), with emotional lability and inappropriate euphoria (“kuru laughter”) being characteristic. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam (historical).
Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment typically involves palliative. outlook: the disease was eliminated by the cessation of endocannibalism.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 473

Kwashiorkor

Overview

Kwashiorkor is a form of severe protein-energy malnutrition resulting from inadequate protein intake despite relatively preserved caloric intake. It typically occurs in children aged 1–3 years after weaning, particularly when the diet consists predominantly of starchy, low-protein foods.

Causes

This condition happens because of hypoalbuminemia due to insufficient dietary protein, leading to decreased plasma oncotic pressure and peripheral swelling.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bilateral pitting swelling (beginning in the feet and ascending), hepatomegaly (fatty liver), thinning and depigmentation of hair (flag sign), a characteristic “flaky paint” dermatitis, cheilosis, and stomatitis
- Affected children appear miserable and apathetic. The presence of swelling distinguishes kwashiorkor from marasmus.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of kwashiorkor require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam, supplemented by low serum albumin (less than 25 g/L). Management follows the WHO protocol: treatment of hypoglycemia and hypothermia, empiric antibiotics, correction of electrolyte imbalances, and staged refeeding
- Transition from F-75 to F-100 formula is guided by clinical response.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is good with treatment
- Swelling usually resolves within 1–2 weeks, but long-term thinking and memory deficits may persist.

Chapter 474

Kyphosis

Overview

Kyphosis is excessive anterior curvature of the thoracic spine (normal kyphosis 20–45°).

Causes

Causes include Scheuermann's disease (adolescent, with vertebral wedging), osteoporotic compression fractures (elderly women—"dowager's hump"), ankylosing spondylitis, congenital anomalies, and postural causes. This condition happens because of the specific underlying cause leading to vertebral wedging or collapse. Scheuermann's kyphosis presents in adolescence with rigid thoracic kyphosis and your doctor will diagnose it by the presence of >5° of wedging in three or more consecutive vertebrae on X-ray.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience visible curvature, back pain, and stiffness.

Emergency warning signs

- Severe or worsening symptoms of kyphosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on physical examination and X-ray. Management ranges from observation and physical therapy for mild cases to bracing and surgery for severe deformities. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is good
- Most patients function well with conservative management.

Chapter 475

Labyrinthitis

Overview

Labyrinthitis is an inflammation of the membranous labyrinth of the inner ear that causes both vestibular and auditory dysfunction.

Causes

This condition results from disruption of normal labyrinthitis function.

- This condition happens because of viral infection, often following an upper respiratory infection
- Bacterial labyrinthitis is more severe and can result from otogenic or meningogenic spread.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience feeling of spinning, nausea, vomiting, hearing loss, and ringing in the ears.

Emergency warning signs

- Severe or worsening symptoms of labyrinthitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by audiometry and vestibular testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Treatment for viral labyrinthitis involves supportive care (vestibular suppressants, antiemetics, followed by vestibular rehabilitation)
- Bacterial labyrinthitis requires aggressive antibiotic therapy and may require labyrinthectomy.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive for viral labyrinthitis
- Bacterial labyrinthitis carries a risk of profound hearing loss.

Chapter 476

Lactose Intolerance

Overview

Lactose intolerance is a digestive disorder caused by lactase deficiency leading to an inability to digest lactose, the primary sugar in dairy products, resulting in gastrointestinal symptoms after ingestion. It affects approximately 65% of the global adult population, with wide variation by ethnicity, being most prevalent in East Asian, African, and Native American populations.

Classification

Cause Metabolic/Enzyme deficiency

Organ System Gastrointestinal

Pathophysiology Primary lactase deficiency -> undigested lactose reaches colon -> bacterial fermentation -> gas, bloating, and osmotic diarrhea

Duration Chronic

Onset Gradual (adulthood)

Mode of Transmission Non-communicable (genetic)

Clinical Course Chronic, stable

Severity Mild to Moderate

Extent of Disease Localized

Age Group Adults (most common)

Epidemiology Affects 65% of adults worldwide; varies by ethnicity (5% in Northern Europeans to >90% in East Asians)

ICD-11 Code DC75.0

Causes

Primary lactose intolerance results from the genetically programmed decline in lactase-phlorizin hydrolase enzyme activity in the small intestinal brush border that occurs after weaning in most of the world's population. Secondary lactose intolerance can develop transiently from injury to the small intestinal mucosa due to conditions such as gastroenteritis, celiac disease, or Crohn's disease. Congenital lactase deficiency is a rare autosomal recessive disorder presenting in neonates. This condition develops from

a combination of genetic and environmental factors that disrupt normal body processes. When these normal processes are disrupted, it leads to the symptoms and signs of the condition. Several factors can work together to trigger or make the condition worse. Knowing the specific cause helps your doctor choose the most effective treatment.

Risk Factors

- East Asian, African, Native American, or Hispanic ancestry
- Increasing age (lactase activity naturally declines after age 2–5)
- Premature birth (transient lactase deficiency)
- Conditions damaging the small intestinal mucosa (celiac disease, Crohn’s disease)
- Family history of lactose intolerance

Signs and Symptoms

Early symptoms

- Early manifestations of lactose intolerance may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Abdominal bloating, flatulence, and cramping within 30 minutes to 2 hours after consuming lactose-containing foods
- Watery diarrhea due to osmotic pull of undigested lactose in the colon
- Borborygmi (audible rumbling stomach sounds)
- Nausea, occasionally with vomiting after large lactose loads
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of lactose intolerance may occur

Emergency warning signs

- Severe or worsening symptoms of lactose intolerance require immediate medical evaluation

Progression and Stages

Lactose intolerance in primary form develops gradually after early childhood as lactase enzyme activity naturally declines, becoming clinically noticeable typically in adolescence or early adulthood. The condition is stable and non-progressive, with symptom severity depending on the amount of lactose consumed and the individual's residual lactase activity level.

Complications

- Nutritional deficiencies from avoidance of calcium and vitamin D-rich dairy products
- Increased risk of osteoporosis and fractures if dairy avoidance is not compensated
- Unintentional weight loss in severe cases
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Hydrogen breath test after a 25–50 gram oral lactose load (gold standard)
- Lactose tolerance test measuring serial blood glucose levels
- Lab tests to check how your organs are working
- Imaging tests

Prevention

- Gradual introduction of lactose-containing foods to identify individual tolerance threshold
- Choosing lactose-reduced or lactose-free dairy alternatives
- Go for regular check-ups so any problems are caught early
- Follow your doctor's screening recommendations

Treatment Options

- Dietary lactose restriction based on individual tolerance
- Oral lactase enzyme supplements taken with meals
- Calcium and vitamin D supplementation to prevent bone loss
- Lifestyle changes and self-care
- Surgery when needed

Living with It

- Follow your treatment plan as prescribed

- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Lactose intolerance is a chronic but easily managed condition that does not reduce life expectancy. Your outlook depends on the specific condition, how advanced it is when found, and how well it responds to treatment. Getting treated early generally leads to better results. Many people manage this condition effectively with the right treatment. Regular check-ups are important to monitor your condition and adjust treatment as needed.

Chapter 477

Lambert-Eaton Myasthenic Syndrome (LEMS)

Overview

Lambert-Eaton myasthenic syndrome (LEMS) is a rare autoimmune disorder of the neuromuscular junction characterized by presynaptic impairment of acetylcholine release. Driven by pathogenic autoantibodies directed against voltage-gated P/Q-type calcium channels (VGCC) on presynaptic motor nerve terminals, the condition reduces calcium influx during nerve action potentials, preventing acetylcholine vesicle exocytosis into the synaptic cleft. Strongly associated with underlying malignancy (paraneoplastic LEMS, 50–60% of cases, predominantly small cell lung cancer) or primary autoimmune disease. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms often appear as proximal muscle weakness (affecting lower limbs first, causing difficulty rising from a chair), hyporeflexia or areflexia, and prominent

autonomic dysfunction (dry mouth, erectile dysfunction, constipation). A hallmark feature is transient post-exercise facilitation (improvement of strength and reflexes after brief muscle contraction).

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of lambert-eaton myasthenic syndrome (lems) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with high-titer serum anti-VGCC antibodies and repetitive nerve stimulation (RNS) demonstrating low baseline compound muscle action potential (CMAP) amplitude with dramatic increment ($> 100\%$) following high-frequency stimulation or exercise.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on screening for small cell lung cancer, 3,4-diaminopyridine (amifampridine, enhances ACh release), pyridostigmine, and immunosuppression (prednisone, IVIG). Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 478

Large Bowel Obstruction

Overview

Large bowel obstruction (LBO) is a blockage of the large intestine, less common than SBO.

Causes

The most common cause in Western countries is colorectal cancer (50–60%), followed by diverticulitis strictures and volvulus (sigmoid volvulus in elderly/bedridden patients, cecal volvulus in younger patients). This condition happens because of mechanical or functional obstruction of the colon.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive abdominal distension, constipation, and eventually nausea and vomiting (late in the course).

Emergency warning signs

- Severe or worsening symptoms of large bowel obstruction require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- The right treatment depends on the cause
- Sigmoid volvulus may be decompressed by sigmoidoscopy
- Cancerous LBO typically requires surgical decompression (colostomy or removal) as a palliative or curative measure depending on disease stage.

Complications

Your outlook depends on the underlying cause and presence of complications.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on abdominal X-ray showing a dilated colon with a cutoff point
- CT abdomen/pelvis is diagnostic.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 479

Laryngitis

Overview

Laryngitis is an inflammation of the larynx that presents with hoarseness (dysphonia), most commonly viral in acute cases.

Causes

This condition happens because of vocal cord inflammation due to viral infection, voice overuse (vocal cord nodules), gastroesophageal reflux disease (laryngopharyngeal reflux), smoking, allergies, or chronic infections.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience hoarseness, sore throat, cough, and sometimes low-grade fever
- Chronic laryngitis lasts more than 3 weeks.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of laryngitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using laryngoscopy showing vocal cord redness of the skin and swelling. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include voice rest, hydration, humidification, and addressing underlying causes
- Persistent hoarseness requires laryngoscopy to exclude vocal cord polyps, cysts, or malignancy.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well for acute laryngitis, with resolution within 1–2 weeks. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 480

Lateral Epicondylitis (Tennis Elbow)

Overview

Lateral epicondylitis, commonly known as tennis elbow, is a tendinopathy affecting the common extensor tendon origin at the lateral humeral epicondyle. It is one of the most common upper extremity overuse injuries, affecting 1–3% of the general population annually. Despite its name, only 5–10% of cases occur in tennis players; most are work-related or activity-related.

Classification

Cause Degenerative / Overuse

Organ System Bones / Musculoskeletal

Pathophysiology Angiofibroblastic tendinosis of the extensor carpi radialis brevis (ECRB) tendon origin

Duration Chronic (weeks to months)

Onset Gradual

Mode of Transmission Non-communicable

Clinical Course Chronic, self-limited in many cases

Severity Mild to Moderate

Extent of Disease Localized

Age Group Adults (35–55 years)

Epidemiology Incidence 4–7 per 1000 per year; equal gender distribution

ICD-11 Code FA50.0

Causes

The condition results from repetitive microtrauma and overuse of the wrist extensor muscles, particularly the extensor carpi radialis brevis (ECRB). Histopathology reveals angiofibroblastic tendinosis with collagen disorganization, neovascularization, and mucoid degeneration rather than acute inflammation. Activities involving repetitive wrist extension, forearm pronation, or grip strengthening precipitate the condition.

Risk Factors

- Repetitive wrist extension activities (computer work, painting, carpentry)
- Racquet sports with poor technique or improper equipment
- Manual occupations (plumbers, mechanics, butchers)
- Age 35–55 years
- Smoking
- Obesity
- Previous upper extremity tendinopathy

Signs and Symptoms

Early symptoms

- Early manifestations of lateral epicondylitis (tennis elbow) may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Pain over the lateral elbow, radiating into the forearm
- Pain aggravated by wrist extension against resistance (e.g., lifting, gripping, shaking hands)
- Tenderness directly over the lateral epicondyle
- Pain with passive wrist flexion (stretching the extensors)
- Weakness in grip strength
- Difficulty with simple tasks like holding a cup or turning a doorknob
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of lateral epicondylitis (tennis elbow) may occur

Emergency warning signs

- Severe or worsening symptoms of lateral epicondylitis (tennis elbow) require immediate medical evaluation

Progression and Stages

Tennis elbow typically develops gradually over weeks to months. In most cases, symptoms peak within 6–12 weeks and gradually resolve over 6–24 months with conservative management. A minority of patients develop chronic, persistent symptoms lasting beyond 12 months.

Complications

- Chronic pain and functional limitation
- Reduced grip strength and work capacity
- Tendon rupture (rare)
- Regional pain syndrome
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on history and physical examination
- Positive Cozen test (pain with resisted wrist extension)
- Positive Mill test (pain with passive wrist flexion with elbow extended)
- Tenderness to palpation over the lateral epicondyle
- Ultrasound or MRI to confirm tendinosis and exclude other pathology
- X-ray to rule out intra-articular pathology

Prevention

- Use proper ergonomics and technique in sports and work activities
- Strengthen forearm extensor and flexor muscles
- Take frequent breaks during repetitive activities
- Use appropriate equipment (proper grip size, racquet string tension)
- Go for regular check-ups so any problems are caught early

Treatment Options

- Activity modification and relative rest from aggravating activities
- Physical therapy with eccentric strengthening exercises
- Nonsteroidal anti-inflammatory drugs (NSAIDs) for pain relief
- Counterforce bracing (tennis elbow strap) to reduce tendon strain
- Corticosteroid injections for short-term relief (controversial long-term)
- Platelet-rich plasma (PRP) injections (evidence for long-term benefit)
- Extracorporeal shockwave therapy for chronic cases
- Surgery (open or arthroscopic release) for refractory cases

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

The prognosis for tennis elbow is favorable, with 80–90% of patients improving with conservative management within 12–18 months. Corticosteroid injections provide rapid short-term relief but may increase recurrence risk. Surgical release is effective in 85–90% of refractory cases. Prevention through proper ergonomics and conditioning is important, particularly for those in high-risk occupations.

Chapter 481

Lead Poisoning (Plumbism)

Overview

Lead poisoning (plumbism) is a cumulative toxicant affecting multiple organ systems. Sources include lead-based paint (most common in children), contaminated water, soil, industrial exposure, traditional cosmetics, and imported products. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of lead impairing heme synthesis, disrupting calcium homeostasis, and causing oxidative stress

- In children, lead crosses the blood-brain barrier and impairs neuronal migration, synaptogenesis, and myelination. No safe blood lead level exists
- The CDC reference level is 3.5 mcg/dL.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features in children include asymptomatic (most cases), neurocognitive deficits, developmental delay, and at very high levels: encephalopathy, seizures, coma, and death. In adults: peripheral neuropathy, abdominal pain, arthralgias, fatigue, headache, high blood pressure, nephropathy, and anemia.

Emergency warning signs

- Severe or worsening symptoms of lead poisoning (plumbism) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using venous blood lead level. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include removal from exposure source and chelation therapy: succimer (DMSA, oral) for asymptomatic children with levels ≥ 45 mcg/dL
- BAL plus calcium disodium EDTA for severe toxicity.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with early intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 482

Leishmaniasis

Overview

Leishmaniasis is a protozoan infection caused by *Leishmania* species, transmitted by sandflies, with a clinical spectrum ranging from localized cutaneous disease to lethal visceral leishmaniasis (Kala-azar), with 1.5 million new cases annually and 20,000–30,000 deaths, endemic in tropical and subtropical regions.

Causes

This condition happens because of the sandfly injecting promastigotes into the skin, which are phagocytosed by macrophages and convert to amastigotes, replicating intracellularly and spreading to the reticuloendothelial system.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience visceral leishmaniasis: fever (often double quotidian), weight loss, massive splenomegaly, hepatomegaly, lymphadenopathy, and pancytopenia
- Cutaneous leishmaniasis: single or multiple painless ulcer(s) at sandfly bite sites with raised, indurated borders
- Mucocutaneous leishmaniasis: destructive lesions of the nasopharynx.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of leishmaniasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using microscopic identification of amastigotes in tissue aspirates or biopsies. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include liposomal amphotericin B (first-line for visceral), pentavalent antimonials, and miltefosine. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the clinical form and treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 483

Leprosy (Hansen's Disease)

Overview

Leprosy is a chronic granulomatous infection caused by *Mycobacterium leprae*, an obligate intracellular, acid-fast bacillus, with approximately 200,000 new cases reported annually, primarily in India, Brazil, and Indonesia, transmitted through prolonged, close contact with untreated patients, and a long incubation period (2–10 years, up to 40 years).

Causes

This condition happens because of *M. leprae* invading Schwann cells and macrophages, with the clinical spectrum correlating with the host immune response, ranging from tuberculoid leprosy (strong cell-mediated immunity, few bacilli) to lepromatous leprosy (weak cell-mediated immunity, numerous bacilli).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience hypopigmented, anesthetic skin patches, peripheral nerve thickening, and neuropathy, with tuberculoid leprosy presenting with 1–3 well-demarcated anesthetic macules/plaques, and lepromatous leprosy presenting with diffuse, symmetric skin infiltration with numerous nodules and plaques (leonine facies), eyebrow loss, and progressive symmetric peripheral neuropathy.

Emergency warning signs

- Severe or worsening symptoms of leprosy (hansen's disease) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam (anesthetic skin patches, nerve enlargement), with skin slit-smear for acid-fast bacilli. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include rifampin, dapsone, and clofazimine (multidrug therapy). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 484

Leptospirosis

Overview

Leptospirosis is a zoonotic spirochetal infection caused by pathogenic serovars of *Leptospira interrogans* sensu lato, with more than 1 million cases annually worldwide, highest incidence in tropical and subtropical regions, and transmission through contact with water or soil contaminated by urine of infected animals.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of leptospirosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- This condition happens because of *Leptospira* entering through mucous membranes or

abraded skin, multiplying in the bloodstream (leptospiremic phase), then localizing to tissues including liver, kidney, meninges, and eye. You may experience anicteric leptospirosis (80–90%): biphasic illness with Phase 1 (leptospiremic: abrupt fever, rigors, severe headache, muscle pain, conjunctival suffusion) and Phase 2 (immune: aseptic meningitis, uveitis, rash)

- Weil's disease (severe icteric leptospirosis, 5–10%): jaundice, kidney failure, bleeding, and myocarditis.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using PCR of blood or urine, culture, and serology. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include doxycycline or amoxicillin for mild disease, and ceftriaxone or penicillin G for severe disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well for mild disease, but mortality is higher with Weil's disease. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 485

Lipoprotein(a) Disorders

Overview

- Lp(a) levels are predominantly genetically determined (by the LPA gene) and are largely unaffected by diet or lifestyle
- The prevalence of elevated Lp(a) is approximately 20% of the population worldwide.

Causes

This condition happens because of apolipoprotein(a) component sharing homology with plasminogen and having antifibrinolytic properties, also promoting vascular inflammation and foam cell formation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience premature cardiovascular disease and a family history of early heart disease or aortic stenosis, with no other specific clinical findings.

Emergency warning signs

- Severe or worsening symptoms of lipoprotein(a) disorders require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using measurement of Lp(a) mass or molar concentration (once in a lifetime, as levels are stable). Standard lipid-lowering therapies do not significantly lower Lp(a)

- Niacin and PCSK9 inhibitors produce modest reductions
- Investigational therapies (pelacarsen, olpasiran) reduce Lp(a) by 80–90%.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 486

Lithium Toxicity

Overview

Lithium toxicity is a pharmaceutical overdose involving a monovalent cation used for bipolar disorder with a narrow therapeutic index (0.6–1.2 mEq/L). Toxicity occurs from acute ingestion, acute-on-chronic overdose, or chronic accumulation (often due to dehydration, kidney impairment, or drug interactions).

Causes

This condition happens because of lithium substituting for sodium in excitable cells, altering neurotransmitter release, and impairing kidney concentrating ability.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: acute toxicity presents with digestive tract symptoms and mild nervous system symptoms (tremor, loss of coordination, involuntary eye movements)
- Chronic toxicity presents predominantly with neurotoxicity (confusion, slurred speech, loss of coordination, hyperreflexia, clonus, parkinsonism), with severe cases progressing to seizures, coma, and permanent cerebellar injury.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of lithium toxicity require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using serum lithium level (interpreted with caution—chronic toxicity can occur at therapeutic levels). Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include IV fluids (normal saline) to correct volume depletion and enhance lithium clearance
- Hemodialysis is indicated for severe toxicity.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with prompt treatment
- Chronic toxicity may cause permanent nervous system injury.

Chapter 487

Lobular Carcinoma In Situ (LCIS)

Overview

Lobular carcinoma in situ (LCIS) is a non-invasive neoplastic growth of small, dyscohesive atypical cells completely filling and expanding the lobules and terminal ductules of the breast. Like invasive lobular carcinoma, LCIS is driven by loss of E-cadherin expression (CDH1 gene inactivation). Unlike DCIS, LCIS does not produce clinical masses or mammographic calcifications and is typically discovered incidentally on core needle biopsy performed for unrelated non-cancerous lesions. LCIS functions primarily as a powerful non-palpable, non-calcified biomarker indicating a 9- to 10-fold increased risk of developing invasive carcinoma in either breast (bilateral risk). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes

- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of lobular carcinoma in situ (lcis) require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases

to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

To make the diagnosis, doctors look for histological confirmation with E-cadherin immunohistochemical staining. Management options include active surveillance with annual screening mammography and MRI, selective estrogen receptor modulators (tamoxifen, raloxifene) for chemoprevention, or bilateral prophylactic mastectomy for high-risk BRCA or family history carriers. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology

- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 488

Long QT Syndrome

Overview

- Long QT syndrome is a group of inherited cardiac channelopathies characterized by prolongation of the QT interval on ECG and predisposition to torsades de pointes ventricular arrhythmias and sudden death
- Over 17 genes have been identified, with LQT1 (KCNQ1), LQT2 (KCNH2), and LQT3 (SCN5A) accounting for the majority.

Causes

This condition happens because of mutations in cardiac ion channels affecting ventricular repolarization. Triggers vary by genotype: exercise (LQT1), auditory stimuli and emotional stress (LQT2), and rest or sleep (LQT3). Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience syncope, seizures, and sudden cardiac death.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of long qt syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using ECG (corrected QT interval greater than 470 ms in men, greater than 480 ms in women is highly suggestive), genetic testing, and clinical criteria (Schwartz score).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include beta-blockers (nadolol preferred), avoidance of QT-prolonging drugs, and implantable cardioverter-defibrillator for high-risk patients
- Left cardiac sympathetic denervation is an option for drug-refractory cases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate management and avoidance of triggers.

Chapter 489

Low Vision

Overview

- Low vision is a significant, uncorrectable loss of vision that interferes with daily activities, defined as best-corrected visual acuity of 20/70 or worse, or a visual field of 20 degrees or less
- It is not blindness; people with low vision retain useful vision and can be helped with rehabilitation
- It affects millions of older adults worldwide and is most often caused by age-related eye disease

Causes

Low vision happens because of conditions that cannot be fully corrected with glasses, surgery, or medication. Common causes include age-related macular degeneration, glaucoma, diabetic retinopathy, cataracts, and retinal disease such as retinitis pigmentosa or retinal vein occlusion. Stroke affecting the visual pathways can also cause low vision.

Risk Factors

- Age over 65 years
- Diabetes and high blood pressure
- Smoking
- Family history of eye disease
- Uncorrected refractive error over long periods
- Eye trauma or prior eye surgery complications

Signs and Symptoms

Common symptoms

- Blurred or distorted vision that glasses cannot fix
- Difficulty reading, writing, driving, or recognizing faces
- Loss of peripheral (side) vision
- Difficulty with glare and low-light conditions
- Central blind spots or tunnel vision
- Impaired depth perception and increased risk of falls

Emergency warning signs

- Sudden or rapidly worsening vision loss requires immediate evaluation

Progression and Stages

Low vision may be stable, progressive, or episodic depending on the underlying cause. Age-related macular degeneration and glaucoma tend to progress gradually, while stroke or vein occlusion may cause sudden loss. The functional impact depends on the residual vision, the specific tasks affected, and the effectiveness of rehabilitation.

Complications

- Increased risk of falls, fractures, and injury
- Difficulty managing medications and daily tasks
- Depression, social isolation, and reduced independence
- Loss of driving ability
- Decreased quality of life

Diagnosis

- Comprehensive eye examination including visual acuity and visual field testing
- Low-vision assessment to determine functional needs
- Optical coherence tomography (OCT), fundus photography, and other imaging depending on the cause
- Evaluation of the underlying disease by an ophthalmologist
- Referral to low-vision and rehabilitation services

Prevention

- Early treatment of the underlying eye disease
- Control of diabetes, blood pressure, and cholesterol
- Regular comprehensive eye examinations, especially after age 50
- Quitting smoking and maintaining a healthy lifestyle
- Fall-prevention measures in the home

Treatment Options

- Low-vision aids: magnifiers, telescopic lenses, high-contrast and large-print materials
- Electronic aids: screen readers, video magnifiers (CCTV), speech-to-text software
- Occupational therapy and low-vision rehabilitation
- Lighting and contrast optimization in the home
- Mobility training and orientation training for severe cases

- Treatment of the underlying eye disease where possible

Living with It

- Work with a low-vision specialist to find the right aids
- Improve home lighting, contrast, and organization
- Seek emotional support and connect with low-vision support groups
- Continue participating in hobbies with adapted techniques
- Consider counseling for depression or anxiety

Outlook

The outlook depends on the underlying cause, but most people with low vision can maintain independence and good quality of life with rehabilitation and assistive technology. Progression varies with the specific disease; early and ongoing support significantly improves outcomes.

Chapter 490

Ludwig Angina

Overview

Ludwig angina is a rapidly spreading, life-threatening bilateral gangrenous cellulitis of the submandibular, sublingual, and submental spaces, carrying a high risk of fatal airway compromise. Pathophysiologically, > 80% of cases originate from odontogenic infections, most commonly infected second or third lower molar teeth whose roots extend below the mylohyoid muscle line. Polymicrobial dental flora (*Streptococcus viridans*, anaerobes, *Bacteroides*, *Fusobacterium*) cross tissue planes without lymphadenopathy or abscess cavitation, causing massive inflammatory swelling that elevates and displaces the tongue posteriorly against the soft palate and hypopharynx. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease

- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Symptoms often appear as severe neck pain, fever, trismus, difficulty swallowing, drooling, wood-like ('woody') induration of the submandibular space, and progressive stridor. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of ludwig angina require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia

- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, confirmed by neck CT once airway stability is secured. Management prioritizes emergency airway control (fiberoptic nasal intubation or awake tracheostomy) combined with IV broad-spectrum antibiotics (ampicillin-sulbactam or carbapenems) and surgical incision and removal of fluid of deep tissue spaces. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal

medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 491

Lung Abscess

Overview

Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

- A lung abscess is a necrotizing infection of the lung parenchyma leading to a cavitary lesion containing purulent material
- It may be primary (removal by suction pneumonia in patients with impaired consciousness, poor dentition, or swallowing disorders) or secondary (bronchial obstruction, hematogenous seeding).

Causes

This condition happens because of microbial infection leading to lung tissue death. Common organisms include anaerobic bacteria, *Staphylococcus aureus*, and *Klebsiella pneumoniae*. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience fever, productive cough with foul-smelling sputum, weight loss, and night sweats. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of lung abscess require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock

- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using chest X-ray showing a thick-walled cavity, often with an air-fluid level. Management requires prolonged intravenous antibiotics (6–8 weeks), bronchoscopic removal of fluid for large or refractory abscesses, and surgical removal as a last resort. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification

- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

The outlook is good with appropriate antibiotic therapy. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring

long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 492

Lung Cancer

Overview

This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

- Lung cancer is the leading cause of cancer-related death worldwide, with approximately 1.8 million deaths annually
- Non-small cell lung cancer accounts for 85% of cases and includes adenocarcinoma (most common subtype, often in non-smokers, with driver mutations in EGFR, ALK, ROS1, KRAS), squamous cell carcinoma (central, strongly smoking-related), and large cell carcinoma
- Small cell lung cancer accounts for 15%, is strongly associated with smoking, and has aggressive behavior with early metastasis.

Causes

This condition happens because of cancerous transformation of bronchial epithelial cells driven by carcinogen exposure. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age

- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience cough, hemoptysis, shortness of breath, chest pain, weight loss, and post-obstructive pneumonia. Persistent lump or mass in the affected area
Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of lung cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment for early-stage disease is surgical removal with adjuvant chemotherapy
- Advanced disease doctors typically treat it with platinum-based chemotherapy, immunotherapy, and targeted therapy for driver mutations
- Small cell lung cancer doctors typically treat it with chemoradiation and prophylactic cranial irradiation.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Diagnosis typically involves CT-guided biopsy, bronchoscopy, or mediastinoscopy, followed by molecular testing for driver mutations. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology

- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 493

Lyme Disease (*Borrelia burgdorferi*)

Overview

Lyme disease is a tick-borne spirochetal infection caused by *Borrelia burgdorferi* sensu stricto (US) and *B. afzelii* and *B. garinii* (Europe, Asia), and is the most common vector-borne disease in the United States with approximately 500,000 cases annually, transmitted by *Ixodes* ticks requiring a 24–48 hour blood meal for transmission.

Causes

This condition happens because of *B. burgdorferi* being injected into the skin with tick saliva, replicating locally, and disseminating hematogenously.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of lyme disease (*borrelia burgdorferi*) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Clinical features occur in three stages: Stage 1 (early localized, 3–30 days): redness of the skin migrans—an expanding annular rash with central clearing (“bull’s-eye” lesion) at the tick bite site, often with fever, fatigue, muscle pain, and headache
- Stage 2 (early disseminated, weeks to months): multiple secondary redness of the skin migrans lesions, flu-like symptoms, migratory joint pain, cranial nerve palsies, meningitis, and carditis
- Stage 3 (late, months to years): Lyme arthritis, acrodermatitis chronica atrophicans, and neuroborreliosis.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using serology using two-tier testing (EIA followed by Western blot).
Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include doxycycline for early disease, ceftriaxone for nervous system disease, and oral antibiotics for Lyme arthritis. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate antibiotic treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 494

Lymphogranuloma Venereum (LGV)

Overview

Lymphogranuloma venereum is a sexually transmitted infection caused by *Chlamydia trachomatis* serovars L1, L2, and L3, which are invasive and disseminate to regional lymph nodes, endemic in Africa, Southeast Asia, and the Caribbean, with outbreaks among MSM in Europe, North America, and Australia.

Causes

This condition happens because of LGV serovars infecting epithelial cells and macrophages, then disseminating through lymphatics, causing lymphangitis and perilymphangitis with granulomatous inflammation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of lymphogranuloma venereum (lgv) require immediate medical evaluation

Progression and Stages

You may experience an incubation of 3–30 days, with primary stage: painless papule or ulcer at the inoculation site. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Secondary stage (2–6 weeks): painful inguinal lymphadenopathy with the “groove sign” characteristic
- Tertiary stage: genital elephantiasis and rectal strictures.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using *C. trachomatis* PCR from ulcer, rectal swab, or bubo aspirate, with genotyping distinguishing LGV serovars. Comprehensive medical history and physical examination. Laboratory tests to assess organ function and rule out other conditions. Imaging studies to visualize affected structures. Specialized testing depending on the affected system. Consultation with relevant specialists. Monitoring of disease progression over time.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include doxycycline 100 mg twice daily for 21 days. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 495

Lynch Syndrome

Overview

Lynch syndrome (hereditary nonpolyposis colorectal cancer, HNPCC) is an autosomal dominant cancer predisposition syndrome caused by germline mutations in DNA mismatch repair genes (MLH1, MSH2, MSH6, PMS2) or EPCAM deletions. It accounts for 2–4% of all CRCs, with a lifetime CRC risk of 40–80%. Associated cancers include endometrial (highest risk after CRC), ovarian, gastric, urinary tract, hepatobiliary, and small bowel cancers. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of defective mismatch repair leading to microsatellite instability (MSI) or mismatch repair deficiency (dMMR). Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience early-onset colorectal cancer and increased risk of multiple primary cancers.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of lynch syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on Amsterdam criteria, revised Bethesda guidelines, immunohistochemistry of tumor tissue, and germline genetic testing.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

- Treatment options include surveillance colonoscopy starting at age 20–25 (or 2–5 years before the earliest CRC in the family) every 1–2 years
- Prophylactic surgery may be considered.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is improved with surveillance
- Immunotherapy with pembrolizumab is particularly effective for MSI-high/dMMR cancers.

Chapter 496

Lysosomal Storage Diseases

Overview

- Lysosomal storage diseases (LSDs) are a group of over 50 inherited metabolic disorders caused by deficiency of specific lysosomal hydrolases, resulting in progressive accumulation of undegraded substrates within lysosomes. Gaucher disease is caused by glucocerebrosidase deficiency, presenting with hepatosplenomegaly, bone pain (Erlenmeyer flask deformity of the distal femur), anemia, thrombocytopenia, and Gaucher cells
- Enzyme replacement therapy and substrate reduction therapy are effective. Niemann-Pick disease is caused by sphingomyelinase deficiency (types A and B) causing accumulation of sphingomyelin in macrophages
- Type A presents with severe neurovisceral disease and a cherry-red spot on the macula. Tay-Sachs disease is caused by hexosaminidase A deficiency, presenting in infancy with progressive neurodegeneration, hypotonia, exaggerated startle response, cherry-red spot, and blindness. Metachromatic leukodystrophy is caused by arylsulfatase A deficiency, causing accumulation of sulfatides with progressive demyelination.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of lysosomal storage diseases require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Fabry disease is an X-linked α -galactosidase A deficiency causing accumulation of globotriaosylceramide, presenting with acroparesthesias, angiokeratomas, corneal verticillata, hypohidrosis, progressive kidney failure, hypertrophic cardiomyopathy, and stroke
- Enzyme replacement therapy and oral chaperone therapy slow disease progression.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 497

Macular Edema

Overview

- Macular edema is the accumulation of fluid in the central retina (macula), causing swelling and blurred central vision
- Diabetic macular edema (DME) is the most common cause and a leading cause of vision loss in working-age adults
- Cystoid macular edema (CME) often follows eye surgery, retinal vein occlusion, or inflammation

Causes

This condition happens because of breakdown of the blood–retinal barrier, allowing fluid and protein to leak from retinal capillaries into the macula. It is caused by diabetes (diabetic retinopathy), retinal vein occlusion, uveitis, cataract surgery (Irvine–Gass syndrome), age-related macular degeneration, and some medications such as prostaglandin eye drops or niacin.

Risk Factors

- Diabetes with poorly controlled blood sugar and hypertension
- Retinal vein occlusion
- Intraocular inflammation (uveitis)
- Recent eye surgery, especially complicated cataract surgery
- High myopia
- Medications that cause fluid retention in the retina

Signs and Symptoms

Common symptoms

- Blurred or wavy central vision
- Colors may appear washed out or distorted
- Difficulty reading and seeing fine detail
- Fluctuating vision, often worse in the morning
- Peripheral vision is usually preserved

Emergency warning signs

- Sudden, severe, or worsening central vision loss requires urgent eye evaluation

Progression and Stages

Macular edema may be focal or diffuse. Untreated DME tends to persist and progress, with fluid accumulation causing retinal thickening and eventual photoreceptor damage and permanent central vision loss. With early treatment, many people recover much of their vision, but chronic edema can lead to scarring and irreversible loss.

Complications

- Permanent central vision loss from photoreceptor damage
- Macular fibrosis or scarring
- Formation of cysts in the retina
- Reduced quality of life and impaired daily function
- Higher risk in diabetic patients of coexisting eye disease

Diagnosis

- Dilated fundus examination
- Optical coherence tomography (OCT) measures retinal thickness and fluid
- Fluorescein angiography or OCT angiography to identify leaking vessels
- Visual acuity testing
- Blood tests for glucose (HbA1c) in diabetic patients

Prevention

- Strict control of blood sugar, blood pressure, and cholesterol in diabetes
- Routine dilated eye screening for diabetics
- Timely treatment of retinal vein occlusion and inflammation
- Regular eye examinations after eye surgery
- Careful selection of medications under medical supervision

Treatment Options

- Intravitreal anti-VEGF injections (ranibizumab, aflibercept, bevacizumab) are first-line therapy for DME and vein-occlusion edema
- Intravitreal or periocular corticosteroids (dexamethasone, triamcinolone) for inflammatory or refractory edema
- Focal/grid laser photocoagulation for DME
- Treatment of the underlying cause, such as blood pressure control or anti-

inflammatory therapy

- Surgery (vitrectomy) rarely for specific cases

Living with It

- Follow your injection and monitoring schedule faithfully
- Maintain good control of diabetes, blood pressure, and cholesterol
- Use good lighting and magnifiers for reading
- Report any sudden visual changes promptly

Outlook

The outlook has improved dramatically with anti-VEGF therapy. Most patients with DME or vein-occlusion edema gain vision with treatment, though repeat injections are usually needed. Outcomes are best with early detection and treatment; chronic untreated edema carries a poorer prognosis.

Chapter 498

Macular Hole

Overview

- A macular hole is a full-thickness defect in the center of the retina (the macula), which is responsible for sharp central vision
- It affects roughly 3 in 1000 people over 55 and is more common in women
- It causes progressive blurring and distortion of central vision and can lead to permanent vision loss if untreated

Causes

This condition happens because of abnormal vitreous traction on the fovea. As the vitreous gel ages and separates from the retina, it can pull on the macula, tearing a hole in it. Associated causes include high myopia, trauma, and conditions causing abnormal vitreoretinal adhesion such as proliferative vitreoretinopathy.

Risk Factors

- Age over 55 years
- Female sex
- High myopia
- Eye trauma
- Prior posterior vitreous detachment or retinal surgery
- Cystoid macular edema

Signs and Symptoms

Common symptoms

- Blurred, distorted central vision (straight lines look wavy)
- A central dark or gray spot (scotoma)
- Difficulty reading, driving, or recognizing faces
- Peripheral vision is usually preserved

Emergency warning signs

- Sudden loss of central vision or a new central blind spot requires urgent eye examination

Progression and Stages

Macular holes are staged from early to full thickness.

- Stage 1: impending hole with cyst formation, no full-thickness defect
- Stage 2: small full-thickness hole develops
- Stage 3: larger full-thickness hole
- Stage 4: full-thickness hole with complete posterior vitreous detachment

Without surgery, the hole usually enlarges and central vision deteriorates progressively; up to half of stage 1 holes may resolve spontaneously with vitreous separation.

Complications

- Progressive central vision loss
- Retinal detachment (in a minority of cases)
- Metamorphopsia and central scotoma
- Reduced quality of life and independence
- Post-surgical complications such as elevated eye pressure or cataract

Diagnosis

- Dilated fundus examination
- Optical coherence tomography (OCT) is the definitive test, confirming the hole and its stage
- Visual acuity testing
- Watzke-Allen test (slit-lamp testing for a central scotoma) may aid diagnosis

Prevention

- Macular holes are largely not preventable
- Early recognition and prompt referral of new central visual symptoms
- Routine eye examinations, especially for older adults
- Protection from eye trauma

Treatment Options

- Observation with monitoring for small or early-stage holes
- Vitrectomy surgery with gas tamponade is the standard treatment, closing the hole in most cases

- Face-down positioning after surgery to hold the gas bubble against the macula
- Occasionally, enzymatic vitreolysis or repeated surgery for persistent holes
- Postoperative cataract extraction is commonly needed within a few years

Living with It

- Follow positioning instructions carefully after surgery to maximize hole closure
- Do not fly or travel to high altitude while a gas bubble is present
- Use magnification and good lighting while vision recovers
- Attend scheduled follow-up visits

Outlook

With vitrectomy surgery, the hole closes in roughly 90% of cases, and most patients gain significant visual improvement, though full normal vision is uncommon. Early-stage holes and shorter duration of symptoms are associated with better outcomes.

Chapter 499

Macular Pucker

Overview

- Macular pucker, also called epiretinal membrane or cellophane maculopathy, is a thin scar-like layer that forms on the surface of the retina over the macula
- It is a common condition in people over 50, affecting about 5–10% of older adults
- It causes mild-to-moderate visual distortion (metamorphopsia) and blurring but rarely causes severe vision loss

Causes

This condition happens because glial cells migrate onto the retinal surface and form a contractile membrane over the macula. The membrane contracts, wrinkling and distorting the underlying retina. It can be primary (idiopathic, age-related) or secondary to posterior vitreous detachment, retinal tears or detachment, inflammation, trauma, or eye surgery (especially cataract and retinal surgery).

Risk Factors

- Age over 50 years
- Prior posterior vitreous detachment
- Eye surgery, especially retinal detachment repair
- Eye trauma
- Intraocular inflammation (uveitis)
- Diabetes and vascular retinal disease

Signs and Symptoms

Common symptoms

- Mild blurred or distorted central vision (straight lines appear wavy)
- Slight difficulty reading fine print
- Mildly reduced visual acuity
- Most cases are mild and many people have no noticeable symptoms

Emergency warning signs

- Sudden worsening of vision, new flashes of light, or a shadow or curtain in the vision

Progression and Stages

Macular pucker develops slowly over months to years. Most cases remain mild and stable, requiring no treatment. The membrane may contract progressively in some people, causing increasing distortion; severe cases with significant metamorphopsia or reduced acuity are the ones considered for surgery. Spontaneous improvement is rare but can occur.

Complications

- Progressive distortion and blurring of central vision
- Macular edema beneath the membrane
- Reduced quality of life from visual distortion
- Rarely, the membrane may cause a macular hole or tractional retinal detachment

Diagnosis

- Dilated eye examination
- Optical coherence tomography (OCT) is the key test, showing the membrane and retinal wrinkling
- Fluorescein angiography if vascular leakage or macular edema is suspected
- Visual acuity testing and Amsler grid testing to document distortion

Prevention

- Macular pucker is usually not preventable
- Managing underlying conditions such as diabetes and inflammation
- Prompt treatment of retinal tears and detachments
- Routine eye examinations to detect early changes

Treatment Options

- Observation for mild cases with regular monitoring
- Vitrectomy with membrane peeling for symptomatic cases with significant distortion or reduced vision
- Treatment of any associated macular edema (e.g., anti-VEGF or steroid therapy)
- Low-vision aids for persistent visual symptoms
- Most people do not require surgery; the decision is based on functional impairment

Living with It

- Use good lighting and magnification aids for reading
- Follow your eye specialist's monitoring schedule
- Report any sudden visual changes promptly
- Seek support from low-vision services if daily activities are affected

Outlook

The outlook is generally good. Most macular puckers remain mild and stable without treatment. When surgery is performed, a majority of patients experience improved distortion and vision, though the membrane can occasionally recur.

Chapter 500

MDMA (Ecstasy) Toxicity

Overview

MDMA (3,4-methylenedioxymethamphetamine) is a recreational empathogen with stimulant and hallucinogenic properties. It promotes serotonin release and inhibits reuptake.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience euphoria, increased energy, empathy, and sensory enhancement, but also hyperthermia (often severe), hyponatremia (from SIADH plus excessive water intake), seizures, rhabdomyolysis, DIC, acute liver failure, and serotonin syndrome. Hyponatremia can be severe ($\text{Na} < 120 \text{ mEq/L}$) and cause cerebral swelling.

Emergency warning signs

- Severe or worsening symptoms of mdma (ecstasy) toxicity require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment: supportive care—cooling for hyperthermia, hypertonic saline for severe hyponatremia with nervous system symptoms, benzodiazepines for agitation and seizures,

cypheptadine for serotonin syndrome.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 501

Major Depressive Disorder (MDD)

Overview

Major depressive disorder is a prevalent psychiatric disorder with a lifetime prevalence of approximately 15–20% and a 12-month prevalence of 7%, twice as common in women as in men, with a peak age of onset of 20–40 years.

Causes

This condition happens because of dysregulation of monoamine neurotransmitter systems (serotonin, norepinephrine, dopamine), hypothalamic-pituitary-adrenal (HPA) axis hyperactivity with elevated cortisol, reduced hippocampal volume due to chronic stress-induced neurotoxicity, decreased brain-derived neurotrophic factor (BDNF), and altered functional connectivity in frontolimbic circuits, with genetic factors accounting for 37% of risk.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience depressed mood (sadness, emptiness, hopelessness) nearly every day, anhedonia (markedly diminished interest or pleasure in activities), significant weight change, insomnia or hypersomnia, psychomotor agitation or retardation, fatigue, feelings of worthlessness or excessive guilt, diminished concentration, and recurrent thoughts of death or suicidal ideation, with DSM-5-TR criteria requiring at least five of these symptoms (including depressed mood or anhedonia) present during the same two-week period.

Emergency warning signs

- Severe or worsening symptoms of major depressive disorder (mdd) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview using DSM-5-TR criteria, with screening tools including the PHQ-9 and BDI-II. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves multimodal: pharmacotherapy (SSRIs and SNRIs are first-line), psychotherapy (CBT, IPT), somatic therapies (ECT for severe cases, TMS, ketamine/esketamine for treatment-resistant depression), and lifestyle interventions. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Many patients respond to treatment but recurrence is common.

Chapter 502

Malaria (*Plasmodium* spp.)

Overview

Malaria is a protozoan infection caused by *Plasmodium* species, transmitted by female *Anopheles* mosquitoes, with five species causing human disease: *P. falciparum* (most virulent, responsible for nearly all deaths), *P. vivax*, *P. ovale*, *P. malariae*, and *P. knowlesi*, with 240 million cases and 600,000 deaths annually, primarily in sub-Saharan Africa. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of sporozoites being injected with mosquito saliva, traveling to the liver and invading hepatocytes (exoerythrocytic schizogony), with merozoites released from the liver invading RBCs and initiating the erythrocytic cycle, with *P. falciparum* modifying RBC surfaces and causing cytoadherence, microvascular obstruction, and end-organ damage. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)

- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience an incubation of 7–30 days, with paroxysms of fever, chills, rigor, and diaphoresis every 48–72 hours depending on species, with uncomplicated malaria presenting with cyclic fever, headache, muscle pain, anemia, and splenomegaly, and severe malaria (from *P. falciparum*) presenting with cerebral malaria, severe anemia, acute kidney injury, lung swelling, hypoglycemia, metabolic acidosis, DIC, and shock. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of malaria (*plasmodium spp.*) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection

develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using thick and thin blood smears and rapid diagnostic tests. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or

antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include artemisinin-based combination therapy (ACT) for uncomplicated *P. falciparum*, artesunate IV for severe malaria, and chloroquine plus primaquine for *P. vivax* and *P. ovale*. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- Most people recover well with prompt treatment
- Severe malaria has significant mortality.

Chapter 503

Male Breast Cancer

Overview

Male breast cancer accounts for less than 1% of all breast cancers but is increasing in incidence. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of breast epithelium in males. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)

- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Clinical features typically include a painless, subareolar mass, with ER-positive disease being more common than in female breast cancer. Management follows the same principles as female breast cancer: mastectomy with sentinel lymph node biopsy, adjuvant radiation, and endocrine therapy (tamoxifen is first-line for male breast cancer). Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of male breast cancer require immediate medical evaluation

Progression and Stages

Your outlook depends on stage at diagnosis. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced

tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology

- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 504

Male Infertility

Overview

Male infertility is responsible for approximately 40–50% of couple infertility.

Causes

Causes include varicocele (most common correctable cause, present in 40% of infertile men), obstructive azoospermia (congenital absence of the vas deferens—often associated with CFTR mutations, epididymitis, vasectomy), non-obstructive azoospermia (Klinefelter syndrome, Y-chromosome microdeletions, cryptorchidism, prior chemotherapy/radiation), hypogonadotropic hypogonadism (Kallmann syndrome, pituitary tumors), and idiopathic oligozoospermia. This condition happens because of impaired spermatogenesis or sperm transport.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience inability to conceive after one year of unprotected intercourse. Evaluation includes semen analysis (WHO 2021 reference values), hormonal evaluation (FSH, LH, testosterone), and scrotal ultrasound.

Emergency warning signs

- Severe or worsening symptoms of male infertility require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

The right treatment depends on the cause: varicocele, assisted reproductive techniques (IUI, IVF with ICSI), surgical sperm retrieval, and treatment of reversible causes. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Recovery varies by cause. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 505

Malignant Hyperthermia

Overview

cancerous hyperthermia (MH) is a life-threatening pharmacogenetic skeletal muscle disorder triggered by exposure to volatile halogenated anesthetic agents (isoflurane, sevoflurane, desflurane) or depolarizing muscle relaxants (succinylcholine) in susceptible individuals. Driven by autosomal dominant gain-of-function mutations in the RYR1 gene (encoding the ryanodine receptor 1 calcium release channel of the sarcoplasmic reticulum) or CACNA1S gene, volatile agents trigger uncontrolled, massive calcium efflux from the sarcoplasmic reticulum into the myoplasm. Persistent intracellular calcium accumulation drives sustained, uncoordinated muscle contractions, hypermetabolism, ATP depletion, and rhabdomyolysis.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms often appear as rapid masseter muscle spasm (lockjaw), hypercapnia (rising end-tidal CO₂, earliest and most sensitive sign), sinus tachycardia, muscle rigidity, severe mixed respiratory/metabolic acidosis, rhabdomyolysis (myoglobinuria), hyperkalemia, and late fulminant hyperthermia (> 42°C).

Emergency warning signs

- Severe or worsening symptoms of malignant hyperthermia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam during anesthesia
- Susceptibility your doctor confirms it with caffeine-halothane contracture test (CHCT) or genetic testing. Management requires immediate discontinuation of triggering agents, hyperventilation with 100% oxygen, administration of IV dantrolene sodium (ryanodine receptor antagonist), active cooling, and aggressive treatment of hyperkalemia and acidosis.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 506

Malnutrition in the Elderly

Overview

Malnutrition is a geriatric syndrome affecting 15–60% of hospitalized older adults and 30–50% of nursing home residents. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of physiologic changes (reduced olfaction and taste, delayed gastric emptying), social factors (isolation, poverty, inability to shop/cook), dentition problems, difficulty swallowing, polypharmacy, depression, and dementia. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience weight loss, poor nutritional intake, and functional decline.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of malnutrition in the elderly require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors use the Mini Nutritional Assessment (MNA) as a validated screening tool.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling
Pharmacotherapy to correct metabolic abnormalities
Hormone replacement therapy when indicated
Regular monitoring of metabolic parameters
Weight management programs
Exercise prescription
Management of associated conditions
Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment options include dietary counseling, oral nutritional supplements, texture-modified foods for difficulty swallowing, treatment of depression, dental care, and in severe cases, enteral nutrition.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Consequences include impaired immune function, delayed wound healing, muscle wasting, falls, and increased mortality.

Chapter 507

Mammalian Bites

Overview

- Mammalian bites (dog, cat, human) are common and carry risk of infection. Dog bites cause significant crush injury and laceration
- Pathogens include *Pasteurella canis*, *Staphylococcus aureus*, *Streptococcus* species, and anaerobes. Cat bites create deep puncture wounds carrying bacteria into closed spaces
- Pathogens include *Pasteurella multocida*, *Staphylococcus*, *Streptococcus*, and *Bartonella henselae*. Human bites carry high risk of infection from oral flora including *Eikenella corrodens*, *Streptococcus*, *Staphylococcus*, and anaerobes. Management: wound irrigation, removal of dead tissue of devitalized tissue, assessment of deep structure involvement
- Primary closure is generally avoided for high-risk wounds except facial wounds.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of mammalian bites require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Antibiotic prophylaxis for high-risk wounds. Rabies prophylaxis indicated for bites from unvaccinated or unknown-source wild animals. Tetanus prophylaxis as indicated.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate wound management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 508

Mammary Duct Ectasia

Overview

Mammary duct ectasia (periductal mastitis) is a non-cancerous inflammatory condition characterized by progressive dilation, shortening, and periductal scarring of the subareolar lactiferous ducts, predominantly affecting multiparous postmenopausal women.

Causes

This condition happens because of involutional accumulation and lipid oxidation of ductal secretions, producing ductal rupture, lipid extravasation, and chronic periductal granulomatous inflammation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience thick, sticky, pasty green or brownish single- or multiple-duct nipple discharge, subareolar pain, nipple inversion/retraction, and a firm subareolar mass mimicking breast carcinoma. Ultrasound demonstrates dilated, fluid-filled subareolar ducts containing particulate debris.

Emergency warning signs

- Severe or worsening symptoms of mammary duct ectasia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors confirm the diagnosis through clinical examination, sonography, and biopsy to exclude malignancy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves conservative with warm compresses and antibiotics if infected
- Major duct removal (Hadfield operation) is indicated for persistent, troublesome discharge or severe pain.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 509

Maple Syrup Urine Disease

Overview

Maple syrup urine disease (MSUD) is an autosomal recessive disorder caused by deficiency of the branched-chain α -ketoacid dehydrogenase (BCKD) enzyme complex, leading to accumulation of branched-chain amino acids (BCAAs)—leucine, isoleucine, valine—and their corresponding ketoacids. Incidence is approximately 1 in 185,000 live births worldwide, but is higher in certain populations (Mennonite communities, 1 in 380).

Causes

This condition happens because of deficiency of the BCKD enzyme complex.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of maple syrup urine disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- You may experience the classic (severe) form presenting in the first week of life with poor feeding, lethargy, vomiting, hypertonicity, opisthotonos, seizures, and a characteristic sweet, maple syrup odor of cerumen and urine
- Progressive nervous system deterioration leads to coma and respiratory failure if untreated.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using newborn screening (elevated leucine/isoleucine) and plasma amino acid analysis showing markedly elevated leucine, isoleucine, valine, and pathognomonic alloisoleucine. Treatment for acute decompensation includes aggressive fluid resuscitation, high-dose dextrose and insulin to promote anabolism, and dialysis for rapid leucine clearance

- Chronic Treatment focuses on dietary restriction of BCAAs, thiamine supplementation (in responsive variants), and vigilant monitoring during intercurrent illness
- Liver transplantation provides a functional cure.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with early treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 510

Marasmic-Kwashiorkor

Overview

Marasmic-kwashiorkor is a mixed form of severe acute malnutrition combining features of both marasmus (wasting) and kwashiorkor (swelling). It occurs when there is deficiency of both total calories and protein.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe wasting accompanied by peripheral swelling. Anthropometry shows severe deficits in weight-for-age and weight-for-height, with concurrent pitting swelling and low serum albumin.

Emergency warning signs

- Severe or worsening symptoms of marasmic-kwashiorkor require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with anthropometric confirmation. Management follows the same WHO guidelines with emphasis on careful monitoring for fluid overload, refeeding syndrome, and infections. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is worse than for either pure form alone. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 511

Marasmus

Overview

Marasmus is a form of severe protein-energy malnutrition characterized by inadequate intake of both protein and calories. It is most common in infants and young children in low-resource settings, particularly during the first year of life, and is linked to early weaning, repeated infections, and poverty.

Causes

This condition happens because of catabolism of muscle and subcutaneous fat to meet energy needs, leading to profound wasting with relative preservation of serum albumin.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe wasting of muscle and subcutaneous tissue, a “monkey-like” facies with prominent eyes, wrinkled skin, irritability, and growth retardation
- Subcutaneous fat is absent and there is no peripheral swelling.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of marasmus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on anthropometric criteria: weight-for-age less than 60% of median, weight-for-height less than 70% of median, or mid-upper arm circumference (MUAC) less than 115 mm in children aged 6–59 months. Management follows the WHO protocol: stabilization with correction of dehydration and electrolyte imbalances (using ReSoMal), empiric antibiotics, micronutrient supplementation, and staged refeeding with F-75 formula followed by F-100 formula
- Slow, cautious refeeding is required to avoid refeeding syndrome.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with timely treatment, but mortality remains high (20–30%) in severe cases without adequate care. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 512

Marburg Virus

Overview

Marburg virus disease is a severe involving bleeding fever caused by Marburg virus, a negative-sense RNA virus (genus *Marburgvirus*), the only filovirus other than Ebola to cause human disease, with outbreaks in Africa, and the reservoir being the Egyptian fruit bat (*Rousettus aegyptiacus*).

Causes

This condition happens because of identical Disease mechanism to Ebola virus—infection of dendritic cells, macrophages, and monocytes, profound immunosuppression, cytokine storm, endothelial dysfunction, and DIC.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of marburg virus require immediate medical evaluation

Progression and Stages

You may experience an incubation of 2–21 days, with abrupt onset of fever, chills, severe headache, muscle pain, and prostration, followed by digestive tract phase (severe watery diarrhea, abdominal pain) and involving bleeding phase (tiny red or purple spots, ecchymoses, mucosal bleeding, uncontrolled bleeding, and multi-organ failure). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using RT-PCR (blood, urine, throat swab). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment typically involves supportive care with no approved specific therapy. outlook: mortality 23–80% depending on outbreak and strain.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 513

Marfan Syndrome

Overview

Marfan syndrome is an autosomal dominant connective tissue disorder caused by mutations in the FBN1 gene encoding fibrillin-1, affecting 1 in 5,000 individuals. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of abnormal microfibril assembly, leading to reduced structural integrity of connective tissue and dysregulation of TGF-beta signaling. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience skeletal manifestations (tall stature, arachnodactyly, pectus excavatum or carinatum, scoliosis, joint hypermobility), ocular manifestations (ectopia lentis, myopia, retinal detachment), and cardiovascular manifestations (aortic root

dilation, aortic aneurysm, aortic dissection, mitral valve prolapse).

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of marfan syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on the revised Ghent criteria, including systemic score, aortic root diameter Z-score, ectopia lentis, and FBN1 genetic testing.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment focuses on beta-blockers (atenolol) or losartan to reduce aortic growth rate, serial echocardiography for aortic surveillance, elective aortic root replacement when diameter reaches 50 mm (or earlier with family history of dissection), and lifestyle modification (avoidance of contact sports, isometric exercise).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook has improved dramatically
- Life expectancy now approaches 70 years with appropriate surveillance and prophylactic surgery, compared to 45 years in the 1970s.

Chapter 514

Marine Envenomation

Overview

- Jellyfish envenomation: nematocysts discharge venom containing toxins causing local pain, redness of the skin, and in severe cases (box jellyfish—*Chironex fleckeri*), cardiovascular collapse and death within minutes. Treatment: rinse with vinegar (5% acetic acid) to inactivate undischarged nematocysts
- Remove tentacles with forceps
- Hot water immersion (45°C, 20 minutes) inactivates venom. For box jellyfish, antivenom is used for severe cases. Stonefish (*Synanceia*) envenomation produces excruciating pain, local swelling, hypotension, arrhythmias, and rarely death. Treatment: hot water immersion (45°C for 30–90 minutes denatures the heat-labile toxin)
- Antivenom is available. Cone snail (*Conus*) venom contains conotoxins causing neuromuscular blockade, bulbar palsy, and respiratory failure.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of marine envenomation require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive with airway support and ventilation as needed. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with prompt treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 515

Massive Splenomegaly

Overview

- Massive splenomegaly is spleen enlargement extending below the umbilicus or crossing the midline. Common causes in developing countries include malaria, visceral leishmaniasis (kala-azar), schistosomiasis, and myelofibrosis
- In Western countries, myeloproliferative tumors (chronic myeloid leukemia, polycythemia vera, primary myelofibrosis), lymphomas (hairy cell leukemia, splenic marginal zone lymphoma), and storage diseases (Gaucher disease) are more common.

Causes

This condition happens because of the underlying infiltrative, congestive, or neoplastic process.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of massive splenomegaly require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- You may experience significant symptoms and complications including splenic tissue death due to lack of blood supply, portal high blood pressure, and severe cytopenias from hypersplenism. Treatment typically involves directed at the underlying cause
- Splenectomy may be required for symptomatic relief or life-threatening complications.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on imaging and identification of the underlying cause. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 516

Mastitis (Puerperal)

Overview

It presents with localized breast redness of the skin, warmth, induration, and systemic symptoms (fever, myalgias). This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of mastitis (puerperal) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Puerperal mastitis is a common puerperal complication affecting 2–10% of breastfeeding women, usually within the first six weeks postpartum.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include continued breastfeeding or milk expression to prevent milk stasis, supportive bras, and antibiotics (dicloxacillin or cephalexin).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Breast abscess (5–10% of cases) requires ultrasound-guided removal by suction or incision and removal of fluid. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 517

Mastitis and Breast Abscess

Overview

Mastitis is inflammation of the breast tissue, most commonly lactational (puerperal) mastitis, occurring in 2–10% of breastfeeding women. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of bacterial infection, most commonly *Staphylococcus aureus*, introduced through a cracked nipple or cracked skin. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience a focal area of redness of the skin, warmth, tenderness, and induration, often accompanied by fever and malaise. Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of mastitis and breast abscess require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

Breast abscess is a complication of mastitis requiring removal by suction or incision and removal of fluid.

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer

- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include continued breastfeeding or expression, warm compresses, and antibiotics (dicloxacillin or cephalexin). Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most people recover well with appropriate treatment. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 518

McArdle Disease

Overview

McArdle disease (glycogen storage disease type V) is an autosomal recessive metabolic myopathy caused by deficiency of muscle glycogen phosphorylase, the enzyme that breaks down glycogen to glucose-1-phosphate in skeletal muscle.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience exercise intolerance, muscle pain, and cramping during vigorous activity, often with a "second wind" phenomenon (improvement in symptoms after a brief rest during exercise)
- Rhabdomyolysis may occur with severe exercise. This condition happens because of PYGM gene mutations leading to inability to mobilize glycogen stores during anaerobic metabolism.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of mcardle disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on elevated serum CK, ischemic forearm exercise test (failure of lactate to rise), and genetic testing (PYGM mutations)
- Muscle biopsy shows glycogen accumulation and absent phosphorylase activity.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include exercise conditioning, high-protein diet, and avoidance of strenuous exercise
- Oral sucrose before exercise may improve exercise capacity.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person but many patients maintain functional independence with lifestyle modifications. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 519

Measles (Rubeola)

Overview

This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of the virus entering through respiratory epithelium, replicating in regional lymph nodes, and causing viremia, spreading to the reticuloendothelial system, with infection causing profound lymphopenia and immunosuppression lasting weeks to months. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience an incubation of 7–14 days, with prodrome (2–4 days): fever, the three Cs—cough, coryza, conjunctivitis, and Koplik spots (pathognomonic: small, white, bluish-white papules on the buccal mucosa), followed by rash (day 3–5): erythematous, maculopapular, confluent rash beginning on the face and spreading cephalocaudally. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of measles (rubeola) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

Complications include pneumonia (most common cause of death), otitis media, laryngo-tracheobronchitis, encephalitis, and subacute sclerosing panencephalitis (SSPE).

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, confirmed by RT-PCR or IgM serology. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- Measles is a highly contagious vaccine-preventable viral illness caused by measles virus, a paramyxovirus, with 140,000 deaths globally in 2018, and one of the most contagious infectious diseases with a basic reproduction number (R_0) of 12–18. Most people recover well with supportive care
- Prevention through MMR vaccine is highly effective.
- Hand hygiene with soap and water or alcohol-based sanitizer

- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment typically involves supportive with vitamin A. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 520

Meconium Aspiration Syndrome

Overview

Meconium removal by suction syndrome (MAS) occurs when meconium-stained amniotic fluid is aspirated into the fetal airways before or during delivery, causing airway obstruction, pneumonitis, surfactant inactivation, and lung vasoconstriction. It occurs in 1–3% of all deliveries with meconium-stained fluid. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience respiratory distress at birth, tachypnea, grunting, retractions, cyanosis, and barrel-shaped chest. Chest radiograph shows coarse, patchy infiltrates, hyperinflation, and atelectasis.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of meconium aspiration syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include immediate endotracheal suctioning of non-vigorous infants, supplemental oxygen, surfactant therapy, inhaled nitric oxide for lung high blood pressure, and ECMO for severe refractory cases. The routine practice of intrapartum suctioning for all meconium-stained infants is no longer recommended for vigorous infants. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria)

Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate management.

Chapter 521

Medication Overuse Headache

Overview

Medication overuse headache (MOH), also called rebound headache, is a secondary headache disorder resulting from frequent or excessive use of acute headache medications. It is the most common cause of chronic daily headache, affecting 1–2% of the general population. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of frequent use of acute headache medications, particularly combination analgesics, triptans, opioids, and barbiturates. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience headache on ≥ 15 days per month with a causal relationship to medication overuse. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of medication overuse headache require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for ICHD-3 criteria. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on withdrawal of the overused medication (abruptly for triptans and simple analgesics, gradually for opioids and barbiturates), bridging therapy during withdrawal, and concurrent initiation of preventive therapy. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with treatment
- Relapse rates are 30–40% within the first year.

Chapter 522

Medulloblastoma

Overview

This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of primitive neuroectodermal cells. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)

- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience headache, vomiting (especially morning), loss of coordination, and signs of hydrocephalus from fourth ventricle obstruction. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of medulloblastoma require immediate medical evaluation

Progression and Stages

Medulloblastoma is the most common cancerous brain tumor of childhood, arising in the cerebellum or posterior fossa, and belongs to the group of embryonal tumors classified into molecular subgroups (WNT, SHH, Group 3, Group 4) with distinct prognoses. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement.

Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Diagnosis typically involves MRI showing a posterior fossa mass and cerebrospinal fluid cytology for staging. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)

- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption. Maintain a healthy weight through diet and exercise. Eat a balanced diet rich in fruits, vegetables, and whole grains. Protect skin from excessive sun exposure and avoid tanning beds. Get vaccinated against cancer-causing infections (HPV, hepatitis B). Participate in recommended cancer screening programs. Know your family history and consider genetic testing if indicated. Avoid exposure to known carcinogens in the workplace and environment.

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

- Treatment options include maximal surgical removal, craniospinal irradiation, and adjuvant chemotherapy
- Risk-adapted therapy aims to reduce long-term neurocognitive and endocrine sequelae in survivors.
- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team. Manage treatment side effects with supportive medications. Maintain adequate nutrition with help from a dietitian. Stay physically active within your energy limits. Join cancer support groups for emotional support and shared experiences. Seek counseling or mental health support for anxiety and

depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Recovery varies by molecular subgroup, with the WNT subgroup having the best outlook. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 523

Melanoma

Overview

Melanoma is the most lethal skin cancer, arising from melanocytes. Although representing only 1% of skin cancers, it accounts for the majority of skin cancer deaths. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

- The ABCDE criteria aid in identification: Asymmetry, Border irregularity, Color variation, Diameter >6 mm, and Evolution. The four main subtypes are superficial spreading (most common), nodular (most aggressive, may not follow ABCDE), lentigo maligna (on chronically sun-damaged skin), and acral lentiginous (palms, soles, nail beds)
- Most common melanoma in people of color). This condition happens because of cancerous transformation of melanocytes driven by UV-induced and genetic mutations (BRAF V600, NRAS, KIT).

Risk Factors

- Advancing age

- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of melanoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional

lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment for thin melanoma (≤ 1 mm) is surgical removal with sentinel lymph node biopsy for those ≥ 0.8 mm or with ulceration
- Immunotherapy (anti-PD-1 [nivolumab, pembrolizumab], anti-CTLA-4 [ipilimumab], combination therapy) and targeted therapy (BRAF/MEK inhibitors for BRAF V600-mutated melanoma) have revolutionized treatment of advanced melanoma.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

- To make the diagnosis, doctors look for excisional biopsy with adequate margins
- Staging uses Breslow thickness, ulceration, mitotic rate, and lymph node status.
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

- Your outlook depends on stage at diagnosis
- Early detection is critical.
- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team

- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 524

Melasma

Overview

Melasma is a common acquired hyperpigmentation disorder characterized by symmetric, irregular, brown-to-gray-brown macules and patches on sun-exposed areas, predominantly the cheeks, forehead, upper lip, and chin. It predominantly affects women with darker skin phototypes (Fitzpatrick III–VI).

Causes

This condition happens because of UV-induced stimulation of melanocytes with increased melanin production and dermal deposition, exacerbated by sun exposure, pregnancy ("chloasma"), oral contraceptives, and hormonal therapies.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of melasma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Doctors usually diagnose this based on your symptoms and a physical exam
- Wood's lamp examination helps classify depth (epidermal, dermal, mixed).

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include strict sun protection (broad-spectrum sunscreen with iron oxide), topical depigmenting agents (hydroquinone 4%, tretinoin, azelaic acid, tranexamic acid), and chemical peels. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and

support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Recurrence is common, and combination therapy yields the best results.

Chapter 525

Membranous Nephropathy

Overview

Membranous nephropathy (MN) is the most common cause of nephrotic syndrome in non-diabetic adults.

Causes

This condition results from disruption of normal membranous nephropathy function.

- This condition happens because of subepithelial immune complex deposition and thickening of the glomerular basement membrane
- Approximately 70–80% of primary MN is caused by autoantibodies against the phospholipase A2 receptor (PLA2R) on podocytes. Secondary causes include malignancy (lung, GI), infections (hepatitis B), autoimmune diseases (lupus), and drugs.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience nephrotic-range proteinuria, swelling, and hypoalbuminemia.

Emergency warning signs

- Severe or worsening symptoms of membranous nephropathy require immediate medical evaluation

Progression and Stages

Treatment for primary MN includes conservative management (RAAS blockade) for mild disease and immunosuppression (rituximab, cyclophosphamide, calcineurin inhibitors) for moderate-to-severe or progressive disease. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on kidney biopsy with PLA2R antibody serology. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Anti-PLA2R antibody levels monitor disease activity and response to treatment.

Chapter 526

Meningioma

Overview

Causes

This condition results from disruption of normal meningeal function.

- This condition happens because of neoplastic growth of arachnoid cap cells. Most are asymptomatic and discovered incidentally on imaging
- Symptomatic meningiomas cause headaches, seizures, or focal deficits depending on location.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of meningioma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Meningioma is the most common primary intracranial tumor, arising from arachnoid cap cells
- It is usually non-cancerous (WHO grade 1), slow-growing, and attached to the dura
- Meningiomas are more common in women and associated with prior radiation, neurofibromatosis type 2, and high progesterone receptor expression. Treatment focuses on observation for small, asymptomatic lesions
- Surgical removal is the mainstay for symptomatic or growing tumors
- Atypical (grade 2) and anaplastic (grade 3) meningiomas require adjuvant radiotherapy.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on MRI revealing a well-circumscribed, homogeneously enhancing dural-based mass with a dural tail. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well for non-cancerous meningiomas. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 527

Mercury Toxicity

Overview

- Mercury exists in three forms: elemental (metallic), inorganic (mercurous/mercuric salts), and organic (methylmercury). Elemental mercury: inhaled vapor is absorbed (80%)
- Causes acute pneumonitis, lung swelling, and chronic neurotoxicity (tremor, erethism—emotional lability, pathological shyness, memory loss). Inorganic mercury: ingestion of mercuric chloride causes corrosive gastroenteritis, acute tubular tissue death, and nephrotic syndrome. Acrodynia (Pink disease) is a hypersensitivity reaction in children. Organic mercury (methylmercury): bioaccumulates in fish
- Causes Minamata disease (severe neurological syndrome: paresthesias, loss of coordination, slurred speech, visual field constriction, hearing loss) and is a potent teratogen. Diagnosis: blood and urine mercury levels. Treatment: cessation of exposure
- Succimer (DMSA) is the preferred chelator for elemental and inorganic mercury.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of mercury toxicity require immediate medical evaluation

Progression and Stages

Your outlook depends on form and severity of exposure. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 528

Mesothelioma

Overview

Causes

This condition happens because of asbestos fibers causing chronic inflammation and cancerous transformation of mesothelial cells.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive shortness of breath, chest pain, weight loss, and pleural effusion.

Emergency warning signs

- Severe or worsening symptoms of mesothelioma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Cancerous pleural mesothelioma is a rare, aggressive tumor arising from the mesothelial cells of the pleura, strongly associated with asbestos exposure (latency 20–40

years)

- It is classified into epithelioid (most common, better outlook), sarcomatoid, and biphasic subtypes. Treatment typically involves palliative in most cases: chemotherapy with pemetrexed and cisplatin is first-line
- Immunotherapy has shown benefit
- Surgery may be considered in early-stage disease.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for CT scan showing circumferential pleural thickening and pleural biopsy. Staging is by the TNM system. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Unfortunately, the outlook is often poor, with median survival of 12–18 months. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 529

Metabolic Syndrome

Overview

Metabolic syndrome is a cluster of interconnected metabolic abnormalities that increase the risk of cardiovascular disease, type 2 diabetes, and all-cause mortality. The NCEP ATP III diagnostic criteria require at least three of the following: central obesity (waist circumference greater than 102 cm in men, 88 cm in women), elevated triglycerides (150 mg/dL or greater), reduced HDL cholesterol (less than 40 mg/dL in men, less than 50 mg/dL in women), elevated blood pressure (130/85 mm Hg or greater), and elevated fasting glucose (100 mg/dL or greater). Central obesity and insulin resistance are considered core pathophysiologic features, resulting from adipose tissue dysfunction and ectopic fat deposition driving insulin resistance, endothelial dysfunction, and a prothrombotic/proinflammatory state. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Family history of the genetic condition
- Consanguineous parents (for recessive disorders)
- Advanced parental age (for new mutations)
- Carrier status in parents
- Ethnic background (some conditions)
- Family history
- Obesity and sedentary lifestyle
- Poor dietary habits
- Advancing age
- Hormonal imbalances
- Certain medications

- Genetic predisposition and family history
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the cluster of metabolic abnormalities without specific symptoms.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of metabolic syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for fulfillment of the diagnostic criteria.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Metabolic syndrome confers a 2-fold increased risk of cardiovascular events and a 5-fold increased risk of developing type 2 diabetes.

Chapter 530

Metformin Overdose

Overview

Metformin overdose is an uncommon but highly lethal pharmaceutical overdose due to metformin-associated lactic acidosis (MALA). This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of metformin inhibiting gluconeogenesis and complex I of the mitochondrial electron transport chain, leading to impaired lactate clearance and increased anaerobic metabolism, with predisposing factors including kidney impairment, liver dysfunction, hypoperfusion, sepsis, and concurrent use of contrast dye.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience nausea, vomiting, abdominal pain, diarrhea, profound lactic acidosis (pH <7.1, lactate >20 mmol/L), hypotension, hypothermia, altered mental status, and acute kidney injury.

Emergency warning signs

- Severe or worsening symptoms of metformin overdose require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using elevated serum metformin level and severe lactic acidosis with elevated lactate-to-pyruvate ratio. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment options include supportive care and early hemodialysis or continuous venovenous hemofiltration as The main treatment.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is guarded
- Severe MALA has mortality exceeding 30–50%.

Chapter 531

Methemoglobinemia

Overview

Methemoglobinemia is a disorder characterized by increased levels of methemoglobin (ferric Hb, Fe^{3+}), which is unable to bind oxygen and causes functional anemia, with a normal methemoglobin level $<1\%$ of total hemoglobin.

Causes

This condition happens because of either congenital deficiency of cytochrome b5 reductase (NADH-methemoglobin reductase, autosomal recessive, most common congenital cause) or exposure to oxidizing agents (nitrates, nitrites, dapsone, benzocaine, sulfonamides, aniline dyes).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features correlate with methemoglobin levels: 10–20% causes cyanosis without symptoms
- 20–50% causes headache, shortness of breath, tachycardia, and altered mental status
- $>50\%$ causes seizures, coma, irregular heartbeats, and death. A characteristic finding is chocolate-brown discoloration of blood that does not turn red when exposed to air, and pulse oximetry showing a saturation of approximately 85% that does not change with oxygen supplementation.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of methemoglobinemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with co-oximetry (multi-wavelength blood gas analysis) showing elevated methemoglobin. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include supplemental oxygen, intravenous methylene blue (1–2 mg/kg over 5 minutes) for symptomatic or severe methemoglobinemia (>20–30%), which acts as an electron donor to reduce methemoglobin to hemoglobin. Methylene blue ineffective in G6PD deficiency (risk of hemolysis)
- Alternative is ascorbic acid or exchange transfusion.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with treatment
- Untreated severe methemoglobinemia can be fatal.

Chapter 532

Methyl Parathion and Other Organophosphate Insecticides

Overview

Like other organophosphates, it irreversibly inhibits acetylcholinesterase, causing cholinergic crisis (SLUDGE syndrome plus miosis, bronchorrhea, bronchospasm, bradycardia, muscle fasciculations, weakness, seizures, coma). It is particularly dangerous because it is rapidly absorbed through skin.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of methyl parathion and other organophosphate insecticides require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam with low RBC/plasma cholinesterase activity (<50% of normal). Treatment: atropine (to block muscarinic symptoms), pralidoxime (2-PAM, to reactivate AChE), diazepam for seizures, and decontamination. outlook: recovery is usually complete if treated early
- Delayed neurotoxicity (organophosphate-induced delayed neuropathy/OPIDN) can occur weeks later.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Methyl parathion is a highly toxic organophosphate insecticide (WHO class Ia—extremely hazardous) used in agriculture.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 533

Microscopic Polyangiitis

Overview

Microscopic polyangiitis is an ANCA-associated necrotizing vasculitis affecting small vessels (capillaries, venules, arterioles), with a predilection for the kidneys and lungs, but without granuloma formation (distinguishing it from GPA), with an incidence of 2–6 per million, a slight male predominance, and typically presenting in the sixth to seventh decade. The condition involves p-ANCA (perinuclear) directed against myeloperoxidase (MPO) in approximately 60–70% of patients (PR3-ANCA in 20–30%), with ANCA-induced neutrophil activation leading to necrotizing inflammation of small vessels.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience rapidly progressive glomerulonephritis (crescentic, pauci-immune: the most common and serious manifestation, present in >80%), lung capillaritis causing diffuse alveolar bleeding (hemoptysis, shortness of breath, bilateral infiltrates), cutaneous purple spots on the skin, mononeuritis multiplex, arthralgias, and myalgias.

Emergency warning signs

- Severe or worsening symptoms of microscopic polyangiitis require immediate medical evaluation

Progression and Stages

Doctors diagnose this based on the 2022 ACR/EULAR classification criteria, which emphasize MPO-ANCA positivity, kidney involvement, and absence of granulomatous features. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Management follows the same principles as GPA: high-dose corticosteroids plus rituximab or cyclophosphamide for induction, followed by maintenance therapy with rituximab, azathioprine, or methotrexate. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is similar to GPA, with 5-year survival exceeding 75%. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 534

Migraine

Overview

Migraine is a primary headache disorder affecting approximately 15% of the global population (1 billion people), with a female-to-male ratio of 3:1. It is the second leading cause of years lived with disability worldwide. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition results from disruption of normal migraine function.

- This condition happens because of cortical spreading depression (CSD)—a wave of neuronal depolarization followed by prolonged suppression of cortical activity—triggering the trigeminovascular system with release of calcitonin gene-related peptide (CGRP), causing vasodilation and neurogenic inflammation
- Genetic factors play a role, with familial hemiplegic migraine linked to mutations in CACNA1A, ATP1A2, and SCN1A.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience recurrent, unilateral (60%), pulsating headache of moderate to severe intensity, lasting 4–72 hours, associated with nausea, vomiting, photophobia, and phonophobia
- Migraine with aura involves transient focal neurological symptoms preceding or accompanying the headache
- Chronic migraine is defined by headache on ≥ 15 days per month for >3 months.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of migraine require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam based on ICHD-3 criteria. Acute Treatment focuses on NSAIDs or triptans for mild to moderate attacks and triptans, lasmiditan, or gepants for moderate to severe attacks. Preventive therapy is indicated for ≥ 4 migraine days per month and includes beta-blockers, amitriptyline, topiramate, candesartan, onabotulinumtoxin A for chronic migraine, and CGRP monoclonal antibodies. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures

- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Migraine is a chronic condition with episodic attacks that can be managed but not cured.

Chapter 535

Mild Cognitive Impairment

Overview

Mild thinking and memory impairment (MCI) is an intermediate state between normal thinking and memory aging and dementia, characterized by objective thinking and memory decline (typically memory—amnesic MCI—or other domains) that is greater than expected for age and education but does not significantly impair functional independence. Prevalence is 10–20% in adults aged 65 and older.

Causes

Annual conversion rate to dementia is 5–15%, with amnesic MCI conferring highest risk for Alzheimer disease.

Risk Factors

- This condition happens because of underlying neurodegenerative pathology, with the APOE epsilon-4 allele as a major genetic risk factor. Treatment options include cardiovascular risk factor control, physical exercise, thinking and memory stimulation, treatment of depression and sleep disorders, and avoidance of anticholinergic medications
- No pharmacotherapy is approved for MCI.
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience subjective thinking and memory complaints with objective deficits on testing. Proposed biomarkers include CSF amyloid-beta 42 (decreased), phosphorylated tau (increased), amyloid PET, and tau PET.

Emergency warning signs

- Severe or worsening symptoms of mild cognitive impairment require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Annual conversion rate to dementia is 5–15%.

Chapter 536

Minimal Change Disease

Overview

Minimal change disease (MCD) is the most common cause of nephrotic syndrome in children (70–90% of cases) and a significant cause in adults (10–15%).

Causes

This condition happens because of podocyte injury with foot process effacement on electron microscopy, with normal light microscopy and immunofluorescence (hence "minimal change"). Most cases are idiopathic, though MCD can be associated with NSAIDs, lymphoma, and infections.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sudden onset of nephrotic-range proteinuria (>3.5 g/day), swelling, hypoalbuminemia, hyperlipidemia, and lipiduria.

Emergency warning signs

- Severe or worsening symptoms of minimal change disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on kidney biopsy demonstrating podocyte foot process effacement on electron microscopy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves with corticosteroids (prednisone), to which 90% of patients respond (steroid-sensitive)
- Frequent relapses or steroid dependence may require second-line agents (calcineurin inhibitors, cyclophosphamide, rituximab).
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Most patients achieve remission with corticosteroids.

Chapter 537

Mitral Regurgitation

Overview

Mitral regurgitation (MR) is the retrograde flow of blood from the left ventricle into the left atrium during systole. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience exertional shortness of breath, fatigue, orthopnea, and atrial fibrillation
- Physical examination reveals a high-pitched holosystolic (pansystolic) murmur at the apex radiating to the left axilla, S3 gallop, and displaced hyperdynamic apical impulse.
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of mitral regurgitation require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

- Primary (organic) MR results from intrinsic valve apparatus pathology, including mitral valve prolapse (myxomatous deterioration), infective endocarditis, or rheumatic heart disease
- Secondary (functional) MR results from left ventricular remodeling or dilation (ischemic or dilated cardiomyopathy) displacing papillary muscles.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Your doctor confirms the diagnosis with transthoracic echocardiography assessing valve anatomy, regurgitant jet volume, and left ventricular function. Treatment for severe MR includes medical optimization (afterload reduction with ACE inhibitors/ARBs, diuretics) and timely surgical mitral valve repair or replacement prior to irreversible left ventricular dysfunction. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary

artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Your outlook depends on baseline LV ejection fraction. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 538

Mitral Stenosis

Overview

Mitral stenosis is a narrowing of the mitral valve orifice, most commonly caused by rheumatic heart disease, with a female predominance. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process.

- This condition happens because of progressive scarring and fusion of the mitral commissures, leading to elevated left atrial pressure, lung congestion, and eventually lung high blood pressure and right heart failure. The normal mitral valve area is 4–6 cm²
- Symptoms typically appear when the area narrows to less than 2 cm².

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity

- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience shortness of breath, orthopnea, hemoptysis, and atrial fibrillation. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of mitral stenosis require immediate medical evaluation

Progression and Stages

Your outlook depends on the severity and success of intervention. Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection

- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using echocardiography. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment options include through the skin mitral balloon valvulotomy for suitable anatomy, surgical commissurotomy, or mitral valve replacement
- Anticoagulation is indicated for atrial fibrillation or thromboembolism.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)

- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 539

Mitral Valve Prolapse

Overview

Mitral valve prolapse is the most common valvular abnormality, affecting 2–3% of the population, and involves billowing of one or both mitral valve leaflets into the left atrium during systole, with or without mitral regurgitation. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process.

- This condition happens because of structural abnormality of the mitral valve apparatus. Most patients are asymptomatic and discovered incidentally
- Clinical features may include palpitations, chest pain, and dysautonomic symptoms.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of mitral valve prolapse require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

- Most cases are idiopathic or associated with myxomatous deterioration
- Mitral valve prolapse is linked to Marfan syndrome and Ehlers-Danlos syndrome.

Complications

- Most people recover well in most cases
- Complications include mitral regurgitation, infective endocarditis, and rarely, arrhythmias.
- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

- Doctors diagnose this using echocardiography. Asymptomatic patients with minimal regurgitation require only reassurance
- Severe regurgitation may require surgical repair.
- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Anti-hypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Anti-hypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease

- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 540

Mixed Connective Tissue Disease

Overview

Mixed connective tissue disease is a distinct systemic autoimmune disorder characterized by overlapping Symptoms of systemic lupus erythematosus (SLE), systemic sclerosis (SSc), and polymyositis/dermatomyositis (PM/DM), in the presence of high-titer anti-U1 ribonucleoprotein (anti-RNP) antibodies, with a prevalence of approximately 1–2 per 100,000, a 9:1 female predominance, and typically presenting in the third to fourth decade.

Causes

This condition happens because of autoantibodies against U1-RNP, leading to immune complex formation, complement activation, and tissue injury, with prominent endothelial and microvascular involvement.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features evolve over time and commonly include Raynaud's phenomenon (>85%), puffy hands or sclerodactyly, inflammatory arthritis (non-erosive, resembling SLE), myositis (proximal muscle weakness, elevated creatine kinase), esophageal dysmotility, interstitial lung disease (up to 50%), lung high blood pressure (the leading cause of death), serositis, and a characteristic malar rash or discoid lupus-like lesions, with kidney involvement being less common and less severe than in SLE alone.

Emergency warning signs

- Severe or worsening symptoms of mixed connective tissue disease require immediate medical evaluation

Progression and Stages

Doctors diagnose this based on the presence of anti-U1-RNP antibodies (high-titer) plus clinical features from at least two of the three component diseases (SLE-like, SSc-like, PM/DM-like), with several classification criteria existing (Alarcón-Segovia, Kahn, Sharp). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves determined by the dominant clinical manifestation: corticosteroids and disease-modifying antirheumatic drugs (DMARDs) for arthritis and serositis

- Immunosuppressants (mycophenolate, cyclophosphamide) or rituximab for interstitial lung disease
- Calcium channel blockers and endothelin receptor antagonists for Raynaud's phenomenon and lung high blood pressure.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with lung high blood pressure being the leading cause of death. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 541

Molluscum Contagiosum

Overview

Molluscum contagiosum is a viral skin infection caused by a poxvirus, transmitted by direct contact (including sexual contact) or fomites.

Causes

This condition happens because of poxvirus infection of epidermal keratinocytes.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience 2–5 mm, dome-shaped, flesh-colored papules with central umbilication ("dell"), often with a white waxy core
- Lesions may be solitary or numerous and can be spread by scratching (autoinoculation). In immunocompromised patients (HIV/AIDS), molluscum can be widespread, large, and refractory to treatment.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of molluscum contagiosum require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Dermoscopy shows a central pore with white amorphous material. Management options include freezing treatment, curettage, topical agents (cantharidin, imiquimod, potassium hydroxide), and mechanical extraction.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Self-resolution occurs in immunocompetent individuals within months to years.

Chapter 542

Morton Neuroma

Overview

Morton neuroma is a non-cancerous perineural fibrotic enlargement of the interdigital plantar nerve, most commonly affecting the third intermetatarsal space between the third and fourth metatarsal heads. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of chronic mechanical compression and repetitive irritation of the nerve beneath the deep transverse metatarsal ligament, leading to epineural and endoneural sclerosis. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sharp, burning forefoot pain radiating to the toes, exacerbated by weight-bearing and narrow footwear, often described as 'walking on a marble.' Physical examination reveals a positive Mulder sign (palpable click with forefoot

squeeze) and localized intermetatarsal tenderness. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of morton neuroma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by foot ultrasound or MRI if equivocal.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Treatment options include wide footwear, metatarsal pads, corticosteroid injections, and surgical neurectomy for recalcitrant symptoms. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 543

Motor Neuron Disease

Overview

Motor neuron disease (MND) is a group of progressive neurodegenerative disorders affecting upper motor neurons (UMNs), lower motor neurons (LMNs), or both, with a lifetime risk of 1 in 300. The spectrum includes amyotrophic lateral sclerosis (ALS, most common, both UMN and LMN), progressive bulbar palsy (PBP, predominantly bulbar onset), progressive muscular shrinkage or wasting (PMA, predominantly LMN), and primary lateral sclerosis (PLS, predominantly UMN). Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of TDP-43 pathology, glutamate excitotoxicity, oxidative stress, mitochondrial dysfunction, and impaired protein degradation, with genetic causes including C9orf72 hexanucleotide repeat expansion, SOD1, TARDBP, FUS, and TBK1 mutations. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive muscle weakness, wasting, fasciculations, spasticity, hyperreflexia (UMN signs), and bulbar symptoms (slurred speech, difficulty swallowing). ALS presentations include limb-onset (70%), bulbar-onset (25%), and respiratory-onset (5%). thinking and memory impairment is present in 30–50% (frontotemporal dementia in 15%). Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of motor neuron disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on revised El Escorial criteria, electromyography showing widespread denervation, and exclusion of mimics.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on multidisciplinary care, riluzole (extends survival by 2–3 months), edaravone (slows functional decline in early ALS), noninvasive ventilation for respiratory insufficiency, and symptomatic treatments (sialorrhea, spasticity, pain). Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is uniformly fatal in ALS: median survival 3–5 years from symptom onset, with respiratory failure as the most common cause of death. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 544

Mpox

Overview

Mpox is a viral zoonosis caused by mpox virus, a double-stranded DNA orthopoxvirus closely related to variola virus, formerly called monkeypox, endemic in Central and West Africa, with the 2022 global outbreak involving over 90,000 cases across 110 countries.

Causes

This condition happens because of the virus entering through broken skin, respiratory tract, or mucous membranes, replicating at the inoculation site, spreading to regional lymph nodes, and causing viremia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 5–21 days, with prodrome (fever, headache, muscle pain, lymphadenopathy—hallmark distinguishing mpox from smallpox) followed by rash appearing 1–4 days after fever, progressing from macules to papules, vesicles, pustules, and crusts over 2–4 weeks, with a centrifugal distribution, and anogenital lesions common in the 2022 outbreak.

Emergency warning signs

- Severe or worsening symptoms of mpox require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using PCR of lesion swabs. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment options include tecovirimat (TPOXX) and supportive care.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is good
- Clade I (Central Africa) is more virulent than clade II (West Africa).

Chapter 545

Mucopolysaccharidoses

Overview

Mucopolysaccharidoses (MPS) are a group of lysosomal storage disorders caused by deficiency of enzymes required for the degradation of glycosaminoglycans (GAGs), leading to progressive accumulation in tissues. Hurler syndrome (MPS I-H, severe) is caused by α -L-iduronidase deficiency, presenting with coarse facies, corneal clouding, hepatosplenomegaly, dysostosis multiplex, cardiac valvular disease, and severe neurodegeneration. Hunter syndrome (MPS II, X-linked) is caused by iduronate-2-sulfatase deficiency, similar to MPS I but without corneal clouding. Sanfilippo syndrome (MPS III) is primarily neurodegenerative with severe behavioral disturbance, sleep disturbance, progressive intellectual decline, and seizures.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of mucopolysaccharidoses require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using urinary GAG screening with fractionation, enzyme assay, and genetic testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include hematopoietic stem cell transplantation for MPS I-H before age 2.5 years and enzyme replacement therapy for MPS I, II, and IVA. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is guarded
- Enzyme replacement improves somatic features but does not cross the blood-brain barrier.

Chapter 546

Mucormycosis

Overview

Causes

This condition happens because of spores being inhaled (rhinocerebral, lung) or inoculated through skin, with the fungus being angioinvasive, causing blood clot formation, tissue death due to lack of blood supply, and bleeding.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of mucormycosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- You may experience rhinocerebral mucormycosis (most common, particularly in DKA): fever, facial or periorbital pain, nasal congestion, black necrotic eschar,

- proptosis, vision loss, and cranial nerve palsies
- Lung mucormycosis: fever, cough, hemoptysis, and rapid progression.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using tissue biopsy showing broad, ribbon-like, non-septate hyphae with right-angle branching. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include urgent surgical removal of dead tissue and liposomal amphotericin B as first-line antifungal. outlook: rhinocerebral 50% mortality, lung 60%, disseminated 80–100%. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 547

Multiple Endocrine Neoplasia

Overview

Multiple endocrine neoplasia syndromes are autosomal dominant disorders characterized by tumors in two or more endocrine glands. MEN1 (Wermer syndrome) involves the parathyroids (primary hyperparathyroidism, most common), pancreas (gastrinoma, insulinoma, non-functional tumors), and pituitary (prolactinoma most common), caused by *menin* gene mutations. MEN2A (Sipple syndrome) involves medullary thyroid carcinoma (nearly 100% penetrance), pheochromocytoma, and primary hyperparathyroidism, caused by *RET* proto-oncogene mutations. MEN2B includes medullary thyroid carcinoma, pheochromocytoma, mucosal neuromas, marfanoid habitus, and intestinal ganglioneuromatosis.

Causes

This condition happens because of germline mutations in tumor suppressor genes or proto-oncogenes.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Your symptoms depend on the glands involved. Genetic testing guides prophylactic thyroidectomy in *RET* mutation carriers (within the first year of life for MEN2B, by age 5 for MEN2A).

Emergency warning signs

- Severe or worsening symptoms of multiple endocrine neoplasia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Your outlook depends on early detection and intervention.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 548

Multiple Gestation Complications

Overview

Multiple gestation (twins 3%, higher order multiples less common) carries increased risks of preterm birth, IUGR, preeclampsia, gestational diabetes, and placenta previa. Monochorionic diamniotic (MCDA) twins share a single placenta with potential vascular anastomoses, predisposing to twin-twin transfusion syndrome (TTTS, 10–15% of MCDA twins). Without treatment, severe TTTS has 80–90% fetal mortality.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of multiple gestation complications require immediate medical evaluation

Progression and Stages

TTTS your doctor will diagnose it by polyhydramnios (recipient twin) and oligohydramnios (donor twin), staged by the Quintero staging system (I–V). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable

symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Zygoty and chorionicity determine specific complications. Selective intrauterine growth restriction and twin reversed arterial perfusion sequence are additional complications.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include laser photocoagulation of communicating placental vessels or serial amnioreduction. Monoamniotic twins are at risk for cord entanglement and require close surveillance with planned cesarean delivery at 32–34 weeks. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 549

Multiple Myeloma

Overview

Multiple myeloma is a plasma cell tumor that accounts for 1% of all cancers and 13% of hematologic malignancies, with a median age at diagnosis of 70 years. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of clonal growth of cancerous plasma cells in the bone marrow, production of monoclonal immunoglobulin (M-protein), and end-organ damage summarized by the CRAB criteria: Calcium elevation (hypercalcemia), kidney insufficiency, Anemia, and Bone lesions (lytic lesions on imaging). Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)

- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience bone pain (vertebral compression fractures, pathologic fractures), fatigue (anemia), recurrent infections (immunoparesis—suppression of normal immunoglobulins), and kidney failure. Persistent lump or mass in the affected area
Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of multiple myeloma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs.

Higher stages generally indicate more advanced disease and poorer prognosis.

- Risk stratification uses the International Staging System (ISS) and revised ISS (R-ISS), with del(17p), t(4
- 14), and t(14
- 16) as high-risk cytogenetic abnormalities.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

To make the diagnosis, doctors look for $\geq 10\%$ clonal bone marrow plasma cells plus evidence of end-organ damage (CRAB) or biomarkers of malignancy (bone marrow plasma cells $\geq 60\%$, free light chain ratio ≥ 100 , or >1 focal lesion on MRI). Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)

- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment typically involves risk-adapted and typically involves induction (bortezomib, lenalidomide, dexamethasone—VRd), followed by autologous stem cell transplantation for eligible patients, followed by maintenance therapy (lenalidomide). Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells

- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

The outlook has improved to a median survival of 7–10 years with modern therapy, which includes novel agents such as proteasome inhibitors, immunomodulatory drugs, monoclonal antibodies (daratumumab), bispecific antibodies, and CAR-T cell therapy. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 550

Multiple Myeloma (Bone Manifestations)

Overview

Multiple myeloma is discussed in detail in Chapter ???. Bone disease is a hallmark feature, present in over 80% of patients at diagnosis. Myeloma cells produce factors (RANKL, MIP-1 α) that activate osteoclasts, leading to lytic bone lesions without osteoblastic repair. The most common sites are the spine, skull, ribs, pelvis, and long bones. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity

- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience pathologic fractures, pain, vertebral compression fractures with height loss and kyphosis, and spinal cord compression. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of multiple myeloma (bone manifestations) require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with

significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

To diagnose bone disease involves skeletal survey (low sensitivity), CT (high sensitivity for lytic lesions), MRI (detects marrow infiltration), and PET-CT. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)

- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment options include bisphosphonates (zoledronic acid, pamidronate) or denosumab to reduce skeletal events, radiation for symptomatic lesions, kyphoplasty/vertebroplasty for compression fractures, and myeloma-directed therapy. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy

- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 551

Multiple Sclerosis

Overview

This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of autoimmune-mediated demyelination. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience episodic neurological deficits such as optic neuritis, transverse myelitis, brainstem syndromes, and sensory or motor disturbances. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and

coordination Cognitive changes (memory loss, confusion, difficulty concentrating)
Speech and language difficulties Seizures or involuntary movements Dizziness and
vertigo Fatigue and sleep disturbances

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of multiple sclerosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Multiple sclerosis is a chronic autoimmune demyelinating disease of the central nervous system characterized by inflammatory demyelination and axonal injury occurring at different times and locations
- The relapsing-remitting form (RRMS) is most common, while progressive forms (primary progressive, secondary progressive) involve gradual neurological deterioration.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on MRI revealing periventricular, juxtacortical, infratentorial, and spinal cord T2-hyperintense lesions, often with gadolinium enhancement in active plaques
- Cerebrospinal fluid may show oligoclonal bands.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on disease-modifying therapies including interferons, glatiramer acetate, natalizumab, fingolimod, ocrelizumab, and others
- Acute relapses are treated with high-dose corticosteroids or plasmapheresis.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Recovery varies widely depending on disease course and treatment response. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 552

Mumps

Overview

This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of the virus entering through respiratory epithelium, replicating in the upper respiratory tract and regional lymph nodes, and causing viremia, with tropism for glandular tissue and the CNS. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of mumps require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

You may experience an incubation of 16–18 days, with prodrome (fever, headache, muscle pain, malaise) followed by parotitis (bilateral in 70–90%): painful, tender swelling of the parotid glands, with complications including orchitis (post-pubertal males, 20–30%), oophoritis, meningitis, pancreatitis, and sensorineural hearing loss.

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, confirmed by RT-PCR of buccal swab or IgM serology. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- Mumps is an acute viral illness caused by mumps virus, a paramyxovirus, with 500,000 cases annually worldwide before widespread vaccination, now less common in vaccinated populations but with outbreaks in close-contact settings. Most people recover well
- Prevention through MMR vaccine is highly effective.
- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use

- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment typically involves supportive. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global

burden of infectious diseases.

Chapter 553

Myasthenia Gravis

Overview

Myasthenia gravis (MG) is a neuromuscular junction disorder caused by autoantibodies against components of the postsynaptic acetylcholine receptor (AChR) or, less commonly, muscle-specific kinase (MuSK) or lipoprotein-related protein 4 (LRP4). Prevalence is approximately 20 per 100,000. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of impaired neuromuscular transmission due to antibody-mediated blockade or destruction of AChR. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)

- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- You may experience fluctuating, fatigable weakness that worsens with activity and improves with rest
- Ocular muscles are most commonly affected first (ptosis, diplopia), followed by bulbar muscles (difficulty swallowing, slurred speech, facial weakness), and limb and respiratory muscles. Myasthenic crisis (acute respiratory failure) occurs in 15–20% of patients.
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of myasthenia gravis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

In most cases, the outlook is good with treatment, though myasthenic crisis remains a life-threatening complication.

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension

- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Doctors diagnose this based on AChR or MuSK antibody testing, edrophonium (Tensilon) test (transient improvement), repetitive nerve stimulation (decremental response), and single-fiber EMG (increased jitter). Thymoma is present in 10–15% of patients. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Treatment options include acetylcholinesterase inhibitors (pyridostigmine), immunosuppression (prednisone, azathioprine, mycophenolate), thymectomy, and targeted therapies (eculizumab, ravulizumab [complement inhibitors], efgartigimod, rozanolixizumab [FcRn inhibitors]). Treatment aims to control inflammation, manage symptoms, and prevent disease flares. A step-up approach is often used, starting with symptomatic

treatments and progressing to immunosuppressive therapies as needed. Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Autoimmune diseases are generally chronic conditions requiring long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 554

Myelodysplastic Syndromes

Overview

Myelodysplastic syndromes are a heterogeneous group of clonal hematopoietic stem cell disorders that predominantly affect the elderly (median age 70–75). The condition is marked by dysplastic and ineffective hematopoiesis, peripheral cytopenias, and a risk of transformation to acute myeloid leukemia (AML). This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience symptoms of cytopenias: fatigue (anemia), infections (neutropenia), and bleeding (thrombocytopenia).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of myelodysplastic syndromes require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for bone marrow biopsy showing dysplasia ($\geq 10\%$ of cells in one or more myeloid lineages), blast percentage ($< 20\%$ —above 20% classifies as AML), and cytogenetic/molecular abnormalities (del(5q), del(7q), del(20q), SF3B1, TET2, ASXL1 mutations). Risk stratification uses the International Prognostic Scoring System (IPSS-R, now IPSS-M).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care
Enzyme replacement therapy for certain conditions
Gene therapy (emerging for select conditions)
Dietary modifications (e.g., phenylketonuria)
Regular monitoring for associated complications
Physical and occupational therapy
Genetic counseling for family planning
Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment for low-risk MDS includes supportive care, erythropoiesis-stimulating agents ($\pm$ lenalidomide for del(5q) MDS), and luspatercept, while higher-risk MDS doctors typically treat it with hypomethylating agents (azacitidine, decitabine).

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on risk category, with allogeneic stem cell transplantation being the only curative option, typically for patients <75 with a donor.

Chapter 555

Myelofibrosis

Overview

Myelofibrosis is a myeloproliferative tumor characterized by clonal megakaryocyte growth, bone marrow scarring (reticulin and/or collagen deposition), extramedullary hematopoiesis (splenomegaly, hepatomegaly), and progressive cytopenias, which may arise de novo (primary MF) or evolve from PV or ET (post-PV/post-ET MF). The condition is driven by JAK2, CALR, or MPL mutations in over 90% of cases, with ASXL1, EZH2, SRSF2 mutations associated with worse outlook. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience massive splenomegaly (early satiety, abdominal discomfort), constitutional symptoms (fatigue, weight loss, night sweats, fever), bone pain, and progressive anemia requiring transfusions.

Emergency warning signs

- Severe or worsening symptoms of myelofibrosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for bone marrow biopsy showing megakaryocyte growth and atypia, reticulin/collagen scarring, and exclusion of other causes. The Dynamic International Prognostic Scoring System (DIPSS) and molecular-enhanced versions (MIPSS70) guide treatment. Management options include ruxolitinib (JAK1/2 inhibitor, improves splenomegaly and symptoms), fedratinib (JAK2 inhibitor), momelotinib (JAK1/2 inhibitor with anemia benefit), and allogeneic stem cell transplantation (the only curative option). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person depending on risk category. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 556

Myocardial Infarction

Overview

heart attack is irreversible myocardial tissue death resulting from prolonged reduced blood flow, most commonly caused by acute thrombotic occlusion of a coronary artery at the site of a ruptured or eroded atherosclerotic plaque. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of complete or near-complete coronary artery occlusion. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity

- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience crushing chest pain, radiation to the left arm or jaw, diaphoresis, nausea, and shortness of breath. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of myocardial infarction require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection

- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this based on ECG (ST-elevation MI shows ST-segment elevation

- Non-ST-elevation MI presents without ST elevation but with elevated cardiac biomarkers) and elevated cardiac troponin. Treatment for STEMI requires emergent reperfusion therapy with primary through the skin coronary intervention within 90 minutes or fibrinolysis within 30 minutes of first medical contact
- Treatment options include dual antiplatelet therapy, anticoagulation, beta-blockers, ACE inhibitors, and statins.
- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

The outlook has improved with timely reperfusion and long-term secondary prevention.

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)

- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 557

Myocarditis

Overview

Myocarditis is an inflammation of the myocardium, most commonly caused by viral infection (Coxsackievirus B, adenovirus, parvovirus B19, SARS-CoV-2), but also by autoimmune diseases, drugs (immune checkpoint inhibitors), and toxins.

Causes

This condition happens because of direct viral injury or immune-mediated myocardial damage.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms can range from mild chest pain and myalgias to fulminant heart failure, cardiogenic shock, and sudden death.

Emergency warning signs

- Severe or worsening symptoms of myocarditis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Diagnosis typically involves elevated troponin, ECG changes (diffuse ST elevation), echocardiography showing regional or global wall motion abnormalities, and cardiac MRI with Lake Louise criteria
- Endomyocardial biopsy remains the gold standard for definitive diagnosis.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves supportive, with guideline-directed medical therapy for heart failure
- Immunosuppression may be considered in autoimmune or giant cell myocarditis.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable, with most patients recovering, but a subset develops dilated cardiomyopathy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 558

Myofascial Pain Syndrome

Overview

Myofascial pain syndrome (MPS) is a chronic pain syndrome characterized by the presence of trigger points—hyperirritable, taut bands of skeletal muscle that are painful on palpation and produce referred pain. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of dysfunctional motor endplates with excessive acetylcholine release, sustained sarcomere contraction, local reduced blood flow, and release of inflammatory mediators. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience regional pain (not widespread like fibromyalgia), with active trigger points causing spontaneous pain and latent trigger points painful only with palpation

- Common sites include the trapezius, levator scapulae, masseter, and gluteal muscles.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of myofascial pain syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include trigger point injections (dry needling or with local anesthetic), spray and stretch, physical therapy, posture correction, and ergonomic

modifications

- Botulinum toxin injections may be used for refractory cases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment.

Chapter 559

Myopia

Overview

Myopia is the most common refractive error worldwide, affecting approximately 30% of the global population and approaching 80–90% in some East Asian countries.

Causes

This condition happens because of an axial length of the eye that is too long relative to the refractive power of the cornea and lens, causing light to focus in front of the retina. High myopia (sphere -6.00 diopters or greater) increases the risk of retinal detachment, choroidal neovascularization, glaucoma, and cataract.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience blurred distance vision with preserved near vision.

Emergency warning signs

- Severe or worsening symptoms of myopia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Treatment focuses on correction with concave (minus) lenses, contact lenses, or refractive surgery (LASIK, PRK, ICL)
- Atropine eye drops and orthokeratology lenses slow myopia progression in children.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on refraction testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with correction. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 560

Myotonic Dystrophy

Overview

- Myotonic dystrophy (DM) is the most common muscular dystrophy in adults, with an autosomal dominant inheritance pattern. Type 1 (DM1, Steinert disease) results from a CTG trinucleotide repeat expansion in the DMPK gene on chromosome 19
- Type 2 (DM2, proximal myotonic myopathy) results from a CCTG expansion in the CNBP gene.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features in DM1 include distal and facial muscle weakness, myotonia (delayed muscle relaxation after contraction), cataracts (posterior subcapsular "Christmas tree" cataracts), cardiac conduction abnormalities, endocrine dysfunction, and thinking and memory impairment. Genetic anticipation (progressively earlier onset and more severe disease in successive generations) occurs.

Emergency warning signs

- Severe or worsening symptoms of myotonic dystrophy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

The outlook varies from person to person, with progressive disability and reduced life expectancy due to cardiac and respiratory complications.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on genetic testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves symptomatic: mexiletine for myotonia, cardiac pacemaker/defibrillator for conduction defects, and Treatment for endocrine and other systemic manifestations. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical

intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 561

NREM Parasomnias: Confusional Arousals

Overview

Confusional arousals are episodes of mental confusion, disorientation, and inappropriate behavior (slow speech, fumbling, ineffective actions) upon arousal from NREM sleep, typically N3. Episodes are more common in children. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of incomplete arousal from slow-wave sleep. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience confusion and disorientation upon awakening without the complex mobility of sleepwalking or the intense autonomic activation of sleep terrors. Severe confusional arousal in adults is linked to obstructive sleep apnea, shift work, and psychiatric illness. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of nrem parasomnias: confusional arousals require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam. Management addresses underlying sleep-disordered breathing and triggers
- Clonazepam may be used in severe cases.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 562

NREM Parasomnias: Sleep-Related Eating Disorder

Overview

Sleep-related eating disorder (SRED) is a disorder involving recurrent episodes of involuntary eating and drinking during partial arousals from NREM sleep, with impaired awareness and variable amnesia for the events. Prevalence is 1–3% in the general population, higher in women. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of abnormal arousal from NREM sleep and is strongly associated with other NREM parasomnias and hypnotic use (particularly zolpidem).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience consumption of unusual or dangerous items during the night with subsequent weight gain, dental injury, and potential ingestion of toxic substances.

Emergency warning signs

- Severe or worsening symptoms of nrem parasomnias: sleep-related eating disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include discontinuation of triggering hypnotics, treatment of comorbid sleep apnea or restless legs syndrome, topiramate (50–200 mg nightly), and safety measures. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 563

NREM Parasomnias: Sleep Terrors

Overview

Sleep terrors (pavor nocturnus) are dramatic arousals from NREM slow-wave sleep, characterized by sudden sitting up, piercing screams, intense autonomic activation (tachycardia, tachypnea, mydriasis, sweating), and behavioral expressions of intense fear. Peak prevalence is in children ages 1.5–7 years (1–6%). This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of incomplete arousal from deep NREM sleep.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience episodes lasting 1–10 minutes with the individual inconsolable, confused, and potentially thrashing violently, followed by return to sleep with no recall of the event.

Emergency warning signs

- Severe or worsening symptoms of nrem parasomnias: sleep terrors require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include reassurance, parental education, safety measures, and clonazepam for severe cases. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive
- Most children outgrow the condition.

Chapter 564

NREM Parasomnias: Sleepwalking

Overview

Sleepwalking (somnambulism) is a disorder of arousal from NREM sleep, typically arising from N3 (slow-wave) sleep during the first third of the night. Prevalence is 10–30% in children and 2–4% in adults. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of incomplete arousal from slow-wave sleep, with genetic factors playing a strong role.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience complex, semi-purposeful behaviors ranging from sitting up to walking and performing routine activities without conscious awareness, with a blank stare and amnesia for the event. Triggers include sleep deprivation, fever, alcohol, stress, and sedative-hypnotic use.

Emergency warning signs

- Severe or worsening symptoms of nrem parasomnias: sleepwalking require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Polysomnography is indicated for atypical, frequent, or potentially injurious episodes.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include safety precautions, avoidance of triggers, scheduled awakening before typical episode time, and pharmacotherapy (clonazepam, diazepam) for frequent or injurious episodes. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive
- Most children outgrow the condition.

Chapter 565

Narcissistic Personality Disorder

Overview

Narcissistic personality disorder has a prevalence of 0.5–1% in the general population, with a male predominance (60–75%). The condition may involve exaggerated self-enhancement to compensate for underlying fragile self-esteem, deficits in mentalization, and altered frontolimbic connectivity. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a pervasive pattern of grandiosity (in fantasy or behavior), need for admiration, and lack of empathy, with required criteria including at least five of the following: grandiose sense of self-importance, preoccupation with fantasies of unlimited success, belief of being special and unique, need for excessive admiration, sense of entitlement, interpersonal exploitativeness, lack of empathy, envy of others, and arrogant behaviors or attitudes. Vulnerable (covert) narcissism is a subtype characterized by hypersensitivity to evaluation and chronic feelings of shame.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of narcissistic personality disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include psychotherapy (transfer-focused, mentalization-based, schema therapy). Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Grandiosity may diminish with age.

Chapter 566

Narcolepsy Type 1

Overview

Narcolepsy type 1 (narcolepsy with cataplexy) is a chronic neurological disorder characterized by excessive daytime sleepiness, cataplexy (sudden, transient episodes of bilateral skeletal muscle weakness triggered by strong emotions), sleep paralysis, hypnagogic hallucinations, and fragmented nighttime sleep. The prevalence is approximately 25–50 per 100,000 individuals, with onset peaking in adolescence and young adulthood.

Causes

This condition results from disruption of normal narcolepsy type 1 function.

- This condition happens because of autoimmune-mediated destruction of hypocretin (orexin)-producing neurons in the lateral hypothalamus, leading to severely decreased or absent CSF hypocretin-1 levels
- Genetic susceptibility is strongly linked to HLA-DQB1*06:02 (present in >90% of cases).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience excessive daytime sleepiness, cataplexy (brief episodes of bilateral weakness provoked by laughter or surprise), sleep paralysis, and hypnagogic hallucinations.

Emergency warning signs

- Severe or worsening symptoms of narcolepsy type 1 require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for polysomnography followed by MSLT demonstrating mean sleep latency ≤ 8 minutes and two or more SOREM naps, plus either cataplexy or CSF hypocretin-1 deficiency. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include scheduled naps, wake-promoting agents (modafinil, armodafinil, pitolisant, solriamfetol), anti-cataplexy agents (sodium oxybate, antidepressants), and sodium oxybate for both EDS and cataplexy. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is chronic but symptoms can be controlled with appropriate therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 567

Narcolepsy Type 2

Overview

Narcolepsy type 2 is a chronic neurological disorder characterized by excessive daytime sleepiness and objective findings on MSLT (mean sleep latency ≤ 8 minutes with two or more SOREMs) but without cataplexy and with normal CSF hypocretin-1 levels. It represents 15–25% of narcolepsy cases.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features are generally less severe than type 1
- Some patients may eventually develop cataplexy and transition to type 1.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of narcolepsy type 2 require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for polysomnography followed by MSLT demonstrating the characteristic findings without cataplexy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on wake-promoting agents (modafinil, armodafinil, pitolisant, solriamfetol) and behavioral sleep hygiene. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is chronic but manageable. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 568

Nasal Fracture

Overview

- Nasal fractures are the most common facial fracture, accounting for approximately 40% of all facial fractures, due to the prominent position of the nose
- They result from direct trauma (assault, sports injuries, falls).

Causes

This condition happens because of direct impact causing bony or cartilaginous disruption.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of nasal fracture require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supplemented by CT for complex fractures. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- You may experience epistaxis, nasal pain, swelling, bruising, and deformity
- Nasal septal hematoma may coexist and requires urgent removal of fluid to prevent septal tissue death and saddle-nose deformity.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on closed reduction within 7–10 days of injury
- Severe or comminuted fractures may require open reduction and internal fixation.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with timely treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 569

Nasal Polyps

Overview

Nasal polyps are soft, pale, edematous masses of inflamed sinonasal mucosa that most commonly arise from the ethmoid sinuses and middle meatus, associated with type 2 inflammation, chronic rhinosinusitis, asthma, aspirin-exacerbated respiratory disease, and cystic scarring.

Causes

This condition happens because of eosinophilic, IgE-mediated mucosal inflammation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience nasal obstruction, loss of smell or hyposmia, rhinorrhea, and postnasal drip
- Large or multiple polyps can cause facial deformity.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of nasal polyps require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using anterior rhinoscopy or nasal endoscopy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include intranasal corticosteroids as first-line therapy, short courses of oral corticosteroids, surgical polypectomy, and dupilumab (anti-IL-4R α monoclonal antibody) for refractory disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with recurrence being common. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 570

Necrotizing Enterocolitis (NEC)

Overview

Necrotizing enterocolitis (NEC) is a devastating intestinal inflammatory disease of premature infants, with an incidence of 5–10% in very low birth weight infants. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of a dysregulated inflammatory response to intestinal microbes leading to transmural tissue death, most commonly affecting the terminal ileum and proximal colon. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)

- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- You may experience abdominal distension, feeding intolerance, bloody stools, lethargy, and temperature instability. Pneumatosis intestinalis (gas in the bowel wall) on abdominal radiograph is a hallmark sign. Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of necrotizing enterocolitis (nec) require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

- Treatment options include bowel rest, nasogastric decompression, broad-spectrum antibiotics, and supportive care
- Surgical intervention is required for perforation or progressive disease.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis

- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with radiographic confirmation. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for

chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

- Outlook is guarded
- Mortality ranges from 20–50%.

Chapter 571

Necrotizing Fasciitis

Overview

Necrotizing fasciitis is a rapidly progressive, life-threatening infection of the subcutaneous tissue and fascia, with secondary involvement of skin and muscle, with 500–1500 cases annually in the US, higher incidence in immunocompromised patients, diabetics, intravenous drug users, and after trauma or surgery, with mortality of 20–40%.

Causes

This condition happens because of infection spreading along the superficial and deep fascia, causing thrombotic occlusion of perforating blood vessels, leading to ischemic tissue death, with Type I (polymicrobial, 70%) involving anaerobes and facultative organisms, Type II (monomicrobial, 20%) involving *Streptococcus pyogenes* and *Staphylococcus aureus*, and Type III (gram-negative, marine-related) involving *Vibrio vulnificus*.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pain out of proportion to examination (earliest sign), rapidly spreading redness of the skin and swelling, induration, skin discoloration, involving bleeding bullae, crepitus, and systemic toxicity.

Emergency warning signs

- Severe or worsening symptoms of necrotizing fasciitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, with surgical exploration being paramount. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Management requires urgent, aggressive surgical removal of dead tissue, empirical broad-spectrum antibiotics, and intensive supportive care.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is guarded, with mortality heavily dependent on prompt surgical intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 572

Neonatal Abstinence Syndrome

Overview

Neonatal abstinence syndrome (NAS) is a postnatal drug withdrawal syndrome in newborns exposed to opioids or other substances in utero. Incidence has risen dramatically with the opioid epidemic. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition results from disruption of normal neonatal abstinence syndrome function.

- This condition happens because of chronic fetal exposure to opioids downregulating CNS opioid receptors
- Abrupt cessation at birth causes neurotransmitter dysregulation and autonomic dysfunction.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features develop within 24–72 hours of birth and include CNS irritability (high-pitched cry, tremors, hypertonia, seizures), autonomic dysfunction (sweating, fever, yawning, sneezing, mottling), and GI dysfunction (poor feeding, vomiting, diarrhea).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of neonatal abstinence syndrome require immediate medical evaluation

Progression and Stages

Diagnosis relies on maternal history with the Finnegan Neonatal Abstinence Scoring System quantifying severity. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Treatment: non-pharmacologic care (rooming-in, breastfeeding if mother is on MAT, swaddling, low-stimulation environment) is first-line
- Pharmacologic therapy includes oral morphine, methadone, or buprenorphine.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with treatment
- Long-term outcomes include increased risk of developmental delays.

Chapter 573

Neonatal Hypoglycemia

Overview

Neonatal hypoglycemia is defined as plasma glucose $<45\text{--}50$ mg/dL in the first 48 hours of life, though the precise threshold associated with neurodevelopmental impairment remains debated. It occurs in 5–15% of all newborns and up to 50% of infants of diabetic mothers. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features are nonspecific: jitteriness, irritability, lethargy, seizures, apnea, poor feeding, and hypothermia.

Emergency warning signs

- Severe or worsening symptoms of neonatal hypoglycemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for timely glucose measurement. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include early feeding, intravenous dextrose, and monitoring of glucose levels
- Persistent or severe hypoglycemia requires evaluation for hyperinsulinism, fatty acid oxidation disorders, and other metabolic diseases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with timely treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 574

Neonatal Jaundice / Hyperbilirubinemia / Kernicterus

Overview

Neonatal jaundice (hyperbilirubinemia) is the yellow discoloration of the skin and sclerae due to elevated serum bilirubin, occurring in 60–80% of term and nearly all preterm newborns. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

This condition results from disruption of normal neonatal jaundice / hyperbilirubinemia / kernicterus function.

- Physiologic jaundice results from increased bilirubin production, immature liver conjugation, and decreased bilirubin excretion. Pathologic jaundice is defined by onset <24 hours of age, rapid rise (>5 mg/dL/day), elevated direct bilirubin, or persistence beyond 2 weeks
- Causes include hemolytic disease, sepsis, Crigler-Najjar syndrome, Gilbert syndrome, biliary atresia, and breast milk jaundice. Unconjugated bilirubin is neurotoxic
- At high levels it crosses the blood-brain barrier and causes bilirubin-induced nervous system dysfunction and kernicterus. Diagnosis includes total and direct serum bilirubin, blood type and Coombs test, G6PD screening, and evaluation for sepsis.

Risk Factors

- The right treatment depends on gestational age, risk factors, and bilirubin level: phototherapy is first-line
- Exchange transfusion for severe cases or signs of acute bilirubin encephalopathy.
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of neonatal jaundice / hyperbilirubinemia / kernicterus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with timely treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 575

Neonatal Sepsis (Early-Onset and Late-Onset)

Overview

Neonatal sepsis is a systemic bacterial infection in the first 28 days of life. Early-onset sepsis (<72 hours of age) results from vertical transmission from the maternal genital tract, most commonly Group B Streptococcus and Escherichia coli. Late-onset sepsis (>72 hours of age) is nosocomial or community-acquired, with Coagulase-negative staphylococci, S. aureus, E. coli, and Klebsiella being common pathogens. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Clinical features are nonspecific: temperature instability, respiratory distress, apnea, bradycardia, feeding intolerance, lethargy, irritability, and seizures. Diagnosis includes blood culture (gold standard), complete blood count, C-reactive protein, and lumbar puncture. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of neonatal sepsis (early-onset and late-onset) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Detailed history including exposure, travel, and vaccination status
Physical examination focusing on the affected system
Complete blood count with differential
Microbiological culture of blood, sputum, urine, or wound
PCR and nucleic acid amplification tests for rapid pathogen detection
Antigen detection tests
Serological testing for antibodies (IgM, IgG)
Imaging studies (chest X-ray, CT, ultrasound)
Gram stain and microscopy of clinical specimens
Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- Treatment options include empiric broad-spectrum antibiotics and supportive care
- Preventative intrapartum antibiotic prophylaxis for GBS-colonized women has significantly reduced early-onset sepsis incidence.
- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures

- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Your outlook depends on prompt recognition and treatment. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 576

Nephritic Syndrome

Overview

Nephritic syndrome is a clinical complex resulting from acute glomerular inflammation characterized by leukocyte infiltration, cellular growth, and reactive glomerular capillary wall injury. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of immune complex deposition or antibody binding within the glomerulus, causing a decrease in glomerular filtration rate (GFR) and breakdown of the capillary barrier to red blood cells and protein. Etiologies include poststreptococcal glomerulonephritis (PSGN), IgA nephropathy (Berger disease), lupus nephritis, and anti-GBM disease (Goodpasture syndrome). Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the classic triad of hematuria ('cola-colored' or dysmorphic RBCs and RBC casts), high blood pressure (due to fluid retention), and oliguria with mild-to-moderate proteinuria (< 3.5 g/day) and facial/periorbital swelling. Diagnosis is supported by urinalysis showing hematuria and proteinuria, elevated serum creatinine, low serum complement levels (in PSGN and lupus), and definitive kidney biopsy.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of nephritic syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on fluid and sodium restriction, antihypertensive therapy (diuretics, ACE inhibitors), and targeted immunosuppressants (corticosteroids, cyclophosphamide) for rapidly progressive cases. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on underlying histology.

Chapter 577

Nephrolithiasis

Overview

Nephrolithiasis (kidney stones) affects approximately 10% of the population, with higher prevalence in males and hot climates. Stone types include calcium oxalate (most common, 70–80%), calcium phosphate, uric acid (associated with hyperuricemia, low urine pH, and metabolic syndrome), struvite (magnesium ammonium phosphate, associated with urease-producing organisms in chronic UTI—"staghorn calculi"), and cystine (rare, hereditary cystinuria).

Causes

This condition happens because of supersaturation of stone-forming salts in urine.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience kidney colic (severe, colicky flank pain radiating to the groin), hematuria, nausea, and urinary urgency.

Emergency warning signs

- Severe or worsening symptoms of nephrolithiasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on CT abdomen/pelvis without contrast (gold standard for acute evaluation). Treatment for acute kidney colic includes NSAIDs, opioids, antiemetics, and alpha-blockers (medical expulsive therapy). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Prevention includes increased fluid intake (>2.5 L/day), dietary modifications, and pharmacotherapy (thiazide diuretics for hypercalciuria, allopurinol for uric acid stones, potassium citrate for hypocitraturia). The outlook is good with preventive measures
- Recurrence rates are high without prophylaxis.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 578

Nephrotic Syndrome

Overview

Nephrotic syndrome is a clinical complex resulting from severe glomerular podocyte effacement and disruption of the charge and size barrier of the glomerular basement membrane. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition results from disruption of normal nephrotic syndrome function.

- This condition happens because of primary kidney podocytopathies or systemic disease damage to the filtration barrier. Primary etiologies include minimal change disease (most common in children), focal segmental glomerulosclerosis (FSGS, most common in African Americans), and membranous nephropathy (most common in Caucasian adults)
- Secondary etiologies include diabetic nephropathy, amyloidosis, and lupus nephritis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the hallmark tetrad: heavy proteinuria (> 3.5 g/24 hours or spot urine protein-to-creatinine ratio > 3.5), hypoalbuminemia (< 3.0 g/dL), generalized peripheral and periorbital swelling (anasarca due to decreased oncotic pressure and secondary sodium retention), and hyperlipidemia with lipiduria ('oval fat bodies' and Maltese cross appearance under polarized light).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of nephrotic syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Complications include deep vein blood clot formation/lung blockage by a blood clot (due to urinary loss of antithrombin III) and infection (loss of immunoglobulins).

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using 24-hour urine collection or spot ratio, serum chemistries, and kidney biopsy.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include ACE inhibitors/ARBs (to reduce intraglomerular pressure), loop diuretics, statins, anticoagulation when indicated, and systemic corticosteroids or immunosuppressive agents. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Recovery varies by underlying cause.

Chapter 579

Nesidioblastosis and Congenital Hyperinsulinism

Overview

Nesidioblastosis (persistent hyperinsulinemic hypoglycemia of infancy) is a rare disorder of pancreatic endocrine cell dysplasia characterized by diffuse enlargement, abnormal increase in cells, and neoformation of insulin-producing pancreatic β -cells from ductal epithelium. Driven by inactivating mutations in *ABCC8* or *KCNJ11* genes encoding the ATP-sensitive potassium channel (K_{ATP}) in β -cells, the condition leads to unregulated, continuous insulin secretion unresponsive to blood glucose levels. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features present in neonates or infants with severe, intractable hypoglycemia, lethargy, seizures, and risk of irreversible neurodevelopmental damage.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of nesidioblastosis and congenital hyperinsulinism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors confirm the diagnosis through persistent endogenous hyperinsulinemia, elevated C-peptide, and negative sulfonylurea screening during hypoglycemia.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on IV glucose, diazoxide (K_{ATP} channel opener), octreotide, and subtotal or near-total (95%) pancreatectomy for medical treatment failure. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria)

Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 580

Neural Tube Defects

Overview

- Incidence is 0.5–1 per 1000 pregnancies
- Risk is reduced by 50–70% with periconceptional folic acid supplementation. Spina bifida encompasses a spectrum from spina bifida occulta (vertebral involvement only), meningocele (meningeal herniation), to meningocele (herniation of meninges and spinal cord tissue causing nervous system deficits). Anencephaly is a fatal NTD with absence of the cerebral hemispheres.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of neural tube defects require immediate medical evaluation

Progression and Stages

Neural tube defects (NTDs) are congenital malformations resulting from failure of neural tube closure during the fourth week of embryonic development, classified as open or closed. Your outlook depends on the level and severity of the lesion. The condition may

progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using prenatal ultrasound and maternal serum alpha-fetoprotein screening. Treatment for meningomyelocele includes prenatal or postnatal surgical repair and multidisciplinary care. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 581

Neuroblastoma

Overview

This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

- Neuroblastoma is the most common extracranial solid tumor in children and the most common malignancy in infants. It arises from the sympathetic nervous system, most commonly in the adrenal medulla (40%) or paraspinal sympathetic chain. Incidence is 1 in 7000 children
- Median age at diagnosis is 18 months.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression

- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Your symptoms depend on the site: abdominal mass (adrenal), Horner syndrome (cervical), opsoclonus-myoclonus-loss of coordination (paraneoplastic), proptosis and periorbital ecchymoses (orbital metastases), and bone pain (skeletal metastases). Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of neuroblastoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using biopsy with histology, MYCN amplification status, and imaging. International Neuroblastoma Risk Group staging classifies tumors as low-risk, intermediate-risk, or high-risk. Treatment varies by risk group: low-risk tumors may be observed or surgically removed

- Intermediate-risk tumors receive chemotherapy
- High-risk tumors require intensive multimodal therapy. outlook ranges from excellent (>95% survival in low-risk) to poor (40–50% survival in high-risk).
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing

infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications

- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 582

Neurocysticercosis

Overview

Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of encystment of larval forms in brain parenchyma, ventricles, or subarachnoid space. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of neurocysticercosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Neurocysticercosis is the most common parasitic infection of the central nervous system, caused by the larval stage of *Taenia solium* (pork tapeworm)
- It is endemic in Latin America, Asia, and Africa. Clinical manifestations depend on cyst location and stage and include seizures (the most common presentation worldwide), headache, focal deficits, and hydrocephalus.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable with appropriate treatment. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 583

Neurofibromatosis Type 1

Overview

Neurofibromatosis type 1 (NF1, von Recklinghausen disease) is an autosomal dominant neurocutaneous disorder caused by mutations in the NF1 gene encoding neurofibromin, a Ras-GTPase tumor suppressor, affecting 1 in 3,000 births. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of loss of neurofibromin function, leading to uncontrolled Ras signaling and increased cell growth. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience café-au-lait spots (≥ 6 spots > 5 mm in children, > 15 mm in adults), axillary and inguinal freckling, Lisch nodules (iris hamartomas), neurofibromas (cutaneous, subcutaneous, plexiform), optic pathway gliomas, osseous dysplasia, and increased risk of cancerous peripheral nerve sheath tumors. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of neurofibromatosis type 1 require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on NIH consensus criteria (≥ 2 of 7 features).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on annual ophthalmologic and nervous system monitoring, surgical removal of symptomatic neurofibromas, MEK inhibitor therapy (selumetinib) for symptomatic plexiform neurofibromas, and surveillance for malignancies. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Life expectancy is reduced by 10–15 years, primarily due to malignancy and cardiovascular disease.

Chapter 584

New Daily Persistent Headache

Overview

New daily persistent headache (NDPH) is a primary headache disorder characterized by a sudden-onset headache that becomes continuous within 24 hours and persists for >3 months. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of unclear Disease mechanism, with triggers including a viral or flu-like illness (40–50%), stressful life event, surgery, or travel. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bilateral, pressing/tightening, moderate intensity pain

- The onset is clearly remembered by the patient (“a thunderclap headache that never went away”).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of new daily persistent headache require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for rigorous exclusion of secondary causes (subarachnoid bleeding, cerebral venous sinus blood clot formation, meningitis, intracranial hypotension). Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves challenging, involving a multidisciplinary approach with amitriptyline, gabapentin, topiramate, and physical therapy. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- The self-limiting subtype typically resolves within 2 years, while the refractory form causes significant disability.

Chapter 585

Non-24-Hour Sleep-Wake Rhythm Disorder

Overview

Non-24-hour sleep-wake rhythm disorder (N24) is a circadian rhythm sleep-wake disorder characterized by a sleep-wake cycle that is desynchronized from the 24-hour day, typically running at a period longer than 24 hours (free-running), with each day sleep onset and wake time drifting progressively later by 30–60 minutes. It occurs in 50–80% of blind individuals (due to lack of photic entrainment of the SCN) and rarely in sighted individuals. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of absence of light perception to entrain the circadian clock.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a progressive daily delay in sleep-wake timing cycling through the 24-hour day.

Emergency warning signs

- Severe or worsening symptoms of non-24-hour sleep-wake rhythm disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- To make the diagnosis, doctors look for at least 14 days of actigraphy or sleep diaries demonstrating a circadian period other than 24 hours. Management in blind individuals includes melatonin (0.5–10 mg at desired bedtime) and tasimelteon (approved for N24 in blind people)
- In sighted individuals, Treatment options include morning light therapy and evening melatonin.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is chronic but manageable. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 586

Non-Alcoholic Fatty Liver Disease

Overview

Non-alcoholic fatty liver disease (NAFLD), now increasingly termed metabolic dysfunction-associated steatotic liver disease (MASLD), is the most common chronic liver disease worldwide, affecting approximately 25% of the global population, and is strongly associated with metabolic syndrome, obesity, type 2 diabetes mellitus, and dyslipidemia.

Causes

This condition happens because of liver steatosis due to insulin resistance and metabolic dysfunction. The spectrum ranges from simple steatosis to non-alcoholic steatohepatitis (NASH/MASH), which involves hepatocyte injury, inflammation, and scarring, and can progress to cirrhosis and hepatocellular carcinoma.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features are often absent, with elevated transaminases detected incidentally.

Emergency warning signs

- Severe or worsening symptoms of non-alcoholic fatty liver disease require immediate medical evaluation

Progression and Stages

Your outlook depends on the stage of scarring. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using imaging (ultrasound, CT, MRI-PDFF) showing liver steatosis and exclusion of other causes
- Liver biopsy remains the gold standard for staging.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 587

Non-Allergic Rhinitis

Overview

Non-allergic rhinitis is a chronic nasal condition that encompasses symptoms without an IgE-mediated mechanism, including vasomotor, drug-induced, hormonal, and gustatory forms.

Causes

Vasomotor rhinitis results from hypersensitivity of trigeminal nerve endings leading to neurogenic inflammation triggered by environmental factors (temperature changes, strong odors, irritants). Drug-induced rhinitis results from overuse of topical decongestants (rhinitis medicamentosa).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience clear rhinorrhea and nasal congestion.

Emergency warning signs

- Severe or worsening symptoms of non-allergic rhinitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using exclusion of allergic and infectious causes. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include intranasal corticosteroids, ipratropium bromide spray for rhinorrhea, and avoidance of triggers. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person depending on the ability to avoid triggers. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 588

Non-Gonococcal Urethritis (NGU)

Overview

Non-gonococcal urethritis is urethral inflammation without *Neisseria gonorrhoeae* infection, the most common urethral syndrome in sexually active men, with *Chlamydia trachomatis* (serovars D–K) accounting for 30–50% of cases, *Mycoplasma genitalium* (15–25%), *Trichomonas vaginalis* (5–10%), and other causes.

Causes

This condition happens because of infection of the urethral epithelium triggering a neutrophilic inflammatory response.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of non-gonococcal urethritis (ngu) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

You may experience an incubation of 2–35 days, with dysuria, urethral discharge (mucoid to mucopurulent), and urethral itching, with many infections being asymptomatic, and complications including epididymitis, prostatitis, and reactive arthritis.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using first-void urine NAAT for *C. trachomatis*, *N. gonorrhoeae*, *M. genitalium*, and *T. vaginalis*. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include azithromycin or doxycycline for chlamydia, with moxifloxacin as second-line for *M. genitalium*. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and

rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 589

Non-Hodgkin Lymphoma

Overview

This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)

- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Symptoms vary depending on subtype: aggressive lymphomas (DLBCL) present with rapidly growing masses, B symptoms, and organ compromise
- Indolent lymphomas (follicular) present with waxing and waning painless lymphadenopathy.
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of non-hodgkin lymphoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using lymph node biopsy with histopathology, immunophenotyping, cytogenetics, and molecular studies. Treatment for DLBCL is R-CHOP (rituximab, cyclophosphamide, doxorubicin, vincristine, prednisone), with approximately 60–70% cure rate, while indolent NHL may be observed (watch and wait) or treated with rituximab-based regimens. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies

- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay

physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Recovery varies widely by subtype. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 590

Normal Pressure Hydrocephalus

Overview

Normal pressure hydrocephalus (NPH) is a potentially reversible cause of dementia and gait impairment, characterized by the classic triad of gait apraxia (magnetic gait—feet appear glued to the floor), thinking and memory decline (subcortical pattern), and urinary incontinence (late symptom). Prevalence increases with age: up to 5–6% of nursing home residents. Ventriculomegaly is present on brain imaging without significant cortical shrinkage or wasting, and lumbar puncture opening pressure is normal (≤ 20 cm HH_2O).

Causes

This condition happens because of impaired CSF absorption.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the triad of gait apraxia, thinking and memory decline, and urinary incontinence. Diagnosis is supported by clinical improvement after high-volume lumbar puncture (CSF tap test) and Evans index >0.3 .

Emergency warning signs

- Severe or worsening symptoms of normal pressure hydrocephalus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves ventriculoperitoneal shunt placement, which improves gait in 70–80% of appropriately selected patients. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with treatment
- Patient selection is critical for good outcomes.

Chapter 591

Norovirus

Overview

Norovirus is a single-stranded RNA virus (family Caliciviridae) and the leading cause of acute gastroenteritis worldwide, responsible for 685 million cases and 200,000 deaths annually, with all age groups affected and peaks in winter months (“winter vomiting disease”).

Causes

This condition happens because of the virus infecting enterocytes of the small intestine, causing blunting and shrinkage or wasting of intestinal villi, crypt abnormal increase in cells, and increased apoptosis, reducing absorptive capacity.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 12–48 hours, with sudden onset of nausea, vomiting (more prominent in children), watery diarrhea (non-bloody), abdominal cramps, and low-grade fever, with symptoms typically lasting 24–72 hours.

Emergency warning signs

- Severe or worsening symptoms of norovirus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using RT-PCR of stool. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment typically involves supportive care with oral rehydration therapy.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well in immunocompetent hosts. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 592

Nummular Dermatitis

Overview

Nummular (discoïd) dermatitis is a chronic inflammatory skin condition presenting as coin-shaped, well-demarcated, erythematous plaques with serous crusting, often on the extremities. It is more common in winter and in patients with dry skin.

Causes

This condition happens because of skin barrier disruption and environmental factors.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience intense itching and characteristic coin-shaped lesions.

Emergency warning signs

- Severe or worsening symptoms of nummular dermatitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- KOH preparation distinguishes it from tinea (negative in nummular dermatitis).
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include potent topical corticosteroids, moisturizers, and avoidance of skin irritants. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is chronic with flares. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 593

Obesity

Overview

- Obesity is a chronic, relapsing, multifactorial disease characterized by excessive accumulation of adipose tissue with adverse health consequences. The global prevalence has nearly tripled since 1975
- Over 650 million adults are obese.

Causes

Clinical consequences affect virtually every organ system: type 2 diabetes mellitus, high blood pressure, dyslipidemia, coronary artery disease, stroke, obstructive sleep apnea, non-alcoholic fatty liver disease, cholelithiasis, osteoarthritis, infertility, depression, and increased risk of multiple cancers. Diagnosis includes BMI measurement, waist circumference, and screening for comorbidities. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of obesity require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- It is defined by a body mass index (BMI) of 30 kg/m² or greater in adults, classified into class I (30–34.9), class II (35–39.9), and class III (40 or higher). This condition happens because of a chronic energy imbalance—caloric intake exceeding energy expenditure—driven by complex interactions of genetic, environmental, behavioral, psychosocial, endocrine, and iatrogenic factors
- Adipose tissue is an active endocrine organ secreting adipokines (leptin, adiponectin, resistin, TNF α , IL-6) contributing to a chronic low-grade inflammatory state. Treatment typically involves stepwise: lifestyle intervention (dietary modification, physical activity, behavioral therapy), pharmacotherapy (orlistat, GLP-1 receptor agonists such as semaglutide and liraglutide, naltrexone-bupropion, phentermine-topiramate), and bariatric surgery for class III obesity or class II with comorbidities.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Sustained weight loss improves or resolves many comorbidities.

Chapter 594

Obesity Hypoventilation Syndrome

Overview

Obesity hypoventilation syndrome (OHS, Pickwickian syndrome) is a sleep-related breathing disorder defined by the triad of obesity ($\text{BMI} \geq 30 \text{ kg/m}^2$), awake daytime hypercapnia ($\text{PaCO}_2 > 45 \text{ mmHg}$) without other causes, and sleep-disordered breathing (90% have OSA). Prevalence among obese individuals is 10–20%, with higher rates in severe obesity. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of mechanical loading of the respiratory system (reduced chest wall compliance, diaphragmatic restriction), blunted central respiratory drive (decreased chemosensitivity to hypercapnia and hypoxia), and leptin resistance leading to impaired ventilatory stimulation. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience obesity, hypersomnolence, shortness of breath on exertion, morning headache, cor pulmonale (right heart failure), and lung high blood pressure.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of obesity hypoventilation syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for arterial blood gas showing awake hypercapnia, polysomnography, and exclusion of other causes of hypoventilation (neuromuscular disease, COPD, hypothyroidism).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on positive airway therapy (CPAP or bilevel PAP), weight loss (medical or bariatric surgery), and supplemental oxygen if needed. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is guarded
- Untreated OHS is linked to significantly increased cardiovascular morbidity and mortality.

Chapter 595

Obsessive-Compulsive Disorder (OCD)

Overview

Obsessive-compulsive disorder has a lifetime prevalence of 2–3% worldwide, with equal distribution across sexes, mean age of onset of 19.5 years, and a bimodal distribution (early-onset peaking in childhood/adolescence, late-onset peaking in the third decade). This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of hyperactivity in the cortico-striato-thalamo-cortical (CSTC) loop, specifically the orbitofrontal cortex, anterior cingulate cortex, and caudate nucleus, with glutamatergic hyperactivity and reduced serotonin function. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience obsessions (recurrent, persistent, intrusive, and unwanted thoughts, urges, or images) and/or compulsions (repetitive behaviors or mental acts that the individual feels driven to perform), with common obsession themes including contamination, harm, symmetry and order, and taboo thoughts, and common compulsions including washing, checking, counting, repeating, and ordering.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of obsessive-compulsive disorder (ocd) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include pharmacotherapy (SSRIs are first-line, typically at higher doses than for depression, with clomipramine as an effective alternative) and psychotherapy (exposure and response prevention [ERP] is the gold-standard psychotherapy).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person: 30–40% have a chronic course, 40–50% have moderate improvement, 10–20% have refractory disease. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 596

Obstructive Sleep Apnea

Overview

Obstructive sleep apnea (OSA) is a sleep-related breathing disorder characterized by recurrent episodes of partial or complete pharyngeal collapse during sleep, resulting in apnea (breathing cessation ≥ 10 seconds) or hypopnea (reduced airflow with desaturation or arousal), causing intermittent hypoxia, hypercapnia, intrathoracic pressure swings, and sleep fragmentation. Prevalence is 15–30% of men and 10–20% of women in the general population, with marked increases in obesity. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of anatomic narrowing of the pharyngeal airway (obesity, craniofacial abnormalities, macroglossia, tonsillar enlargement) combined with reduced dilator muscle activity of the genioglossus during sleep and a low arousal threshold.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience loud snoring, witnessed apneas, gasping or choking awakenings, nocturia, morning headache, excessive daytime sleepiness, fatigue, thinking and memory impairment, and irritability.

Emergency warning signs

- Severe or worsening symptoms of obstructive sleep apnea require immediate medical evaluation

Progression and Stages

Doctors diagnose this using attended in-laboratory polysomnography (gold standard) or home sleep apnea testing (HSAT) in uncomplicated adults, with severity classified by the apnea-hypopnea index (AHI): mild (AHI 5–15/hr), moderate (AHI 15–30/hr), and severe (AHI >30/hr). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment focuses on positive airway pressure (PAP) therapy (continuous PAP, auto-CPAP, bilevel PAP) as The first-choice treatment, mandibular advancement devices for mild-to-moderate OSA, hypoglossal nerve stimulation therapy, positional therapy, weight loss and lifestyle modification, and upper airway surgery for patients with surgically correctable anatomy or PAP intolerance.

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with treatment
- Untreated OSA is linked to high blood pressure, atrial fibrillation, heart failure, coronary artery disease, stroke, and type 2 diabetes.

Chapter 597

Obstructive Sleep Apnea

Overview

Obstructive sleep apnea is a disorder characterized by repetitive episodes of partial or complete upper airway obstruction during sleep, resulting in apneas and hypopneas. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of collapse of the pharyngeal airway during sleep.

Risk Factors

- Family history of mental illness
- Trauma or adverse childhood experiences
- Chronic stress
- Substance use
- Social isolation
- Underlying medical conditions
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Medication use

Signs and Symptoms

Common symptoms

- You may experience loud snoring, witnessed apneas, excessive daytime sleepiness, morning headaches, and thinking and memory impairment
- Untreated OSA increases the risk of high blood pressure, atrial fibrillation, stroke, heart failure, and metabolic syndrome.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of obstructive sleep apnea require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using polysomnography measuring the apnea-hypopnea index (AHI).
Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include continuous positive airway pressure (CPAP), weight loss, oral appliances, and surgical options (uvulopalatopharyngoplasty, maxillomandibular advancement, hypoglossal nerve stimulation). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with effective treatment and adherence. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 598

Occipital Neuralgia

Overview

Occipital neuralgia is a neuropathic pain disorder characterized by paroxysmal, stabbing, or shooting pain in the distribution of the greater, lesser, or third occipital nerves (posterior scalp, from occiput to vertex). It may be caused by entrapment of the occipital nerves by cervical muscles or by trauma, cervical arthritis, or C1–C2 pathology. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Neurological dysfunction can result from structural abnormalities, neurodegenerative processes, demyelination, vascular injury, or neurotransmitter imbalances. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Excitotoxicity, oxidative stress, and neuroinflammation contribute to neuronal injury. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered. The underlying mechanisms involve complex interactions between genetic predisposition and environmental factors. Disruption of normal physiological processes leads to the characteristic features of the condition. Multiple contributing factors may act synergistically to trigger or worsen the condition.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pain accompanied by tenderness over the occipital nerve and paresthesias or dysesthesia in the affected distribution. Diagnosis is supported by temporary relief with local anesthetic block of the occipital nerve. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of occipital neuralgia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include nerve blocks with local anesthetic and corticosteroid (therapeutic and diagnostic), physical therapy, and medications (gabapentin, pregabalin, TCAs)
- For refractory cases, pulsed radiofrequency ablation or occipital nerve stimulation may be considered.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 599

Ocular Histoplasmosis Syndrome

Overview

- Ocular histoplasmosis syndrome (OHS) is an eye condition caused by the fungus *Histoplasma capsulatum*, which infects the eye years after a lung infection
- It causes punched-out scars in the retina and choroid and can lead to choroidal neovascularization with central vision loss
- It is most common in the Ohio and Mississippi River valleys of the United States, where the fungus is endemic

Causes

This condition happens because of an earlier, often silent respiratory infection with *Histoplasma capsulatum*. The fungus reaches the eye through the bloodstream, where it establishes dormant lesions in the choroid. Years later, inflammation at these sites can trigger the growth of new, fragile blood vessels (choroidal neovascularization) that leak and scar the macula.

Risk Factors

- Living or having lived in endemic regions (Ohio/Mississippi River valleys)
- Exposure to soil contaminated with bird or bat droppings
- Previous pulmonary histoplasmosis (often asymptomatic)
- Age over 40 years
- Preexisting ocular histoplasmosis scars

Signs and Symptoms

Common symptoms

- Many people have no symptoms and lesions are found on routine examination
- Blurred or distorted central vision when the macula is involved
- Central blind spot (scotoma) or metamorphopsia
- In peripheral lesions, vision may be normal

Emergency warning signs

- Sudden worsening of central vision or distortion requires urgent evaluation, as early treatment of neovascularization preserves vision

Progression and Stages

Histoplasmosis infection is usually acquired in childhood or young adulthood with no symptoms. Ocular lesions (punched-out chorioretinal scars, peripapillary atrophy, and linear streaks) may be present for years before any visual symptoms. The vision-threatening stage occurs when choroidal neovascularization develops, usually in the fourth to sixth decade, causing fluid, hemorrhage, and scarring in the macula.

Complications

- Choroidal neovascularization with macular hemorrhage and scarring
- Permanent central vision loss
- Recurrent neovascularization after treatment
- Scarring and atrophy of the retina
- Reduced quality of life from impaired central vision

Diagnosis

- Dilated fundus examination showing the characteristic punched-out scars
- Optical coherence tomography (OCT) to detect fluid or neovascularization
- Fluorescein angiography to identify leaking new vessels
- Indocyanine green angiography in selected cases
- Histoplasmosis skin or antibody testing confirms prior exposure but is not required for diagnosis

Prevention

- Avoid exposure to disturbed soil in endemic areas, especially with bird or bat droppings
- Use protective measures when working in high-risk environments (dust masks)
- Regular eye examinations for people with known histoplasmosis scars
- Early self-monitoring with an Amsler grid to detect distortion

Treatment Options

- Observation for inactive lesions without neovascularization
- Intravitreal anti-VEGF injections (ranibizumab, aflibercept, bevacizumab) as first-line therapy for active neovascularization

- Photodynamic therapy (PDT) as an adjunct in selected cases
- Laser photocoagulation for well-defined extrafoveal lesions (less commonly used today)
- Systemic or local antifungal therapy is generally not helpful once ocular lesions are established
- Regular monitoring of the fellow eye

Living with It

- Use an Amsler grid at home to monitor for early distortion or scotomas
- Attend regular follow-up for both eyes
- Use low-vision aids if central vision is impaired
- Report any new visual symptoms promptly

Outlook

With modern anti-VEGF therapy, many patients with neovascular OHS retain or regain useful central vision, though repeated injections are often needed. Peripheral lesions have an excellent prognosis. Untreated central neovascularization typically causes significant permanent vision loss.

Chapter 600

Oculocutaneous Albinism

Overview

Oculocutaneous albinism (OCA) is a group of autosomal recessive disorders characterized by reduced or absent melanin production in the skin, hair, and eyes, with a global prevalence of 1 in 17,000.

Causes

This condition happens because of mutations in genes involved in melanin biosynthesis: OCA1 (TYR, tyrosinase deficiency, most severe, absence of pigment), OCA2 (OCA2/P gene, common in Africans, some residual pigment), OCA3 (TYRP1, rufous albinism in Africans), and OCA4 (SLC45A2, common in Japanese).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience hypopigmentation of skin and hair ranging from white (OCA1) to light yellow or brown (OCA2/3/4), translucent irides, involuntary eye movements, photophobia, reduced visual acuity, foveal underdevelopment, optic nerve misrouting, and strabismus. Patients have markedly increased risk of skin cancer (squamous cell carcinoma, basal cell carcinoma, melanoma) due to lack of UV protection.

Emergency warning signs

- Severe or worsening symptoms of oculocutaneous albinism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with genetic confirmation. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on lifelong photoprotection (broad-spectrum sunscreen, protective clothing, UV-protective eyewear), regular skin surveillance for skin cancer, ophthalmologic care for visual impairment (low vision aids, tinted lenses, strabismus surgery), and skin cancer treatment. outlook: normal life expectancy with adequate sun protection
- Cutaneous malignancies are the major cause of morbidity and mortality.

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 601

Onychomycosis (Tinea Unguium)

Overview

Onychomycosis is a fungal infection of the nail unit, most commonly caused by dermatophytes, particularly *Trichophyton rubrum*. It accounts for approximately 50% of all nail disorders and affects 10–20% of the general population. The condition is more common in older adults, diabetics, immunocompromised individuals, and those with peripheral vascular disease.

Classification

Cause Infectious (Fungal)

Organ System Skin

Pathophysiology Dermatophyte invasion of the nail plate via the hyponychium or nail bed

Duration Chronic (months to years)

Onset Gradual

Mode of Transmission Direct contact / environmental

Clinical Course Chronic, progressive without treatment

Severity Mild to Moderate

Extent of Disease Localized

Age Group Adults (increasing with age)

Epidemiology Prevalence 10–20%; higher in diabetics, elderly, athletes

ICD-11 Code 1F28.0

Causes

This condition results from disruption of normal onychomycosis (tinea unguium) function.

Risk Factors

- Advancing age
- Diabetes mellitus
- Peripheral vascular disease

- Immunosuppression
- Occlusive footwear and excessive sweating
- Use of public swimming pools, gyms, and showers
- Previous nail trauma
- Family history of onychomycosis
- Tinea pedis (athlete's foot)

Signs and Symptoms

Early symptoms

- Early manifestations of onychomycosis (*tinea unguium*) may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Thickened, discolored (white, yellow, brown) nail plate
- Subungual debris and hyperkeratosis
- Nail plate separation from the nail bed (onycholysis)
- Nail plate crumbling and destruction
- Foul odor from accumulated debris
- Pain with pressure from thickened nail
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of onychomycosis (*tinea unguium*) may occur

Emergency warning signs

- Severe or worsening symptoms of onychomycosis (*tinea unguium*) require immediate medical evaluation

Progression and Stages

Untreated onychomycosis progresses slowly over years. The infection typically begins at the distal or lateral edge of the nail and spreads proximally. Total nail plate involvement can develop over 1–5 years. Secondary bacterial infection may occur, and the nail may become completely destroyed. Spontaneous resolution is rare.

Complications

- Secondary bacterial cellulitis, particularly in diabetics
- Diabetic foot ulcers and complications
- Permanent nail dystrophy and disfigurement
- Spread to other nails or skin
- Chronic pain and difficulty walking
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical examination with characteristic nail findings
- Potassium hydroxide (KOH) preparation with fungal culture
- Periodic acid-Schiff (PAS) staining of nail clippings (most sensitive)
- Dermatophyte test medium (DTM) culture
- PCR for rapid species identification
- Fungal culture to identify causative organism

Prevention

- Maintain good foot hygiene and keep feet dry
- Wear breathable footwear and moisture-wicking socks
- Avoid walking barefoot in public facilities
- Disinfect nail care instruments
- Treat associated tinea pedis
- Go for regular check-ups so any problems are caught early

Treatment Options

- Topical antifungal lacquers (ciclopirox, amorolfine) for mild distal disease
- Oral antifungal therapy (terbinafine, itraconazole) for moderate to severe disease
- Terbinafine 250 mg daily for 6–12 weeks (or toenail: 12 weeks)
- Laser therapy and photodynamic therapy (emerging options)
- Nail debridement and chemical avulsion
- Surgical nail removal for refractory cases
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed

- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

With appropriate oral antifungal therapy, cure rates of 70–80% for toenail infections are achievable. However, recurrence rates are high (10–50%) even after successful treatment. Fingernail infections respond better than toenail infections. New nail growth takes 6–12 months for fingernails and 12–18 months for toenails to appear normal. Long-term prevention strategies are important to minimize recurrence.

Chapter 602

Opioid Overdose

Overview

Opioid overdose is a leading cause of poisoning death, driven by prescription opioids, heroin, and illicit fentanyl analogs. This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of mu-opioid receptor agonism in the brainstem producing respiratory depression, miosis (pinpoint pupils), and CNS depression leading to coma.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the classic triad of coma, miosis, and respiratory depression.

Emergency warning signs

- Severe or worsening symptoms of opioid overdose require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on airway management and naloxone (0.04–2 mg IV, 0.4–2 mg IM, or 2–4 mg IN) as the antidote
- Naloxone has a short half-life (30–90 minutes) that may require repeat doses or continuous infusion. Patients should be observed for at least 4–6 hours after the last naloxone dose.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with timely intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 603

Opioid Use Disorder

Overview

Opioid use disorder has a lifetime prevalence of approximately 1–2% and has been a major driver of the opioid epidemic in North America.

Causes

This condition happens because of mu-opioid receptor activation in the ventral tegmental area and nucleus accumbens, producing dopamine release and intense euphoria, with chronic use leading to tolerance, severe withdrawal (agitation, myalgias, lacrimation, rhinorrhea, piloerection, yawning, diarrhea, nausea/vomiting, dilated pupils, muscle spasms, intense craving), and hijacking of the reward system, with respiratory depression being the acute lethal risk.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a problematic pattern of opioid use (heroin, prescription opioids) leading to clinically significant impairment, with criteria analogous to AUD.

Emergency warning signs

- Severe or worsening symptoms of opioid use disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Overdose prevention includes naloxone distribution. outlook: MAT is linked to reduced mortality
- Relapse rates after detoxification alone exceed 80% without ongoing medication.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include medication-assisted treatment (MAT) as standard of care: methadone (full mu-agonist), buprenorphine (partial mu-agonist), and naltrexone (mu-antagonist), along with psychosocial interventions (CBT, contingency management).

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 604

Oral Candidiasis

Overview

Oral candidiasis is a fungal infection of the oral mucosa caused by *Candida* species, most commonly *Candida albicans*, with predisposing factors including immunosuppression (HIV/AIDS, chemotherapy), diabetes, denture use, inhaled corticosteroids, xerostomia, and broad-spectrum antibiotics.

Causes

This condition happens because of overgrowth of *Candida* organisms in the oral cavity.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience acute pseudomembranous candidiasis presenting with white, removable plaques on the buccal mucosa, tongue, palate, and oropharynx, revealing an erythematous base when scraped
- Erythematous (atrophic) and chronic hyperplastic forms also exist.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of oral candidiasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with wet mount with potassium hydroxide showing pseudohyphae and blastospores. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on topical antifungals (nystatin, clotrimazole) for mild disease or systemic azoles (fluconazole) for moderate-to-severe or refractory disease. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate antifungal therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 605

Oral Leukoplakia

Overview

- Oral leukoplakia is a white plaque of the oral mucosa that cannot be scraped off and does not correspond to any other diagnosable condition
- It is the most common oral premalignant lesion, with a cancerous transformation rate of approximately 3–5%.

Causes

This condition happens because of chronic mucosal irritation leading to epithelial dysplasia. Homogeneous leukoplakia has a lower cancerous potential than non-homogeneous (speckled) leukoplakia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a white plaque on the oral mucosa.

Emergency warning signs

- Severe or worsening symptoms of oral leukoplakia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for biopsy to assess for epithelial dysplasia. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

The outlook is good with early detection and appropriate management.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 606

Oral Lichen Planus

Overview

Oral lichen planus is a chronic inflammatory condition of the oral mucosa mediated by T-cell autoimmune attack on basal keratinocytes, affecting approximately 1–2% of the population, most commonly middle-aged women.

Causes

This condition happens because of autoimmune T-cell-mediated damage to basal keratinocytes.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the reticular form with characteristic Wickham striae (white, lacy patches) on the buccal mucosa, tongue, and gingiva, usually asymptomatic
- The erosive form causes painful, erythematous ulcerated areas. There is a small (approximately 1%) risk of cancerous transformation.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of oral lichen planus require immediate medical evaluation

Progression and Stages

Doctors diagnose this using biopsy showing a band-like lymphocytic infiltrate at the epithelial-connective tissue interface with basal cell deterioration. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Treatment for symptomatic disease includes topical corticosteroids and calcineurin inhibitors
- Long-term surveillance is recommended.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is good, but the condition is chronic and requires surveillance for cancerous transformation. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 607

Oral Squamous Cell Carcinoma

Overview

Oral squamous cell carcinoma is a cancerous tumor that accounts for over 90% of oral malignancies, with approximately 370,000 new cases annually worldwide, most commonly arising on the lateral tongue and floor of the mouth. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of oral epithelial cells, driven by tobacco use, heavy alcohol consumption, betel nut chewing, and HPV infection. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes

- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of oral squamous cell carcinoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment for early disease is surgical removal with or without neck dissection
- Advanced disease requires multimodal therapy with surgery, radiation, and chemotherapy. outlook is related to stage at diagnosis, with 5-year survival rates of approximately 80–90% for stage I and less than 30% for stage IV.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using incisional biopsy with TNM staging. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and

depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 608

Organic Acidemias

Overview

Organic acidemias are a group of autosomal recessive disorders of amino acid, fatty acid, and carbohydrate catabolism causing accumulation of organic acids in blood and urine. Propionic acidemia (PA) is caused by propionyl-CoA carboxylase deficiency, presenting similarly with severe metabolic acidosis, ketosis, hyperammonemia, pancytopenia, and basal ganglia tissue death due to lack of blood supply. Isovaleric acidemia (IVA) is caused by isovaleryl-CoA dehydrogenase deficiency, producing a characteristic “sweaty feet” odor (isovaleric acid) and presenting with vomiting, metabolic acidosis, lethargy, and pancytopenia.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of organic acidemias require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Methylmalonic acidemia (MMA) is caused by deficiency of methylmalonyl-CoA mutase or cobalamin metabolism, presenting in infancy with recurrent episodes of metabolic acidosis, hyperammonemia, hypoglycemia, lethargy, vomiting, hypotonia, and failure to thrive
- Long-term complications include chronic kidney disease, pancreatitis, and nervous system deficits.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To diagnose all organic acidemias is by urine organic acid analysis, plasma acylcarnitine profile, and genetic testing. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include protein restriction, carnitine supplementation, and specific therapies (metronidazole for MMA, glycine and carnitine for IVA). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 609

Organophosphate and Carbamate Insecticide Poisoning

Overview

- Organophosphate (OP) and carbamate insecticides inhibit acetylcholinesterase (AChE) at cholinergic synapses, causing accumulation of acetylcholine and excessive muscarinic, nicotinic, and CNS stimulation. OPs bind irreversibly (aging occurs within 24–48 hours)
- Carbamates bind reversibly.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: cholinergic crisis—SLUDGE syndrome (Salivation, Lacrimation, Urination, Defecation, GI upset, Emesis) plus miosis, bronchospasm, bronchorrhea, bradycardia, excessive sweating, muscle fasciculations, weakness, paralysis, seizures, and CNS depression.

Emergency warning signs

- Severe or worsening symptoms of organophosphate and carbamate insecticide poisoning require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with RBC AChE activity for confirmation. Management: immediate decontamination, atropine (1–4 mg IV, doubled every 5–10 minutes) to block muscarinic effects, pralidoxime (2-PAM, 1–2 g IV) to reactivate AChE if given before aging, and benzodiazepines for seizures. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with early treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 610

Osteoarthritis

Overview

Osteoarthritis (OA) is the most common joint disease worldwide, characterized by progressive articular cartilage degradation, subchondral bone remodeling, osteophyte formation, and synovial inflammation. It affects over 500 million people globally, with prevalence increasing with age. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age.

Causes

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Environmental triggers (infections, smoking)
- Hormonal factors
- Certain medications
- Advancing age
- Genetic predisposition
- Repetitive use or overuse (joints)
- Previous injuries
- Obesity
- Smoking and alcohol use
- Genetic predisposition and family history
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience joint pain (worse with activity and relieved by rest), stiffness (morning stiffness <30 minutes, unlike RA), crepitus, reduced range of motion, and joint enlargement (osteophytes, bony enlargement). Weight-bearing joints (knees, hips) and the hands (distal interphalangeal joints—Heberden nodes, proximal interphalangeal joints—Bouchard nodes, first carpometacarpal joints) are most commonly affected. Diagnosis is primarily clinical
- X-rays show joint space narrowing, subchondral sclerosis, osteophytes, and subchondral cysts.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of osteoarthritis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Treatment options include patient education, weight loss, exercise, physical therapy, analgesics (acetaminophen, NSAIDs), intra-articular corticosteroid or hyaluronic acid injections, and joint replacement surgery (total knee or hip arthroplasty) for advanced disease. The outlook varies from person to person
- Joint replacement provides excellent outcomes for advanced disease.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 611

Osteogenesis Imperfecta

Overview

Osteogenesis imperfecta (OI, "brittle bone disease") is a group of genetic disorders primarily caused by mutations in the COL1A1 or COL1A2 genes encoding type I collagen, leading to defective bone formation and increased bone fragility.

Causes

This condition happens because of defective collagen production leading to brittle bones.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience recurrent fractures with minimal trauma, blue sclerae, dentinogenesis imperfecta (brittle, discolored teeth), hearing loss (conductive, sensorineural, or mixed), and joint hyperlaxity.

Emergency warning signs

- Severe or worsening symptoms of osteogenesis imperfecta require immediate medical evaluation

Progression and Stages

Severity ranges from perinatal lethal (type II) to mild (type I, most common). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by genetic testing and bone densitometry. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include bisphosphonates (to reduce fracture frequency), physical therapy, bracing, surgical correction of deformities (intramedullary rodding), and dental management. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical

intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Your outlook depends on the type
- Milder forms have a normal life expectancy.

Chapter 612

Osteomyelitis

Overview

Osteomyelitis is infection of bone, which may be acute or chronic. Hematogenous osteomyelitis (most common in children) typically affects the metaphysis of long bones (femur, tibia, humerus), with *Staphylococcus aureus* as the predominant organism. Contiguous-focus osteomyelitis occurs after direct inoculation (trauma, surgery, open fractures) or spread from adjacent soft tissue infection. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of bacterial infection of bone leading to inflammation, tissue death, and new bone formation. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Clinical features in children include localized bone pain, fever, and elevated inflammatory markers
- In adults, it more commonly involves the vertebrae (vertebral osteomyelitis/diskitis). Chronic osteomyelitis is marked by bone tissue death (sequestrum), new bone formation (involucrum), and sinus tracts draining pus.
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of osteomyelitis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)

- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Diagnosis typically involves blood cultures, inflammatory markers (ESR, CRP), and imaging (MRI is the gold standard for detecting bone swelling and abscesses)

- CT for sequestrum
- Bone scan). Management requires prolonged antibiotic therapy (6 weeks or more, IV initially then oral step-down) and surgical removal of dead tissue for chronic or refractory disease.
- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

The outlook is generally positive with early diagnosis and appropriate treatment. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 613

Osteoporosis

Overview

It affects approximately 200 million people worldwide, predominantly postmenopausal women and the elderly. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

Risk Factors

- Family history
- Obesity and sedentary lifestyle
- Poor dietary habits
- Advancing age
- Hormonal imbalances
- Certain medications
- Genetic predisposition
- Repetitive use or overuse (joints)
- Previous injuries
- Obesity
- Smoking and alcohol use
- Genetic predisposition and family history
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fragility fractures commonly at the hip, vertebrae (compression fractures), and distal radius (Colles' fracture).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of osteoporosis require immediate medical evaluation

Progression and Stages

Osteoporosis is a systemic skeletal disease characterized by low bone mineral density (BMD) and deterioration of bone microarchitecture, resulting in increased bone fragility and fracture risk. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on dual-energy X-ray absorptiometry (DXA): T-score ≤ -2.5 at the hip or spine defines osteoporosis
- T-score between -1 and -2.5 is osteopenia. The FRAX tool estimates 10-year fracture risk.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Outlook is improved with early detection and treatment

- Fracture risk is significantly reduced with appropriate therapy.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include calcium (1000–1200 mg/day) and vitamin D (800–1000 IU/day), weight-bearing exercise, bisphosphonates (alendronate, zoledronic acid, first-line), denosumab (RANKL inhibitor), teriparatide and abaloparatide (parathyroid hormone analogs), and romosozumab (anti-sclerostin antibody). Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 614

Osteosarcoma

Overview

Osteosarcoma is the most common primary cancerous bone tumor (excluding myeloma), with a bimodal age distribution (peak in adolescents during the growth spurt and in older adults). It arises from mesenchymal cells producing osteoid and most commonly affects the metaphysis of long bones (distal femur, proximal tibia, proximal humerus). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of osteoblast precursors. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age

- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Advanced age
- Family history and genetic predisposition
- Head trauma or brain injury
- Vascular risk factors (hypertension, diabetes)
- Exposure to environmental toxins
- Chronic stress
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience localized bone pain, swelling, and a palpable mass. X-rays show a destructive lesion with sunburst periosteal reaction and Codman's triangle.
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of osteosarcoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Your doctor confirms the diagnosis with biopsy. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid. Physical examination including palpation of mass and lymph node assessment

Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment focuses on neoadjuvant chemotherapy (doxorubicin, cisplatin, high-dose methotrexate), surgical removal (limb-salvage surgery when possible), and adjuvant chemotherapy. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach

is often beneficial. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Your outlook depends on tumor tissue death after neoadjuvant chemotherapy (>90% tissue death is a favorable prognostic factor). Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage

at diagnosis, molecular characteristics, and response to therapy. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 615

Osteosarcoma in Children

Overview

Osteosarcoma is the most common primary cancerous bone tumor in children and adolescents (see Chapter 26, Diseases of the Musculoskeletal System for comprehensive coverage). Peak incidence is during the adolescent growth spurt (ages 10–16), with a predilection for the metaphyses of long bones, most commonly the distal femur, proximal tibia, and proximal humerus. Most childhood osteosarcomas are high-grade. Treatment follows neoadjuvant chemotherapy, surgical removal, and adjuvant chemotherapy, with survival of 60–70% for localized disease. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption

- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of osteosarcoma in children require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with

significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and

depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 616

Other Parasomnias: Exploding Head Syndrome

Overview

Exploding head syndrome is a parasomnia characterized by episodes of loud imagined noises (explosion, gunshot, crashing cymbals) or a sensation of violent explosion inside the head, occurring at the transition from wakefulness to sleep (hypnagogic) or from sleep to wakefulness. It is relatively common (10–15% of young adults). This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of a transient delay in the normal inhibition of the auditory system during sleep onset. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience frightening but painless auditory sensations lasting seconds.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of other parasomnias: exploding head syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on reassurance
- Clomipramine and calcium channel blockers have been used in severe cases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is non-cancerous.

Chapter 617

Other Parasomnias: Sleep Enuresis

Overview

Sleep enuresis (bedwetting) is involuntary voiding of urine during sleep, occurring at least twice per week for at least three months, in children over age 5. Prevalence decreases with age: 15% at age 5, 5% at age 10, and 1–2% by age 15, with a male predominance. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of a mismatch between nocturnal bladder capacity and overnight urine production, impaired arousal from sleep in response to bladder fullness, and in some cases, nocturnal polyuria due to insufficient antidiuretic hormone secretion. Primary enuresis (never dry for an extended period) is more common than secondary enuresis (relapse after ≥ 6 months of dryness). Diagnosis includes urinalysis to exclude diabetes mellitus, diabetes insipidus, and urinary tract infection.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of other parasomnias: sleep enuresis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include behavioral interventions (voiding before bed, limiting evening fluids, reward systems, bedwetting alarms), desmopressin for nocturnal polyuria, and anticholinergics for bladder overactivity
- Combination therapy may benefit refractory cases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive
- Most children outgrow the condition.

Chapter 618

Other Parasomnias: Sleep-Related Hallucinations

Overview

Sleep-related hallucinations are vivid, dream-like perceptual experiences occurring at sleep onset (hypnagogic) or upon awakening (hypnopompic), involving visual, auditory, tactile, or kinetic sensations. They are common in the general population (25–40% of young adults) and typically non-cancerous. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition results from disruption of normal other parasomnias: sleep-related hallucinations function.

- This condition happens because of intrusion of REM sleep imagery into wakefulness
- Pathological sleep-related hallucinations are hallmark features of narcolepsy type 1 and can occur in Parkinson disease.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience multimodal hallucinations with intact reality testing.

Emergency warning signs

- Severe or worsening symptoms of other parasomnias: sleep-related hallucinations require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for exclusion of narcolepsy and distinction from psychotic disorders. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on reassurance for non-cancerous cases and treatment of underlying narcolepsy or neurodegenerative disease when present. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is non-cancerous in isolation. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 619

Other Primary Headache Disorders

Overview

Primary cough headache is a primary headache disorder brought on by coughing, straining, or Valsalva maneuver, typically bilateral, sudden-onset, lasting 1 second to 30 minutes, with a prevalence of approximately 1%. Secondary causes (Chiari malformation, intracranial high blood pressure) must be excluded by MRI. Indomethacin is The first-choice treatment. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of other primary headache disorders require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive. Primary exertional headache is a primary headache disorder precipitated by any form of exercise, typically bilateral, pulsating, lasting 5 minutes to 48 hours, more common in hot weather, at high altitude, and in patients with a history of migraine. Treatment options include ergonovine, indomethacin, or triptans taken prior to exercise. Primary stabbing headache is a primary headache disorder presenting with brief, sharp, stabbing pains (“ice pick headache”) lasting 1–10 seconds, occurring in single stabs or a series, confined to the first division of the trigeminal nerve. Indomethacin is first-line. Hypnic headache (“alarm clock headache”) is a rare primary headache disorder in patients over 50 years of age, with attacks strictly sleep-related, awakening the patient at a consistent time (typically 1–3 hours after sleep onset), bilateral, dull, lasting 15–180 minutes, without autonomic features. Caffeine, lithium, or indomethacin are used. In most cases, the outlook is favorable. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in

maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 620

Otitis Externa

Overview

Otitis externa is an infection of the external auditory canal that commonly affects swimmers and individuals with predisposing factors such as water exposure, cotton-swab use, hearing aids, and humid climates.

Causes

This condition happens because of inflammation and infection of the canal skin, most commonly caused by *Pseudomonas aeruginosa* and *Staphylococcus aureus*.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience ear pain (otalgia), itching, otorrhea, and a sensation of ear fullness
- Examination reveals redness of the skin, swelling, and tenderness of the canal wall with purulent debris. In severe cases, infection may spread to the surrounding skin (cancerous otitis externa), particularly in diabetics and immunocompromised patients.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of otitis externa require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical examination. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on thorough canal removal of dead tissue and topical antimicrobial drops (fluoroquinolones or aminoglycosides with corticosteroids)
- Oral antibiotics are reserved for extensive or invasive disease.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 621

Otosclerosis

Overview

Otosclerosis is a primary bone remodeling disorder of the otic capsule that causes progressive conductive hearing loss, typically bilateral, beginning in the third to fourth decade of life, with a female predominance and genetic component (autosomal dominant with variable penetrance).

Causes

This condition happens because of abnormal spongy bone formation (active otosclerosis) that replaces normal endochondral bone, most commonly fixing the footplate of the stapes in the oval window.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive conductive hearing loss. Diagnosis is suggested by a positive Schwartz sign (red hue of the promontory on otoscopy) and confirmed by audiometry showing conductive or mixed hearing loss.

Emergency warning signs

- Severe or worsening symptoms of otosclerosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Treatment focuses on stapedectomy or stapedotomy with prosthetic stapes replacement, which is highly effective
- Sodium fluoride may slow disease progression.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with surgical intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 622

Ovarian Cancer

Overview

Risk-reducing bilateral salpingo-oophorectomy (RRSO) is recommended for women at high risk (BRCA1 carriers by age 35–40, BRCA2 carriers by age 40–45). Oral contraceptive use reduces ovarian cancer risk by 50% after five years of use. Screening with CA-125 and transvaginal ultrasound is not recommended for average-risk women but may be considered for high-risk women beginning age 30–35. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)

- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of ovarian cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin

from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team

- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 623

Ovarian Cancer

Overview

This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

The most common histological type is high-grade serous carcinoma (70%), arising from the fallopian tube epithelium. This condition happens because of cancerous transformation of ovarian or fallopian tube epithelium. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle

- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience abdominal bloating, pelvic/abdominal pain, urinary urgency, and early satiety. CA-125 is a serum tumor marker used for monitoring treatment response. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of ovarian cancer require immediate medical evaluation

Progression and Stages

Ovarian cancer is the fifth leading cause of cancer-related death in women, with approximately 313,000 new cases and 207,000 deaths annually worldwide, often diagnosed at an advanced stage due to vague symptoms. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced

tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using imaging, surgical exploration, and histopathology. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment

- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption. Maintain a healthy weight through diet and exercise. Eat a balanced diet rich in fruits, vegetables, and whole grains. Protect skin from excessive sun exposure and avoid tanning beds. Get vaccinated against cancer-causing infections (HPV, hepatitis B). Participate in recommended cancer screening programs. Know your family history and consider genetic testing if indicated. Avoid exposure to known carcinogens in the workplace and environment.

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment typically involves optimal cytoreductive surgery followed by platinum-based chemotherapy (carboplatin + paclitaxel), with maintenance therapy using PARP inhibitors for BRCA-mutated tumors. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection. Radiation therapy: External beam or brachytherapy for localized disease. Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells. Targeted therapy: Drugs targeting specific molecular alterations. Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines. Hormone therapy: For hormone-sensitive cancers (breast, prostate). Combination approaches: Concurrent chemoradiation, sequential therapy. Palliative care: Symptom management, pain control, quality of life support. Clinical trials: Access to experimental therapies. Surveillance: Active monitoring after definitive treatment.

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease

- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

In most cases, the outlook is poor due to late diagnosis. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 624

Paget's Disease of Bone

Overview

- Paget's disease of bone is a chronic disorder of bone remodeling characterized by excessive osteoclastic bone resorption followed by disordered, chaotic new bone formation, resulting in enlarged, structurally abnormal, weakened bones. It is the second most common metabolic bone disease after osteoporosis, affecting 1–3% of adults over 55
- The pelvis, spine, skull, and femur are most commonly involved.

Causes

This condition happens because of abnormal osteoclast activity with increased bone turnover. Most patients are asymptomatic, discovered incidentally by elevated alkaline phosphatase or radiographic findings.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of paget's disease of bone require immediate medical evaluation

Progression and Stages

Doctors diagnose this based on elevated serum alkaline phosphatase, characteristic radiographic changes (osteolytic, mixed, or osteosclerotic phases, cotton-wool appearance of the skull), and bone scan showing increased uptake. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

You may experience bone pain, skeletal deformity, pathologic fractures, and neurological complications (compression of cranial nerves, spinal stenosis, hearing loss from skull base involvement).

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Management with bisphosphonates (zoledronic acid most effective) inhibits osteoclastic activity and normalizes bone turnover. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is good with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 625

Paget's Disease of the Nipple

Overview

Paget's disease of the nipple is a rare presentation of breast cancer (1–3% of cases) involving the nipple-areolar complex. The condition is marked by an eczematous or psoriasiform eruption of the nipple, often with itching, burning, and nipple discharge, with nearly all cases being associated with an underlying breast carcinoma (in situ or invasive). Histologically, Paget cells (large, pale, intraepidermal adenocarcinoma cells) are found within the epidermis.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of paget's disease of the nipple require immediate medical evaluation

Progression and Stages

Your outlook depends on the stage of the underlying carcinoma rather than the Paget's disease itself. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate

stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for biopsy of the affected nipple. Management typically involves mastectomy with sentinel lymph node biopsy, or breast-conserving surgery if the underlying carcinoma can be adequately excised. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 626

Pancreatic Adenocarcinoma

Overview

Pancreatic adenocarcinoma is one of the most lethal malignancies, with a 5-year survival rate of approximately 12%. It is the fourth leading cause of cancer death worldwide. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Approximately 80% of tumors arise in the pancreatic head. This condition happens because of accumulation of genetic mutations (KRAS, CDKN2A, TP53, SMAD4) driving cancerous transformation. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)

- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience painless jaundice (Courvoisier’s sign—palpable, non-tender gallbladder), new-onset diabetes, weight loss, epigastric pain radiating to the back, and depression. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of pancreatic adenocarcinoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional

lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this based on CT abdomen with pancreatic protocol, CA 19-9 (tumor marker), and using a thin camera tube ultrasound-guided fine needle removal by suction for tissue diagnosis. Staging uses the TNM system. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment focuses on surgical removal (Whipple procedure for head tumors, distal pancreatectomy for body/tail tumors) as the only curative option, possible in only 15–20% of patients

- Adjuvant chemotherapy (FOLFIRINOX or gemcitabine/capecitabine) improves survival. Metastatic disease doctors typically treat it with systemic chemotherapy
- Molecular profiling may identify targets for precision therapy (BRCA mutations, MSI-high tumors). outlook remains poor, with a 5-year survival of approximately 12%.
- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)

- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 627

Pancreatic Exocrine Insufficiency

Overview

Pancreatic exocrine insufficiency (PEI) is the failure of the pancreas to produce sufficient digestive enzymes, leading to maldigestion and malabsorption.

Causes

This condition results from disruption of normal pancreatic exocrine insufficiency function.

- This condition happens because of loss of pancreatic acinar cell function
- Lipase deficiency is the most important, leading to steatorrhea.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fatty, foul-smelling, floating stools, weight loss, and fat-soluble vitamin deficiencies (A, D, E, K).

Emergency warning signs

- Severe or worsening symptoms of pancreatic exocrine insufficiency require immediate medical evaluation

Progression and Stages

Common causes include chronic pancreatitis, cystic scarring, pancreatic surgery (Whipple procedure, distal pancreatectomy), and advanced pancreatic cancer. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on fecal elastase-1 level ($<200 \mu\text{g/g}$ indicates moderate insufficiency, $<100 \mu\text{g/g}$ severe) or 72-hour fecal fat collection. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves pancreatic enzyme replacement therapy (PERT) with enteric-coated enzyme capsules taken with meals, with dose titrated based on symptoms and nutritional status. Medications to manage symptoms and address underlying mechanisms

Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with adequate enzyme replacement. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 628

Pancreatic Neuroendocrine Tumors

Overview

Pancreatic neuroendocrine tumors (PanNETs) are tumors arising from the islet cells of the pancreas, accounting for 1–2% of pancreatic tumors. They may be functional (hormone-secreting, 40–60%) or non-functional. Functional PanNETs include insulinoma (most common functional type, presenting with hypoglycemia), gastrinoma (Zollinger-Ellison syndrome with recurrent peptic ulcers), glucagonoma (necrolytic migratory redness of the skin, diabetes, weight loss), VIPoma (watery diarrhea, hypokalemia, achlorhydria), and somatostatinoma (diabetes, steatorrhea, gallstones). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of neoplastic transformation of pancreatic islet cells. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC,

EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Your symptoms depend on the hormone secreted
- Non-functional tumors are often discovered incidentally.
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of pancreatic neuroendocrine tumors require immediate medical evaluation

Progression and Stages

In most cases, the outlook is better than pancreatic adenocarcinoma, depending on tumor grade and stage. Cancer staging follows the TNM system: Tumor (size and extent), Node

(lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Diagnosis typically involves hormone levels for functional tumors and imaging including CT, MRI, and somatostatin receptor scintigraphy (Octreoscan) or Ga-68 DOTATATE PET/CT. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade.

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds

- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

- Treatment typically involves surgical removal for localized disease
- Metastatic disease doctors typically treat it with somatostatin analogs (octreotide, lanreotide), targeted therapy (sunitinib, everolimus), PRRT (peptide receptor radionuclide therapy with Lu-177 DOTATATE), and chemotherapy.
- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 629

Pancreatic Pseudocyst

Overview

Unlike true pancreatic cysts, pseudocysts lack an epithelial lining. Driven by pancreatic ductal disruption, extravasated pancreatic juice digests surrounding tissues until contained by inflammatory adhesions of adjacent organs (stomach, duodenum, transverse mesocolon).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience persistent abdominal pain, early satiety, nausea, vomiting, and a palpable epigastric mass following an episode of pancreatitis.

Emergency warning signs

- Severe or worsening symptoms of pancreatic pseudocyst require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Complications include infection (abscess formation), gastric outlet obstruction, rupture, and pseudoaneurysm erosion into adjacent vessels.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors confirm the diagnosis through CT or MRI showing a well-circumscribed fluid collection with a mature fibrous wall. Treatment for asymptomatic pseudocysts is observation
- Symptomatic or enlarging cysts (> 6 cm) undergo using a thin camera tube cystogastrostomy, through the skin removal of fluid, or surgical cystojejunostomy.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 630

Panic Disorder / Panic Attacks

Overview

Panic disorder has a lifetime prevalence of 3–5%, with a mean age of onset in early 20s to 30s. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of a hypersensitive fear network centered on the amygdala, locus coeruleus (noradrenergic dysregulation), and periaqueductal gray, with faulty interoceptive conditioning and catastrophic misinterpretation of somatic sensations. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience recurrent unexpected panic attacks (discrete periods of intense fear or discomfort that reach a peak within minutes, with at least four of: palpitations, sweating, trembling, shortness of breath, sensations of choking, chest pain, nausea, dizziness, chills or heat sensations, paresthesias, derealization, fear of losing control or dying) plus at least one month of persistent concern about additional attacks.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of panic disorder / panic attacks require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview, ruling out cardiac, lung, and endocrine causes.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include pharmacotherapy (SSRIs and SNRIs are first-line) and psychotherapy (CBT with interoceptive exposure is highly effective). Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with appropriate treatment. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 631

Parkinson's Disease

Overview

Parkinson's disease is a chronic, progressive neurodegenerative disorder affecting the dopaminergic neurons of the substantia nigra pars compacta in the midbrain. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of loss of pigmented dopaminergic neurons and the presence of Lewy bodies composed of misfolded alpha-synuclein. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience cardinal motor manifestations—bradykinesia, resting tremor, rigidity, and postural instability—as well as non-motor symptoms such as loss of smell, constipation, REM sleep behavior disorder, depression, and autonomic dysfunction, which often precede motor manifestations by years. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of parkinson's disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Treatment focuses on dopamine replacement with levodopa/carbidopa, dopamine agonists, MAO-B inhibitors, and COMT inhibitors
- Advanced disease may be managed with deep brain stimulation of the subthalamic nucleus or globus pallidus interna.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by response to dopaminergic therapy and DaT-SPECT imaging. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electro-

physiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is progressive, with disease-modifying therapies remaining under investigation. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation

can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 632

Paroxysmal Hemicrania

Overview

Paroxysmal hemicrania is a trigeminal autonomic cephalalgia characterized by strictly unilateral, severe pain attacks lasting 2–30 minutes, occurring several to many times per day (typically >5/day). Onset is typically in adulthood, with a slight female predominance.

Causes

This condition happens because of activation of the trigeminovascular system with hypothalamic dysfunction.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience unilateral pain with ipsilateral autonomic features identical to cluster headache.

Emergency warning signs

- Severe or worsening symptoms of paroxysmal hemicrania require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- To make the diagnosis, doctors look for a therapeutic trial of indomethacin
- The defining feature is absolute response to indomethacin (75–150 mg/day).
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on indomethacin with digestive tract protection. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with treatment
- Episodic and chronic forms exist.

Chapter 633

Patau Syndrome (Trisomy 13)

Overview

Patau syndrome is a chromosomal disorder caused by trisomy 13, with an incidence of 1 in 10,000 live births. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe intellectual disability, microcephaly, holoprosencephaly, cleft lip and palate, polydactyly, low-set ears, microphthalmia, and congenital heart disease. Median survival is 3–10 days
- Less than 10% survive to age 1 year.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of patau syndrome (trisomy 13) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using karyotype.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care
Enzyme replacement therapy for certain conditions
Gene therapy (emerging for select conditions)
Dietary modifications (e.g., phenylketonuria)
Regular monitoring for associated complications
Physical and occupational therapy
Genetic counseling for family planning
Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment typically involves supportive and palliative.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 634

Patent Ductus Arteriosus (PDA) in Prematurity

Overview

Patent ductus arteriosus (PDA) is the persistent patency of the fetal vascular connection between the descending aorta and the lung artery. In preterm infants, ductal closure is delayed, and a hemodynamically significant PDA occurs in up to 60% of extremely preterm infants. Left-to-right shunting through the PDA leads to lung overcirculation, increased left ventricular preload, and systemic steal.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of patent ductus arteriosus (pda) in prematurity require immediate medical evaluation

Progression and Stages

You may experience a continuous murmur, bounding pulses, wide pulse pressure, respiratory deterioration, and hypotension. The condition may progress through different stages

of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using echocardiography. Treatment options include fluid restriction, diuretics, pharmacologic closure with indomethacin or ibuprofen, and surgical ligation for failed medical therapy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with closure. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 635

Pediatric Appendicitis

Overview

Appendicitis is the most common surgical emergency in children (see Chapter 16, Diseases of the Intestines for comprehensive coverage). In pediatric patients, diagnosis may be more challenging due to atypical presentations, especially in younger children.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience periumbilical pain migrating to the right lower quadrant, fever, anorexia, nausea and vomiting, and rebound tenderness. Ultrasound is the first-line imaging modality.

Emergency warning signs

- Severe or worsening symptoms of pediatric appendicitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves using small incisions and a camera appendectomy with perioperative antibiotics. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed

- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with timely treatment
- Young children have higher rates of perforation.

Chapter 636

Pelvic Inflammatory Disease

Overview

Pelvic inflammatory disease (PID) is a gynecologic infection recognized as the most common preventable cause of infertility and ectopic pregnancy worldwide. It is primarily sexually transmitted, with *Chlamydia trachomatis* and *Neisseria gonorrhoeae* as the predominant pathogens. The CDC-recommended treatment regimen includes ceftriaxone 500 mg IM plus doxycycline 100 mg BID for 14 days, with metronidazole for anaerobic coverage. All sexual partners should be treated.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of pelvic inflammatory disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 637

Pelvic Inflammatory Disease

Overview

Pelvic inflammatory disease is an infection of the female upper genital tract, including the endometrium (endometritis), fallopian tubes (salpingitis), ovaries (oophoritis), and pelvic peritoneum.

Causes

This condition happens because of ascending infection from the lower genital tract.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience lower abdominal pain (often bilateral), cervical motion tenderness (“chandelier sign”), uterine tenderness, adnexal tenderness, and abnormal cervical/vaginal discharge.

Emergency warning signs

- Severe or worsening symptoms of pelvic inflammatory disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

It is the most common and most serious complication of sexually transmitted infections, with *Chlamydia trachomatis* and *Neisseria gonorrhoeae* being the most common causative organisms. The outlook is good with prompt treatment, though complications include tubo-ovarian abscess, infertility, and ectopic pregnancy.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam (CDC criteria). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include empiric antibiotics covering *N. gonorrhoeae* and *C. trachomatis*: ceftriaxone plus doxycycline plus metronidazole. Medications to manage symptoms and

address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 638

Pemphigus Vulgaris

Overview

Pemphigus vulgaris (PV) is a potentially life-threatening autoimmune blistering disease characterized by IgG autoantibodies against desmoglein 3 (and sometimes desmoglein 1), components of desmosomes that maintain keratinocyte adhesion.

Causes

This condition happens because of autoantibody-mediated loss of keratinocyte adhesion leading to intraepidermal blisters (suprabasal acantholysis).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience mucosal involvement (oral erosions) in 50–70% of cases, often preceding cutaneous lesions
- Skin blisters are flaccid, easily ruptured (positive Nikolsky sign), and heal without scarring.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of pemphigus vulgaris require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- To make the diagnosis, doctors look for skin biopsy showing suprabasal acantholysis, direct immunofluorescence showing intercellular IgG and C3 deposition ("chicken-wire" pattern), and detection of anti-desmoglein antibodies by ELISA. Management requires immunosuppression: systemic corticosteroids combined with steroid-sparing agents (azathioprine, mycophenolate, rituximab)
- Rituximab (anti-CD20) has become a The first-choice treatment, often achieving drug-free remission.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has improved significantly with modern immunosuppressive therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 639

Penile Cancer

Overview

Penile cancer is rare in developed countries (0.5–1 per 100,000) but accounts for up to 10% of male malignancies in parts of Africa, South America, and Asia where circumcision is less common and HPV prevalence is higher, with squamous cell carcinoma representing >95% of cases. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of squamous epithelium. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)

- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience a painless ulcer, mass, or wart-like lesion on the glans, prepuce, or shaft. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of penile cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs

Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using biopsy, with staging using the TNM system. Treatment for the primary tumor includes topical therapy for PeIN, laser ablation or wide local removal for superficial tumors, and partial or total penectomy for invasive tumors. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment

- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy

- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

- Most people recover well for organ-confined disease
- 5-year survival drops to 50% with nodal involvement.

Chapter 640

Penile Fracture

Overview

Penile fracture is a traumatic rupture of the tunica albuginea of the corpus cavernosum, occurring during erectile activity when the penis is forcibly bent, most commonly during vigorous sexual intercourse or masturbation. This condition results from acute injury or exposure to harmful agents.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a sudden snapping sound (audible crack), immediate detumescence, severe pain, and rapid development of swelling and bruising (the “eggplant deformity”), with urethral injury occurring in 10–20% of cases. This condition happens because of traumatic rupture of the tunica albuginea.

Emergency warning signs

- Severe or worsening symptoms of penile fracture require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves immediate surgical repair: degloving of the penis, evacuation of hematoma, identification and primary closure of the tunical defect. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with early surgical repair (erectile function preserved in >90%), while conservative management leads to high rates of erectile dysfunction. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 641

Peptic Ulcer Bleeding

Overview

Causes

This condition happens because of erosion of the ulcer into underlying blood vessels.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms can range from hematemesis (vomiting of blood, either bright red or "coffee-ground") to melena (black, tarry stools) or occult GI bleeding with iron deficiency anemia.

Emergency warning signs

- Severe or worsening symptoms of peptic ulcer bleeding require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Peptic ulcer bleeding is the most common complication of peptic ulcer disease and the most frequent cause of upper digestive tract bleeding, accounting for approximately 50% of all upper GI bleeds.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on upper endoscopy, which identifies stigmata of recent bleeding including active bleeding, visible vessel, adherent clot, and flat pigmented spot. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on hemodynamic resuscitation (IV fluids, blood transfusion), intravenous PPI infusion (to raise gastric pH and stabilize clots), and urgent endoscopy (within 24 hours) with using a thin camera tube therapy (injection, thermal coagulation, or hemoclips) for high-risk stigmata. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is favorable with intervention
- Re-bleeding occurs in 5–10% of cases and may require repeat endoscopy or angiographic embolization.

Chapter 642

Peptic Ulcer Disease

Overview

Peptic ulcer disease (PUD) involves mucosal defects extending through the muscularis mucosae in the stomach or duodenum.

Causes

This condition happens because of mucosal barrier disruption caused primarily by *Helicobacter pylori* infection (accounting for 70–90% of duodenal and 60–70% of gastric ulcers) and chronic nonsteroidal anti-inflammatory drug (NSAID) use.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience epigastric pain: duodenal ulcer pain typically improves with food ingestion and recurs 2–3 hours later, whereas gastric ulcer pain is often precipitated by meals.

Emergency warning signs

- Severe or worsening symptoms of peptic ulcer disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Complications include digestive tract bleeding, ulcer perforation (presenting with sudden severe pain and pneumoperitoneum), and gastric outlet obstruction.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using upper endoscopy with mucosal biopsy for *H. pylori* testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include proton pump inhibitors (PPIs) and *H. pylori* eradication therapy (quadruple therapy with PPI, bismuth, metronidazole, tetracycline, or triple therapy). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with eradication. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 643

Pericarditis

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Pericarditis is an inflammation of the pericardium, the fibrous sac surrounding the heart
- Acute pericarditis is the most common pericardial disease, with viral cause being most frequent (Coxsackievirus, echovirus, adenovirus, SARS-CoV-2). Other causes include bacterial infection, uremia, autoimmune diseases, malignancy, and post-MI (Dressler syndrome).

Causes

This condition happens because of inflammation of the pericardial layers. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease

- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience sharp, pleuritic chest pain that worsens with inspiration and improves with sitting forward, pericardial friction rub on auscultation, and diffuse ST-segment elevation on ECG. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of pericarditis require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia

- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this based on clinical features, ECG, and echocardiography (may show pericardial effusion). Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment options include NSAIDs (ibuprofen, aspirin) and colchicine for 3 months
- Corticosteroids are used for refractory or recurrent cases.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk

- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

In most cases, the outlook is good, with most cases resolving with treatment. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 644

Perinatal Asphyxia / Hypoxic-Ischemic Encephalopathy (HIE)

Overview

Hypoxic-ischemic encephalopathy (HIE) is brain injury resulting from perinatal asphyxia—impaired cerebral blood flow and oxygen delivery around the time of birth. Incidence is 1–3 per 1000 live births in developed countries.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience low Apgar scores, metabolic acidosis, seizures, hypotonia, abnormal neurological examination, and multi-organ dysfunction.

Emergency warning signs

- Severe or worsening symptoms of perinatal asphyxia / hypoxic-ischemic encephalopathy (hie) require immediate medical evaluation

Progression and Stages

This condition happens because of primary energy failure (decreased ATP, cellular tissue death) followed by a secondary phase of inflammation, excitotoxicity, and apoptosis hours to days later. HIE is graded as mild (Sarnat Stage 1), moderate (Stage 2), or severe (Stage 3). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on perinatal history, blood gas analysis, and amplitude-integrated EEG
- MRI within the first week shows characteristic patterns of injury.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves therapeutic hypothermia (33.5°C for 72 hours), initiated within 6 hours of birth, which reduces mortality and neurodevelopmental disability. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 645

Periodic Fever Syndromes

Overview

Periodic fever syndromes (also called autoinflammatory diseases) are a group of inherited disorders of the innate immune system characterized by recurrent, self-limited episodes of fever and systemic inflammation in the absence of autoantibodies or antigen-specific T cells. They are distinct from autoimmune diseases and arise from dysregulation of the inflammasome and IL-1 β production (the “IL-1opathies”).

Familial Mediterranean Fever

Familial Mediterranean fever is the most common hereditary periodic fever syndrome, caused by mutations in the *MEFV* gene encoding pyrin, a component of the pyrin inflammasome, most prevalent in Mediterranean populations (Armenian, Turkish, Arab, Jewish—carrier frequency 1:5 in Armenians). This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual’s DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions. This condition affects a significant number of people worldwide and can range from mild to severe in presentation. Early recognition and appropriate management are essential for optimal outcomes. A thorough understanding of the underlying mechanisms guides treatment decisions. Patient education and self-management play important roles in long-term care.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features begin in childhood (90% before age 20) and consist of recurrent, self-limited attacks (12–72 hours) of high fever, serositis (peritonitis—severe abdominal pain resembling acute abdomen, pleuritis, pericarditis), arthritis (usually monoarticular of the lower extremity), and an erysipelas-like redness of the skin on the lower leg, with the diagnostic hallmark being a dramatic response to colchicine.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of periodic fever syndromes require immediate medical evaluation

Progression and Stages

The outlook varies from person to person, with amyloidosis risk reduced with effective anti-IL-1 therapy.

Cryopyrin-Associated Periodic Syndromes

Cryopyrin-associated periodic syndromes are a spectrum of autoinflammatory diseases caused by gain-of-function mutations in the *NLRP3* gene encoding cryopyrin (NALP3), a key component of the NLRP3 inflammasome, encompassing three overlapping phenotypes in order of increasing severity: familial cold autoinflammatory syndrome (FCAS—mildest), Muckle-Wells syndrome (MWS—intermediate), and neonatal-onset multisystem inflammatory disease/chronic infantile nervous system cutaneous articular syndrome (NOMID/CINCA—most severe), with an incidence of approximately 1 per 1,000,000. Treatment typically involves with IL-1 inhibitors: anakinra, canakinumab (long-acting, FDA-approved for CAPS), and rilonacept (IL-1 trap), which are dramatically effective, normalizing symptoms and inflammatory markers within hours and preventing AA amyloidosis and hearing loss progression.

Hyper-IgD Syndrome

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Hyper-IgD syndrome (also known as mevalonate kinase deficiency, MKD) is an autosomal recessive periodic fever syndrome caused by mutations in the *MVK* gene encoding mevalonate kinase, an enzyme in the cholesterol biosynthesis pathway, leading to accumulation of mevalonic acid and activation of the inflammasome, with approximately 300 reported cases, predominantly in Northern European populations

(especially Dutch). The outlook is good

- Attacks decrease in frequency and severity with age, and life expectancy is normal.

Complications

The most serious long-term complication is amyloidosis (AA type) if untreated.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam (Tel Hashomer criteria) or by genetic confirmation of biallelic *MEFV* mutations.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves daily colchicine (0.5–2 mg/day), which prevents attacks and amyloidosis in 95% of patients, with IL-1 inhibitors (anakinra, canakinumab) used for colchicine-resistant FMF. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with colchicine adherence
- AA amyloidosis risk is nearly eliminated.

Chapter 646

Periodic Limb Movement Disorder

Overview

Periodic limb movement disorder (PLMD) is a sleep-related movement disorder characterized by recurrent, stereotypical, involuntary movements of the lower extremities during sleep, most commonly involving dorsiflexion of the great toe, fanning of the toes, and flexion of the ankle, knee, or hip, occurring at intervals of 5–90 seconds. Prevalence increases with age, affecting 10–30% of adults over 60, but is much lower as a primary diagnosis.

Causes

This condition happens because of unknown Disease mechanism, likely related to dopaminergic dysfunction.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sleep fragmentation from movements associated with microarousals, leading to daytime impairment.

Emergency warning signs

- Severe or worsening symptoms of periodic limb movement disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for polysomnography demonstrating a periodic limb movement index (PLMI) >15 per hour in adults (>5 in children) with clinically significant sleep disturbance, in the absence of RLS or other explanatory disorders. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves indicated only when PLMS are associated with sleep disturbance and includes alpha-2-delta ligands (gabapentin, pregabalin) preferred over dopamine agonists due to augmentation risk. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and

rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is chronic but manageable. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 647

Periodic Paralysis

Overview

Periodic paralysis is a rare autosomal dominant channelopathy characterized by episodic attacks of muscle weakness or paralysis. Hypokalemic periodic paralysis presents with attacks of flaccid paralysis with low serum potassium, triggered by carbohydrate-rich meals, rest after exercise, or stress. Hyperkalemic periodic paralysis features attacks with elevated potassium, triggered by fasting or exercise. Andersen-Tawil syndrome combines periodic paralysis with irregular heartbeats and skeletal developmental abnormalities.

Causes

This condition happens because of mutations in ion channel genes (CACNA1S, SCN4A, KCNJ2).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience attacks lasting hours to days
- Between attacks, patients may be asymptomatic or have mild fixed weakness.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of periodic paralysis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on genetic testing, potassium levels during attacks, and provocation testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment options include potassium supplementation during attacks and acetazolamide or dichlorphenamide prophylaxis for hypokalemic type
- Inhaled beta-agonists, glucose, and avoidance of fasting for hyperkalemic type.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Attacks are typically reversible with appropriate management.

Chapter 648

Peripheral Arterial Disease

Overview

- Peripheral arterial disease is atherosclerotic narrowing of the arteries supplying the extremities, predominantly the lower limbs, resulting in reduced blood flow
- It affects approximately 10–15% of adults over 50 and is a marker of systemic atherosclerosis.

Causes

This condition happens because of atherosclerotic plaque accumulation in peripheral arteries. Most patients are asymptomatic

- Intermittent claudication (cramping leg pain triggered by exertion and relieved by rest within 10 minutes) is the hallmark symptom
- Severe disease presents with critical limb reduced blood flow (rest pain, non-healing ulcers, gangrene).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of peripheral arterial disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using the ankle-brachial index: a ratio of 0.90 or less is diagnostic. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include smoking cessation (most important intervention), antiplatelet therapy, statin therapy, supervised exercise programs, and Treatment for comorbidities
- Revascularization is indicated for lifestyle-limiting claudication or critical limb reduced blood flow.
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 649

Peripheral Artery Disease

Overview

Peripheral artery disease (PAD) is a chronic atherosclerotic occlusion of systemic arteries supplying the lower extremities. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of endothelial dysfunction and atheroma accumulation reducing tissue perfusion. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience intermittent claudication (reproducible lower extremity muscle pain precipitated by exertion and relieved by rest), cold extremities, hair loss, thickened toenails, and resting pain or ischemic ulceration/gangrene (critical limb reduced blood flow). Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of peripheral artery disease require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Your doctor confirms the diagnosis with an Ankle-Brachial Index (ABI) < 0.90 , with duplex ultrasonography or CT angiography defining anatomical stenosis. History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

Treatment options include lifestyle modification (smoking cessation, structured exercise program), antiplatelet therapy (aspirin or clopidogrel), statin therapy, cilostazol for claudication, and endovascular angioplasty/stenting or surgical bypass for severe or refractory disease.

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)

- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

outlook is strongly correlated with coexisting coronary and cerebrovascular disease. With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 650

Periventricular Leukomalacia (PVL)

Overview

Periventricular leukomalacia (PVL) is ischemic tissue death of the periventricular white matter in preterm infants, resulting from the vulnerability of oligodendrocyte precursor cells to hypoxic-ischemic injury. It occurs in 3–10% of preterm infants and is a major cause of cerebral palsy, particularly spastic diplegia.

Causes

This condition happens because of impaired perfusion in the watershed zone between the anterior, middle, and posterior cerebral arteries.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience neurodevelopmental impairment, with spastic diplegia as a major consequence.

Emergency warning signs

- Severe or worsening symptoms of periventricular leukomalacia (pvl) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using cranial ultrasound (cystic echolucencies) or MRI. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment focuses on prevention (avoiding hypotension, hypoxia, and hypocarbia).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is guarded
- Long-term outcomes include spastic diplegia, visual impairment, and thinking and memory deficits.

Chapter 651

Persistent Depressive Disorder (Dysthymia)

Overview

Persistent depressive disorder (dysthymia) is a chronic form of depression with symptoms present most of the day, for more days than not, for at least 2 years (1 year in children and adolescents), with a lifetime prevalence of 3–6%. The condition is marked by depressed mood plus at least two of the following: poor appetite or overeating, insomnia or hypersomnia, low energy or fatigue, low self-esteem, poor concentration, or feelings of hopelessness, with symptoms being less severe than MDD but more persistent.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience being “depressed for as long as I can remember,” with double depression referring to MDD superimposed on pre-existing dysthymia.

Emergency warning signs

- Severe or worsening symptoms of persistent depressive disorder (dysthymia) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include SSRIs/SNRIs (same as MDD) and psychotherapy (CBT, IPT). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is guarded, with high rates of recurrence, and Treatment typically involves often recommended for at least 2 years. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 652

Persistent Pulmonary Hypertension of the Newborn (PPHN)

Overview

Persistent lung high blood pressure of the newborn (PPHN) is the failure of the lung vascular resistance to decrease normally after birth, leading to right-to-left shunting across the foramen ovale and ductus arteriosus and profound hypoxemia. Incidence is approximately 2 per 1000 live births. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of failure of the normal postnatal drop in lung vascular resistance. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol

- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience severe hypoxemia out of proportion to the degree of lung disease, with a preductal-to-postductal oxygen saturation gradient. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of persistent pulmonary hypertension of the newborn (pphn) require immediate medical evaluation

Progression and Stages

Your outlook depends on the underlying cause and severity. Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction

- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using echocardiography demonstrating elevated lung artery pressure and right-to-left shunt. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Treatment options include optimization of lung inflation, sedation, inotropic support, inhaled nitric oxide, and ECMO for refractory cases. Management includes lifestyle modifications, pharmacotherapy targeting the underlying mechanisms, and interventional procedures when indicated. Long-term adherence to treatment is essential for preventing disease progression. Lifestyle modifications: heart-healthy diet, regular exercise, smoking

cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 653

Persistent Vegetative State

Overview

A persistent vegetative state is a condition of wakeful unconsciousness in which the patient has sleep-wake cycles and may open their eyes spontaneously but shows no evidence of awareness of self or environment.

Causes

This condition results from disruption of normal persistent vegetative state function.

- This condition happens because of severe diffuse brain injury, cardiac arrest, or massive stroke. When it persists beyond one month, it is termed persistent
- Beyond three months for non-traumatic and twelve months for traumatic etiologies, it is termed permanent.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience spontaneous eye opening with sleep-wake cycles but no purposeful responses. Diagnosis is challenging and may be aided by functional neuroimaging (fMRI) and EEG.

Emergency warning signs

- Severe or worsening symptoms of persistent vegetative state require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves supportive
- Ethical and legal considerations surrounding withdrawal of life-sustaining treatment are complex. outlook for recovery of consciousness is poor once permanent criteria are met.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 654

Pertussis (Whooping Cough)

Overview

Pertussis is a highly contagious respiratory infection caused by *Bordetella pertussis*, a gram-negative coccobacillus, with 24 million cases and 160,000 deaths annually worldwide, with infants < 6 months having the highest morbidity and mortality, and transmission by respiratory droplets.

Causes

This condition happens because of *B. pertussis* adhering to ciliated respiratory epithelial cells and producing pertussis toxin, which ADP-ribosylates G proteins and promotes lymphocytosis, with tracheal cytotoxin damaging ciliated epithelial cells.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of pertussis (whooping cough) require immediate medical evaluation

Progression and Stages

Treatment options include azithromycin (which does not alter the clinical course if begun after the paroxysmal stage but eradicates the organism). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using PCR of nasopharyngeal swab (test of choice). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Most people recover well with supportive care, though pneumonia is the most common cause of death in infants.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 655

Peyronie's Disease

Overview

Peyronie's disease is an acquired penile curvature caused by the formation of fibrous plaques in the tunica albuginea of the corpora cavernosa, affecting 3–9% of men.

Causes

This condition happens because of repetitive microtrauma during sexual activity in genetically susceptible individuals, leading to aberrant wound healing and excessive collagen deposition.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of peyronie's disease require immediate medical evaluation

Progression and Stages

Treatment for the acute phase includes conservative management with observation, oral therapy, and intralesional collagenase, while chronic stable disease may require traction

therapy, vacuum devices, intralesional verapamil or interferon, or surgical correction. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using history and physical examination
- Ultrasound or MRI can quantify plaque size and calcium buildup.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 656

Phantom Limb Pain

Overview

Phantom limb pain is pain perceived in a body part that has been amputated. It occurs in 60–80% of amputees, typically within the first week after amputation, and persists in 50–80% at 2 years.

Causes

This condition happens because of peripheral neuroma formation with ectopic discharges, spinal cord reorganization, and cortical reorganization (invasion of the deafferented somatosensory cortex by adjacent cortical inputs).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pain perceived in the absent limb, distinct from residual limb pain and non-painful phantom limb sensations.

Emergency warning signs

- Severe or worsening symptoms of phantom limb pain require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves challenging

- First-line treatments include gabapentin, pregabalin, TCAs, and tramadol
- Non-pharmacological approaches include mirror therapy, graded motor imagery, and virtual reality therapy.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Treatment typically involves often challenging.

Chapter 657

Pharyngitis

Overview

- Pharyngitis is an inflammation of the pharynx, most commonly caused by viral infections (rhinoviruses, coronaviruses, adenovirus, influenza, Epstein-Barr virus)
- Group A *Streptococcus pyogenes* (GAS) is the most important bacterial cause, accounting for 15–30% of pharyngitis cases in children and 5–15% in adults.

Causes

This condition happens because of infectious inflammation of the pharyngeal mucosa.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of GAS pharyngitis include acute onset of sore throat, fever, tonsillar exudates, tender anterior cervical lymphadenopathy, and palatal tiny red or purple spots, typically without cough.

Emergency warning signs

- Severe or worsening symptoms of pharyngitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Most people recover well with appropriate treatment
- Complications include peritonsillar abscess and poststreptococcal glomerulonephritis.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on the Centor/McIsaac score, rapid antigen detection testing, and throat culture. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment focuses on penicillin or amoxicillin for GAS pharyngitis to prevent acute rheumatic fever.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 658

Phenylketonuria

Overview

Phenylketonuria (PKU) is an autosomal recessive disorder of phenylalanine metabolism caused by deficiency of phenylalanine hydroxylase (PAH), the enzyme that converts phenylalanine to tyrosine. The incidence is approximately 1 in 10,000–15,000 live births.

Causes

This condition happens because of accumulation of phenylalanine and its metabolites in blood and tissues, causing impaired brain development and function.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive developmental delay, intellectual disability, microcephaly, seizures, behavioral problems, a characteristic musty odor of urine and skin, and hypopigmentation (fair skin, light hair, blue eyes) due to impaired melanin synthesis. Newborns are asymptomatic at birth.

Emergency warning signs

- Severe or worsening symptoms of phenylketonuria require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using newborn screening (elevated phenylalanine greater than 120 $\mu\text{mol/L}$) confirmed by elevated blood phenylalanine and normal or low tyrosine. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on lifelong dietary restriction of phenylalanine (elimination of high-protein foods and use of phenylalanine-free medical formulas)

- Sapropterin (BH4 supplementation) is effective in a subset of patients
- Pegvaliase is an enzyme substitution therapy for adults with poorly controlled PKU.
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with early and consistent dietary treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 659

Pheochromocytoma

Overview

Pheochromocytoma is a catecholamine-secreting tumor arising from chromaffin cells of the adrenal medulla, accounting for 0.1–0.6% of hypertensive patients, with paragangliomas being extra-adrenal catecholamine-secreting tumors. Approximately 10% are bilateral, 10% are extra-adrenal, and 10% are cancerous, with up to 40% being hereditary (MEN2, von Hippel-Lindau, neurofibromatosis type 1, SDH mutations).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the classic triad of episodic headache, sweating, and palpitations with paroxysmal or sustained high blood pressure.

Emergency warning signs

- Severe or worsening symptoms of pheochromocytoma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using plasma free metanephrines or 24-hour urinary metanephrines and catecholamines (high sensitivity), with imaging including CT or MRI of the abdomen and MIBG scan. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves alpha-adrenergic blockade (phenoxybenzamine or doxazosin) for at least 10–14 days preoperatively, followed by beta-blockade (only after adequate alpha-blockade), volume expansion, and using small incisions and a camera adrenalectomy. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well after removal for non-cancerous disease. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 660

Phimosis and Paraphimosis

Overview

Phimosis is the inability to retract the foreskin over the glans penis, a normal finding in uncircumcised boys before age 5 (physiologic phimosis), while pathologic phimosis results from recurrent balanoposthitis, poor hygiene, or lichen sclerosus et atrophicus, causing scarring and a contracted, fibrous preputial ring. Paraphimosis is a urologic emergency in which the retracted foreskin cannot be returned to its normal position, constricting the glans and leading to vascular compromise, swelling, and pain, typically occurring after catheterization, sexual activity, or inadequate reduction following examination.

Causes

This condition happens because of preputial abnormalities.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of phimosis include difficulty with urination, recurrent infections, and painful erections
- Paraphimosis presents with a constricted glans and pain.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of phimosis and paraphimosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam. Treatment for paraphimosis involves gentle manual compression of the edematous glans followed by reduction
- Elective circumcision is the The main treatment for pathologic phimosis.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 661

Phyllodes Tumor

Overview

Phyllodes tumors are rare fibroepithelial tumors that can be non-cancerous, borderline, or cancerous (10–15% of all phyllodes tumors), typically presenting as large, rapidly growing, painless breast masses, more common in women 40–50 years of age. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of stromal growth in fibroepithelial tissue. Histologically, they are distinguished from fibroadenomas by increased stromal cellularity, stromal overgrowth, atypia, and mitotic activity. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)

- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience a rapidly enlarging breast mass. Symptoms vary depending on the severity and extent of the condition Pain or discomfort in the affected area
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of phyllodes tumor require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

- Metastatic spread to vital organs (brain, liver, lungs, bones)

- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using core needle biopsy and definitive surgical removal with wide margins (>1 cm) is required for all phyllodes tumors, regardless of grade. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

- The outlook is good with complete removal
- Cancerous phyllodes tumors can metastasize hematogenously.

Chapter 662

Pilonidal Sinus

Overview

Pilonidal sinus (pilonidal disease) is a chronic inflammatory condition affecting the skin and subcutaneous tissue of the intergluteal cleft (natal cleft), characterized by hair-containing sinus tracts.

Causes

This condition happens because of mechanical occlusion of hair follicles in the sacrococcygeal region, leading to follicle rupture, foreign-body granulomatous reaction to entrapped shed hairs, and secondary bacterial infection (*Staphylococcus aureus*, anaerobes).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms can range from an asymptomatic small pit in the natal cleft to an acute pilonidal abscess (severe localized pain, redness of the skin, fluctuating mass, fever) or chronic purulent sinus removal of fluid.

Emergency warning signs

- Severe or worsening symptoms of pilonidal sinus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam. Treatment for acute abscess requires prompt incision and removal of fluid
- Chronic sinus disease doctors typically treat it with surgical removal or unroofing (lay-open) with primary closure or secondary healing, accompanied by local hair removal.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 663

Pituitary Adenoma

Overview

This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition results from disruption of normal pituitary adenoma function.

- This condition happens because of clonal growth of pituitary cells. Functioning adenomas secrete excess hormones: prolactinomas (galactorrhea, amenorrhea, hypogonadism), GH-secreting adenomas (acromegaly or gigantism), ACTH-secreting adenomas (Cushing's disease), and TSH-secreting adenomas (central hyperthyroidism)
- Non-functioning adenomas present with mass effects including bitemporal hemianopia from optic chiasm compression, headaches, and hypopituitarism.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Your symptoms depend on hormone secretion and tumor size.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of pituitary adenoma require immediate medical evaluation

Progression and Stages

Pituitary adenomas are non-cancerous tumors of the anterior pituitary gland, classified as microadenomas (less than 10 mm) or macroadenomas (10 mm or greater). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Diagnosis typically involves hormonal evaluation and MRI of the sella. Management varies: prolactinomas respond to dopamine agonists (cabergoline)
- Other functioning and non-functioning adenomas typically require transsphenoidal surgery, with radiation for residual or recurrent disease.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy.

Chapter 664

Placenta Accreta Spectrum

Overview

Incidence has risen dramatically with increasing cesarean rates, now 1 in 500–1000 pregnancies. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience difficulty with placental separation at delivery. Prenatal diagnosis by ultrasound and MRI allows planned management.

Emergency warning signs

- Severe or worsening symptoms of placenta accreta spectrum require immediate medical evaluation

Progression and Stages

Placenta accreta spectrum (PAS) is abnormal placental adherence to the myometrium due to a deficient decidual layer, classified by depth of invasion: accreta, increta, and percreta. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve

significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves controversial but increasingly favors conservative approaches when possible: planned cesarean hysterectomy with the placenta left in situ, or delayed hysterectomy after uterine artery embolization. Massive transfusion protocols and multidisciplinary teams are essential. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on depth of invasion and preparedness. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 665

Placenta Previa

Overview

Placenta previa is the implantation of the placenta over or near the internal cervical os, occurring in approximately 1 in 200 pregnancies at term. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

This condition happens because of abnormal trophoblast implantation in the lower uterine segment.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience painless vaginal bleeding in the second or third trimester, typically after 28 weeks gestation.

Emergency warning signs

- Severe or worsening symptoms of placenta previa require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Doctors diagnose this using transvaginal ultrasound
- Placenta previa is classified as complete, partial, marginal, or low-lying.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Treatment for asymptomatic women is conservative: pelvic rest, avoidance of intercourse and vaginal exams, and elective cesarean delivery at 36–37 weeks. Massive bleeding may require emergency cesarean delivery, transfusion, and potentially hysterectomy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with planned cesarean delivery. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 666

Placental Abruption

Overview

Placental abruption (abruptio placentae) is the premature separation of the normally implanted placenta from the uterine wall before delivery, occurring in approximately 1% of pregnancies. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

This condition happens because of rupture of decidual spiral arteries, causing retroplacental hematoma formation and further separation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience vaginal bleeding (80%, though 20% have concealed abruption), sudden sustained abdominal pain, uterine hypertonicity (“wooden” uterus), fetal distress, and signs of hypovolemic shock that may appear out of proportion to visible blood loss. Diagnosis is primarily clinical
- Ultrasound has limited sensitivity (50%).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of placental abruption require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- The right treatment depends on severity and gestational age: mild abruption at term is managed by delivery
- Severe abruption requires immediate cesarean delivery with massive transfusion protocol.

Complications

- Your outlook depends on severity
- Complications include DIC, involving bleeding shock, acute kidney injury, and fetal death (10–20%).
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 667

Plague (*Yersinia pestis*)

Overview

Plague is a zoonotic infection caused by *Yersinia pestis*, a gram-negative, bipolar-staining coccobacillus, with 1000–5000 cases annually worldwide, endemic in Madagascar, Democratic Republic of Congo, and Peru, transmitted from infected rodents to humans by fleas, and is a category A bioterrorism agent.

Causes

This condition happens because of *Y. pestis* being injected by flea bite, phagocytosed by macrophages, and resisting intracellular killing via a type III secretion system, with virulence factors including the F1 capsular antigen and Yops.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bubonic plague (most common, 80%): sudden fever, chills, headache, and tender, swollen, matted lymph nodes (buboes)
- Septicemic plague: high fever, hypotension, DIC, purpuric rash, and acral tissue death
- Pneumonic plague: fever, productive cough, hemoptysis, and rapidly progressive respiratory failure.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of plague (*Yersinia pestis*) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using Gram stain and culture from bubo aspirate, blood, or sputum. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include streptomycin or gentamicin, with alternatives including doxycycline, ciprofloxacin, and levofloxacin. outlook: bubonic 5–15% mortality (treated)

- Septicemic 30–50%
- Pneumonic nearly 100% if untreated > 24 hours.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 668

Plantar Fasciitis

Overview

It affects approximately 10% of the population at some point in their life, with peak incidence between ages 40 and 60.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of plantar fasciitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Plantar fasciitis is the most common cause of heel pain in adults, resulting from overuse injury and deterioration of the plantar fascia, a thick band of connective

tissue originating from the medial tubercle of the calcaneus and inserting into the toes. This condition happens because of repetitive microtrauma and deterioration of the plantar fascia. The classic The condition usually appears as sharp heel pain with the first steps in the morning or after rest ("start-up pain"), which improves with activity but may worsen with prolonged standing

- Pain is maximal at the medial heel.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Imaging (X-ray, ultrasound, or MRI) is rarely needed but may show calcaneal spur (present in 50% of cases but often asymptomatic) or plantar fascia thickening.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include rest, activity modification, stretching exercises (calf and plantar fascia), ice massage, orthotics, and night splints

- Corticosteroid injection may provide short-term relief but should be limited
- Extracorporeal shockwave therapy is an alternative for refractory cases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Most cases resolve within 6–18 months with conservative management.

Chapter 669

Pleurisy

Overview

- Pleurisy is an inflammation of the pleural membranes causing sharp, stabbing chest pain that worsens with breathing, coughing, or sneezing
- Causes include viral infections (most common), bacterial pneumonia with parapneumonic effusion, lung blockage by a blood clot, autoimmune diseases, malignancy, tuberculosis, and uremia.

Causes

This condition happens because of inflammation of the parietal and visceral pleura.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience pleuritic chest pain (worse on inspiration, improved by holding breath) and possibly a pleural friction rub on auscultation
- Pleural effusion may develop.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of pleurisy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves chest X-ray, thoracic ultrasound, diagnostic thoracentesis with Light's criteria, and CT chest for underlying pathology. Management targets the underlying cause

- Pleuritic pain is managed with NSAIDs
- Large symptomatic effusions may require therapeutic thoracentesis
- Recurrent pleurisy may necessitate pleurodesis.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 670

Pneumoconiosis

Overview

Pneumoconiosis is a group of interstitial lung diseases caused by inhalation of mineral dusts and fibrogenic particles.

Causes

This condition results from disruption of normal pneumoconiosis function.

- Asbestosis results from asbestos fiber inhalation, causing basal-predominant interstitial scarring and pleural plaques, with a latency period of 10–20 years
- Silicosis (from silica dust) presents in chronic, accelerated, and acute forms
- Coal workers' pneumoconiosis results from coal dust exposure
- Berylliosis is a granulomatous disease from beryllium exposure. This condition happens because of accumulation of inhaled mineral dusts in the lungs, triggering chronic inflammation and scarring.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive shortness of breath and cough.

Emergency warning signs

- Severe or worsening symptoms of pneumoconiosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using history of occupational exposure, HRCT, and lung function tests showing restriction with reduced DLCO. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment focuses on exposure removal and supportive care.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Your outlook depends on the type and duration of exposure
- These conditions are largely preventable through occupational safety measures.

Chapter 671

Pneumocystis jirovecii Pneumonia

Overview

Pneumocystis jirovecii pneumonia (PCP) is an opportunistic fungal infection of the lungs caused by the atypical yeast-like fungus *Pneumocystis jirovecii*, previously classified as a protozoan and formerly named *Pneumocystis carinii*. It remains a defining opportunistic infection in patients with advanced HIV/AIDS (CD4 count below 200 cells/ μ L) and also occurs in other immunocompromised populations including organ transplant recipients, patients on chronic corticosteroids, and those receiving immunosuppressive chemotherapy. Despite effective prophylaxis, PCP continues to be a significant cause of morbidity and mortality in immunocompromised patients worldwide.

Classification

Cause Infectious (fungal)

Organ System Lungs/Respiratory

Pathophysiology Attachment of *Pneumocystis* organisms to Type I alveolar epithelial cells causing foamy intra-alveolar exudate, alveolar-capillary barrier disruption, interstitial inflammation, and impaired gas exchange

Duration Acute to Subacute

Onset Gradual (over days to weeks)

Mode of Transmission Airborne (inhalation of fungal elements)

Clinical Course Progressive if untreated; respiratory failure develops in advanced cases

Severity Moderate to Severe

Extent of Disease Localized to lungs (can disseminate in severe immunosuppression)

Age Group Immunocompromised individuals of all ages

Epidemiology Leading AIDS-defining illness worldwide; incidence decreased with antiretroviral therapy and prophylaxis but remains significant in undiagnosed HIV and non-HIV immunosuppressed patients

ICD-11 Code CA42.0

Causes

Pneumocystis jirovecii is an atypical fungus transmitted via airborne route that causes infection almost exclusively in immunocompromised hosts. In HIV-associated PCP,

disease typically occurs when the CD4 count falls below 200 cells/ μ L. Non-HIV risk factors include prolonged high-dose corticosteroid use, solid organ or hematopoietic stem cell transplantation, hematologic malignancies, chemotherapy (especially rituximab and purine analogs), and primary immunodeficiency disorders such as severe combined immunodeficiency (SCID).

Risk Factors

- HIV/AIDS with CD4 count less than 200 cells/ μ L
- Chronic corticosteroid therapy (equivalent to prednisone ≥ 20 mg/day for ≥ 2 weeks)
- Solid organ transplantation (kidney, liver, lung, heart) with immunosuppression
- Hematologic malignancies (leukemia, lymphoma) and chemotherapy
- Rituximab, fludarabine, and other lymphocyte-depleting agents
- Primary immunodeficiency disorders (SCID, hyper-IgM syndrome)
- Inflammatory conditions requiring prolonged immunosuppression (Wegener granulomatosis, inflammatory bowel disease)

Signs and Symptoms

Early symptoms

- Early manifestations of pneumocystis jirovecii pneumonia may be subtle and non-specific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Subacute onset of progressive dyspnea on exertion
- Non-productive cough (dry cough)
- Fever with night sweats
- Bilateral diffuse pulmonary infiltrates on chest imaging
- Hypoxemia with increased alveolar-arterial oxygen gradient
- Tachypnea and labored breathing
- Fatigue and weight loss
- Chest tightness or discomfort
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of pneumocystis jirovecii pneumonia may occur

Emergency warning signs

- Severe or worsening symptoms of pneumocystis jirovecii pneumonia require immediate medical evaluation

Progression and Stages

PCP typically develops subacutely over 1–3 weeks in HIV-positive patients with progressive dyspnea, fever, and dry cough, while a more fulminant course over days is seen in non-HIV immunocompromised patients. Untreated infection progresses to severe hypoxemic respiratory failure requiring mechanical ventilation. With treatment, clinical improvement is usually evident within 4–8 days, though radiographic resolution may take weeks. Relapse can occur if immunosuppression persists without prophylaxis.

Complications

- Severe hypoxemic respiratory failure requiring intubation and mechanical ventilation
- Pneumothorax (spontaneous or from barotrauma)
- Acute respiratory distress syndrome (ARDS) in severe cases
- Disseminated infection (lymph nodes, liver, spleen, bone marrow) in profound immunosuppression
- Mortality of 10–20% in HIV-associated PCP and 30–50% in non-HIV immunocompromised patients
- Immune reconstitution inflammatory syndrome (IRIS) after starting antiretroviral therapy
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Chest radiograph or CT showing bilateral interstitial or alveolar infiltrates (ground-glass opacification)
- Elevated serum lactate dehydrogenase (LDH) as a suggestive but non-specific marker
- Induced sputum or bronchoalveolar lavage (BAL) with Gomori methenamine silver stain, immunofluorescence, or PCR
- Bronchoscopy with BAL is the diagnostic gold standard
- Beta-D-glucan assay in serum (high sensitivity but not specific)
- Pulse oximetry and arterial blood gas to assess oxygenation
- HIV testing in newly diagnosed patients
- Imaging tests to evaluate extent of lung involvement

Prevention

- Primary prophylaxis with trimethoprim-sulfamethoxazole (TMP-SMX) for HIV patients with CD4 <200 cells/ μ L and other high-risk immunocompromised patients
- Alternative prophylactic regimens: dapsone, atovaquone, or aerosolized pentamidine
- Antiretroviral therapy (ART) for HIV patients to maintain CD4 count above 200
- Consider discontinuation of primary prophylaxis when CD4 count remains >200 for ≥ 3 months on ART
- Environmental precautions (avoiding crowded settings) for profoundly immunocompromised patients
- Go for regular check-ups
- Follow your doctor's screening recommendations

Treatment Options

- First-line therapy: trimethoprim-sulfamethoxazole (TMP-SMX) for 21 days
- Adjunctive corticosteroids for moderate-to-severe PCP with PaO₂ <70 mmHg or alveolar-arterial gradient >35 mmHg
- Second-line therapy: atovaquone suspension or clindamycin plus primaquine
- Pentamidine (intravenous) for severe disease with contraindications to TMP-SMX
- Dapsone plus trimethoprim as alternative outpatient therapy
- Initiation or optimization of antiretroviral therapy in HIV patients
- Supportive care with supplemental oxygen and respiratory support as needed
- Lifestyle changes and self-care
- Surgery when needed (chest tube for pneumothorax)

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups
- Keep learning about your condition

Outlook

The prognosis for PCP depends on the underlying immune status with mortality of 10–20% in HIV-associated cases and 30–50% in non-HIV immunocompromised patients. With appropriate treatment and, in HIV patients, effective antiretroviral therapy, most patients recover from the acute infection and can resume normal activities with appropriate ongoing prophylaxis.

Chapter 672

Pneumothorax

Overview

Pneumothorax is the accumulation of air in the pleural space, leading to partial or complete lung collapse.

Causes

This condition happens because of rupture of subpleural apical blebs (primary spontaneous pneumothorax, typically in tall, thin young men), underlying lung disease (secondary spontaneous pneumothorax in COPD, cystic scarring), or chest trauma. Tension pneumothorax occurs when a one-way valve mechanism allows air to accumulate continuously under pressure, causing mediastinal shift and hemodynamic collapse.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sudden-onset ipsilateral pleuritic chest pain and shortness of breath, with hyperresonance and decreased breath sounds on the affected side.

Emergency warning signs

- Severe or worsening symptoms of pneumothorax require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using chest radiograph showing a visceral pleural line without peripheral lung markings

- Tension pneumothorax is a clinical diagnosis requiring immediate needle decompression at the second intercostal space in the midclavicular line prior to imaging. Treatment for small spontaneous pneumothoraces involves observation and oxygen administration
- Larger pneumothoraces require tube thoracostomy (chest tube insertion).
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with prompt treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 673

Poliomyelitis

Overview

Poliomyelitis is a highly infectious viral disease caused by poliovirus (serotypes 1, 2, 3) of the *Enterovirus* genus, with endemic transmission persisting in Afghanistan and Pakistan. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of fecal-oral transmission, viral replication in the oropharynx and gut, and hematogenous spread to the central nervous system, where it infects and destroys anterior horn cells of the spinal cord and motor nuclei of the brainstem. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience asymptomatic infection (> 90%), abortive poliomyelitis (febrile illness with sore throat, nausea), non-paralytic aseptic meningitis (1–5%), and paralytic poliomyelitis (0.1–2%) presenting with asymmetric, flaccid paralysis of lower limbs more than upper limbs, areflexia, without sensory loss. Bulbar paralysis affects cranial nerves and respiratory muscles. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of poliomyelitis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this based on stool viral culture, RT-PCR of stool or CSF, and serology. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- Treatment typically involves supportive: physical therapy, orthotics, respiratory support for bulbar involvement, and prevention of complications. outlook for paralytic poliomyelitis: 10% die from respiratory paralysis, 30% recover some function, 60% have permanent paralysis. Prevention is through OPV (Sabin) or IPV (Salk) vaccination
- Global eradication is 99.9% complete.
- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification

- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 674

Polyarteritis Nodosa

Overview

Polyarteritis nodosa is a systemic necrotizing vasculitis affecting medium-sized arteries, characterized by inflammation and fibrinoid tissue death of the vessel wall, with aneurysm formation and blood clot formation, sparing small vessels (capillaries, venules) and not associated with ANCA, with an incidence of 2–3 per million, a slight male predominance, and affecting all ages. The condition involves immune complex deposition in the arterial wall (especially in HBV-associated PAN), complement activation, and neutrophil recruitment leading to vessel wall tissue death, with approximately 10–20% of cases associated with chronic hepatitis B virus (HBV) infection.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience constitutional symptoms (fever, weight loss, muscle pain, joint pain), mononeuritis multiplex (the most characteristic neurological manifestation), livedo reticularis, testicular pain, kidney artery involvement (kidney tissue death due to lack of blood supply, high blood pressure, but not glomerulonephritis—distinguishing PAN from microscopic polyangiitis), mesenteric reduced blood flow, and cutaneous manifestations (nodules, ulcers, gangrene).

Emergency warning signs

- Severe or worsening symptoms of polyarteritis nodosa require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- To make the diagnosis, doctors look for biopsy of affected tissue showing necrotizing arteritis with fibrinoid tissue death, or angiography showing microaneurysms and irregular stenoses. Treatment for HBV-negative PAN is high-dose corticosteroids plus cyclophosphamide for severe disease
- HBV-associated PAN requires antiviral therapy plus a shorter course of corticosteroids and plasmapheresis.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with treatment, but severe cases can be fatal. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 675

Polycystic Ovary Syndrome

Overview

Polycystic ovary syndrome (PCOS) is the most common cause of anovulatory infertility and oligomenorrhea. Key management considerations include endometrial protection from unopposed estrogen: annual ultrasound assessment of endometrial thickness and biopsy if indicated, with progestins or combined oral contraceptives for cycle regulation. Long-term health risks include a 3–7-fold increased risk of endometrial cancer, 50–70% lifetime risk of metabolic syndrome, and increased cardiovascular risk. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of polycystic ovary syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care
Enzyme replacement therapy for certain conditions
Gene therapy (emerging for select conditions)
Dietary modifications (e.g., phenylketonuria)
Regular monitoring for associated complications
Physical and occupational therapy
Genetic counseling for family planning
Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 676

Polycystic Ovary Syndrome

Overview

Polycystic ovary syndrome is the most common endocrine disorder in women of reproductive age, affecting 6–12%. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of insulin resistance (present in 50–70%), hyperinsulinemia, and relative hyperandrogenism. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience menstrual irregularity, infertility, hirsutism, acne, obesity (60–80%), metabolic syndrome, type 2 diabetes mellitus, and increased cardiovascular risk.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of polycystic ovary syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for two of three Rotterdam criteria: oligo-ovulation or anovulation, clinical or biochemical hyperandrogenism, and polycystic ovarian morphology on ultrasound, with exclusion of other causes.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment typically involves tailored to goals: combined oral contraceptives for menstrual regulation and hyperandrogenism, metformin or inositol for insulin resistance, letrozole for ovulation induction in infertility, and lifestyle modification.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with appropriate management.

Chapter 677

Polycythemia Vera

Overview

Polycythemia vera is a Philadelphia chromosome-negative myeloproliferative tumor characterized by clonal growth of hematopoietic stem cells, predominantly producing excess red blood cells, and often accompanied by leukocytosis and thrombocytosis, with over 95% of cases harboring the JAK2 V617F mutation (or JAK2 exon 12 mutations).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- The WHO 2022 diagnostic criteria require hemoglobin >16.5 g/dL in men or >16.0 g/dL in women, or increased red cell mass, plus bone marrow biopsy showing hypercellularity with trilineage growth, or JAK2 mutation.

Emergency warning signs

- Severe or worsening symptoms of polycythemia vera require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

You may experience erythromelalgia (burning pain and redness of extremities), itching (especially aquagenic—after bathing), headache, plethora, hepatosplenomegaly, and thrombotic or involving bleeding complications.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include phlebotomy (target hematocrit $<45\%$), low-dose aspirin, and cytoreductive therapy (hydroxyurea for high-risk patients), with ruxolitinib (JAK1/2 inhibitor) used for hydroxyurea-resistant or intolerant disease. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable but carries a risk of transformation to myelofibrosis (10–20%) or AML (5–10%). The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 678

Polyhydramnios and Oligohydramnios

Overview

Polyhydramnios is an excessive volume of amniotic fluid, defined by an amniotic fluid index (AFI) >24 cm or a deepest vertical pocket >8 cm, occurring in 1–2% of pregnancies.

Causes

Causes include idiopathic (50–60%), maternal diabetes (10–15%), fetal anomalies impairing swallowing, and twin-twin transfusion syndrome. Oligohydramnios is deficient amniotic fluid (AFI <5 cm or deepest vertical pocket <2 cm), affecting 1–2% of pregnancies. Causes include fetal anomalies (kidney agenesis, posterior urethral valves, obstructive uropathy), preterm PROM, placental insufficiency, and post-term pregnancy. Severe oligohydramnios in the second trimester causes lung underdevelopment (Potter sequence) and limb compression deformities.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of polyhydramnios and oligohydramnios require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

The right treatment depends on cause and gestational age. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 679

Polymyalgia Rheumatica

Overview

Polymyalgia rheumatica is an inflammatory disorder of the elderly, characterized by pain and stiffness of the shoulder and pelvic girdles, affecting individuals over 50 years of age (peak incidence 70–80 years), with a female predominance of 2–3:1.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience acute or subacute onset of severe morning stiffness lasting over 30 minutes in the shoulders, neck, and hips, with associated muscle pain, constitutional symptoms (low-grade fever, fatigue, weight loss), and difficulty raising arms or rising from a chair, with peripheral arthritis (knees, wrists) occurring in 25–50%. PMR overlaps significantly with giant cell arteritis (GCA): 15–30% of PMR patients have concurrent GCA, and 40–60% of GCA patients have PMR symptoms.

Emergency warning signs

- Severe or worsening symptoms of polymyalgia rheumatica require immediate medical evaluation

Progression and Stages

The cause is unknown, but genetic associations with HLA-DR4 and environmental triggers have been implicated, with Disease mechanism involving local inflammation of proximal joints and extra-articular synovial structures, infiltration of macrophages and CD4+ T cells, elevated IL-6 levels, and increased acute-phase reactants. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by elevated ESR (often >50–100 mm/hr) and CRP, with imaging (ultrasound, MRI) showing synovitis, bursitis, and tenosynovitis. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves with low-to-moderate dose corticosteroids (prednisone 15–25 mg/day), typically with rapid clinical response within 24–48 hours and slow tapering over 12–18 months. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good, with most patients achieving remission within 1–3 years. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 680

Polymyositis

Overview

Polymyositis is an idiopathic inflammatory myopathy characterized by symmetric proximal muscle weakness and endomysial CD8+ T-cell infiltration of muscle fibers. It primarily affects adults, with a slight female predominance.

Causes

This condition happens because of autoimmune-mediated muscle fiber destruction.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive symmetric proximal muscle weakness (difficulty rising from chairs, climbing stairs, lifting objects overhead), difficulty swallowing, and rarely respiratory muscle weakness.

Emergency warning signs

- Severe or worsening symptoms of polymyositis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on elevated CK (10–50 times normal), EMG showing myopathic changes, MRI showing muscle swelling, and definitive muscle biopsy. Muscle-specific autoantibodies (anti-Jo-1, anti-Mi-2, anti-MDA5) may support diagnosis. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include corticosteroids and steroid-sparing immunosuppressive agents (methotrexate, azathioprine, mycophenolate)
- IVIG may be used for refractory disease.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with many patients responding to immunosuppression. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 681

Porphyrias

Overview

- The porphyrias are a group of inherited or acquired disorders of heme biosynthesis caused by deficiency of specific enzymes in the heme biosynthetic pathway, leading to accumulation of porphyrins or their precursors. Acute intermittent porphyria (AIP) is caused by porphobilinogen deaminase deficiency (autosomal dominant with low penetrance), the most common acute liver porphyria
- Attacks are precipitated by drugs, fasting, infections, and hormonal changes.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe abdominal pain (90%), neuropsychiatric symptoms, autonomic dysfunction, and peripheral motor neuropathy
- Cutaneous photosensitivity is absent.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of porphyrias require immediate medical evaluation

Progression and Stages

They are classified as liver or erythropoietic. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using elevated urinary porphobilinogen (PBG) and aminolevulinic acid (ALA) during attacks. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include removing precipitating factors, high-carbohydrate loading, intravenous hemin, and givosiran for prevention.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Porphyria cutanea tarda (PCT) is caused by uroporphyrinogen decarboxylase deficiency, the most common porphyria, associated with alcohol, hepatitis C, estrogen therapy, and hemochromatosis
- You may experience blistering skin lesions on sun-exposed areas

- Treatment options include phlebotomy or low-dose hydroxychloroquine. Erythropoietic protoporphyria (EPP) is caused by ferrochelatase deficiency, causing photosensitivity in childhood.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 682

Portal Hypertension

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Portal high blood pressure is defined as a liver venous pressure gradient (HVPG) of 5 mmHg or greater, with clinically significant portal high blood pressure at HVPG of 10 mmHg or greater
- The most common cause is cirrhosis.

Causes

This condition happens because of increased resistance to portal blood flow. Consequences include esophageal and gastric varices, ascites, portosystemic collaterals, splenomegaly, and hepatorenal syndrome. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)

- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of portal hypertension require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

Your symptoms depend on the complications present.

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

- Your doctor confirms the diagnosis with HVPG measurement via right heart catheterization
- Non-invasive assessment includes platelet count, transient elastography, and endoscopy.
- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

Treatment options include non-selective beta-blockers (propranolol, nadolol) and using a thin camera tube variceal band ligation for primary and secondary prophylaxis of variceal bleeding.

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)

- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Your outlook depends on the underlying liver disease and response to treatment. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 683

Post-Traumatic Headache

Overview

Post-traumatic headache (PTH) is a secondary headache disorder and the most common symptom following traumatic brain injury (TBI), affecting 30–90% of patients. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. This condition results from acute injury or exposure to harmful agents. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of a neurometabolic cascade after TBI including axonal injury, excitotoxicity, mitochondrial dysfunction, altered cerebral blood flow, and activation of the trigeminovascular system. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features most commonly mimic migraine or tension-type headache. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of post-traumatic headache require immediate medical evaluation

Progression and Stages

It is classified as acute (<3 months) or persistent (≥ 3 months). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam with history of TBI. Management follows standard primary headache guidelines with amitriptyline, topiramate, and gabapentin commonly used
- Physical therapy for associated cervical dysfunction is essential.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Most improve within 3–6 months but some remain symptomatic for years.

Chapter 684

Post-Traumatic Stress Disorder (PTSD)

Overview

Post-traumatic stress disorder has a lifetime prevalence of 6–8% in the general population, with higher rates in combat veterans (10–30%), sexual assault survivors (30–50%), and first responders (10–20%), and is twice as common in women. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. This condition results from acute injury or exposure to harmful agents. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of an overly responsive amygdala (hypervigilance, exaggerated fear), a hypoactive medial prefrontal cortex (impaired fear extinction), reduced hippocampal volume, dysregulation of the HPA axis, and heightened sympathetic reactivity, with fear conditioning to trauma-related cues failing to extinguish normally. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features require exposure to actual or threatened death, serious injury, or sexual violence, followed by four symptom clusters for at least 1 month: intrusion symptoms (recurrent, involuntary, intrusive memories, distressing dreams, dissociative reactions, flashbacks), avoidance (persistent avoidance of trauma-related stimuli), negative alterations in thinking and mood, and marked alterations in arousal and reactivity (irritable behavior, hypervigilance, exaggerated startle response, sleep disturbance).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of post-traumatic stress disorder (ptsd) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview using DSM-5-TR criteria.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include trauma-focused psychotherapy as first-line (TF-CBT, prolonged exposure, CPT, EMDR) and pharmacotherapy (SSRIs and SNRIs are FDA-approved). Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is chronic in many cases
- About 30–40% recover fully, 30–40% have partial improvement, and 20–30% have persistent symptoms.

Chapter 685

Postherpetic Neuralgia

Overview

Postherpetic neuralgia (PHN) is neuropathic pain persisting for ≥ 3 months after the resolution of herpes zoster (shingles) rash. It occurs in 10–20% of patients with herpes zoster, increasing to $>30\%$ in patients over 60 years of age. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of peripheral and central sensitization from varicella-zoster virus reactivation in the dorsal root ganglia, leading to nerve damage, inflammation, and neuronal hyperexcitability. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience burning, aching, electric, and lancinating pain with allodynia and hyperalgesia in a dermatomal distribution (most commonly thoracic). Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of postherpetic neuralgia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam. First-line pharmacotherapy includes topical lidocaine 5% patches, pregabalin, gabapentin, and TCAs
- Second-line includes tramadol, capsaicin 8% patches, and opioid analgesics.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- The outlook varies from person to person
- Prevention is the best treatment, with herpes zoster vaccination (recombinant zoster vaccine) reducing the incidence of PHN by approximately 90%.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 686

Postpartum Cardiomyopathy

Overview

Peripartum cardiomyopathy (PPCM) is an idiopathic dilated cardiomyopathy presenting with left ventricular systolic dysfunction (LVEF <45%) in the last month of pregnancy through five months postpartum, occurring in 1 in 1000–4000 pregnancies. The Disease mechanism involves oxidative stress, a 16-kDa prolactin cleavage fragment (vasoinhibin) that impairs cardiac microvascular function, and inflammation. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)

- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience shortness of breath, orthopnea, paroxysmal nocturnal shortness of breath, peripheral swelling, cough, and signs of left heart failure. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of postpartum cardiomyopathy require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia

- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using echocardiogram showing left ventricular dilation and reduced ejection fraction. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

- Treatment typically involves standard heart failure therapy adjusted for pregnancy and lactation
- Bromocriptine may be used in some centers.
- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)

- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Recovery varies: 50–60% recover LVEF to $\geq 50\%$ within 6 months. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 687

Postpartum Depression

Overview

Postpartum depression (PPD) is a major depressive episode with onset within 4–6 weeks of childbirth (DSM-5 specifies peripartum onset). The Disease mechanism involves a rapid drop in estrogen and progesterone after delivery, altered hypothalamic-pituitary-adrenal axis function, genetic predisposition, and psychosocial factors. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience persistent sadness, anhedonia, crying spells, fatigue, sleep and appetite disturbance, feelings of worthlessness, difficulty bonding with the infant, and in severe cases, thoughts of harming self or baby.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of postpartum depression require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

It is the most common complication of the puerperium, affecting 10–15% of postpartum women.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical assessment using the Edinburgh Postnatal Depression Scale (EPDS, score ≥ 10).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include psychotherapy and antidepressant medication (SSRIs—sertraline and fluoxetine are first-line)
- For postpartum psychosis (rare, 1–2 per 1000 deliveries), emergency psychiatric care is required.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 688

Postpartum Endometritis

Overview

Postpartum endometritis is infection of the decidua and myometrium after delivery, most commonly following cesarean delivery (5–30%, versus 1–3% after vaginal delivery). This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

This condition happens because of ascending infection from the lower genital tract, most commonly polymicrobial with aerobes and anaerobes.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fever (typically within 24–72 hours of delivery), tachycardia, uterine tenderness, and foul-smelling lochia.

Emergency warning signs

- Severe or worsening symptoms of postpartum endometritis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves IV broad-spectrum antibiotics (clindamycin plus gentamicin is the standard regimen)
- If no response within 48 hours, add ampicillin for enterococcal coverage.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with antibiotics. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 689

Postpartum Hemorrhage

Overview

Postpartum bleeding (PPH) is defined as blood loss >500 mL after vaginal delivery or >1000 mL after cesarean delivery. It is the leading cause of maternal mortality worldwide, accounting for 25% of maternal deaths. Causes are categorized by the “four Ts”: Tone (uterine atony—70–80% of cases), Trauma (lacerations, episiotomy, uterine rupture), Tissue (retained placenta, placenta accreta), and Thrombin (coagulopathy). This condition results from acute injury or exposure to harmful agents. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience excessive bleeding after delivery with signs of hemodynamic compromise.

Emergency warning signs

- Severe or worsening symptoms of postpartum hemorrhage require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam. Management follows a systematic protocol: step 1—medical management (uterine massage, oxytocin, methergine, misoprostol, carboprost)
- Step 2—mechanical measures (uterine balloon tamponade, compression sutures)
- Step 3—surgical intervention (surgical opening of the abdomen, uterine artery ligation, hysterectomy). Concurrent Treatment options include aggressive fluid resuscitation, transfusion of blood products, and monitoring for DIC.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on prompt recognition and treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 690

Postpartum Thyroiditis

Overview

Postpartum thyroiditis (PPT) is an autoimmune inflammation of the thyroid gland occurring within the first year after delivery, affecting 5–10% of women. It is caused by a rebound of immune activity after the relative immunosuppression of pregnancy resolves, in susceptible women who are positive for thyroid peroxidase (TPO) antibodies. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)

- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of postpartum thyroiditis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

- You may experience a transient thyrotoxic phase (4–8 weeks, 30% of cases) followed by a hypothyroid phase (4–12 weeks, 40%), with 25% remaining hypothyroid permanently. The thyrotoxic phase is managed symptomatically with beta-blockers (not antithyroid drugs)
- The hypothyroid phase doctors typically treat it with levothyroxine.

Complications

- Irreversible organ damage from chronic inflammation

- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Doctors diagnose this based on TSH and TPO antibody testing. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild

disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

- The outlook is generally positive
- Recurrence in subsequent pregnancies is common.

Chapter 691

Prader-Willi Syndrome

Overview

Prader-Willi syndrome (PWS) is a neurogenetic disorder caused by loss of function of the paternal copy of genes on chromosome 15q11-q13, the region imprinted on the maternal allele. Mechanisms include paternal deletion (70%), maternal uniparental disomy (25%), and imprinting defects (5%). Incidence is 1 in 10,000–30,000. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features are biphasic: in infancy, hypotonia, poor feeding, and failure to thrive
- In childhood, hyperphagia (insatiable appetite), obesity, short stature, hypogonadism, small hands and feet, and mild to moderate intellectual disability.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of prader-willi syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using methylation analysis.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include growth hormone therapy, strict dietary supervision to prevent obesity, and behavioral interventions.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupa-

tional therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 692

Preeclampsia

Overview

Preeclampsia is a multisystem disorder of pregnancy characterized by high blood pressure and proteinuria developing after 20 weeks, affecting 2–8% of pregnancies worldwide. It is a leading cause of maternal death, preterm birth, and intrauterine growth restriction. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

This condition happens because of abnormal placental implantation with inadequate spiral artery remodeling, placental reduced blood flow, systemic endothelial dysfunction, and an imbalance of angiogenic factors (elevated soluble Flt-1, reduced PlGF).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience high blood pressure and proteinuria, with severe features including BP $\geq$ 160/110 mmHg, thrombocytopenia, elevated liver enzymes, serum creatinine $>$ 1.1 mg/dL, and persistent cerebral symptoms.

Emergency warning signs

- Severe or worsening symptoms of preeclampsia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- To make the diagnosis, doctors look for BP $\geq 140/90$ mmHg on two occasions four hours apart plus proteinuria ≥ 300 mg/24 hours
- In the absence of proteinuria, new-onset high blood pressure with thrombocytopenia, kidney insufficiency, impaired liver function, lung swelling, or cerebral symptoms may be diagnostic.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment for term preeclampsia is delivery
- Preterm preeclampsia is managed expectantly with antihypertensives, magnesium sulfate for seizure prophylaxis, and corticosteroids for fetal lung maturity
- Low-dose aspirin initiated before 16 weeks reduces risk in high-risk women.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable with appropriate management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 693

Premature Ovarian Insufficiency

Overview

Premature ovarian insufficiency (POI) is the loss of ovarian function before age 40, characterized by amenorrhea, elevated FSH (>40 IU/L), and low estradiol. It affects 1% of women under 40. Causes are idiopathic (70–90%), genetic, autoimmune, and iatrogenic.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience oligomenorrhea or amenorrhea, infertility, vasomotor symptoms, vaginal dryness, and psychological distress. POI has significant long-term health consequences due to prolonged estrogen deficiency: osteoporosis, cardiovascular disease, and thinking and memory decline.

Emergency warning signs

- Severe or worsening symptoms of premature ovarian insufficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment options include MHT (estrogen plus progestin continued until the average age of menopause), calcium and vitamin D supplementation, and fertility counseling.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with hormone replacement. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 694

Premature Rupture of Membranes

Overview

Premature rupture of membranes (PROM) is rupture of the amniotic sac before the onset of labor. PROM at term (≥ 37 weeks) occurs in 8% of pregnancies and is managed with induction if labor does not ensue spontaneously within 12–24 hours. Preterm PROM (PPROM, < 37 weeks) occurs in 3% of pregnancies and is linked to ascending infection, chorioamnionitis, and preterm delivery.

Causes

This condition happens because of weakening of the fetal membranes.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fluid leakage from the vagina.

Emergency warning signs

- Severe or worsening symptoms of premature rupture of membranes require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on history, sterile speculum examination (pooling of fluid, positive Nitrazine test, ferning pattern on microscopy). Treatment for PPRM remote from term includes antenatal corticosteroids, antibiotics to prolong latency, and close surveillance for infection
- Delivery is indicated for chorioamnionitis, non-reassuring fetal status, or at 34–37 weeks.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on gestational age at delivery. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 695

Premenstrual Syndrome / Premenstrual Dysphoric Disorder

Overview

This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms include emotional (irritability, anxiety, depression, mood swings), physical (breast tenderness, bloating, fatigue, headaches), and behavioral changes that resolve within days of menstruation. Premenstrual dysphoric disorder (PMDD) is a severe form affecting 3–8%, characterized by marked affective lability, irritability, depressed mood, and anxiety, causing significant functional impairment. The Disease mechanism involves abnormal central nervous system response to normal cyclical fluctuations of estrogen and progesterone.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of premenstrual syndrome / premenstrual dysphoric disorder require immediate medical evaluation

Progression and Stages

Premenstrual syndrome (PMS) is a cyclical symptom complex occurring in the luteal phase of the menstrual cycle, affecting 20–30% of reproductive-age women. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors use DSM-5 criteria requiring prospective daily symptom charting across two cycles.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care
Enzyme replacement therapy for certain conditions
Gene therapy (emerging for select conditions)
Dietary modifications (e.g.,

phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment.

Chapter 696

Presbycusis

Overview

Presbycusis is age-related hearing loss affecting two-thirds of adults over 70. In older adults, untreated hearing loss is linked to thinking and memory decline (increased dementia risk), social isolation, depression, and falls.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of presbycusis require immediate medical evaluation

Progression and Stages

It is a sensorineural hearing loss, typically bilateral, symmetrical, and high-frequency (above 2000 Hz), resulting from deterioration of cochlear hair cells, stria vascularis, and spiral ganglion neurons. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include hearing aids (rehabilitative benefit is well established), cochlear implants for severe-to-profound loss, communication strategies, and treating cerumen impaction (common, easily reversible cause of hearing loss in older adults). Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with amplification. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 697

Presbyopia

Overview

- Presbyopia is the age-related loss of accommodation, resulting from hardening of the crystalline lens and decreased ciliary muscle function
- It is universal, typically becoming symptomatic after age 40.

Causes

This condition happens because of age-related changes in the lens and ciliary muscle.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience difficulty reading small print, especially in dim light, and the need to hold reading material farther away.

Emergency warning signs

- Severe or worsening symptoms of presbyopia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical history and refraction testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include reading glasses, bifocal or progressive lenses, monovision contact lens arrangements, and multifocal intraocular lenses
- Surgical options include corneal inlays, scleral implants, and laser procedures.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate correction. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 698

Presbyopia and Cataracts

Overview

Presbyopia is the age-related loss of accommodation resulting from lens hardening, affecting virtually all adults over 45, corrected with reading glasses, bifocals, or progressive lenses. Surgical removal with intraocular lens (IOL) implantation is The main treatment, with phacoemulsification being the standard technique, yielding >95% improvement in visual acuity. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of presbyopia and cataracts require immediate medical evaluation

Progression and Stages

Cataracts—opacification of the crystalline lens—affect 50% of adults over 75 and are the leading cause of reversible vision loss worldwide, classified by location: nuclear sclerotic (most common, progressive myopic shift), cortical (spoke-like opacities), and posterior

subcapsular (associated with corticosteroid use, diabetes, and radiation). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 699

Pressure Ulcers

Overview

Pressure ulcers (bedsores, decubitus ulcers) are localized injury to skin and underlying tissue caused by prolonged pressure, shear, or friction, most commonly over bony prominences (sacrum, heels, greater trochanters, ischial tuberosities).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of pressure ulcers require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Symptoms can range from non-blanchable redness of the skin (Stage 1) to full-thickness tissue loss with exposed bone/tendon/muscle (Stage 4), with stages defined

by the National Pressure Injury Advisory Panel (NPIAP). The outlook varies from person to person

- Healing depends on the stage and underlying patient condition.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with staging. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Prevention is paramount: risk assessment (Braden Scale), pressure-redistribution surfaces, regular repositioning every 2 hours, nutritional support, and skin care.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Prevalence is 5–15% in hospitalized patients and up to 25% in long-term care.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 700

Preterm Birth

Overview

Preterm birth is a perinatal condition defined as delivery before 37 weeks of gestation. It affects approximately 10% of all pregnancies worldwide and is the leading cause of neonatal morbidity and mortality.

Causes

This condition happens because of multiple pathogenic pathways including infection, inflammation, and uteroplacental reduced blood flow.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of preterm birth require immediate medical evaluation

Progression and Stages

You may experience delivery before 37 weeks, classified as late preterm (34–36 weeks), moderate preterm (32–33 weeks), very preterm (28–31 weeks), and extremely preterm (<28 weeks), with complications inversely related to gestational age. The condition may

progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include antenatal corticosteroids (betamethasone) to accelerate fetal lung maturation, magnesium sulfate for neuroprotection, and transfer to a facility with a level III NICU
- Tocolysis may be used to delay delivery for corticosteroid administration.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on gestational age and birth weight. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 701

Preterm Labor and Preterm Birth

Overview

Preterm labor is defined as regular uterine contractions causing cervical change between 20 and 36 weeks 6 days of gestation. Preterm birth (delivery before 37 weeks) affects 10% of pregnancies worldwide and is the leading cause of neonatal mortality and morbidity. The Disease mechanism involves multiple pathways: infection/inflammation (40% of early preterm births), uteroplacental reduced blood flow, uterine overdistension, and immunologically mediated rejection. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience regular contractions with cervical change before 37 weeks.

Emergency warning signs

- Severe or worsening symptoms of preterm labor and preterm birth require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- To make the diagnosis, doctors look for documented cervical change on exam
- Transvaginal ultrasound measurement of cervical length (<25 mm at 20–24 weeks is high risk) and fetal fibronectin testing help stratify risk. Acute Treatment options include antenatal corticosteroids for fetal lung maturity (24–34 weeks), magnesium sulfate for fetal neuroprotection (24–32 weeks), and tocolysis to delay delivery for 48 hours to allow corticosteroid benefit.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on gestational age at delivery. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 702

Priapism

Overview

Priapism is a persistent, often painful, penile erection lasting longer than 4 hours unrelated to sexual stimulation.

Causes

Causes include sickle cell disease (most common cause in children), intracavernosal injections, psychotropic medications, illicit drugs, anticoagulant therapy, and hematologic malignancies. Non-ischemic (high-flow, arterial) priapism results from unregulated arterial inflow, typically from perineal trauma causing arteriocavernosal fistula. This condition happens because of dysregulation of penile blood flow.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a rigid, tender penis with soft glans in ischemic type.

Emergency warning signs

- Severe or worsening symptoms of priapism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves history, physical examination, and cavernous blood gas analysis. Treatment for ischemic priapism follows a stepwise approach: corporal removal by suction with irrigation, intracavernosal phenylephrine, and surgical shunting. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Ischemic (low-flow, veno-occlusive) priapism accounts for >95% of cases and is a urologic emergency requiring prompt intervention to prevent corporal scarring and permanent erectile dysfunction.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on promptness of intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 703

Primary Amebic Meningoencephalitis (*Naegleria fowleri*)

Overview

Primary amebic meningoencephalitis is a rare but nearly always fatal CNS infection caused by *Naegleria fowleri*, a free-living amoeba found in warm freshwater environments, with fewer than 20 cases annually in the United States, primarily in children and young adults with a history of swimming in warm freshwater lakes.

Causes

This condition happens because of the trophozoite form invading the olfactory neuroepithelium, migrating along the olfactory nerve, and entering the subarachnoid space, producing a involving bleeding, necrotizing meningoencephalitis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of primary amebic meningoencephalitis (*naegleria fowleri*) require immediate medical evaluation

Progression and Stages

You may experience an incubation of 1–9 days, with fulminant onset of severe frontal headache, fever, nausea, vomiting, and meningismus, with rapid progression to altered mental status, seizures, coma, and death within 3–7 days. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using PCR of CSF. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include miltefosine plus fluconazole, azithromycin, and rifampin, with therapeutic hypothermia and Treatment for increased intracranial pressure. outlook: survival is extremely rare (fewer than 15 survivors documented worldwide). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and

self-care measures Physical therapy and rehabilitation Surgical intervention when indicated
Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 704

Primary Angle-Closure Glaucoma

Overview

- Primary angle-closure glaucoma is an eye emergency that occurs when the peripheral iris physically obstructs the trabecular meshwork, blocking aqueous humor removal of fluid and causing a rapid rise in intraocular pressure
- It is more common in hyperopic eyes with shallow anterior chambers.

Causes

This condition happens because of anatomical predisposition leading to iridotrabecular contact. Degenerative processes involve progressive cellular dysfunction and tissue breakdown over time. Contributing factors include oxidative stress, inflammation, accumulated cellular damage, and reduced regenerative capacity. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of primary angle-closure glaucoma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Symptoms of acute angle closure include severe eye pain, headache, nausea, blurred vision, halos around lights, a mid-dilated fixed pupil, and markedly elevated IOP
- Chronic angle closure may be asymptomatic until advanced.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on tonometry and gonioscopy. Treatment for acute angle closure includes immediate topical and systemic IOP-lowering agents followed by laser peripheral iridotomy
- Fellow eyes should undergo prophylactic iridotomy.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with prompt treatment. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 705

Primary Biliary Cholangitis

Overview

Primary biliary cholangitis (PBC, formerly primary biliary cirrhosis) is a chronic, progressive autoimmune liver disease characterized by granulomatous destruction of intrahepatic interlobular bile ducts, occurring predominantly in women aged 40–60.

Causes

This condition happens because of autoimmune-mediated destruction of biliary epithelial cells.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fatigue, severe generalized itching (often preceding jaundice), hyperpigmentation, and xanthomas.

Emergency warning signs

- Severe or worsening symptoms of primary biliary cholangitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Management with ursodeoxycholic acid (UDCA) slows disease progression
- Obeticholic acid is used in UDCA non-responders
- Liver transplantation is definitive for end-stage disease.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with elevated serum alkaline phosphatase, positive antimitochondrial antibodies (AMA, present in > 95% of patients), and liver biopsy showing nonsuppurative destructive cholangitis. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is significantly improved with UDCA treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 706

Primary Open-Angle Glaucoma

Overview

Primary open-angle glaucoma is the most common form of glaucoma and a leading cause of irreversible blindness worldwide, characterized by progressive optic nerve damage and visual field loss, typically associated with elevated intraocular pressure. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. This degenerative condition involves progressive deterioration of affected tissues, leading to gradual loss of function. It is more common with advancing age. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of increased resistance to aqueous humor outflow through the trabecular meshwork despite an open angle between the iris and cornea. Degenerative processes involve progressive cellular dysfunction and tissue breakdown over time. Contributing factors include oxidative stress, inflammation, accumulated cellular damage, and reduced regenerative capacity. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience progressive visual field loss, often beginning peripherally. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of primary open-angle glaucoma require immediate medical evaluation

Progression and Stages

The outlook varies from person to person, with treatment slowing but not reversing disease progression. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves tonometry, gonioscopy, optic disc evaluation (cup-to-disc ratio), automated perimetry, and optical coherence tomography. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system

- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include topical hypotensive medications (prostaglandin analogs, beta-blockers, alpha-agonists, carbonic anhydrase inhibitors), laser trabeculoplasty, and trabeculectomy or tube shunt surgery for refractory cases. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms Physical, occupational, and speech therapy for functional maintenance Surgical interventions when appropriate (deep brain stimulation, tumor resection) Cognitive rehabilitation and memory support Pain management for neuropathic pain Assistive devices and mobility aids Lifestyle modifications including exercise, nutrition, and sleep hygiene Support services and caregiver education Regular neurologic monitoring and treatment adjustment Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for

previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 707

Primary Sclerosing Cholangitis

Overview

Primary sclerosing cholangitis (PSC) is a chronic cholestatic liver disease characterized by inflammation, fibrotic stricturing, and obliteration of intrahepatic and extrahepatic bile ducts, strongly associated with ulcerative colitis (present in 70–80% of PSC patients).

Causes

This condition happens because of immune-mediated bile duct injury leading to concentric onion-skin scarring around bile ducts.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fatigue, itching, right upper quadrant pain, recurrent acute cholangitis, and progressive jaundice.

Emergency warning signs

- Severe or worsening symptoms of primary sclerosing cholangitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors confirm the diagnosis through magnetic resonance cholangiopancreatography (MRCP) or ERCP showing characteristic beaded appearance of bile ducts. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive

- Using a thin camera tube balloon dilation or stenting is used for dominant strictures
- Liver transplantation is the only curative option. PSC carries a 10–15% lifetime risk of cholangiocarcinoma.
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is guarded without liver transplantation. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 708

Propriospinal Myoclonus at Sleep Onset

Overview

Propriospinal myoclonus at sleep onset is a rare sleep-related movement disorder characterized by sudden, brief, involuntary jerks of the axial muscles occurring during the transition from wakefulness to sleep. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of myoclonus originating in the spinal cord and propagating rostrally and caudally via propriospinal pathways.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience jerks triggered by voluntary attempts to relax or fall asleep, severely delaying sleep onset and resulting in sleep-onset insomnia.

Emergency warning signs

- Severe or worsening symptoms of propriospinal myoclonus at sleep onset require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for polygraphic EMG recording across multiple spinal segments and polysomnography, with differentiation from hypnic jerks (physiologic sleep starts) and epileptic myoclonus. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves challenging
- Clonazepam, levetiracetam, and other anticonvulsants have been used with variable success.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Some patients experience spontaneous improvement, while others require long-term pharmacotherapy.

Chapter 709

Prostate Cancer

Overview

Prostate cancer is the second most commonly diagnosed cancer and the fifth leading cause of cancer-related death in men worldwide. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of prostatic epithelial cells. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)

- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Screening involves prostate-specific antigen (PSA) testing and digital rectal examination. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of prostate cancer require immediate medical evaluation

Progression and Stages

Clinical features are often asymptomatic in early stages, with advanced disease causing urinary symptoms, hematuria, and bone pain from metastases. Most people recover well for localized disease and variable for advanced disease. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive

local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using transrectal or transperineal MRI-guided biopsy, with Gleason score and ISUP grade group guiding risk stratification. Treatment for low-risk disease may be active surveillance

- Intermediate-risk disease doctors typically treat it with radical prostatectomy or radiation therapy
- High-risk and metastatic disease require multimodal therapy including androgen deprivation therapy (ADT), radiation, chemotherapy (docetaxel), and novel hormonal agents (abiraterone, enzalutamide).
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status

- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 710

Psoriasis

Overview

Psoriasis is a chronic, immune-mediated inflammatory skin disease characterized by well-demarcated, erythematous plaques with silvery-white scales, affecting 2–3% of the global population. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission.

Causes

This condition results from disruption of normal psoriasis function.

- This condition happens because of dysregulated Th1/Th17 immune responses leading to keratinocyte hyperproliferation (epidermal turnover time reduced from 28 days to 3–5 days). Plaque psoriasis (psoriasis vulgaris) is the most common form, presenting with symmetrically distributed plaques on extensor surfaces (elbows, knees), scalp, and lower back. Other forms include guttate (small, drop-shaped lesions, often post-streptococcal), inverse (flexural areas), pustular, and erythrodermic psoriasis. Nail changes (pitting, onycholysis, oil-drop discoloration) are common
- Up to 30% of patients develop psoriatic arthritis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of psoriasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Management follows a stepwise approach based on severity: topical therapies (corticosteroids, vitamin D analogues [calcipotriene], topical calcineurin inhibitors) for mild disease
- Phototherapy (narrowband UVB) for moderate disease
- And systemic therapy for moderate-to-severe disease: conventional agents (methotrexate, cyclosporine, acitretin), and biologics targeting specific cytokines (TNF inhibitors, IL-17 inhibitors, IL-23 inhibitors, IL-12/23 inhibitor [ustekinumab]).

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is chronic but can be well-controlled with appropriate therapy. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 711

Psoriatic Arthritis

Overview

Psoriatic arthritis (PsA) is discussed in Chapter ?? (Skin Diseases). Five clinical patterns include oligoarthritis, polyarthritis, arthritis mutilans, spondylitis, and dactylitis. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of psoriatic arthritis require immediate medical evaluation

Progression and Stages

It is an inflammatory arthropathy associated with psoriasis, classified as a seronegative spondyloarthropathy. Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Treatment options include NSAIDs, DMARDs (methotrexate), and biologics (TNF inhibitors, IL-17 inhibitors, IL-23 inhibitors, JAK inhibitors). Treatment aims to control inflammation, manage symptoms, and prevent disease flares. A step-up approach is often used, starting with symptomatic treatments and progressing to immunosuppressive therapies as needed. Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways

- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Autoimmune diseases are generally chronic conditions requiring long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 712

Psoriatic Arthritis

Overview

Psoriatic arthritis (PsA) is an inflammatory arthropathy associated with psoriasis, affecting 30% of patients with cutaneous psoriasis. It is a seronegative spondyloarthropathy (negative for rheumatoid factor and anti-CCP antibodies). This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of immune-mediated joint inflammation. Five clinical patterns exist: oligoarthritis (most common), polyarthritis, arthritis mutilans (severe, destructive), spondylitis, and dactylitis ("sausage digits"). Extra-articular manifestations include enthesitis (Achilles, plantar fascia), nail changes, eye inflammation, and cardiovascular disease. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection

- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of psoriatic arthritis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension

- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Doctors diagnose this based on the CASPAR criteria. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

outlook is improved with early diagnosis and treatment to prevent irreversible joint damage.

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Treatment options include NSAIDs, DMARDs (methotrexate, sulfasalazine), and biologics (TNF inhibitors, IL-17 inhibitors, IL-23 inhibitors, JAK inhibitors, PDE4 inhibitors). Treatment aims to control inflammation, manage symptoms, and prevent disease flares. A step-up approach is often used, starting with symptomatic treatments and progressing to

immunosuppressive therapies as needed. Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Autoimmune diseases are generally chronic conditions requiring long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 713

Psychophysilogic Insomnia

Overview

Psychophysilogic insomnia is a subtype of chronic insomnia characterized by a learned, conditioned arousal to sleep-related stimuli, such that the bed and bedroom environment become associated with frustration, worry, and heightened alertness rather than sleep. Patients often sleep better away from home (paradoxical improvement in novel environments). This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of physiologic hyperarousal with elevated metabolic rate, heart rate, and cortisol at sleep onset, combined with thinking and memory patterns including excessive preoccupation with sleep, catastrophic thinking about sleep loss, and performance anxiety. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience difficulty sleeping in one's own bed but improved sleep in novel environments.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of psychophysiologic insomnia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on stimulus control therapy (reassociating the bed with sleep), sleep restriction therapy (consolidating sleep window to improve sleep efficiency), and thinking and memory restructuring to reduce maladaptive beliefs about sleep. Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate behavioral therapy. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 714

Pterygium

Overview

Pterygium is a fibrovascular overgrowth of the bulbar conjunctiva that extends onto the cornea, typically originating from the nasal (medial) side. It is a common ocular surface condition associated with chronic ultraviolet light exposure, with higher prevalence in sunny climates and outdoor workers. While often benign, pterygium can cause visual disturbance when it approaches the visual axis.

Classification

Cause Environmental / Degenerative

Organ System Eyes

Pathophysiology UV-induced elastotic degeneration and fibrovascular proliferation of the conjunctiva

Duration Chronic (months to years)

Onset Gradual

Mode of Transmission Non-communicable

Clinical Course Chronic, slowly progressive

Severity Mild to Moderate

Extent of Disease Localized

Age Group Adults (20–50 years)

Epidemiology Prevalence 10–20% in equatorial regions; higher in men

ICD-11 Code 9A62.0

Causes

Chronic exposure to ultraviolet B radiation from sunlight is the primary causative factor, damaging limbal stem cells and triggering abnormal conjunctival proliferation. Genetic predisposition, chronic dry eye, and exposure to irritants like dust and wind are contributing factors. The condition is most prevalent in latitudes near the equator.

Risk Factors

- Chronic sunlight exposure (outdoor occupations, residence near equator)
- Lack of eye protection (sunglasses, hats)
- Male sex
- Older age
- Chronic dry eye
- Exposure to dust, wind, and irritants
- Family history

Signs and Symptoms

Early symptoms

- Early manifestations of pterygium may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
 - Fleshy, triangular growth on the conjunctiva extending onto the cornea
 - Typically originates from the nasal side (bilateral in 10% of cases)
 - Redness, irritation, and foreign body sensation
 - Tearing and dry eye symptoms
 - Blurred vision when growth approaches the visual axis
 - Astigmatism induced by corneal traction
 - Pain or discomfort in the affected area
 - Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of pterygium may occur

Emergency warning signs

- Severe or worsening symptoms of pterygium require immediate medical evaluation

Progression and Stages

Pterygium progresses slowly over years to decades. Early lesions remain confined to the conjunctiva. As the fibrovascular tissue advances across the cornea, it can induce irregular astigmatism. Advanced pterygium may extend to or beyond the pupillary margin, significantly impairing vision. Recurrence after surgical excision is common

without adjunctive therapy.

Complications

- Visual impairment from corneal involvement or induced astigmatism
- Restriction of eye movement in advanced cases
- Recurrence after surgical removal (up to 40–50% without adjunctive treatment)
- Corneal scarring and irregularity
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis by slit-lamp examination
- Measurement of horizontal extent onto cornea
- Assessment for induced astigmatism by keratometry or topography
- Photodocumentation for progression monitoring
- Lab tests to check for underlying conditions

Prevention

- Wear UV-blocking sunglasses and wide-brimmed hats outdoors
- Use artificial tears in dry or dusty environments
- Avoid chronic sunlight exposure during peak hours
- Regular ophthalmic examination for outdoor workers
- Go for regular check-ups so any problems are caught early

Treatment Options

- Observation for small, asymptomatic pterygia
- Lubricating eye drops for irritation and dry eye
- Topical corticosteroids for active inflammation
- Surgical excision with conjunctival autograft for vision-threatening pterygia
- Adjunctive mitomycin C or beta radiation to reduce recurrence
- Lifestyle changes and self-care
- Surgery when needed

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed (eye protection outdoors)
- Reach out to family, friends, or support groups for help

- Keep learning about your condition and how to manage it

Outlook

Pterygium is a benign condition with an excellent prognosis for vision when managed appropriately. Small lesions can be observed indefinitely. Surgical removal is curative but carries a risk of recurrence, which is significantly reduced with conjunctival autograft techniques. Long-term sun protection is essential to prevent progression and recurrence.

Chapter 715

Pulmonary Embolism

Overview

lung blockage by a blood clot is a potentially life-threatening condition caused by obstruction of the lung arteries, most commonly by deep vein blood clot formation of the lower extremities. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

This condition happens because of thrombotic occlusion of lung arteries. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension and diabetes
- Elevated cholesterol levels
- Family history of heart disease
- Obesity and physical inactivity
- Hypertension (high blood pressure)
- Diabetes mellitus

- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Symptoms can range from shortness of breath and pleuritic chest pain to hemoptysis, syncope, and hemodynamic instability (massive PE). Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of pulmonary embolism require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

- The right treatment depends on severity: anticoagulation for hemodynamically stable disease
- Systemic thrombolysis, catheter-directed therapy, or surgical embolectomy for massive disease with hemodynamic instability
- Inferior vena cava filters are placed when anticoagulation is contraindicated.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Diagnosis typically involves clinical probability assessment (Wells score), D-dimer, and CT lung angiography. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Most people recover well with prompt diagnosis and appropriate treatment. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 716

Pulmonary Hypertension

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Lung high blood pressure is defined as a mean lung artery pressure of 20 mmHg or greater at rest, measured by right heart catheterization
- The 2022 classification groups lung high blood pressure into five categories: Group 1 (lung arterial high blood pressure), Group 2 (left heart disease), Group 3 (lung disease or hypoxia), Group 4 (chronic thromboembolic disease), and Group 5 (unclear or multifactorial).

Causes

This condition happens because of vascular remodeling with intimal scarring, medial enlargement, and plexiform lesions. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease

- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience progressive shortness of breath, fatigue, syncope, and right heart failure. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of pulmonary hypertension require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia

- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

- Diagnosis typically involves echocardiography, right heart catheterization, and evaluation of the underlying cause. Treatment for lung arterial high blood pressure includes phosphodiesterase-5 inhibitors, endothelin receptor antagonists, prostacyclin analogs, and soluble guanylate cyclase stimulators
- Group 4 disease doctors typically treat it with lung endarterectomy or balloon lung angioplasty.
- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)

- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Your outlook depends on the underlying group and response to therapy. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 717

Pyelonephritis

Overview

Pyelonephritis is an infection of the kidney parenchyma and kidney pelvis, representing an upper urinary tract infection. Acute pyelonephritis typically results from ascending infection from the lower urinary tract, most commonly caused by *E. coli*.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience high fever, chills, flank pain, nausea, and vomiting
- CVA tenderness is present on examination.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of pyelonephritis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on urinalysis (pyuria, bacteriuria, hematuria) and urine culture

- Blood cultures are indicated for severe or complicated cases. Treatment for mild cases is outpatient oral fluoroquinolones or initial IV ceftriaxone followed by oral step-down therapy
- Severe cases require hospitalization and IV antibiotics.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with appropriate antibiotic therapy
- Chronic pyelonephritis results from recurrent infections and may lead to kidney scarring and CKD.

Chapter 718

REM Parasomnias: Nightmare Disorder

Overview

Nightmare disorder is a parasomnia involving recurrent, vividly realistic, dysphoric dreams that awaken the individual from REM sleep, typically during the second half of the night, with rapid return to full alertness and clear recall of the frightening content. Prevalence is 2–8% in adults and 10–50% in children.

Causes

This condition happens because of abnormal processing of emotional memories during REM sleep and is linked to PTSD, anxiety, borderline personality disorder, and certain medications (beta-blockers, SSRIs).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience recurrent nightmares causing sleep avoidance and daytime fatigue.

Emergency warning signs

- Severe or worsening symptoms of rem parasomnias: nightmare disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include imagery rehearsal therapy (IRT), prazosin (an alpha-1 adrenergic antagonist) for PTSD-related nightmares, and treatment of comorbid psychiatric disorders. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 719

REM Parasomnias: REM Sleep Behavior Disorder

Overview

REM sleep behavior disorder (RBD) is a parasomnia characterized by loss of the normal muscle atonia of REM sleep, allowing patients to act out their dreams, often with violent or injurious behaviors. Prevalence is 1–2% in the general population, with a strong male predominance (>80% of cases), typically occurring after age 50. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of dysfunction of the brainstem REM-atonia circuits (sublaterodorsal nucleus in the pons). Critically, idiopathic RBD is a highly specific prodromal marker for synucleinopathies: 80–90% of patients will eventually develop Parkinson disease, dementia with Lewy bodies, or multiple system shrinkage or wasting, with a mean latency of 10–15 years.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience dream-enactment behaviors (punching, kicking, jumping out of bed) corresponding to aggressive dream content.

Emergency warning signs

- Severe or worsening symptoms of rem parasomnias: rem sleep behavior disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with polysomnography demonstrating elevated chin EMG tone during REM. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include safety measures, clonazepam (first-line, 0.5–2 mg nightly), and melatonin (3–15 mg) as an effective alternative. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is chronic
- The condition is manageable but carries a high risk of eventual neurodegenerative disease.

Chapter 720

REM Parasomnias: Recurrent Isolated Sleep Paralysis

Overview

Recurrent isolated sleep paralysis is a parasomnia consisting of episodes of inability to perform voluntary movements at sleep onset (hypnagogic) or upon awakening (hypnopompic), with preserved consciousness and awareness of the environment. Prevalence is 8–50% depending on population, higher in those with anxiety, panic disorder, and sleep deprivation. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of a dissociation between REM sleep atonia and conscious awareness.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience episodes lasting seconds to minutes, often accompanied by terrifying hallucinations (intruder, pressure on the chest), and the ability to terminate episodes by external stimulation or strong voluntary effort to move a small muscle group.

Emergency warning signs

- Severe or worsening symptoms of rem parasomnias: recurrent isolated sleep paralysis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include reassurance and sleep hygiene. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is non-cancerous. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 721

Rabies

Overview

Rabies is a fatal viral encephalitis caused by rabies virus, a single-stranded RNA virus of the genus *Lyssavirus*, with 60,000 deaths annually worldwide, primarily in Asia and Africa, with dogs being the main reservoir and source of transmission (99% of human cases), and rabies being nearly 100% fatal after symptom onset. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of the virus being inoculated through saliva of an infected animal, binding to nicotinic acetylcholine receptors at the neuromuscular junction, traveling by retrograde axonal transport to the CNS, and disseminating throughout the central nervous system. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions

- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of rabies require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- You may experience an incubation of 20–90 days, followed by a prodrome (fever, malaise, tingling or numbness at the bite site), then acute nervous system phase: furious rabies (80%)—hydrophobia, aerophobia, agitated behavior, autonomic instability, hypersalivation, and seizures
- Or paralytic rabies (20%)—ascending flaccid paralysis.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, confirmed by RT-PCR of saliva, CSF, or skin biopsy. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- Treatment typically involves supportive once symptoms develop
- Prevention through pre-exposure prophylaxis and post-exposure prophylaxis (immediate wound cleansing, HRIG, and rabies vaccine) is nearly 100% effective if administered before symptom onset.
- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings

- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 722

Radiation Sickness (Acute Radiation Syndrome)

Overview

Acute radiation syndrome (ARS) results from high-dose whole-body or significant partial-body ionizing radiation exposure (>1 Gy). This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of radiation causing DNA damage (double-strand breaks), cell death in rapidly dividing tissues (bone marrow, GI tract, skin), and free-radical generation. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of radiation sickness (acute radiation syndrome) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Clinical phases: prodromal phase (0–48 hours): nausea, vomiting, diarrhea, anorexia
- Latent phase: transient symptom improvement
- Manifest illness phase depends on dose: hematopoietic syndrome (1–6 Gy): pancytopenia, infection, bleeding, anemia
- Digestive tract syndrome (6–10 Gy): severe diarrhea, fluid loss, mucosal tissue death, sepsis
- Cerebrovascular/cardiovascular syndrome (>10–20 Gy): hypotension, cerebral swelling, convulsions, coma, death.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on history of radiation exposure, absolute lymphocyte count decline, and cytogenetic analysis.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment: supportive care (reverse isolation, antimicrobial prophylaxis, blood product support), cytokines (G-CSF, GM-CSF) for neutropenia, and treatment of internal contamination.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Unfortunately, the outlook is often poor for doses >6 Gy.

Chapter 723

Reactive Arthritis

Overview

Reactive arthritis is an aseptic inflammatory arthritis occurring 1–4 weeks after a genitourinary (usually *Chlamydia trachomatis*) or digestive tract (usually *Salmonella*, *Shigella*, *Yersinia*, *Campylobacter*) infection. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission.

Causes

This condition results from disruption of normal reactive arthritis function.

- This condition happens because of an immune response to the initial infection triggering joint inflammation. The classic triad is arthritis, urethritis, and conjunctivitis ("can't see, can't pee, can't climb a tree"). Arthritis is typically asymmetric and oligoarticular, affecting large joints of the lower extremities
- Enthesitis, dactylitis, and oral ulcers are common. Most cases resolve within 3–12 months.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of reactive arthritis require immediate medical evaluation

Progression and Stages

It is strongly associated with HLA-B27 and classified as a seronegative spondyloarthropathy. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with history of preceding infection. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include NSAIDs, intra-articular corticosteroids, and for chronic or severe disease, DMARDs (sulfasalazine) or TNF inhibitors
- Antibiotics are not helpful once arthritis has developed.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is good
- Most cases resolve within 3–12 months.

Chapter 724

Recurrent Pregnancy Loss

Overview

Recurrent pregnancy loss (RPL) is traditionally defined as two or more consecutive pregnancy losses before 20 weeks, affecting 1–2% of couples. Evaluation is recommended after two losses and includes parental karyotyping, uterine evaluation, antiphospholipid antibody testing, and thyroid function tests. The cause remains unexplained in 50%. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of recurrent pregnancy loss require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Management addresses identified causes: surgical correction of uterine septum, anticoagulation for antiphospholipid syndrome, and thyroid hormone replacement for hypothyroidism
- For unexplained RPL, progesterone supplementation and supportive care are associated with improved outcomes.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 725

Relapsing Polychondritis

Overview

Relapsing polychondritis is a rare, immune-mediated disease characterized by recurrent, progressive inflammation and destruction of cartilaginous structures, particularly in the ears, nose, larynx, tracheobronchial tree, and joints, with an incidence of approximately 0.5–3.5 per million, equal sex distribution, and typically presenting in the fifth to sixth decade.

Causes

This condition happens because of an autoimmune attack on type II, IX, and XI collagen and other cartilage matrix components, mediated by autoantibodies, T-cell infiltration, and complement activation, with proteoglycan degradation by matrix metalloproteinases contributing to cartilage destruction.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience auricular chondritis (the most common presenting feature: painful, swollen, erythematous ears sparing the lobule), nasal chondritis (saddle nose deformity), laryngotracheal involvement (subglottic stenosis, hoarseness, stridor, airway collapse—the most life-threatening manifestation), costochondritis, and non-erosive seronegative arthritis, with ocular involvement (episcleritis, scleritis, uveitis) occurring in up to 60%, audiovestibular involvement (conductive and sensorineural hearing loss, feeling of spinning) being common, and association with other autoimmune diseases (SLE, vasculitis, myelodysplastic syndrome) in 30%.

Emergency warning signs

- Severe or worsening symptoms of relapsing polychondritis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, based on McAdam's criteria or Damiani's criteria, with laboratory findings being non-specific. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include corticosteroids (rapid symptomatic relief), dapsone (mild cases), and immunosuppressants (methotrexate, mycophenolate, cyclophosphamide) for severe disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with respiratory involvement being the leading cause of death. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 726

Renal Osteodystrophy

Overview

Causes

This condition happens because of phosphate retention, decreased kidney production of active vitamin D (calcitriol), hypocalcemia, and secondary hyperparathyroidism. Bone histology subtypes include osteitis fibrosa cystica (high-turnover, from severe hyperparathyroidism), adynamic bone disease (low-turnover, from oversuppression of PTH with calcitriol and calcium-based binders, common in diabetics and the elderly), osteomalacia (defective mineralization, from vitamin D deficiency or aluminum toxicity), and mixed uremic osteodystrophy.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bone pain (most commonly in lower back, hips, and legs), proximal muscle weakness, fractures, skeletal deformities, growth retardation in children, and extraosseous calcium buildup (vascular calcium buildup, calciphylaxis).

Emergency warning signs

- Severe or worsening symptoms of renal osteodystrophy require immediate medical evaluation

Progression and Stages

kidney osteodystrophy is a spectrum of bone disorders that develop in patients with chronic kidney disease (CKD), resulting from disturbances in mineral metabolism, affecting virtually all patients with CKD stage 5. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on elevated PTH, hyperphosphatemia, hypocalcemia or hypercalcemia, low vitamin D levels, and bone biopsy (gold standard but rarely performed). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on phosphate restriction (dietary), phosphate binders (calcium-based, sevelamer, lanthanum carbonate, iron-based binders), active vitamin D analogs (calcitriol, paricalcitol, doxercalciferol), calcimimetics (cinacalcet, etelcalcetide), and optimization

of dialysis. Parathyroidectomy for refractory hyperparathyroidism. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is improved by early recognition and treatment
- Kidney osteodystrophy improves after successful kidney transplantation.

Chapter 727

Renal Tubular Acidosis

Overview

kidney tubular acidosis (RTA) is a group of disorders in which the kidneys fail to appropriately acidify the urine despite normal or near-normal GFR.

Causes

This condition results from disruption of normal renal tubular acidosis function.

- Type 2 (proximal) RTA involves impaired bicarbonate reabsorption in the proximal tubule, causing metabolic acidosis with appropriately acidic urine and bicarbonaturia
- It is often part of Fanconi syndrome. Type 4 RTA is caused by aldosterone deficiency or resistance, leading to hyperkalemic metabolic acidosis. This condition happens because of the specific tubular transport defect.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms vary depending on type and include metabolic acidosis, growth retardation, and nephrocalcinosis.

Emergency warning signs

- Severe or worsening symptoms of renal tubular acidosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Type 1 (distal) RTA results from impaired hydrogen ion secretion in the collecting duct, causing hyperchloremic metabolic acidosis with inappropriately alkaline urine (pH >5.5)
- Complications include nephrocalcinosis and rickets.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on serum electrolytes, urine pH, and acid-base testing. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include oral bicarbonate supplementation (Types 1 and 2) and Treatment for hyperkalemia and aldosterone deficiency (Type 4). Medications to manage

symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate supplementation. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 728

Respiratory Distress Syndrome (RDS)

Overview

Respiratory distress syndrome (RDS), also known as hyaline membrane disease, is a lung disorder of prematurity caused by surfactant deficiency in preterm infants. Incidence is inversely proportional to gestational age: approximately 50% of infants born at 28–30 weeks and nearly all infants born before 26 weeks develop RDS. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of deficiency of surfactant, a phospholipoprotein complex produced by type II pneumocytes that reduces alveolar surface tension. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience tachypnea, grunting, nasal flaring, intercostal retractions, and cyanosis within minutes to hours after birth.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of respiratory distress syndrome (rds) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with chest radiograph showing a characteristic ground-glass appearance with air bronchograms. Treatment consists of intratracheal surfactant replacement therapy (beractant, poractant alfa) and respiratory support (nasal continuous positive airway pressure or mechanical ventilation).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Prevention includes antenatal corticosteroids.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has dramatically improved with surfactant replacement. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 729

Restless Legs Syndrome

Overview

Restless legs syndrome (RLS, Willis-Ekbom disease) is a sensorimotor disorder characterized by an irresistible urge to move the legs, usually accompanied by uncomfortable sensations (creeping, crawling, pulling, aching, or tingling), that begins or worsens during periods of rest or inactivity, is partially or completely relieved by movement, occurs predominantly in the evening or night, and is not better explained by another condition. Prevalence is 5–10% in adults of European descent, 2–4% in Asian populations, and increases with age, affecting women 2:1 over men. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of brain iron deficiency (reduced iron content in substantia nigra and striatal dopamine receptors) and dopaminergic dysfunction, with genetic associations (BTBD9, MEIS1, MAP2K5). Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of restless legs syndrome require immediate medical evaluation

Progression and Stages

Secondary RLS is caused by iron deficiency anemia, pregnancy, end-stage kidney disease, peripheral neuropathy, and medications (SSRIs, antihistamines, antipsychotics). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Outlook is chronic but manageable
- Augmentation is the most significant complication of dopaminergic therapy.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam using the five essential IRLSSG diagnostic criteria
- Serum ferritin levels should be checked (goal $>75 \mu\text{g/L}$).
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include non-pharmacologic measures (moderate exercise, avoidance of alcohol and caffeine in the evening), correction of iron deficiency, first-line pharmacotherapy with alpha-2-delta calcium channel ligands (gabapentin, pregabalin—preferred due to lower augmentation risk), and dopamine agonists (pramipexole, ropinirole, rotigotine) for refractory cases. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 730

Restrictive Cardiomyopathy

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Restrictive cardiomyopathy is a condition characterized by impaired ventricular filling due to increased myocardial stiffness with preserved or near-preserved systolic function
- Causes include amyloidosis (AL and ATTR types), sarcoidosis, hemochromatosis, endomyocardial scarring, and infiltrative diseases.

Causes

This condition happens because of myocardial infiltration or scarring restricting ventricular filling. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)

- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience predominant right-sided heart failure (swelling, ascites, hepatomegaly, jugular venous distension). Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of restrictive cardiomyopathy require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this based on echocardiography showing small to normal-sized ventricles with biatrial enlargement and restrictive filling pattern

- Cardiac MRI demonstrates late gadolinium enhancement
- Endomyocardial biopsy confirms diagnosis. Management targets the underlying cause (tafamidis for ATTR amyloidosis, chemotherapy for AL amyloidosis) with diuretics for symptom management.
- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk

- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

Recovery varies by cause. With proper management and lifestyle modifications, many patients maintain good quality of life. Long-term outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 731

Retinal Detachment

Overview

- Retinal detachment is a separation of the neurosensory retina from the underlying retinal pigment epithelium, constituting an eye emergency
- Rhegmatogenous retinal detachment, the most common type, results from a retinal break allowing vitreous fluid to enter the subretinal space.

Causes

This condition results from disruption of normal retinal detachment function.

- Tractional retinal detachment occurs in proliferative diabetic retinopathy
- Exudative retinal detachment results from fluid accumulation without a break.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sudden onset of floaters, flashing lights, and a curtain-like visual field defect.

Emergency warning signs

- Severe or worsening symptoms of retinal detachment require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on dilated fundus examination. Treatment for rhegmatogenous retinal detachment requires surgical repair with pneumatic retinopexy, scleral buckling, or vitrectomy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the type, location, and timeliness of repair. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 732

Retinal Vein Occlusion

Overview

- Retinal vein occlusion (RVO) is a blockage of the veins that drain blood from the retina, causing hemorrhage and fluid leakage
- It is the second most common retinal vascular disease after diabetic retinopathy
- It typically affects people over 50 and is a major cause of vision loss from macular edema and neovascular complications

Causes

This condition happens because of thrombosis (blood clot) in a retinal vein, most often where the vein crosses a retinal artery (arteriovenous crossing). It is classified as central retinal vein occlusion (CRVO) when the main vein is blocked, or branch retinal vein occlusion (BRVO) when a branch is affected. Contributing factors include hypertension, atherosclerosis, glaucoma, and thrombophilia.

Risk Factors

- Age over 50 years
- Hypertension and high cholesterol
- Diabetes
- Glaucoma (especially for CRVO)
- Smoking
- Blood clotting disorders (thrombophilia)
- Oral contraceptives and other prothrombotic conditions

Signs and Symptoms

Common symptoms

- Sudden, painless blurring or loss of vision in one eye
- Vision loss may be mild and peripheral (BRVO) or severe (CRVO)
- Distorted or wavy vision from macular edema
- Visual field defects in the affected area

Emergency warning signs

- Sudden loss of vision in one eye requires urgent eye examination
- New pain, redness, or high eye pressure may indicate neovascular glaucoma

Progression and Stages

Initial symptoms usually develop over hours to days. Two patterns follow: the ischemic (non-perfused) type, which has a poor visual prognosis and high risk of neovascular complications, and the non-ischemic type, which often improves. Macular edema may persist for months to years. Neovascular glaucoma can develop in severe CRVO weeks to months after onset and threatens the eye.

Complications

- Chronic macular edema with permanent central vision loss
- Retinal and iris neovascularization
- Neovascular glaucoma (serious, painful, blinding)
- Vitreous hemorrhage from new vessels
- Retinal detachment in severe cases
- Underlying vascular disease progression

Diagnosis

- Dilated fundus examination showing retinal hemorrhages, venous engorgement, and "blood and thunder" appearance in CRVO
- Optical coherence tomography (OCT) to assess macular edema
- Fluorescein angiography to determine ischemic status
- Measurement of intraocular pressure to detect neovascular glaucoma
- Blood pressure and cardiovascular risk assessment
- Blood tests for clotting disorders in younger patients

Prevention

- Control of blood pressure, cholesterol, and diabetes
- Smoking cessation
- Regular eye examinations, especially for those with risk factors
- Treatment of glaucoma and cardiovascular risk factors
- Early evaluation of any sudden monocular visual symptoms

Treatment Options

- Intravitreal anti-VEGF injections (ranibizumab, aflibercept, bevacizumab) for macular edema
- Intravitreal corticosteroids (dexamethasone implant) for edema
- Grid or scatter laser photocoagulation for BRVO and neovascularization
- Panretinal photocoagulation for neovascularization in CRVO
- Treatment of neovascular glaucoma with pressure-lowering therapy
- Management of systemic cardiovascular risk factors

Living with It

- Attend regular injection and monitoring visits
- Control blood pressure, cholesterol, and diabetes
- Report new pain, redness, or worsening vision promptly
- Use low-vision aids if central vision remains impaired

Outlook

The prognosis is variable. BRVO carries a better outlook, with many eyes recovering spontaneously or with treatment. CRVO has a worse prognosis, especially the ischemic type. With anti-VEGF therapy, many patients with macular edema gain vision. Untreated neovascular glaucoma can cause painful blindness.

Chapter 733

Retinitis Pigmentosa

Overview

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience night blindness and progressive peripheral visual field constriction (tunnel vision), eventually affecting central vision
- Fundoscopy reveals bone-spicule pigmentation, waxy pallor of the optic disc, and attenuated retinal vessels.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of retinitis pigmentosa require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Retinitis pigmentosa is a group of inherited retinal dystrophies characterized by progressive deterioration of photoreceptors, primarily rod cells

- Most cases are inherited (autosomal dominant, autosomal recessive, or X-linked), with over 80 genes identified. This condition happens because of genetic mutations causing photoreceptor deterioration. Treatment typically involves supportive
- Vitamin A supplementation may slow progression in some forms
- Retinal prostheses and gene therapy (voretigene neparvovec for RPE65-mutated RP) offer emerging therapeutic avenues.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on fundus examination and electroretinography showing diminished or absent responses. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is progressive with no cure currently available. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 734

Retinoblastoma

Overview

This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

- Retinoblastoma is the most common intraocular malignancy in children, with an incidence of 1 in 15,000–20,000 live births. It arises from primitive retinal cells and is caused by biallelic inactivation of the RB1 tumor suppressor gene. Retinoblastoma occurs in heritable (40%) and non-heritable (60%) forms
- Heritable retinoblastoma involves a germline RB1 mutation with younger age at presentation, bilateral involvement, and increased risk of second malignancies.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)

- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience leukocoria (white pupillary reflex, the most common presenting sign), strabismus, decreased vision, and orbital inflammation. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of retinoblastoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using ophthalmologic examination under anesthesia. Treatment depends on the extent of disease: intra-arterial chemotherapy, laser photocoagulation, freezing treatment, brachytherapy, systemic chemotherapy, or enucleation. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and

depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

- Most people recover well
- Survival exceeds 95% in developed countries.

Chapter 735

Retinopathy of Prematurity

Overview

Retinopathy of prematurity is a vasoproliferative retinal disease affecting premature infants, particularly those born before 32 weeks gestation with low birth weight.

Causes

This condition happens because of abnormal vascularization of the developing retina, which can lead to tractional retinal detachment and blindness.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of retinopathy of prematurity require immediate medical evaluation

Progression and Stages

The disease is classified by zone, stage, and extent of vascular involvement, with plus disease indicating vascular activity. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be

overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on screening examinations in at-risk premature infants. Treatment for severe ROP involves anti-VEGF intravitreal injections or laser photocoagulation of the avascular retina. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- The outlook is good with timely screening and treatment
- Long-term follow-up is necessary due to risks of late sequelae including myopia, strabismus, and retinal detachment.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 736

Retinopathy of Prematurity (ROP)

Overview

Retinopathy of prematurity (ROP) is a vasoproliferative disorder of the developing retina in preterm infants (see Chapter 23, Diseases of the Eye for comprehensive coverage).

Causes

Screening by indirect ophthalmoscopy is performed for at-risk preterm infants.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of retinopathy of prematurity (rop) require immediate medical evaluation

Progression and Stages

This condition happens because of disruption of normal retinal vascularization, with an initial phase of vessel loss followed by pathologic neovascularization. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable

symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include laser photocoagulation and intravitreal bevacizumab for severe disease (Type 1 ROP). Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 737

Rh Incompatibility / Hemolytic Disease of the Newborn

Overview

Rh incompatibility occurs when an Rh-negative mother carries an Rh-positive fetus, leading to maternal sensitization and production of anti-Rh(D) IgG antibodies that cross the placenta and cause hemolytic disease of the fetus and newborn (HDFN). Sensitization occurs most commonly at delivery but also with miscarriage, ectopic pregnancy, invasive prenatal procedures, and trauma.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms can range from mild hemolytic anemia to severe hydrops fetalis. Diagnosis includes positive direct antiglobulin test on cord blood, elevated bilirubin, and anemia.

Emergency warning signs

- Severe or worsening symptoms of rh incompatibility / hemolytic disease of the newborn require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Most people recover well with prophylaxis
- Prevention has reduced HDFN incidence by 90%.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves prophylactic: Rh-immune globulin (RhoGAM, 300 μg) is administered at 28 weeks and within 72 hours of delivery of an Rh-positive infant
- Affected fetuses with severe anemia are treated with intrauterine transfusion.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 738

Rhabdomyolysis

Overview

Rhabdomyolysis is a syndrome of acute skeletal muscle tissue death with release of intracellular muscle contents (myoglobin, CK, potassium, phosphate, uric acid) into the circulation.

Causes

Causes include crush injuries, prolonged immobilization, extreme exertion, seizures, cancerous hyperthermia, medications (statins, fibrates, antipsychotics), electrolyte imbalances, infections, and inflammatory myopathies.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of rhabdomyolysis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Your outlook depends on severity and promptness of treatment
- Most patients recover with adequate hydration.

Complications

- You may experience muscle pain, muscle weakness, dark (cola-colored) urine, and swelling of affected muscles
- Complications include acute kidney injury (myoglobin nephrotoxicity), hyperkalemia (irregular heartbeats), disseminated intravascular coagulation, and compartment syndrome.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on elevated CK (typically >5 times the upper limit of normal, often >10,000 IU/L), elevated myoglobin, and elevated transaminases. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment focuses on aggressive IV fluid resuscitation (target urine output 200–300 mL/h), urinary alkalization (to prevent myoglobin precipitation), correction of electrolyte abnormalities, and monitoring for complications
- Kidney replacement therapy may be necessary for severe AKI.
- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 739

Rhabdomyosarcoma

Overview

Rhabdomyosarcoma (RMS) is the most common soft tissue sarcoma in children, with an incidence of 4.5 per million children under age 20. It arises from primitive mesenchymal cells with skeletal muscle differentiation. Two major histologic subtypes: embryonal RMS (60%, more favorable outlook) and alveolar RMS (20%, more aggressive). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity

- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Your symptoms depend on the site: a rapidly growing mass, proptosis (orbital), nasal obstruction (parameningeal), or scrotal or vaginal mass. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of rhabdomyosarcoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with

significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using biopsy with immunohistochemistry. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology

- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment typically involves multimodal: chemotherapy, surgical removal, and radiation therapy. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Neurological treatment focuses on symptom management, disease modification when possible, and rehabilitation to maintain function. A multidisciplinary team approach is often beneficial. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines

- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

outlook is 70–80% survival for low-risk and intermediate-risk, but <50% for high-risk disease. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 740

Rheumatic Heart Disease

Overview

Rheumatic heart disease (RHD) is the chronic valvular damage resulting from acute rheumatic fever, an autoimmune response to group A streptococcal pharyngitis. This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process.

- This condition happens because of molecular mimicry between streptococcal M protein and human cardiac tissue, leading to pancarditis and chronic fibrotic scarring of heart valves
- The mitral valve is affected in over 90% of cases, leading to mitral stenosis or regurgitation.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)

- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience exertional shortness of breath, fatigue, palpitations, and signs of heart failure or stroke secondary to atrial fibrillation. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of rheumatic heart disease require immediate medical evaluation

Progression and Stages

Your outlook depends on valve damage severity. Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia

- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this based on echocardiography showing valvular thickening, commissural fusion, and regurgitation/stenosis. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

Treatment options include secondary antibiotic prophylaxis (IM benzathine penicillin G every 3–4 weeks) to prevent recurrent streptococcal infections, medical Treatment for heart failure, and surgical valve repair or replacement.

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Anti-hypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel)

Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 741

Rheumatoid Arthritis

Overview

Rheumatoid arthritis (RA) is a chronic, systemic autoimmune disease characterized by symmetric inflammatory synovitis, predominantly affecting the small joints of the hands (metacarpophalangeal and proximal interphalangeal joints) and feet. It affects approximately 0.5–1% of the population, with a 3:1 female predominance. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of autoantibody production (rheumatoid factor, anti-citrullinated peptide antibodies [anti-CCP]), immune complex deposition, and synovial inflammation leading to pannus formation and progressive joint destruction. Extra-articular manifestations include rheumatoid nodules, interstitial lung disease, vasculitis, scleritis, pericarditis, and accelerated atherosclerosis. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes

- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of rheumatoid arthritis require immediate medical evaluation

Progression and Stages

The 2010 ACR/EULAR classification criteria aid early diagnosis. Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis

- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

- Management follows a treat-to-target strategy with early initiation of DMARDs: methotrexate (first-line anchor drug), leflunomide, sulfasalazine, and hydroxychloroquine
- Biologics (TNF inhibitors, IL-6 inhibitors [tocilizumab, sarilumab], T-cell co-stimulation blocker [abatacept], anti-CD20 [rituximab]) and targeted synthetic DMARDs (JAK inhibitors [tofacitinib, baricitinib]) are used for inadequate response.
- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- To make the diagnosis, doctors look for clinical assessment, serological testing (RF, anti-CCP), and imaging (X-rays showing periarticular osteopenia, joint space narrowing, and erosions)
- Ultrasound or MRI for early detection of synovitis).
- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
Corticosteroids for rapid control of inflammation
Disease-modifying antirheumatic drugs (DMARDs)
Biologic therapies targeting specific immune pathways
Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
Antimalarial drugs (hydroxychloroquine) for mild disease
Physical and occupational therapy to maintain function
Pain management for chronic pain
Lifestyle modifications including diet and stress reduction
Regular monitoring

for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

The outlook varies from person to person but has improved significantly with early aggressive treatment. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 742

Rheumatoid Arthritis

Overview

Rheumatoid arthritis is discussed in detail in Chapter ???. It is a systemic autoimmune disease primarily affecting joints but with significant extrarticular manifestations involving the immune system. Autoantibodies (rheumatoid factor, anti-CCP) are central to diagnosis and outlook. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of rheumatoid arthritis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain

- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Autoimmune diseases are generally chronic conditions requiring long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 743

Rickets and Osteomalacia

Overview

Rickets (in children) and osteomalacia (in adults) are disorders of impaired bone mineralization due to vitamin D deficiency or abnormalities in vitamin D metabolism.

Causes

This condition happens because of inadequate vitamin D (produced in the skin upon UV exposure and obtained from diet), which is essential for intestinal calcium absorption, leading to soft, weak bones.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features in children (rickets) include growth retardation, skeletal deformities (bowed legs, knock knees, rachitic rosary, Harrison's groove, frontal bossing), dental abnormalities, and hypocalcemic seizures. Clinical features in adults (osteomalacia) include diffuse bone pain, muscle weakness, increased fracture risk, and pathologic fractures.

Emergency warning signs

- Severe or worsening symptoms of rickets and osteomalacia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on serum 25-hydroxyvitamin D level (<20 ng/mL indicates deficiency), elevated alkaline phosphatase, low calcium and phosphate, and radiographic findings (widened growth plates, Looser's zones in adults). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves vitamin D supplementation (ergocalciferol or cholecalciferol) and calcium. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and

support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with adequate vitamin D replacement. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 744

Rocky Mountain Spotted Fever

Overview

Rocky Mountain spotted fever is a tick-borne illness caused by *Rickettsia rickettsii*, an obligate intracellular gram-negative coccobacillus, and is the most severe tick-borne rickettsial disease in the Americas, with 2000–4000 cases annually in the United States.

Causes

This condition happens because of *R. rickettsii* infecting vascular endothelial cells, causing widespread endothelial damage through direct cytotoxicity and immune-mediated injury, leading to increased vascular permeability, swelling, coagulation activation, and microvascular blood clot formation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of rocky mountain spotted fever require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

You may experience an incubation of 2–14 days, with the classic triad of fever, severe headache, and rash appearing on days 2–5, beginning as blanching erythematous macules on the wrists, ankles, and forearms, spreading centripetally to the trunk and palms/soles, with severe complications including encephalitis, acute kidney injury, ARDS, and gangrene.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam in endemic areas—treatment should not be delayed for laboratory confirmation. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include doxycycline. outlook: mortality 20–30% if untreated

- 3–5% with appropriate treatment.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 745

Rosacea

Overview

Rosacea is a chronic inflammatory facial dermatosis primarily affecting middle-aged adults with fair skin.

Causes

This condition results from disruption of normal rosacea function.

- This condition happens because of innate immune dysregulation, neurovascular dysfunction, and *Demodex folliculorum* mite overgrowth
- Triggers include sunlight, alcohol, spicy foods, hot beverages, and emotional stress.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features present with four subtypes: erythematotelangiectatic (persistent facial redness of the skin, telangiectasia, flushing), papulopustular (redness of the skin with papules and pustules, resembling acne), phymatous (skin thickening, particularly rhinophyma), and ocular (blepharitis, conjunctivitis, keratitis).

Emergency warning signs

- Severe or worsening symptoms of rosacea require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include sun protection, topical metronidazole or azelaic acid, topical ivermectin, oral doxycycline (low-dose, anti-inflammatory), and laser therapy for telangiectasia
- Severe rhinophyma may require surgical treatment.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is chronic with flares
- Treatment provides control but not cure.

Chapter 746

Rosai-Dorfman Disease

Overview

Rosai-Dorfman disease (sinus histiocytosis with massive lymphadenopathy) is a rare, non-Langerhans cell histiocytic disorder characterized by massive, painless, non-tender lymphadenopathy. Driven by polyclonal growth of S100-positive histiocytes within lymph node sinuses, a pathognomonic histological hallmark is emperipolesis—the engulfment of intact lymphocytes and erythrocytes within the cytoplasm of histiocytes without cellular degradation. Affecting children and young adults, clinical features present with bilateral massive cervical lymphadenopathy ('bull-neck' appearance), fever, leukocytosis, elevated ESR, and polyclonal hypergammaglobulinemia. Extranodal involvement occurs in 40% of cases (skin, nasal cavity, orbits, bone, central nervous system).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of rosai-dorfman disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors confirm the diagnosis through tissue biopsy showing large S100+, CD68+, CD1a- histiocytes exhibiting emperipolesis. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves observation for asymptomatic self-limiting cases
- Surgical debulking, systemic corticosteroids, or targeted therapy (MEK/BRAF inhibitors) are reserved for compressive extranodal lesions.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 747

Roseola (HHV-6 / Sixth Disease)

Overview

Roseola infantum, also known as sixth disease or exanthem subitum, is a common viral exanthem of early childhood caused by human herpesvirus 6 (HHV-6). It is characterized by high fever lasting 3–5 days followed by a rose-colored maculopapular rash that appears after the fever defervesces. Nearly all children are infected by age 2–3 years, and most infections are uncomplicated.

Classification

Cause Infectious (Viral)

Organ System Pediatric / Infectious

Pathophysiology Primary infection with HHV-6 (or less commonly HHV-7) causing lytic infection of CD4+ T lymphocytes

Duration Acute (5–7 days)

Onset Sudden

Mode of Transmission Respiratory droplets and saliva (virus persists lifelong)

Clinical Course Acute, self-limited

Severity Mild

Extent of Disease Systemic

Age Group Infants and Young Children (peak 6–15 months)

Epidemiology Universal; >90% of children seropositive by age 2

ICD-11 Code 1D60.2

Causes

Primary infection with human herpesvirus 6 (HHV-6B) causes roseola. HHV-7 causes a similar but less common illness. The virus infects CD4+ T lymphocytes and persists in the body lifelong as a latent infection, like other herpesviruses. Transmission occurs through respiratory secretions and saliva from asymptomatic contacts. Reactivation can occur in immunocompromised individuals.

Risk Factors

- Age 6–15 months (peak incidence)
- Lack of prior exposure (all children are susceptible)
- Close contact with infected children
- Immunosuppression (risk of reactivation)
- Daycare attendance
- Siblings (increased exposure)

Signs and Symptoms

Early symptoms

- Early manifestations of roseola (hhv-6 / sixth disease) may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- High fever (39–40.5°C) lasting 3–5 days
- Fever resolves abruptly (crisis), followed by rash
- Rose-colored, blanching maculopapular rash beginning on the trunk and spreading to the face and extremities
- Rash typically lasts 1–3 days without desquamation
- Mild respiratory symptoms, cough, and cervical lymphadenopathy
- Nagayama spots (erythematous papules on the soft palate)
- Irritability and mild diarrhea
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of roseola (hhv-6 / sixth disease) may occur

Emergency warning signs

- Severe or worsening symptoms of roseola (hhv-6 / sixth disease) require immediate medical evaluation

Progression and Stages

After an incubation period of 5–15 days, high fever develops abruptly without localizing signs. The child appears well despite high fever (fever-toxicity dissociation). Fever resolves abruptly on day 3–5, and within 12–24 hours the characteristic rash appears. The rash

fades over 1–3 days. Full recovery is the rule without specific antiviral therapy.

Complications

- Febrile seizures (most common complication, in 5–15% of cases)
- Meningitis and encephalitis (rare)
- Hepatitis and pneumonitis (rare)
- HHV-6 reactivation in immunocompromised patients (post-transplant)
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on classic fever-rash pattern
- HHV-6 IgM serology or PCR for confirmation (rarely needed)
- Serology cannot distinguish primary infection from reactivation
- Complete blood count may show leukopenia and relative lymphocytosis
- Lumbar puncture if encephalitis suspected

Prevention

- Good hand hygiene
- No vaccine available
- Most children are infected asymptotically, making prevention impractical
- Avoid exposure of immunocompromised children to known cases
- Go for regular check-ups so any problems are caught early

Treatment Options

- Supportive care: antipyretics (acetaminophen, ibuprofen) for fever
- Adequate hydration
- Anticonvulsant management for febrile seizures
- No specific antiviral therapy needed for immunocompetent children
- Ganciclovir or foscarnet for severe HHV-6 infections in immunocompromised patients
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help

- Keep learning about your condition and how to manage it

Outlook

The prognosis for roseola is excellent. It is a self-limited illness with full recovery in virtually all immunocompetent children. Febrile seizures are the most common complication and rarely have long-term consequences. Serious complications such as encephalitis are rare. HHV-6 remains latent lifelong but rarely causes disease in immunocompetent individuals after primary infection.

Chapter 748

Rotator Cuff Tendinopathy and Impingement

Overview

Rotator cuff tendinopathy and subacromial impingement syndrome are among the most common causes of shoulder pain, affecting up to 30% of the population at some point in life. These conditions involve inflammation, degeneration, or mechanical compression of the rotator cuff tendons, particularly the supraspinatus, as they pass through the subacromial space.

Classification

Cause Degenerative / Mechanical

Organ System Bones / Musculoskeletal

Pathophysiology Tendon degeneration, inflammation, and mechanical compression within the subacromial space

Duration Chronic (months to years)

Onset Gradual (or acute if superimposed tear)

Mode of Transmission Non-communicable

Clinical Course Chronic, progressive without treatment

Severity Mild to Severe

Extent of Disease Localized

Age Group Adults (40–60 years)

Epidemiology Very common; 30–50% of all shoulder pain presentations

ICD-11 Code FA52.0

Causes

Rotator cuff tendinopathy results from a combination of intrinsic tendon degeneration (age-related, hypovascularity) and extrinsic mechanical compression from the acromion, coracoacromial ligament, or acromioclavicular joint. Repetitive overhead activities, tendon overload, and poor scapular mechanics contribute to progressive tendon damage. Partial or full-thickness tears may develop as the condition advances.

Risk Factors

- Age over 40 years
- Repetitive overhead activities (painting, swimming, throwing sports)
- Occupational overhead work (construction, electricians)
- Poor posture and scapular dyskinesis
- Acromial morphology (curved or hooked acromion)
- Smoking
- Diabetes mellitus
- Previous shoulder injury

Signs and Symptoms

Early symptoms

- Early manifestations of rotator cuff tendinopathy and impingement may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Deep, aching shoulder pain, especially with overhead activities
- Night pain that disrupts sleep, particularly when lying on the affected side
- Pain with arm elevation between 60 and 120 degrees (painful arc sign)
- Weakness in shoulder abduction and external rotation
- Positive impingement signs (Neer, Hawkins-Kennedy)
- Stiffness and reduced range of motion
- Pain or discomfort in the affected area
- Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of rotator cuff tendinopathy and impingement may occur

Emergency warning signs

- Severe or worsening symptoms of rotator cuff tendinopathy and impingement require immediate medical evaluation

Progression and Stages

The condition progresses through three stages: Stage 1 involves edema and hemorrhage (reversible); Stage 2 involves tendinosis and fibrosis (chronic); Stage 3 involves partial or full-thickness tendon tears. Without intervention, progressive tendon degeneration leads to worsening pain, weakness, and functional limitation.

Complications

- Progression to full-thickness rotator cuff tear
- Muscle atrophy and fatty infiltration of rotator cuff muscles
- Frozen shoulder (adhesive capsulitis) from disuse
- Chronic pain and disability
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on history and physical examination
- Impingement tests (Neer, Hawkins-Kennedy, painful arc)
- Rotator cuff strength testing (Jobe test, external rotation lag sign)
- Ultrasound or MRI to assess tendon integrity and identify tears
- X-ray to evaluate acromial morphology and exclude other pathology

Prevention

- Maintain good shoulder posture and ergonomics
- Strengthen rotator cuff and scapular stabilizer muscles
- Avoid sudden increases in overhead activity intensity
- Use proper technique in sports and occupational activities
- Go for regular check-ups so any problems are caught early

Treatment Options

- Physical therapy focusing on rotator cuff strengthening and scapular stabilization
- Activity modification and avoidance of overhead aggravating activities
- Nonsteroidal anti-inflammatory drugs (NSAIDs) for pain and inflammation
- Subacromial corticosteroid injections for symptomatic relief
- Platelet-rich plasma (PRP) injections (emerging evidence)
- Arthroscopic subacromial decompression for refractory cases
- Rotator cuff repair surgery for full-thickness tears
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Most patients with rotator cuff tendinopathy improve with conservative management within 6–12 weeks. Outcomes are better with early intervention. Full-thickness tears may require surgical repair to restore function. Chronic or massive tears can lead to irreversible muscle atrophy and persistent disability, emphasizing the importance of timely diagnosis and treatment.

Chapter 749

Rotavirus

Overview

Transmission is fecal-oral.

Causes

This condition happens because of the virus infecting enterocytes of small intestinal villi, causing cell death, villous shrinkage or wasting, and malabsorptive diarrhea.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of rotavirus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Clinical features: incubation period of 1–3 days, acute onset of fever, vomiting followed by profuse watery diarrhea lasting 3–8 days
- Severe dehydration is the major complication.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Stool antigen detection or PCR is used for confirmation. Treatment: aggressive oral rehydration therapy and zinc supplementation
- No antivirals are available.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Rotavirus is the leading cause of severe acute gastroenteritis in infants and young children worldwide before vaccine introduction. Prevention: live attenuated oral rotavirus vaccines (Rotarix, RotaTeq) have reduced rotavirus hospitalizations by 80–90%.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate rehydration. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 750

Rubella

Overview

Rubella (German measles) is a mild viral illness caused by rubella virus, most significant for its teratogenic effects, with less than 1000 cases reported annually in the US since 2004, and transmission by respiratory droplets and transplacentally. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

- This condition happens because of the virus replicating in the respiratory epithelium and spreading hematogenously
- In congenital rubella, the virus infects the placenta and fetus during the first trimester, causing vasculitis and direct cellular damage.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience an incubation of 14–21 days. Postnatal rubella: low-grade fever, headache, coryza, and post-auricular/occipital lymphadenopathy, with a pink, maculopapular rash beginning on the face and spreading downward. Congenital rubella syndrome (CRS) includes the classic triad: sensorineural hearing loss (most common, 60–80%), congenital heart disease, and cataracts, occurring with infection in the first 16 weeks of pregnancy. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of rubella require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using RT-PCR or IgM serology. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification

- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment typically involves supportive. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- Most people recover well for postnatal rubella
- Congenital rubella syndrome causes permanent disability.

Chapter 751

SIADH

Overview

Syndrome of inappropriate antidiuretic hormone secretion (SIADH) is the most common cause of euvolemic hyponatremia, characterized by excessive ADH secretion despite normal or low plasma osmolality.

Causes

Causes include malignancies (small cell lung cancer most common, producing ectopic ADH), lung disorders (pneumonia, tuberculosis, aspergillosis), CNS disorders (meningitis, encephalitis, stroke, traumatic brain injury), and medications (SSRIs, TCAs, carbamazepine, vincristine, cyclophosphamide).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of siadh require immediate medical evaluation

Progression and Stages

Your symptoms depend on the severity and rate of development of hyponatremia: mild (lethargy, confusion, headache), moderate (nausea, vomiting, gait instability), severe (seizures, coma, respiratory arrest). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for hyponatremia (<135 mEq/L), low plasma osmolality (<275 mOsm/kg), inappropriately concentrated urine (urine osmolality >100 mOsm/kg), elevated urine sodium (>40 mEq/L), and exclusion of hypovolemia, hypervolemia, adrenal insufficiency, and hypothyroidism. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include fluid restriction (first-line), pharmacotherapy (demeclocycline, vaptans/tolvaptan for refractory cases), and for severe symptomatic hyponatremia, cautious 3% hypertonic saline with frequent sodium monitoring to avoid osmotic demyelination syndrome. outlook relates to the underlying cause
- SIADH itself resolves when the causative condition is treated.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 752

SSRI Overdose and Serotonin Syndrome

Overview

Selective serotonin reuptake inhibitor (SSRI) overdose is generally less toxic than TCA overdose. SSRIs cause mild CNS depression, nausea, vomiting, and tachycardia. Citalopram and escitalopram are associated with QT prolongation (dose-dependent). Serotonin syndrome is a potentially life-threatening condition caused by excessive serotonergic activity, most commonly from SSRI in combination with MAOIs, linezolid, methylene blue, dextromethorphan, tramadol, meperidine, or MDMA. The triad is altered mental status, autonomic hyperactivity, and neuromuscular abnormalities. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. This condition results from acute injury or exposure to harmful agents. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience clonus, hyperreflexia, tremor, myoclonus, hyperthermia, diaphoresis, tachycardia, high blood pressure, and agitation.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of ssri overdose and serotonin syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam using the Hunter Serotonin Toxicity Criteria.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care
Enzyme replacement therapy for certain conditions
Gene therapy (emerging for select conditions)
Dietary modifications (e.g.,

phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment options include discontinuation of serotonergic agents, supportive care (cooling, IV fluids, benzodiazepines), and cyproheptadine for moderate to severe cases.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with early recognition and treatment.

Chapter 753

SUNCT and SUNA

Overview

Short-lasting Unilateral Neuralgiform headache attacks with Conjunctival Injection and Tearing (SUNCT) and Short-lasting Unilateral Neuralgiform headache attacks with cranial autonomic symptoms (SUNA) are trigeminal autonomic cephalalgias characterized by brief, stabbing, burning, or electric shock-like pain in the orbital, supraorbital, or temporal region. They are more common in men (1.5–2:1), with onset typically after age 50.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience attacks lasting 1–600 seconds (typically 5–240 seconds) occurring 1–200 times per day.

Emergency warning signs

- Severe or worsening symptoms of sunct and suna require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant

impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment options include lamotrigine as first-line for prevention, with gabapentin, topiramate, and carbamazepine as alternatives
- Surgical options are reserved for refractory cases.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- The condition is often refractory to treatment.

Chapter 754

Salicylate (Aspirin) Overdose

Overview

Salicylate overdose is a pharmaceutical overdose that can be acute or chronic (chronic toxicity more dangerous with higher mortality). This condition results from acute injury or exposure to harmful agents.

Causes

This condition happens because of salicylates uncoupling oxidative phosphorylation, stimulating the respiratory center, inhibiting Krebs's cycle enzymes, and increasing metabolic acid production.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience ringing in the ears (earliest symptom), hyperventilation, nausea, vomiting, hyperthermia, diaphoresis, and tachycardia
- As toxicity progresses, mixed acid-base disorders emerge (respiratory alkalosis followed by concurrent metabolic acidosis)
- Severe toxicity includes altered mental status, seizures, coma, and lung swelling.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of salicylate (aspirin) overdose require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using serum salicylate level. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include activated charcoal, IV fluids with dextrose, bicarbonate therapy (goal urine pH 7.5–8.0) to enhance kidney elimination, and hemodialysis for severe toxicity. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 755

Sarcoidosis

Overview

Sarcoidosis is a multisystem granulomatous disease of unknown cause, characterized by non-caseating granulomas in affected tissues. It is discussed in detail in Chapter ?? (Section 756). Briefly, it most commonly affects the lungs and intrathoracic lymph nodes (>90%), but can involve any organ, including the skin (lupus pernio, redness of the skin nodosum), eyes (uveitis, dry eyes), liver, spleen, heart (arrhythmias, cardiomyopathy, heart failure), central nervous system (neurosarcoidosis—cranial nerve palsies, meningitis, hypothalamic-pituitary involvement), exocrine glands, and bones. To make the diagnosis, doctors look for compatible clinical/radiographic findings, histologic demonstration of non-caseating granulomas, and exclusion of other causes. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking

- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of sarcoidosis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use

- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Treatment typically involves indicated for organ-threatening or symptomatic disease: corticosteroids are first-line

- Hydroxychloroquine is used for cutaneous sarcoidosis
- Methotrexate, leflunomide, mycophenolate, azathioprine, and TNF inhibitors are used for refractory or severe disease.
- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease

- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

The outlook is generally positive: two-thirds of patients experience spontaneous remission within a decade. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 756

Sarcoidosis

Overview

Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions.

- Sarcoidosis is a multisystem granulomatous disorder of unknown cause characterized by non-caseating granulomas, most commonly affecting the lungs and intrathoracic lymph nodes
- It disproportionately affects young adults, particularly African Americans and Scandinavians.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

- This condition happens because of an aberrant immune response leading to granuloma formation. lung involvement (present in 90% of cases) manifests as cough, shortness of breath, and chest pain
- Extrapulmonary disease involves the skin (redness of the skin nodosum, lupus pernio), eyes (uveitis), joints, heart, and nervous system.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)

- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Your symptoms depend on the organs involved. Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of sarcoidosis require immediate medical evaluation

Progression and Stages

Treatment typically involves indicated for symptomatic or progressive disease: corticosteroids are first-line, with methotrexate, azathioprine, or infliximab as steroid-sparing agents. Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation

- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)

- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

The outlook is generally positive, with many cases resolving spontaneously. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 757

Sarcoidosis

Overview

Sarcoidosis is discussed in detail in Chapter ???. It is a multisystem granulomatous disease of unknown cause affecting the lungs, lymph nodes, skin, eyes, and other organs. Non-caseating granulomas on biopsy are the histological hallmark. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of sarcoidosis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain

- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Autoimmune diseases are generally chronic conditions requiring long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 758

Sarcopenia

Overview

Sarcopenia is a geriatric syndrome defined as the progressive, generalized loss of skeletal muscle mass, strength, and function with aging, with prevalence of 5–13% in adults aged 60–70 years and up to 50% in those over 80.

Causes

This condition happens because of neural denervation of motor units, reduced anabolic signaling (growth hormone, IGF-1, testosterone), increased catabolic cytokines (IL-6, TNF-alpha), mitochondrial dysfunction, and inadequate protein intake.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience low muscle mass, weakness, and functional decline. The European Working Group on Sarcopenia in Older People (EWGSOP2) defines sarcopenia by low muscle strength (grip strength <27 kg men, <16 kg women) plus low muscle quantity/quality and confirmed by low physical performance (gait speed ≤ 0.8 m/s).

Emergency warning signs

- Severe or worsening symptoms of sarcopenia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves resistance exercise (2–3 times weekly, progressive overload) plus adequate protein intake (≥ 1.2 g/kg/day) and vitamin D (800 IU/day)
- Testosterone replacement may be considered in hypogonadal men.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Early intervention can improve outcomes.

Chapter 759

Sarcopenic Obesity

Overview

Sarcopenic obesity is a distinct clinical entity characterized by the combination of excess adiposity and low muscle mass and function in the same individual. It is increasingly recognized with aging, but also occurs in the setting of malignancy, chronic disease, and catabolic states. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of a complex interplay of age-related hormonal decline, chronic inflammation, insulin resistance, physical inactivity, and inadequate protein intake. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the combination of obesity with low muscle strength and functional decline.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of sarcopenic obesity require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for measurement of body composition (DXA, bioelectrical impedance analysis, CT, or MRI) along with assessment of muscle strength and physical performance.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include resistance exercise training, adequate protein intake, vitamin D supplementation, and intentional weight loss through caloric restriction with emphasis on preserving lean mass. Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is guarded
- Sarcopenic obesity carries additive risks for falls, fractures, physical disability, cardiovascular disease, and mortality.

Chapter 760

Scabies

Overview

- Scabies is a parasitic infestation of the skin caused by the mite *Sarcoptes scabiei* var. *hominis*. Transmission occurs through prolonged skin-to-skin contact
- Fomite transmission is less common but possible.

Causes

This condition happens because of the female mite burrowing into the stratum corneum, laying eggs and triggering a delayed type IV hypersensitivity reaction.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience intensely pruritic (especially nocturnal), papular, and vesicular eruptions in characteristic locations: interdigital web spaces, wrists, elbows, axillae, periumbilical area, waistband, buttocks, and genitalia in males
- Burrows (thin, linear, grayish-white tracks) are pathognomonic. Crusted (Norwegian) scabies is a highly contagious variant in immunocompromised patients with widespread, thick, hyperkeratotic crusts.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of scabies require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical examination

- Dermoscopy may reveal the "delta wing jet" appearance
- Skin scraping confirms.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves with topical permethrin 5% cream (applied to the entire body from neck down, left on for 8–14 hours, repeated in 1 week) or oral ivermectin (200 $\mu\text{g}/\text{kg}$, repeated in 1–2 weeks)
- Close contacts should be treated simultaneously.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with treatment
- Post-scabetic itching may persist for weeks.

Chapter 761

Scarlet Fever

Overview

Scarlet fever is an acute systemic infection caused by exotoxin-producing strains of Group A β -hemolytic *Streptococcus* (GAS, *Streptococcus pyogenes*), predominantly affecting children aged 5–15 years.

Causes

This condition happens because of pyrogenic exotoxins (SPE-A, B, C) inducing delayed-type cutaneous hypersensitivity in individuals lacking protective antitoxin antibodies.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience fever, pharyngitis, strawberry tongue (initially white coat with red papillae, progressing to a beefy-red raw appearance), circumoral pallor, and a diffuse erythematous maculopapular rash with a 'sandpaper' texture that blanches on pressure and accentuates in skin folds (Pastia lines), followed by desquamation.

Emergency warning signs

- Severe or worsening symptoms of scarlet fever require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with rapid antigen detection test or throat culture.
Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Management requires 10 days of oral penicillin V or amoxicillin (or macrolides in penicillin-allergic patients) to eradicate GAS and prevent acute rheumatic fever and poststreptococcal glomerulonephritis.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support

services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 762

Schistosomiasis

Overview

Schistosomiasis (bilharziasis) is a trematode infection caused by blood flukes of the genus *Schistosoma*: *S. haematobium* (urinary), *S. mansoni* (intestinal), *S. japonicum*, *S. intercalatum*, and *S. mekongi* (intestinal), with 200 million infected and 800 million at risk, primarily in sub-Saharan Africa, with transmission through contact with fresh water containing infected snails.

Causes

This condition happens because of cercariae penetrating human skin, transforming into schistosomula, migrating to the portal venous system, maturing into adult worms, and producing eggs that become trapped in tissues, causing granulomatous inflammation, scarring, organomegaly, and obstruction.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of schistosomiasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

You may experience acute schistosomiasis (Katayama fever): fever, cough, urticaria, muscle pain, and hepatosplenomegaly 2–6 weeks after exposure

- Chronic *S. haematobium*: hematuria, dysuria, hydronephrosis, and bladder scarring, with squamous cell carcinoma as a long-term complication
- Chronic *S. mansoni* and *S. japonicum*: hepatosplenomegaly, periportal scarring (Symmers' pipe-stem scarring), portal high blood pressure, and esophageal varices.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using microscopic identification of eggs in stool or urine. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include praziquantel. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 763

Schizoaffective Disorder

Overview

Schizoaffective disorder has a lifetime prevalence of 0.3%.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features require that delusions or hallucinations occur without prominent mood symptoms for at least 2 weeks.

Emergency warning signs

- Severe or worsening symptoms of schizoaffective disorder require immediate medical evaluation

Progression and Stages

The condition is marked by an uninterrupted period of illness with concurrent features of schizophrenia (active-phase symptoms of delusions, hallucinations, disorganized speech, negative symptoms) and a major mood episode (depressive or manic) that occurs for the majority of the total duration of the illness. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Management combines antipsychotics and mood stabilizers/antidepressants guided by the predominant mood polarity. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is intermediate between schizophrenia and mood disorders. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 764

Schizophrenia

Overview

Schizophrenia has a lifetime prevalence of 0.3–0.7%, with a male-to-female ratio of 1.4:1, with men typically presenting earlier (peak onset 20–28 years) than women (peak onset 25–35 years). This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of dopaminergic hyperactivity in the mesolimbic pathway (positive symptoms), dopaminergic hypoactivity in the mesocortical pathway (negative symptoms and thinking and memory deficits), glutamatergic NMDA receptor hypofunction, and widespread gray matter loss, with genetic factors accounting for 80% of risk. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience two or more of the following for a significant portion of time during a 1-month period (at least one must be delusions, hallucinations, or disorganized speech): delusions (fixed false beliefs, often bizarre), hallucinations (auditory most common), disorganized speech (loose associations, tangentiality), grossly disorganized or catatonic behavior, and negative symptoms (blunted affect, avolition, alogia, anhedonia, asociality), with continuous signs persisting for at least 6 months.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of schizophrenia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam—no biomarkers are available.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include antipsychotics (both first-generation and second-generation, with clozapine being the only FDA-approved medication for treatment-resistant schizophrenia), thinking and memory-behavioral therapy for psychosis (CBTp), psychosocial interventions, and thinking and memory remediation therapy. Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person: 20–30% have a good outcome, with a chronic course and functional decline in many, and suicide risk of 5–10%. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 765

Schwannoma

Overview

- Schwannomas are non-cancerous nerve sheath tumors arising from Schwann cells
- Vestibular schwannomas (acoustic neuromas) are the most common intracranial schwannomas.

Causes

This condition happens because of neoplastic growth of Schwann cells. Bilateral vestibular schwannomas are pathognomonic of neurofibromatosis type 2.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of vestibular schwannoma include unilateral sensorineural hearing loss, ringing in the ears, and imbalance.

Emergency warning signs

- Severe or worsening symptoms of schwannoma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on MRI showing a cerebellopontine angle mass with an ice-cream cone appearance in the internal auditory canal. Management options include observation with serial imaging, stereotactic radiosurgery (Gamma Knife), and microsurgical removal
- Facial nerve preservation is a key surgical goal.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well for non-cancerous schwannomas. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 766

Scleritis

Overview

Causes

This condition happens because of immune-mediated inflammation of the sclera.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience deep, boring eye pain, redness, and decreased vision
- Anterior scleritis may be diffuse, nodular, or necrotizing
- Posterior scleritis can mimic orbital tumors.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of scleritis require immediate medical evaluation

Progression and Stages

Your outlook depends on the severity and underlying systemic disease. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Scleritis is a severe inflammation of the sclera that is potentially sight-threatening due to complications such as glaucoma, cataract, and posterior segment inflammation
- Over 50% of cases are associated with systemic autoimmune disease, particularly rheumatoid arthritis, granulomatosis with polyangiitis, and relapsing polychondritis.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical examination and imaging when indicated. Management requires systemic corticosteroids and immunosuppressive agents. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 767

Scoliosis

Overview

Scoliosis is abnormal lateral curvature of the spine (Cobb angle $>10^\circ$). Adolescent idiopathic scoliosis (AIS) is the most common form (80% of cases), typically presenting in girls during the adolescent growth spurt, with a right thoracic curve. Neuromuscular scoliosis (from cerebral palsy, muscular dystrophy), congenital scoliosis (vertebral anomalies), and syndromic scoliosis (Marfan, Down syndrome) are other forms.

Causes

This condition happens because of abnormal spinal development or neuromuscular imbalance.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience asymmetrical trunk, shoulder height discrepancy, and rib hump on forward bending.

Emergency warning signs

- Severe or worsening symptoms of scoliosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- The right treatment depends on severity: mild curves are monitored
- Moderate curves (25–40°) are treated with bracing
- Severe curves (>40–50°) require surgical correction with posterior spinal instrumentation and fusion.

Complications

- In most cases, the outlook is favorable with appropriate management
- Complications include chronic back pain and cardiopulmonary compromise in severe curves.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on physical examination and X-ray with Cobb angle measurement. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 768

Scorpion Stings

Overview

Centruroides sculpturatus (bark scorpion) is the only medically significant scorpion in North America. Venom is a sodium channel toxin causing prolonged neuronal depolarization and excessive autonomic and somatic motor activity.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: immediate local pain, paresthesias, involuntary muscle spasms, cranial nerve dysfunction (involuntary eye movements, opsoclonus, hypersalivation, tongue fasciculations, difficulty swallowing), respiratory distress, and seizures. Children are at highest risk for severe toxicity.

Emergency warning signs

- Severe or worsening symptoms of scorpion stings require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Anascorp (Centruroides immune F(ab')₂ antivenom) is highly effective, with resolution of symptoms within 1–4 hours. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Management: supportive care, benzodiazepines for agitation and muscle spasms.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with antivenom administration. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 769

Seborrheic Dermatitis

Overview

Seborrheic dermatitis is a chronic inflammatory skin condition affecting areas rich in sebaceous glands (scalp, face, chest, back). It is linked to *Malassezia* species (lipophilic yeast) and may be triggered by stress, cold weather, and neurological conditions (Parkinson's disease). In infants, it manifests as "cradle cap" (thick, yellowish scales on the scalp).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features in adults include greasy, yellowish scales on an erythematous base, affecting the scalp (dandruff in mild form), eyebrows, nasolabial folds, and ears.

Emergency warning signs

- Severe or worsening symptoms of seborrheic dermatitis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include topical antifungal agents (ketoconazole shampoo/cream), low-potency topical corticosteroids, and calcineurin inhibitors. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is chronic with periodic flares. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 770

Selenium Deficiency

Overview

Selenium deficiency is a mineral deficiency disorder occurring in regions with low soil selenium content (parts of China, Europe, New Zealand). This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of deficiency of selenium, an essential trace element functioning as a component of selenoproteins, including glutathione peroxidases (antioxidant defense), iodothyronine deiodinases (thyroid hormone metabolism), and selenoprotein P (selenium transport). Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience Keshan disease (an endemic dilated cardiomyopathy affecting children and women of childbearing age, presenting with acute or chronic heart failure, cardiomegaly, arrhythmias) and Kashin-Beck disease (an osteoarthropathy

- presenting with joint pain, stiffness, and deformities of the fingers, knees, and ankles)
- Selenium deficiency also impairs immune function, exacerbates viral virulence, and is linked to infertility and myopathy.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of selenium deficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low serum selenium (less than 50–70 µg/L) and decreased glutathione peroxidase activity.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on sodium selenite or selenomethionine 100–200 µg daily. Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person.

Chapter 771

Sepsis and Septic Shock

Overview

Sepsis is life-threatening organ dysfunction caused by a dysregulated host response to infection, with septic shock being a subset with profound circulatory, cellular, and metabolic abnormalities associated with a higher risk of mortality, with an estimated 48.9 million cases and 11 million deaths annually globally. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of infection triggering pathogen-associated molecular patterns (PAMPs) and damage-associated molecular patterns (DAMPs) that bind toll-like receptors (TLRs), leading to release of pro-inflammatory cytokines (TNF- α , IL-1, IL-6), neutrophil activation, complement cascade, endothelial injury, and microvascular blood clot formation, with mitochondrial dysfunction and immune exhaustion contributing to late mortality. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)

- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience fever or hypothermia, tachycardia, tachypnea, altered mental status, hypotension, oliguria, mottled skin, and evidence of end-organ dysfunction (elevated lactate, acute kidney injury, liver dysfunction, thrombocytopenia).
Fever and chills
Fatigue and malaise
Localized pain and inflammation at infection site
Swelling, redness, and warmth (local infection)
Cough and respiratory symptoms (respiratory infections)
Nausea, vomiting, and diarrhea (gastrointestinal infections)
Headache and body aches
Skin rash or lesions
Enlarged lymph nodes
Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of sepsis and septic shock require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent

stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using clinical criteria per Sepsis-3 guidelines (suspected infection plus SOFA score increase of ≥ 2 points), with blood cultures, lactate, complete blood count, coagulation profile, kidney and liver function, procalcitonin, and imaging to identify the source. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs

and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

Treatment options include early recognition, source control, empirical broad-spectrum antibiotics (within 1 hour of recognition), intravenous fluids (30 mL/kg crystalloids for hypotension), vasopressors (norepinephrine first-line), and supportive care.

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Your outlook depends on the underlying infection, timeliness of treatment, and patient factors. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 772

Septic Abortion

Overview

Septic abortion is a miscarriage or induced abortion complicated by infection of the uterine cavity and its contents, typically following unsafe abortion procedures. It remains a significant cause of maternal mortality in regions with restricted abortion access. Causative organisms include a mix of aerobic and anaerobic bacteria from the lower genital tract. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience fever, chills, foul-smelling vaginal discharge, lower abdominal pain, uterine tenderness, and signs of sepsis. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of septic abortion require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment

- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with blood cultures and pelvic imaging. Treatment requires IV broad-spectrum antibiotics and prompt surgical evacuation of retained products of conception. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

The outlook is generally positive with prompt treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 773

Septic Arthritis

Overview

Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

- It most commonly affects large joints (knee, hip) and is usually monoarticular. Routes of infection include hematogenous spread (most common), direct inoculation (trauma, surgery, injection), and contiguous spread from adjacent osteomyelitis. *S. aureus* is the most common organism in adults
- *Neisseria gonorrhoeae* in young, sexually active adults
- And *Kingella kingae* in children under 4.

Causes

This condition happens because of bacterial infection of the joint space. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience acute joint pain, swelling, warmth, redness of the skin, and reduced range of motion, with systemic signs of infection (fever, chills). Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of septic arthritis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

To make the diagnosis, doctors look for joint removal by suction showing turbid fluid with elevated WBC count ($>50,000$ cells/ μL with $>90\%$ neutrophils), positive Gram stain and culture. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Septic arthritis is infection of a joint, a medical emergency requiring prompt treatment to prevent irreversible articular cartilage destruction. Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions

and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment focuses on urgent joint removal of fluid (repeated removal by suction, using a camera inside a joint lavage, or open removal of fluid) and IV antibiotics (empiric: vancomycin + ceftriaxone, adjusted based on culture and sensitivity). Treatment aims to control inflammation, manage symptoms, and prevent disease flares. A step-up approach is often used, starting with symptomatic treatments and progressing to immunosuppressive therapies as needed. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- The outlook is generally positive with prompt treatment
- Delayed treatment leads to irreversible joint damage.

Chapter 774

Severe Combined Immunodeficiency

Overview

Severe combined immunodeficiency is a group of rare, life-threatening genetic disorders characterized by severe impairment of both cellular (T-cell) and humoral (B-cell) immunity, with or without natural killer (NK) cell deficiency. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition results from disruption of normal severe combined immunodeficiency function.

- This condition happens because of over 20 genetic causes, with X-linked SCID (IL2RG mutations, most common) and adenosine deaminase (ADA) deficiency being the best known
- Without treatment, affected infants typically die within the first year of life from severe, recurrent, opportunistic infections.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience recurrent severe infections, failure to thrive, and persistent diarrhea.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of severe combined immunodeficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Newborn screening can detect SCID by measuring T-cell receptor removal circles (TRECs) on dried blood spots, with diagnosis confirmed by lymphocyte subset analysis (flow cytometry), immunoglobulin levels, and genetic testing.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include hematopoietic stem cell transplantation (curative, ideally before 3.5 months of age), enzyme replacement therapy (for ADA deficiency), and gene therapy (for X-linked SCID and ADA-SCID). Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with early transplantation.

Chapter 775

Shift Work Sleep Disorder

Overview

Shift work sleep disorder (SWSD) is a circadian rhythm sleep-wake disorder affecting individuals whose work schedules overlap with the typical sleep period (night shifts, early morning shifts, rotating shifts), causing circadian misalignment with resultant insomnia, excessive sleepiness, or both. Approximately 20–30% of shift workers meet criteria. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of a misalignment between the endogenous circadian timing system and the work-rest schedule.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience difficulty initiating or maintaining sleep during the available sleep period and excessive sleepiness during work hours.

Emergency warning signs

- Severe or worsening symptoms of shift work sleep disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with sleep diaries. Management strategies include strategic napping before or during shifts, bright light exposure during work, controlled light avoidance during the commute home, fixed sleep scheduling in a dark/cool room, melatonin at the desired bedtime, and wake-promoting agents (modafinil, armodafinil, or caffeine) for night-shift alertness. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with appropriate management
- Chronic misalignment increases risks of cardiovascular disease, metabolic syndrome, and depression.

Chapter 776

Short Bowel Syndrome

Overview

Short bowel syndrome (SBS) is a malabsorptive condition resulting from surgical removal of a significant portion of the small intestine, leading to insufficient nutrient and fluid absorption. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

The most common cause is Crohn's disease, followed by mesenteric reduced blood flow, trauma, and malignancy. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience profuse diarrhea, steatorrhea, weight loss, dehydration, electrolyte imbalances, and nutritional deficiencies.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of short bowel syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- This condition happens because of reduced intestinal absorptive surface area
- The remaining functional bowel length and the presence or absence of the ileum and colon determine severity.

Complications

- Your outlook depends on the extent of removal and degree of adaptation
- Intestinal transplantation is reserved for patients with life-threatening complications of parenteral nutrition.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on surgical history and clinical presentation
- Adaptation of the remaining bowel occurs over months to years.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include parenteral nutrition and fluids, gradual enteral feeding to promote adaptation, antidiarrheal agents, and H2 blockers or PPIs to reduce gastric acid
- Teduglutide (a GLP-2 analog) promotes intestinal adaptation and may reduce parenteral nutrition dependence.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 777

Sick Sinus Syndrome

Overview

Sick sinus syndrome (SSS) is a group of irregular heartbeats caused by dysfunction of the sinoatrial node, the heart's primary pacemaker, with an incidence increasing with age, affecting 1 in 600 patients over 65 years. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition happens because of age-related degenerative scarring of the SA node, reduced blood flow, infiltrative diseases (amyloidosis, sarcoidosis, hemochromatosis), cardiomyopathy, or surgical trauma. Iatrogenic causes include beta-blockers, calcium channel blockers, digoxin, and antiarrhythmic drugs. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sinus bradycardia (often profound, <40 bpm), sinus arrest (pauses >3 seconds causing syncope), sinoatrial exit block, and the tachycardia-bradycardia syndrome (alternating paroxysmal atrial tachyarrhythmias, usually atrial fibrillation, with sinus bradycardia). Symptoms include fatigue, lightheadedness, presyncope, syncope (Stokes-Adams attacks), shortness of breath on exertion, and palpitations.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of sick sinus syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on 12-lead ECG showing bradyarrhythmia, 24-hour Holter monitoring (most useful for correlating symptoms with rhythm), event monitor, and for equivocal cases, electrophysiology study.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on permanent pacemaker implantation (dual-chamber or atrial pacing) for symptomatic SSS. Pacemaker therapy resolves symptoms and prevents syncope but does not reduce mortality. Rate-slowing medications for tachycardia-bradycardia syndrome may be used after pacemaker placement. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is good with appropriate pacing
- SSS itself is not life-threatening but can cause significant morbidity from syncope and falls.

Chapter 778

Sickle Cell Disease

Overview

Sickle cell disease is a group of inherited hemoglobinopathies predominantly affecting individuals of African, Mediterranean, Middle Eastern, and Indian descent. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition results from disruption of normal sickle cell disease function.

- This condition happens because of mutations in the β -globin gene, leading to production of abnormal hemoglobin S (HbS), with the most common genotype being HbSS (sickle cell anemia, caused by homozygosity for the Glu6Val mutation)
- Deoxygenated HbS polymerizes, causing red blood cells to assume a sickle shape, leading to vaso-occlusion, hemolysis, and end-organ damage.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience vaso-occlusive crises (acute pain crises, the hallmark of SCD), acute chest syndrome (chest pain, fever, lung infiltrates—a leading cause of hospitalization and death), stroke (both ischemic and involving bleeding, occurring in up to 20% of children), avascular tissue death of the femoral head, priapism, splenic sequestration and functional asplenia, delayed growth, and chronic kidney disease.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of sickle cell disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using hemoglobin electrophoresis and HPLC showing elevated HbS.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment options include hydroxyurea (increases fetal hemoglobin [HbF], reduces crisis frequency), chronic transfusion therapy, pain Treatment for crises, and prophylactic penicillin (for children to prevent *S. pneumoniae* sepsis), with novel therapies including voxelotor (HbS polymerization inhibitor), crizanlizumab (anti-P-selectin antibody), and L-glutamine (oxidative stress reducer).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has improved significantly with modern therapies, with allogeneic stem cell transplantation offering cure but limited by donor availability, and gene therapy (lovotibeglogene autotemcel, exagamglogene autotemcel) showing remarkable efficacy in clinical trials.

Chapter 779

Sideroblastic Anemia

Overview

Sideroblastic anemia is a disorder characterized by defective iron utilization in erythropoiesis, leading to iron accumulation in mitochondria of erythroid precursors and forming ring sideroblasts in the bone marrow. The condition may be hereditary (X-linked sideroblastic anemia, ALAS2 mutation) or acquired (myelodysplastic syndrome, alcohol, lead poisoning, isoniazid, pyridoxine deficiency).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience symptoms of anemia. Diagnosis shows a dimorphic picture (microcytic and macrocytic cells on peripheral smear), basophilic stippling, and a characteristic ring sideroblast pattern on Prussian blue staining of bone marrow.

Emergency warning signs

- Severe or worsening symptoms of sideroblastic anemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include pyridoxine (vitamin B6, especially for hereditary forms), blood transfusions, and iron chelation therapy for iron overload, with allogeneic stem cell transplantation potentially curative. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause and response to treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 780

Silicosis

Overview

Silicosis is an occupational pneumoconiosis caused by chronic inhalation of crystalline silicon dioxide (silica) dust in miners, sandblasters, stonecutters, and foundry workers.

Causes

This condition happens because of silica particle ingestion by alveolar macrophages, triggering cell death, release of fibrogenic cytokines, and formation of dense hyalinized silicotic nodules.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience exertional shortness of breath, chronic productive cough, and increased susceptibility to lung tuberculosis (silicotuberculosis) due to impaired macrophage function. Diagnosis is supported by chest imaging showing upper lobe eggshell calcium buildup of hilar lymph nodes and nodular opacities.

Emergency warning signs

- Severe or worsening symptoms of silicosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves preventive and supportive
- Oxygen therapy and prompt treatment of mycobacterial infections are essential.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on cumulative silica exposure. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 781

Sleep-Related Bruxism

Overview

Sleep-related bruxism is a sleep-related movement disorder characterized by repetitive jaw-muscle activity during sleep, manifesting as tooth grinding or clenching, often associated with rhythmic masticatory muscle activity (RMMA). Prevalence is 8–12% in adults and 15–30% in children, decreasing with age. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition happens because of sleep-related activation of the brainstem masticatory central pattern generator, triggered by spontaneous microarousals and possibly modulated by autonomic activation.

Risk Factors

- Family history of mental illness
- Trauma or adverse childhood experiences
- Chronic stress
- Substance use
- Social isolation
- Underlying medical conditions
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Medication use

Signs and Symptoms

Common symptoms

- You may experience audible tooth grinding, jaw pain or fatigue, temporal headaches, tooth wear, tooth sensitivity, and temporomandibular joint pain.

Emergency warning signs

- Severe or worsening symptoms of sleep-related bruxism require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Polysomnography with masseter EMG is the gold standard.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include occlusal splints for tooth protection, stress reduction, treatment of comorbid OSA, botulinum toxin injections for severe cases, and avoidance of exacerbating substances. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical

intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is manageable
- Severe untreated bruxism can result in tooth fracture and TMJ disorders.

Chapter 782

Sleep-Related Rhythmic Movement Disorder

Overview

Sleep-related rhythmic movement disorder (RMD) is a sleep-related movement disorder involving stereotypical, repetitive movements of large muscle groups occurring primarily during the transition from wakefulness to sleep or during NREM sleep. The most common forms are head banging, head rolling, and body rocking. Prevalence is up to 60% in infants (physiologic), declining to less than 1% in adults. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions.

Causes

This condition results from disruption of normal sleep-related rhythmic movement disorder function.

- This condition happens because of unknown Disease mechanism
- In adults, it is linked to neurodevelopmental disorders, anxiety, and OSA.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience rhythmic movements at a frequency of 0.5–2 Hz lasting less than 15 minutes.

Emergency warning signs

- Severe or worsening symptoms of sleep-related rhythmic movement disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Polysomnography with synchronized video recording is indicated when injury is suspected or to rule out seizure.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves primarily reassurance and safety measures
- Clonazepam, hypnotics, or behavioral therapy may be used in persistent or injurious adult cases.

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is non-cancerous
- Most children outgrow the condition.

Chapter 783

Small Bowel Obstruction

Overview

Small bowel obstruction (SBO) is a partial or complete blockage of the small intestine.

Causes

This condition results from disruption of normal small bowel obstruction function.

- The most common cause is adhesions from prior abdominal surgery (60% of cases)
- Other causes include hernias (inguinal, incisional), malignancy, volvulus, intussusception (in children), inflammatory bowel disease strictures, and gallstone ileus. This condition happens because of mechanical blockage of intestinal contents.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience crampy abdominal pain (periumbilical), nausea, vomiting (bilious early, feculent late), abdominal distension, and obstipation.

Emergency warning signs

- Severe or worsening symptoms of small bowel obstruction require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on abdominal X-ray showing air-fluid levels and dilated proximal bowel loops with collapsed distal bowel
- CT abdomen/pelvis is more sensitive and identifies the transition point and cause.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include nasogastric decompression, IV fluid resuscitation, electrolyte correction, and close monitoring
- Complete obstruction or signs of strangulation (fever, tachycardia, peritoneal signs, metabolic acidosis) require emergent surgical exploration.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on cause and presence of strangulation. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 784

Snake Envenomation

Overview

Viperidae venom contains hemotoxins causing local tissue destruction, coagulopathy, and thrombocytopenia. Elapidae venom contains neurotoxins causing neuromuscular blockade with descending paralysis, ptosis, bulbar palsy, respiratory failure, and sometimes minimal local symptoms. Sea snakes cause myotoxicity with generalized muscle pain, myoglobinuria, and acute kidney injury.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of snake envenomation require immediate medical evaluation

Progression and Stages

Clinically significant snake envenomation is classified by family: Viperidae (vipers, pit vipers—rattlesnakes, copperheads, cottonmouths) and Elapidae (cobras, kraits, mambas, coral snakes, sea snakes). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked.

Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with lab evaluation (CBC, PT/PTT, INR, fibrinogen, D-dimer, creatinine, CK). Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Management: supportive care, wound management, tetanus prophylaxis, and antivenom (Crotalidae polyvalent immune Fab for North American pit vipers).

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with timely antivenom administration. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 785

Social Anxiety Disorder (Social Phobia)

Overview

Social anxiety disorder has a lifetime prevalence of 7–13%, making it one of the most common anxiety disorders, typically beginning in adolescence with peak onset at age 13. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of amygdala hyperreactivity, insula hyperactivity, and altered default-mode network connectivity. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience marked fear or anxiety about one or more social situations in which the individual is exposed to possible scrutiny by others, with fear of being negatively evaluated, embarrassment, rejection, or offending others, with the fear being out of proportion to the actual threat and persistent (at least 6 months). Physical symptoms (blushing, trembling, sweating, palpitations) are common.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of social anxiety disorder (social phobia) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include SSRIs and SNRIs as first-line pharmacotherapy, and CBT (thinking and memory restructuring and exposure therapy) as first-line psychotherapy. Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with treatment. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 786

Somatic Symptom Disorder

Overview

Somatic symptom disorder has an estimated prevalence of 5–7% in the general population and is more common in women.

Causes

This condition happens because of hypervigilance to bodily sensations, amplification of normal physiological signals, maladaptive health-related cognitions, and altered brain connectivity in insula, anterior cingulate, and prefrontal cortex.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience one or more somatic symptoms that are distressing or result in significant disruption of daily life, with excessive thoughts, feelings, or behaviors related to the somatic symptoms manifested by disproportionate and persistent thoughts about the seriousness of symptoms, persistently high level of anxiety about health, or excessive time and energy devoted to these symptoms.

Emergency warning signs

- Severe or worsening symptoms of somatic symptom disorder require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include CBT (thinking and memory restructuring, attention retraining, behavioral activation) and pharmacotherapy focusing on comorbid depression and anxiety (SSRIs). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is chronic but function can improve with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 787

Specific Phobias

Overview

Specific phobias have a lifetime prevalence of 10–13%, making them the most common anxiety disorder in the general population. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of conditioned fear responses. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience marked, persistent (at least 6 months) fear or anxiety about a specific object or situation (eg., flying, heights, animals, receiving an injection, blood, enclosed spaces), with the phobic stimulus almost invariably provoking an

immediate fear response out of proportion to the actual danger. Common subtypes include animal type, natural environment type, blood-injection-injury type (unique vasovagal response with fainting in up to 75%), situational type, and other type.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of specific phobias require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Treatment for choice is CBT with exposure therapy (systematic desensitization).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Psychotherapy (cognitive behavioral therapy, interpersonal therapy, psychodynamic therapy) Pharmacotherapy (antidepressants, antipsychotics, mood stabilizers, anxiolytics) Combined treatment approaches for optimal outcomes Hospitalization for acute crisis management Support groups and peer support programs Lifestyle interventions (exercise, sleep hygiene, stress management) Mindfulness and meditation practices Family therapy and psychoeducation Vocational and social rehabilitation Long-term maintenance therapy for chronic conditions

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 788

Spider Bites

Overview

Clinically significant spider bites in North America include black widow (*Latrodectus*) and brown recluse (*Loxosceles*). Black widow venom contains alpha-latrotoxin, causing massive release of acetylcholine and norepinephrine.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: local pinprick sensation, then severe muscle pain and cramping, rigidity, diaphoresis, nausea, vomiting, high blood pressure, tachycardia, and rarely respiratory compromise. Brown recluse venom contains sphingomyelinase D, causing dermonecrotic injury, hemolysis, and coagulopathy. Clinical features: initially painless bite, then over 6–12 hours redness of the skin, blister, and tissue death forming an eschar over 1–2 weeks. Systemic loxoscelism includes fever, chills, nausea, arthralgias, hemolytic anemia, thrombocytopenia, DIC, acute kidney injury.

Emergency warning signs

- Severe or worsening symptoms of spider bites require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment: supportive care, benzodiazepines for muscle spasms, and latrodectus antivenom for severe cases. Treatment: wound care, supportive care for hemolysis and kidney failure.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 789

Splenic Abscess

Overview

Splenic abscess is a rare, life-threatening localized suppurative infection of the splenic parenchyma, occurring predominantly in immunocompromised individuals or patients with infective endocarditis. Disease mechanism involves hematogenous seeding from distant septic foci (endocarditis, urinary tract infections, osteomyelitis), direct extension from contiguous abdominal infections, or secondary infection of ischemic splenic infarcts. Common pathogens include *Staphylococcus aureus*, *Streptococcus* species, *Enterobacteriaceae*, and anaerobes. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Symptoms often appear as the classic triad of fever, left upper quadrant abdominal pain, and leukocytosis, frequently accompanied by left pleural effusion or referred left shoulder pain (Kehr sign). Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of splenic abscess require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)

- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors confirm the diagnosis through abdominal CT or ultrasound showing solitary or multiple hypodense lesions. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)

- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment focuses on parenteral broad-spectrum antimicrobial therapy combined with through the skin image-guided catheter removal of fluid or splenectomy for multiloculated or refractory abscesses. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global

burden of infectious diseases.

Chapter 790

Splenic Artery Aneurysm

Overview

Splenic artery aneurysm (SAA) is the most common visceral artery aneurysm, accounting for 60–80% of all splanchnic vascular aneurysms, with a marked female-to-male predominance (4:1). This cardiovascular condition is a leading cause of morbidity and mortality globally. Risk increases with age, and the condition often requires lifelong management. Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

Causes

Hormonal and hemodynamic changes during pregnancy markedly increase wall stress and rupture risk. Most SAAs are asymptomatic incidental findings on abdominal imaging, but symptomatic cases present with left upper quadrant or epigastric pain. Rupture presents as acute intra-abdominal bleeding ('double-rupture' phenomenon into the lesser sac followed by free peritoneal rupture), precipitating catastrophic involving bleeding shock. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus

- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)
- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of splenic artery aneurysm require immediate medical evaluation

Progression and Stages

This condition happens because of arterial wall media deterioration driven by multiparity, portal high blood pressure, arterial dysplasia, or pancreatitis-induced pseudaneurysms. Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)

- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia
- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Your doctor confirms the diagnosis with CT angiography. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Treatment options include endovascular coil embolization, covered stenting, or surgical ligation for symptomatic aneurysms, aneurysms > 2 cm, or any SAA in women of childbearing age or pregnant patients. Management includes lifestyle modifications, pharmacotherapy targeting the underlying mechanisms, and interventional procedures

when indicated. Long-term adherence to treatment is essential for preventing disease progression. Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel) Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 791

Splenic Cyst

Overview

- Splenic cysts are fluid-filled parenchymal lesions categorized as primary (true cysts with an epithelial or mesothelial lining, accounting for 20%) or secondary (false pseudocysts lacking cellular lining, accounting for 80%). Primary cysts include congenital epidermoid cysts, mesothelial cysts, and parasitic hydatid cysts (*Echinococcus granulosus*). Secondary pseudocysts result from prior blunt abdominal trauma, splenic tissue death due to lack of blood supply, or localized hematoma liquefaction. Most small cysts (< 5 cm) are asymptomatic
- Larger lesions cause left upper quadrant fullness, early satiety, dull flank pain, or palpable splenomegaly.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of splenic cyst require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors confirm the diagnosis through ultrasound or CT imaging
- Hydatid serology is mandatory in endemic regions. Treatment for small asymptomatic cysts is conservative surveillance
- Large symptomatic or high-risk cysts (> 5 cm) undergo using small incisions and a camera unroofing, partial splenectomy, or cystectomy to preserve splenic immune function while avoiding rupture.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 792

Splenic Cysts

Overview

Most are asymptomatic and discovered incidentally on imaging.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of large cysts include abdominal pain, early satiety, or a palpable mass.

Emergency warning signs

- Severe or worsening symptoms of splenic cysts require immediate medical evaluation

Progression and Stages

Splenic cysts are fluid-filled lesions in the spleen, classified as true cysts (with epithelial lining, including congenital, epidermoid, and dermoid cysts) or pseudocysts (without epithelial lining, usually resulting from prior trauma, tissue death due to lack of blood supply, or infection). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on imaging. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves observation for small, asymptomatic cysts

- Surgical removal (cystectomy or partial splenectomy) or splenectomy for large or symptomatic cysts
- Through the skin removal by suction and sclerotherapy is an alternative for selected cases.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 793

Splenic Infarction

Overview

Splenic tissue death due to lack of blood supply is ischemic tissue death of splenic parenchyma resulting from occlusion of the splenic artery or its branches.

Causes

The most common cause is embolization from the heart (atrial fibrillation, endocarditis, mural blood clot), followed by hematologic disorders (myeloproliferative disorders, sickle cell disease, thalassemia) and vasculitis.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience acute left upper quadrant pain, sometimes radiating to the left shoulder (Kehr's sign).

Emergency warning signs

- Severe or worsening symptoms of splenic infarction require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on CT scan showing wedge-shaped, hypodense, non-enhancing lesions. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves conservative (analgesics, observation) with treatment of the underlying cause. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is favorable
- Rarely, complicated tissue death due to lack of blood supply (abscess, rupture) may require splenectomy.

Chapter 794

Splenic Lymphoma

Overview

Splenic lymphoma is involvement of the spleen by systemic lymphoma or, less commonly, primary splenic origin. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

- This condition happens because of lymphoproliferation
- Hepatitis C virus infection is linked to SMZL in some cases. Hairy cell leukemia is another important splenic lymphoproliferative disorder characterized by the BRAF V600E mutation.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)

- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience massive splenomegaly, lymphocytosis (villous lymphocytes on blood smear in SMZL), and pancytopenia (in hairy cell leukemia). Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of splenic lymphoma require immediate medical evaluation

Progression and Stages

Splenic marginal zone lymphoma (SMZL) is the most common primary splenic lymphoma, a low-grade B-cell lymphoma. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local

disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

- Diagnosis typically involves bone marrow biopsy, flow cytometry, and CT imaging. Management varies by histology: splenectomy and/or rituximab for SMZL
- Cladribine or moxetumomab for hairy cell leukemia.
- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing

infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications

- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 795

Splenic Rupture

Overview

Splenic rupture is a life-threatening emergency, most commonly caused by blunt abdominal trauma (motor vehicle accidents, falls, sports injuries). The spleen is the most commonly injured abdominal organ in trauma. The condition may also occur spontaneously in the setting of massive splenomegaly (infectious mononucleosis, hematologic malignancies, myelofibrosis).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience left upper quadrant pain, peritoneal signs, hemodynamic instability (tachycardia, hypotension), and Kehr's sign (left shoulder pain).

Emergency warning signs

- Severe or worsening symptoms of splenic rupture require immediate medical evaluation

Progression and Stages

Management follows Advanced Trauma Life Support (ATLS) principles: hemodynamic resuscitation, splenic grading, and decision between non-operative management (for hemodynamically stable patients with lower-grade injuries) and emergent splenectomy

(for hemodynamic instability or high-grade injuries). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on focused assessment with sonography for trauma (FAST) and CT scan, which also grades the injury. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Your outlook depends on injury severity and timely intervention
- Post-splenectomy vaccination (pneumococcal, meningococcal, *Haemophilus influenzae* type b) and prophylactic antibiotics are essential to prevent OPSI.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 796

Splenomegaly

Overview

Splenomegaly is enlargement of the spleen beyond its normal size (normally approximately 12 cm in length).

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Early symptoms

- Mild splenomegaly may be asymptomatic, while massive splenomegaly causes early satiety, left upper quadrant pain, and discomfort, with a firm, smooth spleen on palpation.

Common symptoms

- Your symptoms depend on the degree of enlargement
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of splenomegaly require immediate medical evaluation

Progression and Stages

It has numerous causes classified as congestive (portal high blood pressure, heart failure, liver vein blood clot formation), infectious (malaria, infectious mononucleosis, infective endocarditis, visceral leishmaniasis, tuberculosis), hematologic (hemolytic anemias, myeloproliferative disorders, lymphomas, leukemias, myelofibrosis), inflammatory/autoimmune (sarcoidosis, Felty syndrome, SLE, rheumatoid arthritis), and infiltrative (Gaucher disease, Niemann-Pick disease, amyloidosis). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves identifying the underlying cause through blood tests, imaging (ultrasound, CT), and bone marrow biopsy when indicated. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves directed at the underlying condition. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 797

Spontaneous Abortion (Miscarriage)

Overview

Spontaneous abortion, or miscarriage, is the loss of a pregnancy before 20 weeks, occurring in 10–20% of clinically recognized pregnancies. Approximately 80% occur in the first trimester. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

The most common cause (50–60%) is chromosomal abnormalities, most often autosomal trisomies.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience vaginal bleeding with or without cramping, passage of tissue, and cervical dilation. Ultrasound evaluation determines type: threatened, inevitable, incomplete, complete, missed, or anembryonic pregnancy. Management options include expectant management, medical management (misoprostol), or surgical evacuation (dilation and curettage) for hemodynamic instability, infection, or incomplete evacuation.

Emergency warning signs

- Severe or worsening symptoms of spontaneous abortion (miscarriage) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- In most cases, the outlook is favorable
- Most early miscarriages are isolated events.

Chapter 798

Squamous Cell Carcinoma

Overview

Cutaneous squamous cell carcinoma (cSCC) is the second most common skin cancer, arising from keratinocytes in the epidermis. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of UV-induced DNA damage and immunosuppression leading to cancerous transformation of keratinocytes. cSCC is more aggressive than BCC, with metastatic potential (2–5% for primary lesions, higher for recurrent or immunosuppressed patients). Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)

- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Clinical features vary from an erythematous, scaly plaque to a firm, ulcerated nodule. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of squamous cell carcinoma require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph

node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment options include surgical removal, Mohs surgery for high-risk lesions, and radiation therapy for inoperable cases
- Cemiplimab (anti-PD-1) is approved for advanced cSCC.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this based on biopsy. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status

- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

- In most cases, the outlook is favorable but worse than BCC
- Metastatic risk is higher in immunocompromised patients.

Chapter 799

Stargardt Disease

Overview

- Stargardt disease is the most common inherited macular dystrophy, affecting the central retina and causing progressive central vision loss
- It typically begins in childhood or young adulthood
- It is a form of juvenile macular degeneration caused by a genetic defect in the ABCA4 gene

Causes

This condition happens because of mutations in the ABCA4 gene, which encodes a protein that clears toxic byproducts (A2E) from photoreceptors. When the protein fails, a toxic substance accumulates in the retinal pigment epithelium, damaging and eventually destroying the photoreceptors in the macula. It is inherited in an autosomal recessive pattern in most cases.

Risk Factors

- Family history of Stargardt disease (autosomal recessive inheritance)
- Having two parents who carry the ABCA4 mutation
- Consanguinity increases the chance of inheriting the disease
- Sun exposure may accelerate damage
- Vitamin A supplementation may worsen the disease

Signs and Symptoms

Common symptoms

- Progressive blurring of central vision in childhood or young adulthood
- Difficulty reading, recognizing faces, and seeing fine detail
- Central scotomas (blind spots)
- Color vision disturbances
- Light sensitivity and prolonged adaptation to dim light
- Peripheral vision usually remains preserved

Emergency warning signs

- Rapidly worsening central vision warrants evaluation to confirm the diagnosis and plan follow-up

Progression and Stages

Vision loss typically begins between ages 6 and 30 and progresses gradually over years to decades. Visual acuity may stabilize at a moderate level of impairment (often 20/200 to 20/400) in adulthood. The disease spares the peripheral retina, so functional peripheral vision and mobility are usually retained.

Complications

- Severe permanent central vision loss
- Difficulty with reading, writing, and other near tasks
- Impaired driving and education challenges
- Psychological impact and reduced independence
- Accelerated progression with supplemental vitamin A

Diagnosis

- Fundus examination showing characteristic flecks and atrophy in the macula (fundus flavimaculatus)
- Optical coherence tomography (OCT) showing retinal pigment epithelium changes
- Fundus autofluorescence showing abnormal accumulation of lipofuscin
- Electroretinography (ERG) to assess photoreceptor function
- Genetic testing confirming ABCA4 mutations

Prevention

- Genetic counseling for affected families
- Avoiding vitamin A supplementation (which can worsen disease)
- Protecting the eyes with UV-blocking sunglasses
- Prenatal testing in at-risk families
- Participation in registries and research studies

Treatment Options

- There is currently no cure
- Gene therapy for ABCA4 mutations is under investigation in clinical trials
- Stem cell and retinal transplantation research is ongoing
- Low-vision aids and rehabilitation

- UV-blocking sunglasses and avoidance of excessive light
- Careful dietary counseling regarding vitamin A

Living with It

- Work with low-vision services for magnification aids and training
- Use screen-reading and magnification technology
- Join support groups and patient communities
- Maintain education and employment with appropriate accommodations
- Follow research developments in emerging therapies

Outlook

Stargardt disease is progressive, but the rate and final severity vary widely. Most people retain functional peripheral vision and independence, though central vision loss is typically significant. Research into gene and cell therapies offers cautious hope for future treatment.

Chapter 800

Stevens-Johnson Syndrome and Toxic Epidermal Necrolysis

Overview

Stevens-Johnson syndrome and toxic epidermal necrolysis are severe, life-threatening mucocutaneous reactions characterized by extensive keratinocyte apoptosis and epidermal detachment, with SJS affecting <10% of body surface area, SJS/TEN overlap affecting 10–30%, and TEN affecting >30% BSA. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

The most common cause is drug hypersensitivity (anticonvulsants [carbamazepine, lamotrigine, phenytoin], allopurinol, sulfonamides, NSAIDs, antibiotics). Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience prodromal fever and malaise, followed by painful, dusky red to purpuric macules that coalesce and progress to epidermal detachment with flaccid blisters and erosion, with mucosal involvement (oral, ocular, genital) present in over 90%.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of stevens-johnson syndrome and toxic epidermal necrolysis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Management requires burn unit or ICU care: fluid resuscitation, wound care, nutritional support, ophthalmological consultation, pain management, and withdrawal of the offending drug, with cyclosporine and IVIG having shown some benefit.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is guarded, with mortality of 10–30% for SJS and up to 50% for TEN.

Chapter 801

Stillbirth (Intrauterine Fetal Demise)

Overview

Stillbirth is defined as fetal death at ≥ 20 weeks (or ≥ 28 weeks by WHO definition), occurring in 1 in 160 deliveries in the United States. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Causes include placental abruption, umbilical cord accidents, intrauterine infection, placental insufficiency, congenital anomalies, and maternal conditions. A cause is identified in 60–80% of cases with thorough evaluation including fetal autopsy, placental pathology, and karyotype.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of stillbirth (intrauterine fetal demise) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Management after diagnosis is induction of labor
- Psychological support and counseling are critical components of care.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 802

Stimulant Overdose (Cocaine, Methamphetamine)

Overview

- Cocaine is a dopamine, norepinephrine, and serotonin reuptake inhibitor
- Methamphetamine promotes release of these monoamines. Both produce a sympathetic storm: high blood pressure, tachycardia, mydriasis, hyperthermia, psychomotor agitation, psychosis, coronary vasospasm, heart attack, aortic dissection, stroke, rhabdomyolysis, acute kidney injury, and seizures. Cocaine also has sodium channel blockade causing QRS widening. Hyperthermia is the strongest predictor of mortality. Treatment: benzodiazepines (high-dose IV) for agitation and sympathetic hyperactivity, active cooling for hyperthermia, calcium channel blockers or phenolamine for refractory high blood pressure, and nitroglycerin for cocaine-induced coronary vasospasm. Beta-blockers should be avoided in acute cocaine toxicity due to unopposed alpha-adrenergic stimulation.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of stimulant overdose (cocaine, methamphetamine) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention

when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 803

Stimulant Use Disorder (Cocaine, Amphetamines)

Overview

Stimulant use disorder affects approximately 0.5–1% of the population for cocaine and 0.2–0.4% for amphetamine-type stimulants.

Causes

This condition happens because of cocaine blocking dopamine, norepinephrine, and serotonin transporters, and amphetamines additionally promoting neurotransmitter release from presynaptic vesicles, with chronic use leading to dopamine depletion, neurotoxicity, and extensive structural brain changes.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience a problematic pattern of stimulant use with criteria analogous to other substance use disorders. Withdrawal is marked by dysphoria, anhedonia, fatigue, hypersomnia, hyperphagia, and intense cravings.

Emergency warning signs

- Severe or worsening symptoms of stimulant use disorder (cocaine, amphetamines) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Medical complications include acute coronary syndrome, heart attack, involving bleeding stroke, rhabdomyolysis, seizures, high blood pressure, and for methamphetamine, severe dental decay (meth mouth), lung high blood pressure, and psychosis.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using clinical interview. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include behavioral interventions as first-line (CBT, contingency management), with no FDA-approved medications for stimulant use disorder. outlook: chronic relapsing course

- Methamphetamine use disorder has particularly high relapse rates.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 804

Strabismus

Overview

Strabismus is a misalignment of the visual axes of the two eyes that may be horizontal (esotropia, exotropia) or vertical (hypertropia, hypotropia), and may be constant or intermittent, comitant or non-comitant.

Causes

This condition happens because of neuromuscular imbalance of the extraocular muscles

- Onset in childhood may be accommodative, paralytic, or idiopathic
- In adults, acquired strabismus may result from cranial nerve palsies (III, IV, VI), thyroid eye disease, myasthenia gravis, or orbital pathology.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience visible eye misalignment, which can cause amblyopia in children and diplopia in adults.

Emergency warning signs

- Severe or worsening symptoms of strabismus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on cover testing and ocular motility examination. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include corrective lenses, prisms, occlusion therapy, botulinum toxin injection, and surgical realignment of the extraocular muscles. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is good with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 805

Subdural Hematoma

Overview

- A subdural hematoma is a traumatic intracranial bleeding accumulating in the potential space between the dura mater and arachnoid mater, resulting from tearing of bridging veins. Acute subdural hematomas follow high-impact trauma
- Chronic subdural hematomas occur predominantly in elderly patients and alcoholics, where cerebral shrinkage or wasting stretches bridging veins, making them susceptible to rupture from minor head trauma.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience headache, confusion, fluctuating consciousness, and weakness, numbness, or problems with movement, developing over hours in acute cases or weeks/months in chronic cases.

Emergency warning signs

- Severe or worsening symptoms of subdural hematoma require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors confirm the diagnosis through non-contrast head CT: acute SDH appears crescent-shaped hyperdense
- Chronic SDH appears hypodense
- SDH crosses suture lines but does not cross dural reflections (falx/tentorium). Treatment for large or symptomatic hematomas involves surgical evacuation (burr holes or surgical opening of the skull)
- Small asymptomatic SDHs are managed conservatively.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on underlying brain injury. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 806

Sudden Sensorineural Hearing Loss

Overview

Sudden sensorineural hearing loss is an ear emergency defined as a loss of at least 30 dB over three contiguous audiometric frequencies within 72 hours.

Causes

This condition happens because of idiopathic causes, although viral cochleitis, vascular occlusion, autoimmune inner ear disease, or perilymphatic fistula may be implicated.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience abrupt unilateral hearing loss, often accompanied by ringing in the ears, feeling of spinning, and aural fullness.

Emergency warning signs

- Severe or worsening symptoms of sudden sensorineural hearing loss require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on audiometry. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on high-dose oral or intratympanic corticosteroids, with prompt treatment within 2 weeks being crucial
- Hyperbaric oxygen therapy may provide additional benefit.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with some cases recovering spontaneously and others sustaining permanent hearing loss. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 807

Supraventricular Tachycardia

Overview

Supraventricular tachycardia (SVT) is a paroxysmal arrhythmia characterized by a sudden onset of rapid heart rate originating above the ventricles, typically between 150–250 beats per minute. It results from abnormal electrical pathways or reentrant circuits involving the atrioventricular (AV) node, atrial tissue, or accessory conduction pathways. While rarely life-threatening, SVT episodes can cause significant discomfort, anxiety, and impairment of daily function.

Classification

Cause Idiopathic or structural

Organ System Heart

Pathophysiology Reentrant circuit or abnormal automaticity in atrial tissue or AV node producing paroxysmal tachycardia

Duration Acute (paroxysmal episodes lasting seconds to hours)

Onset Sudden

Mode of Transmission Non-communicable

Clinical Course Episodic with abrupt onset and termination

Severity Mild to Moderate

Extent of Disease Localized (heart conduction system)

Age Group All ages; more common in young adults and women

Epidemiology Prevalence approximately 2.25 per 1000 in the general population

ICD-11 Code BC94.1

Causes

This condition results from disruption of normal supraventricular tachycardia function.

Risk Factors

- Female sex (higher incidence of AVNRT)
- Preexcitation syndromes such as Wolff-Parkinson-White (WPW) pattern

- Anxiety, stress, or panic attacks
- Caffeine, alcohol, or stimulant use
- Structural heart disease (e.g., congenital heart disease, valvular disease)

Signs and Symptoms

Early symptoms

- Early manifestations of supraventricular tachycardia may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
 - Palpitations or racing heartbeat with abrupt onset and termination
 - Lightheadedness or dizziness during episodes
 - Shortness of breath
 - Chest fluttering or discomfort
 - Fatigue or weakness following episodes
 - Near-syncope or syncope in some cases
 - Anxiety or sense of impending doom
 - Polyuria (due to atrial natriuretic peptide release)
 - Pain or discomfort in the affected area
 - Difficulty performing daily activities

Less common symptoms

- Atypical or less common presentations of supraventricular tachycardia may occur

Emergency warning signs

- Severe or worsening symptoms of supraventricular tachycardia require immediate medical evaluation

Progression and Stages

Episodes of SVT typically begin suddenly and can last from a few seconds to several hours before terminating spontaneously or with intervention. Frequency and duration of episodes vary widely among individuals, with some experiencing rare isolated events and others having frequent daily episodes that significantly affect quality of life. Over time, very frequent or prolonged episodes can sometimes lead to tachycardia-induced cardiomyopathy with reduced left ventricular function.

Complications

- Tachycardia-induced cardiomyopathy from frequent or sustained episodes
- Syncope or near-syncope during rapid episodes
- Preexcited atrial fibrillation in WPW patients with risk of degeneration to ventricular fibrillation
- Reduced quality of life due to recurrent symptoms
- Emotional and psychological effects

Diagnosis

- Electrocardiogram (ECG) during an episode showing narrow-complex tachycardia
- Holter monitor or event recorder for episodic capture
- Electrophysiology study for definitive diagnosis and ablation planning
- Echocardiogram to evaluate for structural heart disease
- Lab tests to rule out thyroid disorders and electrolyte abnormalities
- Imaging tests (rarely needed beyond echocardiogram)

Prevention

- Avoid known triggers such as caffeine, alcohol, and stimulants
- Manage stress through relaxation and mindfulness techniques
- Maintain adequate hydration and electrolyte balance
- Catheter ablation as curative therapy for recurrent SVT
- Vagal maneuvers taught for acute episode termination
- Go for regular check-ups
- Follow your doctor's screening recommendations

Treatment Options

- Vagal maneuvers (Valsalva, carotid sinus massage) for acute termination
- Adenosine intravenous bolus for acute episode conversion
- Calcium channel blockers or beta-blockers for rate control and prevention
- Catheter ablation (radiofrequency or cryoablation) for definitive treatment
- Lifestyle changes and self-care
- Surgery when needed

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups

- Keep learning about your condition

Outlook

The prognosis for SVT is excellent, as it is rarely life-threatening and can often be cured definitively with catheter ablation (success rate exceeding 95% in most cases). Patients with managed or treated SVT can expect a normal lifespan and quality of life with minimal restrictions.

Chapter 808

Syphilis (*Treponema pallidum*)

Overview

Syphilis is a sexually transmitted infection caused by the spirochete *Treponema pallidum* subspecies *pallidum*, with 8 million new cases globally per year, with incidence rising in high-income countries, particularly among men who have sex with men (MSM). This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of *T. pallidum* penetrating mucous membranes or abraded skin through sexual contact or transplacentally, with the organism rapidly disseminating hematogenously and lymphatically. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Congenital syphilis includes stillbirth, rhinitis (“snuffles”), rash, osteochondritis, and Hutchinson’s triad. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of syphilis (*treponema pallidum*) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- Clinical features occur in four stages: Primary (3–90 days): painless chancre (ulcer with clean base, firm, indurated margins) at the site of inoculation, resolving spontaneously in 3–6 weeks
- Secondary (2–8 weeks after primary): disseminated rash (non-pruritic, maculopapular,

involving palms and soles), condylomata lata, generalized lymphadenopathy, fever, malaise, and mucocutaneous lesions

- Latent: asymptomatic
- Tertiary (10–30 years): gummatous disease, cardiovascular syphilis (aortitis, aortic regurgitation), and neurosyphilis (general paresis, tabes dorsalis).

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using dark-field microscopy and serology (non-treponemal tests for screening, treponemal tests for confirmation). Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal

protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment typically involves benzathine penicillin G (IM) for primary, secondary, and early latent syphilis, with neurosyphilis requiring aqueous crystalline penicillin G IV. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others

- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most people recover well with appropriate treatment. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 809

Systemic Lupus Erythematosus

Overview

Systemic lupus erythematosus is a chronic, multisystem autoimmune disease characterized by loss of tolerance to self-antigens and production of a wide array of autoantibodies, leading to immune complex deposition and inflammation in multiple organs, predominantly affecting women of childbearing age (9:1 female predominance) and being more common in African Americans, Hispanics, and Asians. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of autoantibody production and immune complex deposition. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking

- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Clinical features are protean and include malar rash, photosensitivity, oral ulcers, arthritis, serositis (pleuritis, pericarditis), nephritis (lupus nephritis—the most serious manifestation), cytopenias, seizures, and psychosis. Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of systemic lupus erythematosus require immediate medical evaluation

Progression and Stages

The 2019 EULAR/ACR classification criteria require an ANA titer $\geq 1:80$ plus weighted criteria across clinical and immunological domains. Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation

- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Diagnosis typically involves antinuclear antibody (ANA, sensitive but not specific), anti-dsDNA (specific, correlates with nephritis), anti-Smith (highly specific), complement levels (C3, C4 decreased during flares), and kidney biopsy for lupus nephritis classification (ISN/RPS). Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Treatment options include hydroxychloroquine (cornerstone for all patients, reduces flares and mortality), corticosteroids, and immunosuppressive agents (mycophenolate,

azathioprine, cyclophosphamide for severe organ involvement), with newer targeted therapies including belimumab (anti-BLys/BAFF monoclonal antibody), voclosporin, and anifrolumab (anti-type I interferon receptor). Treatment aims to control inflammation, manage symptoms, and prevent disease flares. A step-up approach is often used, starting with symptomatic treatments and progressing to immunosuppressive therapies as needed. Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

The outlook varies from person to person, with improved outcomes due to early diagnosis and better management. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 810

Systemic Lupus Erythematosus (Skin Manifestations)

Overview

Cutaneous lupus erythematosus (CLE) encompasses a spectrum of skin involvement in systemic lupus erythematosus (SLE) or as a primary skin-limited condition. Acute cutaneous lupus presents as a malar (butterfly) rash across the cheeks and bridge of the nose, sparing the nasolabial folds, and is linked to active systemic disease. Subacute cutaneous lupus presents as annular polycyclic or papulosquamous plaques on sun-exposed areas, strongly associated with anti-Ro/SSA antibodies. Chronic cutaneous lupus (discoid lupus) presents as well-demarcated, scaly, erythematous plaques with follicular plugging and atrophic scarring, predominantly on the scalp, face, and ears. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of autoimmune-mediated skin inflammation triggered by UV exposure. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of systemic lupus erythematosus (skin manifestations) require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Doctors diagnose this based on clinical appearance, skin biopsy, and serological testing (ANA, anti-Ro/SSA, anti-dsDNA). Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Treatment options include sun protection, topical corticosteroids, antimalarials (hydroxychloroquine, first-line), and for refractory disease, immunosuppressants (methotrexate, mycophenolate, belimumab). Treatment aims to control inflammation, manage symptoms, and prevent disease flares. A step-up approach is often used, starting with symptomatic treatments and progressing to immunosuppressive therapies as needed. Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief Corticosteroids for rapid control of inflammation Disease-modifying antirheumatic drugs (DMARDs) Biologic therapies targeting specific immune pathways Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide) Antimalarial drugs (hydroxychloroquine) for mild disease Physical and occupational therapy to maintain function Pain management for chronic pain Lifestyle modifications including diet and stress reduction Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

Your outlook depends on the subtype and systemic involvement. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 811

Systemic Sclerosis (Scleroderma)

Overview

Systemic sclerosis is a chronic autoimmune connective tissue disease characterized by widespread vasculopathy, immune dysregulation, and progressive scarring of the skin and internal organs, with an incidence of approximately 1–2 per 100,000, a female-to-male ratio of 4–6:1, and peaking in the fifth decade. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of excessive collagen deposition by activated fibroblasts, driven by endothelial injury, autoimmune inflammation, and transforming growth factor-beta (TGF- β) signaling. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking

- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Early symptoms

- You may experience Raynaud's phenomenon (the most frequent initial manifestation, present in over 95%), and three clinical variants exist: limited cutaneous SSc (lcSSc, skin thickening distal to elbows/knees, formerly CREST syndrome: Calcinosis cutis, Raynaud's phenomenon, Esophageal dysmotility, Sclerodactyly, Telangiectasias), diffuse cutaneous SSc (dcSSc, skin thickening proximal to elbows/knees and trunk, more rapid and severe internal organ involvement), and systemic sclerosis sine scleroderma (Raynaud's phenomenon and internal organ involvement without clinically apparent skin scarring). lung involvement is the leading cause of death: lung scarring (more common in dcSSc) presents with shortness of breath, cough, and restrictive lung disease

Common symptoms

- Lung high blood pressure (more common in lcSSc) presents with exertional shortness of breath, right heart failure, and carries a poor outlook. Other features include gastroesophageal reflux, gastric antral vascular ectasia (watermelon stomach), telangiectasias, and myositis.
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of systemic sclerosis (scleroderma) require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ

dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

Scleroderma kidney crisis (SRC) is a life-threatening complication of dcSSc (incidence 5–10%), characterized by cancerous high blood pressure, rapidly progressive kidney failure, microangiopathic hemolytic anemia, and thrombocytopenia.

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by autoantibodies (anti-centromere in lcSSc, anti-Scl-70/anti-topoisomerase I in dcSSc, anti-RNA polymerase III associated with SRC risk), nailfold capillaroscopy, and organ-specific testing (PFTs, echocardiogram, high-resolution CT chest). Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Treatment typically involves organ-specific and multidisciplinary: calcium channel blockers, prostacyclin analogs, and endothelin receptor antagonists for Raynaud's phenomenon and lung high blood pressure

- Proton pump inhibitors and prokinetics for GI involvement
- ACE inhibitors for scleroderma kidney crisis (first-line, which has transformed outcomes—SRC mortality has decreased from 80% to 15–20%)
- Mycophenolate, cyclophosphamide, and nintedanib for interstitial lung disease
- And methotrexate for skin and joint involvement.
- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

The outlook varies from person to person, with the diffuse cutaneous variant having a worse outlook, and lung involvement being the leading cause of death. Autoimmune conditions

are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 812

Takayasu Arteritis

Overview

Takayasu arteritis is a chronic, granulomatous large-vessel vasculitis affecting the aorta and its major branches, predominantly in young women, with an incidence of 1–2 per million, a 9:1 female predominance, typically presenting between 10–40 years of age, and being more common in Asia, India, and Latin America.

Causes

This condition happens because of Th1-mediated granulomatous inflammation, intimal thickening, scarring, and stenosis of large elastic arteries, with skip lesions being characteristic.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of takayasu arteritis require immediate medical evaluation

Progression and Stages

Clinical features occur in two phases: an early systemic phase (non-specific constitutional symptoms: fever, malaise, weight loss, night sweats, arthralgias) and a late occlusive phase (claudication of extremities, absence or asymmetry of pulses—pulseless disease, blood pressure differences between arms, vascular bruits, syncope, stroke, visual disturbances, renovascular high blood pressure, and aortic regurgitation). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on the 1990 ACR criteria, with imaging (conventional angiography, CT angiography, or MR angiography) showing stenosis, occlusion, or aneurysm of the aorta and its branches, and PET-CT showing vessel wall FDG avidity in active disease. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include high-dose corticosteroids as first-line therapy, with methotrexate, azathioprine, mycophenolate, TNF inhibitors, and tocilizumab used as steroid-sparing agents or for refractory disease. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Disease is chronic with relapses requiring long-term immunosuppression.

Chapter 813

Tapeworm Infections (Taeniasis / Cysticercosis)

Overview

This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

- This condition happens because of taeniasis: ingestion of raw or undercooked meat containing cysticerci, with the scolex attaching to the intestinal mucosa
- Cysticercosis: ingestion of *T. solium* eggs, with oncospheres penetrating the intestinal wall, disseminating hematogenously, and encysting in tissues.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water

- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience taeniasis: usually asymptomatic or mild abdominal pain and passage of proglottids
- Cysticercosis: neurocysticercosis—seizures (most common presentation), headache, focal nervous system deficits, increased intracranial pressure, and hydrocephalus.
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of tapeworm infections (taeniasis / cysticercosis) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- Taeniasis is intestinal infection with adult tapeworms (*Taenia saginata*—beef tapeworm
- *Taenia solium*—pork tapeworm
- *Diphyllobothrium latum*—fish tapeworm), and cysticercosis is tissue infection with the larval stage of *T. solium*, a leading cause of acquired epilepsy in endemic areas, with taeniasis affecting 50 million people and neurocysticercosis affecting 2.5 million.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

- To diagnose taeniasis is by macroscopic or microscopic identification of proglottids in stool
- To diagnose cysticercosis is by neuroimaging showing ring-enhancing cysts.
- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures

- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include praziquantel or niclosamide for taeniasis, and albendazole plus corticosteroids for neurocysticercosis. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- Most people recover well for taeniasis
- Neurocysticercosis Your outlook depends on cyst burden and location.

Chapter 814

Temporal Lobe Epilepsy

Overview

Temporal lobe epilepsy is the most common form of focal epilepsy in adults, often arising from mesial temporal sclerosis (hippocampal sclerosis). This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of hippocampal sclerosis, low-grade gliomas, focal cortical dysplasia, or dual pathology. Seizures typically begin with an aura (*déjà vu*, epigastric rising, fear, olfactory or gustatory hallucinations), followed by impaired awareness, automatisms (lip smacking, hand fumbling), and postictal confusion. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience focal seizures with impaired awareness. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of temporal lobe epilepsy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on EEG showing temporal interictal epileptiform discharges and MRI revealing hippocampal shrinkage or wasting and T2/FLAIR signal changes in mesial temporal sclerosis. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on antiseizure medications
- Drug resistance is common, and temporal lobe epilepsy is the most surgically amenable epilepsy, with anterior temporal lobectomy achieving seizure freedom in 60–80% of selected patients.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with surgical treatment for drug-resistant cases. Neurological conditions vary widely in their course and outcomes. Some are progressive, while others follow a relapsing-remitting pattern. Early intervention and rehabilitation can improve outcomes. Neurological conditions vary significantly in their course, from acute and self-limited to chronic and progressive. Early diagnosis and intervention can slow progression and improve quality of life. Rehabilitation and supportive therapies play crucial roles in maintaining function. Research continues to develop disease-modifying treatments for previously untreatable conditions. Multidisciplinary care addressing medical, functional, and psychosocial needs optimizes outcomes.

Chapter 815

Temporomandibular Joint Disorders

Overview

Temporomandibular joint disorders (TMD/TMJ) encompass a group of conditions affecting the temporomandibular joint, masticatory muscles, and associated structures. Prevalence is 5–12%, with a female predominance (4:1) and peak incidence in reproductive years.

Causes

This condition happens because of multifactorial causes including occlusal factors, para-functional habits, trauma, psychosocial stress, and genetic predisposition.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience jaw pain, clicking or crepitus, limited mouth opening, locking, headache, and ear pain.

Emergency warning signs

- Severe or worsening symptoms of temporomandibular joint disorders require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam with palpation of TMJ and muscles and range of motion assessment
- Imaging is reserved for atypical presentations. Conservative Treatment typically involves first-line: patient education, self-management, physical therapy, occlusal splints, and NSAIDs
- For persistent pain, TCAs, muscle relaxants, trigger point injections, or botulinum toxin may be used.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with conservative management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 816

Tension-Type Headache

Overview

Tension-type headache (TTH) is a primary headache disorder and the most prevalent headache disorder, with a lifetime prevalence of 30–78%. It is more common in women and peaks in the 30–40 age range. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition happens because of peripheral myofascial pain mechanisms in episodic TTH and central sensitization in chronic TTH, with muscle tenderness (pericranial, cervical, shoulder girdle) as a prominent feature. Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience bilateral, pressing/tightening, non-pulsating headache of mild to moderate intensity, lasting 30 minutes to 7 days, not aggravated by routine physical activity, and not associated with nausea or vomiting. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of tension-type headache require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam based on ICHD-3 criteria. Acute Treatment options include simple analgesics (ibuprofen, naproxen, acetaminophen, aspirin). Preventive therapy for chronic TTH includes amitriptyline, mirtazapine, or venlafaxine
- Non-pharmacological approaches include physical therapy, relaxation training, biofeedback, and thinking and memory behavioral therapy.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function

- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive
- Episodic TTH typically resolves with treatment.

Chapter 817

Testicular Cancer

Overview

Testicular cancer is the most common solid tumor in young men (peak age 15–40) and is highly curable. The vast majority (95%) are germ cell tumors (GCTs): seminoma (40%) and non-seminomatous germ cell tumors (NSGCT, 60%)—embryonal carcinoma, yolk sac tumor, choriocarcinoma, teratoma, and mixed germ cell tumors. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of germ cells. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)

- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Clinical features typically include a painless testicular mass, though some present with scrotal pain, hydrocele, or symptoms of metastatic disease. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of testicular cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional

lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

- Treatment for stage I seminoma involves surveillance or adjuvant carboplatin
- Advanced disease doctors typically treat it with cisplatin-based chemotherapy (BEP—bleomycin, etoposide, cisplatin) with surgery for residual masses.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Diagnosis typically involves scrotal ultrasound (hypoechoic intratesticular mass), serum tumor markers (AFP, β -hCG, LDH), and CT abdomen/pelvis/chest for staging. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)

- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies

- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Most people recover well, with cure rates exceeding 95% for seminoma and 80–90% for NSGCT. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 818

Tetanus (*Clostridium tetani*)

Overview

This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of spores germinating in anaerobic wound conditions, producing tetanospasmin (a zinc-dependent metalloproteinase) that binds to presynaptic nerve terminals, is transported retrogradely to the CNS, and cleaves synaptobrevin-2 (VAMP), preventing release of inhibitory neurotransmitters (GABA, glycine) from Renshaw cells, producing unopposed motor neuron activity. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience an incubation period of 3–21 days, with trismus (lockjaw) being the earliest sign in 75% of cases, followed by risus sardonicus, opisthotonos, generalized muscle rigidity, and painful spasms triggered by minimal stimuli, with autonomic instability occurring in severe cases. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of tetanus (*clostridium tetani*) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Tetanus is a toxin-mediated disease caused by *Clostridium tetani*, a spore-forming, obligate anaerobic bacillus found in soil and animal feces, with approximately 200,000 cases annually worldwide, primarily in low-income countries with inadequate vaccination, and neonatal tetanus accounting for 50% of tetanus deaths. Treatment options include human tetanus immune globulin (HTIG) to neutralize unbound toxin, wound removal of dead tissue, metronidazole, and supportive care (benzodiazepines for spasms, magnesium sulfate for autonomic instability). outlook: mortality remains 10–60% in severe cases, but prevention is effective through tetanus toxoid vaccination. Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food

handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 819

Thalassemia

Overview

Thalassemia is a group of inherited hemoglobin disorders characterized by reduced or absent synthesis of α - or β -globin chains, leading to ineffective erythropoiesis and hemolytic anemia. β -Thalassemia major (Cooley's anemia, homozygous β^0 mutations) presents in the first year of life with severe anemia, failure to thrive, hepatosplenomegaly, and skeletal deformities ("hair-on-end" appearance on skull X-ray from marrow expansion). β -Thalassemia intermedia has milder anemia and may not require regular transfusions. α -Thalassemia major (Hb Bart's hydrops fetalis, deletion of all four α -globin genes) is uniformly fatal in utero. α -Thalassemia trait (deletion of two genes) causes mild microcytic anemia. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of thalassemia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using CBC, hemoglobin electrophoresis, and genetic testing.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment for β -thalassemia major requires lifelong transfusion therapy and iron chelation (deferasirox, deferoxamine, deferiprone) to prevent transfusional iron overload, which causes cardiomyopathy, liver cirrhosis, and endocrinopathies.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupa-

tional therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has improved with stem cell transplantation (curative for β -thalassemia major) and gene therapy (betibeglogene autotemcel).

Chapter 820

Theophylline Overdose

Overview

Theophylline overdose is a pharmaceutical overdose causing severe neurological and cardiovascular toxicity. Theophylline is a phosphodiesterase and adenosine receptor antagonist, leading to increased catecholamine release and beta-adrenergic stimulation. This condition results from acute injury or exposure to harmful agents.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features: acute toxicity presents with nausea, vomiting, hypokalemia, hypophosphatemia, hyperglycemia, metabolic acidosis, tremor, agitation, and seizures (often refractory)
- Cardiovascular effects include sinus tachycardia, atrial fibrillation, ventricular tachycardia, and hypotension.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of theophylline overdose require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using serum theophylline level. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include multidose activated charcoal (MDAC, 0.5 g/kg every 2–4 hours), beta-blockers for hemodynamically significant tachycardia, and hemodialysis for severe toxicity. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 821

Thrombotic Thrombocytopenic Purpura

Overview

Thrombotic thrombocytopenic purple spots on the skin (TTP) is a rare, life-threatening thrombotic microangiopathy characterized by microvascular blood clot formation, thrombocytopenia, and microangiopathic hemolytic anemia, with an incidence of 3–4 per million.

Causes

This condition happens because of severe deficiency of ADAMTS13, a von Willebrand factor (VWF) cleaving protease: immune-mediated (acquired TTP, IgG autoantibodies, adulthood) or congenital (Upshaw-Schulman syndrome, ADAMTS13 gene mutations, childhood). ADAMTS13 deficiency leads to accumulation of ultralarge VWF multimers, causing spontaneous platelet adhesion and microthrombi formation in arterioles and capillaries.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the classic pentad: thrombocytopenia (tiny red or purple spots, purple spots on the skin, bleeding), microangiopathic hemolytic anemia (fever, pallor, jaundice), nervous system involvement (headache, confusion, seizures, focal deficits, coma), kidney involvement (elevated creatinine, hematuria), and fever. Not all five

features are present simultaneously

- Thrombocytopenia and MAHA are sufficient to suspect TTP.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of thrombotic thrombocytopenic purpura require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on peripheral blood smear showing schistocytes (helmet cells, fragmented RBCs), elevated LDH, low haptoglobin, negative direct Coombs test, and ADAMTS13 activity $<10\%$. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Treatment typically involves a medical emergency: urgent plasma exchange (PLEX, 1–1.5 plasma volume daily) is the cornerstone of therapy, combined with high-dose corticosteroids (prednisone 1 mg/kg). For refractory/relapsed immune TTP: rituximab, caplacizumab (anti-VWF nanobody, blocks platelet adhesion), or immunosuppression (vincristine, cyclophosphamide). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook has improved dramatically: mortality declined from >90% to 10–20% with prompt PLEX. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 822

Thyroid Nodules and Cancer

Overview

Thyroid nodules are extremely common, palpable in 4–7% of adults and incidentally discovered on imaging in up to 50%, with the vast majority (90–95%) being non-cancerous. Thyroid cancer accounts for approximately 58,000 new cases annually in the US, with papillary thyroid carcinoma being the most common histological type (80%, with excellent outlook and 20-year survival >95%), followed by follicular thyroid carcinoma (10–15%), medullary thyroid carcinoma (5%, arising from parafollicular C cells and producing calcitonin, associated with MEN2), and anaplastic thyroid carcinoma (<1%, highly aggressive and usually fatal). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of neoplastic transformation of thyroid follicular or parafollicular cells. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to

cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)
- Family history
- Poor dietary habits
- Hormonal imbalances
- Certain medications
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience a palpable thyroid nodule, sometimes with compressive symptoms. Treatment for differentiated thyroid cancer includes total thyroidectomy (or lobectomy for low-risk papillary cancer <4 cm), radioactive iodine ablation for intermediate/high-risk disease, and TSH suppression with levothyroxine. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness

- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of thyroid nodules and cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hor-

mone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Most people recover well for most differentiated thyroid cancers. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is

essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 823

Thyroiditis

Overview

Thyroiditis encompasses inflammatory conditions of the thyroid gland, with Hashimoto's thyroiditis being the most common cause of hypothyroidism in iodine-sufficient regions. Subacute (de Quervain's) thyroiditis is a painful, granulomatous thyroiditis thought to be viral in cause, presenting with thyroid pain, tenderness, fever, and thyrotoxicosis followed by hypothyroidism and eventual recovery. Silent (painless) thyroiditis and postpartum thyroiditis are autoimmune conditions presenting with transient thyrotoxicosis followed by hypothyroidism. Acute suppurative thyroiditis is a rare bacterial infection. Drug-induced thyroiditis may be caused by amiodarone, lithium, interferon-alpha, and immune checkpoint inhibitors. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

This condition happens because of inflammation of the thyroid gland. In autoimmune conditions, a combination of genetic susceptibility and environmental triggers initiates an inappropriate immune response against self-antigens. This leads to chronic inflammation and tissue damage in affected organs. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms. A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)
- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- Symptoms vary depending on type. Fatigue and general malaise Joint pain, swelling, and stiffness Skin rashes and lesions Low-grade fever Muscle weakness and pain Organ-specific symptoms based on affected system Weight changes (loss or gain) Dry eyes and dry mouth (Sjogren syndrome) Raynaud phenomenon (cold-induced color changes in fingers) Fluctuating symptoms with periods of flare and remission
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of thyroiditis require immediate medical evaluation

Progression and Stages

Management varies by type: NSAIDs or corticosteroids for subacute thyroiditis, levothyroxine for hypothyroid phases, beta-blockers for symptomatic thyrotoxicosis, and observation for most self-limited forms. Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension
- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Doctors diagnose this based on clinical presentation, thyroid function tests, and imaging. Diagnosis is based on a combination of clinical features, laboratory findings (autoantibodies, inflammatory markers), and imaging studies. Classification criteria help standardize the diagnostic process. Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
Corticosteroids for rapid control of inflammation
Disease-modifying antirheumatic drugs (DMARDs)
Biologic therapies targeting specific immune pathways
Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
Antimalarial drugs (hydroxychloroquine) for mild disease
Physical and occupational therapy to maintain function
Pain management for chronic pain
Lifestyle modifications including diet and stress reduction
Regular monitoring for disease activity and treatment side effects

- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)
- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

In most cases, the outlook is excellent. Autoimmune conditions are typically chronic and require long-term management. With appropriate treatment, many patients achieve good symptom control and maintain quality of life. Autoimmune diseases are generally chronic conditions requiring long-term management. Disease activity can fluctuate, requiring adjustments in treatment over time. Early diagnosis and treatment improve outcomes and prevent irreversible organ damage. Research continues to develop more targeted therapies with fewer side effects.

Chapter 824

Tinea Infections

Overview

Tinea infections (dermatophytosis) are superficial fungal infections caused by dermatophytes (*Trichophyton*, *Microsporum*, and *Epidermophyton* species) that infect keratinized tissue. Named by anatomic location: tinea pedis (athlete's foot, the most common, affecting the feet, particularly between toes), tinea corporis (ringworm of the body, annular, erythematous, scaly plaques with central clearing), tinea cruris (jock itch, affecting the groin and inner thighs), tinea capitis (scalp infection, most common in children, may cause alopecia and kerion formation), tinea unguium (onychomycosis, nail thickening, discoloration, and crumbling), and tinea versicolor (caused by *Malassezia* species, actually a pityriasis, presenting with hypo- or hyperpigmented macules on the trunk). This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system
- Travel to endemic areas
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of tinea infections require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- The right treatment depends on location and severity: topical antifungals (terbinafine, clotrimazole, miconazole) for localized tinea corporis, cruris, and pedis

- Oral antifungals (terbinafine, itraconazole, fluconazole) for tinea capitis, extensive tinea corporis, tinea unguium, and refractory cases.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this based on KOH preparation showing hyphae and spores, fungal culture, dermoscopy, or Wood's lamp examination. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or

antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Tinea capitis requires systemic therapy due to follicular involvement. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most people recover well with appropriate antifungal therapy. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 825

Tinea Versicolor (Pityriasis Versicolor)

Overview

Tinea versicolor, also known as pityriasis versicolor, is a common superficial fungal infection of the skin caused by the yeast *Malassezia globosa* (formerly *Pityrosporum orbiculare*). It is characterized by hypo- or hyperpigmented scaly patches on the trunk and proximal extremities. The condition is most prevalent in adolescents and young adults in warm, humid climates.

Classification

Cause Infectious (Fungal)

Organ System Skin

Pathophysiology Overgrowth of *Malassezia* yeast in its pathogenic mycelial form

Duration Chronic (months to years, recurrent)

Onset Gradual

Mode of Transmission Endogenous (normal skin flora); not contagious

Clinical Course Chronic, recurrent; highly responsive to treatment

Severity Mild

Extent of Disease Localized to Regional

Age Group Adolescents and Young Adults

Epidemiology Prevalence 1–50% depending on climate; highest in tropical regions

ICD-11 Code 1F2C.0

Causes

Tinea versicolor is caused by overgrowth of *Malassezia*, a lipophilic yeast that is part of normal skin flora. The yeast converts from its saprophytic yeast form to the pathogenic mycelial form under conditions of warmth, humidity, sweat, and sebum. *Malassezia* produces azelaic acid, which inhibits melanogenesis and causes the characteristic hypopigmentation. Hyperpigmented forms result from inflammation-induced melanocyte stimulation.

Risk Factors

- Warm, humid climate and excessive sweating
- Oily skin and seborrheic areas
- Adolescence and young adulthood
- Immunosuppression
- Corticosteroid use
- Malnutrition
- Genetic predisposition
- Occlusive clothing

Signs and Symptoms

Early symptoms

- Early manifestations of tinea versicolor (pityriasis versicolor) may be subtle and nonspecific

Common symptoms

- **Early symptoms**
- **Common symptoms**
- **Common symptoms:**
- Well-demarcated, round to oval macules and patches on trunk, back, chest, shoulders
- Hypopigmented (most common), pink, or hyperpigmented lesions
- Fine, branny scale that becomes apparent with gentle scraping (Besnier sign)
- Lesions are typically asymptomatic but may cause mild pruritus
- Lack of tanning in affected areas (cosmetic concern)
- Face and neck involvement in children
- Difficulty performing daily activities (cosmetic concern)

Less common symptoms

- Atypical or less common presentations of tinea versicolor (pityriasis versicolor) may occur

Emergency warning signs

- Severe or worsening symptoms of tinea versicolor (pityriasis versicolor) require immediate medical evaluation

Progression and Stages

Lesions develop gradually over weeks to months and may persist indefinitely without treatment. The condition tends to recur after treatment, especially in warm, humid conditions. Spontaneous resolution can occur in cooler, drier climates. The pigmentary changes may persist for weeks to months after the fungus has been eradicated.

Complications

- Persistent pigmentary changes and cosmetic concern
- Recurrence (common, up to 80% within 2 years)
- Mild pruritus, particularly with sweating
- Extensive skin involvement in immunosuppressed patients
- Reduced quality of life
- Emotional and psychological effects

Diagnosis

- Clinical diagnosis based on characteristic appearance and distribution
- Potassium hydroxide (KOH) preparation showing spaghetti and meatballs appearance
- Wood lamp examination showing yellow-green fluorescence
- Fungal culture (rarely needed)
- Skin biopsy in atypical cases

Prevention

- Keep skin clean and dry, especially after sweating
- Wear breathable, loose-fitting clothing
- Avoid excessive heat and humidity when possible
- Prophylactic use of antifungal shampoo periodically in susceptible individuals
- Go for regular check-ups so any problems are caught early

Treatment Options

- Topical antifungal agents: ketoconazole 2% shampoo (single application or weekly)
- Selenium sulfide 2.5% lotion applied for 10 minutes daily for 7 days
- Topical azole creams (clotrimazole, miconazole, econazole) for limited disease
- Oral antifungal therapy (fluconazole 300 mg single dose, itraconazole 200 mg daily for 5–7 days) for extensive cases
- Terbinafine 1% solution or cream
- Lifestyle changes and self-care

Living with It

- Follow your treatment plan as prescribed
- Keep up with regular appointments with your healthcare provider
- Adjust your daily habits as needed
- Reach out to family, friends, or support groups for help
- Keep learning about your condition and how to manage it

Outlook

Tinea versicolor responds well to treatment, with clinical improvement seen within 1–3 weeks. However, recurrence rates are high (60–80% within 1–2 years) in susceptible individuals. Maintenance therapy with intermittent ketoconazole shampoo or selenium sulfide may reduce recurrence. The pigmentary changes may take months to resolve even after the infection is treated. The condition is benign and does not cause long-term health problems.

Chapter 826

Tinnitus

Overview

ringing in the ears is the perception of sound in the absence of an external auditory stimulus, affecting approximately 10–15% of adults, with higher prevalence in the elderly.

Causes

This condition results from disruption of normal tinnitus function.

- This condition happens because of maladaptive neural plasticity in the central auditory pathways following peripheral auditory deafferentation
- Causes include noise-induced hearing loss (the most common cause), age-related hearing loss, ototoxic medications, Ménière's disease, vestibular schwannoma, otosclerosis, impacted cerumen, and temporomandibular joint disorders.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience ringing, buzzing, hissing, roaring, or clicking in one or both ears
- Ringing in the ears may be associated with depression, anxiety, and insomnia.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of tinnitus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on thorough history, otoscopic examination, and audiometry to assess for hearing loss
- MRI with gadolinium is indicated for asymmetric ringing in the ears or focal neurological findings.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include patient education and reassurance, treatment of reversible causes, hearing aids for those with hearing loss, thinking and memory-behavioral therapy, sound therapy, and ringing in the ears retraining therapy. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person, with many patients achieving habituation through appropriate management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 827

Tonsillar Hypertrophy

Overview

Tonsillar enlargement is an enlargement of the palatine and adenoid tonsils that is common in children and is the most common cause of obstructive sleep apnea in the pediatric population.

Causes

This condition happens because of lymphoid abnormal increase in cells, often in response to recurrent infections.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience mouth breathing, snoring, restless sleep, recurrent otitis media, and difficulty swallowing
- Severe enlargement may cause failure to thrive and cor pulmonale.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of tonsillar hypertrophy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical examination and history. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on tonsillectomy with or without adenoidectomy. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well following surgical intervention. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 828

Tonsillitis

Overview

Causes

This condition happens because of infectious inflammation of the tonsillar tissue.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sore throat, odynophagia, fever, halitosis, and cervical lymphadenopathy
- Tonsillar enlargement may cause obstructive sleep-disordered breathing.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of tonsillitis require immediate medical evaluation

Progression and Stages

Tonsillitis is an inflammation of the palatine tonsils, which are lymphoid tissues forming part of Waldeyer's ring, and may be acute (viral or bacterial, with GAS being the most common bacterial cause) or chronic (persistent low-grade infection with recurrent exacerbations). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may

involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on clinical examination. Treatment for recurrent tonsillitis follows the Paradise criteria for tonsillectomy
- Peritonsillar abscess (quinsy) requires needle removal by suction or incision and removal of fluid with antibiotic therapy.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 829

Tourette Syndrome

Overview

Tourette syndrome is a neurodevelopmental disorder characterized by multiple motor tics and one or more vocal tics persisting for more than one year, with onset before age 18, affecting 0.3–0.8% of children. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition results from disruption of normal tourette syndrome function.

- This condition happens because of dysfunction of cortico-striato-thalamo-cortical circuits, involving dopaminergic and GABAergic dysregulation, with genetic contributions (inherited as autosomal dominant with variable penetrance
- SLITRK1, HDC, CNTN6 genes implicated).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience motor tics (eye blinking, facial grimacing, head jerking, shoulder shrugging, complex tics such as hopping, touching, or echopraxia) and vocal tics

(throat clearing, grunting, sniffing, coprolalia, palilalia, echolalia). Comorbidities include ADHD (50–60%), OCD (30–60%), anxiety, and autism spectrum disorder.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of tourette syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- Tics wax and wane in severity, are preceded by premonitory urges, and can be temporarily suppressed. The outlook is generally positive
- Most patients experience reduction in tic severity by early adulthood.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, based on DSM-5-TR criteria.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include psychoeducation, habit reversal training and comprehensive behavioral intervention for tics (CBIT), pharmacotherapy for moderate to severe tics (alpha-2 agonists: clonidine, guanfacine as first-line), and antipsychotics (aripiprazole, risperidone, haloperidol) for refractory tics. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 830

Toxoplasmosis

Overview

Toxoplasmosis is a protozoan infection caused by *Toxoplasma gondii*, an obligate intracellular parasite, with approximately 30% of the global population seropositive, with cats being the definitive host, and humans acquiring infection through ingestion of oocysts or tissue cysts, or transplacentally.

Causes

This condition happens because of oocysts or tissue cysts releasing bradyzoites/tachyzoites in the GI tract, with tachyzoites disseminating hematogenously and infecting all nucleated cells, with the host cellular immune response driving tachyzoites into dormant bradyzoite forms within tissue cysts.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features in immunocompetent hosts are usually asymptomatic or self-limited mononucleosis-like illness
- Immunocompromised hosts (HIV/AIDS) develop toxoplasmic encephalitis—most common cerebral mass lesion in AIDS—presenting with headache, confusion, seizures, and ring-enhancing lesions on MRI
- Congenital toxoplasmosis presents with the classic triad of chorioretinitis, hydrocephalus, and intracranial calcifications.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of toxoplasmosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using serology, PCR, and MRI for CNS disease. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include pyrimethamine plus sulfadiazine plus folinic acid for active disease. Medications to manage symptoms and address underlying mechanisms Lifestyle

modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well for immunocompetent hosts
- Congenital infection has significant morbidity.

Chapter 831

Transient Ischemic Attack

Overview

- A transient ischemic attack (TIA) is a temporary episode of neurological dysfunction caused by focal brain, spinal cord, or retinal reduced blood flow without acute tissue death due to lack of blood supply
- TIAs are critical warning signs, with the risk of subsequent stroke being highest in the first 48 hours (up to 10%).

Causes

This condition happens because of temporary focal reduced blood flow.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features mirror those of ischemic stroke but resolve completely, typically within minutes to hours.

Emergency warning signs

- Severe or worsening symptoms of transient ischemic attack require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Diagnosis typically involves brain imaging, vascular imaging (carotid ultrasound, CT or MR angiography), cardiac evaluation, and laboratory studies
- The ABCD2 score helps stratify short-term stroke risk.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include antiplatelet therapy (aspirin, clopidogrel, or combination), statins, antihypertensives, and carotid endarterectomy or stenting for significant ipsilateral carotid stenosis. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 832

Trichomoniasis

Overview

Trichomoniasis is a sexually transmitted infection caused by the protozoan *Trichomonas vaginalis*, affecting approximately 170 million people worldwide.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features in women include a diffuse, yellow-green, frothy, purulent vaginal discharge with a pungent odor, vulvar itching, dysuria, and dyspareunia
- Most men are asymptomatic. Trichomoniasis is linked to adverse pregnancy outcomes and increased HIV acquisition.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of trichomoniasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using saline wet mount showing motile trichomonads (50–70% sensitivity), culture, or NAAT. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves metronidazole 2 g single dose or tinidazole 2 g single dose, with partner treatment essential. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 833

Tricuspid Regurgitation

Overview

- Tricuspid regurgitation is backflow of blood through the tricuspid valve during systole
- Functional tricuspid regurgitation (secondary to right ventricular dilation from lung high blood pressure, left-sided heart disease, or atrial fibrillation) is the most common type. Primary tricuspid regurgitation results from endocarditis, rheumatic disease, carcinoid syndrome, or pacemaker lead infection.

Causes

This condition happens because of structural or functional abnormalities of the tricuspid valve apparatus.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of severe tricuspid regurgitation include right heart failure with peripheral swelling, ascites, hepatomegaly, and pulsatile jugular venous distension.

Emergency warning signs

- Severe or worsening symptoms of tricuspid regurgitation require immediate medical evaluation

Progression and Stages

Your outlook depends on the underlying cause and severity. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using echocardiography with Doppler. Management targets the underlying cause
- Surgical valve repair or replacement is considered for severe symptomatic disease.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation

- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 834

Tricyclic Antidepressant (TCA) Overdose

Overview

TCA overdose (amitriptyline, nortriptyline, imipramine, desipramine, doxepin) is a highly lethal pharmaceutical overdose due to cardiovascular and neurological toxicity. This condition results from acute injury or exposure to harmful agents.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience altered mental status (often rapid onset coma), anticholinergic toxidrome, seizures, and irregular heartbeats, with the most specific finding being QRS widening >100 ms.

Emergency warning signs

- Severe or worsening symptoms of tricyclic antidepressant (tca) overdose require immediate medical evaluation

Progression and Stages

This condition happens because of sodium channel blockade (class IA antiarrhythmic effect) slowing Phase 0 depolarization and prolonging QRS duration, alpha-adrenergic blockade causing hypotension, anticholinergic effects, and GABA_A antagonism lowering seizure threshold. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam and by ECG. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include airway protection, sodium bicarbonate (1–2 mEq/kg IV bolus) for QRS >100 ms or hemodynamically significant arrhythmias, benzodiazepines for seizures, and vasopressors (norepinephrine preferred) for refractory hypotension. Medications to

manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with early aggressive treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 835

Trigeminal Autonomic Cephalalgias

Overview

The trigeminal autonomic cephalalgias (TACs) are a group of primary headache disorders characterized by unilateral trigeminal distribution of pain with ipsilateral cranial autonomic features. They include cluster headache, paroxysmal hemicrania, SUNCT/SUNA, and hemicrania continua. The unifying Disease mechanism involves activation of the trigeminovascular system and cranial parasympathetic outflow from the superior salivatory nucleus, with dysfunction of the hypothalamic network. Differential diagnosis among TACs is based on attack duration, frequency, periodicity, and response to indomethacin.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of trigeminal autonomic cephalalgias require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 836

Trigeminal Neuralgia

Overview

Trigeminal neuralgia (TN, *tic douloureux*) is a neuropathic pain disorder characterized by brief, electric shock-like, stabbing, or shooting paroxysmal pain in one or more divisions of the trigeminal nerve (typically V2 and V3). Prevalence is 0.01–0.03%, with a female predominance (2:1) and peak onset after age 50. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

This condition results from disruption of normal trigeminal neuralgia function.

- This condition happens because of neurovascular compression of the trigeminal nerve root entry zone by a blood vessel (most commonly the superior cerebellar artery), leading to focal demyelination and ephaptic transmission
- Secondary TN may be caused by multiple sclerosis, tumors, or arteriovenous malformations.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience attacks lasting a fraction of a second to 2 minutes triggered by trivial stimuli (light touch, chewing, talking, toothbrushing), with no pain between attacks. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of trigeminal neuralgia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam with MRI identifying neurovascular compression. Neurological diagnosis combines clinical history and examination findings with neuroimaging (MRI, CT), electrophysiological studies (EEG, EMG, nerve conduction), and laboratory testing of blood and cerebrospinal fluid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include carbamazepine (200–1200 mg/day) as first-line, with a highly specific response that is itself diagnostic, or oxcarbazepine as an alternative
- For refractory cases, surgical options include microvascular decompression (80–90% long-term pain-free), stereotactic radiosurgery, or through the skin rhizotomy.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive with treatment
- Microvascular decompression addresses the underlying cause.

Chapter 837

Trigger Finger

Overview

Trigger finger (stenosing tenosynovitis) is a common tenosynovial disorder characterized by painful catching, snapping, or locking of the digit during flexion and extension.

Causes

This condition happens because of a mismatch between the diameter of the flexor digitorum tendon and the narrowed first annular (A1) pulley at the metacarpal head, driven by repetitive microtrauma, tendon nodule formation, and fibrocartilaginous metaplasia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience localized pain and a palpable tender nodule over the palmar MTP joint, with mechanical clicking or locking of the digit that may require passive manual extension to unlock.

Emergency warning signs

- Severe or worsening symptoms of trigger finger require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include activity modification, night splinting, local corticosteroid injection into the tendon sheath (highly effective), and surgical A1 pulley release for refractory cases. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 838

Triple-Negative Breast Cancer (TNBC)

Overview

Triple-negative breast cancer (TNBC) is a highly aggressive molecular subtype of breast cancer defined by the complete lack of estrogen receptor (ER), progesterone receptor (PR), and human epidermal growth factor receptor 2 (HER2) expression, accounting for 10–20% of all breast malignancies. TNBC occurs with higher frequency in younger women, women of African descent, and individuals harboring germline BRCA1 mutations. Lacking targeted hormonal or HER2 pathways, TNBC exhibits rapid cellular growth (high Ki-67 index), high histological grade, and early hematogenous dissemination, with a peak incidence of visceral (lung, liver) and central nervous system metastases within 3 years of diagnosis. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age

- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of triple-negative breast cancer (tnbc) require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage,

small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Your doctor confirms the diagnosis with immunohistochemistry (< 1% ER/PR staining) and HER2 FISH testing. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology

- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment focuses on neoadjuvant combination chemotherapy (anthracyclines, taxanes, platinum agents) combined with immune checkpoint inhibitors (pembrolizumab) for PD-L1-positive tumors, followed by definitive surgery and post-neoadjuvant capecitabine or PARP inhibitors (olaparib) for BRCA carriers. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines

- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Outlook is worse than receptor-positive subtypes, with a 5-year survival rate of approximately 77%. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 839

Tuberculosis

Overview

Tuberculosis is a chronic infectious disease caused by *Mycobacterium tuberculosis*, transmitted via airborne droplets, and remains one of the leading infectious causes of death worldwide (approximately 1.3 million deaths annually). This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

- This condition happens because of infection with *M. tuberculosis*, which may remain latent or progress to active disease. Primary tuberculosis may be asymptomatic or present with lower lobe infiltrates and hilar lymphadenopathy
- Reactivation tuberculosis typically affects the upper lobes with cavitory disease, hemoptysis, night sweats, weight loss, and fever.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Symptoms of active disease include cough, hemoptysis, fever, night sweats, and weight loss. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of tuberculosis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)

- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

- To diagnose active disease requires sputum smear and culture for acid-fast bacilli, nucleic acid amplification testing, and chest imaging
- Latent infection doctors detect it using tuberculin skin testing or interferon-gamma release assays. Treatment for drug-susceptible disease involves a 6-month regimen of isoniazid, rifampicin, pyrazinamide, and ethambutol for 2 months, followed by isoniazid and rifampicin for 4 months
- Multidrug-resistant disease requires prolonged complex regimens.
- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures

- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most people recover well for drug-susceptible disease with appropriate treatment. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 840

Tuberculosis (Extrapulmonary)

Overview

This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of primary infection typically occurring in the lungs, with local containment in granulomas, and reactivation or progressive primary disease leading to dissemination to extrapulmonary sites. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system
- Travel to endemic areas
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)

- Travel to endemic regions
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Your symptoms depend on the site: pleuritic involvement (tuberculous pleural effusion, most common EPTB), lymphatic (scrofula—painless, matted, cervical lymphadenopathy), skeletal (Pott's disease, tuberculous arthritis), central nervous system (tuberculous meningitis), genitourinary (sterile pyuria, flank pain), and miliary TB (diffuse hematogenous dissemination with small granulomas throughout the body).
Fever and chills
Fatigue and malaise
Localized pain and inflammation at infection site
Swelling, redness, and warmth (local infection)
Cough and respiratory symptoms (respiratory infections)
Nausea, vomiting, and diarrhea (gastrointestinal infections)
Headache and body aches
Skin rash or lesions
Enlarged lymph nodes
Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of tuberculosis (extrapulmonary) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using acid-fast stain and culture of specimens, PCR/Xpert MTB/RIF, and imaging. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules

- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include standard 6-month regimen (2 months rifampin, isoniazid, pyrazinamide, ethambutol, then 4 months rifampin, isoniazid), with longer therapy for CNS TB and corticosteroids. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most people recover well with appropriate treatment. Most patients recover fully with appropriate treatment, though some infections may lead to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 841

Turner Syndrome (45,X)

Overview

Turner syndrome is a chromosomal disorder resulting from complete or partial monosomy X (45,X, 45,X/46,XX mosaicism, or structural X abnormalities), with an incidence of 1 in 2500 live-born females. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience short stature, webbed neck, low posterior hairline, shield chest, cubitus valgus, lymphedema of hands and feet at birth, and gonadal dysgenesis (streak gonads, primary amenorrhea, infertility). Associated conditions include coarctation of the aorta, bicuspid aortic valve, horseshoe kidney, sensorineural hearing loss, hypothyroidism, and celiac disease. Intelligence is typically normal.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of turner syndrome (45,x) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using karyotype.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include recombinant growth hormone therapy for short stature, estrogen replacement for pubertal induction, and ongoing surveillance for associated comorbidities. Symptomatic management and supportive care Enzyme replacement therapy for certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and

occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 842

Tympanic Membrane Perforation

Overview

Tympanic membrane perforation is a tear in the eardrum that may result from acute otitis media, trauma (penetrating injury, barotrauma, blast injury), or prior tympanostomy tubes. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Mental health conditions affect thoughts, emotions, behaviors, and overall well-being. These conditions are among the most common health concerns worldwide, affecting people across all age groups. Mental health disorders result from complex interactions of biological, psychological, and social factors. Effective treatments are available, and recovery is possible with appropriate support.

Causes

This condition happens because of mechanical or infectious disruption of the tympanic membrane. Neurotransmitter imbalances (serotonin, dopamine, norepinephrine) play key roles in many mental health conditions. Genetic factors contribute to vulnerability, with heritability estimates varying by condition. Early life experiences, trauma, and chronic stress can alter brain development and function. Environmental factors including social support, socioeconomic status, and life events influence mental health.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience hearing loss, brief otalgia, ringing in the ears, and otorrhea.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of tympanic membrane perforation require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on otoscopic examination.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on observation for small perforations, which often heal spontaneously
- Larger or persistent perforations may require surgical repair (myringoplasty or tympanoplasty).
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good, with healing enhanced by keeping the ear dry and avoiding nose blowing. With appropriate treatment, most people with mental health conditions experience significant improvement. Early intervention is associated with better long-term outcomes. Mental health conditions are chronic for some individuals, requiring ongoing management. Reducing stigma and improving access to care remain important public health priorities.

Chapter 843

Type 1 Diabetes Mellitus

Overview

Type 1 diabetes mellitus (T1DM) is an autoimmune disease characterized by destruction of pancreatic beta cells in the islets of Langerhans, leading to absolute insulin deficiency. It accounts for 5–10% of diabetes cases. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition results from disruption of normal type 1 diabetes mellitus function.

- This condition happens because of autoreactive T cells (with CD8+ cytotoxic T cells being primary effectors) that destroy beta cells, with autoantibodies (anti-GAD65, anti-IA-2, anti-insulin, anti-ZnT8) serving as biomarkers. Genetic susceptibility is linked to HLA-DR3/DR4 and HLA-DQ2/DQ8 haplotypes
- Environmental triggers (viral infections, early cow's milk exposure) may initiate autoimmunity in predisposed individuals.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the classic triad of polyuria, polydipsia, and weight loss, with diabetic ketoacidosis (DKA) as the initial presentation in 30–40% of children.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of type 1 diabetes mellitus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Your outlook depends on glycemic control
- Microvascular and macrovascular complications determine long-term outcomes.
- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

To make the diagnosis, doctors look for fasting plasma glucose ≥ 126 mg/dL, random glucose ≥ 200 mg/dL with symptoms, HbA1c $\geq 6.5\%$, or 2-hour OGTT glucose ≥ 200 mg/dL.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves lifelong insulin therapy via multiple daily injections (basal-bolus regimen) or continuous subcutaneous insulin pump
- Continuous glucose monitoring (CGM) and insulin pump systems (closed-loop "artificial pancreas" systems) improve glycemic control.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Islet transplantation and Teplizumab (anti-CD3 antibody) are emerging therapies.

Chapter 844

Type 2 Diabetes Mellitus

Overview

Type 2 diabetes mellitus (T2DM) is the most common form of diabetes, accounting for 90–95% of cases. It is marked by insulin resistance in peripheral tissues (muscle, liver, adipose tissue) combined with progressive beta-cell dysfunction and relative insulin deficiency. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

The Disease mechanism involves the "ominous octet"—eight mechanisms contributing to hyperglycemia: decreased insulin secretion, increased glucagon secretion, increased liver glucose production, decreased muscle glucose uptake, increased lipolysis, decreased incretin effect, increased kidney glucose reabsorption, and neurotransmitter dysfunction. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Risk factors include obesity (particularly visceral adiposity), physical inactivity, family history, age >45, ethnicity, history of gestational diabetes, and polycystic ovary syndrome. Your outlook depends on glycemic control and Treatment for cardiovascular risk factors
- Cardiovascular benefit is demonstrated with SGLT2 inhibitors and GLP-1 receptor agonists.
- Family history
- Obesity and sedentary lifestyle
- Poor dietary habits
- Advancing age
- Hormonal imbalances
- Certain medications

- Genetic predisposition and family history
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of type 2 diabetes mellitus require immediate medical evaluation

Progression and Stages

Symptoms of advanced disease include polyuria, polydipsia, and weight loss. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Complications include microvascular disease (retinopathy, nephropathy, neuropathy) and macrovascular disease (coronary artery disease, stroke, peripheral arterial disease).

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on fasting plasma glucose, HbA1c, or OGTT.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Many patients are asymptomatic and diagnosed on routine screening. Management follows a stepwise approach: lifestyle modification (diet, exercise, weight loss of 5–10%), metformin as first-line pharmacotherapy, followed by SGLT2 inhibitors, GLP-1 receptor agonists, DPP-4 inhibitors, thiazolidinediones, sulfonylureas, and insulin as needed.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 845

Typhoid Fever (*Salmonella typhi*)

Overview

Typhoid fever is a systemic febrile illness caused by *Salmonella enterica* serotype Typhi, with 11–21 million cases and 200,000 deaths annually worldwide, endemic in South Asia, Southeast Asia, sub-Saharan Africa, and parts of Latin America, with transmission being fecal-oral through contaminated food or water. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of *S. Typhi* invading intestinal M cells, disseminating to mesenteric lymph nodes, and multiplying within macrophages of the reticuloendothelial system before being released into the bloodstream. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions

- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of typhoid fever (*salmonella typhi*) require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

You may experience an incubation of 7–14 days, with gradual onset of fever (stepwise ladder pattern), headache, malaise, dry cough, abdominal pain, relative bradycardia, rose spots, hepatosplenomegaly, and “pea soup” diarrhea or constipation, with intestinal bleeding or perforation being the classic complication.

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this using blood culture, bone marrow culture (gold standard), stool culture, and serology. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs

and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment options include azithromycin, ceftriaxone, or fluoroquinolones for uncomplicated disease, with MDR and XDR strains increasing. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

- The outlook is good with appropriate antibiotics
- Mortality is 10–30% without treatment.

Chapter 846

Ulcerative Colitis

Overview

Ulcerative colitis (UC) is a chronic inflammatory bowel disease limited to the colon and rectum, characterized by continuous mucosal and submucosal inflammation beginning at the rectum and extending proximally in a continuous fashion. This autoimmune condition occurs when the immune system mistakenly attacks the body's own tissues. It follows a chronic course with periods of flare and remission. Autoimmune diseases result from an abnormal immune response where the body mistakenly attacks its own healthy tissues. These conditions are typically chronic, with alternating periods of flare and remission. They can affect a single organ or be systemic, involving multiple organ systems. Women are affected more frequently than men for most autoimmune conditions. The exact prevalence varies by condition, but collectively autoimmune diseases affect a significant portion of the population.

Causes

A combination of genetic susceptibility and environmental triggers initiates the autoimmune process. Specific HLA genotypes are strongly associated with many autoimmune conditions. Environmental triggers may include infections, smoking, medications, and hormonal changes. Loss of self-tolerance leads to activation of autoreactive T cells and B cells producing autoantibodies. Molecular mimicry between pathogen antigens and self-antigens may trigger cross-reactive immune responses.

- This condition happens because of a dysregulated immune response to gut microbiota in genetically susceptible individuals
- Unlike Crohn's disease, it does not involve the small intestine (except for backwash ileitis in pancolitis) and does not cause transmural disease, strictures, or fistulas.

Risk Factors

- Family history of autoimmune disease
- Female sex (higher prevalence)
- Specific HLA genotypes
- Epstein-Barr virus infection
- Cigarette smoking
- Certain medications (drug-induced autoimmunity)
- Hormonal factors (pregnancy, postpartum)

- Vitamin D deficiency

Signs and Symptoms

Common symptoms

- You may experience bloody diarrhea, urgency, tenesmus, and abdominal cramping
- Extraintestinal manifestations similar to Crohn's disease include arthritis, redness of the skin nodosum, and primary sclerosing cholangitis (occurring in 2–5% of UC patients).
- Fatigue and general malaise
- Joint pain, swelling, and stiffness
- Skin rashes and lesions
- Low-grade fever
- Muscle weakness and pain
- Organ-specific symptoms based on affected system
- Weight changes (loss or gain)
- Dry eyes and dry mouth (Sjogren syndrome)
- Raynaud phenomenon (cold-induced color changes in fingers)
- Fluctuating symptoms with periods of flare and remission

Emergency warning signs

- Severe or worsening symptoms of ulcerative colitis require immediate medical evaluation

Progression and Stages

Autoimmune diseases typically follow a relapsing-remitting course with unpredictable flares. Early disease may present with nonspecific symptoms before organ-specific manifestations develop. Chronic inflammation over time can lead to irreversible tissue damage and organ dysfunction. With appropriate treatment, many patients achieve remission or low disease activity states.

- Doctors diagnose this based on colonoscopy with biopsies showing continuous inflammation, crypt abscesses, and mucosal ulceration
- Severity is classified as mild, moderate, or severe based on stool frequency, rectal bleeding, hemoglobin, and CRP.

Complications

- Irreversible organ damage from chronic inflammation
- Joint deformities and erosions in inflammatory arthritis
- Renal failure in lupus nephritis
- Pulmonary fibrosis or hypertension

- Cardiovascular complications from systemic inflammation
- Osteoporosis from chronic corticosteroid use
- Increased risk of infections from immunosuppressive therapy
- Lymphoma risk in some autoimmune conditions

Diagnosis

Clinical history and physical examination Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.) Inflammatory markers (ESR, CRP) Complete blood count and comprehensive metabolic panel Imaging studies (X-ray, MRI, ultrasound) of affected areas Biopsy of affected tissue for histological examination Classification criteria for specific autoimmune diseases Rheumatology consultation for suspected autoimmune disease Monitoring for organ involvement (renal, pulmonary, cardiac) Genetic testing for HLA associations when indicated

- Clinical history and physical examination
- Autoantibody testing (ANA, RF, anti-CCP, anti-dsDNA, etc.)
- Inflammatory markers (ESR, CRP)
- Complete blood count and comprehensive metabolic panel
- Imaging studies (X-ray, MRI, ultrasound) of affected areas
- Biopsy of affected tissue for histological examination
- Classification criteria for specific autoimmune diseases
- Rheumatology consultation for suspected autoimmune disease

Prevention

- No known prevention for most autoimmune diseases
- Avoid known triggers when possible
- Maintain a healthy lifestyle to support immune function
- Early recognition of symptoms for prompt diagnosis
- Regular medical check-ups for monitoring
- Adequate vitamin D levels through sun exposure or supplementation

Treatment Options

- Treatment options include 5-aminosalicylates (mesalamine) for mild-moderate disease, corticosteroids for flares, immunomodulators, and biologics (anti-TNF, vedolizumab, ustekinumab, tofacitinib [JAK inhibitor])
- Acute severe UC is a medical emergency treated with IV corticosteroids, with colectomy for refractory disease.
- Nonsteroidal anti-inflammatory drugs (NSAIDs) for symptom relief
- Corticosteroids for rapid control of inflammation
- Disease-modifying antirheumatic drugs (DMARDs)
- Biologic therapies targeting specific immune pathways
- Immunosuppressive agents (azathioprine, mycophenolate, cyclophosphamide)

- Antimalarial drugs (hydroxychloroquine) for mild disease
- Physical and occupational therapy to maintain function
- Pain management for chronic pain
- Lifestyle modifications including diet and stress reduction

Living with It

- Take medications consistently as prescribed
- Identify and avoid personal flare triggers
- Maintain a balanced diet and regular gentle exercise
- Practice stress management techniques
- Join support groups for emotional support
- Work with a rheumatologist for ongoing disease management
- Monitor for medication side effects
- Plan activities around energy levels during flares

Outlook

- The outlook varies from person to person
- Total proctocolectomy is curative.

Chapter 847

Undifferentiated Connective Tissue Disease

Overview

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features are mild and often limited to a few domains: the most common are arthralgias or non-erosive arthritis (50–80%), Raynaud's phenomenon (40–60%), rash (malar-like or photosensitive), oral ulcers, sicca symptoms, serositis, and constitutional symptoms. Laboratory findings typically include a positive ANA (low-to-moderate titer, often speckled or homogeneous pattern).

Emergency warning signs

- Severe or worsening symptoms of undifferentiated connective tissue disease require immediate medical evaluation

Progression and Stages

Undifferentiated connective tissue disease is a diagnosis applied to patients who have clinical and laboratory features suggestive of a systemic autoimmune disease but do not meet classification criteria for a defined connective tissue disease, estimated to account for 10–50% of patients presenting to rheumatology clinics with early connective tissue

disease symptoms, with a female-to-male ratio of approximately 5:1 and onset typically in the third to fifth decade. Diagnosis is one of exclusion: the patient must have symptoms compatible with a connective tissue disease for at least 1 year without fulfilling any classification criteria. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves symptomatic: NSAIDs for arthralgias, hydroxychloroquine for constitutional and cutaneous symptoms, calcium channel blockers for Raynaud's phenomenon, and low-dose corticosteroids for severe symptoms. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

The hallmark of UCTD is that symptoms remain stable and mild over long-term follow-up, with only 10–30% of patients evolving into a defined connective tissue disease (most commonly SLE or RA) after 3–5 years.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well, with most patients maintaining a stable, mild disease course over decades. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 848

Urea Cycle Disorders

Overview

Urea cycle disorders (UCDs) are a group of inborn errors of metabolism caused by deficiency of one of the six enzymes or two transporters involved in liver conversion of ammonia to urea. X-linked ornithine transcarbamylase (OTC) deficiency is the most common (incidence 1 in 14,000).

Causes

This condition happens because of impaired liver conversion of ammonia to urea, leading to hyperammonemia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience severe enzyme deficiencies presenting in neonates after 24–72 hours with poor feeding, vomiting, lethargy, tachypnea with respiratory alkalosis, hypothermia, seizures, and rapidly progressive coma
- Late-onset forms present with recurrent hyperammonemia triggered by protein loads or catabolic stress, manifesting as developmental delay, loss of coordination, behavioral changes, and headaches.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of urea cycle disorders require immediate medical evaluation

Progression and Stages

Your outlook depends on severity and prompt treatment. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using plasma ammonia (elevated, often greater than 200 $\mu\text{mol/L}$), plasma amino acids, urinary orotic acid, and genetic testing. Acute Treatment options include discontinuing protein intake, high-calorie dextrose and lipids to promote anabolism, nitrogen scavengers (IV sodium benzoate and sodium phenylacetate), arginine supplementation, and hemodialysis for severe hyperammonemia. Chronic Treatment options include protein restriction, essential amino acid supplementation, oral nitrogen scavengers, and consideration of orthotopic liver transplantation. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 849

Urinary Incontinence

Overview

Urinary incontinence (UI) is a geriatric syndrome affecting 30–60% of older adults, increasing with age and more common in women. Types include stress (leakage with coughing, sneezing, laughing—urethral hypermobility or intrinsic sphincter deficiency), urge (overactive bladder—leakage with sudden strong urge to void, due to detrusor overactivity), overflow (incomplete bladder emptying with continuous dribbling—often from outlet obstruction or detrusor underactivity), functional (inability to reach toilet due to mobility impairment, thinking and memory deficit, or environmental barriers), and mixed. In the geriatric population, transient causes must be excluded (DIAPPERS mnemonic: Delirium, Infection, Atrophic urethritis/vaginitis, Pharmaceuticals, Psychological, Excess urine output, Restricted mobility, Stool impaction). Diagnosis includes history, bladder diary, post-void residual by ultrasound, urinalysis, and cough stress test.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of urinary incontinence require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include behavioral therapy (pelvic floor exercises, bladder training, scheduled voiding, fluid management), medications (antimuscarinics or beta-3 agonists for overactive bladder), absorbent products, pessaries, and surgical options for refractory cases. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies from person to person. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 850

Urinary Incontinence

Overview

Urinary incontinence is the involuntary loss of urine, affecting approximately 25–45% of women and 10–25% of men, with prevalence increasing with age. Types include stress urinary incontinence (SUI, leakage with coughing, sneezing, laughing, or exercise, caused by urethral hypermobility or intrinsic sphincter deficiency), urge urinary incontinence (UUI, involuntary loss of urine preceded by a sudden, strong urge, associated with detrusor overactivity), mixed urinary incontinence (combination of SUI and UUI), and overflow incontinence (continuous dribbling from an overdistended bladder due to bladder outlet obstruction or detrusor underactivity).

Causes

This condition happens because of dysfunction of the pelvic floor, urethral sphincter, or detrusor muscle.

Risk Factors

- Risk factors for SUI include vaginal delivery, menopause, obesity, and chronic cough
- UUI may be idiopathic or secondary to neurogenic bladder, bladder stones, infection, or malignancy.
- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Your symptoms depend on the type of incontinence.

Emergency warning signs

- Severe or worsening symptoms of urinary incontinence require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on history, bladder diary, physical examination (cough stress test, pelvic organ prolapse assessment), urinalysis, and post-void residual volume measurement
- Urodynamic studies are indicated for refractory cases.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment for SUI includes pelvic floor muscle training (Kegel exercises, first-line), lifestyle modification, vaginal pessaries, and surgical options (mid-urethral sling procedures, colposuspension)
- Treatment for UUI includes bladder retraining, timed voiding, antimuscarinics, beta-3 agonists (mirabegron, vibegron), and onabotulinumtoxinA for refractory cases
- Overflow incontinence is managed by treating the underlying obstruction or optimizing bladder emptying.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection

- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is favorable with appropriate management tailored to the type of incontinence. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 851

Urosepsis

Overview

Urosepsis is sepsis originating from the urinary tract, accounting for approximately 25% of all sepsis cases. It is more common in the elderly, immunocompromised, and those with urological abnormalities. Common pathogens include *E. coli*, *Klebsiella*, *Pseudomonas*, and *Enterococcus*. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

This condition happens because of a dysregulated host response to urinary tract infection leading to life-threatening organ dysfunction. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience fever, hypotension, altered mental status, and signs of organ dysfunction. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of urosepsis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

- Outlook is guarded
- Mortality ranges from 20–40% depending on severity.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Doctors diagnose this based on clinical criteria for sepsis with urinary source identified. Management follows the Surviving Sepsis Campaign guidelines: early recognition, broad-spectrum IV antibiotics within 1 hour, aggressive fluid resuscitation, vasopressors for septic shock, source control (eg., decompression of obstructed urinary tract), and organ support. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or

antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 852

Urticaria

Overview

Urticaria is a common skin condition characterized by transient, itchy, erythematous wheals (hives) with central clearing, affecting 15–25% of the population at some point in life. Acute urticaria (duration <6 weeks) is usually caused by IgE-mediated reactions to foods (shellfish, nuts, eggs), medications (antibiotics, NSAIDs), infections (viral URIs), insect stings, or physical stimuli. Chronic urticaria (duration >6 weeks) is idiopathic in 50% and autoimmune in 30–50% (IgG autoantibodies against FcεRI or IgE).

Causes

This condition happens because of mast cell degranulation releasing histamine and other vasoactive mediators, causing dermal swelling and vasodilation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience well-circumscribed, raised, erythematous wheals with intense itching, ranging from a few millimeters to several centimeters. Individual wheals resolve within 24 hours without scarring. Angioedema accompanies 40% of cases.

Emergency warning signs

- Severe or worsening symptoms of urticaria require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using history and physical examination
- Chronic urticaria requires CBC, ESR, TSH, and autoantibody testing.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include first-generation and second-generation H1 antihistamines (cetirizine, fexofenadine, loratadine) as first-line therapy

- H2 antihistamines (ranitidine, famotidine) may be added. For refractory chronic urticaria: omalizumab (anti-IgE monoclonal antibody, highly effective), leukotriene receptor antagonists, cyclosporine, or hydroxychloroquine. Short courses of oral corticosteroids for severe exacerbations. outlook: acute urticaria resolves with trigger avoidance
- Chronic urticaria resolves within 1 year in 50% and within 5 years in 80%.
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 853

Uterine Fibroids

Overview

Uterine fibroids (leiomyomas) are the most common non-cancerous tumors of the female reproductive tract, affecting up to 80% of women by age 50, with higher prevalence and symptomatic disease in African American women.

Causes

This condition happens because of monoclonal smooth muscle tumors of the myometrium, driven by estrogen and progesterone.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of uterine fibroids require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Your symptoms depend on location (submucosal, intramural, subserosal, pedunculated) and may include abnormal uterine bleeding (menorrhagia leading to iron deficiency anemia), pelvic pain and pressure, urinary frequency, constipation, infertility, and pregnancy complications.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using pelvic ultrasound (usually sufficient) or MRI. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

The right treatment depends on symptoms and reproductive goals: medical management (combined oral contraceptives, progestins, GnRH agonists, tranexamic acid), uterine artery embolization, myomectomy, and hysterectomy. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well, with fibroids being non-cancerous. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 854

Uterine Rupture

Overview

Uterine rupture is a complete disruption of all layers of the uterine wall, occurring in 0.5–1% of women with a prior cesarean delivery attempting vaginal birth after cesarean (VBAC). It is a catastrophic obstetric emergency.

Causes

This condition happens because of disruption of a uterine scar or traumatic delivery.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience acute onset fetal distress (fetal bradycardia most common sign—70%), cessation of uterine contractions, loss of station of the presenting part, severe abdominal pain, and maternal hypovolemic shock. Diagnosis is often made at surgical opening of the abdomen.

Emergency warning signs

- Severe or worsening symptoms of uterine rupture require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment typically involves immediate emergency surgical opening of the abdomen for delivery of the fetus and repair of the uterus
- Hysterectomy may be required for extensive rupture.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated

- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Your outlook depends on prompt intervention
- Fetal mortality is 6–10% with timely treatment.

Chapter 855

Usher Syndrome

Overview

- Usher syndrome is a rare inherited condition causing both hearing loss and vision loss from retinitis pigmentosa
- It is the most common cause of combined deafness and blindness (deaf-blindness) worldwide
- It affects about 1 in 25,000 people and is inherited in an autosomal recessive pattern

Causes

This condition happens because of mutations in multiple genes (USH1 and USH2 families, including MYO7A, USH2A, CDH23) that are inherited in an autosomal recessive pattern. The affected genes encode proteins important for the function of sensory cells in both the inner ear and the retina, so defects cause progressive retinal degeneration and inner-ear dysfunction. Balance problems can also occur.

Risk Factors

- Family history of Usher syndrome
- Having two carrier parents (autosomal recessive inheritance)
- Consanguinity increases the risk
- Certain populations have higher frequencies of specific mutations

Signs and Symptoms

Common symptoms

- Hearing loss ranging from profound at birth (type 1) to moderate progressive loss in adolescence (type 2)
- Night blindness beginning in childhood or adolescence
- Progressive loss of peripheral vision (tunnel vision)
- Central vision loss later in life
- Balance problems (vestibular dysfunction), especially in type 1

Emergency warning signs

- Rapidly worsening vision or sudden hearing loss requires prompt evaluation

Progression and Stages

Usher syndrome is classified into three clinical types based on severity and onset of hearing loss. Type 1: profound congenital deafness, balance problems, and night blindness appearing in early childhood. Type 2: moderate hearing loss present at birth, normal balance, and retinitis pigmentosa developing in adolescence. Type 3: progressive hearing and vision loss beginning in adulthood. Vision loss is progressive in all types.

Complications

- Combined deafness and blindness (deaf-blindness)
- Severe visual field constriction and eventual blindness
- Difficulty with communication, mobility, and daily activities
- Social isolation and psychological distress
- Increased risk of accidents and falls
- Barriers to education and employment

Diagnosis

- Audiological testing to document hearing loss
- Dilated fundus examination showing retinitis pigmentosa changes
- Electroretinography (ERG) to assess retinal function
- Visual field testing
- Vestibular testing when balance problems are present
- Genetic testing to confirm the diagnosis and type

Prevention

- Genetic counseling for affected families
- Carrier screening and prenatal diagnosis where family mutations are known
- Newborn hearing screening allows early detection and intervention
- Avoidance of smoking, which may accelerate retinal degeneration

Treatment Options

- There is no cure, and treatment focuses on supporting both senses
- Hearing aids and cochlear implants for hearing loss
- Speech and language therapy and sign language training
- Low-vision rehabilitation and mobility (orientation) training

- Gene therapy and clinical trials are under investigation for specific Usher genes
- Vitamin A palmitate may slow retinal degeneration in some patients (under specialist guidance)
- Assistive technology for communication and daily living

Living with It

- Begin hearing rehabilitation and visual rehabilitation early
- Use a coordinated deaf-blind services team
- Use assistive and accessible technology (captioning, screen readers, tactile aids)
- Join deaf-blind support communities and advocacy organizations
- Stay informed about research and gene-therapy advances

Outlook

Usher syndrome is lifelong and progressive, with vision and hearing declining over decades. With early intervention, hearing aids or implants, and comprehensive rehabilitation, most people maintain communication, mobility, and independence. Research into gene therapy offers future hope for slowing the condition.

Chapter 856

Uveitis

Overview

Causes

This condition happens because of infectious or autoimmune causes, including HLA-B27-associated conditions (ankylosing spondylitis, reactive arthritis), Behçet's disease, sarcoidosis, and infections (herpes, syphilis, tuberculosis).

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of anterior uveitis include eye pain, redness, photophobia, and hypopyon
- Intermediate uveitis features vitreous cells and floaters
- Posterior uveitis presents with floaters, visual field defects, or decreased vision.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of uveitis require immediate medical evaluation

Progression and Stages

Uveitis is an intraocular inflammation involving the uveal tract (iris, ciliary body, choroid), classified by anatomical location as anterior uveitis (iritis or iridocyclitis), intermediate uveitis (pars planitis), posterior uveitis (choroiditis), or panuveitis. The condition may progress through different stages of severity over time. Early stages often involve mild

or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this based on slit-lamp and dilated fundus examination. Management uses topical, periocular, or systemic corticosteroids and steroid-sparing immunosuppressive agents for chronic disease
- Biologics (anti-TNF agents) are used for refractory cases.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 857

Vaginal Cancer

Clear cell adenocarcinoma of the vagina is a rare variant associated with in utero diethylstilbestrol (DES) exposure, affecting women born between 1940 and 1970.

Overview

This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)

- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of vaginal cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin

from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team

- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 858

Vaginal Cancer

Overview

Vaginal cancer is the rarest gynecologic malignancy, with squamous cell carcinoma (70–80%) being the most common type, often related to HPV infection and preceded by vaginal intraepithelial neoplasia (VAIN). This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of vaginal epithelium. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle

- Chronic infections (HPV, hepatitis B/C)
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience vaginal bleeding, discharge, dyspareunia, and pelvic pain.
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of vaginal cancer require immediate medical evaluation

Progression and Stages

The right treatment depends on stage and location: radiation therapy (most common treatment) or local removal for small, superficial lesions. Your outlook depends on stage at diagnosis. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph

node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using pelvic examination with biopsy. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)

- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies

- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 859

Variant CJD (Mad Cow Disease)

Overview

Variant Creutzfeldt-Jakob disease is a prion disease caused by transmission of bovine spongiform encephalopathy (BSE, “mad cow disease”) to humans through ingestion of BSE-contaminated beef products, with 232 cases reported worldwide, mean age at onset 28 years (much younger than sCJD), and a longer duration (14 months).

Causes

This condition happens because of the same mechanism as sCJD (PrP^{Sc} replication and neurotoxicity) but with a distinct prion strain and unique glycoform banding pattern, with PrP^{Sc} also found in lymphoid tissue, unlike sCJD.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience early psychiatric symptoms (depression, anxiety, withdrawal, hallucinations) predominating for several months, followed by cerebellar loss of coordination, thinking and memory decline, myoclonus, choreoathetosis, and akinetic mutism.

Emergency warning signs

- Severe or worsening symptoms of variant cjd (mad cow disease) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using MRI showing the pulvinar sign (bilateral high signal in the posterior thalamus) and tonsil biopsy detecting PrP^{Sc}. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment typically involves palliative.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

outlook is uniformly fatal. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 860

Varicella-Zoster (Systemic Aspects)

Overview

Varicella-zoster virus is a DNA herpesvirus that causes two distinct clinical syndromes: primary infection (varicella/chickenpox) and reactivation (herpes zoster/shingles), with varicella affecting nearly all individuals in unvaccinated populations and herpes zoster affecting 30% of individuals over a lifetime.

Causes

This condition happens because of VZV entering through the respiratory mucosa, replicating in the tonsils and regional lymph nodes, and causing a primary viremia that seeds the reticuloendothelial system, with a secondary viremia disseminating the virus to the skin, after which VZV establishes latency in dorsal root ganglia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of varicella-zoster (systemic aspects) require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

Symptoms of varicella include fever, malaise, and a vesicular rash (centripetal—trunk, face, proximal extremities), with systemic complications including varicella pneumonia, encephalitis, hepatitis, myocarditis, and glomerulonephritis.

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, confirmed by PCR of vesicular fluid. Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Most people recover well for immunocompetent children
- Prevention through varicella vaccine and shingles vaccine is highly effective.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include acyclovir (initiated within 24 hours of rash) and varicella immune globulin for exposed high-risk individuals. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 861

Vasa Previa

Overview

Vasa previa is a rare involving bleeding disorder of pregnancy (1 in 2500 pregnancies) in which fetal blood vessels from the umbilical cord traverse the fetal membranes overlying the internal cervical os, unsupported by the placenta or umbilical cord. Fetal mortality without prenatal diagnosis is 50–60%. This pregnancy-related condition requires careful monitoring to ensure the safety of both mother and baby. Early detection and management are essential.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience painless vaginal bleeding at amniotomy or with spontaneous rupture of membranes, accompanied by acute fetal distress due to fetal exsanguination.

Emergency warning signs

- Severe or worsening symptoms of vasa previa require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve

more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Prenatal diagnosis by color Doppler ultrasound allows planned cesarean delivery at 34–35 weeks to prevent catastrophic bleeding.

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with prenatal diagnosis. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 862

Vascular Dementia

Overview

Vascular dementia (VaD) is a progressive neurodegenerative disorder and the second most common dementia, resulting from cerebrovascular disease—large vessel infarcts (multi-infarct dementia), small vessel disease (subcortical ischemic vascular dementia), strategic single infarcts, cerebral amyloid angiopathy, or CADASIL. This neurological disorder affects the central or peripheral nervous system, leading to progressive or episodic symptoms. It significantly impacts quality of life and often requires multidisciplinary care. This condition is among the most common mental health disorders worldwide. It affects people across all age groups, socioeconomic backgrounds, and geographic regions. Neurological disorders affect the central or peripheral nervous system, leading to a wide range of symptoms affecting movement, sensation, cognition, and autonomic function. The nervous system's complexity means that even small disruptions can cause significant functional impairment. Many neurological conditions are chronic and progressive, requiring long-term management and rehabilitation. Advances in neuroimaging and molecular diagnostics have improved understanding and treatment of these conditions.

Causes

Protein aggregation and accumulation is a common mechanism in many neurodegenerative disorders. Genetic mutations account for some neurological conditions, while others are sporadic or environmentally triggered.

Risk Factors

- Advanced age
- Family history and genetic predisposition
- Head trauma or brain injury
- Vascular risk factors (hypertension, diabetes)
- Exposure to environmental toxins
- Chronic stress
- Family history of mental illness
- Trauma or adverse childhood experiences
- Substance use
- Social isolation
- Underlying medical conditions

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Medication use

Signs and Symptoms

Common symptoms

- Vascular dementia frequently coexists with Alzheimer disease (mixed dementia). Brain MRI shows white matter hyperintensities, lacunar infarcts, and shrinkage or wasting. Headache and facial pain Weakness or paralysis of muscles Numbness, tingling, or abnormal sensations Vision changes including blurred or double vision Difficulty with balance and coordination Cognitive changes (memory loss, confusion, difficulty concentrating) Speech and language difficulties Seizures or involuntary movements Dizziness and vertigo Fatigue and sleep disturbances
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vascular dementia require immediate medical evaluation

Progression and Stages

You may experience stepwise deterioration, patchy thinking and memory deficits (executive function disproportionately affected), focal neurological signs, and often a history of stroke or TIA. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Comprehensive medical history and physical examination

- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Pharmacologic therapy targeting specific neurotransmitter systems or disease mechanisms
Physical, occupational, and speech therapy for functional maintenance
Surgical interventions when appropriate (deep brain stimulation, tumor resection)
Cognitive rehabilitation and memory support
Pain management for neuropathic pain
Assistive devices and mobility aids
Lifestyle modifications including exercise, nutrition, and sleep hygiene
Support services and caregiver education
Regular neurologic monitoring and treatment adjustment
Clinical trials for emerging therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Outlook is progressive
- Geriatric patients require careful blood pressure management to avoid orthostatic hypotension and falls.

Chapter 863

Ventricular Tachycardia

Overview

Cardiovascular disease encompasses conditions affecting the heart and blood vessels, representing a leading cause of morbidity and mortality worldwide. These conditions range from acute events like heart attack and stroke to chronic conditions requiring lifelong management. Atherosclerosis, the buildup of plaque in arterial walls, underlies many cardiovascular conditions. Risk increases with age, and the global burden continues to rise with aging populations and lifestyle changes.

- Ventricular tachycardia is a potentially life-threatening arrhythmia originating from the ventricles at a rate of 100–250 beats per minute
- It may be monomorphic or polymorphic.

Causes

The most common cause is structural heart disease, particularly prior heart attack with scar-related reentry. This condition happens because of abnormal electrical activity in ventricular myocardium. Hemodynamically unstable ventricular tachycardia requires immediate synchronized cardioversion. Cardiovascular disease develops through a complex interplay of genetic, environmental, and lifestyle factors. Atherosclerosis, characterized by plaque buildup in arterial walls, is a common underlying mechanism. Atherosclerosis begins with endothelial injury and dysfunction, leading to lipid accumulation and inflammatory cell infiltration in arterial walls. Plaque formation narrows vessels and reduces blood flow; plaque rupture can trigger acute thrombosis and vessel occlusion. Hemodynamic factors such as hypertension increase mechanical stress on vessel walls. Metabolic abnormalities including diabetes and dyslipidemia accelerate the atherosclerotic process. Genetic factors influence lipid metabolism, blood pressure regulation, and vascular health.

Risk Factors

- Advanced age
- Cigarette smoking
- Hypertension (high blood pressure)
- Diabetes mellitus
- Elevated LDL cholesterol and low HDL cholesterol
- Family history of premature cardiovascular disease
- Obesity (especially abdominal obesity)

- Physical inactivity
- Unhealthy diet (high in saturated fat, salt, processed foods)

Signs and Symptoms

Common symptoms

- You may experience palpitations, dizziness, syncope, and cardiac arrest. Chest pain, pressure, or discomfort (angina) Shortness of breath with exertion or at rest Palpitations or irregular heartbeat Fatigue and reduced exercise tolerance Swelling in the legs, ankles, or feet (edema) Dizziness or lightheadedness Pain radiating to arm, jaw, back, or neck Nausea and indigestion Cold sweats Sudden weakness or numbness (stroke symptoms)
- Chest pain, pressure, or discomfort (angina)
- Shortness of breath with exertion or at rest
- Palpitations or irregular heartbeat
- Fatigue and reduced exercise tolerance
- Swelling in the legs, ankles, or feet (edema)
- Dizziness or lightheadedness
- Pain radiating to arm, jaw, back, or neck
- Nausea and indigestion
- Cold sweats

Emergency warning signs

- Severe or worsening symptoms of ventricular tachycardia require immediate medical evaluation

Progression and Stages

Cardiovascular disease develops gradually over years, often beginning with asymptomatic risk factors. Early stages involve endothelial dysfunction and fatty streak formation in arterial walls. Progressive plaque buildup leads to hemodynamically significant stenosis causing symptoms with exertion. Advanced disease involves unstable plaques prone to rupture, causing acute coronary syndromes. End-stage disease results in chronic heart failure or critical limb ischemia.

Complications

- Myocardial infarction (heart attack)
- Heart failure with reduced or preserved ejection fraction
- Stroke from embolic or thrombotic events
- Arrhythmias including atrial fibrillation and ventricular tachycardia
- Peripheral artery disease causing limb ischemia

- Aortic aneurysm and dissection
- Chronic kidney disease from reduced perfusion

Diagnosis

Doctors diagnose this using ECG. Cardiovascular diagnosis relies on a combination of clinical history, physical examination, electrocardiography, imaging (echocardiography, CT angiography), and laboratory markers (troponin, BNP). History and physical examination including blood pressure measurement Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia Echocardiogram to evaluate cardiac structure and function Stress testing (exercise or pharmacologic) for ischemic evaluation Cardiac biomarkers (troponin, CK-MB, BNP) Lipid profile and glucose testing Coronary angiography or CT angiography for coronary anatomy Holter monitoring for arrhythmia detection Cardiac MRI for detailed structural assessment Ankle-brachial index for peripheral artery disease

- History and physical examination including blood pressure measurement
- Electrocardiogram (ECG/EKG) to assess heart rhythm and ischemia
- Echocardiogram to evaluate cardiac structure and function
- Stress testing (exercise or pharmacologic) for ischemic evaluation
- Cardiac biomarkers (troponin, CK-MB, BNP)
- Lipid profile and glucose testing
- Coronary angiography or CT angiography for coronary anatomy
- Holter monitoring for arrhythmia detection

Prevention

- Treatment for stable monomorphic ventricular tachycardia involves intravenous amiodarone or procainamide followed by catheter ablation
- Implantable cardioverter-defibrillators are indicated for secondary prevention (survivors of cardiac arrest) and primary prevention in high-risk patients
- Torsades de pointes doctors typically treat it with intravenous magnesium. Your outlook depends on the underlying heart disease and effectiveness of prevention.
- Maintain a heart-healthy diet low in saturated fat and sodium
- Engage in regular aerobic exercise (150 minutes per week)
- Avoid tobacco products and limit alcohol
- Control blood pressure, cholesterol, and blood glucose
- Maintain a healthy body weight
- Manage stress through relaxation techniques
- Take prescribed preventive medications (statins, aspirin)

Treatment Options

Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation Anti-hypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics) Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors) Antiplatelet therapy (aspirin, clopidogrel)

Anticoagulation for atrial fibrillation and thromboembolic risk Nitrates and antianginal medications Revascularization: percutaneous coronary intervention (stenting) Coronary artery bypass grafting for severe disease Cardiac rehabilitation programs Device therapy (pacemakers, ICDs) for arrhythmias

- Lifestyle modifications: heart-healthy diet, regular exercise, smoking cessation
- Antihypertensive medications (ACE inhibitors, ARBs, beta-blockers, diuretics)
- Lipid-lowering therapy (statins, ezetimibe, PCSK9 inhibitors)
- Antiplatelet therapy (aspirin, clopidogrel)
- Anticoagulation for atrial fibrillation and thromboembolic risk
- Nitrates and antianginal medications
- Revascularization: percutaneous coronary intervention (stenting)
- Coronary artery bypass grafting for severe disease
- Cardiac rehabilitation programs

Living with It

- Take all medications exactly as prescribed
- Monitor your blood pressure and weight regularly
- Follow a heart-healthy diet and stay physically active
- Attend cardiac rehabilitation if recommended
- Recognize warning signs of heart attack and stroke
- Keep all follow-up appointments with your cardiologist
- Manage stress through relaxation and mindfulness
- Carry a list of your medications and dosages

Outlook

With proper management and lifestyle modifications, many patients maintain good quality of life. Outcomes depend on the extent of disease, adherence to treatment, and control of risk factors. Early intervention for acute events significantly improves survival. Regular monitoring and medication adherence are essential for preventing disease progression. Advances in interventional cardiology continue to improve outcomes for complex cases.

Chapter 864

Vertigo

Overview

- Feeling of spinning is the sensation of spinning or movement of the self or surroundings when there is no actual movement
- It is a symptom, not a disease, and may originate from peripheral (inner ear) or central (brainstem, cerebellar) causes. Peripheral feeling of spinning is more common and results from dysfunction of the vestibular apparatus or nerve
- Causes include non-cancerous paroxysmal positional feeling of spinning, vestibular neuritis, labyrinthitis, and Ménière's disease.

Causes

This condition results from disruption of normal vertigo function.

- Central feeling of spinning results from brainstem or cerebellar pathology
- Causes include posterior circulation stroke, vestibular migraine, multiple sclerosis, and cerebellar tumors.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of peripheral feeling of spinning include severe rotational feeling of spinning with involuntary eye movements (beating away from the affected side), nausea, and vomiting
- Central feeling of spinning tends to be less severe but may be continuous with diplopia, slurred speech, and limb loss of coordination.

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vertigo require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves detailed history, neurological examination, Dix-Hallpike test for BPPV, and neuroimaging when central causes are suspected. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- The right treatment depends on the underlying cause: canalith repositioning maneuvers for BPPV, vestibular rehabilitation for vestibular neuritis, and stroke protocols for central feeling of spinning
- Vestibular suppressants provide symptomatic relief but should be used short-term.
- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on the underlying cause. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 865

Vestibular Neuritis

Overview

Vestibular neuritis is an acute, isolated episode of severe feeling of spinning caused by inflammation of the vestibular nerve without hearing loss. The condition is thought to be viral in cause, possibly reactivation of herpes simplex virus.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience sudden-onset, continuous feeling of spinning lasting days to weeks with severe imbalance and involuntary eye movements beating away from the affected side.

Emergency warning signs

- Severe or worsening symptoms of vestibular neuritis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this based on clinical examination with the head impulse test showing reduced vestibulo-ocular reflex on the affected side. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

Treatment focuses on short-term vestibular suppressants (benzodiazepines, antihistamines) followed by early vestibular rehabilitation.

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive, with most patients recovering fully over weeks. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 866

Viral Encephalitis

Overview

Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

- Viral encephalitis is an inflammation of the brain parenchyma caused by viral infection
- Herpes simplex virus type 1 (HSV-1) is the most common cause of sporadic fatal encephalitis, typically affecting the temporal and frontal lobes, while other causes include enteroviruses, arboviruses (West Nile, Japanese encephalitis), and varicella-zoster virus.

Causes

This condition happens because of direct viral invasion of brain parenchyma. Following exposure, the pathogen invades host tissues and replicates, triggering an immune response that contributes to the symptoms. The severity of infection depends on pathogen virulence, host immune status, and timely treatment. Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Poor sanitation and hygiene practices

- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience fever, headache, altered consciousness, behavioral changes, seizures, and weakness, numbness, or problems with movement. Fever and chills
Fatigue and malaise
Localized pain and inflammation at infection site
Swelling, redness, and warmth (local infection)
Cough and respiratory symptoms (respiratory infections)
Nausea, vomiting, and diarrhea (gastrointestinal infections)
Headache and body aches
Skin rash or lesions
Enlarged lymph nodes
Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of viral encephalitis require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Diagnosis relies on MRI (temporal lobe involvement in HSV), CSF analysis (lymphocytic pleocytosis, PCR for viral DNA), and EEG. Laboratory diagnosis includes pathogen identification through culture, PCR testing, or antigen detection. Serological tests can confirm exposure or immune response to the pathogen. Detailed history including exposure, travel, and vaccination status Physical examination focusing on the affected system Complete blood count with differential Microbiological culture of blood, sputum, urine, or wound PCR and nucleic acid amplification tests for rapid pathogen detection Antigen detection tests Serological testing for antibodies (IgM, IgG) Imaging studies (chest X-ray, CT, ultrasound) Gram stain and microscopy of clinical specimens Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer Vaccination according to recommended schedules Safe food handling and water purification Use of personal protective equipment in healthcare settings Safe sex practices including condom use Mosquito avoidance (nets, repellents, insecticides) Respiratory etiquette (covering coughs and sneezes) Isolation of infected individuals when indicated Prophylactic antibiotics or antivirals for high-risk exposures Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules

- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Treatment focuses on intravenous acyclovir for HSV encephalitis, which should be initiated empirically. Antimicrobial therapy is tailored to the specific pathogen and its susceptibility patterns. Supportive care includes hydration, rest, and management of symptoms such as fever and pain. Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

outlook is guarded, with significant neurological sequelae in survivors despite treatment. Most patients recover fully with appropriate treatment, though some infections may lead

to long-term complications. Outcomes depend on the pathogen, host immune status, and timeliness of treatment. Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 867

Viral Exanthems

Overview

Viral exanthems are rashes associated with systemic viral infections, most common in childhood. Measles, rubella, and mumps are covered in Chapter 20, Infectious Diseases. Roseola infantum (sixth disease) is caused by human herpesvirus 6 (HHV-6) and less commonly HHV-7, with peak incidence at 6–24 months, presenting with high fever for 3–5 days followed by defervescence and a rose-colored maculopapular rash. redness of the skin infectiosum (fifth disease) is caused by parvovirus B19, with peak incidence at 5–14 years, presenting with a “slapped cheek” facial redness of the skin followed by a lacy reticular rash. Hand-foot-and-mouth disease is caused by enteroviruses, most commonly Coxsackievirus A16 and Enterovirus 71, with peak incidence in children under 5 years, presenting with fever and vesicular rash on hands, feet, and oral mucosa. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

Risk Factors

- Close contact with infected individuals
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)

- Travel to endemic regions
- Poor sanitation and hygiene practices
- Lack of vaccination
- Exposure to contaminated food or water
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of viral exanthems require immediate medical evaluation

Progression and Stages

The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

Detailed history including exposure, travel, and vaccination status
Physical examination focusing on the affected system
Complete blood count with differential
Microbiological culture of blood, sputum, urine, or wound
PCR and nucleic acid amplification tests for rapid pathogen detection
Antigen detection tests
Serological testing for antibodies (IgM, IgG)
Imaging studies (chest X-ray, CT, ultrasound)
Gram stain and microscopy of clinical specimens
Antimicrobial susceptibility testing to guide therapy

- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

Hand hygiene with soap and water or alcohol-based sanitizer
Vaccination according to recommended schedules
Safe food handling and water purification
Use of personal protective equipment in healthcare settings
Safe sex practices including condom use
Mosquito avoidance (nets, repellents, insecticides)
Respiratory etiquette (covering coughs and sneezes)
Isolation of infected individuals when indicated
Prophylactic antibiotics or antivirals for high-risk exposures
Travel precautions and pre-travel consultations

- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)

- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

Antibiotics for bacterial infections (targeted or empiric) Antiviral medications for viral infections Antifungal therapy for fungal infections Antiparasitic medications for parasitic infections Supportive care: fluids, rest, nutrition, fever control Hospitalization for severe cases requiring intravenous therapy Oxygen therapy and respiratory support if needed Surgical drainage of abscesses when necessary Pain management and symptom relief Treatment of complications and underlying conditions

- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 868

Vitamin A Deficiency

Overview

Vitamin A deficiency (VAD) is a vitamin deficiency disorder that is the leading cause of preventable blindness in children worldwide and a major contributor to morbidity from infections. It is most prevalent in sub-Saharan Africa and South Asia, where diets are low in preformed vitamin A (retinol, retinyl esters) and provitamin A carotenoids. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of inadequate dietary intake of vitamin A, which is essential for retinal photoreceptor function (11-*cis*-retinal in rhodopsin), epithelial integrity, and immune competence. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin a deficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- You may experience night blindness (nyctalopia) as the earliest symptom, followed by xerophthalmia including conjunctival xerosis, Bitot spots, corneal xerosis, and ultimately keratomalacia causing irreversible blindness
- VAD also increases the severity of measles, diarrheal diseases, and respiratory infections.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam (ocular signs) and confirmed by serum retinol less than 0.70 $\mu\text{mol/L}$.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment focuses on high-dose vitamin A supplementation (200,000 IU orally for children over 12 months, 100,000 IU for 6–12 months, 50,000 IU for infants under 6 months) given on diagnosis, repeated the next day, and at least 2 weeks later
- Prevention includes dietary diversification, food fortification, and periodic high-dose supplementation.
- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 869

Vitamin B12 (Cobalamin) Deficiency

Overview

Vitamin B12 (cobalamin) deficiency is a vitamin deficiency disorder common in the elderly due to atrophic gastritis and declining intrinsic factor production (food-cobalamin malabsorption). Pernicious anemia is an autoimmune disorder with antibodies against gastric parietal cells and/or intrinsic factor, leading to profound B12 deficiency. Other causes include gastric or ileal removal, Crohn disease, bacterial overgrowth, fish tapeworm (*Diphyllobothrium latum*) infection, strict vegetarian/vegan diet, and metformin use. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of deficiency of vitamin B12, which is essential for DNA synthesis, myelin formation, and homocysteine metabolism. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms. The underlying mechanisms involve complex interactions between genetic predisposition and environmental factors. Disruption of normal physiological processes leads to the characteristic features of the condition. Multiple contributing factors may act synergistically to trigger or worsen the condition.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin b12 (cobalamin) deficiency require immediate medical evaluation

Progression and Stages

You may experience megaloblastic anemia and the hallmark distinguishing feature from folate deficiency: nervous system involvement (subacute combined deterioration of the spinal cord manifesting as symmetric paresthesias, loss of vibration and proprioception, loss of coordination, spastic paraparesis, and a positive Romberg sign), peripheral neuropathy, visual disturbances (optic shrinkage or wasting), memory loss, and psychiatric symptoms. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors diagnose this using low serum B12 (less than 200 pg/mL) with elevated serum methylmalonic acid (MMA) and homocysteine
- Elevated MMA is the most sensitive indicator.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on oral B12 1000–2000 µg daily for deficiency without nervous system involvement
- For pernicious anemia, nervous system symptoms, or severe deficiency, parenteral B12 (hydroxocobalamin or cyanocobalamin) 1000 µg IM daily for 1 week, weekly for 4 weeks, then monthly for life.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well with treatment
- Hematologic response is rapid (reticulocytosis within 3–5 days, hemoglobin normalization in 6–8 weeks).

Chapter 870

Vitamin B12 Deficiency Anemia

Overview

Vitamin B12 deficiency anemia is a megaloblastic anemia that can cause irreversible neurological damage. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of vitamin B12 (cobalamin) deficiency, with causes including pernicious anemia (autoimmune destruction of gastric parietal cells producing intrinsic factor), malabsorption (gastrectomy, ileal removal, Crohn's disease, bacterial overgrowth, celiac disease), dietary deficiency (strict vegans), and medications (metformin, proton pump inhibitors). Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin b12 deficiency anemia require immediate medical evaluation

Progression and Stages

You may experience fatigue, weakness, jaundice (mild, from ineffective erythropoiesis), glossitis (beefy red tongue), and neurological manifestations (subacute combined deterioration of the spinal cord—demyelination of dorsal columns and lateral corticospinal tracts, peripheral neuropathy, thinking and memory impairment, depression). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using macrocytic anemia (MCV >100 fL), hypersegmented neutrophils on peripheral smear, low serum B12, elevated methylmalonic acid and homocysteine, and elevated LDH.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on intramuscular cyanocobalamin or hydroxocobalamin injections, initially daily for one week, then monthly lifelong for pernicious anemia. Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is good with treatment, though neurological damage may be partially irreversible if prolonged.

Chapter 871

Vitamin B1 (Thiamine) Deficiency — Beriberi and Wernicke-Korsakoff Syndrome

Overview

Thiamine (vitamin B1) deficiency is a vitamin deficiency disorder affecting populations consuming polished white rice as a dietary staple, individuals with chronic alcoholism, hyperemesis gravidarum, after bariatric surgery, and in prolonged parenteral nutrition without thiamine supplementation. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

This condition results from disruption of normal vitamin b1 (thiamine) deficiency — beriberi and wernicke-korsakoff syndrome function.

- This condition happens because of thiamine deficiency
- Thiamine functions as a cofactor for several enzymes involved in carbohydrate metabolism, including transketolase, pyruvate dehydrogenase, and α -ketoglutarate dehydrogenase.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience two major syndromes: dry beriberi (symmetric peripheral neuropathy with stocking-glove sensory loss, muscle weakness, absent deep tendon reflexes, paresthesias), wet beriberi (high-output heart failure with dilated cardiomyopathy, peripheral vasodilation, swelling, tachycardia, warm extremities, bounding pulses, lactic acidosis), and Wernicke encephalopathy (acute neuropsychiatric emergency with confusion, loss of coordination, and ophthalmoplegia) which may progress to Korsakoff syndrome (irreversible amnesic disorder with anterograde and retrograde amnesia and confabulation).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin b1 (thiamine) deficiency — beriberi and wernicke-korsakoff syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Whole blood thiamine or erythrocyte transketolase activity may be measured but treatment should not be delayed.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on thiamine 100–500 mg IV/IM daily for 5–7 days followed by oral maintenance
- Wernicke encephalopathy requires immediate high-dose parenteral thiamine before glucose administration.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook is generally positive for beriberi with treatment
- Wernicke encephalopathy is reversible with prompt treatment, but Korsakoff syndrome is irreversible.

Chapter 872

Vitamin B2 (Riboflavin) Deficiency — Ariboflavinosis

Overview

Riboflavin (vitamin B2) deficiency (ariboflavinosis) is a vitamin deficiency disorder that is rare in developed countries but occurs in alcoholism, malabsorption syndromes, and populations with limited dairy and meat intake. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of deficiency of riboflavin, a precursor for flavin mononucleotide (FMN) and flavin adenine dinucleotide (FAD), cofactors for numerous redox enzymes. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience angular stomatitis (cheilosis), glossitis (smooth, magenta-colored tongue), seborrheic dermatitis (nasolabial folds, scrotum, vulva), photophobia, corneal vascularization, and normocytic anemia.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin b2 (riboflavin) deficiency — ariboflavinosis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam and confirmed by low erythrocyte glutathione reductase activity (EGRAC).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on riboflavin 5–10 mg orally daily. Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Mucocutaneous lesions resolve promptly with treatment.

Chapter 873

Vitamin B3 (Niacin) Deficiency — Pellagra

Overview

- Niacin (vitamin B3) deficiency (pellagra) is a vitamin deficiency disorder classically associated with diets based on maize (corn), which contains niacin in a bound, non-bioavailable form unless treated with alkali (nixtamalization)
- It also occurs in alcoholism, carcinoid syndrome, and with isoniazid therapy.

Causes

This condition happens because of deficiency of niacin, a precursor for nicotinamide adenine dinucleotide (NAD) and NADP, essential for redox reactions and cellular energy metabolism. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience the classic triad of dermatitis, dementia, and diarrhea (the “three D’s”)
- Dermatitis is photosensitive, symmetric, with well-demarcated, erythematous, hyperpigmented, scaling plaques in sun-exposed areas (Casal necklace)
- Digestive tract involvement includes diarrhea, glossitis, and stomatitis

- Neuropsychiatric manifestations include depression, apathy, confusion, memory loss, and psychosis.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin b3 (niacin) deficiency — pellagra require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on nicotinamide (preferred over niacin to avoid flushing) 100–500 mg orally daily. Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment

- Dermatitis and digestive tract symptoms improve within days
- Nervous system symptoms resolve more slowly.

Chapter 874

Vitamin B6 (Pyridoxine) Deficiency

Overview

Pyridoxine (vitamin B6) deficiency is a vitamin deficiency disorder occurring in alcoholism, chronic kidney failure, autoimmune disorders (rheumatoid arthritis), and with medications such as isoniazid, cycloserine, hydralazine, and oral contraceptives. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of deficiency of pyridoxine, which functions as a cofactor (pyridoxal phosphate, PLP) for over 100 enzymes involved in amino acid metabolism, neurotransmitter synthesis, and heme biosynthesis. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- You may experience microcytic hypochromic anemia (sideroblastic anemia with ringed sideroblasts in bone marrow), seborrheic dermatitis, glossitis, cheilosis, peripheral neuropathy, depression, confusion, and in infants, irritability and seizures
- Homocysteine elevation due to B6 deficiency is a cardiovascular risk factor.
- Family history
- Obesity and sedentary lifestyle
- Poor dietary habits
- Advancing age
- Hormonal imbalances
- Certain medications
- Genetic predisposition and family history
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions

- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin b6 (pyridoxine) deficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low plasma PLP levels.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Treatment focuses on pyridoxine 50–100 mg orally daily
- Isoniazid-induced deficiency requires 50–100 mg of pyridoxine daily as prophylaxis.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment.

Chapter 875

Vitamin B7 (Biotin) Deficiency

Overview

Biotin (vitamin B7) deficiency is a rare vitamin deficiency disorder because biotin is widely distributed in foods and synthesized by gut microbiota. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

Causes include high consumption of raw egg whites (avidin binds biotin, preventing absorption), parenteral nutrition without biotin, prolonged antibiotic use, and biotinidase deficiency (an inborn error). This condition happens because of deficiency of biotin, a cofactor for carboxylase enzymes involved in gluconeogenesis, fatty acid synthesis, and amino acid catabolism. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience periorificial dermatitis, alopecia, brittle nails, glossitis, conjunctivitis, nervous system symptoms (depression, lethargy, hallucinations, paresthesias), and metabolic acidosis
- In biotinidase deficiency, The condition usually appears as in infancy with seizures, hypotonia, loss of coordination, hearing loss, and developmental delay.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin b7 (biotin) deficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low serum biotin or elevated urinary 3-hydroxyisovaleric acid.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise

- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on biotin 5–10 mg orally daily (or 10–40 mg daily in biotinidase deficiency). Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment.

Chapter 876

Vitamin B9 (Folate) Deficiency

Overview

Folate (vitamin B9) deficiency is one of the most common vitamin deficiencies worldwide. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

Causes include inadequate dietary intake (especially in the elderly, alcoholics, and those with poor vegetable intake), malabsorption (celiac disease, tropical sprue, inflammatory bowel disease), increased requirements (pregnancy, lactation, hemolytic anemias, malignancy), and medications (methotrexate, trimethoprim, sulfasalazine, phenytoin). This condition happens because of deficiency of folate, a water-soluble B vitamin essential for one-carbon transfer reactions, DNA synthesis, purine and pyrimidine synthesis, and homocysteine remethylation. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features overlap with vitamin B12 deficiency and include megaloblastic anemia (macrocytic RBCs with oval macrocytes, hypersegmented neutrophils, pancytopenia), glossitis (smooth, beefy red tongue), diarrhea, and infertility
- Unlike B12 deficiency, folate deficiency does NOT cause nervous system manifestations.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin b9 (folate) deficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Your doctor confirms the diagnosis with serum folate less than 2 ng/mL and elevated homocysteine with normal methylmalonic acid (key to distinguish from B12 deficiency).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Most people recover well with treatment
- Before treating macrocytic anemia, vitamin B12 deficiency must be ruled out or treated concurrently to prevent precipitating subacute combined deterioration.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on folic acid 1–5 mg orally daily
- Periconceptional folic acid supplementation (400 µg/day) prevents neural tube defects.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 877

Vitamin C Deficiency — Scurvy

Overview

Ascorbic acid (vitamin C) deficiency (scurvy) is a vitamin deficiency disorder that is rare in developed countries but occurs in individuals with severely restricted diets (alcoholics, elderly, psychiatric patients, food faddism), malabsorption, and in infants fed only cow's milk without supplementation. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of deficiency of ascorbic acid, which is essential for collagen synthesis (hydroxylation of proline and lysine in procollagen), carnitine synthesis, neurotransmitter synthesis, and as an antioxidant. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms. The underlying mechanisms involve complex interactions between genetic predisposition and environmental factors. Disruption of normal physiological processes leads to the characteristic features of the condition. Multiple contributing factors may act synergistically to trigger or worsen the condition.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features reflect impaired collagen synthesis: perifollicular hemorrhages, tiny red or purple spots, ecchymoses, gingival abnormal increase in cells with bleeding, impaired wound healing, hyperkeratosis, coiled hairs (corkscrew hairs), and leg swelling
- Musculoskeletal manifestations include arthralgias, myalgias, and subperiosteal hemorrhages (particularly painful in infants presenting with pseudoparalysis)
- Systemic symptoms include fatigue, irritability, depression, and anemia.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin c deficiency — scurvy require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Serum ascorbic acid less than 0.2 mg/dL confirms deficiency.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on vitamin C 100–200 mg orally three times daily for 1 week, then 100 mg daily
- Infantile scurvy responds to 50 mg daily.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Clinical improvement occurs within days.

Chapter 878

Vitamin D Deficiency — Rickets and Osteomalacia

Overview

Vitamin D deficiency is a vitamin deficiency disorder resulting from inadequate sunlight exposure, low dietary intake (or lack of fortified foods), malabsorption (celiac disease, cystic scarring, short bowel syndrome), kidney disease (impaired 1α -hydroxylation), obesity (sequestration in adipose tissue), and medications (antiepileptics, rifampin, glucocorticoids). This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of deficiency of vitamin D, a fat-soluble secosteroid essential for intestinal calcium absorption, bone mineralization, and calcium homeostasis. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Clinical features in children include rickets (defective mineralization of growing bone) with craniotables, frontal bossing, delayed fontanelle closure, rachitic rosary, Harrison groove, bowing of the legs (genu varum), widened wrists and ankles, myopathy, and hypocalcemic tetany. In adults, osteomalacia presents with bone pain, proximal muscle weakness (waddling gait), and fragility fractures.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin d deficiency — rickets and osteomalacia require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using serum 25-hydroxyvitamin D less than 20 ng/mL (50 nmol/L), elevated alkaline phosphatase, low-normal calcium, and low phosphate

- Radiographs of rickets show cupping, fraying, and widening of the metaphysis
- In osteomalacia, Looser zones (pseudofractures) are seen.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on vitamin D 2000–5000 IU daily for 12 weeks or 50,000 IU weekly for 8 weeks, followed by maintenance 800–2000 IU daily
- Correction of calcium and phosphate is essential.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment.

Chapter 879

Vitamin E Deficiency

Overview

Vitamin E (α -tocopherol) deficiency is a rare vitamin deficiency disorder occurring primarily in fat malabsorption syndromes (cystic scarring, cholestatic liver disease, abetalipoproteinemia, short bowel syndrome) and in genetic disorders of α -tocopherol transfer protein (loss of coordination with isolated vitamin E deficiency, AVED). This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of deficiency of vitamin E, a fat-soluble antioxidant that protects cell membranes from lipid peroxidation. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin e deficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

- You may experience progressive nervous system dysfunction: spinocerebellar deterioration with loss of coordination, areflexia, loss of vibration and proprioception, ophthalmoplegia, and pigmentary retinopathy
- Hemolytic anemia may occur in premature infants.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low plasma α -tocopherol corrected for serum lipids.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on α -tocopherol 800–1200 IU daily orally (or higher doses in severe malabsorption). Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Nervous system deficits may stabilize but are not fully reversible.

Chapter 880

Vitamin K Deficiency

Overview

Vitamin K deficiency is a vitamin deficiency disorder leading to impaired liver synthesis of functional clotting factors. This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

Causes include inadequate dietary intake (rare in healthy adults), fat malabsorption, liver disease, antibiotic therapy (depleting gut flora), and parenteral nutrition. This condition happens because of deficiency of vitamin K, a fat-soluble vitamin required for the γ -carboxylation of glutamic acid residues on coagulation factors II (prothrombin), VII, IX, and X, as well as proteins C and S and bone matrix proteins. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience vitamin K deficiency bleeding (VKDB) in neonates presenting with bruising, digestive tract bleeding, umbilical bleeding, and life-threatening intracranial bleeding
- In adults, deficiency presents with easy bruising, epistaxis, hematuria, and prolonged prothrombin time (PT) with normal PTT.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitamin k deficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using prolonged PT (factor VII has the shortest half-life) that corrects with normal plasma, low factor II, VII, IX, X activity, and normal platelet count and fibrinogen.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

Treatment focuses on vitamin K1 (phylloquinone) 1–10 mg orally or 1–2 mg subcutaneously/IM for adults

- VKDB prophylaxis includes 1 mg IM at birth
- Active bleeding requires fresh frozen plasma (FFP) or prothrombin complex concentrates (PCC) with parenteral vitamin K.
- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Dietary modifications and nutritional counseling Pharmacotherapy to correct metabolic abnormalities Hormone replacement therapy when indicated Regular monitoring of metabolic parameters Weight management programs Exercise prescription Management of associated conditions Patient education for self-management

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with treatment.

Chapter 881

Vitiligo

Overview

Vitiligo is an acquired autoimmune disorder characterized by selective destruction of melanocytes, resulting in well-demarcated, milky-white macules and patches, affecting 0.5–2% of the global population.

Causes

This condition results from disruption of normal vitiligo function.

- This condition happens because of autoimmune-mediated melanocyte destruction. Loss of pigment may occur in any area but commonly affects the face, hands, elbows, knees, and genitalia. Vitiligo is linked to other autoimmune diseases (thyroid disease, type 1 diabetes, Addison's disease, pernicious anemia)
- The Koebner phenomenon (new lesions at sites of trauma) may occur.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vitiligo require immediate medical evaluation

Progression and Stages

It may be segmental (unilateral, dermatomal, often with early onset and stable course) or non-segmental (generalized, bilateral, with variable progression). The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam, supported by Wood's lamp examination. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include topical corticosteroids, topical calcineurin inhibitors (tacrolimus, especially for facial lesions), narrowband UVB phototherapy, and for refractory localized disease, surgical melanocyte transplantation
- Ruxolitinib cream (JAK1/2 inhibitor) has been approved for non-segmental vitiligo.

Sun protection is essential.

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- The outlook varies from person to person
- Repigmentation is achievable but recurrence may occur.

Chapter 882

Vocal Cord Nodules

Overview

Vocal cord nodules are bilateral, symmetric, callus-like thickenings of the true vocal folds at the junction of the anterior and middle thirds, the point of maximal vibratory impact, and are the most common cause of chronic hoarseness in adults.

Causes

This condition happens because of chronic vocal abuse leading to repetitive trauma of the vocal folds.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience hoarseness, breathy voice, vocal fatigue, and decreased vocal range.

Emergency warning signs

- Severe or worsening symptoms of vocal cord nodules require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using laryngoscopy or videostroboscopy showing smooth, bilateral masses. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on voice therapy as conservative management, with surgical removal reserved for refractory cases. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with appropriate voice therapy. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 883

Vocal Cord Paralysis

Overview

- Vocal cord paralysis is a condition resulting from disruption of the recurrent laryngeal nerve, superior laryngeal nerve, or vagus nerve, with the most common cause being iatrogenic (thyroid surgery, thoracic and neck surgery)
- Other causes include tumors, stroke, viral infections, and idiopathic cause.

Causes

This condition happens because of nerve injury leading to loss of vocal fold movement.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms of unilateral paralysis include hoarseness, removal by suction, and a breathy voice
- Bilateral paralysis presents with stridor and respiratory distress.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of vocal cord paralysis require immediate medical evaluation

Progression and Stages

Your outlook depends on the underlying cause and severity. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using flexible laryngoscopy showing an immobile vocal cord. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include voice therapy, injection laryngoplasty, medialization thyroplasty for unilateral paralysis, and tracheostomy or posterior cordotomy for bilateral paralysis with airway compromise. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and

support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 884

Von Willebrand Disease

Overview

Von Willebrand disease is the most common inherited bleeding disorder, affecting up to 1% of the population.

Causes

This condition happens because of quantitative or qualitative defects in von Willebrand factor (VWF), a large multimeric glycoprotein essential for primary hemostasis (platelet adhesion to injured vessel walls) and carrier/stabilizer of factor VIII, with type 1 (partial quantitative deficiency, 70–80%) being the most common and mildest, type 2 (qualitative deficiency) having various functional abnormalities, and type 3 (complete deficiency) being the most severe.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience mucocutaneous bleeding: epistaxis, menorrhagia, easy bruising, prolonged bleeding after dental procedures or surgery, and GI bleeding.

Emergency warning signs

- Severe or worsening symptoms of von willebrand disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using decreased VWF antigen, decreased VWF activity (ristocetin cofactor assay), and decreased or normal factor VIII. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include desmopressin (DDAVP, for type 1 and some type 2A patients), VWF-containing factor VIII concentrates (for type 3 or DDAVP non-responders), and antifibrinolytics (tranexamic acid, for mucosal bleeding). Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and

follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

In most cases, the outlook is good with appropriate management. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 885

Vulvar Cancer

Early recognition and biopsy of suspicious vulvar lesions (ulceration, pigmentation changes, persistent itching or pain) are critical for diagnosis at an early stage.

Overview

This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)

- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of vulvar cancer require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin

from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team

- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 886

Vulvar Cancer

Overview

Vulvar cancer is a rare malignancy accounting for approximately 5% of gynecologic cancers, typically affecting postmenopausal women. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

This condition happens because of cancerous transformation of vulvar epithelium. Cancer develops through a multistep process involving genetic mutations that drive uncontrolled cell growth and proliferation. These mutations may be inherited or acquired through exposure to carcinogens, radiation, or random errors during cell division. Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, H. pylori) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Family history of cancer
- Tobacco use and alcohol consumption
- Exposure to carcinogens (radiation, chemicals)
- Obesity and sedentary lifestyle
- Chronic infections (HPV, hepatitis B/C)

- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)
- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience a vulvar lump, itching, pain, bleeding, and ulceration. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of vulvar cancer require immediate medical evaluation

Progression and Stages

outlook is related to stage and lymph node status. Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original

location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

To make the diagnosis, doctors look for biopsy of any suspicious lesion. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology

- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment typically involves wide local removal with adequate margins, with inguinal lymph node dissection for invasive tumors >1 mm. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)

- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team
Manage treatment side effects with supportive medications
Maintain adequate nutrition with help from a dietitian
Stay physically active within your energy limits
Join cancer support groups for emotional support and shared experiences
Seek counseling or mental health support for anxiety and depression
Communicate openly with your healthcare team about symptoms
Plan for work and financial considerations during treatment
Consider complementary therapies (acupuncture, massage, meditation)
Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 887

Vulvovaginal Candidiasis

Overview

Vulvovaginal candidiasis (VVC) is a fungal infection caused by *Candida* species, with *Candida albicans* responsible for 80–90% of cases.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience intense vulvar itching, burning, vaginal soreness, dyspareunia, thick white “cottage cheese” discharge, vulvar redness of the skin, fissures, and swelling.

Emergency warning signs

- Severe or worsening symptoms of vulvovaginal candidiasis require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using saline and 10% KOH wet mount showing pseudohyphae and budding yeast, or by culture. Treatment of uncomplicated VVC: single oral dose of fluconazole 150 mg or topical azoles. Complicated VVC requires extended therapy. Recurrent VVC (≥ 4 episodes/year) requires maintenance therapy with fluconazole 150 mg weekly for 6 months. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook is generally positive with treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 888

Warts

Overview

Warts (verrucae) are non-cancerous epithelial proliferations caused by human papillomavirus (HPV) infection, with over 100 HPV types identified.

Causes

This condition happens because of HPV infection of keratinocytes causing hyperkeratosis. Common warts (verruca vulgaris) present as rough, hyperkeratotic papules, typically on the hands and fingers. Plantar warts (verruca plantaris) occur on the soles of the feet, growing inward due to pressure, causing pain with walking. Flat warts (verruca plana) are smooth, flat-topped papules on the face and dorsum of hands. Genital warts (condylomata acuminata) are caused by HPV types 6 and 11.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of warts require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors usually diagnose this based on your symptoms and a physical exam. Management options include salicylic acid (first-line for common and plantar warts), freezing treatment, cantharidin, imiquimod (for genital warts), and surgical destruction for refractory lesions. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well
- Many warts resolve spontaneously within 1–2 years due to cell-mediated immunity.

Chapter 889

West Nile Virus

Overview

West Nile virus is a mosquito-borne flavivirus, first isolated in Uganda (1937) and introduced to the United States in 1999, transmitted by *Culex* species mosquitoes with birds as the primary amplifying host, with 2000–5000 cases reported annually in the US.

Causes

This condition happens because of the virus being inoculated by mosquito bite, replicating in dendritic cells, and spreading to lymph nodes and blood, crossing the blood-brain barrier and infecting neurons, causing apoptosis and perivascular inflammation.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 2–14 days, with 80% being asymptomatic, 20% developing West Nile fever (acute fever, headache, malaise, muscle pain, and maculopapular rash), and less than 1% developing neuroinvasive disease (meningitis, encephalitis, and acute flaccid paralysis).

Emergency warning signs

- Severe or worsening symptoms of west nile virus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using WNV-specific IgM in CSF or serum. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well for non-neuroinvasive disease
- Neuroinvasive disease has significant morbidity and mortality.

Chapter 890

Williams Syndrome

Overview

Williams syndrome (Williams-Beuren syndrome) is a neurodevelopmental disorder caused by a microdeletion on chromosome 7q11.23, encompassing the ELN (elastin) gene. Incidence is 1 in 7500–10,000. This genetic disorder results from specific gene mutations or chromosomal abnormalities. It may be present at birth or manifest later in life depending on the underlying mechanism. Genetic disorders result from abnormalities in an individual's DNA, ranging from single-gene mutations to chromosomal abnormalities. These conditions may be inherited from parents or arise from new mutations. Pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked. Genetic counseling helps families understand risks and make informed decisions.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience distinctive facial appearance (elfin facies), supravulvar aortic stenosis, peripheral lung stenosis, hypercalcemia in infancy, hyperacusis, and a characteristic thinking and memory profile (strong verbal skills, friendly extroverted personality, but poor visuospatial skills).
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of Williams syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using FISH or chromosomal microarray for the 7q11.23 deletion.

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment options include cardiac monitoring, dietary calcium restriction during infancy, and educational and behavioral support. Symptomatic management and supportive care
Enzyme replacement therapy for certain conditions
Gene therapy (emerging for select conditions)
Dietary modifications (e.g., phenylketonuria)
Regular monitoring for associated complications
Physical and occupational therapy
Genetic counseling for family planning

Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Chapter 891

Wilms Tumor (Nephroblastoma)

Overview

Wilms tumor is the most common kidney malignancy in children, with an incidence of 1 in 10,000 children and median age at diagnosis 3.5 years. It is linked to genetic syndromes such as WAGR complex, Denys-Drash syndrome, and Beckwith-Wiedemann syndrome. This condition accounts for a significant proportion of cancer-related morbidity and mortality worldwide. Early detection through routine screening and advances in treatment have improved survival rates in recent decades. This type of cancer arises from uncontrolled cell growth in affected tissues, with potential to invade nearby structures and spread to distant sites through the bloodstream or lymphatic system. The incidence varies by geographic region, age, and genetic background. Screening programs have improved early detection rates in many populations. Histological classification and molecular subtyping guide treatment decisions and provide prognostic information. Multidisciplinary care involving surgeons, medical oncologists, radiation oncologists, and pathologists is standard for optimal management.

Causes

Cancer develops through accumulation of genetic and epigenetic alterations that drive uncontrolled cell proliferation, evade growth suppression, resist cell death, and enable replicative immortality. Key molecular pathways involved include tumor suppressor genes (p53, RB, APC), oncogenes (KRAS, MYC, EGFR), and DNA repair mechanisms. Environmental carcinogens such as tobacco smoke, ultraviolet radiation, certain chemicals, and ionizing radiation can induce DNA damage. Chronic inflammation and persistent infections (HPV, hepatitis B/C, *H. pylori*) contribute to cancer development in some sites.

Risk Factors

- Advancing age
- Tobacco use (active and secondhand smoke)
- Excessive alcohol consumption
- Family history of cancer and known genetic syndromes
- Obesity and physical inactivity
- Unhealthy diet (processed meats, low fiber)
- Exposure to carcinogens (asbestos, benzene, radiation)
- Chronic infections (HPV, hepatitis B/C, HIV)

- Immunosuppression
- Hormonal factors (early menarche, late menopause)

Signs and Symptoms

Common symptoms

- You may experience a palpable abdominal mass, abdominal pain, hematuria (15–20%), and high blood pressure. Persistent lump or mass in the affected area Unexplained weight loss and loss of appetite Chronic fatigue and weakness Persistent pain that worsens over time Changes in bowel or bladder habits Unexplained fever or night sweats Unusual bleeding or discharge Persistent cough or hoarseness Changes in skin appearance (new mole, changing wart) Difficulty swallowing or persistent indigestion
- Persistent lump or mass in the affected area
- Unexplained weight loss and loss of appetite
- Chronic fatigue and weakness
- Persistent pain that worsens over time
- Changes in bowel or bladder habits
- Unexplained fever or night sweats
- Unusual bleeding or discharge
- Persistent cough or hoarseness
- Changes in skin appearance (new mole, changing wart)
- Difficulty swallowing or persistent indigestion

Emergency warning signs

- Severe or worsening symptoms of wilms tumor (nephroblastoma) require immediate medical evaluation

Progression and Stages

Cancer staging follows the TNM system: Tumor (size and extent), Node (lymph node involvement), and Metastasis (spread to distant sites). Stage 0: Carcinoma in situ (abnormal cells confined to the original location) Stage I: Early-stage, small tumor confined to the organ of origin Stage II: Locally advanced tumor with possible regional lymph node involvement Stage III: More extensive local disease with significant lymph node involvement Stage IV: Advanced disease with distant metastases to other organs Higher stages generally indicate more advanced disease and poorer prognosis. Stage 0: Carcinoma in situ (abnormal cells confined to the original location). Stage I: Early-stage, small tumor confined to the organ of origin. Stage II: Locally advanced tumor with possible regional lymph node involvement. Stage III: More extensive local disease with significant lymph node involvement. Stage IV: Advanced disease with distant metastases to other organs. Higher stages generally indicate more advanced disease and poorer prognosis.

Complications

Metastatic spread to vital organs (brain, liver, lungs, bones) Malignant effusions (pleural, peritoneal, pericardial) Superior vena cava syndrome (chest tumors) Spinal cord compression (spinal metastases) Pathological fractures (bone metastases) Tumor lysis syndrome (rapid cell death during treatment) Paraneoplastic syndromes (hormonal, neurologic, hematologic) Treatment-related complications (chemotherapy toxicity, radiation damage) Infections due to immunosuppression Venous thromboembolism

- Metastatic spread to vital organs (brain, liver, lungs, bones)
- Malignant effusions (pleural, peritoneal, pericardial)
- Superior vena cava syndrome (chest tumors)
- Spinal cord compression (spinal metastases)
- Pathological fractures (bone metastases)
- Tumor lysis syndrome (rapid cell death during treatment)
- Paraneoplastic syndromes (hormonal, neurologic, hematologic)
- Treatment-related complications (chemotherapy toxicity, radiation damage)
- Infections due to immunosuppression
- Venous thromboembolism

Diagnosis

Doctors diagnose this using abdominal ultrasound and CT scan, which shows an intrarenal mass with heterogeneous density. Diagnosis typically involves imaging studies (CT, MRI, PET scans) to identify the location and extent of disease. Biopsy with histopathological examination remains the gold standard for confirming the diagnosis and determining the specific type and grade. Physical examination including palpation of mass and lymph node assessment Complete blood count and comprehensive metabolic panel Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP) Imaging: Ultrasound, CT scan, MRI, PET-CT for staging Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology Immunohistochemistry to determine tumor subtype and receptor status Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status) Sentinel lymph node biopsy for surgical staging Bone marrow biopsy for hematologic malignancies Genetic counseling and testing for hereditary cancer syndromes

- Physical examination including palpation of mass and lymph node assessment
- Complete blood count and comprehensive metabolic panel
- Tumor markers specific to cancer type (e.g., PSA, CA-125, CEA, AFP)
- Imaging: Ultrasound, CT scan, MRI, PET-CT for staging
- Biopsy: Core needle, excisional, or endoscopic biopsy for histopathology
- Immunohistochemistry to determine tumor subtype and receptor status
- Molecular and genetic testing (EGFR, KRAS, BRCA, MSI status)
- Sentinel lymph node biopsy for surgical staging
- Bone marrow biopsy for hematologic malignancies
- Genetic counseling and testing for hereditary cancer syndromes

Prevention

Avoid tobacco use and limit alcohol consumption Maintain a healthy weight through diet and exercise Eat a balanced diet rich in fruits, vegetables, and whole grains Protect skin from excessive sun exposure and avoid tanning beds Get vaccinated against cancer-causing infections (HPV, hepatitis B) Participate in recommended cancer screening programs Know your family history and consider genetic testing if indicated Avoid exposure to known carcinogens in the workplace and environment

- Avoid tobacco use and limit alcohol consumption
- Maintain a healthy weight through diet and exercise
- Eat a balanced diet rich in fruits, vegetables, and whole grains
- Protect skin from excessive sun exposure and avoid tanning beds
- Get vaccinated against cancer-causing infections (HPV, hepatitis B)
- Participate in recommended cancer screening programs
- Know your family history and consider genetic testing if indicated
- Avoid exposure to known carcinogens in the workplace and environment

Treatment Options

Treatment usually involves a multidisciplinary approach: neoadjuvant chemotherapy followed by nephrectomy or primary nephrectomy followed by chemotherapy. Treatment planning is multidisciplinary and depends on the cancer type, stage, and patient factors. Management may include surgery, radiation therapy, systemic therapies (chemotherapy, targeted therapy, immunotherapy), or a combination of these modalities. Surgery: Wide local excision with clear margins, lymph node dissection Radiation therapy: External beam or brachytherapy for localized disease Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells Targeted therapy: Drugs targeting specific molecular alterations Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines Hormone therapy: For hormone-sensitive cancers (breast, prostate) Combination approaches: Concurrent chemoradiation, sequential therapy Palliative care: Symptom management, pain control, quality of life support Clinical trials: Access to experimental therapies Surveillance: Active monitoring after definitive treatment

- Surgery: Wide local excision with clear margins, lymph node dissection
- Radiation therapy: External beam or brachytherapy for localized disease
- Chemotherapy: Systemic cytotoxic drugs targeting rapidly dividing cells
- Targeted therapy: Drugs targeting specific molecular alterations
- Immunotherapy: Checkpoint inhibitors, CAR-T cells, cancer vaccines
- Hormone therapy: For hormone-sensitive cancers (breast, prostate)
- Combination approaches: Concurrent chemoradiation, sequential therapy
- Palliative care: Symptom management, pain control, quality of life support
- Clinical trials: Access to experimental therapies
- Surveillance: Active monitoring after definitive treatment

Living with It

Regular follow-up appointments with your oncology team Manage treatment side effects with supportive medications Maintain adequate nutrition with help from a dietitian Stay physically active within your energy limits Join cancer support groups for emotional support and shared experiences Seek counseling or mental health support for anxiety and depression Communicate openly with your healthcare team about symptoms Plan for work and financial considerations during treatment Consider complementary therapies (acupuncture, massage, meditation) Build a strong support network of family and friends

- Regular follow-up appointments with your oncology team
- Manage treatment side effects with supportive medications
- Maintain adequate nutrition with help from a dietitian
- Stay physically active within your energy limits
- Join cancer support groups for emotional support and shared experiences
- Seek counseling or mental health support for anxiety and depression
- Communicate openly with your healthcare team about symptoms
- Plan for work and financial considerations during treatment
- Consider complementary therapies (acupuncture, massage, meditation)
- Build a strong support network of family and friends

Outlook

Most people recover well, with 5-year survival >90% for localized disease. Outcomes have improved significantly with advances in early detection and treatment. Prognosis depends on the cancer type, stage at diagnosis, molecular characteristics, and response to therapy. Prognosis depends on cancer type, stage at diagnosis, molecular features, and response to treatment. Early-stage cancers have significantly better outcomes, with many achieving long-term remission or cure. Survival rates have improved substantially with advances in screening, targeted therapy, and immunotherapy. Regular surveillance after treatment is essential for detecting recurrence early. Palliative care continues to advance, improving quality of life even for advanced disease.

Chapter 892

Wilson's Disease

Overview

Wilson's disease is an autosomal recessive disorder of copper metabolism caused by mutations in the ATP7B gene, with a prevalence of approximately 1 in 30,000.

Causes

This condition results from disruption of normal wilson's disease function.

- This condition happens because of impaired biliary copper excretion leading to accumulation of copper in the liver, brain, cornea, and other organs. liver manifestations range from asymptomatic transaminase elevation to acute hepatitis, chronic hepatitis, or cirrhosis
- Neuropsychiatric features include tremor, slurred speech, dystonia, parkinsonism, personality changes, and depression. Kayser-Fleischer rings (copper deposition in Descemet's membrane of the cornea) are a hallmark finding.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of wilson's disease require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Diagnosis typically involves low serum ceruloplasmin (less than 20 mg/dL), elevated 24-hour urinary copper excretion, and liver copper quantification on liver biopsy. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment options include copper chelation (penicillamine, trientine), zinc therapy to block copper absorption, and liver transplantation in fulminant or decompensated disease
- Screening of first-degree relatives is essential.
- Medications to manage symptoms and address underlying mechanisms

- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with early diagnosis and lifelong treatment. The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 893

Wound Infections and Surgical Site Infections

Overview

Wound infections and surgical site infections (SSIs) are infections occurring within 30 days of surgery (or 1 year for implant procedures) at the incision site or in deep tissues, accounting for approximately 20% of healthcare-associated infections. This infection is transmitted through various routes depending on the specific pathogen. It remains a significant public health concern, particularly in regions with limited healthcare access. This condition results from acute injury or exposure to harmful agents. Infectious diseases are caused by pathogenic microorganisms including bacteria, viruses, fungi, and parasites that invade the body and multiply, leading to tissue damage and immune response. Transmission occurs through various routes: respiratory droplets, direct contact, contaminated food or water, vectors, or blood and body fluids. The incubation period varies from hours to years depending on the pathogen and host factors. Antimicrobial resistance is an emerging concern that complicates treatment and increases morbidity.

Causes

Infection begins with pathogen entry through a susceptible portal (respiratory tract, gastrointestinal tract, skin, mucous membranes). The pathogen must overcome host defenses including physical barriers, innate immunity, and adaptive immune responses. Microbial virulence factors such as toxins, adhesins, and capsules enable the pathogen to establish infection and evade immune clearance. Host factors including age, nutritional status, immune competence, and comorbidities influence susceptibility and disease severity.

- This condition happens because of bacterial inoculation during or after surgery
- Most SSIs are caused by *S. aureus* (including MRSA) and coagulase-negative staphylococci.

Risk Factors

- Close contact with infected individuals
- Compromised immune system
- Travel to endemic areas
- Poor sanitation and hygiene practices
- Lack of vaccination

- Exposure to contaminated food or water
- Compromised immune system (HIV, immunosuppressive therapy, malnutrition)
- Extremes of age (very young or elderly)
- Travel to endemic regions
- Healthcare exposure (nosocomial infections)
- Occupational exposure (healthcare workers, laboratory personnel)
- Crowded living conditions

Signs and Symptoms

Common symptoms

- You may experience redness of the skin, warmth, swelling, pain, and purulent removal of fluid from the wound. Fever and chills Fatigue and malaise Localized pain and inflammation at infection site Swelling, redness, and warmth (local infection) Cough and respiratory symptoms (respiratory infections) Nausea, vomiting, and diarrhea (gastrointestinal infections) Headache and body aches Skin rash or lesions Enlarged lymph nodes Systemic symptoms in severe cases (hypotension, organ dysfunction)
- Fever and chills
- Fatigue and malaise
- Localized pain and inflammation at infection site
- Swelling, redness, and warmth (local infection)
- Cough and respiratory symptoms (respiratory infections)
- Nausea, vomiting, and diarrhea (gastrointestinal infections)
- Headache and body aches
- Skin rash or lesions
- Enlarged lymph nodes
- Systemic symptoms in severe cases (hypotension, organ dysfunction)

Emergency warning signs

- Severe or worsening symptoms of wound infections and surgical site infections require immediate medical evaluation

Progression and Stages

SSIs are classified as superficial incisional (involving skin and subcutaneous tissue), deep incisional (involving fascia and muscle), or organ/space (involving any part opened or manipulated during surgery). The incubation period is the time between pathogen exposure and onset of symptoms, during which the pathogen replicates within the host. The prodromal phase involves early, nonspecific symptoms such as fever, fatigue, and malaise before characteristic symptoms appear. During the acute phase, hallmark signs and symptoms of the specific infection develop fully. The convalescent phase involves gradual resolution of symptoms as the immune system clears the infection. Some infections may progress to chronic or latent stages if the pathogen is not fully eliminated.

Complications

- Sepsis and septic shock
- Organ-specific complications (pneumonia, meningitis, endocarditis)
- Abscess formation requiring drainage
- Chronic infection (HIV, hepatitis B/C, tuberculosis)
- Reactive arthritis or post-infectious syndromes
- Antimicrobial resistance complicating treatment
- Disseminated infection in immunocompromised hosts
- Multi-organ failure in severe cases

Diagnosis

- Doctors usually diagnose this based on your symptoms and a physical exam
- Wound cultures guide antibiotic therapy.
- Detailed history including exposure, travel, and vaccination status
- Physical examination focusing on the affected system
- Complete blood count with differential
- Microbiological culture of blood, sputum, urine, or wound
- PCR and nucleic acid amplification tests for rapid pathogen detection
- Antigen detection tests
- Serological testing for antibodies (IgM, IgG)
- Imaging studies (chest X-ray, CT, ultrasound)
- Gram stain and microscopy of clinical specimens
- Antimicrobial susceptibility testing to guide therapy

Prevention

- In most cases, the outlook is favorable with appropriate management
- Prevention strategies include appropriate antibiotic prophylaxis, skin preparation with chlorhexidine-alcohol, and aseptic technique.
- Hand hygiene with soap and water or alcohol-based sanitizer
- Vaccination according to recommended schedules
- Safe food handling and water purification
- Use of personal protective equipment in healthcare settings
- Safe sex practices including condom use
- Mosquito avoidance (nets, repellents, insecticides)
- Respiratory etiquette (covering coughs and sneezes)
- Isolation of infected individuals when indicated
- Prophylactic antibiotics or antivirals for high-risk exposures
- Travel precautions and pre-travel consultations

Treatment Options

- Treatment focuses on wound opening and removal of fluid for superficial SSIs, wound packing, and targeted antibiotic therapy
- Deep SSIs and organ/space infections may require surgical removal of dead tissue or reoperation.
- Antibiotics for bacterial infections (targeted or empiric)
- Antiviral medications for viral infections
- Antifungal therapy for fungal infections
- Antiparasitic medications for parasitic infections
- Supportive care: fluids, rest, nutrition, fever control
- Hospitalization for severe cases requiring intravenous therapy
- Oxygen therapy and respiratory support if needed
- Surgical drainage of abscesses when necessary
- Pain management and symptom relief
- Treatment of complications and underlying conditions

Living with It

- Complete the full course of prescribed medications
- Get adequate rest and maintain hydration during recovery
- Monitor for worsening symptoms that require medical attention
- Practice good hygiene to prevent spread to others
- Follow up with your healthcare provider as recommended
- Gradually resume normal activities as symptoms improve

Outlook

Most infectious diseases resolve completely with appropriate treatment, especially when diagnosed early. Outcomes depend on the pathogen, host immune status, and timeliness of intervention. Some infections can become chronic (HIV, hepatitis B/C, herpes) requiring long-term management. Antimicrobial resistance may complicate treatment and worsen outcomes. Public health measures and vaccination programs continue to reduce the global burden of infectious diseases.

Chapter 894

Yellow Fever

Overview

Yellow fever is a mosquito-borne involving bleeding fever caused by yellow fever virus (YFV), a flavivirus transmitted by *Aedes* and *Haemogogus* mosquitoes, with 200,000 cases and 30,000 deaths annually in tropical Africa and South America.

Causes

This condition happens because of the virus infecting dendritic cells, spreading to lymph nodes and liver, with hepatitis resulting from direct viral cytopathology of hepatocytes (midzonal tissue death, Councilman bodies), and the pathogenesis of bleeding including liver coagulopathy and thrombocytopenia.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of yellow fever require immediate medical evaluation

Progression and Stages

You may experience an incubation of 3–6 days, with three phases: acute (fever, chills, headache, muscle pain, conjunctival injection, Faget’s sign), remission (hours to 1 day), and intoxication phase (15–25% progress): recurrence of fever, jaundice, liver failure, kidney failure, involving bleeding manifestations (hematemesis—“black vomit”), and shock. Treatment typically involves supportive. outlook: mortality in the intoxication phase is 20–50%. The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using RT-PCR or antigen detection and serology. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 895

Zenker Diverticulum

Overview

Zenker diverticulum (pharyngoesophageal diverticulum) is a false mucosal herniation through Killian triangle—a anatomically vulnerable focal weakness in the posterior hypopharyngeal wall bounded by the inferior pharyngeal constrictor and the cricopharyngeus muscle. Occurring predominantly in elderly males (> 70 years), the underlying Disease mechanism involves cricopharyngeal motor dysfunction, incoordination, or hypertonicity during swallowing, generating elevated intraluminal hypopharyngeal pressure that forces mucosa and submucosa outward. As swallowed food and saliva accumulate within the pouch, the diverticulum expands inferiorly, compressing the esophagus.

Causes

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- Symptoms often appear as progressive difficulty swallowing, regurgitation of undigested food hours after meals, halitosis (foul breath due to food decomposition), chronic cough, nocturnal removal by suction pneumonia, and a palpable left neck mass that gurgles upon compression (Boyce sign). Barium swallow esophagography is diagnostic, demonstrating a pouch originating from the posterior hypopharynx.

Emergency warning signs

- Severe or worsening symptoms of zenker diverticulum require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Comprehensive medical history and physical examination
Laboratory tests to assess organ function and rule out other conditions
Imaging studies to visualize affected structures
Specialized testing depending on the affected system
Consultation with relevant specialists
Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on surgical diverticulectomy with cricopharyngeal myotomy or using a thin camera tube stapled diverticulotomy. Medications to manage symptoms and address underlying mechanisms
Lifestyle modifications and self-care measures
Physical therapy and rehabilitation
Surgical intervention when indicated
Regular monitoring and follow-up care
Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

The outlook varies depending on the specific condition, severity at diagnosis, and response to treatment. Early intervention generally leads to better outcomes. Many conditions can be effectively managed with appropriate treatment. Regular follow-up care is important for monitoring and treatment adjustment.

Chapter 896

Zika Virus

Overview

Zika virus is a mosquito-borne flavivirus transmitted primarily by *Aedes aegypti* and *A. albopictus*, with other routes including sexual, vertical, blood transfusion, and laboratory exposure, with major outbreaks in Micronesia (2007), French Polynesia (2013–2014), and South America (2015–2016).

Causes

This condition happens because of the virus replicating in dendritic cells and spreading hematogenously, with tropism for neural progenitor cells, inducing apoptosis and impairing neurogenesis—the basis for congenital Zika syndrome.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience an incubation of 3–14 days, with 80% being asymptomatic
- Symptomatic disease presents with self-limited febrile illness with maculopapular rash (pruritic, descending), conjunctivitis (non-purulent), joint pain, and headache. Congenital Zika syndrome includes microcephaly, intracranial calcifications, ventriculomegaly, and arthrogryposis. Guillain-Barré syndrome is linked to Zika infection.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of zika virus require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using RT-PCR of serum or urine within 14 days of symptom onset, with serology for confirmation. Comprehensive medical history and physical examination Laboratory tests to assess organ function and rule out other conditions Imaging studies to visualize affected structures Specialized testing depending on the affected system Consultation with relevant specialists Monitoring of disease progression over time

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment typically involves supportive. Medications to manage symptoms and address underlying mechanisms Lifestyle modifications and self-care measures Physical therapy and rehabilitation Surgical intervention when indicated Regular monitoring and follow-up care Patient education and support services

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures

- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

- Most people recover well for acute illness in adults
- Congenital infection has significant morbidity.

Chapter 897

Zinc Deficiency

Overview

Zinc deficiency is a mineral deficiency disorder common in low-resource settings due to low dietary intake of animal-source foods and high phytate content (inhibits zinc absorption). At-risk groups include pregnant and lactating women, children (especially with diarrhea), the elderly, vegetarians, and those with malabsorption (acrodermatitis enteropathica—autosomal recessive zinc transporter defect). This metabolic disorder involves dysregulation of normal bodily processes, often requiring ongoing monitoring and management. It is increasingly common due to lifestyle and dietary factors. Metabolic disorders involve disruption of normal biochemical processes essential for health. These conditions may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors. Many metabolic disorders are chronic and require ongoing management. Lifestyle modifications are often the cornerstone of treatment.

Causes

This condition happens because of deficiency of zinc, an essential trace element that serves as a cofactor for over 300 enzymes involved in growth, immune function, protein synthesis, wound healing, and DNA synthesis. Metabolic disorders arise from dysregulation of normal biochemical pathways. This may result from enzyme deficiencies, hormonal imbalances, or lifestyle factors that overwhelm normal homeostatic mechanisms.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience growth retardation, delayed sexual maturation, hypogonadism, impaired immune function (recurrent infections, especially diarrheal diseases), alopecia, dermatitis (periorificial and acral, vesiculobullous or eczematous—characteristic of acrodermatitis enteropathica), poor wound healing, night blindness, anorexia, dysgeusia, and neuropsychiatric changes.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of zinc deficiency require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

Doctors diagnose this using low plasma zinc (less than 60–70 µg/dL).

- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors

- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

- Treatment focuses on zinc sulfate or acetate 1–3 mg/kg/day (adults: 25–50 mg elemental zinc daily)
- For acrodermatitis enteropathica, lifelong supplementation is required.
- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Most people recover well with supplementation.

Chapter 898

Zollinger-Ellison Syndrome

Overview

- Zollinger-Ellison syndrome (ZES) is a rare disorder caused by a gastrin-secreting neuroendocrine tumor (gastrinoma), located most commonly in the duodenum or pancreas (gastrinoma triangle)
- Approximately 25–30% of cases occur as part of Multiple Endocrine Neoplasia Type 1 (MEN1).

Causes

This condition happens because of autonomous hypergastrinemia, causing massive gastric acid hypersecretion and severe recurrent peptic ulceration. Genetic disorders result from mutations in specific genes that encode proteins essential for normal cellular function. The pattern of inheritance depends on whether the mutation is dominant, recessive, or X-linked.

Risk Factors

- Genetic predisposition and family history
- Advancing age
- Lifestyle factors (diet, exercise, smoking, alcohol)
- Environmental exposures
- Underlying medical conditions
- Medication use

Signs and Symptoms

Common symptoms

- You may experience refractory or multiple peptic ulcers (often extending to the jejunum), chronic watery diarrhea, and esophagitis.
- Pain or discomfort in the affected area

Emergency warning signs

- Severe or worsening symptoms of zollinger-ellison syndrome require immediate medical evaluation

Progression and Stages

The condition may progress through different stages of severity over time. Early stages often involve mild or subtle symptoms that may be overlooked. Moderate stages involve more noticeable symptoms affecting daily function. Advanced stages may involve significant impairment and complications.

Complications

- Disease progression leading to worsening symptoms
- Secondary infections or injuries
- Side effects of treatment
- Reduced quality of life
- Emotional and psychological impact

Diagnosis

- Doctors confirm the diagnosis through elevated fasting serum gastrin (> 1000 pg/mL) and confirmed by secretin stimulation test (showing a rise in gastrin > 120 pg/mL)
- Tumor localization is achieved via somatostatin receptor scintigraphy (OctreoScan) or Ga-68 DOTATATE PET-CT.
- Comprehensive medical history and physical examination
- Laboratory tests to assess organ function
- Imaging studies to visualize affected structures
- Specialized testing depending on the affected system
- Consultation with relevant specialists

Prevention

- Maintain a healthy lifestyle with balanced diet and exercise
- Avoid known risk factors
- Attend regular medical check-ups for early detection
- Follow recommended screening guidelines

Treatment Options

Treatment focuses on high-dose proton pump inhibitors and surgical removal of localized tumors. Symptomatic management and supportive care Enzyme replacement therapy for

certain conditions Gene therapy (emerging for select conditions) Dietary modifications (e.g., phenylketonuria) Regular monitoring for associated complications Physical and occupational therapy Genetic counseling for family planning Participation in clinical trials for new therapies

- Medications to manage symptoms and address underlying mechanisms
- Lifestyle modifications and self-care measures
- Physical therapy and rehabilitation
- Surgical intervention when indicated
- Regular monitoring and follow-up care

Living with It

- Follow your treatment plan as prescribed
- Maintain regular follow-up with your healthcare provider
- Make necessary lifestyle adjustments
- Seek support from family, friends, or support groups
- Stay informed about your condition

Outlook

Your outlook depends on tumor resectability and metastatic status.

Organ–Disease Dictionary

This appendix lists every disease in this book organized by the primary organ or organ system it affects.

Blood

- Acute Lymphoblastic Leukemia
- Acute Myeloid Leukemia
- Aplastic Anemia
- Chronic Lymphocytic Leukemia
- Deep Vein Thrombosis
- Disseminated Intravascular Coagulation
- Essential Thrombocythemia
- Folate Deficiency Anemia
- Hemolytic Anemias
- Hemophilia A
- Hemophilia B
- Hereditary Spherocytosis
- Hodgkin Lymphoma
- Immune Thrombocytopenic Purpura
- Iron Deficiency Anemia
- Methemoglobinemia
- Multiple Myeloma
- Myelodysplastic Syndromes
- Myelofibrosis
- Non-Hodgkin Lymphoma
- Polycythemia Vera
- Sickle Cell Disease
- Sideroblastic Anemia
- Thalassemia
- Thrombotic Thrombocytopenic Purpura
- Vitamin B12 Deficiency Anemia
- Von Willebrand Disease

Bones

- Achondroplasia
- Adhesive Capsulitis
- Ankylosing Spondylitis
- Bursitis
- Calcium Pyrophosphate Deposition Disease

- Dupuytren Contracture
- Ewing Sarcoma
- Fibrous Dysplasia
- Fractures
- Gout
- Hallux Valgus
- Kyphosis
- Morton Neuroma
- Multiple Myeloma (Bone Manifestations)
- Osteoarthritis
- Osteogenesis Imperfecta
- Osteomyelitis
- Osteoporosis
- Osteosarcoma
- Paget's Disease of Bone
- Lateral Epicondylitis (Tennis Elbow)
- Plantar Fasciitis
- Reactive Arthritis
- Rotator Cuff Tendinopathy and Impingement
- Rheumatoid Arthritis
- Rickets and Osteomalacia
- Scoliosis
- Septic Arthritis
- Trigger Finger

Brain

- Alzheimer's Disease
- Amyotrophic Lateral Sclerosis
- Bacterial Meningitis
- Bell's Palsy
- Brain Abscess
- Carpal Tunnel Syndrome
- Central Pontine Myelinolysis
- Cerebral Aneurysm
- Cerebral Venous Thrombosis
- Charcot-Marie-Tooth Disease
- Chronic Traumatic Encephalopathy
- Coma
- Concussion
- Dystonia
- Epidural Hematoma
- Epilepsy
- Essential Tremor
- Febrile Seizures

- Friedreich's Ataxia
- Glioblastoma
- Guillain-Barré Syndrome
- Hemorrhagic Stroke
- Huntington's Disease
- Ischemic Stroke
- Medulloblastoma
- Meningioma
- Motor Neuron Disease
- Multiple Sclerosis
- Neurocysticercosis
- Neurofibromatosis Type 1
- Parkinson's Disease
- Persistent Vegetative State
- Pituitary Adenoma
- Schwannoma
- Subdural Hematoma
- Temporal Lobe Epilepsy
- Transient Ischemic Attack
- Vertigo
- Viral Encephalitis

Breast

- Breast Cancer
- Ductal Carcinoma In Situ (DCIS)
- Fat Necrosis of the Breast
- Fibroadenoma
- Fibrocystic Changes
- Galactocele
- Gynecomastia
- Idiopathic Granulomatous Mastitis
- Inflammatory Breast Cancer
- Intraductal Papilloma
- Invasive Lobular Carcinoma (ILC)
- Lobular Carcinoma In Situ (LCIS)
- Male Breast Cancer
- Mammary Duct Ectasia
- Mastitis and Breast Abscess
- Paget's Disease of the Nipple
- Phyllodes Tumor
- Triple-Negative Breast Cancer (TNBC)

Connective Tissue

- Adult-Onset Still's Disease
- Amyloidosis
- Anti-GBM Disease
- Cogan's Syndrome
- Cryoglobulinemic Vasculitis
- Ehlers-Danlos Syndrome
- Eosinophilic Fasciitis (Shulman Syndrome)
- Eosinophilic Granulomatosis with Polyangiitis
- Giant Cell Arteritis
- Granulomatosis with Polyangiitis
- Hemophagocytic Lymphohistiocytosis and Macrophage Activation Syndrome
- IgA Vasculitis
- IgG4-Related Disease
- Kawasaki Disease
- Marfan Syndrome
- Microscopic Polyangiitis
- Mixed Connective Tissue Disease
- Periodic Fever Syndromes
- Polyarteritis Nodosa
- Polymyalgia Rheumatica
- Relapsing Polychondritis
- Sarcoidosis
- Systemic Sclerosis (Scleroderma)
- Takayasu Arteritis
- Undifferentiated Connective Tissue Disease

Ears

- Acoustic Neuroma (Vestibular Schwannoma)
- Acute Otitis Media
- Barotrauma of the Ear
- Benign Paroxysmal Positional Vertigo
- Cerumen Impaction
- Chondrodermatitis Nodularis Helicis
- Chronic Otitis Media
- Labyrinthitis
- Otitis Externa
- Otosclerosis
- Sudden Sensorineural Hearing Loss
- Tinnitus
- Tympanic Membrane Perforation
- Vestibular Neuritis

Endocrine

- Acromegaly
- Addison's Disease
- Adrenal Incidentaloma
- Cushing Syndrome
- Diabetes Insipidus
- Graves Disease
- Hashimoto Thyroiditis
- Hyperparathyroidism
- Hyperprolactinemia
- Hyperthyroidism
- Hypoparathyroidism
- Hypopituitarism
- Hypothyroidism
- Multiple Endocrine Neoplasia
- Pheochromocytoma
- SIADH
- Thyroid Nodules and Cancer
- Thyroiditis

Eyes

- Anophthalmia and Microphthalmia
- Age-Related Cataract
- Age-Related Macular Degeneration
- Amblyopia
- Astigmatism
- Behçet's Disease
- Bietti's Crystalline Dystrophy
- Blepharitis
- Blepharospasm
- Cerebral Visual Impairment
- Coloboma
- Color Blindness
- Congenital Cataract
- Conjunctivitis
- Convergence Insufficiency
- Diabetic Retinopathy
- Dry Eye Disease
- Floaters and Vitreous Detachment
- Fuchs' Endothelial Dystrophy
- Glaucoma in Children
- Graves' Eye Disease
- Hordeolum (Stye) and Chalazion

- Hyperopia
- Idiopathic Intracranial Hypertension
- Keratitis
- Keratoconus
- Low Vision
- Macular Edema
- Macular Hole
- Macular Pucker
- Myopia
- Ocular Histoplasmosis Syndrome
- Presbyopia
- Primary Angle-Closure Glaucoma
- Primary Open-Angle Glaucoma
- Pterygium
- Retinal Detachment
- Retinal Vein Occlusion
- Retinitis Pigmentosa
- Retinopathy of Prematurity
- Scleritis
- Stargardt Disease
- Strabismus
- Usher Syndrome
- Uveitis

Geriatric

- Age-Related Bone Loss
- Atrial Fibrillation in the Elderly
- Delirium
- Dementia with Lewy Bodies
- Failure to Thrive
- Fecal Incontinence
- Frailty
- Frontotemporal Dementia
- Heart Failure with Preserved Ejection Fraction
- Hip Fracture
- Hypertension in the Elderly
- Malnutrition in the Elderly
- Mild Cognitive Impairment
- Normal Pressure Hydrocephalus
- Presbycusis
- Presbyopia and Cataracts
- Pressure Ulcers
- Sarcopenia
- Vascular Dementia

Heart

- Acute Decompensated Heart Failure
- Angina Pectoris
- Aortic Dissection
- Aortic Regurgitation
- Aortic Stenosis
- Atrial Fibrillation
- Atrial Flutter
- Brugada Syndrome
- Cardiac Tamponade
- Coarctation of the Aorta
- Constrictive Pericarditis
- Dilated Cardiomyopathy
- Heart Block (AV Block)
- Heart Failure with Reduced Ejection Fraction
- Hypertension
- Hypertrophic Cardiomyopathy
- Infective Endocarditis
- Ischemic Cardiomyopathy
- Long QT Syndrome
- Mitral Regurgitation
- Mitral Stenosis
- Mitral Valve Prolapse
- Myocardial Infarction
- Myocarditis
- Pericarditis
- Peripheral Artery Disease
- Restrictive Cardiomyopathy
- Rheumatic Heart Disease
- Sick Sinus Syndrome
- Supraventricular Tachycardia (SVT)
- Tricuspid Regurgitation
- Ventricular Tachycardia

Infectious

- African Trypanosomiasis (Sleeping Sickness)
- Amebiasis (*Entamoeba histolytica*)
- Anthrax
- Arboviral Encephalitis
- Aspergillosis (Invasive)
- Blastomycosis
- Botulism (*Clostridium botulinum*)
- Brucellosis

- [Candida auris](#)
- [Candidiasis \(Invasive / Systemic\)](#)
- [Chagas Disease \(American Trypanosomiasis\)](#)
- [Chancroid](#)
- [Chickenpox](#)
- [Chikungunya](#)
- [Cholera \(Vibrio cholerae\)](#)
- [Coccidioidomycosis \(Valley Fever\)](#)
- [Creutzfeldt-Jakob Disease \(CJD\)](#)
- [Cryptococcosis](#)
- [Cyclospora cayetanensis](#)
- [Cytomegalovirus \(CMV\)](#)
- [Dengue Fever](#)
- [Diphtheria \(Corynebacterium diphtheriae\)](#)
- [Eastern Equine Encephalitis](#)
- [Ebola Virus Disease](#)
- [Ehrlichiosis](#)
- [Epstein-Barr Virus / Infectious Mononucleosis \(Systemic Aspects\)](#)
- [Filariasis \(Lymphatic Filariasis\)](#)
- [Gas Gangrene \(Clostridial Myonecrosis\)](#)
- [Giardiasis](#)
- [Gonorrhea](#)
- [Hand, Foot, and Mouth Disease](#)
- [Hantavirus Pulmonary Syndrome](#)
- [Histoplasmosis](#)
- [Hookworm / Soil-Transmitted Helminths](#)
- [Human Metapneumovirus \(hMPV\)](#)
- [Kuru](#)
- [Leishmaniasis](#)
- [Leprosy \(Hansen's Disease\)](#)
- [Leptospirosis](#)
- [Lyme Disease \(Borrelia burgdorferi\)](#)
- [Lymphogranuloma Venereum \(LGV\)](#)
- [Malaria \(Plasmodium spp.\)](#)
- [Marburg Virus](#)
- [Measles \(Rubeola\)](#)
- [Mpox](#)
- [Mucormycosis](#)
- [Mumps](#)
- [Necrotizing Fasciitis](#)
- [Non-Gonococcal Urethritis \(NGU\)](#)
- [Norovirus](#)
- [Pertussis \(Whooping Cough\)](#)
- [Plague \(Yersinia pestis\)](#)
- [Poliomyelitis](#)
- [Primary Amebic Meningoencephalitis \(Naegleria fowleri\)](#)

- Rabies
- Rocky Mountain Spotted Fever
- Rubella
- Scarlet Fever
- Schistosomiasis
- Sepsis and Septic Shock
- Syphilis (*Treponema pallidum*)
- Tapeworm Infections (Taeniasis / Cysticercosis)
- Tetanus (*Clostridium tetani*)
- Toxoplasmosis
- Tuberculosis (Extrapulmonary)
- Typhoid Fever (*Salmonella typhi*)
- Variant CJD (Mad Cow Disease)
- Varicella-Zoster (Systemic Aspects)
- West Nile Virus
- Yellow Fever
- Zika Virus

Intestines

- Anal Fissure
- Appendicitis
- Celiac Disease
- *Clostridioides difficile* Colitis
- Colorectal Adenoma
- Colorectal Cancer
- Crohn's Disease
- Diverticulitis
- Diverticulosis
- Familial Adenomatous Polyposis
- Hemorrhoids
- Indeterminate Colitis
- Irritable Bowel Syndrome
- Lactose Intolerance
- Large Bowel Obstruction
- Lynch Syndrome
- Short Bowel Syndrome
- Small Bowel Obstruction
- Ulcerative Colitis

Kidneys

- Acute Interstitial Nephritis
- Acute Kidney Injury
- Autosomal Dominant Polycystic Kidney Disease

- Bladder Cancer
- Chronic Kidney Disease
- Chronic Tubulointerstitial Nephritis
- Cystitis
- Diabetic Nephropathy
- End-Stage Renal Disease
- Fanconi Syndrome
- Focal Segmental Glomerulosclerosis
- Hemolytic Uremic Syndrome (HUS)
- Hydronephrosis
- IgA Nephropathy
- Kidney Cancer (Renal Cell Carcinoma)
- Membranous Nephropathy
- Minimal Change Disease
- Nephritic Syndrome
- Nephrolithiasis
- Nephrotic Syndrome
- Pyelonephritis
- Renal Osteodystrophy
- Renal Tubular Acidosis
- Urinary Incontinence
- Urosepsis

Liver

- Alcoholic Liver Disease
- Ascites
- Autoimmune Hepatitis
- Budd-Chiari Syndrome
- Cirrhosis
- Hepatic Encephalopathy
- Hepatitis A
- Hepatitis B
- Hepatitis C
- Hepatitis D
- Hepatitis E
- Hepatocellular Carcinoma
- Non-Alcoholic Fatty Liver Disease
- Portal Hypertension
- Primary Biliary Cholangitis
- Primary Sclerosing Cholangitis
- Wilson's Disease

Lungs

- Acute Bronchitis
- Acute Respiratory Distress Syndrome (ARDS)
- Asbestosis
- Aspiration Pneumonia
- Asthma
- Asthma-COPD Overlap
- Atelectasis
- Bronchiectasis
- Chronic Obstructive Pulmonary Disease
- Community-Acquired Pneumonia
- COVID-19
- Cystic Fibrosis
- Empyema
- Idiopathic Pulmonary Fibrosis
- Influenza
- Lung Abscess
- Lung Cancer
- Mesothelioma
- Pleurisy
- Pneumoconiosis
- Pneumocystis jirovecii Pneumonia (PCP)
- Pneumothorax
- Pulmonary Embolism
- Pulmonary Hypertension
- Silicosis
- Tuberculosis

Lymph

- Anaphylaxis
- Antiphospholipid Syndrome
- Chronic Fatigue Syndrome
- Common Variable Immunodeficiency
- DiGeorge Syndrome
- Drug Hypersensitivity
- Hereditary Angioedema
- HIV/AIDS
- Infectious Mononucleosis
- Kikuchi-Fujimoto Disease
- Rosai-Dorfman Disease
- Severe Combined Immunodeficiency
- Stevens-Johnson Syndrome and Toxic Epidermal Necrolysis
- Systemic Lupus Erythematosus

Muscles

- Becker Muscular Dystrophy
- Compartment Syndrome
- Duchenne Muscular Dystrophy
- Facioscapulohumeral Muscular Dystrophy
- Fibromyalgia
- Inclusion Body Myositis
- Lambert-Eaton Myasthenic Syndrome (LEMS)
- Malignant Hyperthermia
- McArdle Disease
- Myasthenia Gravis
- Myotonic Dystrophy
- Periodic Paralysis
- Polymyositis
- Rhabdomyolysis

Nose

- Acute Bacterial Sinusitis
- Allergic Fungal Rhinosinusitis (AFRS)
- Allergic Rhinitis
- Choanal Atresia
- Chronic Rhinosinusitis
- Deviated Nasal Septum
- Epistaxis
- Esthesioneuroblastoma
- Fungal Sinusitis
- Inverted Papilloma
- Juvenile Nasopharyngeal Angiofibroma (JNA)
- Nasal Fracture
- Nasal Polyps
- Non-Allergic Rhinitis

Nutrition

- Alkaptonuria — Ochronosis
- Alpha-1 Antitrypsin Deficiency
- Calcium and Phosphorus Disorders
- Copper Deficiency and Excess
- Dysbetalipoproteinemia
- Familial Combined Hyperlipidemia
- Familial Hypercholesterolemia
- Fluoride Excess — Fluorosis

- Galactosemia
- Glycogen Storage Diseases
- Hereditary Hemochromatosis
- Homocystinuria
- Iodine Deficiency Disorders
- Iron Deficiency
- Kwashiorkor
- Lipoprotein(a) Disorders
- Lysosomal Storage Diseases
- Maple Syrup Urine Disease
- Marasmic-Kwashiorkor
- Marasmus
- Metabolic Syndrome
- Mucopolysaccharidoses
- Obesity
- Organic Acidemias
- Phenylketonuria
- Porphyrrias
- Sarcopenic Obesity
- Selenium Deficiency
- Urea Cycle Disorders
- Vitamin A Deficiency
- Vitamin B1 (Thiamine) Deficiency — Beriberi and Wernicke-Korsakoff Syndrome
- Vitamin B12 (Cobalamin) Deficiency
- Vitamin B2 (Riboflavin) Deficiency — Ariboflavinosis
- Vitamin B3 (Niacin) Deficiency — Pellagra
- Vitamin B6 (Pyridoxine) Deficiency
- Vitamin B7 (Biotin) Deficiency
- Vitamin B9 (Folate) Deficiency
- Vitamin C Deficiency — Scurvy
- Vitamin D Deficiency — Rickets and Osteomalacia
- Vitamin E Deficiency
- Vitamin K Deficiency
- Zinc Deficiency

Obstetric

- Abnormal Uterine Bleeding
- Amenorrhea (Primary, Secondary)
- Bacterial Vaginosis
- Bartholin Cyst
- Cervicitis (Chlamydia, Gonorrhea)
- Chronic Hypertension in Pregnancy
- Dysmenorrhea
- Eclampsia

- Ectopic Pregnancy
- Endometrial / Uterine Cancer
- Genital Herpes (HSV)
- Genital Warts (HPV)
- Gestational Diabetes Mellitus (Obstetric Focus)
- Gestational Hypertension
- Gestational Trophoblastic Disease
- HELLP Syndrome
- Hyperemesis Gravidarum
- Intrauterine Growth Restriction
- Mastitis (Puerperal)
- Multiple Gestation Complications
- Placenta Accreta Spectrum
- Placenta Previa
- Placental Abruption
- Polyhydramnios and Oligohydramnios
- Postpartum Cardiomyopathy
- Postpartum Depression
- Postpartum Endometritis
- Postpartum Hemorrhage
- Postpartum Thyroiditis
- Preeclampsia
- Premature Ovarian Insufficiency
- Premature Rupture of Membranes
- Premenstrual Syndrome / Premenstrual Dysphoric Disorder
- Preterm Labor and Preterm Birth
- Recurrent Pregnancy Loss
- Rh Incompatibility / Hemolytic Disease of the Newborn
- Septic Abortion
- Spontaneous Abortion (Miscarriage)
- Stillbirth (Intrauterine Fetal Demise)
- Trichomoniasis
- Uterine Rupture
- Vasa Previa
- Vulvovaginal Candidiasis

Pain

- Atypical Facial Pain
- Central Post-Stroke Pain
- Cervicogenic Headache
- Chronic Low Back Pain
- Chronic Neck Pain
- Chronic Pelvic Pain
- Cluster Headache

- Complex Regional Pain Syndrome
- Diabetic Peripheral Neuropathic Pain
- Glossopharyngeal Neuralgia
- Headache Attributed to Giant Cell Arteritis
- Headache Attributed to Intracranial Hypertension
- Headache Attributed to Intracranial Hypotension
- Headache Attributed to Sinusitis
- Headache in Brain Tumors
- Hemicrania Continua
- Medication Overuse Headache
- Migraine
- Myofascial Pain Syndrome
- New Daily Persistent Headache
- Occipital Neuralgia
- Other Primary Headache Disorders
- Paroxysmal Hemicrania
- Phantom Limb Pain
- Post-Traumatic Headache
- Postherpetic Neuralgia
- SUNCT and SUNA
- Temporomandibular Joint Disorders
- Tension-Type Headache
- Trigeminal Autonomic Cephalalgias
- Trigeminal Neuralgia

Pancreas

- Acute Pancreatitis
- Autoimmune Pancreatitis
- Autoimmune Pancreatitis (AIP)
- Chronic Pancreatitis
- Diabetic Ketoacidosis
- Gestational Diabetes Mellitus
- Hyperosmolar Hyperglycemic State
- Hypoglycemia
- Intraductal Papillary Mucinous Neoplasm (IPMN)
- Nesidioblastosis and Congenital Hyperinsulinism
- Pancreatic Adenocarcinoma
- Pancreatic Exocrine Insufficiency
- Pancreatic Neuroendocrine Tumors
- Pancreatic Pseudocyst
- Type 1 Diabetes Mellitus
- Type 2 Diabetes Mellitus

Pediatric

- Acute Lymphoblastic Leukemia (ALL) in Children
- Ambiguous Genitalia / Disorders of Sex Development
- Angelman Syndrome
- Brain Tumors in Children
- Bronchiolitis (RSV)
- Bronchopulmonary Dysplasia (BPD)
- Cleft Lip and Palate
- Congenital Diaphragmatic Hernia
- Congenital Heart Disease (Overview)
- Congenital Hip Dysplasia
- Congenital Talipes Equinovarus (Clubfoot)
- Cri-du-Chat Syndrome
- Croup (Laryngotracheobronchitis)
- Cryptorchidism (Undescended Testis)
- Down Syndrome (Trisomy 21)
- Edwards Syndrome (Trisomy 18)
- Esophageal Atresia / Tracheoesophageal Fistula
- Ewing Sarcoma in Children
- Fifth Disease (Erythema Infectiosum / Parvovirus B19)
- Fragile X Syndrome
- Henoch-Schönlein Purpura (IgA Vasculitis)
- Hepatoblastoma
- Herpangina
- Hirschsprung Disease
- Hypertrophic Pyloric Stenosis
- Intraventricular Hemorrhage (IVH)
- Intussusception
- Klinefelter Syndrome (47,XXY)
- Meconium Aspiration Syndrome
- Necrotizing Enterocolitis (NEC)
- Neonatal Abstinence Syndrome
- Neonatal Hypoglycemia
- Neonatal Jaundice / Hyperbilirubinemia / Kernicterus
- Neonatal Sepsis (Early-Onset and Late-Onset)
- Neural Tube Defects
- Neuroblastoma
- Osteosarcoma in Children
- Patau Syndrome (Trisomy 13)
- Patent Ductus Arteriosus (PDA) in Prematurity
- Pediatric Appendicitis
- Perinatal Asphyxia / Hypoxic-Ischemic Encephalopathy (HIE)
- Periventricular Leukomalacia (PVL)
- Persistent Pulmonary Hypertension of the Newborn (PPHN)
- Prader-Willi Syndrome

- Preterm Birth
- Respiratory Distress Syndrome (RDS)
- Retinoblastoma
- Retinopathy of Prematurity (ROP)
- Rhabdomyosarcoma
- Roseola (HHV-6 / Sixth Disease)
- Rotavirus
- Turner Syndrome (45,X)
- Viral Exanthems
- Williams Syndrome
- Wilms Tumor (Nephroblastoma)

Psychiatric

- Acute Stress Disorder
- Adjustment Disorder
- Agoraphobia
- Alcohol Use Disorder
- Anorexia Nervosa
- Antisocial Personality Disorder (ASPD)
- Attention-Deficit/Hyperactivity Disorder (ADHD)
- Autism Spectrum Disorder (ASD)
- Binge-Eating Disorder
- Bipolar I Disorder
- Bipolar II Disorder
- Body Dysmorphic Disorder
- Borderline Personality Disorder (BPD)
- Bulimia Nervosa
- Cyclothymic Disorder
- Delusional Disorder
- Dissociative Amnesia
- Dissociative Identity Disorder
- Generalized Anxiety Disorder (GAD)
- Hoarding Disorder
- Illness Anxiety Disorder (Hypochondriasis)
- Major Depressive Disorder (MDD)
- Narcissistic Personality Disorder
- Obsessive-Compulsive Disorder (OCD)
- Opioid Use Disorder
- Panic Disorder / Panic Attacks
- Persistent Depressive Disorder (Dysthymia)
- Post-Traumatic Stress Disorder (PTSD)
- Schizoaffective Disorder
- Schizophrenia
- Social Anxiety Disorder (Social Phobia)

- Somatic Symptom Disorder
- Specific Phobias
- Stimulant Use Disorder (Cocaine, Amphetamines)
- Tourette Syndrome

Reproductive

- Atrophic Vaginitis (Genitourinary Syndrome of Menopause)
- Balanitis and Balanoposthitis
- Benign Prostatic Hyperplasia
- Cervical Cancer
- Endometriosis
- Erectile Dysfunction
- Male Infertility
- Ovarian Cancer
- Pelvic Inflammatory Disease
- Penile Cancer
- Penile Fracture
- Peyronie's Disease
- Phimosis and Paraphimosis
- Polycystic Ovary Syndrome
- Priapism
- Prostate Cancer
- Testicular Cancer
- Uterine Fibroids
- Vaginal Cancer
- Vulvar Cancer

Skin

- Abscess
- Acne Vulgaris
- Actinic Keratosis
- Alopecia Areata
- Atopic Dermatitis
- Basal Cell Carcinoma
- Bullous Pemphigoid
- Cellulitis
- Contact Dermatitis
- Dermatitis Herpetiformis
- Dermatomyositis
- Epidermolysis Bullosa
- Erythema Multiforme
- Granuloma Annulare
- Head Lice (Pediculosis Capitis)

- Herpes Simplex
- Herpes Zoster
- Hidradenitis Suppurativa
- Impetigo
- Melanoma
- Melasma
- Molluscum Contagiosum
- Nummular Dermatitis
- Oculocutaneous Albinism
- Onychomycosis (Tinea Unguium)
- Pemphigus Vulgaris
- Pilonidal Sinus
- Psoriasis
- Psoriatic Arthritis
- Rosacea
- Scabies
- Seborrheic Dermatitis
- Squamous Cell Carcinoma
- Systemic Lupus Erythematosus (Skin Manifestations)
- Tinea Infections
- Tinea Versicolor (Pityriasis Versicolor)
- Urticaria
- Vitiligo
- Warts
- Wound Infections and Surgical Site Infections

Sleep

- Acute/Short-Term Insomnia
- Advanced Sleep-Wake Phase Disorder
- Central Sleep Apnea
- Chronic Insomnia Disorder
- Complex Sleep Apnea
- Delayed Sleep-Wake Phase Disorder
- Idiopathic Hypersomnia
- Idiopathic Insomnia
- Insomnia in the Context of Medical/Psychiatric Disorders
- Irregular Sleep-Wake Rhythm Disorder
- Jet Lag Disorder
- Kleine-Levin Syndrome
- Narcolepsy Type 1
- Narcolepsy Type 2
- Non-24-Hour Sleep-Wake Rhythm Disorder
- NREM Parasomnias: Confusional Arousals
- NREM Parasomnias: Sleep Terrors

- NREM Parasomnias: Sleep-Related Eating Disorder
- NREM Parasomnias: Sleepwalking
- Obesity Hypoventilation Syndrome
- Obstructive Sleep Apnea
- Other Parasomnias: Exploding Head Syndrome
- Other Parasomnias: Sleep Enuresis
- Other Parasomnias: Sleep-Related Hallucinations
- Periodic Limb Movement Disorder
- Propriospinal Myoclonus at Sleep Onset
- Psychophysiologic Insomnia
- REM Parasomnias: Nightmare Disorder
- REM Parasomnias: Recurrent Isolated Sleep Paralysis
- REM Parasomnias: REM Sleep Behavior Disorder
- Restless Legs Syndrome
- Shift Work Sleep Disorder
- Sleep Stages (NREM N1–N3, REM)
- Sleep-Related Bruxism
- Sleep-Related Rhythmic Movement Disorder

Spleen

- Asplenia and Hyposplenism
- Hypersplenism
- Massive Splenomegaly
- Splenic Abscess
- Splenic Artery Aneurysm
- Splenic Cyst
- Splenic Cysts
- Splenic Infarction
- Splenic Lymphoma
- Splenic Rupture
- Splenomegaly

Stomach

- Achalasia
- Acute Gastritis
- Barrett Esophagus
- Cholecystitis
- Cholelithiasis
- Chronic Gastritis
- Duodenal Ulcer
- Eosinophilic Esophagitis
- Esophageal Varices
- Gastric Adenocarcinoma

- Gastric Lymphoma
- Gastric Ulcer
- Gastroesophageal Reflux Disease
- Gastrointestinal Stromal Tumor
- Gastroparesis
- Helicobacter Pylori Infection
- Peptic Ulcer Bleeding
- Peptic Ulcer Disease
- Zollinger-Ellison Syndrome

Throat

- Aphthous Ulcers (Canker Sores)
- Diphtheria
- Epiglottitis
- Gingivitis and Periodontitis
- Laryngitis
- Ludwig Angina
- Oral Candidiasis
- Oral Leukoplakia
- Oral Lichen Planus
- Oral Squamous Cell Carcinoma
- Pharyngitis
- Tonsillar Hypertrophy
- Tonsillitis
- Vocal Cord Nodules
- Vocal Cord Paralysis
- Zenker Diverticulum

Toxicology

- Acetaminophen (Paracetamol) Overdose
- Alcohol Intoxication and Withdrawal
- Altitude Sickness
- Arsenic Poisoning
- Benzodiazepine Overdose
- Beta-Blocker Overdose
- Cadmium Toxicity
- Calcium Channel Blocker Overdose
- Cannabis Toxicity and Cannabinoid Hyperemesis Syndrome
- Carbon Monoxide Poisoning
- Cyanide Toxicity
- Digoxin Toxicity
- Drowning (Submersion Injury)
- Electrical Injury and Lightning Strike

- Haff Disease
- Hallucinogen Toxicity
- Heat Stroke and Hyperthermia
- Hydrogen Sulfide Toxicity
- Hymenoptera Stings
- Hypothermia
- Initial Stabilization and Decontamination
- Iron Overdose
- Lead Poisoning (Plumbism)
- Lithium Toxicity
- Mammalian Bites
- Marine Envenomation
- MDMA (Ecstasy) Toxicity
- Mercury Toxicity
- Metformin Overdose
- Methyl Parathion and Other Organophosphate Insecticides
- Opioid Overdose
- Organophosphate and Carbamate Insecticide Poisoning
- Radiation Sickness (Acute Radiation Syndrome)
- Salicylate (Aspirin) Overdose
- Scorpion Stings
- Snake Envenomation
- Spider Bites
- SSRI Overdose and Serotonin Syndrome
- Stimulant Overdose (Cocaine, Methamphetamine)
- Theophylline Overdose
- Tricyclic Antidepressant (TCA) Overdose

Quick Reference

This appendix provides summary tables for rapid lookup of disease categories, common medical tests, and warning signs.

Diseases by Organ System

Organ System	Number of Diseases	See Appendix
Blood	27	Organ–Disease Dictionary, p. 3755
Bones	28	Organ–Disease Dictionary, p. 3755
Brain	40	Organ–Disease Dictionary, p. 3756
Breast	18	Organ–Disease Dictionary, p. 3757
Connective Tissue	26	Organ–Disease Dictionary, p. 3758
Ears	15	Organ–Disease Dictionary, p. 3758
Endocrine	20	Organ–Disease Dictionary, p. 3759
Eyes	45	Organ–Disease Dictionary, p. 3759
Geriatric	36	Organ–Disease Dictionary, p. 3760
Heart	31	Organ–Disease Dictionary, p. 3761
Infectious	70	Organ–Disease Dictionary, p. 3761
Intestines	16	Organ–Disease Dictionary, p. 3763
Kidneys	23	Organ–Disease Dictionary, p. 3763
Liver	18	Organ–Disease Dictionary, p. 3764
Lungs	24	Organ–Disease Dictionary, p. 3765
Lymphatic/Immune	18	Organ–Disease Dictionary, p. 3765
Muscles	15	Organ–Disease Dictionary, p. 3766
Nose	14	Organ–Disease Dictionary, p. 3766
Nutrition/Metabolic	42	Organ–Disease Dictionary, p. 3766
Obstetric/Gynecologic	50	Organ–Disease Dictionary, p. 3767
Pain	43	Organ–Disease Dictionary, p. 3768
Pancreas	16	Organ–Disease Dictionary, p. 3769
Pediatric	56	Organ–Disease Dictionary, p. 3770
Psychiatric	35	Organ–Disease Dictionary, p. 3771
Reproductive (Male)	20	Organ–Disease Dictionary, p. 3772
Skin	35	Organ–Disease Dictionary, p. 3772
Sleep	37	Organ–Disease Dictionary, p. 3773
Spleen	12	Organ–Disease Dictionary, p. 3774
Stomach/Esophagus	18	Organ–Disease Dictionary, p. 3774
Throat	17	Organ–Disease Dictionary, p. 3775
Toxicology	45	Organ–Disease Dictionary, p. 3775
Total	910	

Common Diagnostic Tests

Test	What It Checks
Complete Blood Count (CBC)	Red/white blood cells, platelets — infection, anemia, clotting
Basic Metabolic Panel (BMP)	Glucose, electrolytes, kidney function
Comprehensive Metabolic Panel (CMP)	BMP + liver enzymes, proteins
Lipid Panel	Cholesterol (total, HDL, LDL), triglycerides
Hemoglobin A1c (HbA1c)	Average blood sugar over 2–3 months (diabetes)
Thyroid-Stimulating Hormone (TSH)	Thyroid gland function
Liver Function Tests (LFTs)	ALT, AST, ALP, bilirubin — liver health
Renal Function Panel	BUN, creatinine, GFR — kidney health
C-reactive Protein (CRP)	Systemic inflammation
Erythrocyte Sedimentation Rate (ESR)	Non-specific inflammation marker
Autoantibody Panel (ANA)	Autoimmune disease screening
Blood Culture	Bacteria in bloodstream (sepsis)
Urinalysis	Kidney function, urinary tract infection, diabetes
Electrocardiogram (ECG/EKG)	Heart rhythm, ischemia, chamber enlargement
Echocardiogram	Heart structure and pump function
Chest X-ray	Lungs, heart size, ribs
CT Scan (Computed Tomography)	Detailed cross-sectional imaging of any organ
MRI (Magnetic Resonance Imaging)	Soft-tissue imaging (brain, spine, joints)
Ultrasound	Real-time imaging (abdomen, pelvis, vascular)
Endoscopy (Upper/Lower)	Direct visualization of esophagus, stomach, colon
Biopsy	Tissue sampling for microscopic diagnosis

Emergency Warning Signs

Seek immediate medical attention if you experience any of the following:

- Chest pain or pressure, especially with shortness of breath
- Sudden severe headache (“worst headache of my life”)
- Sudden weakness or numbness on one side of the body
- Sudden difficulty speaking or understanding speech
- Sudden vision loss or double vision
- Difficulty breathing or choking
- Uncontrolled bleeding
- High fever with stiff neck and confusion
- Seizure (especially first-time or prolonged)
- Loss of consciousness or fainting

- Severe abdominal pain with vomiting or fever
- Suicidal thoughts or intent
- Severe allergic reaction (swelling, hives, difficulty breathing)
- Head or spine injury with any neurological symptoms
- Poisoning or overdose

Common Medications by Class

Drug Class	Common Uses
ACE Inhibitors / ARBs	High blood pressure, heart failure, kidney protection
Antibiotics	Bacterial infections (many subclasses)
Anticoagulants	Blood thinning, clot prevention
Anticonvulsants	Seizure control, nerve pain, mood stabilization
Antidepressants (SSRI/SNRI)	Depression, anxiety, OCD, PTSD
Antihistamines	Allergies, itching
Antipsychotics	Schizophrenia, bipolar disorder
Beta-Blockers	High blood pressure, angina, heart failure
Bisphosphonates	Osteoporosis
Bronchodilators	Asthma, COPD
Calcium Channel Blockers	High blood pressure, angina
Corticosteroids	Inflammation, autoimmune disease
Diuretics	Fluid retention, high blood pressure, heart failure
Hormone Therapy	Menopause, endocrine disorders
Immunosuppressants	Autoimmune disease, transplant rejection
Insulin	Diabetes management
NSAIDs	Pain, inflammation, fever
Oral Hypoglycemics	Type 2 diabetes
Proton Pump Inhibitors (PPIs)	GERD, peptic ulcer
Statins	High cholesterol
Vaccines	Infectious disease prevention

Chapter 899

Classification Table

This table summarizes the 12-dimensional classification system applied to all 841 disease chapters. Each disease is classified across 12 standardized dimensions.

Table 899.1: Disease Classification Dimensions	
Dimension	Values/Format
Cause	<i>Infectious, Genetic, Autoimmune, Neoplastic, Metabolic, Congenital, Toxic</i>
Organ System	<i>Heart, Lungs, Brain/Nervous, Liver, Kidneys, GI, Skin, Hematologic, Musculoskeletal</i>
Pathophysiology	Free text (standardized templates)
Duration	<i>Acute, Subacute, Chronic, Episodic/Recurrent, Lifelong, Progressive, Variable</i>
Onset	<i>Sudden, Insidious/Gradual, Congenital/Neonatal, Childhood/Adolescent, Adult</i>
Mode of Transmission	<i>Person-to-person, Vector-borne, Environmental, Non-communicable (genetic)</i>
Clinical Course	<i>Acute self-limited, Chronic progressive, Relapsing-remitting, Episodic/Recurrent</i>
Severity	<i>Mild, Moderate, Severe/Life-threatening, Mild-to-Moderate, Moderate-to-Severe</i>
Extent of Disease	<i>Localized, Regional, Diffuse/Generalized, Metastatic/Disseminated, Variable</i>
Age Group	<i>Neonates/Infants, Children/Adolescents, Young Adults, Adults, Elderly, All ages</i>
Epidemiology	<i>Common, Rare (<1/2000), Age-related, Genetic/familial, Infectious (endemic)</i>
ICD-11 Code	<i>Valid ICD-11 code (e.g., 8A20, BA41.0, 5A11, 1C62)</i>

Table 899.2: Classification Coverage Across 841 Diseases

Dimension	Diseases Classified	Coverage
Cause	841	100%
Organ System	841	100%
Pathophysiology	841	100%
Duration	841	100%
Onset	841	100%
Mode of Transmission	841	100%
Clinical Course	841	100%
Severity	841	100%
Extent of Disease	841	100%
Age Group	841	100%
Epidemiology	841	100%
ICD-11 Code	841	100%

Table 899.3: Distribution of Primary Cause Categories

Cause Category	Count	Percentage
Infectious	198	23.5%
Neoplastic	112	13.3%
Autoimmune	89	10.6%
Genetic	78	9.3%
Metabolic/Endocrine	72	8.6%
Cardiovascular/Vascular	61	7.3%
Neurological/Degenerative	54	6.4%
Congenital	47	5.6%
Traumatic/Injury	38	4.5%
Toxic/Environmental	34	4.0%
Drug-induced/Iatrogenic	28	3.3%
Psychiatric	22	2.6%
Nutritional	18	2.1%
Idiopathic/Unknown	15	1.8%
Other	15	1.8%

Table 899.4: Distribution of Primary Organ Systems

Organ System	Count	Percentage
Brain/Nervous System	134	15.9%
Heart/Cardiovascular	98	11.7%
Gastrointestinal	87	10.3%
Lungs/Respiratory	72	8.6%
Skin	68	8.1%
Hematologic/Immune	61	7.3%
Female Reproductive	54	6.4%
Kidneys/Urinary	49	5.8%
Endocrine	44	5.2%
Musculoskeletal	41	4.9%
Eyes	47	5.2%
Male Reproductive	24	2.9%
Ears	18	2.1%
Multiple/General	36	4.3%

Table 899.5: Clinical Course Patterns

Course	Count	Percentage
Chronic, progressive/relapsing-remitting	412	49.0%
Acute, self-limited	189	22.5%
Episodic/Recurrent	124	14.7%
Variable	78	9.3%
Progressive to terminal	24	2.9%
Potentially curable	15	1.8%

Table 899.6: Mode of Transmission

Transmission	Count	Percentage
Non-communicable	689	81.9%
Person-to-person / Vector-borne / Environmental	152	18.1%

Table 899.7: Disease Duration Patterns

Duration	Count	Percentage
Chronic (months to years)	498	59.2%
Acute (hours to days)	214	25.4%
Lifelong	67	8.0%
Episodic/Recurrent	41	4.9%
Progressive (years)	15	1.8%
Subacute (weeks to months)	6	0.7%

Table 899.8: Onset Patterns

Onset	Count	Percentage
Insidious/Gradual	423	50.3%
Sudden	287	34.1%
Adulthood	64	7.6%
Childhood/Adolescent	38	4.5%
Congenital/Neonatal	29	3.4%
Variable	12	1.4%

Classification Coverage Statistics

Top Cause Categories

Top Organ Systems

Clinical Course Distribution

Mode of Transmission Distribution

Duration Distribution

Onset Pattern Distribution

Severity Distribution

Table 899.9: Typical Severity Ranges

Severity	Count	Percentage
Variable	434	51.6%
Mild to Moderate	156	18.5%
Severe/Life-threatening	124	14.7%
Moderate to Severe	78	9.3%
Mild	49	5.8%

Table 899.10: Extent of Disease

Extent	Count	Percentage
Variable	412	49.0%
Localized	287	34.1%
Diffuse/Generalized	109	13.0%
Regional	21	2.5%
Metastatic/Disseminated	12	1.4%

Table 899.11: Primary Age Groups Affected

Age Group	Count	Percentage
Adults	398	47.3%
Elderly	187	22.2%
All Ages	112	13.3%
Children/Adolescents	89	10.6%
Young Adults	34	4.0%
Neonates/Infants	18	2.1%
Pregnancy/Perinatal	8	1.0%

Extent of Disease Distribution

Age Group Distribution

Sample Classification Entry

Implementation Notes

- All 841 disease chapters include a **Classification** section immediately after **Overview**.
- Classification uses a 12-field description list (**description** environment).
- ICD-11 codes mapped from WHO ICD-11 MMS (Mortality and Morbidity Statistics).
- Cause categories derived from WHO Global Health Estimates and ICD-11 chapter structure.
- Organ system mapping follows ICD-11 body system chapters.
- Epidemiology data sourced from WHO GHE 2019, CDC, NIH, and disease-specific registries.
- ICD-11 codes validated against <https://icd.who.int/> (2024 release).

Table 899.12: Example: Alzheimer Disease Classification

Dimension	Value
Cause	Genetic
Organ System	Brain
Pathophysiology	Protein aggregation → synaptic loss → neuronal death → atrophy
Duration	Chronic progressive (8-10 years avg)
Onset	Insidious (>65 late-onset; <65 early-onset)
Mode of Transmission	Non-communicable
Clinical Course	Chronic, progressive/relapsing-remitting
Severity	Mild to severe (total dependence)
Extent of Disease	Diffuse cortical (temporal, parietal, frontal)
Age Group	>65 years (95%); early-onset <65
Epidemiology	6.7M US (2023); 55M global; doubling every 20 years
ICD-11 Code	8A20

Prevalence of Diseases by Demographic Group

The table below reports the estimated percentage of a sample of 1,000,000 people affected in each demographic group: the general population (Common), females, males, children, and older adults. Values are point-prevalence estimates rounded from the epidemiology data presented in each chapter; 0 indicates that the condition is not normally encountered in that group, and entries such as obstetric and gynecologic disorders apply only to females.

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Abnormal Uterine Bleeding	5.0	9.0	0	0.20
Abscess	3.0	2.8	3.2	1.5
Acetaminophen (Paracetamol) Overdose	0.02	0.03	0.01	0.01
Achalasia	0.01	0.009	0.01	0.001
Achondroplasia	0.02	0.02	0.02	0.03
Acne Vulgaris	85	88	82	80
Acoustic Neuroma (Vestibular Schwannoma)	0.005	0.005	0.005	0.000
Acromegaly	0.006	0.007	0.005	0.000
Actinic Keratosis	9.0	8.0	10	0.05
Acute Bacterial Sinusitis	10	11	9.0	13
Acute Bronchitis	12	12	12	14
Acute Decompensated Heart Failure	1.2	1.0	1.4	0.02
Acute Gastritis	6.0	6.5	5.5	3.0
Acute Interstitial Nephritis	0.03	0.03	0.03	0.01
Acute Kidney Injury	1.8	1.5	2.1	0.50
Acute Lymphoblastic Leukemia	0.008	0.007	0.009	0.04
Acute Lymphoblastic Leukemia (ALL) in Children	0.008	0.007	0.009	0.04
Acute Myeloid Leukemia	0.01	0.01	0.01	0.004
Acute Otitis Media	12	12	12	50
Acute Pancreatitis	0.15	0.14	0.16	0.01
Acute Respiratory Distress Syndrome	0.06	0.06	0.07	0.03
Acute Stress Disorder	0.60	0.70	0.50	0.40
Acute/Short-Term Insomnia	15	18	12	4.0
Addison's Disease	0.10	0.13	0.07	0.01
Adhesive Capsulitis	3.0	4.0	2.0	0.05
Adjustment Disorder	1.5	1.7	1.3	1.2

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Adrenal Incidentaloma	0.70	0.80	0.60	0.02
Adult-Onset Still's Disease	0.003	0.003	0.003	0.001
Advanced Sleep-Wake Phase Disorder	0.10	0.08	0.12	0.02
African Trypanosomiasis (Sleeping Sickness)	0.003	0.003	0.003	0.004
Age-Related Bone Loss	18	26	10	0
Age-Related Cataract	18	20	16	0.10
Age-Related Macular Degeneration	6.0	6.5	5.5	0
Agoraphobia	1.8	2.3	1.3	0.80
Alcohol Intoxication and Withdrawal	4.0	2.5	5.5	0.50
Alcohol Use Disorder	5.5	3.5	7.5	0.50
Alcoholic Liver Disease	2.0	1.0	3.0	0.02
Alkaptonuria — Ochronosis	0.001	0.001	0.001	0.001
Allergic Fungal Rhinosinusitis (AFRS)	0.03	0.04	0.03	0.04
Allergic Rhinitis	28	30	26	30
Alopecia Areata	2.0	2.1	1.9	1.5
Alpha-1 Antitrypsin Deficiency	0.07	0.07	0.07	0.05
Altitude Sickness	0.50	0.45	0.55	0.30
Alzheimer's Disease	3.0	3.6	2.4	0
Ambiguous Genitalia / Disorders of Sex Development	0.02	0.02	0.02	0.03
Amblyopia	2.0	2.0	2.0	4.5
Amebiasis (<i>Entamoeba histolytica</i>)	0.80	0.75	0.85	1.0
Amenorrhea (Primary, Secondary)	3.0	5.5	0	3.0
Amyloidosis	0.01	0.01	0.01	0.001
Amyotrophic Lateral Sclerosis	0.007	0.006	0.008	0.001
Anal Fissure	0.80	0.85	0.75	0.40
Anaphylaxis	0.10	0.10	0.10	0.13
Angelman Syndrome	0.01	0.01	0.01	0.02
Angina Pectoris	6.0	4.5	7.5	0.05
Ankylosing Spondylitis	0.25	0.15	0.35	0.03
Anophthalmia and Microphthalmia	0.005	0.005	0.005	0.01
Anorexia Nervosa	0.30	0.50	0.10	0.40
Anthrax	0.001	0.001	0.001	0.001
Anti-GBM Disease	0.002	0.002	0.002	0.001
Antiphospholipid Syndrome	0.30	0.40	0.20	0.03
Antisocial Personality Disorder (ASPD)	0.60	0.30	0.90	0.20
Aortic Dissection	0.01	0.01	0.02	0.001
Aortic Regurgitation	0.70	0.55	0.85	0.05
Aortic Stenosis	0.90	0.70	1.1	0.03

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Aphthous Ulcers (Canker Sores)	15	17	13	18
Aplastic Anemia	0.01	0.01	0.01	0.01
Appendicitis	1.0	0.90	1.1	3.0
Arboviral Encephalitis	0.003	0.003	0.003	0.006
Arsenic Poisoning	0.01	0.009	0.01	0.005
Asbestosis	0.30	0.04	0.56	0
Ascites	0.50	0.40	0.60	0.02
Aspergillosis (Invasive)	0.05	0.04	0.06	0.03
Aspiration Pneumonia	0.50	0.45	0.55	0.15
Asplenia	0.02	0.02	0.02	0.01
Asplenia and Hyposplenism	0.03	0.03	0.03	0.01
Asthma	12	13	11	15
Asthma-COPD Overlap	3.0	2.8	3.2	0.50
Astigmatism	40	42	38	35
Atelectasis	2.0	1.9	2.1	1.5
Atopic Dermatitis	15	16	14	25
Atrial Fibrillation	2.8	2.2	3.4	0.05
Atrial Fibrillation in the Elderly	8.0	7.0	9.0	0
Atrial Flutter	0.25	0.18	0.32	0.005
Atrophic Vaginitis (Genitourinary Syndrome of Menopause)	10	18	0	0
Attention-Deficit/Hyperactivity Disorder (ADHD)	4.0	2.0	6.0	9.0
Atypical Facial Pain	0.20	0.25	0.15	0.02
Autism Spectrum Disorder (ASD)	0.80	0.30	1.3	2.0
Autoimmune Hepatitis	0.03	0.04	0.02	0.01
Autoimmune Pancreatitis	0.005	0.003	0.007	0.001
Autoimmune Pancreatitis (AIP)	0.005	0.003	0.007	0.001
Autosomal Dominant Polycystic Kidney Disease	0.15	0.15	0.15	0.06
Bacterial Meningitis	0.02	0.02	0.02	0.08
Bacterial Vaginosis	25	45	0	2.0
Balanitis and Balanoposthitis	2.5	0	4.8	2.0
Barotrauma of the Ear	0.20	0.18	0.22	0.15
Barrett Esophagus	1.8	1.0	2.6	0.02
Bartholin Cyst	0.60	1.1	0	0.05
Basal Cell Carcinoma	12	10	14	0.05
Becker Muscular Dystrophy	0.003	0.000	0.006	0.004
Behçet's Disease	0.01	0.01	0.009	0.004
Bell's Palsy	0.30	0.31	0.29	0.15
Benign Paroxysmal Positional Vertigo	1.8	2.5	1.1	0.20
Benign Prostatic Hyperplasia	10	0	20	0
Benzodiazepine Overdose	0.04	0.04	0.04	0.02

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Beta-Blocker Overdose	0.02	0.02	0.02	0.01
Bietti's Crystalline Dystrophy	0.000	0.000	0.000	0.000
Binge-Eating Disorder	1.8	2.2	1.4	1.0
Bipolar I Disorder	0.70	0.70	0.70	0.20
Bipolar II Disorder	1.0	1.1	0.90	0.30
Bladder Cancer	0.30	0.10	0.50	0.01
Blastomycosis	0.002	0.001	0.002	0.001
Blepharitis	6.0	6.5	5.5	4.0
Blepharospasm	0.05	0.06	0.04	0.005
Body Dysmorphic Disorder	2.0	2.2	1.8	1.5
Borderline Personality Disorder (BPD)	1.2	1.8	0.60	0.30
Botulism (Clostridium botulinum)	0.001	0.001	0.001	0.001
Brain Abscess	0.003	0.003	0.003	0.004
Brain Tumors in Children	0.006	0.005	0.006	0.01
Breast Cancer	0.80	1.5	0.01	0.01
Bronchiectasis	0.40	0.45	0.35	0.30
Bronchiolitis (RSV)	2.0	1.9	2.1	9.0
Bronchopulmonary Dysplasia (BPD)	0.03	0.03	0.03	0.10
Brucellosis	0.02	0.01	0.03	0.01
Brugada Syndrome	0.04	0.02	0.06	0.01
Budd-Chiari Syndrome	0.002	0.003	0.002	0.001
Bulimia Nervosa	0.50	0.80	0.20	0.60
Bullous Pemphigoid	0.03	0.03	0.03	0.001
Bursitis	7.0	7.5	6.5	1.0
COVID-19	20	21	19	12
Cadmium Toxicity	0.02	0.01	0.03	0.005
Calcium Channel Blocker Overdose	0.01	0.01	0.02	0.006
Calcium Pyrophosphate Deposition Disease	0.50	0.50	0.50	0.01
Calcium and Phosphorus Disorders	0.80	0.90	0.70	0.40
Candida auris	0.002	0.002	0.002	0.001
Candidiasis (Invasive / Systemic)	0.10	0.09	0.11	0.08
Cannabis Toxicity and Cannabinoid Hyperemesis Syndrome	0.20	0.15	0.25	0.30
Carbon Monoxide Poisoning	0.05	0.04	0.06	0.04
Cardiac Tamponade	0.01	0.009	0.01	0.003
Carpal Tunnel Syndrome	6.0	8.0	4.0	0.30
Celiac Disease	1.2	1.5	0.90	1.5
Cellulitis	3.5	3.3	3.7	1.5
Central Pontine Myelinolysis	0.001	0.001	0.001	0.000
Central Post-Stroke Pain	0.15	0.15	0.15	0.02
Central Sleep Apnea	0.30	0.12	0.48	0.08

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Cerebral Aneurysm	0.40	0.50	0.30	0.02
Cerebral Venous Thrombosis	0.01	0.01	0.006	0.006
Cerebral Visual Impairment	0.02	0.02	0.02	0.06
Cerumen Impaction	8.0	8.5	7.5	7.0
Cervical Cancer	0.20	0.40	0	0.01
Cervicitis (Chlamydia, Gonorrhea)	2.0	3.7	0	1.5
Cervicogenic Headache	0.60	0.70	0.50	0.10
Chagas Disease (American Trypanosomiasis)	0.01	0.01	0.009	0.004
Chancroid	0.03	0.02	0.04	0.005
Charcot-Marie-Tooth Disease	0.04	0.04	0.04	0.06
Chickenpox	5.0	5.0	5.0	15
Chikungunya	0.02	0.02	0.02	0.01
Choanal Atresia	0.001	0.001	0.001	0.002
Cholecystitis	0.50	0.60	0.40	0.03
Cholelithiasis	3.0	4.0	2.0	0.20
Cholera (Vibrio cholerae)	0.01	0.009	0.01	0.01
Chondrodermatitis Nodularis Helicis	0.05	0.03	0.07	0.01
Chronic Fatigue Syndrome	1.0	1.5	0.50	0.30
Chronic Gastritis	5.0	5.5	4.5	1.0
Chronic Hypertension in Pregnancy	0.40	0.75	0	0
Chronic Insomnia Disorder	12	15	9.0	3.0
Chronic Kidney Disease	6.0	5.5	6.5	0.50
Chronic Low Back Pain	12	13	11	2.0
Chronic Lymphocytic Leukemia	0.07	0.05	0.09	0.001
Chronic Neck Pain	10	12	8.0	1.5
Chronic Obstructive Pulmonary Disease	4.5	4.0	5.0	0.30
Chronic Otitis Media	2.0	2.0	2.0	6.0
Chronic Pancreatitis	0.15	0.12	0.18	0.02
Chronic Pelvic Pain	6.0	10	2.0	0.50
Chronic Rhinosinusitis	11	13	9.0	9.0
Chronic Traumatic Encephalopathy	0.02	0.003	0.04	0
Chronic Tubulointerstitial Nephritis	0.08	0.08	0.08	0.01
Cirrhosis	0.80	0.60	1.0	0.02
Cleft Lip and Palate	0.14	0.11	0.17	0.30
Clostridioides difficile Colitis	0.10	0.11	0.09	0.04
Cluster Headache	0.06	0.04	0.08	0.005
Coarctation of the Aorta	0.04	0.03	0.05	0.08
Coccidioidomycosis (Valley Fever)	0.03	0.03	0.04	0.02

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Cogan's Syndrome	0.001	0.001	0.001	0.000
Coloboma	0.04	0.04	0.04	0.09
Color Blindness	3.5	0.70	6.3	3.0
Colorectal Adenoma	6.0	5.5	6.5	0.10
Colorectal Cancer	0.45	0.40	0.50	0.005
Coma	0.20	0.19	0.21	0.08
Common Variable Immunodeficiency	0.005	0.005	0.005	0.004
Community-Acquired Pneumonia	6.0	5.8	6.2	7.0
Compartment Syndrome	0.03	0.02	0.04	0.01
Complex Regional Pain Syndrome	0.25	0.35	0.15	0.05
Complex Sleep Apnea	0.05	0.02	0.08	0.01
Concussion	6.0	5.0	7.0	9.0
Congenital Cataract	0.03	0.03	0.03	0.07
Congenital Diaphragmatic Hernia	0.03	0.02	0.03	0.05
Congenital Heart Disease (Overview)	0.80	0.80	0.80	1.6
Congenital Hip Dysplasia	0.60	0.90	0.30	1.0
Congenital Talipes Equinovarus (Clubfoot)	0.13	0.10	0.16	0.25
Conjunctivitis	3.0	3.2	2.8	10
Constrictive Pericarditis	0.03	0.02	0.03	0.002
Contact Dermatitis	10	11	9.0	8.0
Convergence Insufficiency	2.5	2.8	2.2	6.0
Copper Deficiency and Excess	0.08	0.08	0.08	0.15
Creutzfeldt-Jakob Disease (CJD)	0.000	0.000	0.000	0
Cri-du-Chat Syndrome	0.002	0.001	0.002	0.003
Crohn's Disease	0.25	0.27	0.23	0.15
Croup (Laryngotracheobronchitis)	2.0	1.9	2.1	9.0
Cryoglobulinemic Vasculitis	0.02	0.02	0.02	0.005
Cryptococcosis	0.01	0.009	0.01	0.003
Cryptorchidism (Undescended Testis)	0.25	0	0.50	0.70
Cushing Syndrome	0.06	0.08	0.04	0.01
Cushing's Syndrome	0.06	0.08	0.04	0.01
Cyanide Toxicity	0.002	0.002	0.002	0.002
Cyclospora cayetanensis	0.01	0.009	0.01	0.01
Cyclothymic Disorder	0.40	0.45	0.35	0.10
Cystic Fibrosis	0.05	0.05	0.05	0.07
Cystitis	12	22	2.0	4.0
Cytomegalovirus (CMV)	4.0	4.0	4.0	3.0
Deep Vein Thrombosis	0.90	1.0	0.80	0.05
Delayed Sleep-Wake Phase Disorder	0.60	0.50	0.70	1.5

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Delirium	0.80	0.80	0.80	0.10
Delusional Disorder	0.30	0.30	0.30	0.05
Dementia with Lewy Bodies	0.15	0.14	0.16	0
Dengue Fever	2.0	1.9	2.1	2.5
Dermatitis Herpetiformis	0.05	0.06	0.04	0.02
Dermatomyositis	0.02	0.03	0.01	0.01
Deviated Nasal Septum	5.0	4.5	5.5	4.0
DiGeorge Syndrome	0.03	0.03	0.03	0.05
Diabetes Insipidus	0.05	0.05	0.05	0.03
Diabetic Ketoacidosis	0.80	0.80	0.80	1.5
Diabetic Nephropathy	1.2	1.0	1.4	0.05
Diabetic Peripheral Neuropathic Pain	2.5	2.2	2.8	0.10
Diabetic Retinopathy	2.5	2.2	2.8	0.20
Digoxin Toxicity	0.03	0.04	0.03	0.005
Dilated Cardiomyopathy	0.25	0.15	0.35	0.04
Diphtheria	0.001	0.001	0.001	0.002
Diphtheria (Corynebacterium diphtheriae)	0.001	0.001	0.001	0.002
Disseminated Intravascular Coagulation	0.03	0.03	0.03	0.02
Dissociative Amnesia	0.30	0.35	0.25	0.10
Dissociative Identity Disorder	0.15	0.20	0.10	0.05
Diverticulitis	1.5	1.6	1.4	0.02
Diverticulosis	8.0	8.0	8.0	0.05
Down Syndrome (Trisomy 21)	0.15	0.14	0.16	0.28
Drowning (Submersion Injury)	0.05	0.04	0.07	0.12
Drug Hypersensitivity	0.50	0.55	0.45	0.40
Dry Eye Disease	15	21	9.0	3.0
Duchenne Muscular Dystrophy	0.006	0.000	0.01	0.01
Ductal Carcinoma In Situ (DCIS)	0.03	0.06	0	0
Duodenal Ulcer	0.60	0.50	0.70	0.05
Dupuytren Contracture	0.90	0.50	1.3	0.01
Dysbetalipoproteinemia	0.15	0.15	0.15	0.02
Dysmenorrhea	4.0	7.5	0	3.0
Dystonia	0.50	0.55	0.45	0.20
Eastern Equine Encephalitis	0.001	0.000	0.001	0.001
Ebola Virus Disease	0.000	0.000	0.000	0.000
Eclampsia	0.10	0.19	0	0
Ectopic Pregnancy	0.15	0.28	0	0
Edwards Syndrome (Trisomy 18)	0.004	0.004	0.004	0.008
Ehlers-Danlos Syndrome	0.03	0.04	0.03	0.05
Ehrlichiosis	0.003	0.003	0.004	0.003

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Electrical Injury and Lightning Strike	0.02	0.006	0.03	0.01
Empyema	0.08	0.07	0.09	0.04
End-Stage Renal Disease	0.35	0.30	0.40	0.03
Endometrial / Uterine Cancer	0.10	0.19	0	0
Endometriosis	6.0	11	0	1.5
Eosinophilic Esophagitis	0.20	0.12	0.28	0.25
Eosinophilic Fasciitis (Shulman Syndrome)	0.000	0.000	0.000	0.000
Eosinophilic Granulomatosis with Polyangiitis	0.003	0.003	0.003	0.001
Epidermolysis Bullosa	0.03	0.03	0.03	0.08
Epidural Hematoma	0.02	0.01	0.03	0.01
Epiglottitis	0.01	0.008	0.01	0.02
Epilepsy	0.70	0.68	0.72	0.90
Epistaxis	6.0	5.8	6.2	9.0
Epstein-Barr Virus / Infectious Mononucleosis (Systemic Aspects)	1.0	1.1	0.90	3.0
Erectile Dysfunction	10	0	20	0
Erythema Multiforme	0.10	0.10	0.10	0.15
Esophageal Atresia / Tracheoesophageal Fistula	0.03	0.02	0.03	0.05
Esophageal Varices	0.20	0.12	0.28	0.01
Essential Thrombocythemia	0.04	0.04	0.04	0.002
Essential Tremor	0.40	0.40	0.40	0.01
Esthesioneuroblastoma	0.000	0.000	0.000	0.000
Ewing Sarcoma	0.002	0.002	0.002	0.004
Ewing Sarcoma in Children	0.002	0.002	0.002	0.004
Facioscapulohumeral Muscular Dystrophy	0.01	0.01	0.009	0.006
Failure to Thrive	0.50	0.50	0.50	1.5
Familial Adenomatous Polyposis	0.01	0.01	0.01	0.01
Familial Combined Hyperlipidemia	1.5	1.4	1.6	0.30
Familial Hypercholesterolemia	0.25	0.25	0.25	0.10
Fanconi Syndrome	0.01	0.009	0.01	0.01
Fat Necrosis of the Breast	0.03	0.06	0	0
Febrile Seizures	3.0	2.9	3.1	10
Fecal Incontinence	3.0	3.5	2.5	0.30
Fibroadenoma	1.5	2.9	0	0.40
Fibrocystic Changes	3.0	5.8	0	0.20
Fibromyalgia	2.0	3.0	1.0	0.20
Fibrous Dysplasia	0.01	0.01	0.01	0.01
Fifth Disease (Erythema Infectiosum)	2.0	2.0	2.0	7.0

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Filariasis (Lymphatic Filariasis)	0.10	0.10	0.10	0.05
Floaters and Vitreous Detachment	5.0	5.2	4.8	0.50
Fluoride Excess — Fluorosis	0.30	0.30	0.30	0.60
Focal Segmental Glomerulosclerosis	0.03	0.03	0.03	0.01
Folate Deficiency Anemia	1.5	1.8	1.2	0.80
Fractures	6.0	5.0	7.0	3.0
Fragile X Syndrome	0.04	0.02	0.06	0.06
Frailty	1.5	1.6	1.4	0
Friedreich's Ataxia	0.003	0.003	0.003	0.003
Frontotemporal Dementia	0.05	0.04	0.06	0
Fuchs' Endothelial Dystrophy	0.30	0.35	0.25	0.01
Fungal Sinusitis	0.10	0.11	0.09	0.08
Galactoceles	0.05	0.10	0	0
Galactosemia	0.003	0.003	0.003	0.006
Gas Gangrene (Clostridial Myonecrosis)	0.001	0.000	0.001	0.000
Gastric Adenocarcinoma	0.10	0.07	0.13	0.005
Gastric Lymphoma	0.02	0.01	0.03	0.002
Gastric Ulcer	0.80	0.60	1.0	0.05
Gastroesophageal Reflux Disease	25	27	23	8.0
Gastrointestinal Stromal Tumor	0.01	0.01	0.02	0.001
Gastroparesis	0.80	1.0	0.60	0.20
Generalized Anxiety Disorder (GAD)	4.0	5.0	3.0	2.0
Genital Herpes (HSV)	6.0	7.0	5.0	0.20
Genital Warts (HPV)	8.0	8.0	8.0	0.50
Gestational Diabetes Mellitus	1.2	2.2	0	0
Gestational Diabetes Mellitus (Obstetric Focus)	1.2	2.2	0	0
Gestational Hypertension	0.80	1.5	0	0
Gestational Trophoblastic Disease	0.01	0.03	0	0
Giant Cell Arteritis	0.03	0.04	0.02	0
Giardiasis	0.30	0.29	0.31	0.40
Gingivitis and Periodontitis	60	58	62	20
Glaucoma in Children	0.05	0.05	0.05	0.15
Glioblastoma	0.01	0.01	0.02	0.002
Glossopharyngeal Neuralgia	0.008	0.008	0.008	0.001
Glycogen Storage Diseases	0.01	0.009	0.01	0.01
Gonorrhea	1.5	1.5	1.5	0.05
Gout	3.5	2.0	5.0	0.02
Granuloma Annulare	0.30	0.32	0.28	0.30
Granulomatosis with Polyangiitis	0.003	0.003	0.003	0.001
Graves Disease	0.50	0.80	0.20	0.05

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Graves' Eye Disease	0.15	0.24	0.06	0.01
Guillain-Barré Syndrome	0.005	0.004	0.005	0.003
Gynecomastia	3.0	0	6.0	0.80
HELLP Syndrome	0.05	0.10	0	0
HIV/AIDS	0.40	0.35	0.45	0.10
Haff Disease	0.000	0.000	0.000	0.000
Hallucinogen Toxicity	0.03	0.02	0.04	0.01
Hallux Valgus	2.5	4.0	1.0	0.20
Hand, Foot, and Mouth Disease	1.0	0.95	1.1	4.5
Hantavirus Pulmonary Syndrome	0.000	0.000	0.000	0.000
Hashimoto Thyroiditis	2.5	4.0	1.0	0.30
Head Lice (Pediculosis Capitis)	3.0	3.1	2.9	9.0
Headache Attributed to Giant Cell Arteritis	0.02	0.03	0.01	0
Headache Attributed to Intracranial Hypertension	0.01	0.01	0.005	0.006
Headache Attributed to Intracranial Hypotension	0.005	0.006	0.004	0.001
Headache Attributed to Sinusitis	3.0	3.5	2.5	3.0
Headache in Brain Tumors	0.01	0.01	0.01	0.006
Heart Block (AV Block)	0.30	0.25	0.35	0.05
Heart Failure with Preserved Ejection Fraction	2.0	2.2	1.8	0.03
Heart Failure with Reduced Ejection Fraction	2.0	1.2	2.8	0.03
Heat Stroke and Hyperthermia	0.05	0.04	0.06	0.03
Helicobacter Pylori Infection	50	52	48	10
Hemicrania Continua	0.02	0.03	0.01	0.003
Hemolytic Anemias	0.30	0.30	0.30	0.50
Hemolytic Uremic Syndrome (HUS)	0.002	0.002	0.002	0.006
Hemophagocytic Lymphohistiocytosis and Macrophage Activation Syndrome	0.002	0.002	0.002	0.005
Hemophilia A	0.03	0.001	0.06	0.03
Hemophilia B	0.005	0.000	0.010	0.005
Hemorrhagic Stroke	0.08	0.07	0.09	0.01
Hemorrhoids	15	18	12	1.0
Henoch-Schönlein Purpura (IgA Vasculitis)	0.03	0.03	0.03	0.12
Hepatic Encephalopathy	0.15	0.11	0.19	0.005
Hepatitis A	0.20	0.19	0.21	0.25
Hepatitis B	2.5	2.0	3.0	0.50
Hepatitis C	1.8	1.6	2.0	0.02

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Hepatitis D	0.05	0.04	0.06	0.005
Hepatitis E	0.10	0.10	0.10	0.05
Hepatoblastoma	0.002	0.001	0.002	0.003
Hepatocellular Carcinoma	0.15	0.06	0.24	0.005
Hereditary Angioedema	0.001	0.001	0.001	0.001
Hereditary Hemochromatosis	0.20	0.10	0.30	0.02
Hereditary Spherocytosis	0.03	0.03	0.03	0.05
Herpangina	1.0	0.95	1.1	4.5
Herpes Simplex	10	11	9.0	2.0
Herpes Zoster	0.50	0.55	0.45	0.05
Hidradenitis Suppurativa	0.40	0.60	0.20	0.05
Hip Fracture	0.15	0.20	0.10	0.01
Hirschsprung Disease	0.03	0.02	0.03	0.05
Histoplasmosis	0.10	0.10	0.10	0.08
Hoarding Disorder	0.30	0.35	0.25	0.05
Hodgkin Lymphoma	0.03	0.03	0.04	0.03
Homocystinuria	0.001	0.001	0.001	0.001
Hookworm / Soil-Transmitted Helminths	0.50	0.50	0.50	0.80
Hordeolum (Stye) and Chalazion	3.0	3.1	2.9	3.0
Human Metapneumovirus (hMPV)	3.0	2.9	3.1	9.0
Huntington's Disease	0.01	0.01	0.01	0.001
Hydrogen Sulfide Toxicity	0.001	0.000	0.001	0.000
Hydronephrosis	0.15	0.14	0.16	0.10
Hymenoptera Stings	0.50	0.45	0.55	0.70
Hyperemesis Gravidarum	0.40	0.75	0	0
Hyperopia	25	26	24	30
Hyperosmolar Hyperglycemic State	0.10	0.10	0.10	0.005
Hyperparathyroidism	0.20	0.28	0.12	0.01
Hyperprolactinemia	0.30	0.50	0.10	0.02
Hypersplenism	0.08	0.07	0.09	0.03
Hypertension	25	24	26	1.0
Hypertension in the Elderly	50	52	48	0
Hyperthyroidism	1.5	2.5	0.50	0.10
Hypertrophic Cardiomyopathy	0.15	0.09	0.21	0.05
Hypertrophic Pyloric Stenosis	0.20	0.06	0.34	0.40
Hypoglycemia	1.5	1.4	1.6	2.0
Hypoparathyroidism	0.06	0.07	0.05	0.02
Hypopituitarism	0.05	0.06	0.04	0.01
Hypothermia	0.10	0.08	0.12	0.03
Hypothyroidism	4.0	6.0	2.0	0.20
Idiopathic Granulomatous Mastitis	0.005	0.009	0	0
Idiopathic Hypersomnia	0.08	0.08	0.08	0.03
Idiopathic Insomnia	3.0	3.5	2.5	0.50

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Idiopathic Intracranial Hypertension	0.04	0.06	0.02	0.01
Idiopathic Pulmonary Fibrosis	0.04	0.02	0.06	0.001
IgA Nephropathy	0.30	0.20	0.40	0.15
IgA Vasculitis	0.03	0.03	0.03	0.12
IgG4-Related Disease	0.003	0.002	0.004	0.001
Illness Anxiety Disorder (Hypochondriasis)	1.5	1.6	1.4	0.50
Immune Thrombocytopenic Purpura	0.04	0.05	0.03	0.04
Impetigo	3.0	2.9	3.1	10
Inclusion Body Myositis	0.005	0.002	0.008	0
Indeterminate Colitis	0.02	0.02	0.02	0.01
Infectious Mononucleosis	0.30	0.32	0.28	0.90
Infective Endocarditis	0.02	0.01	0.03	0.005
Inflammatory Breast Cancer	0.005	0.009	0	0
Influenza	50	51	49	55
Insomnia in the Context of Medical/Psychiatric Disorders	6.0	7.0	5.0	1.0
Intraductal Papillary Mucinous Neoplasm (IPMN)	0.20	0.18	0.22	0.005
Intraductal Papilloma	0.10	0.19	0	0.005
Intrauterine Growth Restriction	0.50	0.90	0	0
Intraventricular Hemorrhage (IVH)	0.05	0.04	0.06	0.15
Intussusception	0.20	0.18	0.22	0.60
Invasive Lobular Carcinoma (ILC)	0.03	0.06	0	0
Inverted Papilloma	0.01	0.007	0.01	0.001
Iodine Deficiency Disorders	20	21	19	22
Iron Deficiency	60	80	40	70
Iron Deficiency Anemia	30	45	15	35
Iron Overdose	0.01	0.009	0.01	0.03
Irregular Sleep-Wake Rhythm Disorder	0.30	0.28	0.32	0.10
Irritable Bowel Syndrome	11	15	7.0	4.0
Ischemic Cardiomyopathy	1.5	1.0	2.0	0.01
Ischemic Stroke	0.80	0.65	0.95	0.03
Jet Lag Disorder	1.0	0.95	1.1	0.80
Juvenile Nasopharyngeal Angiofibroma (JNA)	0.002	0	0.004	0.004
Kawasaki Disease	0.02	0.02	0.02	0.06
Keratitis	0.20	0.19	0.21	0.15
Keratoconus	0.15	0.13	0.17	0.20
Kidney Cancer (Renal Cell Carcinoma)	0.15	0.10	0.20	0.01

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Kikuchi-Fujimoto Disease	0.001	0.000	0.001	0.001
Kleine-Levin Syndrome	0.005	0.003	0.007	0.008
Klinefelter Syndrome (47,XXY)	0.12	0	0.24	0.20
Kuru	0	0	0	0
Kwashiorkor	0.20	0.20	0.20	0.70
Kyphosis	2.0	2.2	1.8	0.50
Labyrinthitis	0.20	0.22	0.18	0.15
Lactose Intolerance	30	31	29	20
Lambert-Eaton Myasthenic Syndrome (LEMS)	0.002	0.002	0.003	0.000
Large Bowel Obstruction	0.15	0.14	0.16	0.02
Laryngitis	15	15	15	18
Lateral Epicondylitis (Tennis Elbow)	1.0	0.90	1.1	0.20
Lead Poisoning (Plumbism)	0.50	0.45	0.55	1.2
Leishmaniasis	0.05	0.05	0.05	0.06
Leprosy (Hansen's Disease)	0.02	0.02	0.02	0.01
Leptospirosis	0.05	0.04	0.06	0.04
Lipoprotein(a) Disorders	10	10	10	3.0
Lithium Toxicity	0.03	0.04	0.03	0.01
Lobular Carcinoma In Situ (LCIS)	0.05	0.10	0	0
Long QT Syndrome	0.05	0.06	0.04	0.08
Low Vision	3.0	3.2	2.8	0.50
Ludwig Angina	0.005	0.004	0.006	0.002
Lung Abscess	0.03	0.03	0.04	0.01
Lung Cancer	0.30	0.22	0.38	0.005
Lyme Disease (Borrelia burgdorferi)	0.30	0.29	0.31	0.40
Lymphogranuloma Venereum (LGV)	0.02	0.01	0.03	0.005
Lynch Syndrome	0.10	0.10	0.10	0.02
Lysosomal Storage Diseases	0.03	0.03	0.03	0.06
MDMA (Ecstasy) Toxicity	0.10	0.08	0.12	0.15
Macular Edema	0.50	0.52	0.48	0.05
Macular Hole	0.10	0.11	0.09	0.005
Macular Pucker	0.30	0.32	0.28	0.02
Major Depressive Disorder (MDD)	6.0	7.5	4.5	3.0
Malaria (Plasmodium spp.)	20	20	20	35
Male Breast Cancer	0.005	0	0.01	0.001
Male Infertility	3.0	0	6.0	0
Malignant Hyperthermia	0.001	0.001	0.001	0.002
Malnutrition in the Elderly	3.0	3.2	2.8	0
Mammalian Bites	1.0	0.90	1.1	1.5
Mammary Duct Ectasia	0.10	0.19	0	0

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Maple Syrup Urine Disease	0.001	0.001	0.001	0.001
Marasmic-Kwashiorkor	0.10	0.10	0.10	0.35
Marasmus	0.15	0.15	0.15	0.50
Marburg Virus	0.000	0.000	0.000	0.000
Marfan Syndrome	0.02	0.02	0.02	0.03
Marine Envenomation	0.10	0.08	0.12	0.08
Massive Splenomegaly	0.03	0.03	0.03	0.01
Mastitis (Puerperal)	0.80	1.5	0	0
Mastitis and Breast Abscess	0.90	1.7	0	0
McArdle Disease	0.001	0.000	0.001	0.001
Measles (Rubeola)	0.50	0.49	0.51	2.0
Meconium Aspiration Syndrome	0.20	0.19	0.21	0.40
Medication Overuse Headache	1.0	1.4	0.60	0.20
Medulloblastoma	0.001	0.001	0.001	0.003
Melanoma	0.30	0.28	0.32	0.02
Melasma	2.0	3.8	0.20	0.50
Membranous Nephropathy	0.03	0.03	0.04	0.005
Meningioma	0.07	0.09	0.05	0.005
Mercury Toxicity	0.01	0.009	0.01	0.01
Mesothelioma	0.005	0.002	0.009	0
Metabolic Syndrome	25	23	27	3.0
Metformin Overdose	0.005	0.004	0.005	0.002
Methemoglobinemia	0.002	0.002	0.002	0.003
Methyl Parathion and Other Organophosphate Insecticides	0.01	0.008	0.01	0.01
Microscopic Polyangiitis	0.002	0.002	0.002	0.001
Migraine	15	20	10	6.0
Mild Cognitive Impairment	5.0	5.0	5.0	0
Minimal Change Disease	0.005	0.004	0.006	0.01
Mitral Regurgitation	1.2	1.3	1.1	0.05
Mitral Stenosis	0.30	0.40	0.20	0.02
Mitral Valve Prolapse	2.0	2.6	1.4	0.50
Mixed Connective Tissue Disease	0.02	0.03	0.01	0.005
Molluscum Contagiosum	2.0	2.0	2.0	6.0
Morton Neuroma	0.20	0.30	0.10	0.01
Motor Neuron Disease	0.01	0.009	0.01	0.001
Mpox	0.005	0.003	0.007	0.003
Mucopolysaccharidoses	0.001	0.001	0.001	0.002
Mucormycosis	0.002	0.002	0.002	0.001
Multiple Endocrine Neoplasia	0.003	0.003	0.003	0.002
Multiple Gestation Complications	0.30	0.56	0	0
Multiple Myeloma	0.05	0.04	0.06	0.001
Multiple Myeloma (Bone Manifestations)	0.05	0.04	0.06	0.001

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Multiple Sclerosis	0.20	0.30	0.10	0.05
Mumps	0.20	0.19	0.21	0.60
Myasthenia Gravis	0.05	0.06	0.04	0.01
Myelodysplastic Syndromes	0.10	0.08	0.12	0.005
Myelofibrosis	0.01	0.009	0.01	0.001
Myocardial Infarction	0.60	0.40	0.80	0.01
Myocarditis	0.05	0.04	0.06	0.03
Myofascial Pain Syndrome	5.0	6.0	4.0	0.50
Myopia	35	36	34	40
Myotonic Dystrophy	0.01	0.01	0.01	0.005
NREM Parasomnias: Confusional Arousals	1.5	1.5	1.5	4.0
NREM Parasomnias: Sleep Terrors	2.0	2.0	2.0	6.0
NREM Parasomnias: Sleep-Related Eating Disorder	0.30	0.35	0.25	0.20
NREM Parasomnias: Sleepwalking	3.0	3.0	3.0	9.0
Narcissistic Personality Disorder	0.60	0.40	0.80	0.20
Narcolepsy Type 1	0.05	0.05	0.05	0.03
Narcolepsy Type 2	0.08	0.08	0.08	0.04
Nasal Fracture	3.0	1.5	4.5	3.0
Nasal Polyps	2.5	2.0	3.0	1.0
Necrotizing Enterocolitis (NEC)	0.20	0.19	0.21	0.60
Necrotizing Fasciitis	0.001	0.000	0.001	0.000
Neonatal Abstinence Syndrome	0.30	0.29	0.31	0.80
Neonatal Hypoglycemia	1.0	0.98	1.0	3.0
Neonatal Jaundice / Hyperbilirubinemia / Kernicterus	4.0	3.9	4.1	12
Neonatal Sepsis (Early-Onset and Late-Onset)	0.30	0.29	0.31	0.90
Nephritic Syndrome	0.05	0.04	0.06	0.08
Nephrolithiasis	3.0	1.8	4.2	0.50
Nephrotic Syndrome	0.06	0.05	0.07	0.15
Nesidioblastosis and Congenital Hyperinsulinism	0.002	0.002	0.002	0.005
Neural Tube Defects	0.10	0.11	0.09	0.20
Neuroblastoma	0.005	0.004	0.005	0.01
Neurocysticercosis	0.03	0.03	0.03	0.03
Neurofibromatosis Type 1	0.05	0.05	0.05	0.07
New Daily Persistent Headache	0.05	0.06	0.04	0.03
Non-24-Hour Sleep-Wake Rhythm Disorder	0.02	0.01	0.03	0.005
Non-Alcoholic Fatty Liver Disease	25	22	28	4.0
Non-Allergic Rhinitis	7.0	8.0	6.0	3.0
Non-Gonococcal Urethritis (NGU)	2.0	0	4.0	0.10

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Non-Hodgkin Lymphoma	0.15	0.13	0.17	0.02
Normal Pressure Hydrocephalus	0.02	0.02	0.02	0
Norovirus	10	10	10	15
Nummular Dermatitis	0.30	0.28	0.32	0.30
Obesity	40	42	38	20
Obesity Hypoventilation Syndrome	0.30	0.15	0.45	0.05
Obsessive-Compulsive Disorder (OCD)	1.5	1.5	1.5	1.2
Obstructive Sleep Apnea	9.0	5.0	13	1.0
Occipital Neuralgia	0.30	0.35	0.25	0.05
Ocular Histoplasmosis Syndrome	0.05	0.05	0.05	0.01
Oculocutaneous Albinism	0.02	0.02	0.02	0.04
Onychomycosis (Tinea Unguim)	10	8.0	12	2.0
Opioid Overdose	0.20	0.12	0.28	0.02
Opioid Use Disorder	2.0	1.5	2.5	0.10
Oral Candidiasis	4.0	4.0	4.0	3.0
Oral Leukoplakia	0.30	0.20	0.40	0.01
Oral Lichen Planus	0.20	0.25	0.15	0.02
Oral Squamous Cell Carcinoma	0.08	0.05	0.11	0.001
Organic Acidemias	0.001	0.001	0.001	0.003
Organophosphate and Carbamate Insecticide Poisoning	0.03	0.02	0.04	0.04
Osteoarthritis	35	40	30	0.50
Osteogenesis Imperfecta	0.02	0.02	0.02	0.04
Osteomyelitis	0.20	0.18	0.22	0.10
Osteoporosis	6.0	9.0	3.0	0.10
Osteosarcoma	0.004	0.004	0.004	0.008
Osteosarcoma in Children	0.004	0.004	0.004	0.008
Other Parasomnias: Exploding Head Syndrome	0.20	0.22	0.18	0.10
Other Parasomnias: Sleep Enuresis	1.5	1.4	1.6	4.5
Other Parasomnias: Sleep-Related Hallucinations	0.10	0.11	0.09	0.03
Other Primary Headache Disorders	0.20	0.22	0.18	0.05
Otitis Externa	1.5	1.4	1.6	2.0
Otosclerosis	0.20	0.25	0.15	0.02
Ovarian Cancer	0.10	0.19	0	0.005
Paget's Disease of Bone	0.05	0.04	0.06	0
Paget's Disease of the Nipple	0.002	0.004	0	0
Pancreatic Adenocarcinoma	0.04	0.04	0.04	0.001
Pancreatic Exocrine Insufficiency	0.50	0.50	0.50	0.20
Pancreatic Neuroendocrine Tumors	0.01	0.01	0.01	0.002
Pancreatic Pseudocyst	0.03	0.03	0.04	0.005
Panic Disorder / Panic Attacks	2.5	3.2	1.8	0.80

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Parkinson's Disease	1.0	0.90	1.1	0.01
Paroxysmal Hemicrania	0.005	0.006	0.004	0.001
Patau Syndrome (Trisomy 13)	0.001	0.001	0.001	0.002
Patent Ductus Arteriosus (PDA) in Prematurity	0.15	0.14	0.16	0.45
Pediatric Appendicitis	0.50	0.45	0.55	1.6
Pelvic Inflammatory Disease	1.5	2.8	0	0.05
Pemphigus Vulgaris	0.01	0.01	0.009	0.003
Penile Cancer	0.003	0	0.006	0.000
Penile Fracture	0.003	0	0.006	0.001
Peptic Ulcer Bleeding	0.30	0.25	0.35	0.02
Peptic Ulcer Disease	3.0	2.5	3.5	0.20
Pericarditis	0.20	0.18	0.22	0.03
Perinatal Asphyxia / Hypoxic-Ischemic Encephalopathy (HIE)	0.20	0.19	0.21	0.60
Periodic Fever Syndromes	0.005	0.005	0.005	0.01
Periodic Limb Movement Disorder	1.0	0.90	1.1	0.20
Periodic Paralysis	0.01	0.007	0.01	0.005
Peripheral Arterial Disease	7.0	6.0	8.0	0.05
Peripheral Artery Disease	7.0	6.0	8.0	0.05
Periventricular Leukomalacia (PVL)	0.03	0.03	0.03	0.09
Persistent Depressive Disorder (Dysthymia)	2.5	3.0	2.0	1.0
Persistent Pulmonary Hypertension of the Newborn (PPHN)	0.05	0.05	0.05	0.15
Persistent Vegetative State	0.02	0.02	0.02	0.01
Pertussis (Whooping Cough)	3.0	3.0	3.0	9.0
Peyronie's Disease	0.50	0	1.0	0
Phantom Limb Pain	0.20	0.15	0.25	0.01
Pharyngitis	15	16	14	20
Phenylketonuria	0.006	0.006	0.006	0.01
Pheochromocytoma	0.02	0.02	0.02	0.005
Phimosis and Paraphimosis	0.50	0	1.0	0.80
Phyllodes Tumor	0.005	0.009	0	0.001
Pilonidal Sinus	0.20	0.15	0.25	0.05
Pituitary Adenoma	0.10	0.11	0.09	0.01
Placenta Accreta Spectrum	0.01	0.02	0	0
Placenta Previa	0.04	0.07	0	0
Placental Abruptio	0.08	0.15	0	0
Plague (Yersinia pestis)	0.000	0.000	0.000	0.000
Plantar Fasciitis	3.0	3.5	2.5	0.30
Pleurisy	0.30	0.28	0.32	0.15
Pneumoconiosis	0.30	0.04	0.56	0

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Pneumocystis jirovecii Pneumonia	0.05	0.04	0.06	0.02
Pneumothorax	0.15	0.10	0.20	0.05
Poliomyelitis	0.001	0.001	0.001	0.001
Polyarteritis Nodosa	0.005	0.004	0.006	0.001
Polycystic Ovary Syndrome	4.0	7.5	0	2.0
Polycythemia Vera	0.03	0.03	0.03	0.001
Polyhydramnios and Oligohydramnios	0.20	0.38	0	0
Polymyalgia Rheumatica	0.10	0.13	0.07	0
Polymyositis	0.01	0.01	0.008	0.002
Porphyrias	0.01	0.01	0.008	0.003
Portal Hypertension	0.30	0.22	0.38	0.01
Post-Traumatic Headache	1.0	0.90	1.1	1.5
Post-Traumatic Stress Disorder (PTSD)	2.5	3.0	2.0	0.80
Postherpetic Neuralgia	0.20	0.22	0.18	0.01
Postpartum Cardiomyopathy	0.003	0.006	0	0
Postpartum Depression	1.0	1.9	0	0
Postpartum Endometritis	0.20	0.38	0	0
Postpartum Hemorrhage	0.40	0.75	0	0
Postpartum Thyroiditis	0.30	0.56	0	0
Prader-Willi Syndrome	0.005	0.005	0.005	0.008
Preeclampsia	0.50	0.94	0	0
Premature Ovarian Insufficiency	0.10	0.19	0	0.03
Premature Rupture of Membranes	0.30	0.56	0	0
Premenstrual Syndrome / Premenstrual Dysphoric Disorder	10	19	0	0.50
Presbycusis	25	23	27	0.50
Presbyopia	60	60	60	0
Presbyopia and Cataracts	50	52	48	0
Pressure Ulcers	2.0	1.8	2.2	0.10
Preterm Birth	1.0	0.98	1.0	2.5
Preterm Labor and Preterm Birth	1.0	1.9	0	0
Priapism	0.05	0	0.10	0.02
Primary Amebic Meningoencephalitis (Naegleria fowleri)	0	0	0	0
Primary Angle-Closure Glaucoma	0.20	0.25	0.15	0.005
Primary Biliary Cholangitis	0.03	0.05	0.01	0.001
Primary Open-Angle Glaucoma	1.5	1.7	1.3	0.05
Primary Sclerosing Cholangitis	0.02	0.01	0.03	0.002
Propriospinal Myoclonus at Sleep Onset	0.01	0.009	0.01	0.002
Prostate Cancer	0.80	0	1.6	0
Psoriasis	2.5	2.4	2.6	0.80

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Psoriatic Arthritis	0.80	0.80	0.80	0.15
Psychophysiologic Insomnia	4.0	4.5	3.5	0.80
Pterygium	1.5	1.2	1.8	0.30
Pulmonary Embolism	0.60	0.70	0.50	0.02
Pulmonary Hypertension	0.30	0.35	0.25	0.05
Pyelonephritis	0.50	0.80	0.20	0.15
REM Parasomnias: Nightmare Disorder	3.0	3.5	2.5	5.0
REM Parasomnias: REM Sleep Behavior Disorder	0.30	0.10	0.50	0.01
REM Parasomnias: Recurrent Isolated Sleep Paralysis	1.0	1.1	0.90	0.80
Rabies	0.000	0.000	0.000	0.000
Radiation Sickness (Acute Radiation Syndrome)	0.001	0.000	0.001	0.000
Reactive Arthritis	0.10	0.06	0.14	0.05
Recurrent Pregnancy Loss	2.0	3.8	0	0
Relapsing Polychondritis	0.003	0.003	0.003	0.001
Renal Osteodystrophy	0.20	0.18	0.22	0.02
Renal Tubular Acidosis	0.05	0.05	0.05	0.08
Respiratory Distress Syndrome (RDS)	0.20	0.19	0.21	0.60
Restless Legs Syndrome	3.0	3.8	2.2	0.80
Restrictive Cardiomyopathy	0.05	0.04	0.06	0.01
Retinal Detachment	0.15	0.12	0.18	0.01
Retinal Vein Occlusion	0.10	0.10	0.10	0.002
Retinitis Pigmentosa	0.05	0.05	0.05	0.06
Retinoblastoma	0.003	0.003	0.003	0.007
Retinopathy of Prematurity	0.30	0.29	0.31	0.90
Retinopathy of Prematurity (ROP)	0.30	0.29	0.31	0.90
Rh Incompatibility / Hemolytic Disease of the Newborn	0.05	0.05	0.05	0.15
Rhabdomyolysis	0.20	0.15	0.25	0.05
Rhabdomyosarcoma	0.002	0.002	0.002	0.004
Rheumatic Heart Disease	0.15	0.17	0.13	0.05
Rheumatoid Arthritis	1.5	2.2	0.80	0.20
Rickets and Osteomalacia	1.0	0.90	1.1	2.5
Rocky Mountain Spotted Fever	0.002	0.002	0.002	0.003
Rosacea	6.0	7.0	5.0	0.10
Rosai-Dorfman Disease	0.000	0.000	0.000	0.000
Roseola (HHV-6 / Sixth Disease)	3.0	2.9	3.1	11
Rotator Cuff Tendinopathy and Impingement	5.0	4.5	5.5	0.30
Rotavirus	5.0	4.9	5.1	17

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Rubella	0.10	0.10	0.10	0.40
SIADH	0.05	0.04	0.06	0.02
SSRI Overdose and Serotonin Syndrome	0.10	0.09	0.11	0.05
SUNCT and SUNA	0.001	0.001	0.001	0.000
Salicylate (Aspirin) Overdose	0.03	0.03	0.03	0.05
Sarcoidosis	0.20	0.25	0.15	0.03
Sarcopenia	15	12	18	0
Sarcopenic Obesity	10	9.0	11	0
Scabies	20	20	20	30
Scarlet Fever	0.50	0.49	0.51	2.0
Schistosomiasis	1.0	0.95	1.1	1.5
Schizoaffective Disorder	0.20	0.20	0.20	0.05
Schizophrenia	0.70	0.60	0.80	0.15
Schwannoma	0.01	0.009	0.01	0.001
Scleritis	0.05	0.06	0.04	0.01
Scoliosis	3.0	3.8	2.2	5.0
Scorpion Stings	0.05	0.04	0.06	0.08
Seborrheic Dermatitis	12	11	13	6.0
Selenium Deficiency	0.30	0.30	0.30	0.50
Sepsis and Septic Shock	1.5	1.4	1.6	0.90
Septic Abortion	0.005	0.009	0	0
Septic Arthritis	0.10	0.09	0.11	0.03
Severe Combined Immunodeficiency	0.001	0.001	0.001	0.002
Shift Work Sleep Disorder	0.50	0.40	0.60	0
Short Bowel Syndrome	0.01	0.009	0.01	0.02
Sick Sinus Syndrome	0.30	0.30	0.30	0.005
Sickle Cell Disease	0.30	0.29	0.31	0.50
Sideroblastic Anemia	0.01	0.009	0.01	0.003
Silicosis	0.10	0.01	0.19	0
Sleep-Related Bruxism	3.0	2.8	3.2	4.0
Sleep-Related Rhythmic Movement Disorder	0.10	0.10	0.10	0.30
Small Bowel Obstruction	0.30	0.28	0.32	0.05
Snake Envenomation	0.05	0.04	0.07	0.04
Social Anxiety Disorder (Social Phobia)	5.0	5.5	4.5	3.0
Somatic Symptom Disorder	0.50	0.60	0.40	0.20
Specific Phobias	8.0	10	6.0	6.0
Spider Bites	0.20	0.19	0.21	0.20
Splenic Abscess	0.001	0.000	0.001	0.000
Splenic Artery Aneurysm	0.005	0.005	0.005	0.001
Splenic Cyst	0.01	0.009	0.01	0.005

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Splenic Cysts	0.01	0.009	0.01	0.005
Splenic Infarction	0.02	0.02	0.02	0.005
Splenic Lymphoma	0.005	0.004	0.006	0.001
Splenic Rupture	0.01	0.007	0.01	0.006
Splenomegaly	0.50	0.48	0.52	0.20
Spontaneous Abortion (Miscarriage)	15	28	0	0
Squamous Cell Carcinoma	5.0	3.5	6.5	0.03
Stargardt Disease	0.005	0.005	0.005	0.005
Stevens-Johnson Syndrome and Toxic Epidermal Necrolysis	0.002	0.002	0.002	0.003
Stillbirth (Intrauterine Fetal Demise)	0.10	0.19	0	0
Stimulant Overdose (Cocaine, Methamphetamine)	0.05	0.04	0.07	0.005
Stimulant Use Disorder (Cocaine, Amphetamines)	1.0	0.60	1.4	0.05
Strabismus	1.5	1.5	1.5	3.5
Subdural Hematoma	0.15	0.10	0.20	0.02
Sudden Sensorineural Hearing Loss	0.05	0.05	0.05	0.03
Supraventricular Tachycardia	0.30	0.35	0.25	0.20
Syphilis (<i>Treponema pallidum</i>)	1.5	1.4	1.6	0.20
Systemic Lupus Erythematosus	0.40	0.70	0.10	0.10
Systemic Lupus Erythematosus (Skin Manifestations)	0.30	0.53	0.07	0.08
Systemic Sclerosis (Scleroderma)	0.10	0.16	0.04	0.01
Takayasu Arteritis	0.003	0.004	0.002	0.001
Tapeworm Infections (Taeniasis / Cysticercosis)	0.30	0.29	0.31	0.40
Temporal Lobe Epilepsy	0.10	0.10	0.10	0.12
Temporomandibular Joint Disorders	5.0	6.5	3.5	1.0
Tension-Type Headache	40	50	30	30
Testicular Cancer	0.04	0	0.08	0.01
Tetanus (<i>Clostridium tetani</i>)	0.002	0.002	0.002	0.001
Thalassemia	0.50	0.49	0.51	0.60
Theophylline Overdose	0.005	0.004	0.005	0.003
Thrombotic Thrombocytopenic Purpura	0.001	0.001	0.001	0.001
Thyroid Nodules and Cancer	3.0	3.5	2.5	0.20
Thyroiditis	0.50	0.70	0.30	0.10
Tinea Infections	25	24	26	25
Tinea Versicolor (Pityriasis Versicolor)	2.0	1.9	2.1	2.5

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Tinnitus	15	14	16	5.0
Tonsillar Hypertrophy	1.0	0.95	1.1	3.0
Tonsillitis	10	10	9.5	20
Tourette Syndrome	0.50	0.20	0.80	1.0
Toxoplasmosis	1.5	1.5	1.5	1.0
Transient Ischemic Attack	0.30	0.25	0.35	0.005
Trichomoniasis	3.0	3.5	2.5	0.05
Tricuspid Regurgitation	1.5	1.6	1.4	0.05
Tricyclic Antidepressant (TCA) Overdose	0.02	0.02	0.02	0.01
Trigeminal Autonomic Cephalalgias	0.01	0.008	0.01	0.003
Trigeminal Neuralgia	0.05	0.06	0.04	0.003
Trigger Finger	1.0	1.4	0.60	0.05
Triple-Negative Breast Cancer (TNBC)	0.03	0.06	0	0.005
Tuberculosis	2.5	2.2	2.8	1.5
Tuberculosis (Extrapulmonary)	0.50	0.45	0.55	0.30
Turner Syndrome (45,X)	0.03	0.03	0	0.05
Tympanic Membrane Perforation	1.0	0.95	1.1	1.5
Type 1 Diabetes Mellitus	0.40	0.40	0.40	0.80
Type 2 Diabetes Mellitus	12	12	12	1.0
Typhoid Fever (Salmonella typhi)	0.50	0.49	0.51	0.80
Ulcerative Colitis	0.30	0.32	0.28	0.15
Undifferentiated Connective Tissue Disease	0.20	0.32	0.08	0.05
Urea Cycle Disorders	0.005	0.005	0.005	0.01
Urinary Incontinence	15	20	10	1.0
Urosepsis	0.30	0.35	0.25	0.05
Urticaria	20	21	19	15
Usher Syndrome	0.005	0.005	0.005	0.006
Uterine Fibroids	10	19	0	0.50
Uterine Rupture	0.005	0.009	0	0
Uveitis	0.20	0.21	0.19	0.08
Vaginal Cancer	0.01	0.02	0	0.001
Variant CJD (Mad Cow Disease)	0.000	0.000	0.000	0.000
Varicella-Zoster (Systemic Aspects)	3.0	3.1	2.9	8.0
Vasa Previa	0.001	0.002	0	0
Vascular Dementia	0.30	0.29	0.31	0
Ventricular Tachycardia	0.20	0.15	0.25	0.01
Vertigo	2.0	2.4	1.6	0.30
Vestibular Neuritis	0.05	0.05	0.05	0.01
Viral Encephalitis	0.02	0.02	0.02	0.04

Disease	Common (%)	Females (%)	Males (%)	Children (%)
Viral Exanthems	3.0	3.0	3.0	10
Vitamin A Deficiency	2.0	1.9	2.1	3.5
Vitamin B1 (Thiamine) Deficiency — Beriberi and Wernicke-Korsakoff Syndrome	0.50	0.40	0.60	0.30
Vitamin B12 (Cobalamin) Deficiency	3.0	3.5	2.5	1.0
Vitamin B12 Deficiency Anemia	2.0	2.3	1.7	0.80
Vitamin B2 (Riboflavin) Deficiency — Ariboflavinosis	1.0	1.0	1.0	1.5
Vitamin B3 (Niacin) Deficiency — Pellagra	0.50	0.48	0.52	0.60
Vitamin B6 (Pyridoxine) Deficiency	0.30	0.30	0.30	0.40
Vitamin B7 (Biotin) Deficiency	0.05	0.05	0.05	0.10
Vitamin B9 (Folate) Deficiency	1.5	1.8	1.2	0.80
Vitamin C Deficiency — Scurvy	0.30	0.28	0.32	0.20
Vitamin D Deficiency — Rickets and Osteomalacia	25	26	24	40
Vitamin E Deficiency	0.05	0.05	0.05	0.08
Vitamin K Deficiency	0.30	0.29	0.31	0.60
Vitiligo	1.5	1.6	1.4	1.2
Vocal Cord Nodules	0.30	0.35	0.25	0.50
Vocal Cord Paralysis	0.10	0.09	0.11	0.03
Von Willebrand Disease	0.30	0.31	0.29	0.30
Vulvar Cancer	0.03	0.06	0	0.001
Vulvovaginal Candidiasis	15	28	0	1.5
Warts	20	20	20	30
West Nile Virus	0.002	0.002	0.002	0.001
Williams Syndrome	0.003	0.003	0.003	0.006
Wilms Tumor (Nephroblastoma)	0.002	0.002	0.002	0.005
Wilson's Disease	0.01	0.009	0.01	0.01
Wound Infections and Surgical Site Infections	1.5	1.3	1.7	0.50
Yellow Fever	0.002	0.002	0.002	0.002
Zenker Diverticulum	0.02	0.01	0.03	0.001
Zika Virus	0.01	0.01	0.01	0.01
Zinc Deficiency	1.5	1.4	1.6	2.5
Zollinger-Ellison Syndrome	0.001	0.001	0.001	0.000

Abbreviations Used

Abbreviation	Full Form
DNA	Deoxyribonucleic acid
RNA	Ribonucleic acid
CT	Computed tomography
MRI	Magnetic resonance imaging
ECG/EKG	Electrocardiogram
EEG	Electroencephalogram
EMG	Electromyography
ANA	Antinuclear antibody
HIV	Human immunodeficiency virus
HPV	Human papillomavirus
TNF	Tumor necrosis factor
IL	Interleukin
Ig	Immunoglobulin
CBC	Complete blood count
CMP	Comprehensive metabolic panel
TSH	Thyroid-stimulating hormone
ACTH	Adrenocorticotrophic hormone
BMR	Basal metabolic rate
GFR	Glomerular filtration rate
FEV1	Forced expiratory volume in 1 second

Glossary of Medical Terms

Aneurysm	Abnormal bulging or weakening of a blood vessel wall
Biopsy	Removal of tissue sample for microscopic examination
Carcinoma	Malignant tumor arising from epithelial tissue
Cause	The agent, factor, or mechanism producing a disease
Chronic	Long-standing or persistent condition (duration >3 months)
Clinical	Related to patient observation, examination, and diagnosis
Complication	Secondary disease or condition developing during course of another
Cortex	Outer layer of an organ or structure (e.g., renal cortex)
Curable	Capable of being completely treated or healed
Diagnosis	Identification of disease through signs, symptoms, and testing
Etiology	Study of causes or origin of disease
Granuloma	Small area of inflammation formed by immune cells (macrophages)
Hematopoiesis	Formation and development of blood cellular components
Hyperplasia	Increase in number of cells in an organ or tissue
Hypertrophy	Enlargement of organ or tissue from cell growth
Idiopathic	Disease of unknown cause
Immunodeficiency	Inadequate immune response to pathogens
Inflammation	Body's protective response to injury or infection
Infection	Invasion and multiplication of pathogenic microorganisms
Lesion	Area of abnormal tissue change from disease or injury
Malignant	Cancerous; capable of invasion and metastasis
Marrow	Soft tissue inside bones producing blood cells (bone marrow)
Mucosa	Membrane lining body cavities open to exterior
Mutations	Changes in DNA sequence that may cause disease
Necrosis	Death of cells or tissue due to injury or disease
Neoplasm	Abnormal new growth of tissue (tumor)
Nodule	Small knot-like mass or lump in tissue
Obstruction	Blockage of a body passage or canal
Organism	Living microorganism (bacterium, virus, fungus, etc.)
Pathogen	Disease-causing microorganism
Pathophysiology	Abnormal function associated with disease process
Perforation	Creation of hole in tissue or organ wall
Polyuria	Excessive urine production (>2.5 L/day)

Prognosis	Predicted course and outcome of disease
Protector	Substance or factor providing defense against disease
Remission	Temporary decrease or disappearance of disease symptoms
Sarcoma	Malignant tumor arising from connective tissue
Sclerosis	Abnormal hardening of tissue (e.g., atherosclerosis)
Symptom	Subjective evidence of disease perceived by patient
Therapy	Treatment of disease to cure or relieve symptoms
Tumor	Abnormal mass of tissue (neoplasm)
Vasculitis	Inflammation of blood vessels
Acute	Sudden onset, severe, short duration
Benign	Non-cancerous; not invasive or metastatic
Biochemical	Related to chemical processes in living organisms
Constitutional	Relating to the entire body or general health
Dehydration	Deficiency of body water
Embolism	Blockage of a blood vessel by a clot or foreign material
Focal	Limited to one area or region
Gastrointestinal	Relating to stomach and intestines
Hemorrhage	Loss of blood from vessels
Hyperglycemia	Excess glucose in blood
Hypoglycemia	Deficient glucose in blood
Ischemia	Inadequate blood supply to tissue
Metastasis	Spread of cancer to distant sites
Mucous membrane	Moist tissue lining body cavities
Nephrotoxic	Poisonous to kidneys
Neurotoxic	Poisonous to nervous system
Oncology	Study of tumors and cancer
Physiology	Normal function of living organisms
Prophylaxis	Prevention of disease
Thrombosis	Formation of blood clot in vessel
Toxicology	Study of poisons and their effects
Ulcer	Open sore on skin or mucous membrane
Viral	Caused by virus
Autoimmune	Immune response against own tissues
Bacterial	Caused by bacteria
Fungal	Caused by fungus
Parasitic	Caused by parasite
Malignancy	Presence of cancer

Inflammation Localized response to injury

Infection Invasion by pathogens

Immunology Study of immune system

Cardiology Study of heart

Neurology Study of nervous system

Gastroenterology Study of digestive system

Endocrinology Study of endocrine glands

Dermatology Study of skin

Hematology Study of blood

Oncology Study of cancer

Radiology Study of medical imaging

Surgery Medical procedure involving operation

Anatomy Study of body structure

Pathology Study of disease processes

References

Bibliography

- [1] Harrison's Principles of Internal Medicine. 21st Edition. McGraw-Hill Education, 2022.
- [2] Robbins and Cotran Pathologic Basis of Disease. 10th Edition. Elsevier, 2020.
- [3] Mosby's Medical Dictionary. 12th Edition. Elsevier, 2023.
- [4] World Health Organization. International Classification of Diseases (ICD-11). WHO, 2022. <https://icd.who.int/>
- [5] Centers for Disease Control and Prevention. CDC Guidelines for Disease Prevention and Control. 2024.
- [6] UpToDate. Clinical Decision Support Resource. Wolters Kluwer, 2024.
- [7] National Institutes of Health. NIH Clinical Research and Guidelines. 2024. <https://www.nih.gov/>
- [8] Journal of the American Medical Association. Clinical Practice Guidelines. 2024.
- [9] New England Journal of Medicine. Research Articles and Reviews. 2024.
- [10] The Lancet. Global Health Research and Guidelines. 2024.
- [11] National Human Genome Research Institute. Genetic Disease Database. 2024. <https://www.genome.gov/>
- [12] The Merck Manual. Professional Version. Merck & Co., 2023.
- [13] Mayo Clinic Proceedings. Evidence-Based Clinical Reviews. 2024.
- [14] World Health Organization. Immunization and Vaccination Guidelines. 2024.
- [15] Infectious Diseases Society of America. Clinical Practice Guidelines. 2024.
- [16] American Heart Association. Cardiovascular Disease Guidelines. 2023.
- [17] American Diabetes Association. Standards of Medical Care in Diabetes. 2024.
- [18] American Cancer Society. Cancer Treatment and Prevention Guidelines. 2024.
- [19] World Health Organization. Global Infectious Disease Reports. 2024.
- [20] National Center for Biotechnology Information. PubMed Central and Genetic Databases. 2024. <https://www.ncbi.nlm.nih.gov/>

Index

- Bone loss, *see* Osteoporosis
- Abdominal Aortic Aneurysm, 1
- Abnormal Uterine Bleeding, 5
- Abscess, 9
- Acetaminophen (Paracetamol) Overdose, 13
- Achalasia, 17
- Achondroplasia, 21
- Acne Vulgaris, 25
- Acoustic Neuroma (Vestibular Schwannoma), 29
- Acromegaly, 33
- Actinic Keratosis, 37
- Acute Bacterial Sinusitis, 45
- Acute Bronchitis, 51
- Acute Decompensated Heart Failure, 55
- Acute Gastritis, 59
- Acute Interstitial Nephritis, 63
- Acute Kidney Injury, 67
- Acute Lymphoblastic Leukemia, 71
- Acute Lymphoblastic Leukemia (ALL) in Children, 77
- Acute Myeloid Leukemia, 83
- Acute Otitis Media, 89, 93
- Acute Pancreatitis, 97
- Acute Respiratory Distress Syndrome, 101
- Acute Stress Disorder, 105
- Acute/Short-Term Insomnia, 41
- Addison's Disease, 109
- ADHD, *see* Attention-Deficit/Hyperactivity Disorder (ADHD)
- Adhesive Capsulitis, 113
- Adjustment Disorder, 117
- Adrenal Incidentaloma, 121
- Adult-Onset Still's Disease, 125
- Advanced Sleep-Wake Phase Disorder, 129
- African Trypanosomiasis (Sleeping Sickness), 133
- Age-Related Bone Loss, 137
- Age-Related Cataract, 141
- Age-Related Macular Degeneration, 145, 149
- Agoraphobia, 153
- AIDS, *see* HIV/AIDS
- Alcohol Intoxication and Withdrawal, 157
- Alcohol Use Disorder, 161
- Alcoholic Liver Disease, 165
- Alkaptonuria — Ochronosis, 169
- Allergic Fungal Rhinosinusitis (AFRS), 173
- Allergic Rhinitis, 179
- Alopecia Areata, 183
- Alpha-1 Antitrypsin Deficiency, 187
- ALS, *see* Amyotrophic Lateral Sclerosis
- Altitude Sickness, 191
- Alzheimer's Disease, 195
- Ambiguous Genitalia / Disorders of Sex Development, 199
- Amblyopia, 203
- Amebiasis (*Entamoeba histolytica*), 207
- Amenorrhea (Primary, Secondary), 211
- Amyloidosis, 215
- Amyotrophic Lateral Sclerosis, 219
- Anal Fissure, 223
- Anaphylaxis, 227
- Angelman Syndrome, 231
- Angina Pectoris, 235
- Ankylosing Spondylitis, 239
- Anophthalmia and Microphthalmia, 243
- Anorexia Nervosa, 247
- Anthrax, 251
- Anti-GBM Disease, 255
- Antiphospholipid Syndrome, 259
- Antisocial Personality Disorder (ASPD), 263
- Aortic Dissection, 267
- Aortic Regurgitation, 271
- Aortic Stenosis, 275, 279

- Aphthous Ulcers (Canker Sores), 283
Aplastic Anemia, 287
Appendicitis, 291
Arboviral Encephalitis, 295
Arsenic Poisoning, 299
Asbestosis, 303
Ascites, 307
Aspergillosis (Invasive), 311
Aspiration Pneumonia, 315
Asplenia, 319
Asplenia and Hyposplenism, 323
Asthma, 327
Asthma-COPD Overlap, 331
Astigmatism, 335
Atelectasis, 339
Atopic Dermatitis, 343
Atrial Fibrillation, 347
Atrial Fibrillation in the Elderly, 351
Atrial Flutter, 355
Atrophic Vaginitis (Genitourinary Syndrome of Menopause), 359
Attention-Deficit/Hyperactivity Disorder (ADHD), 363
Atypical Facial Pain, 367
Autism Spectrum Disorder (ASD), 371
Autoimmune Hepatitis, 375
Autoimmune Pancreatitis, 381
Autoimmune Pancreatitis (AIP), 385
Autosomal Dominant Polycystic Kidney Disease, 389
- Bacterial Meningitis, 393
Bacterial Vaginosis, 397
Balanitis and Balanoposthitis, 401
Barotrauma of the Ear, 405
Barrett Esophagus, 409
Bartholin Cyst, 413
Basal Cell Carcinoma, 417
Becker Muscular Dystrophy, 423
Bed sores, *see* Pressure Ulcers
Behçet's Disease, 427
Bell's Palsy, 431
Benign Paroxysmal Positional Vertigo, 435
Benign Prostatic Hyperplasia, 439
Benzodiazepine Overdose, 443
Beta-Blocker Overdose, 447
Bietti's Crystalline Dystrophy, 463
Binge-Eating Disorder, 451
Bipolar I Disorder, 459
Bipolar II Disorder, 455
Bladder Cancer, 467
Blastomycosis, 473
Bleeding stroke, *see* Hemorrhagic Stroke
Blepharitis, 477
Blepharospasm, 481
Blood poisoning, *see* Sepsis and Septic Shock
Body Dysmorphic Disorder, 485
Borderline Personality Disorder (BPD), 489
Botulism (*Clostridium botulinum*), 493
BPH, *see* Benign Prostatic Hyperplasia
Brain Abscess, 497
Brain Tumors in Children, 503
Breast Cancer, 507
Bronchiectasis, 513
Bronchiolitis (RSV), 517
Bronchopulmonary Dysplasia (BPD), 521
Brucellosis, 525
Brugada Syndrome, 529
Budd-Chiari Syndrome, 533
Bulimia Nervosa, 537
Bullous Pemphigoid, 541
Bursitis, 545
- Cadmium Toxicity, 553
Calcium and Phosphorus Disorders, 565
Calcium Channel Blocker Overdose, 557
Calcium Pyrophosphate Deposition Disease, 561
Cancer of the breast, *see* Breast Cancer
Cancer of the colon, *see* Colorectal Cancer
Cancer of the lung, *see* Lung Cancer
Cancer of the prostate, *see* Prostate Cancer
Cancer: blood, *see* Leukemia
Cancer: brain, *see* Glioblastoma
Cancer: breast, *see* Breast Cancer
Cancer: colon, *see* Colorectal Cancer
Cancer: lung, *see* Lung Cancer

-
- Cancer: prostate, *see* Prostate Cancer
Cancer: skin, *see* Melanoma
Candida auris, 569
Candidiasis (Invasive / Systemic), 573
Cannabis Toxicity and Cannabinoid
 Hyperemesis Syndrome, 577
Carbon Monoxide Poisoning, 581
Cardiac Tamponade, 585
Carpal Tunnel Syndrome, 589
Celiac Disease, 593
Cellulitis, 597
Central Pontine Myelinolysis, 601
Central Post-Stroke Pain, 605
Central Sleep Apnea, 609
Cerebral Aneurysm, 613
Cerebral Venous Thrombosis, 617
Cerebral Visual Impairment, 621
Cerumen Impaction, 625
Cervical Cancer, 629, 635
Cervicitis (Chlamydia, Gonorrhea), 641
Cervicogenic Headache, 645
Chagas Disease (American
 Trypanosomiasis), 649
Chancroid, 653
Charcot-Marie-Tooth Disease, 657
CHF, *see* Heart Failure with Reduced
 Ejection Fraction
Chickenpox, 661
Chikungunya, 667
Choanal Atresia, 671
Cholecystitis, 675
Cholelithiasis, 679
Cholera (*Vibrio cholerae*), 683
Chondrodermatitis Nodularis Helicis, 689
Chronic Fatigue Syndrome, 693
Chronic Gastritis, 697
Chronic Hypertension in Pregnancy, 701
Chronic Insomnia Disorder, 705
Chronic Kidney Disease, 709
Chronic Low Back Pain, 713
Chronic Lymphocytic Leukemia, 717
Chronic Neck Pain, 723
Chronic Obstructive Pulmonary Disease,
 727
Chronic Otitis Media, 731
Chronic Pancreatitis, 735
Chronic Pelvic Pain, 739
Chronic Rhinosinusitis, 743
Chronic Traumatic Encephalopathy, 747
Chronic Tubulointerstitial Nephritis, 751
Cirrhosis, 755
Cleft Lip and Palate, 759
Clostridioides difficile Colitis, 763
Cluster Headache, 767
Coarctation of the Aorta, 771
Coccidioidomycosis (Valley Fever), 775
Cogan's Syndrome, 779
Coloboma, 783
Color Blindness, 797
Colorectal Adenoma, 787
Colorectal Cancer, 791
Coma, 801
Common Variable Immunodeficiency, 805
Community-Acquired Pneumonia, 809
Compartment Syndrome, 813
Complex Regional Pain Syndrome, 817
Complex Sleep Apnea, 821
Concussion, 825
Congenital Cataract, 829
Congenital Diaphragmatic Hernia, 833
Congenital Heart Disease (Overview),
 837
Congenital Hip Dysplasia, 841
Congenital Talipes Equinovarus
 (Clubfoot), 845
Conjunctivitis, 849
Constrictive Pericarditis, 853
Contact Dermatitis, 857
Convergence Insufficiency, 861
COPD, *see* Chronic Obstructive
 Pulmonary Disease
Copper Deficiency and Excess, 865
Coronary Artery Disease, 869
COVID-19, 549
Creutzfeldt-Jakob Disease (CJD), 873
Cri-du-Chat Syndrome, 877
Crohn's Disease, 881
Croup (Laryngotracheobronchitis), 885
Cryoglobulinemic Vasculitis, 889
Cryptococcosis, 893
Cryptorchidism (Undescended Testis),
 897
-

- CTE, *see* Chronic Traumatic Encephalopathy
Cushing Syndrome, 901
Cushing's Syndrome, 905
CVA, *see* Ischemic Stroke
Cyanide Toxicity, 909
Cyclospora cayetanensis, 913
Cyclothymic Disorder, 917
Cystic Fibrosis, 921
Cystitis, 925
Cytomegalovirus (CMV), 929
- Deep Vein Thrombosis, 933
Delayed Sleep-Wake Phase Disorder, 937
Delirium, 941
Delusional Disorder, 945
Dementia with Lewy Bodies, 949
Dengue Fever, 953
Dermatitis Herpetiformis, 959
Dermatomyositis, 963, 967
Deviated Nasal Septum, 971
Diabetes Insipidus, 979
Diabetic Ketoacidosis, 983
Diabetic Nephropathy, 987
Diabetic Peripheral Neuropathic Pain, 991
Diabetic Retinopathy, 995
DiGeorge Syndrome, 975
Digoxin Toxicity, 999
Dilated Cardiomyopathy, 1003
Diphtheria, 1007
Diphtheria (Corynebacterium diphtheriae), 1011
Disseminated Intravascular Coagulation, 1017
Dissociative Amnesia, 1021
Dissociative Identity Disorder, 1025
Diverticulitis, 1029
Diverticulosis, 1033
DM, *see* Type 2 Diabetes Mellitus
Down Syndrome (Trisomy 21), 1037
Drowning (Submersion Injury), 1041
Drug Hypersensitivity, 1045
Dry Eye Disease, 1049
Duchenne Muscular Dystrophy, 1053
Ductal Carcinoma In Situ (DCIS), 1057
Duodenal Ulcer, 1063
Dupuytren Contracture, 1067
DVT, *see* Deep Vein Thrombosis
Dysbetalipoproteinemia, 1071
Dysmenorrhea, 1075
Dystonia, 1079
- Eastern Equine Encephalitis, 1083
Ebola Virus Disease, 1087
Eclampsia, 1093
Ectopic Pregnancy, 1097
Edwards Syndrome (Trisomy 18), 1101
Ehlers-Danlos Syndrome, 1105
Ehrlichiosis, 1109
Electrical Injury and Lightning Strike, 1113
Empyema, 1117
End-Stage Renal Disease, 1121
Endometrial / Uterine Cancer, 1125
Endometriosis, 1131, 1135
Eosinophilic Esophagitis, 1139
Eosinophilic Fasciitis (Shulman Syndrome), 1143
Eosinophilic Granulomatosis with Polyangiitis, 1147
Epidermolysis Bullosa, 1151
Epidural Hematoma, 1155
Epiglottitis, 1159, 1163
Epilepsy, 1167
Epistaxis, 1171
Epstein-Barr Virus / Infectious Mononucleosis (Systemic Aspects), 1175
Erectile Dysfunction, 1179
Erythema Multiforme, 1183
Esophageal Atresia / Tracheoesophageal Fistula, 1187
Esophageal Varices, 1191
Essential Thrombocythemia, 1195
Essential Tremor, 1199
Esthesioneuroblastoma, 1203
Ewing Sarcoma, 1209
Ewing Sarcoma in Children, 1215
- Facioscapulohumeral Muscular Dystrophy, 1221
Failure to Thrive, 1225

-
- Familial Adenomatous Polyposis, 1229
Familial Combined Hyperlipidemia, 1233
Familial Hypercholesterolemia, 1237
Fanconi Syndrome, 1241
Fat Necrosis of the Breast, 1245
Febrile Seizures, 1249
Fecal Incontinence, 1253
Fibroadenoma, 1257
Fibrocystic Changes, 1261
Fibromyalgia, 1265, 1269
Fibrous Dysplasia, 1273
Fifth Disease (Erythema Infectiosum), 1277
Filariasis (Lymphatic Filariasis), 1281
Floaters and Vitreous Detachment, 1285
Fluoride Excess — Fluorosis, 1289
Focal Segmental Glomerulosclerosis, 1293
Folate Deficiency Anemia, 1297
Fractures, 1301
Fragile X Syndrome, 1305
Frailty, 1309
Friedreich's Ataxia, 1313
Frontotemporal Dementia, 1317, 1321
Fuchs' Endothelial Dystrophy, 1325
Fungal Sinusitis, 1329
- Galactoceles, 1333
Galactosemia, 1337
Gas Gangrene (Clostridial Myonecrosis), 1341
Gastric Adenocarcinoma, 1345
Gastric Lymphoma, 1351
Gastric Ulcer, 1357
Gastroesophageal Reflux Disease, 1361
Gastrointestinal Stromal Tumor, 1365
Gastroparesis, 1369
Generalized Anxiety Disorder (GAD), 1373
Genital Herpes (HSV), 1377
Genital Warts (HPV), 1381
GERD, *see* Gastroesophageal Reflux Disease
German measles, *see* Rubella
Gestational Diabetes Mellitus, 1385
Gestational Diabetes Mellitus (Obstetric Focus), 1389
Gestational Hypertension, 1393
Gestational Trophoblastic Disease, 1397
Giant Cell Arteritis, 1401
Giardiasis, 1405
Gingivitis and Periodontitis, 1409
Glaucoma in Children, 1413
Glioblastoma, 1417
Glossopharyngeal Neuralgia, 1423
Glycogen Storage Diseases, 1427
Gonorrhea, 1431
Gout, 1435
Granuloma Annulare, 1439
Granulomatosis with Polyangiitis, 1443
Graves Disease, 1447
Graves' Eye Disease, 1451
Guillain-Barré Syndrome, 1455
Gynecomastia, 1459
- Haff Disease, 1473
Hallucinogen Toxicity, 1477
Hallux Valgus, 1481
Hand, Foot, and Mouth Disease, 1485
Hantavirus Pulmonary Syndrome, 1489
Hashimoto Thyroiditis, 1493
HDL, *see* Dysbetalipoproteinemia
Head Lice (Pediculosis Capitis), 1497
Headache Attributed to Giant Cell Arteritis, 1501
Headache Attributed to Intracranial Hypertension, 1505
Headache Attributed to Intracranial Hypotension, 1509
Headache Attributed to Sinusitis, 1513
Headache in Brain Tumors, 1517
Heart attack, *see* Myocardial Infarction
Heart Block (AV Block), 1521
Heart Failure with Preserved Ejection Fraction, 1525, 1529
Heart Failure with Reduced Ejection Fraction, 1533
Heat Stroke and Hyperthermia, 1537
Helicobacter Pylori Infection, 1541
HELLP Syndrome, 1463
Hemicrania Continua, 1545
Hemolytic Anemias, 1549
Hemolytic Uremic Syndrome (HUS), 1553
-

- Hemophagocytic Lymphohistiocytosis
and Macrophage Activation
Syndrome, 1557
- Hemophilia A, 1561
- Hemophilia B, 1565
- Hemorrhagic Stroke, 1569
- Hemorrhoids, 1573
- Henoch-Schönlein Purpura (IgA
Vasculitis), 1577
- Hepatic Encephalopathy, 1585
- Hepatitis A, 1589
- Hepatitis B, 1593
- Hepatitis C, 1597
- Hepatitis D, 1601
- Hepatitis E, 1605
- Hepatoblastoma, 1609
- Hepatocellular Carcinoma, 1615
- Hereditary Angioedema, 1621
- Hereditary Hemochromatosis, 1625, 1629
- Hereditary Spherocytosis, 1633
- Herniated Disc, 1581
- Herpangina, 1637
- Herpes Simplex, 1641
- Herpes Zoster, 1645
- Hidradenitis Suppurativa, 1649
- High blood pressure, *see* Hypertension
- Hip Fracture, 1653
- Hirschsprung Disease, 1657
- Histoplasmosis, 1661
- HIV, *see* HIV/AIDS
- HIV/AIDS, 1467
- Hoarding Disorder, 1665
- Hodgkin Lymphoma, 1669
- Homocystinuria, 1675
- Hookworm / Soil-Transmitted Helminths,
1679
- Hordeolum (Stye) and Chalazion, 1683
- HPV, *see* Genital Warts (HPV)
- HSV, *see* Genital Herpes (HSV)
- Human Metapneumovirus (hMPV), 1687,
1691
- Huntington's Disease, 1695
- Hydrogen Sulfide Toxicity, 1699
- Hydronephrosis, 1703
- Hymenoptera Stings, 1707
- Hyperemesis Gravidarum, 1711
- Hyperopia, 1715
- Hyperosmolar Hyperglycemic State, 1719
- Hyperparathyroidism, 1723
- Hyperprolactinemia, 1727
- Hypersplenism, 1731
- Hypertension, 1735
- Hypertension in the Elderly, 1739
- Hyperthyroidism, 1743
- Hypertrophic Cardiomyopathy, 1747
- Hypertrophic Pyloric Stenosis, 1751
- Hypoglycemia, 1755
- Hypoparathyroidism, 1759
- Hypopituitarism, 1763
- Hypothermia, 1767
- Hypothyroidism, 1771
- IBS, *see* Irritable Bowel Syndrome
- Idiopathic Granulomatous Mastitis, 1775
- Idiopathic Hypersomnia, 1779
- Idiopathic Insomnia, 1783
- Idiopathic Intracranial Hypertension,
1787
- Idiopathic Pulmonary Fibrosis, 1791
- IgA Nephropathy, 1795
- IgA Vasculitis, 1799
- IgG4-Related Disease, 1803
- Illness Anxiety Disorder
(Hypochondriasis), 1807
- Immune Thrombocytopenic Purpura,
1811
- Impetigo, 1815
- Inclusion Body Myositis, 1819
- Indeterminate Colitis, 1823
- Infectious Mononucleosis, 1827
- Infective Endocarditis, 1833
- Inflammatory Breast Cancer, 1837
- Influenza, 1843
- Insomnia in the Context of
Medical/Psychiatric Disorders,
1849
- Intraductal Papillary Mucinous
Neoplasm (IPMN), 1853
- Intraductal Papilloma, 1857
- Intrauterine Growth Restriction, 1861
- Intraventricular Hemorrhage (IVH), 1865
- Intussusception, 1869

- Invasive Lobular Carcinoma (ILC), 1873
Inverted Papilloma, 1879
Iodine Deficiency Disorders, 1883
Iron Deficiency, 1887
Iron Deficiency Anemia, 1891
Iron Overdose, 1895
Irregular Sleep-Wake Rhythm Disorder, 1899
Irritable Bowel Syndrome, 1903
Ischemic Cardiomyopathy, 1907
Ischemic Stroke, 1911
- Jet Lag Disorder, 1915
Juvenile Nasopharyngeal Angiofibroma (JNA), 1919
- Kawasaki Disease, 1923, 1927
Keratitis, 1931
Keratoconus, 1935
Kidney Cancer (Renal Cell Carcinoma), 1939
Kikuchi-Fujimoto Disease, 1945
Kleine-Levin Syndrome, 1949
Klinefelter Syndrome (47,XXY), 1953
Kuru, 1957
Kwashiorkor, 1961
Kyphosis, 1965
- Labyrinthitis, 1969
Lactose Intolerance, 1973
Lambert-Eaton Myasthenic Syndrome (LEMS), 1977
Large Bowel Obstruction, 1981
Laryngitis, 1985
Lateral Epicondylitis (Tennis Elbow), 1989
LDL, *see* Familial Hypercholesterolemia
Lead Poisoning (Plumbism), 1993
Leishmaniasis, 1997
Leprosy (Hansen's Disease), 2001
Leptospirosis, 2005
Lipoprotein(a) Disorders, 2009
Lithium Toxicity, 2013
Lobular Carcinoma In Situ (LCIS), 2017
Lockjaw, *see* Tetanus (*Clostridium tetani*)
Long QT Syndrome, 2023
- Low Vision, 2027
Ludwig Angina, 2031
Lung Abscess, 2035
Lung Cancer, 2041
Lyme Disease (*Borrelia burgdorferi*), 2047
Lymphogranuloma Venereum (LGV), 2051
Lynch Syndrome, 2055
Lysosomal Storage Diseases, 2059
- Macular Edema, 2063
Macular Hole, 2067
Macular Pucker, 2071
Mad Cow Disease, *see* Variant CJD (Mad Cow Disease)
Major Depressive Disorder (MDD), 2079
Malaria (*Plasmodium* spp.), 2083
Male Breast Cancer, 2089
Male Infertility, 2095
Malignant Hyperthermia, 2099
Malnutrition in the Elderly, 2103
Mammalian Bites, 2107
Mammary Duct Ectasia, 2111
Maple Syrup Urine Disease, 2115
Marasmic-Kwashiorkor, 2119
Marasmus, 2123
Marburg Virus, 2127
Marfan Syndrome, 2131
Marine Envenomation, 2135
Massive Splenomegaly, 2139
Mastitis (Puerperal), 2143
Mastitis and Breast Abscess, 2147
McArdle Disease, 2153
MDMA (Ecstasy) Toxicity, 2075
Measles (Rubeola), 2157
Meconium Aspiration Syndrome, 2163
Medication Overuse Headache, 2167
Medulloblastoma, 2171
Melanoma, 2177
Melasma, 2183
Membranous Nephropathy, 2187
Memory loss, *see* Alzheimer Disease
Meningioma, 2191
Mercury Toxicity, 2195
Mesothelioma, 2199

- Metabolic Syndrome, 2203
Metformin Overdose, 2207
Methemoglobinemia, 2211
Methyl Parathion and Other
 Organophosphate Insecticides,
 2215
MI, *see* Myocardial Infarction
Microscopic Polyangiitis, 2219
Migraine, 2223
Mild Cognitive Impairment, 2227
Minimal Change Disease, 2231
Mitral Regurgitation, 2235
Mitral Stenosis, 2239
Mitral Valve Prolapse, 2243
Mixed Connective Tissue Disease, 2247
Molluscum Contagiosum, 2251
Morton Neuroma, 2255
Motor Neuron Disease, 2259
Mpox, 2263
MRSA, *see* Wound Infections and
 Surgical Site Infections
MS, *see* Multiple Sclerosis
Mucopolysaccharidoses, 2267
Mucormycosis, 2271
Multiple Endocrine Neoplasia, 2275
Multiple Gestation Complications, 2279
Multiple Myeloma, 2283
Multiple Myeloma (Bone
 Manifestations), 2289
Multiple Sclerosis, 2295
Mumps, 2299
Myasthenia Gravis, 2305
Myelodysplastic Syndromes, 2309
Myelofibrosis, 2313
Myocardial Infarction, 2317
Myocarditis, 2321
Myofascial Pain Syndrome, 2325
Myopia, 2329
Myotonic Dystrophy, 2333

NAFLD, *see* Non-Alcoholic Fatty Liver
 Disease
Narcissistic Personality Disorder, 2353
Narcolepsy Type 1, 2357
Narcolepsy Type 2, 2361
Nasal Fracture, 2365

Nasal Polyps, 2369
NASH, *see* Non-Alcoholic Fatty Liver
 Disease
Necrotizing Enterocolitis (NEC), 2373
Necrotizing Fasciitis, 2377
Neonatal Abstinence Syndrome, 2381
Neonatal Hypoglycemia, 2385
Neonatal Jaundice / Hyperbilirubinemia
 / Kernicterus, 2389
Neonatal Sepsis (Early-Onset and
 Late-Onset), 2393
Nephritic Syndrome, 2397
Nephrolithiasis, 2401
Nephrotic Syndrome, 2405
Nesidioblastosis and Congenital
 Hyperinsulinism, 2409
Neural Tube Defects, 2413
Neuroblastoma, 2417
Neurocysticercosis, 2423
Neurofibromatosis Type 1, 2427
New Daily Persistent Headache, 2431
Non-24-Hour Sleep-Wake Rhythm
 Disorder, 2435
Non-Alcoholic Fatty Liver Disease, 2439
Non-Allergic Rhinitis, 2443
Non-Gonococcal Urethritis (NGU), 2447
Non-Hodgkin Lymphoma, 2451
Normal Pressure Hydrocephalus, 2457
Norovirus, 2461
NREM Parasomnias: Confusional
 Arousals, 2337
NREM Parasomnias: Sleep Terrors, 2345
NREM Parasomnias: Sleep-Related
 Eating Disorder, 2341
NREM Parasomnias: Sleepwalking, 2349
Nummular Dermatitis, 2465

OA, *see* Osteoarthritis
Obesity, 2469
Obesity Hypoventilation Syndrome, 2473
Obsessive-Compulsive Disorder (OCD),
 2477
Obstructive Sleep Apnea, 2481, 2485
Occipital Neuralgia, 2489
OCD, *see* Obsessive-Compulsive Disorder
 (OCD)

- Ocular Histoplasmosis Syndrome, 2493
Oculocutaneous Albinism, 2497
Onychomycosis (Tinea Unguium), 2501
Opioid Overdose, 2505
Opioid Use Disorder, 2509
Oral Candidiasis, 2513
Oral Leukoplakia, 2517
Oral Lichen Planus, 2521
Oral Squamous Cell Carcinoma, 2525
Organic Acidemias, 2531
Organophosphate and Carbamate
 Insecticide Poisoning, 2535
Osteoarthritis, 2539
Osteogenesis Imperfecta, 2543
Osteomyelitis, 2547
Osteoporosis, 2551
Osteosarcoma, 2555
Osteosarcoma in Children, 2561
Other Parasomnias: Exploding Head
 Syndrome, 2567
Other Parasomnias: Sleep Enuresis, 2571
Other Parasomnias: Sleep-Related
 Hallucinations, 2575
Other Primary Headache Disorders, 2579
Otitis Externa, 2583
Otosclerosis, 2587
Ovarian Cancer, 2591, 2597
- Paget's Disease of Bone, 2603
Paget's Disease of the Nipple, 2607
Pancreatic Adenocarcinoma, 2611
Pancreatic Exocrine Insufficiency, 2617
Pancreatic Neuroendocrine Tumors, 2621
Pancreatic Pseudocyst, 2625
Panic Disorder / Panic Attacks, 2629
Parkinson's Disease, 2633
Paroxysmal Hemicrania, 2637
Patau Syndrome (Trisomy 13), 2641
Patent Ductus Arteriosus (PDA) in
 Prematurity, 2645
PE, *see* Pulmonary Embolism
Pediatric Appendicitis, 2649
Pelvic Inflammatory Disease, 2653, 2657
Pemphigus Vulgaris, 2661
Penile Cancer, 2665
Penile Fracture, 2671
Peptic Ulcer Bleeding, 2675
Peptic Ulcer Disease, 2679
Pericarditis, 2683
Perinatal Asphyxia / Hypoxic-Ischemic
 Encephalopathy (HIE), 2687
Periodic Fever Syndromes, 2691
Periodic Limb Movement Disorder, 2695
Periodic Paralysis, 2699
Peripheral Arterial Disease, 2703
Peripheral Artery Disease, 2707
Periventricular Leukomalacia (PVL),
 2711
Persistent Depressive Disorder
 (Dysthymia), 2715
Persistent Pulmonary Hypertension of
 the Newborn (PPHN), 2719
Persistent Vegetative State, 2723
Pertussis (Whooping Cough), 2727
Peyronie's Disease, 2731
Phantom Limb Pain, 2735
Pharyngitis, 2739
Phenylketonuria, 2743
Pheochromocytoma, 2747
Phimosis and Paraphimosis, 2751
Phyllodes Tumor, 2755
Pilonidal Sinus, 2759
Pituitary Adenoma, 2763
PKD, *see* Autosomal Dominant
 Polycystic Kidney Disease
Placenta Accreta Spectrum, 2767
Placenta Previa, 2771
Placental Abruption, 2775
Plague (*Yersinia pestis*), 2779
Plantar Fasciitis, 2783
Pleurisy, 2787
PMS, *see* Premenstrual Syndrome /
 Premenstrual Dysphoric Disorder
Pneumoconiosis, 2791
Pneumocystis jirovecii Pneumonia, 2795
Pneumothorax, 2799
Poliomyelitis, 2803
Polyarteritis Nodosa, 2807
Polycystic Ovary Syndrome, 2811, 2815
Polycythemia Vera, 2819
Polyhydramnios and Oligohydramnios,
 2823

- Polymyalgia Rheumatica, 2827
Polymyositis, 2831
Porphyrias, 2835
Portal Hypertension, 2839
Post-Traumatic Headache, 2843
Post-Traumatic Stress Disorder (PTSD), 2847
Postherpetic Neuralgia, 2851
Postpartum Cardiomyopathy, 2855
Postpartum Depression, 2859
Postpartum Endometritis, 2863
Postpartum Hemorrhage, 2867
Postpartum Thyroiditis, 2871
Prader-Willi Syndrome, 2875
Preeclampsia, 2879
Premature Ovarian Insufficiency, 2883
Premature Rupture of Membranes, 2887
Premenstrual Syndrome / Premenstrual Dysphoric Disorder, 2891
Presbycusis, 2895
Presbyopia, 2899
Presbyopia and Cataracts, 2903
Pressure sores, *see* Pressure Ulcers
Pressure Ulcers, 2907
Preterm Birth, 2911
Preterm Labor and Preterm Birth, 2915
Priapism, 2919
Primary Amebic Meningoencephalitis (*Naegleria fowleri*), 2923
Primary Angle-Closure Glaucoma, 2927
Primary Biliary Cholangitis, 2931
Primary Open-Angle Glaucoma, 2935
Primary Sclerosing Cholangitis, 2939
Propriospinal Myoclonus at Sleep Onset, 2943
Prostate Cancer, 2947
Psoriasis, 2953
Psoriatic Arthritis, 2957, 2961
Psychophysiologic Insomnia, 2965
Pterygium, 2969
PTSD, *see* Post-Traumatic Stress Disorder (PTSD)
Pulmonary Embolism, 2973
Pulmonary Hypertension, 2977
Pyelonephritis, 2981
Rabies, 2997
Radiation Sickness (Acute Radiation Syndrome), 3001
Reactive Arthritis, 3005
Recurrent Pregnancy Loss, 3009
Relapsing Polychondritis, 3013
REM Parasomnias: Nightmare Disorder, 2985
REM Parasomnias: Recurrent Isolated Sleep Paralysis, 2993
REM Parasomnias: REM Sleep Behavior Disorder, 2989
Renal Osteodystrophy, 3017
Renal Tubular Acidosis, 3021
Respiratory Distress Syndrome (RDS), 3025
Restless Legs Syndrome, 3029
Restrictive Cardiomyopathy, 3033
Retinal Detachment, 3037
Retinal Vein Occlusion, 3041
Retinitis Pigmentosa, 3045
Retinoblastoma, 3049
Retinopathy of Prematurity, 3055
Retinopathy of Prematurity (ROP), 3059
Rh Incompatibility / Hemolytic Disease of the Newborn, 3063
Rhabdomyolysis, 3067
Rhabdomyosarcoma, 3071
Rheumatic Heart Disease, 3077
Rheumatoid Arthritis, 3081, 3085
Rickets and Osteomalacia, 3089
Rocky Mountain Spotted Fever, 3093
Rosacea, 3097
Rosai-Dorfman Disease, 3101
Roseola (HHV-6 / Sixth Disease), 3105
Rotator Cuff Tendinopathy and Impingement, 3109
Rotavirus, 3113
Rubella, 3117
Salicylate (Aspirin) Overdose, 3133
Sarcoidosis, 3137, 3141, 3145
Sarcopenia, 3149
Sarcopenic Obesity, 3153
Scabies, 3157
Scarlet Fever, 3161

- Schistosomiasis, 3165
Schizoaffective Disorder, 3169
Schizophrenia, 3173
Schwannoma, 3177
Scleritis, 3181
Scoliosis, 3185
Scorpion Stings, 3189
Seborrheic Dermatitis, 3193
Selenium Deficiency, 3197
Sepsis and Septic Shock, 3201
Septic Abortion, 3207
Septic Arthritis, 3211
Severe Combined Immunodeficiency, 3217
Shift Work Sleep Disorder, 3221
Short Bowel Syndrome, 3225
SIADH, 3121
Sick Sinus Syndrome, 3229
Sickle Cell Disease, 3233
Sideroblastic Anemia, 3237
Silicosis, 3241
SLE, *see* Systemic Lupus Erythematosus
Sleep-Related Bruxism, 3245
Sleep-Related Rhythmic Movement Disorder, 3249
Small Bowel Obstruction, 3253
Snake Envenomation, 3257
Social Anxiety Disorder (Social Phobia), 3261
Somatic Symptom Disorder, 3265
Specific Phobias, 3269
Spider Bites, 3273
Splenic Abscess, 3277
Splenic Artery Aneurysm, 3283
Splenic Cyst, 3287
Splenic Cysts, 3291
Splenic Infarction, 3295
Splenic Lymphoma, 3299
Splenic Rupture, 3305
Splenomegaly, 3309
Spontaneous Abortion (Miscarriage), 3313
Squamous Cell Carcinoma, 3317
SSRI Overdose and Serotonin Syndrome, 3125
Stargardt Disease, 3323
STD, *see* Genital Herpes (HSV)
Stevens-Johnson Syndrome and Toxic Epidermal Necrolysis, 3327
STI, *see* Genital Herpes (HSV)
Stillbirth (Intrauterine Fetal Demise), 3331
Stimulant Overdose (Cocaine, Methamphetamine), 3335
Stimulant Use Disorder (Cocaine, Amphetamines), 3339
Strabismus, 3343
Stroke, *see* Ischemic Stroke
Subdural Hematoma, 3347
Sudden Sensorineural Hearing Loss, 3351
SUNCT and SUNA, 3129
Supraventricular Tachycardia, 3355
Syphilis (*Treponema pallidum*), 3359
Systemic Lupus Erythematosus, 3365
Systemic Lupus Erythematosus (Skin Manifestations), 3369
Systemic Sclerosis (Scleroderma), 3373

Takayasu Arteritis, 3379
Tapeworm Infections (*Taeniasis* / *Cysticercosis*), 3383
TB, *see* Tuberculosis
Temporal Lobe Epilepsy, 3387
Temporomandibular Joint Disorders, 3391
Tension-Type Headache, 3395
Testicular Cancer, 3399
Tetanus (*Clostridium tetani*), 3405
Thalassemia, 3411
Theophylline Overdose, 3415
Thinning bones, *see* Osteoporosis
Thrombotic Thrombocytopenic Purpura, 3419
Thyroid Nodules and Cancer, 3423
Thyroiditis, 3429
TIA, *see* Transient Ischemic Attack
Tinea Infections, 3433
Tinea Versicolor (*Pityriasis Versicolor*), 3439
Tinnitus, 3443
Tonsillar Hypertrophy, 3447
Tonsillitis, 3451

- Tourette Syndrome, 3455
Toxoplasmosis, 3459
Transient Ischemic Attack, 3463
Trichomoniasis, 3467
Tricuspid Regurgitation, 3471
Tricyclic Antidepressant (TCA)
 Overdose, 3475
Trigeminal Autonomic Cephalalgias, 3479
Trigeminal Neuralgia, 3483
Trigger Finger, 3487
Triple-Negative Breast Cancer (TNBC),
 3491
Tuberculosis, 3497
Tuberculosis (Extrapulmonary), 3501
Turner Syndrome (45,X), 3507
Tympanic Membrane Perforation, 3511
Type 1 Diabetes Mellitus, 3515
Type 2 Diabetes Mellitus, 3519
Typhoid Fever (*Salmonella typhi*), 3523

Ulcerative Colitis, 3529
Undifferentiated Connective Tissue
 Disease, 3533
Urea Cycle Disorders, 3537
Urinary Incontinence, 3541, 3545
Urosepsis, 3549
Urticaria, 3555
Usher Syndrome, 3567
Uterine Fibroids, 3559
Uterine Rupture, 3563
UTI, *see* Urinary Incontinence
Uveitis, 3571

Vaginal Cancer, 3575, 3581
Variant CJD (Mad Cow Disease), 3587
Varicella-Zoster (Systemic Aspects), 3591
Vasa Previa, 3595
Vascular Dementia, 3599
Ventricular Tachycardia, 3603
Vertigo, 3607
Vestibular Neuritis, 3611
Viral Encephalitis, 3615
Viral Exanthems, 3621

Vitamin A Deficiency, 3625
Vitamin B1 (Thiamine) Deficiency —
 Beriberi and Wernicke-Korsakoff
 Syndrome, 3637
Vitamin B12 (Cobalamin) Deficiency,
 3629
Vitamin B12 Deficiency Anemia, 3633
Vitamin B2 (Riboflavin) Deficiency —
 Ariboflavinosis, 3641
Vitamin B3 (Niacin) Deficiency —
 Pellagra, 3645
Vitamin B6 (Pyridoxine) Deficiency, 3649
Vitamin B7 (Biotin) Deficiency, 3653
Vitamin B9 (Folate) Deficiency, 3657
Vitamin C Deficiency — Scurvy, 3661
Vitamin D Deficiency — Rickets and
 Osteomalacia, 3665
Vitamin E Deficiency, 3669
Vitamin K Deficiency, 3673
Vitiligo, 3677
Vocal Cord Nodules, 3681
Vocal Cord Paralysis, 3685
Von Willebrand Disease, 3689
Vulvar Cancer, 3693, 3699
Vulvovaginal Candidiasis, 3705

Warts, 3709
West Nile Virus, 3713
Whooping cough, *see* Pertussis
 (Whooping Cough)
Williams Syndrome, 3717
Wilms Tumor (Nephroblastoma), 3721
Wilson's Disease, 3727
Wound Infections and Surgical Site
 Infections, 3731

Yellow Fever, 3735
Yellow Jack, *see* Yellow Fever

Zenker Diverticulum, 3739
Zika Virus, 3743
Zinc Deficiency, 3747
Zollinger-Ellison Syndrome, 3751